

ZHOU XUN

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THE
PEOPLE'S HEALTH

*Health Intervention
and Delivery
in Mao's China,
1949–1983*

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Series editors Rosalind Crone and Heather Shore

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周迅

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Health Intervention and Delivery

in Mao's China, 1949-1983

ZHOU XUN

McGill-Queen's University Press
Montreal & Kingston • London • Chicago

人民健康

健康干预和交付

在毛泽东时代的中国，1949-1983 年

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McGill-Queen's University Press
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*I dedicate this book to my parents, who with many
of their colleagues worked day and night for more than fifty years
trying to bring parasitic diseases such as schistosomiasis
under some degree of control.*

谨以此书献给我的父母，他们和许多同事一起，五十多年来
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THE PEOPLE'S HEALTH

THE PEOPLE'S HEALTH

Introduction

WHAT IS THE PEOPLE'S HEALTH?

This is a book about the people's health in the modern age.¹ From the medieval practice of quarantining a ship to control the Black Death in 1377 in Dubrovnik to John Snow locking the water pumps in Victorian London in 1854 to prevent the spread of cholera, the idea of preventing the spread of disease demanded looking at social control as providing some sort of immediate intervention to preserve life. The goal was, and is, the gradual elimination of those factors that are seen as damaging to health, but the underlying idea is the creation of a world in which health is a right guaranteed by the state. The pursuit of this goal was to be accomplished through institutions such as public health authorities and via credentialed medical practitioners entrusted with improving the people's health. This book looks at the debates about and the practices that were introduced to improve the people's health in the People's Republic of China (the PRC) as a product of the politics of the Cold War, from the Communist Party's seizure and coalescing of power in 1949 to the period immediately after the death of Mao Zedong in 1976, in a global context. Health played an outsized role in the internal politics of the new Communist state and came in complex ways to define the PRC globally. My focus is on how the state imagines medicine being utilized as a tool of political action. This has been examined before, if not in the same detail. What lies at the core of my interest is to examine how the people who were impacted by the politics of health responded to control from 'above' and to examine the experts' views and their national and

导言

什么是人民健康？

这是一本关于现代人民健康的书。¹从1377年杜布罗夫尼克中世纪隔离船舶以控制黑死病的做法，到1854年约翰·斯诺关闭维多利亚时期伦敦的水泵以防止霍乱蔓延，防止疾病传播的想法需要将社会控制视为提供某种直接干预以保护生命。目标过去是，现在仍然是，逐步消除那些被视为损害健康的因素，但其根本思想是创造一个健康是国家保障的权利的世界。这一目标的实现将通过公共卫生机构等机构以及受托改善人民健康的具有资质的医疗从业人员来实现。本书着眼于关于改善中华人民共和国（prc）人民健康的辩论和实践，将其视为冷战政治的产物，从中国共产党1949年夺取和巩固政权到1976年毛泽东去世后的时期，置于全球背景下。健康在新共产主义国家的内部政治中发挥了巨大的作用，并以复杂的方式定义了prc在全球的地位。我关注的是国家如何设想将医学用作政治行动的工具。这一点以前已经研究过，即使不是以同样的细节。我感兴趣的核心是考察受健康政治影响的人民如何回应来自“上方”的控制，并考察专家的观点及其国家和

global reception, as well as looking at how such interventions were experienced from 'below' by listening to those impacted by disease and the policies developed to confront its causes and to cure those people who were/are impacted by the disease. Such a perspective is not an attempt merely to imagine a centre/periphery model of tension between the state and the individual but to outline how resistance or acquiescence among those impacted by state policies concerning health and illness continued to shape and reshape health policy in the PRC under Mao. Simultaneously, such varying levels of power reflected the use from 'above' and 'below' of both 'expert' and 'popular' medicine, of models of allopathic medicine that arose in the first great age of Western biology at the close of the nineteenth century, and systems of folk medicine (as defined from the perspective of allopathic medicine) that jumbled religious or ritual practices, systems of popular medicine, and folklore, as practised among the various population groups living in China. At the core are the shifting patterns of state power, of how health policy came to be one of the central means of impacting and influencing the 'masses' – and how both expert and popular medicine became tools of political power. The reality in the end is how those who were ill or at risk believed in the efficacy of such claims, for good or for ill.

This book is neither intended to be a comparative history of disease eradication across different countries nor 'merely' an internal history of the people's health in the PRC. Such a history, as I shall show in detail, reflects the shifts on the national level as a result of changes in the global understanding of the people's health and the role of the state as its provider and protector. Globally, such shifts occurred in the shadow of the nineteenth and early twentieth centuries. Claims about the role of the state as an active agent in intervening to control the spread of disease and the concomitant colonial medical practice were developed to improve the people's health in Asia and Africa as a means of improving productivity (in a new field labelled 'tropical medicine'). But after the collapse of both Imperial Japan and Nazi Germany in 1945 – states that used health as a rationale for the cruellest and most horrific experiments in promoting the people's health and for many of the same goals as the colonial powers as well as in the context of genocidal racial policies – such models of the 'white man's burden' in public health became untenable. Following the trials of the Nazi doctors at Nuremberg

全球的接受情况, 以及通过倾听受疾病影响的人以及为应对疾病原因和治愈受疾病影响的人而制定的政策, 从“下方”了解这些干预措施的经验。这种视角并非仅仅试图想象国家与个人之间紧张关系的中心/边缘模式, 而是要概述在毛泽东领导下的中国, 那些受国家健康和疾病政策影响的人的抵抗或默许如何继续塑造和重塑卫生政策。与此同时, 这种不同的权力水平反映了自19世纪末西方生物学第一个伟大时代兴起的同种疗法医学模式, 以及民间医学体系(从同种疗法医学的角度定义)中, 来自“上方”和“下方”的“专家”和“大众”医学的使用, 这些民间医学体系混合了宗教或仪式活动、大众医学体系以及在中国居住的各种人群中实践的民俗。核心是国家权力的转变模式, 即卫生政策如何成为影响和影响“群众”的主要手段之一——以及专家和大众医学如何成为政治权力的工具。最终的现实是那些生病或面临风险的人如何相信这些主张的有效性, 无论好坏。

本书既非旨在对不同国家之间的疾病根除进行比较历史研究, 也非“仅仅”是对中华人民共和国人民健康状况的内部历史研究。正如我将详细展示的那样, 这样的历史反映了国家层面的转变, 这是由于全球对人民健康的理解以及国家作为其提供者和保护者的角色发生了变化。在全球范围内, 这种转变发生在19世纪和20世纪初的阴影下。关于国家作为积极行动者干预控制疾病传播以及随之而来的殖民医学实践的主张, 是为了改善亚洲和非洲人民的健康, 以此作为提高生产力的一种手段(在一个被称为“热带医学”的新领域)。但在1945年日本帝国和纳粹德国垮台后, 这些国家以健康为借口, 进行了最残酷和最可怕的实验, 以促进人民的健康, 并为了与殖民列强相同的目标, 以及在种族灭绝政策的背景下——这种“白人的负担”公共卫生模式变得站不住脚。在纽伦堡对纳粹医生进行审判之后

in 1947, there was a search globally for a positive model for what had become a discredited claim that the state needed to protect, preserve, and further the people's health, and this came to be found in Communist China under Mao.

The image of the PRC, the country most identified by many international health stakeholders as a 'positive deviant' in the 'Long 1970s' in achieving better health outcomes with limited resources, is a prime example of the search for answers to the people's health in the mid-twentieth century and beyond. My goal here is to illustrate how politics and health are inextricably linked in the PRC. Policy shifts were not merely internal debates about specific approaches to the people's health among specialists in public health in the PRC. They were at once responses to global attitudes toward health systems, in particular health care delivery and public health intervention but also, in turn, helped to shape them. While the PRC was a developing country during the Cold War, the claims about health and health care in China also reflected the concerns in many developing countries about the efficacy of their own health systems. Cold War politics impacted medical practice and public health initiatives, and China became the touchstone for such discussions.

HOW DO WE EXAMINE THE PEOPLE'S HEALTH?

What is different about what I am undertaking here is both the reach of the evidence and my methodology. Social scientists (specifically those working in anthropology, development studies, and policy design), as well as historians, deploy history in different ways to achieve different insights. In this book, I have tried to find a balance between the pragmatic needs of the social scientist and the demands of the historian that will contribute to a 'more rounded' history of health and medicine in the PRC. It begins with techniques familiar to social historians of medicine that describe the human stories behind the statistics. More specifically, it uses an integrated approach by analyzing the accounts of local implementation and individual testimonies about health care against the state-sanctioned statistical analyses and official party rhetoric. Local and personal stories, for example, shed crucial light on issues of access, inequalities, and so on, while statistics, as a larger evaluation of impact across populations, may point to a different assessment of individual opportunity and entitlement. Official rhetoric and

1947年，全球范围内都在寻找一个积极的模式，以取代已经声名狼藉的“国家需要保护、维护和增进人民健康”的主张，而这个模式最终在毛泽东领导下的共产主义中国找到了。

在许多国际卫生利益相关者眼中，中华人民共和国（prc）是“20世纪70年代漫长时期”中以有限资源实现更好健康结果的“积极离经者”的典型代表，是20世纪中叶及以后寻找人民健康答案的绝佳例证。我在这里的目标是阐明政治和健康在中国是密不可分的。政策转变不仅仅是中国公共卫生专家之间关于人民健康的具体方法的内部辩论。它们既是对全球对卫生系统态度的回应，特别是医疗保健服务和公共卫生干预，同时也反过来帮助塑造了这些态度。尽管中国在冷战时期是一个发展中国家，但关于中国健康和医疗保健的主张也反映了许多发展中国家对其自身卫生系统有效性的担忧。冷战政治影响了医疗实践和公共卫生倡议，中国成为此类讨论的试金石。

我们如何考察人民的健康？

我在这里所做的事情的不同之处在于证据的范围和我的方法。社会科学家（特别是那些在人类学、发展研究和政策设计领域工作的人）以及历史学家，以不同的方式运用历史来实现不同的见解。在本书中，我试图在社会科学家的务实需求和历史学家的要求之间找到平衡，这将有助于更全面地了解中华人民共和国的健康和医学史。它从医学社会历史学家熟悉的技巧开始，描述了统计数据背后的人的故事。更具体地说，它采用了一种综合方法，通过分析地方实施的记录和关于医疗保健的个人证词，对照国家批准的统计分析和官方党的言论。例如，地方和个人的故事为获取、不平等问题等提供了重要的启示，而统计数据作为对跨人群影响的更大评估，可能会指向对个人机会和权利的不同评估。官方言论和

reports from the archives provide the necessary background information, chronology, and statistics that are often missing in the oral interviews. The oral interviews, on the other hand, tell of individual experiences of and interactions with, as well as resistance to, such large-scale government health campaigns. Through the interviews,* we learn the particular way in which individuals remember these past events but not how they were actually implemented or intended to function. Juxtapose one to the other and there is a fuller sense of what was occurring at the time in different places, as well as of how historical memory shaped individual memories and how individuals' remembrances of what happened have continued to impact their lives up to the present day. Based on hundreds of files from rarely seen party archives in eight impacted provinces across China, and on freshly collected oral testimonies from experts, local cadres, and villagers in those provinces, we can now illustrate the huge diversity of everyday interactions and the diverse aspects of support, accommodation, and resistance.† By analyzing local implementations and personal testimonies against statistical analysis and political rhetoric, we can better understand the many complexities involved in the implementation of large-scale public health initiatives. While analysis of these cases provides immediate lessons for those seeking to strengthen health systems, it also generates background information for future research. Since health systems are, essentially, collections of people, this book is as much about the human story of innovations and responsiveness to changing politics and human needs as it is about the politics of health. The oral testimonies, as with my earlier work on the Great Leap famine as well as Gail Hershatter's work on the lives of rural women in the early PRC, open a unique window into what life was like for different individuals living under communism.²

* I have concealed the true identities of the people interviewed. This is to protect their privacy and for political considerations.

† I selected eight county and provincial archives where relatively rich documentation can be deemed representative of the many dimensions of state interaction in everyday life. All these provinces present a measure of geographical and political diversity, and they went through very different experiences under Mao. These provinces are also interesting because they contain areas where the Anti-Schistosomiasis Campaign or the Barefoot Doctor program were most vigorously carried out.

来自档案馆的报告提供了必要的背景信息、年表和统计数据，而这些信息在口头访谈中常常缺失。另一方面，口头访谈讲述了个人经历、与大规模政府卫生运动的互动以及对这些运动的抵制。通过访谈，* 我们了解了个人回忆这些过去事件的特定方式，而不是这些事件的实际实施方式或预期功能。将两者并列比较，可以更全面地了解当时不同地方发生的事情，以及历史记忆如何塑造个人记忆，以及个人对所发生事情的回忆如何持续影响他们现在的生活。基于中国八个受影响省份中罕见的党档案中的数百份文件，以及从这些省份的专家、地方干部和村民那里收集的最新版口头证词，我们现在可以说明日常互动的巨大多样性以及支持、适应和抵制的各个方面。† 通过对照统计分析和政治辞令来分析地方实施情况和个人证词，我们可以更好地理解大规模公共卫生举措实施过程中涉及的许多复杂性。虽然对这些案例的分析为那些寻求加强卫生系统的人提供了直接的经验教训，但它也为未来的研究生成了背景信息。由于卫生系统本质上是人的集合，因此本书既是关于创新和对不断变化的政治和人类需求做出反应的人的故事，也是关于卫生政治的。口头证词，就像我早期关于大跃进饥荒的作品以及盖尔·赫沙特关于早期中华人民共和国农村妇女生活的研究一样，为我们打开了一扇独特的窗口，让我们了解在共产主义制度下生活的不同个体的生活状况。

²

* 我对受访者的真实身份进行了隐瞒。这是为了保护他们的隐私以及出于政治考虑。

† 我选择了八个县级和省级档案馆，这些档案馆拥有相对丰富的文献，可以被认为是国家在日常生活中互动的多个维度的代表。所有这些省份都呈现出一定程度的地理和政治多样性，并且在毛泽东领导下经历了截然不同的经历。这些省份也很有趣，因为它们包含血吸虫病防治运动或赤脚医生计划开展得最积极的地区。

MY TWO CASE STUDIES

To illustrate the complexity of both the voices heard from ‘below’ and the political and expert culture from ‘above’, I focus on the two internationally acclaimed public health initiatives in the PRC under Mao Zedong’s leadership – namely, the Anti-Schistosomiasis Campaign and the Barefoot Doctor program – as case studies to explore the complex and dynamic relationships between politics and health and, equally important, between individual lives and the political system as well as the health system. It is clear that public health objectives became an integral part of the Chinese revolution from its outset in 1911 through the 1920s. Medicine had been part of European ‘soft power’ in China from the time of Western missionaries such as Peter Parker in the early nineteenth century* to the various medical undertakings of the Rockefeller Foundation in the early twentieth century. The world of post-World War II China, which came under Mao’s leadership after 1949, presented continuities as well as ruptures with this already revolutionary political and public health world. I do not assert that the contested claims of the success and failures of the PRC’s health system as reflected by these two case studies are wholly accurate or inaccurate; rather, these claims must be understood through the political visions that formed the public health initiatives and shaped the accounts about them both within and beyond the PRC. We can develop a better and more nuanced understanding of the Chinese approach to health by exploring the processes through which the Maoist health system was conceived and the political context in which they were, and could be, evaluated. My perspective in evaluating the Maoist health system ranges from the political (and poetic) pronouncements attributed to Mao to the popular experience on the ground of the Anti-Schistosomiasis Campaign and the Barefoot Doctor programme.

The centrepiece of the PRC’s public health campaign was the Anti-Schistosomiasis Campaign. This campaign began in 1955 when Mao was mounting the ‘Socialist High Tide’ to bring the socialist revolution to the Chinese countryside. The Communist state singled out schistosomiasis because it is a disease transmitted by working in the

* Peter Parker (1804–1888) was the first American Protestant medical missionary to China. In 1835, he opened Ophthalmic Hospital in Canton. This was the first Western-style hospital in China.

MY TWO CASE STUDIES

为了说明来自“底层”的声音以及来自“上层”的政治和专家文化的复杂性，我重点研究了毛泽东领导下中华人民共和国两项国际公认的公共卫生倡议——即血吸虫病防治运动和赤脚医生项目——作为案例研究，以探讨政治与卫生之间复杂而动态的关系，以及同样重要的，个体生活与政治体制以及卫生体制之间的关系。很明显，公共卫生目标从1911年到1920年代的中国革命之初就成为了其不可分割的一部分。从19世纪初的西方传教士彼得·帕克*到20世纪初的洛克菲勒基金会的各种医疗事业，医学一直是欧洲在华“软实力”的一部分。二战后的中国在1949年后由毛泽东领导，呈现出与这个已经具有革命性的政治和公共卫生世界的连续性和断裂性。我并不断言，这两项案例研究反映的关于中华人民共和国卫生体制成功与失败的有争议的主张是完全准确或不准确的；相反，必须通过形成公共卫生倡议并在中国国内外塑造关于它们的叙述的政治愿景来理解这些主张。通过探索毛泽东时代的卫生体制构想的过程以及评估它们的政治背景，我们可以更好地、更细致地理解中国对卫生问题的处理方法。我评估毛泽东时代卫生体制的视角范围从归因于毛泽东的政治（和诗意）声明到血吸虫病防治运动和赤脚医生计划的民众基层经验。

中华人民共和国公共卫生运动的核心是血吸虫病防治运动。这场运动始于1955年，当时毛泽东正掀起“社会主义高潮”，以将社会主义革命带到中国农村。共产党国家特别挑选出血吸虫病，因为它是一种通过在水中工作传播的疾病。

* 彼得·帕克(1804–1888)是第一位到中国的美国新教医学传教士。1835年，他在广州开设了眼科医院。这是中国第一家西式医院。

paddy fields and is aided in this by the traditional way of life of the farming communities living in rice-growing regions. It was decided that the national goal should be to eradicate it. Schistosomiasis is a disease that afflicted people in many parts of the underdeveloped world, and it was thought that it would bring prestige to the country if the PRC could be the first to eliminate it. The central party leadership declared that the goal of eradication would be achieved in seven years – although this obviously did not happen. In fact, toward the end of the Great Leap Forward in the early 1960s, the morbidity rate was sharply on the rise. By this time, the rural health system had essentially collapsed, and people had turned back to local traditional practitioners. From these localized initiatives emerged the ‘Barefoot Doctors’, and the Chinese Communist Party (CCP) more or less adopted a system that people had already shown that they wanted and rolled it out locally – all of which speaks to the legitimacy that the CCP lost in the aftermath of the famine that had resulted from the Great Leap Forward.

The PRC's public health initiatives in the Anti-Schistosomiasis Campaign and the Barefoot Doctor program are well known around the world because they became the touchstone for public health innovations during the Cold War. A historical account of how these campaigns were implemented and received at the local level throughout China on the political as well as on the experiential level that also addresses their impact on global health is long overdue. Until now there has been no systematic study of the health system in the PRC under Mao Zedong's leadership. The reason for this is simple: from 1949 to 1976, China was largely closed to the West. Needless to say, there were many difficulties in undertaking extensive, nationwide oral histories. Even after the country opened its doors following the death of Mao, access to party archives remained extremely difficult, forcing researchers to rely on published official and semi-official literature. Lack of access to primary data other than policy formulations and party propaganda meant that the earlier historical literature could do no more than provide a very general survey of the field, unable to offer any concrete evidence or critical insights regarding how health care was actually experienced by the population at large.³ Social scientists have focused rather narrowly on state policy formulations alone, and therefore little is known about how these grand schemes were actually carried out on the ground and experienced by the communities involved.⁴ Even the

水稻田，并受益于水稻种植区农民社区的传统生活方式。当时决定将消灭血吸虫病作为国家目标。血吸虫病是困扰许多发展中国家人民的疾病，如果中国能够成为第一个消灭它的国家，那么这将为中国带来声望。中央领导层宣布将在七年内实现消灭目标——尽管这显然没有发生。事实上，在 20 世纪 60 年代初大跃进即将结束时，发病率急剧上升。到那时，农村卫生系统基本上已经崩溃，人们又重新求助于当地的传统医生。从这些地方性的倡议中，涌现出了“赤脚医生”，中国共产党（中共）或多或少地采纳了一个人民已经表明他们想要的制度，并在地方上推广开来——所有这些都表明了中共在大跃进造成的饥荒之后失去的合法性。

中国在血吸虫病防治运动和赤脚医生计划中的公共卫生举措举世闻名，因为它们成为冷战时期公共卫生创新的试金石。长期以来，人们一直期待着从历史的角度，在政治和经验层面，考察这些运动在中国各地如何实施和接受，以及它们对全球卫生的影响。到目前为止，还没有人系统地研究毛泽东领导下的中国公共卫生体系。原因很简单：从 1949 年到 1976 年，中国基本上对西方关闭了大门。毋庸置疑，进行广泛的、全国性的口述历史非常困难。即使在毛泽东去世后，中国打开了国门，查阅党的档案仍然极其困难，迫使研究人员依赖已出版的官方和半官方文献。除了政策制定和党的宣传之外，缺乏获取一手数据的途径意味着早期的历史文献只能对该领域进行非常概括的调查，无法提供任何关于广大民众实际体验医疗保健的具体证据或批判性见解。³ 社会科学家们的研究重点主要集中在国家政策的制定上，因此，对于这些宏伟的计划实际上是如何在基层执行以及相关社区如何体验这些计划知之甚少。⁴ 甚至

few detailed historical studies of traditional Chinese medicine, such as that by Kim Taylor, are based on published policy documents and propaganda material.⁵ Furthermore, Taylor's analysis relied heavily on what is now agreed by historians of the PRC as an outdated historiography in which the CCP was credited with establishing a stable government after decades of war and political turmoil and with carrying out a successful social transformation program, including public health initiatives. To date, there are only a small handful of excellent ethnographically based case studies of the Barefoot Doctor program, but these studies are focused on specific villages.⁶ There is one historical assessment of the Anti-Schistosomiasis Campaign in a particular region in eastern China.⁷ All of these studies have a narrow geographic focus, yet it is all too apparent that no broad historical analysis can be based on a mere handful of archival files narrowly selected from one single holding or on interviews from a single village. This present book does not enter into the debate about whether the Chinese public health policy was a success or a failure. Instead, I focus on how people experienced these public health campaigns by ranging more widely across China and listening to the participants, as well as reading archival materials that only became available in the late 1990s. As a result of the new national regulations regarding archives, introduced in the 1990s, this book is able to offer a more nuanced account of the debates about the people's health in the PRC under Mao Zedong and how they were perceived on multiple levels in society. My intent is neither to praise nor to condemn the activities but to provide an account of the often-unintended consequences attendant to them.

THE COURSE OF THE PEOPLE'S HEALTH IN THE PRC

From the onset of the Chinese Communist revolution in the 1920s to the end of World War II in the mid-1940s, shortly before the CCP seized power, public health objectives were an integral part of the ideology of the revolution. The overwhelming poverty in the Chinese countryside, which signalled China's backwardness to the world, was linked to the poor hygiene and poor health of the rural population. China was the 'Sick Man of Asia' in more than one way. Leaders of the revolution, including Mao Zedong, who was to become the supreme head of the CCP and chairman of the PRC in 1949, believed that improving rural health and medical care would therefore open

对传统中医进行详细历史研究的文献不多，比如 Kim Taylor 的研究就是基于已发布的政策文件和宣传材料。⁵此外，Taylor 的分析在很大程度上依赖于中华人民共和国历史学家现在公认的过时史学，该史学认为，中国共产党在经历了数十年的战争和政治动荡后，建立了一个稳定的政府，并实施了一项成功的社会转型计划，包括公共卫生举措。迄今为止，只有少数几个以人种学为基础的赤脚医生项目案例研究，但这些研究都集中在特定的村庄。⁶有一篇关于中国东部特定地区血吸虫病防治运动的历史评估。⁷所有这些研究都具有狭窄的地理焦点，但很明显，任何广泛的历史分析都不能仅仅基于从单个馆藏中狭隘选择的少数档案文件或来自单个村庄的访谈。本书不参与关于中国公共卫生政策是成功还是失败的争论。相反，我专注于人们如何通过更广泛地在中国各地考察和倾听参与者的意见，以及阅读 20 世纪 90 年代末才可获得的档案材料来体验这些公共卫生运动。由于 20 世纪 90 年代引入的关于档案馆的新国家法规，本书能够更细致地描述毛泽东领导下的中华人民共和国关于人民健康的辩论，以及这些辩论在社会多个层面上的看法。我的目的既不是赞扬也不是谴责这些活动，而是提供一个关于这些活动常常伴随的意想不到后果的叙述。

中华人民共和国人民健康历程

从 20 世纪 20 年代中国共产党革命开始到 20 世纪 40 年代中期第二次世界大战结束，即中共夺取政权前不久，公共卫生目标是革命意识形态不可分割的一部分。中国农村的普遍贫困，向世界表明了中国的落后，这与农村人口的恶劣卫生条件和糟糕的健康状况有关。在很多方面，中国都是“亚洲病夫”。包括毛泽东在内的革命领导人（他将于 1949 年成为中共最高领导人和中华人民共和国主席）认为，改善农村卫生和医疗保健将因此开启

the door for modernization. Between the late 1920s and the 1940s, in the CCP-controlled Soviet bases, public health work became a priority for the party. The communist praxis was to change manners of living, moving toward the modernization of the collective. To reach such goals, including those of public health, it was inevitable that the whole population had to be involved as part of the transformative vision of the CCP.

After the CCP seized power in 1949, it quickly took on the challenge of eradicating the snail-borne parasite *Schistosomiasis japonica* (usually labelled simply as schistosomiasis). The national Anti-Schistosomiasis Campaign linked allopathic medicine and public health work with the ongoing legitimization of the new order. Interrelated with the predominant agricultural way of life, especially for rural, rice-growing farmers, schistosomiasis has been endemic in much of China's rice-growing regions along and south of the Yangtze River. Historically, the disease had evoked widespread fear in these regions. It was believed to have contributed to infertility and thus was perceived as the cause of under-population and poor productivity, the underlying causes of China's rural poverty. Immediately upon the founding of the PRC in 1949, schistosomiasis was elevated to one of the most serious diseases that needed to be combated. Fighting the disease was viewed as vital to national security, productivity, and the military manpower of the new state and came to stand for fighting for socialism.

Furthermore, the difficulty of controlling the disease, widely acknowledged internationally, proved a real test for the new Communist state. To eradicate something and succeed where others, especially 'Western' (which often stands for 'imperialist') experts, had failed was a prize worth trying for. Public health teams, trained and led by experts, took the first steps in the campaign to control the disease. Their job was to discover the extent of the problem, and for them this was a proper and necessary public health objective. At the same time, conquering an intractable medical and public health problem would also bring them global scientific prestige. In this respect, both the CCP and the experts potentially had a win/win situation – if they could succeed.

Central economic and political planning was extended helter-skelter across the country during the first decade of the PRC. Concurrent with this, the central government's sense of urgency with regard to schistosomiasis control, as with many other aspects involved in

为现代化敞开大门。在 20 世纪 20 年代末至 40 年代，在中共控制的苏维埃根据地，公共卫生工作成为党的优先事项。共产主义的实践是改变生活方式，朝着集体现代化迈进。为了实现这些目标，包括公共卫生目标，整个民众都必须参与其中，这是中共改造愿景的一部分。

1949 年中共夺取政权后，迅速着手消除血吸虫病这种由钉螺传播的寄生虫（通常简称为血吸虫病）。全国血吸虫病防治运动将对抗疗法和公共卫生工作与新秩序的持续合法化联系起来。血吸虫病与主要的农业生活方式，特别是农村种植水稻的农民的生活息息相关，长期以来在中国长江及其以南的大部分稻米种植区流行。历史上，这种疾病在这些地区引起了广泛的恐惧。人们认为它会导致不孕，因此被认为是人口不足和生产力低下的原因，也是中国农村贫困的根本原因。1949 年中华人民共和国成立后，血吸虫病立即被提升为需要防治的最严重的疾病之一。与疾病作斗争被认为是国家安全、生产力和新国家的军事力量至关重要的事情，并代表着为社会主义而奋斗。

此外，国际上普遍承认控制这种疾病的难度，这对新成立的共产主义国家来说是一次真正的考验。如果能够根除这种疾病，并在其他国家，特别是“西方”（通常代表“帝国主义”）专家失败的地方取得成功，那将是一项值得努力的荣誉。由专家培训和领导的公共卫生团队在控制这种疾病的运动中迈出了第一步。他们的工作是发现问题的严重程度，对他们来说，这是一个恰当和必要的公共卫生目标。与此同时，征服一个棘手的医疗和公共卫生问题也将为他们带来全球科学声誉。在这方面，如果他们能够成功，中国共产党和专家们都有可能实现双赢。

在中华人民共和国成立的第一个十年里，中央经济和政治计划在全国范围内迅速扩展。与此同时，中央政府对血吸虫病控制的紧迫感，以及与此相关的许多其他方面，

centralization, caused the local responses to the complexities of the hasty implementation of such a massive public health campaign to be fragmented and often contradictory. By bringing those directly impacted by the campaign (from the villagers involved to the cadres in charge of implementing the campaign) back into this history and seeing them as active participants in this state public health campaign, we can see how great plans on paper were transformed into makeshift solutions that bore little or no resemblance to the original political project as soon as they encountered the wide variety of human experiences of health, disease, and, indeed, changing politics. To overcome obstacles, in the early stage of the campaign, the front-line workers, or the CCP cadres managing the day-to-day control of the disease, understood the need to be persuasive rather than authoritarian in their approach. This was reflected in the extensive public health propaganda as well as in the more detailed and local organizational work during this period.

Beginning in 1956, as an integral part of the transformative project of the CCP to end rural poverty, the mass population in the Chinese countryside was mobilized in a nationwide eradication campaign aimed at the various vectors of disease, in particular the water-borne snails that transmitted the infectious agent that causes *Schistosomiasis japonica*. With the advent of the Great Leap Forward between 1957 and 1960, there was an even greater political demand to speed up the process of eliminating the snails. Land reclamation and irrigation schemes were methods used to bury and hence kill the snails. Not only did this prove extremely costly and wasteful, but these schemes also ran into conflict with the system for water conservation, contributing to severe flooding as well as causing alkalization of the soil. In the end, the reclamation initiatives had to be abandoned, and hence the snails returned. The subsequent switch to killing the snails using molluscicides further damaged the ecosystem, with long-lasting negative consequences on the environment.

This period of radical and rapid political change also saw intensification of direct treatment through the application of shorter courses of the highly toxic antimony tartrate (tartar emetic), an accepted intervention of the time known to control the infection. Such an approach was not only limited in its effectiveness, but many people died from exposure to the substance. At the same moment, the utopian project of the Great Leap Forward, aimed at transforming the Chinese countryside into a communist paradise, subsequently led

中央集权导致在仓促实施如此大规模的公共卫生运动时，地方应对措施变得分散，并且常常是矛盾的。通过将直接受到该运动影响的人（从参与其中的村民到负责实施该运动的干部）重新带入这段历史，并将他们视为国家公共卫生运动的积极参与者，我们可以看到，纸上谈兵的宏伟计划是如何转变为临时解决方案的，一旦它们遇到人类在健康、疾病以及不断变化的政治方面的各种各样的经验，这些解决方案与最初的政治项目几乎没有或根本没有相似之处。为了克服障碍，在运动的早期阶段，一线工作人员，或负责疾病日常控制的中共干部，明白有必要采取说服而非专制的做法。这反映在广泛的公共卫生宣传以及在此期间更详细和本地化的组织工作中。

从1956年开始，作为中国共产党旨在消除农村贫困的变革性项目的一个组成部分，中国农村的大量人口被动员起来，参加了一场旨在消灭各种疾病媒介的全国性运动，特别是传播导致日本血吸虫病的传染性病原体的水生蜗牛。随着1957年至1960年大跃进的到来，在政治上对加速消除蜗牛的进程提出了更高的要求。土地开垦和灌溉计划被用来掩埋并杀死蜗牛。这不仅被证明是极其昂贵和浪费的，而且这些计划还与水利系统发生冲突，导致严重的洪水，并导致土壤碱化。最终，开垦计划不得不放弃，因此蜗牛又回来了。随后转向使用杀软体动物剂杀死蜗牛进一步破坏了生态系统，对环境造成了长期负面影响。

在这个激进和快速的政治变革时期，人们也看到了通过应用短期疗程的高毒性酒石酸锑钾（吐酒石）进行直接治疗的强化，这在当时是被接受的控制感染的干预措施。这种方法不仅效果有限，而且许多人死于接触该物质。与此同时，旨在将中国农村改造为共产主义天堂的乌托邦式的大跃进运动，随后导致了

to the worst famine in history, which lasted for four years between 1958 and 1962 and claimed tens of millions of lives as well as many animals. This, needless to say, also complicated the earlier eradication initiatives. At the height of the Great Leap Forward, and in the aftermath of the famine, millions of new agricultural migrants as well as farm animals were sent to the infected regions to help with agricultural production. A huge number of people and animals were infected by schistosomiasis, while many who had been cured of the disease became re-infected. In the meantime, famine-related oedema, gynaecological problems, and child malnutrition became even more widespread in the countryside. Unable to cope with the crisis, the PRC's rural health system literally collapsed. In the face of diminishing returns, in order to control the collapse of the public health system, ever more authoritarian and centralized methods came into play. With increasing desperation and despite the claims being made, officials saw the promised goal of schistosomiasis eradication and the rural utopia it was to help create slipping away from them.

Since it became clear that the centralized approach to people's health was neither politically effective nor even feasible, the result, whether intentional or not, was the decentralization of the PRC's health care delivery in response to the post-famine rural health crisis. From 1965 to 1977, the shift was away from expert, centralized programs that dealt with the people's health, such as the Anti-Schistosomiasis Campaign, to a focus on what would come to be called the 'Barefoot Doctor' program. With the collapse of the rural health system, villagers took their health and well-being into their own hands. From stories that emerged from previously unseen archive documents and from interviews with a wide swathe of those involved in the Barefoot Doctor program, as well as with the experts, some of whom continued to undertake their specialized research on schistosomiasis control during this period, we can learn much about a number of local health and medical innovations. Many of these local innovations involved the quiet acquiescence, if not active cooperation, of the local cadres. These initially quiet and local health initiatives in villages were accelerated by the post-famine decentralization of rural health care delivery in the PRC. They also coincided with a series of political campaigns during this period that tried to re-establish more centralized control in the wake of the chaos caused by the famine. The climax was the Great Proletarian Cultural Revolution, which began in 1966 and lasted until Mao's

导致了历史上最严重的饥荒，这场饥荒从 1958 年持续到 1962 年，夺走了数千万人的生命以及许多动物的生命。不用说，这也使早期的根除行动复杂化了。在大跃进的高峰期以及饥荒之后，数百万新的农业移民以及农场动物被送往疫区以帮助农业生产。大量的人和动物感染了血吸虫病，而许多已经治愈该疾病的人再次被感染。与此同时，与饥荒有关的水肿、妇科疾病和儿童营养不良在农村地区变得更加普遍。由于无法应对危机，中华人民共和国的农村卫生系统实际上已经崩溃。面对收益递减，为了控制公共卫生系统的崩溃，越来越多专制和集中的方法开始发挥作用。随着绝望情绪的增加，尽管官方声称，官员们看到承诺的血吸虫病根除目标以及它本应帮助创造的农村乌托邦正在从他们手中溜走。

自从明确了以集中方式保障人民健康既没有政治效益，也不可行之后，无论是有意还是无意，其结果都是中华人民共和国医疗卫生服务的权力下放，以应对饥荒后的农村卫生危机。从 1965 年到 1977 年，转变的方向是从处理人民健康的专家式、集中式项目

（如血吸虫病防治运动）转向关注后来被称为“赤脚医生”的项目。随着农村卫生系统的崩溃，村民们将他们的健康和福祉掌握在自己手中。从先前未公开的档案文件中涌现的故事，以及对广泛参与赤脚医生计划的人员以及专家的采访（其中一些专家在此期间继续进行血吸虫病防治的专门研究），我们可以了解到许多关于当地卫生和医疗创新的信息。许多地方创新都涉及到当地干部的默默默许，如果不是积极合作的话。这些最初在村庄里悄然兴起的当地卫生倡议，因中华人民共和国饥荒后农村卫生保健服务的权力下放而加速。与此同时，在此期间也发生了一系列政治运动，试图在饥荒造成的混乱之后重建更加集中的控制。高潮是始于 1966 年并持续到毛泽东时代的无产阶级文化大革命。

death in 1976. At this time, the Ministry of Health, as well as the ‘expert’ allopathic medicine and public health models, came under fire as part of Mao’s effort to dismantle the technocratic and bureaucratic structure that had by then been firmly established in the PRC. The Barefoot Doctor program and other local innovations, including the training of village health workers and the revival of some local healing practices, came to represent those ‘newly emerged revolutionary things’ intended to replace the ‘old bourgeois’ expert- and hospital-centred health system.

In 1968, at the height of the Great Proletarian Cultural Revolution, the Barefoot Doctor program, coupled with the Rural Co-operative Medical System (RCMS), was officially launched. By 1976, as we are told, 90 per cent of China was covered by the RCMS, and Barefoot Doctors could be seen throughout rural China providing invaluable if often unequal care and aid to millions of villagers. The radical differences in training and resources, as recounted by the participants, often led to the provision of inefficient and insufficient health care, but this sometimes encouraged relatively untrained practitioners to acquire further knowledge, in the realms of both allopathic and alternative systems of medicine, from the codified ‘Traditional Chinese Medicine’ (TCM) to folk medicines.

What had begun as a hodgepodge of local initiatives, together with fragments of expert medicine that evolved in response to the collapse of the state health care system during the years of famine, came by the Long 1970s to be the public face of the PRC in its relationship with the ‘First’ and ‘Third’ Worlds. During this period of the Cold War, the ‘Chinese approach to health’ came to hold out the promise of a true alternative to the crumbling, single disease-centred ‘vertical’ health care program advocated by the United States or the centralized health care structure of the Soviet Union. Having seen that neither model was truly effective in improving the people’s health, Western public health professionals and policy-makers sought a way out of the perceived Western post-war health crisis. The PRC’s Barefoot Doctor program and the Anti-Schistosomiasis Campaign came to be seen as the ‘best show in town’. They were widely cited by international public health experts as evidence of the PRC’s purportedly superior primary health care system. This was simultaneously an implied critique of the earlier approaches taken by the British and the American health authorities and other international health organizations, which denied the participation of the

1976 年去世。此时，卫生部以及“专家”对抗疗法和公共卫生模式受到了抨击，这是毛泽东瓦解当时已在中国牢固建立的技术官僚和官僚机构的一部分努力。赤脚医生计划和其他地方创新，包括乡村卫生工作者的培训和一些地方医疗实践的复兴，代表了那些旨在取代以“旧资产阶级”专家和医院为中心的卫生系统的“新出现的革命事物”。

1968 年，在文化大革命的高潮时期，赤脚医生计划与农村合作医疗制度（rcms）相结合，正式启动。正如我们所知，到 1976 年，中国 90% 的地区都覆盖了农村合作医疗制度，在整个中国农村都可以看到赤脚医生，他们为数百万村民提供宝贵的（即使往往是不平等的）医疗保健和援助。参与者讲述的培训和资源方面的巨大差异，往往导致提供低效和不足的医疗保健，但这有时会鼓励相对未经培训的从业者从编纂的“传统中医药”（tcm）到民间医学，获得更多对抗疗法和替代医学体系方面的知识。

最初由地方倡议的大杂烩以及在饥荒年代为应对国家医疗保健系统崩溃而发展起来的零星专家医学，到了 20 世纪 70 年代，成为了中华人民共和国与“第一”和“第三”世界关系的公众形象。在冷战时期，“中国医疗卫生模式”被认为是能够真正替代美国倡导的崩溃的、以单一疾病为中心的“垂直”医疗保健项目，以及苏联集中式医疗保健结构的一种选择。西方公共卫生专业人士和政策制定者看到这两种模式都不能真正有效地改善人民的健康，因此寻求摆脱他们所认为的西方战后医疗卫生危机的方法。中华人民共和国的赤脚医生计划和血吸虫病防治运动被视为“城里最好的戏”。国际公共卫生专家广泛引用它们，作为中国据说优越的初级卫生保健体系的证据。这同时也是对英国和美国卫生当局以及其他国际卫生组织早期做法的一种含蓄的批评，因为他们拒绝了人民的参与。

impacted communities in health care delivery. Neither the post-war American public health system, exemplified by the creation of the Centers for Disease Control in 1946 (as the Communicable Disease Center) nor William Beveridge's model of the National Health Service (NHS), which was introduced in the UK by Aneurin Bevan in the same year, bore the same sort of utopian promise for the economically strapped 'Third' World. Halfdan Mahler, the new director general of the World Health Organization (WHO) at the time, and his colleague Kenneth Newell felt strongly that the Chinese approach to tackling health problems with limited financial, technological, and human resources should be promoted around the world. In 1975, Mahler proposed 'Health for All', and this was adopted by the Twenty-Ninth World Health Assembly in 1976 as its goal, to be achieved by the year 2000. Two years later, in 1978, it was formally included in the Declaration of Alma Ata.

In the meantime, the PRC leadership realized that health initiatives could be 'inexpensive but profitable' undertakings, which could boost its effort to promote a new international order: a 'people's revolutionary movement' against colonialism, imperialism, and hegemonism. Between 1963 and 1989, the PRC sent medical teams to more than forty countries in Africa. In addition, China gave medical and health training to a large number of students from Africa, Southeast Asia, and Latin America, competing with the extensive parallel programs for training physicians in the Soviet Union. With this expansion, the PRC's influence on health systems in Africa, Asia, and Latin America grew exponentially. But the reputation of the medical model evolved in the PRC turned out to be phantasmagoric. All of the praise by physicians and public health specialists who visited China in the Long 1970s was based, at best, on selective reporting and visits and, at worst, on a level of self-deception that was rooted in the desire for a utopian model that seamlessly integrated medical, social, and economic goals. Yet the PRC's model became the gold standard for global public health.

In 1977, Kenneth Warren was appointed director of health science at the Rockefeller Foundation. Warren and his colleague Julia Walsh argued that the goal set at Alma Ata was 'unattainable because of the cost and numbers of trained personnel required'. They proposed a Selective Primary Health Care model that introduced a package of low-cost technical interventions to tackle the main disease problems as an interim strategy for disease control in developing countries

对医疗保健服务中受影响的社区产生了影响。无论是战后美国的公共卫生系统,例如1946年成立的疾病控制中心(作为传染病中心),还是威廉·贝弗里奇的国家医疗服务体系(nhs)模型,该模型于同年由阿内林·贝文在英国引入,都没有为经济拮据的“第三”世界带来同样的乌托邦式的希望。当时的世界卫生组织(who)新任总干事哈尔夫丹·马勒和他的同事肯尼斯·纽厄尔强烈认为,中国以有限的财政、技术和人力资源解决健康问题的方法应在全球范围内推广。1975年,马勒提出了“人人享有健康”,并在1976年被第二十九届世界卫生大会采纳为目标,计划在2000年前实现。两年后,即1978年,它被正式纳入《阿拉木图宣言》。

与此同时,中国领导人意识到,卫生倡议可能是一项“廉价但有利可图”的事业,这可以促进其推动建立新的国际秩序的努力:一场反对殖民主义、帝国主义和霸权主义的“人民革命运动”。1963年至1989年间,中国向非洲40多个国家派遣了医疗队。此外,中国还为来自非洲、东南亚和拉丁美洲的大量学生提供了医疗和卫生培训,与苏联为培训医生而开展的大规模平行项目展开竞争。随着这种扩张,中国对非洲、亚洲和拉丁美洲卫生系统的影响呈指数级增长。但中国医疗模式的声誉最终变得虚幻。20世纪70年代长期访问中国的医生和公共卫生专家所给予的所有赞扬,充其量是基于选择性的报告和访问,最坏的情况是,这种自我欺骗的程度源于对一个能够无缝整合医疗、社会和经济目标的乌托邦模式的渴望。然而,中国的模式成为了全球公共卫生的黄金标准。

1977年,肯尼斯·沃伦被任命为洛克菲勒基金会的健康科学主任。沃伦和他的同事朱莉娅·沃尔什认为,阿拉木图设定的目标“由于成本和所需训练有素的人员数量而无法实现”。他们提出了一种选择性初级卫生保健模式,引入一揽子低成本技术干预措施,以解决主要的疾病问题,作为发展中国家疾病控制的临时战略。

(1979).⁸ A number of United Nation agencies quickly adopted this selective approach, since it was less costly than the more integrated approach advocated by Mahler and Newell. Known to be obsessed with health quality and information, Warren advocated this model of ‘good healthcare at low cost’, with an emphasis on expert medicine. As an expert on schistosomiasis, he favoured controlling the disease with chemotherapy. According to him, using newer single-dose, oral, non-toxic chemotherapeutic agents in treating the disease simply cost less than attempts to eliminate snails. Warren used the PRC’s Anti-Schistosomiasis Campaign as his counter-argument. He argued that the Chinese approach of involving the rural masses to kill the snails by burying old canals and digging new ones was too costly and ineffective in controlling the disease, at least for the interim period.⁹ Warren’s study also cast doubt on the earlier claim that the Chinese had successfully eradicated the disease. The subsequent re-evaluation in the West, as well as in China itself, of the PRC’s health system of the Mao era (1949–76) suggested that the original claims for the superior health care system, and for the eradication of schistosomiasis, were, indeed, to a great extent invalid.

Coincidentally, the post-Mao PRC leadership under Deng Xiaoping after 1978 gradually embraced the neoliberal market economy and opted for a selective primary health care model, with an emphasis on specialized and hospital-centred health care as well as a focus on short-term economic gain. Health professionals in the PRC were prompted to ‘put their shoes on’ in order to catch up with developments in medicine and health care in the West. In 1983, the internationally acclaimed Barefoot Doctor program was officially abandoned, and millions of disadvantaged rural villagers, together with urban migrants, were deprived of access to any form of health care. My extensive interviews with experts, health officials, former Barefoot Doctors, and individuals who lived through this transition demonstrate that there has been a deepening inequality between urban and rural health care. These interviews also demonstrate that, once again, in some quiet corners of China, individuals and communities took health and well-being into their own hands. Yet, with little support and even disapproval from the authorities, many of the local health initiatives and health workers could not sustain either their efforts or their livelihood. The PRC may serve as a model for public health dysfunction as well as for successful adaptation, depending on the level of analysis and the historical context in which it is found.

(1979)。⁸许多联合国机构迅速采用了这种选择性的方法，因为它比马勒和纽维尔倡导的更综合的方法成本更低。沃伦以痴迷于健康质量和信息而闻名，他倡导这种“低成本的优质医疗保健”模式，强调专家医学。作为血吸虫病专家，他倾向于用化疗控制这种疾病。他认为，使用更新的单剂量、口服、无毒的化疗药物治疗该疾病，比试图消灭蜗牛的成本更低。沃伦用中国血吸虫病防治运动作为他的反驳论点。他认为，中国动员农村群众通过掩埋旧河道和挖掘新河道来杀死蜗牛的方法，在控制疾病方面成本太高且效率低下，至少在过渡时期是如此。⁹沃伦的研究也对早先关于中国已成功根除该疾病的说法提出了质疑。随后西方以及中国自身对毛泽东时代（1949-76年）中国卫生系统的重新评估表明，最初关于卓越的医疗保健系统以及根除血吸虫病的说法在很大程度上是无效的。

巧合的是，1978年后邓小平领导下的后毛泽东时代中国逐渐接受了新自由主义市场经济，并选择了选择性的初级卫生保健模式，强调专业化和以医院为中心的医疗保健，以及关注短期经济效益。中国的卫生专业人员被敦促“穿上鞋子”，以便赶上西方医学和卫生保健的发展。1983年，国际知名的赤脚医生计划被正式废弃，数百万处境不利的农村村民以及城市移民被剥夺了获得任何形式医疗保健的机会。我对专家、卫生官员、前赤脚医生以及经历过这一转变的个人的大量访谈表明，城乡医疗保健之间的不平等现象日益加剧。这些访谈还表明，在中国的一些偏远角落，个人和社区再次将健康和福祉掌握在自己手中。然而，由于缺乏支持，甚至受到当局的反对，许多地方卫生倡议和卫生工作者都无法维持他们的努力或生计。中华人民共和国可以作为公共卫生功能失调以及成功适应的模型，这取决于分析的水平以及它所处的历史背景。

In modern times, a well-functioning health system has been seen as intrinsic to the well-being of any society, but we know little about how to achieve such a system. Health systems, by definition, are complex and dynamic. In the end, they are the work of collections of people and are designed for the people. Inevitably, all health systems carry within them human strengths and weaknesses. While political vision and the leadership's ability to mobilize the masses may contribute to a better health system and ultimately better health, political factors can also have unintended yet real negative consequences. The story of the debates about improving the people's health in the first three decades of the People's Republic of China can serve as much a model for such complexity as it served as an ideal, if badly flawed, model for the improvement of people's health throughout the world during many decades of the twentieth century. Today, such problems continue to haunt developed as well as developing countries: no model has turned out to be sufficient to cover all of the problems confronting people's health. The utopian vision of Mao's programs have given way to the pragmatics of dealing with each social and medical problem in its existing context rather than applying a universal panacea that guarantees the people's health. The desire for such programs (the 'mirage of health', as Rene Dubois put it¹⁰) remains, and the notion that the goal of the public's health is static and that approaches need only be sharpened and improved, which haunted all of the attempts in the PRC, is still a dream being pursued, even if the history of such approaches is littered not only with half-successes but also with greater failures and unintended, often fatal, consequences.

在现代社会，运作良好的医疗卫生体系被视为任何社会福祉的内在组成部分，但我们对于如何建立这样一个体系知之甚少。从定义上讲，医疗卫生体系是复杂且动态的。归根结底，它们是人们共同努力的成果，并且是为人民服务的。不可避免地，所有医疗卫生体系都蕴含着人性的优点和缺点。虽然政治远见和领导人动员群众的能力可能有助于建立更好的医疗卫生体系，并最终改善健康状况，但政治因素也可能产生意想不到的、但却是真实的负面后果。在中华人民共和国成立后的最初三十年里，关于改善人民健康的辩论故事，既可以作为这种复杂性的一个典范，也可以作为一个理想化的（尽管存在严重缺陷）典范，在二十世纪的几十年里为全世界改善人民健康服务。今天，这些问题继续困扰着发达国家和发展中国家：没有任何一种模式能够充分解决人们健康面临的所有问题。毛泽东计划的乌托邦愿景已经让位于在现有背景下处理每一个社会和医疗问题的实用主义，而不是应用一种保证人民健康的万能药。对这类计划的渴望（正如勒内·杜博斯所说的那样，“健康的幻影”¹⁰）依然存在，并且认为公众健康的目标是静态的，只需改进和完善方法即可，这种观念一直困扰着中华人民共和国的所有尝试，即使这种方法的历史不仅充满了半成功，也充满了更大的失败和意想不到的、往往是致命的后果，但这仍然是一个正在追逐的梦想。

Declaring War on Schistosomiasis

THE MILITARY ENCOUNTERS DISEASE

In the extremely cold winter of January 1949, the Communist Party's Eastern China Field Army under the leadership of General Su Yu, in collaboration with the Central Plain Field Army, successfully completed the third phase of the Huai River (military) Campaign. The battle, which lasted sixty-six days during the winter of 1948 and 1949, has been widely regarded as the most decisive victory for the Communist army during the Chinese Civil War. It resulted in the Nationalists losing more than 550,000 soldiers, almost half of their entire army. The end of the ten-year civil war brought hope to millions living on the war-torn northern and central China plain. In the meantime, on 15 January 1949, the Communists' Northeastern and Northern China Field Armies, led by Marshall Gao Gang, captured Tianjin, one of the most important port cities in northern China. Defenceless, Fu Zuoyi, the Nationalist general in charge of northern China, surrendered. On 31 January, the third day of the Year of the Earth Ox in the Chinese calendar, the Communists' People's Liberation Army (PLA) marched peacefully into Peking, the 'City of Northern Peace' (today it is called Beijing, 'the Northern Capital'). As the early warm spring wind swept across the north China plain, people in Peking and the surrounding regions danced to celebrate the New Year as well as to welcome the Communist soldiers. Many civilians saw the coming of the PLA soldiers as an auspicious sign – the Earth Ox sent by heaven. In Chinese mythology, the arrival of the Earth Ox represents heavenly promise. To realize this heavenly promise, however, would involve paying a price in hard labour.¹

向血吸虫病宣战

军队遭遇疾病

1949年1月，在极其寒冷的冬季，由粟裕将军领导的中国共产党华东野战军与中原野战军合作，成功完成了淮海（军事）战役的第三阶段。这场战役持续了1948年和1949年冬季的66天，被广泛认为是国共内战期间共产党军队最决定性的胜利。它导致国民党损失了超过55万士兵，几乎占其军队总数的一半。十年内战的结束给生活在饱受战争蹂躏的华北和华中平原上的数百万人带来了希望。与此同时，1949年1月15日，由高岗元帅领导的共产党东北和华北野战军攻占了华北最重要的港口城市之一天津。负责华北的国民党将领傅作义在毫无防御能力的情况下投降了。1月31日，中国历法中的土牛年正月初三，中国人民解放军（PLA）和平进入北平，即“北方和平之城”（今天称为北京，即“北方首都”）。当早春的暖风吹过华北平原时，北京及周边地区的人们跳舞庆祝新年，并欢迎共产党的士兵。许多平民认为解放军士兵的到来是一个吉祥的兆头——上天派来的土牛。在中国神话中，土牛的到来代表着上天的承诺。然而，要实现这一上天的承诺，需要付出艰苦的劳动才能实现。¹

For those who lived south of the Yangtze River, as well as in China's deep southwest, the conflict and its concomitant uncertainty dragged on. After the spectacular victory at the Huai River Campaign, the Chinese Communist Party (CCP) and the PLA leadership did not rest. The dream of unifying China entailed more difficult struggles. In February, the Eastern China Field Army underwent serious reorganization and changed its name to the Third Field Army. Once again, General Su Yu was put in charge of the critical military campaign to cross the Yangtze. In anticipation of the campaign, Su selected some of his finest young soldiers for preparatory training. To ford the river would entail fighting in the water, but the majority of Su's soldiers were from Shandong province and were not used to fighting on waterways. A major part of their training was therefore intensive swimming lessons in the labyrinth of creeks on the edge of the Yangtze near Anhui province. Yang Qiaoyun oversaw the army's health activities. Sixty years later, in a memoir, Qiaoyun recalled how General Su's soldiers were spirited and motivated and how they quickly immersed themselves in the aquatic training. Soon, however, many of them suffered from severe diarrhoea and high fever, making them unfit for action. Unprepared for this, Qiaoyun remembered, the army doctors, most of whom were also from northern China, misdiagnosed the problem as dysentery and malaria and treated it accordingly. Unable to see any improvement, the army's health unit engaged some medical experts from local hospitals and the Nationalist government's Central Health Station. After examining their faeces, the experts determined that the soldiers were infected with a locally endemic disease, schistosomiasis japonicum.²

Schistosomiasis japonicum (schistosomiasis, hereafter), also known as 'oriental bilharzia', was linked to the rural way of life and especially rice-growing farmers. It is caused by the blood fluke *Schistosomiasis japonica*. The larval form of this parasite lives in snails, but it can penetrate the human body via the skin when contacted in water. In the first instance, those infected would often experience minor symptoms of abdominal pain, diarrhoea, and blood in their stools. If left untreated, the disease can be debilitating as the trapped larvae or eggs increasingly introduce inflammatory reactions in various human organs. This can cause liver enlargement as well as, in more severe cases, enlargement of the spleen, nervous disorders, and potentially death. Historically, for the villagers who lived in affected regions, schistosomiasis, as well as other diseases such as malaria and otitis,

对于居住在长江以南以及中国西南腹地的人们来说，冲突及其伴随的不确定性仍在持续。在淮海战役取得辉煌胜利后，中国共产党（ccp）和中国人民解放军（pla）领导层并没有停下脚步。统一中国的梦想需要更加艰苦的斗争。2月，华东野战军进行了认真的重组，并更名为第三野战军。苏玉将军再次负责渡江的关键军事行动。为了迎接战役，苏挑选了一些他最优秀的年轻士兵进行准备训练。渡过这条河需要在水中作战，但苏的大部分士兵来自山东省，不习惯在水路上作战。因此，他们训练的一个主要部分是在安徽省附近长江边缘的迷宫般的小溪中进行强化游泳训练。杨巧云负责军队的健康活动。六十年后，巧云在一篇回忆录中回忆起苏将军的士兵们是多么的意气风发和充满动力，以及他们是如何迅速投入到水上训练中的。然而，很快，他们中的许多人就患上了严重的腹泻和高烧，使他们无法行动。巧云回忆说，由于对此毫无准备，军队的医生，他们中的大多数也来自中国北方，将这个问题误诊为痢疾和疟疾，并据此进行了治疗。由于看不到任何改善，军队的卫生队聘请了当地医院和国民政府中央卫生站的一些医学专家。在检查了他们的粪便后，专家们确定士兵感染了一种地方性疾病，即日本血吸虫病。²

日本血吸虫病（以下简称血吸虫病），又称“东方血吸虫病”，与农村生活方式，尤其是水稻种植农民息息相关。它是由血吸虫日本血吸虫引起的。这种寄生虫的幼虫生活在蜗牛体内，但当人在水中接触时，它可以通过皮肤进入人体。最初，感染者通常会出现轻微的腹痛、腹泻和便血症状。如果未经治疗，这种疾病可能会使人衰弱，因为被困的幼虫或卵子会越来越多地在人体各个器官中引起炎症反应。这可能导致肝脏肿大，在更严重的情况下，还会导致脾脏肿大、神经紊乱，甚至死亡。历史上，对于生活在受影响地区的村民来说，血吸虫病以及其他疾病，如疟疾和中耳炎，

were understood as divine retributions for misdeeds or were caused by bad *fengshui*. During the outbreak seasons, villagers would light incense at their associated temples and make offerings to the gods, asking for forgiveness and protection. The Confucian temple at Buyun Township in Zhejiang province's Jiaxing County was conveniently situated near a bridge. As one of the larger temples in the region and one of the busiest, it had more rooms, better facilities, and quite a few resident nuns. During the disease outbreak seasons, the temple served as a refuge for those local people who fell sick. After the Communist army takeover in Jiaxing, the temple, along with many other religious institutions, were converted into mass treatment centres for schistosomiasis victims, and in 1952 the temple was officially configured into Jiaxing region's schistosomiasis prevention centre.³

The Yangtze River delta and the outflow areas of the river (which are also known as 'the lake regions') are called 'China's rice basket'. The lush paddy fields, the large areas of wetland, and the humid climate conditions provide, in addition to rich crops, a fertile environment for the snails that are the carriers of schistosomiasis. When the PLA's Third Field Army was preparing for the river crossing, it was estimated that the disease was endemic in some 1,300 townships near the prospective battlefield and that there were more than 11 billion square metres of snail-infested areas.⁴ The disease had already impacted more than 10 million of the local population.⁵ Since spring was the outbreak season, it was not surprising that many of the soldiers, being from the north and with little or no resistance to the disease, became infected. The serious schistosomiasis outbreaks amongst the PLA soldiers as they were preparing for crossing the Yangtze also demonstrated how quickly the disease could become endemic. As mentioned, the blood fluke can penetrate the human body via the skin when contacted in water and cause infection immediately. The regions along the lower reaches of the Yangtze are covered with extensive networks of waterways. Traditionally, local populations built their houses along the water. Families used the same water for cooking and for washing vegetables and clothing as well as for cleaning their 'night stool' or chamber pots. Boat people who lived on their boats had no toilet facilities and they excreted directly into the river, as did their dogs. Such life habits meant that the water could easily become contaminated, and the disease could spread quickly through this contaminated water. In addition, local farmers used a mixture of river mud, green manure, and human

被认为是神明对罪行的惩罚，或是由不良的风水引起的。在疫情爆发的季节，村民会在相关的寺庙里点香，并向神灵供奉祭品，祈求宽恕和庇护。浙江省嘉兴市步云乡的孔庙地理位置优越，靠近一座桥梁。作为该地区规模较大、香火最旺的寺庙之一，它拥有更多的房间、更好的设施和相当多的常驻尼姑。在疾病爆发季节，该寺庙是当地患病民众的避难所。共产党军队接管嘉兴后，该寺庙与许多其他宗教机构一起，被改造成血吸虫病患者的治疗中心，1952年，该寺庙正式被配置为嘉兴地区的血吸虫病防治中心。³

长江三角洲和河流的流出区（也称为“湖区”）被称为“中国的米粮仓”。茂盛的稻田、大面积的湿地和潮湿的气候条件，除了提供丰富的农作物外，还为血吸虫病的传播媒介钉螺提供了肥沃的环境。当中国人民解放军第三野战军准备渡江时，据估计，在未来的战场附近，约有1300个乡镇流行这种疾病，并且有超过110亿平方米的螺 infested 地区。⁴ 该疾病已经影响了当地超过1000万人口。⁵ 由于春季是疫情爆发的季节，因此来自北方且对该疾病几乎没有抵抗力的士兵中，有许多人被感染也就不足为奇了。解放军战士在准备渡江时爆发的严重血吸虫病也表明了该疾病传播的速度有多快。如前所述，血吸虫在水中接触时可以通过皮肤侵入人体并立即引起感染。长江下游地区布满了广泛的水道网络。传统上，当地居民沿着水边建造房屋。家庭用同一水源做饭、洗菜、洗衣服以及清洗“夜壶”。住在船上的船民没有厕所设施，他们及其狗直接在河里排泄。这种生活习惯意味着水源很容易被污染，并且疾病可以通过这种被污染的水迅速传播。此外，当地农民还使用河泥、绿肥和人类的混合物

excrement as fertilizer in their paddy fields.⁶ After the schistosoma eggs entered the paddy fields, when the weather turned warmer they would quickly hatch into snails and multiply in great numbers. Prior to the incident of outbreak amongst the PLA soldiers, there had been frequent reports of outbreaks in previous years that had wiped out some entire villages in the regions.⁷

Despite being severely weakened by the disease, this was no time for the army, including the sick soldiers, to retreat. After the Nationalists turned down the proposal from the United States to sign a peace agreement with its Communist rival, on 20 April 1949 the PLA Third Field Army fired at HMS *Amethyst*, the British Royal Navy frigate sailing down the Yangtze to the Nationalist capital Nanjing on a mission to protect British interests there.⁸ This first gunfire marked the beginning of the PLA's Yangtze River-Crossing Campaign, also known as the Nanjing, Shanghai, and Hangzhou Military Campaign. The campaign signalled that the Communists were approaching the point where they would take over the rest of China. On 23 April, the PLA Third Field Army captured Nanjing, and in the next five weeks it rallied and successfully seized Hangzhou and Shanghai, two of the most important cities on the Yangtze River. The CCP's red flag with its five stars replaced the blue sky-white sun flag of Republican China. In the Yangtze River delta, people welcomed peace, but they did not see the Communist army as fulfilling any auspicious heavenly promise. Many of them were still tormented by the traumatic memory of being terrorized by the civil war that had waged between the Taiping heavenly army and the Qing imperial army some eighty years earlier and, more recently, by the Japanese occupation. Would these young peasant soldiers from northern China who spoke strange dialects be any different? The local populations observed them with fear and distrust that was mingled with a certain amount of curiosity. Some were impressed by how disciplined the PLA soldiers were. Others were intrigued to see female cadres walking on the street dressed in their grey uniforms.⁹

For the PLA as well as for the CCP's southbound officers, the relatively easy conquest was, however, followed by further complications. Just as the local population found these northern peasant cadres and soldiers strange, the CCP officials and soldiers found the southern climate and culture challenging, as they did the incomprehensible and varied dialects spoken in the region. Unable to tell the difference between green and black tea, some CCP officials

在稻田里用粪便作肥料。⁶血吸虫卵进入稻田后，天气转暖时会迅速孵化成钉螺，并大量繁殖。在解放军战士爆发疫情之前，此前曾多次报道过该地区一些村庄全村覆灭的疫情事件。⁷

尽管受到疾病的严重削弱，但这并不是军队（包括生病的士兵）撤退的时候。在国民党拒绝了美国提出的与共产党对手签署和平协议的提议后，1949年4月20日，解放军第三野战军向英国皇家海军护卫舰紫石英号开火，该护卫舰沿长江驶往国民党首都南京，执行保护英国在那里的利益的任务。⁸这第一声枪响标志着解放军渡江战役的开始，也称为南京、上海、杭州战役。这场战役预示着共产党即将接管中国其他地区。4月23日，解放军第三野战军占领了南京，并在接下来的五个星期里集结力量，成功夺取了长江上最重要的两个城市杭州和上海。中国共产党的五星红旗取代了中华民国的青天白日旗。在长江三角洲，人们欢迎和平，但他们并不认为共产党军队正在履行任何吉祥的天命。他们中的许多人仍然受到太平天国军队与清朝皇军之间大约八十年前爆发的内战，以及最近的日本占领所造成的创伤记忆的折磨。这些来自中国北方、说着奇怪方言的年轻农民士兵会有什么不同吗？当地居民怀着恐惧和不信任的心情观察着他们，但也夹杂着一定程度的好奇。有些人对解放军士兵的纪律性印象深刻。另一些人则对看到穿着灰色制服走在街上的女干部感到好奇。⁹

对于 PLA 以及中共的南下干部来说，相对容易的征服之后，随之而来的是进一步的复杂情况。正如当地居民觉得这些北方农民干部和士兵很奇怪一样，中共官员和士兵也觉得南方的气候和文化充满挑战，他们也觉得该地区令人费解且多种多样的方言难以理解。由于无法分辨绿茶和红茶，一些中共官员

dismissed the tea-drinking culture in the south as decadent and thus ‘unhealthy’.¹⁰ For the CCP, however, the more pressing issue was the military resistance of various local non-Communist groups, including the remnants of Nationalist loyalists. At the same time, although Chiang Kai-shek and the Nationalist army had been driven to the small island of Formosa (Taiwan, as it is known today), they continued to present a serious military threat. There was a constant fear that they would attack the mainland. In August 1949, less than three months after the Third Field Army entered Shanghai, the CCP Central Committee, led by Chairman Mao Zedong, gave an order to General Su Yu to train his men to be prepared to attack Taiwan as well as to guard the security of Shanghai and the surrounding regions. The Ninth Regiment’s Third Field Army (the PLA Twentieth Group Army) was assigned to the task. To invade Taiwan would involve the soldiers being able to swim, since they would have to fight battles in waterways. This entailed at least an hour of intensive daily swimming lessons for the soldiers in the waterways in Zhejiang’s Jiaxing and Pinghu and at Jinshan, a small port of great military importance just southwest of Shanghai. Since these were schistosomiasis-endemic regions, not surprisingly more than one-third of the soldiers from the three army units were quickly reported to have been infected with schistosomiasis japonicum. The news of yet another serious schistosomiasis outbreak shocked the CCP leadership. They began to see this strange southern disease as an additional threat to their newly gained victory. Could this shatter their mission to unify China?

Would the Battle of Red Cliff, recounted in the classics of Chinese history, be repeated? In one of China’s most important history books, the *Comprehensive Mirror to Aid in Government* (资治通鉴), compiled by the northern Song scholar official Sima Guang (1019–1086), as well as in other works, there is the account of the famous Battle of Red Cliff (third century CE). The book tells how disease and poor health had contributed to General-in-Chief Cao Cao’s defeat: in the winter of 208–09, Cao Cao moved to unify the Han Empire. While preparing for the Battle of Red Cliff at Dongting Lake on the Yangtze River in central China, his soldiers succumbed to a mysterious illness. Like the Third Field Army soldiers in the twentieth century, Cao Cao’s men in the third century were also northerners from the region along the Yellow River delta. Unaccustomed to the strange environment and humid climate of the Yangtze region, they

他们认为南方饮茶文化是颓废的，因此是“不健康的”。¹⁰然而，对于中国共产党来说，更为紧迫的问题是各个地方非共产党团体的军事抵抗，包括国民党效忠者的残余势力。与此同时，尽管蒋介石和国民党军队已被赶到福摩萨小岛（即今天的台湾），但他们仍然构成了严重的军事威胁。人们一直担心他们会袭击大陆。1949年8月，在第三野战军进入上海不到三个月后，由毛泽东主席领导的中共中央委员会命令粟裕将军训练他的部队，准备进攻台湾，并保卫上海及周边地区的安全。第九团第三野战军（中国人民解放军第二十集团军）被分配到这项任务。入侵台湾将涉及到士兵能够游泳，因为他们将不得不在水域中作战。这需要士兵们每天至少进行一小时的高强度游泳课程，地点在浙江嘉兴、平湖以及上海西南方向具有重要军事意义的小港口金山。由于这些都是血吸虫病流行区域，毫不奇怪，来自三个军队单位的三分之一以上的士兵很快就被报告感染了日本血吸虫病。又一次严重的血吸虫病爆发的消息震惊了中共领导层。他们开始将这种奇怪的南方疾病视为对其新取得胜利的又一威胁。这会粉碎他们统一中国的使命吗？

中国历史典籍中记载的赤壁之战会重演吗？在中国最重要的史书之一《资治通鉴》（*Comprehensive Mirror to Aid in Government*）中，由北宋学者司马光（1019-1086）编纂，以及其他著作中，都有对著名的赤壁之战（公元三世纪）的记载。书中讲述了疾病和不良健康状况是如何导致总司令曹操战败的：公元208-09年冬季，曹操试图统一汉朝。在长江中部的洞庭湖一带准备赤壁之战时，他的士兵们得了一种神秘的疾病。就像20世纪的第三野战军士兵一样，三世纪的曹操士兵也是来自黄河三角洲沿线的北方人。他们不习惯长江地区陌生的环境和潮湿的气候，

had become severely ill. Cao Cao and his remaining army were forced to flee.¹¹ Their defeat at the Battle of Red Cliff is widely acknowledged as one of the most catastrophic moments in Chinese history. Some recent studies by Chinese researchers have identified schistosomiasis as the ‘mysterious’ illness that caused the defeat of Cao Cao’s army.¹² As result of Cao Cao’s defeat at the Battle of Red Cliff, China was divided into three warring countries, known as the Three Kingdoms. The Three Kingdom Period (220–80), also known as the ‘Period of Disunity’, is remembered in China as one of the bloodiest periods in history.

The CCP and its PLA were determined that they would be immortalized in the history books not as failed rebels but as victorious conquerors. Proven unbeatable so far in their struggle against the Nationalist armies, they now embarked on a battle to defeat the ‘enemy of the natural world’. The newly founded CCP Shanghai Municipal Government, led by Marshall Chen Yi, appointed Cui Yitian, a southern-bound CCP cadre and graduate of the Liaoning Medical University, to take charge of the battle against this strange southern parasitic disease: schistosomiasis japonicum. Cui was a native of Liaoning province in Manchuria. In September 1931, his home region was lost to the Japanese. Believing that wartime medical and health work was essential in saving his country and the Chinese people from becoming ‘stateless slaves’ (colonized by Japan), he joined the CCP and volunteered to go to the battlefield to treat sick and wounded soldiers. Before the PLA entered Shanghai, Cui, along with thousands of patriotic youth who supported the Communist revolution, underwent a month-long intensive cadre-training program. During this time, he worked closely with Marshall Chen Yi to plan future medical and health work in Shanghai and the surrounding regions. Schistosomiasis had not been on their agenda; as a northerner, Cui claimed little or no knowledge of it. Now, in his position as the new director of the Shanghai Municipal Health Bureau, Cui appealed to medical and health professionals in Shanghai and Zhejiang province, where the PLA soldiers were infected, for help.¹³

The local experts quickly joined forces with their new Communist patron. Linked to the rural way of life and particularly to rice-growing farmers, schistosomiasis was of political, economic, and cultural importance in this region. Local records tell us that from the fifteenth to the nineteenth century, several outbreaks along the Yangtze River delta had devastated the region. The dramatic increase in the

已经病得很重。曹操和他的残余军队被迫逃跑。¹¹ 他们在赤壁之战中的失败被广泛认为是中国历史上最具灾难性的时刻之一。中国研究人员最近的一些研究表明，血吸虫病是导致曹操军队失败的“神秘”疾病。¹² 由于曹操在赤壁之战中战败，中国分裂为三个交战国，即三国。三国时期（公元 220-80 年），又称“分裂时期”，在中国被认为是历史上最血腥的时期之一。

中国共产党及其解放军决心在史书上永垂不朽，不是作为失败的叛军，而是作为胜利的征服者。到目前为止，他们在与国民党军队的斗争中已被证明是不可战胜的，现在他们开始了与“自然界的敌人”作斗争。新成立的中共上海市人民政府在陈毅元帅的领导下，任命南下干部、辽宁医科大学毕业生崔义田负责对抗这种奇怪的南方寄生虫病：日本血吸虫病。崔是满洲里辽宁省人。1931 年 9 月，他的家乡沦陷于日本人之手。他认为战时医疗卫生工作对于拯救国家和中国人民免于成为“无国籍奴隶”（被日本殖民统治）至关重要，因此他加入了中国共产党，自愿奔赴战场，医治伤病员。在解放军进入上海之前，崔和数千名支持共产主义革命的爱国青年一起，接受了为期一个月的强化干部培训计划。在此期间，他与陈毅元帅密切合作，计划未来在上海及周边地区的医疗卫生工作。血吸虫病并不在他们的议事日程上；作为一个北方人，崔声称对它知之甚少。现在，作为上海市卫生局新任局长，崔呼吁上海和浙江省（解放军士兵感染血吸虫病的地方）的医疗卫生专业人员提供帮助。¹³

当地专家迅速与他们新的共产主义赞助人联手。血吸虫病与乡村生活方式，特别是与种植水稻的农民息息相关，在该地区具有政治、经济和文化重要性。当地记录告诉我们，从 15 世纪到 19 世纪，长江三角洲沿岸的几次爆发摧毁了该地区。急剧增加的

number of people infected by the disease was linked to the agricultural expansion driven by the imperial government and backed by local elites, with the aim of relieving the chronic problem of famine. Large-scale irrigation projects and the newly opened paddy fields, as well as the increased amount of river mud being used as fertilizer, created an ideal environment for snails – the vector for schistosomiasis – to breed. At the same time, intense agricultural activities increased the chance of infection as more and more people were exposed to the vector. The steady population growth, on the one hand, as a result of agricultural migration and famine control, and the increase in agricultural livestock, on the other, speeded up transmission. In the second half of the seventeenth century, the disease was said to have become so prevalent that it was regarded by the local people as a real menace. While some died of the disease, even more fled in fear of being infected. The disease, coupled with the Taiping Rebellion in the 1860s – the worst civil war in Chinese history – resulted in large parts of the countryside being left uninhabited and agricultural fields abandoned.¹⁴

Social reformers and modernizers from the endemic regions linked the disease to infertility and thus to under-population and poor productivity. They viewed it as a serious handicap to rural development, contributing to the overwhelming rural poverty and more generally to the backwardness of China. Experts trained in Western allopathic ‘tropical medicine’ and public health were called in to begin a top-down attack on the spread of the infection. One of these was Chen Fangzhi, a graduate of the Imperial Tokyo University’s Medical School created under the leadership of German physicians in the late nineteenth century. In 1928, Chen was appointed the first director of the Sanitary Department in the Nationalists’ Ministry of Interior. When the Sanitary Department was incorporated into the newly established Ministry of Health later that year, Chen passed the directorship to his former student from the Zhejiang Public School of Medicine (founded by doctors trained under the German and Japanese system 德日系), Yao Yongzheng. Chen himself went to Shanghai to head the Ministry of Health’s new agency, the Central Hygiene Laboratory (CHL). The task of the CHL was to conduct chemical and pharmaceutical analysis, bacteriological and pathological examinations, and laboratory research, as well as to exercise control over drugs and patent medicine.¹⁵ During his tenure, Chen conducted extensive field research on the problem of schistosomiasis

感染该疾病的人数与帝国政府推动并得到地方精英支持的农业扩张有关，其目的是缓解长期的饥荒问题。大规模的灌溉工程和新开垦的稻田，以及用作肥料的河泥数量的增加，为血吸虫病的传播媒介——钉螺的繁殖创造了理想的环境。与此同时，随着越来越多的人暴露于传播媒介，密集的农业活动增加了感染的机会。一方面，农业移民和饥荒控制导致人口稳定增长，另一方面，农业牲畜的增加加速了传播。据说在 17 世纪下半叶，这种疾病变得非常普遍，以至于当地人认为它是一种真正的威胁。虽然有些人死于这种疾病，但更多的人因害怕被感染而逃离。这种疾病，加上 1860 年代的太平天国叛乱——中国历史上最严重的内战——导致大片乡村无人居住，农田被废弃。¹⁴

来自流行地区的社会改革家和现代化人士将这种疾病与不孕症联系起来，从而与人口不足和生产力低下联系起来。他们认为这是农村发展的一个严重障碍，导致普遍的农村贫困，更普遍地导致中国的落后。接受过西方对抗疗法“热带医学”和公共卫生培训的专家被召集来，开始自上而下地攻击这种感染的传播。其中一位是陈方之，他是帝国东京大学医学院的毕业生，该医学院在 19 世纪末在德国医生的领导下创建。1928 年，陈被任命为国民党内政部卫生司第一任司长。当年早些时候，卫生司被纳入新成立的卫生部后，陈将司长的职位传给了他在浙江公立医学院（由接受过德日系医生培训的医生创办）的学生姚永正。陈本人去了上海，负责卫生部的新机构，中央卫生实验所 (chl)。chl 的任务是进行化学和药物分析、细菌学和病理学检查以及实验室研究，以及对药物和专利药进行控制。¹⁵ 在任职期间，陈对血吸虫病问题进行了广泛的实地调查

in the Shanghai and Zhejiang region, which made him one of the foremost Chinese experts on the subject. His research established the most authoritative data used by the new Communist authority as well as by the experts engaged by the CCP to tackle the disease in the initial stage of its anti-schistosomiasis crusade. A native of rural Zhejiang, Chen grew up witnessing the extensive damage caused by the disease. He called schistosomiasis ‘the disease of national humiliation’ (国耻病) because it injured China’s national economy as well as people’s livelihood – two keys to China’s rejuvenation, according to Sun Yatsen, a trained physician who became the founder of modern China. For Chen, schistosomiasis control work had to be a responsibility shared by all Chinese persons.¹⁶

Chen’s concern linked the disease to the survival of China as a country and a ‘race’. This concern was also widely felt by other members of local medical communities, most of whom were Western-trained and who advocated modernizing China through bio-medicine. Xu Xifan, who also grew up in rural Zhejiang province, was a graduate of the Rockefeller Foundation’s Peking Union Medical College (PUMC) and had a PhD in parasitology (financed by the Rockefeller Foundation International Health Commission) from the University of Neuchatel in Switzerland. Reinhard Hoeppli, a former doctor for the German navy during World War I who later became a distinguished parasitologist and headed the PUMC’s Division of Parasitology, once remarked that Xu was ‘one of the best qualified of the younger workers in the field of parasitology in China’.¹⁷ In 1948 and 1949, Xu, by then a world-class helminthologist, was the head of the Parasitology Laboratory of the Nationalist government’s National Institute of Health in Nanjing. In 1948, after he witnessed how a major schistosomiasis outbreak that year had wiped out the entire population of a village in Zhejiang province’s Qu County and left lush agricultural land in waste, he compared schistosomiasis to the ‘opium plague’: ‘If not treated and prevented, there will be no one left in the countryside to tend the farming fields.’¹⁸ By linking the disease to people’s livelihood, a key principle of the Chinese revolution, tackling the schistosomiasis problem was not only of military urgency for the CCP but a critical issue of national concern. Successful control of the disease and reducing the mortality from it was an effective way to consolidate the CCP’s rule in this ‘disease-swamped land’ – which was the image of China that was depicted by many Western observers as well China’s modernizers.

在上海和浙江地区，这使他成为中国该领域最重要的专家之一。他的研究确立了新的共产主义当局以及中共聘请的专家在血吸虫病防治运动初期所使用的最权威的数据。陈出生于浙江农村，亲眼目睹了该疾病造成的广泛损害。他称血吸虫病为“国耻病”，因为它损害了中国的国民经济和人民的生计——根据训练有素的医生、现代中国的创始人孙中山的说法，这是中国复兴的两个关键。对于陈来说，血吸虫病防治工作必须是中国人共同承担的责任。¹⁶

陈的担忧将这种疾病与中国作为一个国家和一个“种族”的生存联系起来。当地医学界的其他成员也普遍感受到这种担忧，他们中的大多数人都受过西方培训，并主张通过生物医学实现中国的现代化。徐锡凡也出生于浙江农村，毕业于洛克菲勒基金会的北京协和医学院（pumc），并在瑞士纳沙泰尔大学获得了寄生虫学博士学位（由洛克菲勒基金会国际健康委员会资助）。Reinhard Hoeppli 曾担任第一次世界大战期间德国海军的医生，后来成为著名的寄生虫学家，并担任 pumc 的寄生虫学系主任，他曾评论说，徐是“中国寄生虫学领域最有资格的年轻工作者之一”。¹⁷ 1948 年和 1949 年，当时已是世界级的蠕虫学家的徐锡凡担任国民政府设在南京的国立卫生研究所寄生虫学实验室主任。1948 年，他亲眼目睹了当年血吸虫病的大爆发如何摧毁了浙江省衢县的一个村庄的全部人口，并使肥沃的农田荒芜，他将血吸虫病比作“鸦片瘟疫”：“如果不治疗和预防，农村将没有人耕种农田。”¹⁸ 通过将这种疾病与人民的生计联系起来，这是中国革命的关键原则，解决血吸虫病问题不仅对中共具有军事上的紧迫性，而且是国家关注的关键问题。成功控制这种疾病并降低其死亡率是巩固中共在这个“疾病泛滥的土地”上的统治的有效途径——这是许多西方观察家和中国现代化人士所描绘的中国形象。

POLITICS OF THE DISEASE

After World War II, a global interest in fighting schistosomiasis emerged. This budding interest had more than just a purely scientific impetus. The increasing presence of the United States in the Pacific, as well as the British involvement in Egypt, meant that this previously little-known ‘Oriental disease’ and its ‘African cousins’ were now of geopolitical and global importance.

A decade earlier, in 1938, at the twenty-ninth session of the League of Nations’ Health Committee (LNHC), Dr Hilmy Bey, then Egypt’s under-secretary of state for public health (medical affairs) and the country’s official delegate to the League of Nations Health Organization (LNHO), had warned the LNHC that the problem of schistosomiasis was becoming more prevalent in many parts of the world. He proposed that the LNHC undertake serious investigation of the disease ‘on lines similar to those followed by the [League of Nations] Malaria Commission’ and organize international collaborations to ensure its prevention.¹⁹ Responding to the Egyptian proposal, the LNHC called upon a handful of Western experts, led by Robert Leiper, the director of the Department of Parasitology at the London School of Hygiene and Tropical Medicine.

At the start of his career, Leiper had spent 1906–07 in Cairo studying under Robert Looss, the celebrated German helminthologist. It was in Egypt, where a third of population was infected by schistosomiasis, that Leiper had developed a research interest in the disease. In November 1913, the British Colonial Office made a special grant to the relatively new London School of Tropical Medicine (which was founded in 1899 and later changed its name to the London School of Hygiene and Tropical Medicine following a Rockefeller grant to rebuild and fund a school of public health) to allow Leiper, the school’s Wandsworth Scholar, to go to the Far East ‘to study the mode of spread of bilharziasis [an alternative European name for schistosomiasis] and to obtain ... definite experimental evidence on the subject’. Leiper’s ‘Oriental’ expedition also gained support from the British Royal Navy, since *Schistosomiasis japonica* was said to be a concern for the crews of British ships operating on the Yangtze River. It seconded the Navy Surgeon-Lieutenant Atkinson to assist Leiper’s scientific expedition. Although the expedition was ‘abruptly’ interrupted by the outbreak of World War I, Leiper was reported to have obtained enough evidence to ‘throw discredit’ upon Looss’s

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二战后，全球对抗血吸虫病的兴趣日益浓厚。这种萌芽中的兴趣不仅仅是纯粹的科学动力。美国在太平洋地区日益增加的存在，以及英国在埃及的参与，意味着这种以前鲜为人知的“东方疾病”及其“非洲表亲”现在具有地缘政治和全球重要性。

十年前，1938年，在国际联盟卫生委员会（lnhc）第二十九届会议上，时任埃及公共卫生（医疗事务）国务秘书兼该国国际联盟卫生组织（lnho）官方代表 Hilmy Bey 博士警告 lnhc，血吸虫病问题在世界许多地方变得越来越普遍。他建议 lnhc 对该疾病进行认真的调查，“类似于 [国际联盟] 疟疾委员会”所采取的路线，并组织国际合作以确保其预防。¹⁹ 为了回应埃及的提议，lnhc 召集了少数西方专家，由伦敦卫生与热带医学院寄生虫学系主任罗伯特·莱珀领导。

在职业生涯的早期，莱珀曾在 1906-07 年在开罗师从著名的德国蠕虫学专家罗伯特·卢斯。正是在埃及，那里三分之一的人口感染了血吸虫病，莱珀对该疾病产生了研究兴趣。1913 年 11 月，英国殖民部向相对较新的伦敦热带医学院（成立于 1899 年，后来在获得洛克菲勒基金会的资助重建并资助一所公共卫生学院后，更名为伦敦卫生与热带医学院）提供了一项特别拨款，允许该学院的万兹沃思学者莱珀前往远东“研究血吸虫病的传播方式 [，血吸虫病是血吸虫病的另一种欧洲名称]，并获得……关于该主题的明确实验证据”。莱珀的“东方”探险也得到了英国皇家海军的支持，因为据说日本血吸虫病是英国在长江上作业的船只船员的担忧。日本血吸虫病被认为是英国在长江上作业的船只船员的担忧。海军外科中尉阿特金森被调来协助莱珀的科学考察。尽管这次探险因第一次世界大战的爆发而“突然”中断，但据报道，莱珀获得了足够的证据来“否定”卢斯的观点。

highly controversial hypothesis of human-to-human transmission and argued that the disease was not communicable directly from person to person.²⁰ In 1915, the British War Office sent Leiper and his two colleagues from the London School of Tropical Medicine to ‘investigate bilharzia disease in that country [Egypt] and advise on the preventive measures to be adopted in connection with the troops’.²¹ This Egyptian mission allowed Leiper to solve the main problems of the life cycles of the African species of human schistosomes, caused by blood flukes of either *Schistosoma haematobium* or *Schistosoma mansoni*. Leiper’s study showed that the transmission of each species of schistosome was firmly tied to the distribution of their permissive snail hosts (freshwater *Biomphalaria* and *Bulinus* snails), which had been historically confined to the Nile delta region. It led Leiper to conclude that schistosomiasis could be eradicated by destroying its intermediate host, the snail. This was important from the War Office’s perspective because Leiper’s discovery showed that an infected soldier could not be re-infected and could not spread the disease directly to other soldiers or to the home population in Britain where there were no local molluscs to act efficiently as carriers. In addition, according to Leiper, the disease could be effectively prevented by supplying British troops with filtered water: ‘it is obvious ... [that] in large towns where filtered water is supplied for drinking and bathing, there is practically no risk to Europeans.’ Leiper’s findings had lifted the British Army’s fear of schistosomiasis as a burden to colonial expansion.²² More than twenty years later, with Leiper as their leading expert, the LNHc took the view that the disease was only a local condition and therefore not directly related to the health of a broader Western community.²³

This attitude persisted until 1944. In late October 1944, the Allied forces entered the Leyte Gulf off the Philippines and fought what came to be the largest naval battle of World War II – perhaps the largest naval battle in history. Once the troops landed, they found themselves in one of the most endemic regions of *Schistosomiasis japonica* in Asia. Over the next few months, some 1,700 cases of *Schistosomiasis japonica* were reported amongst the US Army and the Royal Australian Air Force. It was also reported that 30 per cent of the US prisoners of war who survived internment at the Davao Penal Colony on Mindanao, Philippine Islands, were infected. Despite earlier warnings by the surgeon general, *Schistosomiasis japonica* was a disease relatively new to the US War Department. Fearing a major epidemic amongst its forces,

人与人之间传播的高度争议性假设，并认为该疾病不会直接在人与人之间传播。²⁰ 1915年，英国陆军部派遣伦敦热带医学院的莱珀和他的两位同事前往“调查[埃及]的血吸虫病，并为部队采取的预防措施提供建议”。²¹ 这次埃及之行使莱珀得以解决非洲人血吸虫生命周期的主要问题，这种血吸虫是由埃及血吸虫或曼氏血吸虫引起的。莱珀的研究表明，每种血吸虫的传播都与它们的可容许蜗牛宿主（淡水钉螺和布林蜗牛）的分布紧密相关，这些蜗牛在历史上一直局限于尼罗河三角洲地区。这使得莱珀得出结论，血吸虫病可以通过摧毁其中间宿主——蜗牛来根除。从陆军部的角度来看，这很重要，因为莱珀的发现表明，受感染的士兵不会被再次感染，也不会直接将疾病传播给其他士兵或英国的国内民众，因为英国没有可以作为有效携带者的当地软体动物。此外，根据莱珀的说法，通过向英国军队提供过滤水可以有效地预防该疾病：“很明显……[在]提供过滤水用于饮用和沐浴的大城市中，欧洲人几乎没有风险。”莱珀的发现消除了英国军队对血吸虫病作为殖民扩张负担的恐惧。²² 二十多年后，在莱珀作为其首席专家的领导下，Lnhc 认为该疾病只是一种局部疾病，因此与更广泛的西方群体的健康没有直接关系。²³

这种态度一直持续到1944年。1944年10月下旬，盟军进入菲律宾附近的莱特湾，并进行了二战中最大规模的海战——也可能是历史上最大规模的海战。部队登陆后，他们发现自己身处亚洲日本血吸虫病最流行的地区之一。在接下来的几个月里，美国陆军和澳大利亚皇家空军报告了大约1700例日本血吸虫病病例。据报道，在棉兰老岛达沃监狱殖民地幸存下来的美国战俘中，有30%的人受到感染。尽管外科医生早些时候发出了警告，但日本血吸虫病对美国战争部来说是一种相对较新的疾病。由于担心军队中爆发大规模流行病，

in early 1945 the US Army Epidemiological Board in Washington, DC, appointed a Sub-commission on Schistosomiasis to go to Leyte to undertake further investigations. Ernest Carroll Faust, a parasitologist at the Department of Tropical Medicine at Tulane University in New Orleans, was appointed its director.²⁴

Interestingly, in his earlier career Faust had spent several years in China teaching and researching. It was in China that he began to develop an interest in *Schistosomiasis japonica*. In 1924, twenty years after the Japanese pathologist Fujiro Katsurada discovered a trematode that was the etiological agent of a disease that had been a major health hazard in the Katayama region of Japan and named it ‘schistosomia japonicum’, Faust and his colleague Henry Meleney at the Rockefeller Foundation’s China enterprise – the Peking Union Medical College – published a monograph, *Studies on Schistosomiasis Japonica*.²⁵ This work was based on their meticulous field research in the central Yangtze Valley. It remained the most important and comprehensive study on the disease for the subsequent two decades. It was through their work that the world learned that the disease was a major health risk for the entire Yangtze Valley’s waterways, as well as for the coastal river valleys south of Shanghai. Faust and Meleney became widely acknowledged in the international and Chinese medical communities as the most authoritative voice on the disease, from the life cycle of the disease, to its distribution, to its treatment.

In 1945, as World War II was coming to an end, there was a strong possibility that an Allied force invading Japan would be launched from the coastal region of China, where schistosomia japonicum was endemic. Faust and Meleney’s earlier work grew ever more important, even though the dropping of atomic bombs on Hiroshima and Nagasaki made an invasion unnecessary. There remained, however, further urgent need for more epidemiological surveys, and for studies of prevention and treatment of the disease, because of the unstable political situation in China. It became necessary to disseminate such knowledge amongst the army and civilian health practitioners in a China racked by civil war. This made schistosomiasis and its control an urgent but also a fashionable field for study. In 1947, the renowned parasitologist and president of the American Society of Parasitology, Norman Stoll, another of the Rockefeller Foundation’s ‘old China hands’, produced the first assessment of the total world distribution of schistosomiasis. In his seminal publication “This Wormy World” (1947), Stoll argued

1945 年初，美国陆军流行病学委员会在华盛顿特区任命了一个血吸虫病小组委员会前往莱特岛进行进一步调查。新奥尔良杜兰大学热带医学系的寄生虫学家欧内斯特·卡罗尔·福斯特被任命为该委员会的主任。²⁴

有趣的是，在他的早期职业生涯中，福斯特曾在中国教学和研究多年。正是在中国，他开始对日本血吸虫病产生兴趣。1924 年，在日本病理学家葛西富士郎发现了一种吸虫，它是日本片山地区一种主要健康危害的病原体，并将其命名为“日本血吸虫”20 年后，福斯特和他在洛克菲勒基金会中国企业——北京协和医学院的同事亨利·梅勒尼出版了一本专著，日本血吸虫病研究。²⁵ 这项工作是基于他们在长江中游地区细致的实地调查。在随后的二十年里，它仍然是对该疾病最重要和最全面的研究。通过他们的工作，世界了解到该疾病是整个长江流域水道以及上海以南沿海河谷的主要健康风险。福斯特和梅勒尼在国际和中国医学界被广泛认为是该疾病最权威的声音，从该疾病的生命周期到其分布再到其治疗。

1945 年，随着第二次世界大战接近尾声，盟军部队很有可能从中国沿海地区发起入侵日本的行动，而日本血吸虫病在该地区是地方性流行病。福斯特和梅莱尼早期的工作变得越来越重要，即使在广岛和长崎投下原子弹使得入侵变得没有必要。然而，由于中国不稳定的政治局势，对该疾病进行更多的流行病学调查，以及预防和治疗研究的需求依然十分迫切。有必要在中国这个饱受内战蹂躏的国家里，向军队和民间的卫生从业人员传播这些知识。这使得血吸虫病及其控制成为一个紧迫但也时髦的研究领域。1947 年，著名的寄生虫学家、美国寄生虫学学会主席诺曼·斯托尔（Norman Stoll），洛克菲勒基金会的另一位“中国通”，对血吸虫病在全球的总分布情况进行了首次评估。在他的开创性出版物《这个多虫的世界》（1947）中，斯托尔认为

that schistosomiasis, a complex and little-known disease affecting 114 million people across three different regions – Asia, Africa, and Latin America – was ‘a mountain worth conquering’. Quoting an old Chinese saying, ‘If you do not scale the mountain, you cannot view the plain’, Stoll urged the international scientific community that there should be more professional engagement with the disease to ‘help reduce the prospect of having the world forge ahead to more than 3,000 million human helminthiasis by the year 2000’.²⁶ Stoll’s article became a rallying call for an international effort to tackle the disease.

Around the same time, Hilmy Bey’s successor, Dr Aly Tewlik Shousha of Egypt, who was also a member of the technical preparatory committee of the newly founded World Health Organization (WHO), pushed hard for the WHO to include the issue of schistosomiasis as one of the items on the agenda for the First World Health Assembly. On 8 July 1948 in Geneva, the question of schistosomiasis was discussed during the assembly’s eleventh meeting under the item titled ‘Special Endemic Diseases’. During the meeting, Shousha argued that after malaria, schistosomiasis was the most widespread and destructive disease in the world.²⁷ His draft resolution to establish an expert committee on the disease was supported by India, Iran, France, and Ireland, as well as the Food and Agricultural Organization of the UN (FAO) – another newly created post-war international agency. For the French and the FAO, schistosomiasis was of economic importance because it would affect agricultural production in the Mediterranean basin as well as in many other colonial food-growing regions. The FAO had actively sought to collaborate with the WHO on its plan to develop irrigation projects in the Near East (including the Middle East and parts of northern and eastern Africa) – the ‘desert belt lying along the Tropic of Cancer’. This gave an immediate urgency to the control of schistosomiasis.²⁸ But the representatives of the Eastern Bloc countries, including the USSR, Romania, Poland, and Czechoslovakia, opposed the resolution by upholding the LNHc’s earlier view that schistosomiasis was only a local condition and of only regional concern and argued that the WHO should not finance such an expert committee from its own budget.²⁹ This reflected the USSR’s overall foreign policy at the time, since it took a rather negative view on social and economic development as advocated by the UN and the United States – hence, the USSR never joined the FAO.³⁰

血吸虫病是一种复杂的、鲜为人知的疾病，影响着亚洲、非洲和拉丁美洲三个不同地区的 1.14 亿人，是 “一座值得征服的山”。斯托尔引用中国的一句老话 “不登山，不见平原”，敦促国际科学界更多地参与到疾病的专业研究中，以 “帮助减少到 2000 年全球人类蠕虫感染超过 30 亿的预期”。²⁶ 斯托尔的文章成为国际社会努力解决这一疾病的集结号。

大约在同一时间，希尔米·贝的继任者，埃及的阿里·特夫利克·舒沙博士（也是新成立的世界卫生组织 (who) 的技术筹备委员会成员）大力推动世界卫生组织将血吸虫病问题列为第一届世界卫生大会的议程项目之一。1948 年 7 月 8 日，在日内瓦举行的世界卫生大会第十一次会议上，在题为 “特殊地方性疾病” 的项目下讨论了血吸虫病问题。在会议期间，舒沙认为，继疟疾之后，血吸虫病是世界上最广泛和最具破坏性的疾病。²⁷ 他关于成立该疾病专家委员会的决议草案得到了印度、伊朗、法国和爱尔兰以及联合国粮食及农业组织 (fao) 的支持 —— 这是另一个新成立的战后国际机构。对于法国和粮农组织来说，血吸虫病具有重要的经济意义，因为它会影响地中海盆地以及许多其他殖民地粮食种植地区的农业生产。粮农组织积极寻求与世界卫生组织合作，制定其在近东（包括中东和非洲北部和东部部分地区）开发灌溉项目的计划 —— “沿北回归线延伸的沙漠带”。这立即给血吸虫病的控制带来了紧迫性。²⁸ 但包括苏联、罗马尼亚、波兰和捷克斯洛伐克在内的东欧集团国家反对该决议，他们坚持国际卫生署此前的观点，即血吸虫病只是一种地方病，只是区域关注的问题，并认为世界卫生组织不应从自己的预算中资助这样一个专家委员会。²⁹ 这反映了当时苏联的总体外交政策，因为它对联合国和美国倡导的社会和经济发展持相当消极的看法 —— 因此，苏联从未加入粮农组织。³⁰

Although the opposition to Shousha's draft resolution numbered only a small minority and the health of the world was fundamental to the WHO's claim, the opinion of the USSR and the Eastern Bloc nevertheless mattered politically. In 1947, prior to the WHO's formal inauguration, Rolf Struthers, associate director of the Rockefeller's Medical Sciences Division, had expressed deep anxiety that 'If ... Russia will not join ... it will not be a World Health Organization.'³¹ Politics and economic interests have always played a crucial role in international health. From the outset – as Dr Sze Szeming, one of the WHO's founding fathers, remembered – the WHO really had more to do with politics than medicine.³² Schistosomiasis, like malaria, had become as much a political phenomenon as a biological one. In the end, the First World Health Assembly Programme Committee compromised by agreeing to send observers to the Fourth International Congress of Tropical Medicine and to establish a study group to carry out further research and make recommendations on the control of the disease.³³

Shousha did not give up, for in a post-war Egypt anxious about its national unity and stability and concerned with rejuvenation as well as international recognition, turning fighting schistosomiasis into a global undertaking was a battle worth winning. Shousha was a strong advocate for vector control, since his teams had succeeded in wiping out malarial mosquitos after seven months of the routine application of Paris Green, a powerful larvicide for anopheline larvae.³⁴ In 1921, following the French entomologist Emile Roubaud's success in using trioxymethylene to poison anopheline larvae, Marshall Barber, a specialist working for the United States Public Health Service as well as a former and future employee of IHB and subsequently the International Health Division (IHD),* along with his assistant Theodore Brevard Hayne and the entomologist William Komp, developed this powerful larvicide. They called it Paris Green because of its green colour. With the discovery of Paris Green, the vector control approach increasingly became a key component in malaria as well as other parasitic disease control projects around the world.³⁵ For Shousha, an international war on schistosomiasis

* The International Health Division of the Rockefeller Foundation was created in 1913 with the name International Health Commission (IHC). Throughout its history, it underwent several mandate and name changes, becoming the International Health Board (IHB) in 1916 and the International Health Division (IHD) in 1927.

尽管反对舒沙决议草案的只占少数，而且世界卫生组织的根本主张是世界人民的健康，但苏联和东方集团的意见在政治上仍然很重要。1947年，在世界卫生组织正式成立之前，洛克菲勒医学科学部副主任罗尔夫·斯特鲁瑟斯曾深感担忧，“如果……俄国不加入……它将不会是一个世界卫生组织”。³¹ 政治和经济利益在国际卫生中一直发挥着至关重要的作用。正如世界卫生组织的创始人之一施思明博士回忆的那样，从一开始，世界卫生组织实际上与政治的关系比与医学的关系更大。³² 血吸虫病和疟疾一样，已经变成了一种政治现象，同时也变成了一种生物现象。最后，第一届世界卫生大会规划委员会妥协了，同意派观察员参加第四届国际热带医学大会，并成立一个研究小组，以开展进一步的研究，并就控制该疾病提出建议。³³

舒沙没有放弃，因为在战后埃及，人们既对国家统一和稳定感到焦虑，又关心国家复兴和国际认可，将血吸虫病防治转变为一项全球事业是一场值得赢得的战斗。舒沙是媒介控制的坚定倡导者，因为他的团队在常规施用巴黎绿（一种针对疟蚊幼虫的强效杀幼虫剂）七个月后，成功消灭了疟蚊。³⁴ 1921年，在法国昆虫学家埃米尔·鲁博（Emile Roubaud）成功使用三聚甲醛毒杀疟蚊幼虫之后，为美国公共卫生服务部门工作的专家马歇尔·巴伯（Marshall Barber），以及曾任职于国际卫生局（IHB）以及后来的国际卫生部门（IHD）* 的雇员，与他的助手西奥多·布雷瓦德·海恩（Theodore Brevard Hayne）和昆虫学家威廉·孔普（William Komp）一起，开发了这种强效杀幼虫剂。他们因其绿色而称其为巴黎绿。随着巴黎绿的发现，媒介控制方法日益成为全球疟疾以及其他寄生虫病控制项目的关键组成部分。³⁵ 对于舒沙来说，一场国际性的血吸虫病战争

* 洛克菲勒基金会国际卫生部于1913年成立，最初名为国际卫生委员会（IHC）。在其发展历程中，它经历了数次职权和名称变更，于1916年更名为国际卫生委员会（IHB），并于1927年更名为国际卫生部（IHD）。

would allow him to push his vector control program further forward and afield.³⁶ A few months later, under his chairmanship, the Executive Board submitted a proposed program and budget for 1950 to be discussed at the Second World Health Assembly. Accentuating the severity of schistosomiasis, which ‘after malaria, is perhaps the most widespread debilitating disease in many parts of the world’ and ‘an international problem from the social and economic, as well as from the health, point of view’, the proposed program included field action to control snails in endemic regions, therapeutic trials, and engaging relevant stakeholders to get involved in controlling the disease.³⁷ Two months later, in June 1949, the WHO Programme Committee at the Second World Health Assembly adopted this proposed program for 1950.³⁸

MEDICINE, HEALTH, AND NATION-BUILDING

Despite China's role in the creation of the WHO, the Chinese representatives kept their distance from the debate over the issue of a schistosomiasis control program within the WHO. Remember that China, where the disease was endemic, was regularly mentioned in these very international discussions about schistosomiasis, but at this point, the Nationalist government of China, which held the ‘Chinese seat’ at the United Nations until 1971, had just been driven out of the Chinese mainland by their Communist opposition and had no immediate political interest in dealing with this matter on the mainland. Yet it would soon find schistosomiasis prevalent on the island of Formosa (Taiwan). In comparison, the CCP and its new People's Republic of China (PRC) government, which was not a member of the UN until 1971, quickly became involved in schistosomiasis control. It would later use the struggle against the disease in a propaganda war against the Nationalist enemy.³⁹

Adding to the military urgency that first sparked their engagement in the issue, as was the case with the US Army in the Philippines; schistosomiasis control was also a matter of national survival for the new China.⁴⁰ Widely acknowledged as one of the worst health risks to the 8 per cent of the population living in China's agricultural heartland, the disease was linked to a ‘feudal’ way life that was seen by many of the modernizing elites to have contributed to China's overwhelming poverty. Since the health of the nation was labelled as intrinsic to China's national survival and thus high on

这将使他能够进一步推进其病媒控制计划。³⁶几个月后，在他的主持下，执行委员会提交了一份关于1950年拟议方案和预算，供第二届世界卫生大会讨论。该方案强调了血吸虫病的严重性，认为其“继疟疾之后，可能是世界许多地区最普遍的使人衰弱的疾病”，并且“从社会、经济以及健康的角度来看，都是一个国际问题”。拟议方案包括采取实地行动来控制流行地区的蜗牛，进行治疗试验，并让相关利益攸关方参与到疾病控制中来。³⁷两个月后，即1949年6月，第二届世界卫生大会上的世卫组织规划委员会通过了这项关于1950年的拟议方案。³⁸

医学、健康与国家建设

尽管中国在世界卫生组织的创建中发挥了作用，但中国代表在世界卫生组织内部关于血吸虫病控制项目问题的辩论中与该问题保持了距离。请记住，中国是该疾病的流行地区，经常在这些关于血吸虫病的国际讨论中被提及，但此时，在1971年之前一直占据联合国“中国席位”的中国国民政府刚刚被其共产主义反对派赶出中国大陆，并且没有立即的政治利益来处理大陆上的这件事。然而，它很快就会发现血吸虫病在福尔摩沙岛（台湾）上很流行。相比之下，中国共产党及其新的中华人民共和国（prc）政府，直到1971年才成为联合国成员，迅速参与了血吸虫病的控制。它后来将利用与疾病的斗争来进行一场反对国民党敌人的宣传战。³⁹

与最初激发他们参与该问题的军事紧迫性相比，正如美国军队在菲律宾的情况一样；血吸虫病控制对于新中国来说也是一个国家生存的问题。⁴⁰血吸虫病被广泛认为是生活在中国农业腹地8%人口所面临的最严重的健康风险之一，该疾病与一种“封建”的生活方式联系在一起，许多现代化精英认为这种生活方式是造成中国普遍贫困的原因。由于国民健康被认为是国家生存的内在要素，因此在中国受到高度重视

the CCP's revolutionary agenda, its pledge of a war against schistosomiasis was understood as integral to achieving the revolutionary goal of transforming China into a 'new, independent, and prosperous society'.⁴¹ Such a vow would help the CCP to legitimize its newly acquired power in the affected regions along the Yangtze regions and in southern China and in turn allow it to bring about political changes. On the other hand, the difficulty of eradicating the disease – now widely acknowledged internationally – proved a real test for the CCP, which had just succeeded in achieving the 'unachievable' by driving the Nationalists out of the Chinese mainland. This victory had stunned the world. To eradicate a disease that the capitalist West and the enemies of the world communist movement had failed to accomplish would further allow the new government to win international recognition. Future success at disease control would be the manifestation of the revolution's success on the world stage, so it was therefore a worthy prize. Shanghai and the surrounding regions near China's east coast were areas with a long history of health and medical outreach work by missionary groups as well as a concentration of Western-trained parasitologists and epidemiologists from the Nationalist era. For the public health campaigners and health officials from the Nationalist era, controlling schistosomiasis had been an integral part of a modernizing project that sought to redeem their diseased, backward country through bio-medicine and public health activities. For the medical communities there, it was also in their professional interest to support the CCP. To conquer the apparently intractable medical problem of schistosomiasis would bring them international scientific prestige and therefore status with the new regime. This was potentially a win/win situation, and they quickly began to work in concert against schistosomiasis in a political war as well as a public health one.

In December 1949, leading public health figures and medical experts in Shanghai worked alongside the new Communist authority to set up a special schistosomiasis task force, the Shanghai Region Schistosomiasis Prevention and Treatment Committee. This committee of experts consisted of thirty-six members and twenty-nine researchers. Some were specialists in the field, while others were active members of the Shanghai medical and public health community. The committee's leadership was made up of a mixture of party officials and renowned names in the public health and medical world. Amongst them was Yan Fuqing, a graduate of Yale Medical

中国共产党（CCP）的革命议程及其对血吸虫病宣战的承诺被理解为实现将中国转变为“新的、独立的、繁荣的社会”这一革命目标不可或缺的一部分。⁴¹ 这样的誓言将有助于中国共产党在长江沿岸地区和华南地区巩固其新获得的权力，并进而促成政治变革。另一方面，根除这种疾病的难度——现在在国际上已得到广泛认可——对刚刚成功地将国民党赶出中国大陆，从而实现了“不可能实现的目标”的中国共产党来说，是一个真正的考验。这场胜利震惊了世界。根除资本主义西方和世界共产主义运动的敌人都未能完成的疾病，将进一步使新政府赢得国际认可。未来在疾病控制方面的成功将是革命在世界舞台上取得成功的体现，因此这是一项值得争夺的奖项。上海和中国东海岸附近的周边地区，长期以来一直是传教团体开展卫生和医疗外展工作的地区，也是国民党时期受过西方培训的寄生虫学家和流行病学家的集中地。对于国民党时期的公共卫生运动者和卫生官员来说，控制血吸虫病一直是现代化项目不可或缺的一部分，该项目旨在通过生物医学和公共卫生活动来拯救他们患病的、落后的国家。对于那里的医疗界来说，支持中国共产党也符合他们的职业利益。征服看似棘手的血吸虫病医学问题将为他们带来国际科学声望，从而在新政权中获得地位。这可能是一个双赢的局面，他们很快开始在政治战争和公共卫生战争中共同对抗血吸虫病。

1949年12月，上海的公共卫生界领袖和医学专家与新成立的共产党政府合作，成立了一个血吸虫病防治特别工作组，即上海地区血吸虫病防治委员会。这个专家委员会由36名成员和29名研究人员组成。其中一些是该领域的专家，另一些则是上海医疗和公共卫生界的积极成员。委员会的领导层由党政官员和公共卫生和医疗界的知名人士混合组成。其中包括毕业于耶鲁医学院的颜福庆。

School and one of the most prominent educators and advocates of bio-medicine in China. Others included leading medical cadres of the PLA: Song Shilun, the former commander of the Ninth Regiment of the Third Field Army, and Cui Yitian and his colleague Gong Naiquan. Liu Shenwang, a veteran revolutionary from the Yangtze region, was appointed secretary, and Su Delong, an Oxford-trained epidemiologist, was the vice-secretary.⁴² Su had first become involved in schistosomiasis control in the 1930s after graduating from the Shanghai Medical School. Inspired by Yan Fuqing, his former dean, he had worked in a rural health experimental program in Shanghai County that aimed to introduce modern health care to the Chinese countryside. During this period, he gained first-hand experience of treating patients with advanced-stage schistosomiasis. This kind of leadership structure – small numbers of senior party military officials mixed with a group of professional experts – served as an organizational template for all professional organizations in the early PRC. This was not only to ensure that the CCP's takeover of the existing professional bodies went as smoothly as possible, it was also a mirror of the idea of the 'United Front' that had begun some thirty years earlier during the New Cultural Movement – a movement aimed at the rejuvenation of China by breaking away from traditional ethics and culture, which were blamed for preventing China from developing into a modern industrial nation. Many of those who took part were young intellectuals – college and university students – who sought to save their troubled and backward country by introducing 'Western Democracy, Science and Culture' to China.⁴³ The battle against schistosomiasis allowed them to combine all of these disparate modernizing strands into a single, focused political undertaking.

As with Chen Fangzhi and Xu Xifan, whom we met earlier, Yan Fuqing and Su Delong, as well as the non-CCP committee members, shared with the Communist revolutionaries the same New Cultural spirit that was characterized by anti-imperialism, anti-feudalism, and the desire for China's rejuvenation (or the Dream of a Strong China 强国梦).⁴⁴ Their professional honour and self-respect derived from their sense of duty to the Chinese nation and the Chinese people and had little to do with governmental changes and class interests. Such individual professional integrity was embedded in the universal love for their 'sick' country and people; hence, their desire to modernize China through medical and health work made them loyal supporters

学校以及中国最杰出的生物医学教育家和倡导者之一。其他人包括人民解放军的主要医疗干部：第三野战军第九团前指挥员宋时轮，以及崔义田和他的同事龚乃泉。来自长江地区的革命老兵刘申旺被任命为秘书，曾在牛津大学接受培训的流行病学家苏德隆担任副秘书长。⁴² 苏德隆在上海医学院毕业后，于20世纪30年代首次参与血吸虫病防治工作。受其前任院长颜福庆的启发，他曾在上海县的一个农村卫生实验项目中工作，旨在将现代医疗保健引入中国农村。在此期间，他获得了治疗晚期血吸虫病患者的第一手经验。这种领导结构——少数高级党内军方官员与一群专业专家相结合——成为新中国成立初期所有专业组织的组织模板。这不仅是为了确保中国共产党对现有专业机构的接管尽可能顺利地进行，也是对大约三十年前新文化运动期间开始的“统一战线”思想的反映——这场运动旨在通过打破传统伦理和文化来复兴中国，这些传统伦理和文化被认为是阻碍中国发展成为现代工业国家的原因。许多参与者是年轻的知识分子——学院和大学的学生——他们试图通过将“西方民主、科学和文化”引入中国来拯救他们饱受困扰和落后的国家。⁴³ 与血吸虫病的斗争使他们能够将所有这些不同的现代化潮流结合成一个单一的、重点明确的政治事业。

正如我们之前了解的陈方之和徐锡藩一样，颜福庆和苏德隆，以及非中共党员，都与共产主义革命者怀有共同的新文化精神，其特点是反帝国主义、反封建主义和渴望中华民族复兴（或强国梦）。⁴⁴ 他们的职业荣誉和自尊源于他们对中华民族和中国人民的责任感，与政府更迭和阶级利益关系不大。这种个人的职业操守植根于对他们“病态”国家和人民的普遍热爱；因此，他们希望通过医疗卫生工作实现中国的现代化，这使他们成为忠实的支持者。

of the CCP's revolutionary utopian claim to transform China into a new, independent, and prosperous modern nation, eventually free of all diseases.

Yan Fuqing's biographers also tell of an earlier encounter between Yan and the youthful Mao Zedong during the New Cultural period. At the time, Yan was the president of Hunan-Yale (Xiang-Ya 湘雅) Medical College and the vice-president of its associated hospital. Both the medical college and the hospital were a Yale missionary endeavour in Changsha (in Hunan province in central China), first set up by the medical missionary Dr Edward Hume in the early twentieth century. The aim was to 'save' China through bio-medicine and modern education. After World War I, the Treaty of Versailles (1919) granted the rights to German colonial properties in China to Japan. It sparked the May the Fourth Students Demonstration in Peking. This incident, known in Chinese historiography as the May Fourth Incident, marked the turning point of the New Cultural Movement from the movement of cultural renewal to an anti-imperialist one. Mao Zedong returned to his native Hunan province in September 1919 and took up the editorship of *New Hunan*, the journal of the Hunan-Yale Medical College that advocated the New Cultural Movement. Mao used it to propagate the spirit of New Culture as well as Bolshevism and anti-imperialism. Newly married, Mao sought help from Yan for his young wife's gynaecological problems. Yan not only treated her but treated her for free. This personal experience undoubtedly made a deep impression on Mao. Yet Yan's mass public health experiment a few years earlier in Pingxiang's Anyuan colliery in central China, bordering Hunan and Jiangxi, impressed Mao even more. From 1917 to 1919, the Rockefeller Foundation's International Health Board (IHB) sponsored a hookworm control and sanitary program in the Anyuan colliery, China's largest colliery, 'as a means of entering public health fieldwork in China'.⁴⁵ Here, they replicated their earlier successful work in hookworm control in the southern United States.⁴⁶ If it worked in the poverty-ridden areas of Jim Crow America, the assumption was that it would be successful in the poverty-stricken areas of China. The IHB appointed Yan Fuqing to take charge of the work. As with most of the IHB's public health campaigns, Yan's team began with mass treatment and health education, followed by the setting up of a permanent sanitary board to build latrines and enforce a strict discipline of hygiene.⁴⁷ In 1921, Mao, who had recently joined the CCP, and his comrades Li Lishan

这是中国共产党革命乌托邦式的主张，即要把中国改造成为一个崭新的、独立的、繁荣的现代国家，并最终摆脱一切疾病。

颜福庆的传记作者还讲述了颜与青年毛泽东在新文化运动时期的早期相遇。当时，颜是湘雅医学院的院长及其附属医院的副院长。这两所医疗机构都是耶鲁大学在长沙（位于中国中部湖南省）的传教事业，最初由医学传教士爱德华·休谟博士在二十世纪初建立。其目的是通过生物医学和现代教育来“拯救”中国。第一次世界大战后，《凡尔赛条约》（1919年）将德国在华殖民财产的权利授予日本。这引发了北京五四学生示威运动。这一事件在中国史学中被称作五四运动，标志着新文化运动从文化复兴运动转向反帝国主义运动。1919年9月，毛泽东回到家乡湖南省，担任倡导新文化运动的湘雅医学院刊物《新湖南》的编辑。毛利用它来宣传新文化精神以及布尔什维克主义和反帝国主义。新婚燕尔的毛泽东向颜寻求帮助，以解决他年轻妻子的妇科问题。颜不仅为她治疗，而且还免费治疗。这段亲身经历无疑给毛泽东留下了深刻的印象。然而，几年前颜在湖南和江西交界处，中国中部萍乡安源煤矿进行的大规模公共卫生实验给毛泽东留下了更深刻的印象。从1917年到1919年，洛克菲勒基金会的国际卫生委员会(ihb)在中国最大的煤矿安源煤矿赞助了一项钩虫防治和卫生项目，“以此作为进入中国公共卫生领域实地考察的一种手段”。⁴⁵在这里，他们复制了他们早期在美国南部钩虫防治方面的成功经验。⁴⁶如果它在吉姆·克劳美国的贫困地区有效，那么假设它在中国贫困地区也会有效。ihb任命颜福庆负责这项工作。与ihb的大多数公共卫生运动一样，颜的团队从大规模治疗和健康教育开始，然后设立一个永久性的卫生委员会，以建造厕所并执行严格的卫生纪律。⁴⁷1921年，刚加入中国共产党的毛泽东和他的同志李立三

and Liu Shaoqi chose the same colliery for their mass mobilization work. Existing scholarship on Mao and Li's Anyuan activities in this period stresses their involvement in the mass education campaigns that were advocated by leading modernizers such as Liao Qichao, Hu Shi, and Jimmy Yen, but a closer reading of the two episodes also shows that Mao and his colleagues' field strategy bore a remarkable resemblance to that employed by Yan's team during the IHB's hook-worm control campaign.⁴⁸ Along with public lectures, they made use of pamphlets, posters, and brochures to disseminate the message of revolutionary change (in place of the transmission of health and hygiene information during the IHB campaign). Although the miners at Anyuan colliery distrusted bio-medicine, the IHB campaign raised their awareness of how poor work and living conditions had contributed to their ill health and that by pushing the colliery authority to improve hygiene standards, along with their general material condition, they would, in turn, improve their own health and well-being. Mao and his colleagues capitalized on this.

The post-World War I period was an age of growing confidence in public health. In China, as well as in the Soviet Union and South America, the binary of healthy and diseased became an integral part of the revolutionary rhetoric. Hygiene rituals, on the other hand, were also a means of introducing revolutionary discipline. Mao was first introduced to the miners by a distant relative named Mao Ziyun, a folk healer who had won the miners' trust by healing their minor ailments such as sore throats. Mao, however, took it much further in showing the miners a more radical path: that they could achieve betterment as well as human dignity through revolutionary upheaval that was 'out of the barrel of gun'.

NEW STATE, OLD CHALLENGES

In 1949, the CCP took the world by surprise by driving the Nationalists out of the Chinese mainland in spite of the American support that the Nationalists had been given. Having achieved power 'out of the barrel of gun', the CCP leadership was presented with the challenge of continuing to fight disease and of improving the people's health, with both being part of its revolutionary goal from the outset. In the new China, disease and ill health, once seen as the enemy of the revolution, continued to threaten the security of this new and fragile nation-state as well as drain its limited resources.

刘少奇也选择了同一煤矿进行群众动员工作。现有关于毛泽东和李在这一时期的安源活动的学术研究强调他们参与了由梁启超、胡适和晏阳初等主要现代化人士倡导的群众教育运动，但仔细阅读这两个事件也可以看出，毛泽东及其同事的实地战略与晏阳初团队在ihb的钩虫控制运动中采用的战略非常相似。⁴⁸除了公开讲座外，他们还利用小册子、海报和宣传册来传播革命变革的信息（而不是在ihb运动期间传播健康和卫生信息）。尽管安源煤矿的矿工不信任生物医学，但ihb运动提高了他们对恶劣工作和生活条件如何导致他们身体不适的认识，并且通过推动煤矿当局提高卫生标准以及改善他们的总体物质条件，他们反过来也会改善自己的健康和福祉。毛泽东和他的同事们利用了这一点。

第一次世界大战后的时期是公众健康信心日益增长的时代。在中国、苏联和南美，健康与疾病的二元对立成为了革命言论不可或缺的一部分。另一方面，卫生习惯也是引入革命纪律的一种手段。毛第一次被一位名叫毛子云的远房亲戚介绍给矿工，毛子云是一位民间医生，他通过治疗矿工的轻微疾病（如喉咙痛）赢得了矿工的信任。然而，毛更进一步地向矿工展示了一条更为激进的道路：他们可以通过“枪杆子里面出政权”的革命动荡来实现改善和人的尊严。

新国家，老挑战

1949年，尽管国民党得到了美国的支持，但中国共产党将国民党赶出中国大陆，令世界感到惊讶。在“枪杆子里面出政权”后，中国共产党领导人面临着继续与疾病作斗争和改善人民健康的挑战，这两者从一开始就是其革命目标的一部分。在新中国，疾病和不良健康状况曾经被视为革命的敌人，现在继续威胁着这个新兴和脆弱的民族国家安全，并消耗其有限的资源。



Figure 1.1 On 18 July 1953, Pinghu County Patriotic Health Campaign Propaganda Team parade through Chengguan township while holding the banner 'To Eradicate Schistosomiasis Is to Support the War in Korea to Fight the Americans'.

Along with natural disasters and famine, poor health and disease presented a huge burden to an economically weakened state struggling to recover from the damage of war as well as guard its security against the ongoing Nationalist threat. Of equal importance, since revolutionaries viewed ill health and disease as expressions of socio-economic inequality, medical advancement, together with the improvement of material conditions and the environment, were linked to the legitimacy of the new order. Declaring a war against a disease such as schistosomiasis would open up a field for the CCP to work with the medical community and with public health and social hygiene campaigners, giving the new authorities access to the social capital associated with such earlier undertakings.

With the beginning of the Korean War in the latter half of 1950, the need for a healthy and efficient army added a further political and military urgency to the PRC's health work. An official estimate says that more than forty million people died of infectious diseases and other illnesses between 1949 and 1950 and that many more lives



图 1.1: 1953 年 7 月 18 日, 平湖县爱国卫生运动宣传队在城关镇游行, 手举“消灭血吸虫病就是支援抗美援朝战争”的旗帜。

与自然灾害和饥荒一样, 糟糕的健康状况和疾病给一个经济上疲软、努力从战争破坏中恢复并防范持续存在的国民党威胁的国家带来了巨大的负担。同样重要的是, 由于革命者将不良健康状况和疾病视为社会经济不平等的表现, 因此医学进步与物质条件和环境的改善联系在一起, 这关系到新秩序的合法性。向血吸虫病等疾病宣战将为中国共产党与医学界以及公共卫生和社会卫生运动者合作开辟领域, 使新当局能够获得与此类早期事业相关的社会资本。

随着 1950 年下半年朝鲜战争的开始, 对一支健康和高效军队的需求, 进一步增加了中华人民共和国卫生工作的政治和军事紧迫性。据官方估计, 1949 年至 1950 年间, 有超过 4000 万人死于传染病和其他疾病, 还有更多人的生命

were permanently impacted.⁴⁹ Ill health was not only an enormous drain on social and human resources, it also seriously undermined military manpower and economic productivity. The crusade against schistosomiasis, together with other public health initiatives such as the Anti-Germ Warfare Campaign and the Patriotic Health Campaign during the Korean War (1952), became an integral part of national defence.⁵⁰

Along the Yangtze regions, it was reported that 90 per cent of the local population was infected with schistosomiasis, leaving virtually no healthy young men to join the army or to engage in agricultural activities. Near Shanghai, in Jiangsu province's Qingpu County, for example, more than 90 per cent of young men were excluded from army service after testing positive for schistosomiasis.⁵¹ Anhui province's She County was the birthplace of Ke Qingshi, another revolutionary of the New Cultural generation who would become the party boss overseeing the PRC's nationwide Anti-Schistosomiasis Campaign between 1955 and 1965. She County was at one time central China's most important cultural and trading centre and the birthplace of the Peking opera. Between 1522 and 1820, She County was reckoned to be one of China's most prosperous areas, and the local merchants were widely held in awe for their immense wealth.⁵² Yet by the time the CCP took over the region, sixty-two villages, some of which had had more than a hundred families, were deserted and without any trace of human habitation. Many formerly grand and revered memorial halls were left in ruins, and cobbled stone streets were covered in moss. Investigations have shown that severe outbreaks of schistosomiasis in 1948 and in earlier years had killed and debilitated many of the local inhabitants. Those who managed to survive had abandoned their homes and run away, leaving more than 1,600 hectares of agricultural land laid to waste.⁵³ As in earlier times, schistosomiasis continued to contribute to wasted land and deserted villages. The successful political revolution still had to deal with precisely the same health quandaries as its predecessor.

At the same time, disease, in particular schistosomiasis, and poor health were understood to have contributed to high rates of infertility and infant mortality in China. Sharing the language of the social Darwinists in Europe, social reformers and cultural and revolutionary theorists, as well as public health campaigners in China, viewed maternal and child health as indexes to China's national reconstruction or the 'Dream of a Strong China'. In September 1949, on the eve of the

受到了永久性的影响。⁴⁹ 不良的健康状况不仅是对社会和人力资源的巨大消耗,而且严重削弱了军事力量和经济生产力。与其他公共卫生举措(如抗细菌战运动和朝鲜战争期间的爱国卫生运动(1952年))一道,开展血吸虫病防治运动成为国防的重要组成部分。⁵⁰

据报道,在长江沿岸地区,当地90%的人口感染了血吸虫病,几乎没有健康的年轻人可以参军或从事农业活动。例如,在上海附近的江苏省青浦县,90%以上的年轻人因血吸虫病检测呈阳性而被排除在兵役之外。⁵¹ 安徽省歙县是柯庆施的出生地,他是新文化运动的另一位革命者,后来成为党魁,负责1955年至1965年间中华人民共和国在全国范围内的血吸虫病防治运动。歙县一度是华中地区最重要的文化和贸易中心,也是京剧的发源地。1522年至1820年间,歙县被认为是当时中国最繁荣的地区之一,当地的商人因其巨额财富而广受敬畏。⁵² 然而,当中国共产党接管该地区时,六十二个村庄(其中一些村庄有一百多个家庭)已荒无人烟,没有任何人类居住的痕迹。许多以前宏伟而受人尊敬的纪念馆变成了废墟,鹅卵石街道上长满了苔藓。调查显示,1948年和更早年份爆发的严重血吸虫病疫情已导致当地许多居民死亡和衰弱。那些设法幸存下来的人放弃了家园,逃离了家园,留下了1600多公顷的农业用地被废弃。⁵³ 与早期一样,血吸虫病继续导致土地荒芜和村庄荒废。成功的政治革命仍然必须处理与其前任完全相同的健康难题。

与此同时,疾病,特别是血吸虫病和恶劣的健康状况,被认为是导致中国高不孕率和婴儿死亡率的原因。中国的社会改良家、文化和革命理论家,以及公共卫生运动者,与欧洲的社会达尔文主义者持有相同的观点,他们将妇幼健康视为中国民族重建或“强国梦”的指标。1949年9月,在

founding of the People's Republic of China – the PRC – the top leadership of the CCP drafted their new country's provisional constitution entitled 'The Common Program of the Chinese People's Consultative Congress'. Presented by the new premier Zhou Enlai and approved by acclamation by the Chinese People's Consultative Congress, the Common Program listed child and maternal health as one of its sixty articles.⁵⁴ In this new China, children not only were seen as the building blocks of a strong China, as Yan Fuqing had once stated, but they had become 'The Future of China', 'the Little Masters', and 'the Heirs of Revolution'. To guard their health was thus to guard the fruit of the revolution and to build the future utopian state.

Because it is part of communist praxis to seek to move patterns of living toward the collective, it was inevitable that the war on disease as well as other public health initiatives in the PRC would involve the entire population. But how could the new political leadership motivate the 'masses' to take part in such a 'revolutionary' health endeavour? This conundrum would prove a real test for the CCP and its health officials. As observed at the turn of the century by the Scottish parasitologist Patrick Manson, widely acknowledged as one of the founders of 'tropical medicine' as a discipline and who had lived in the Far East for nearly twenty-five years, 'to give water taps, traps, drains, ventilator to the Chinaman ... [who were] unappreciative of their purpose and ignorant of their use ... was like giving a monkey a fiddle; they did not understand their purpose and they broke them.' In other words, these 'Chinamen' would never change their unsanitary habits.⁵⁵ For the next fifty years, despite concerted efforts by Western missionaries, the IHB, and China's modernizers, there was little improvement to be seen regarding changing the 'unclean habits' of the 'ignorant' and 'retarded' 'Chinaman'. The vast countryside continued to be rampant with disease, and the unclean habits of the Chinese peasantry were believed to have contributed to the high rate of morbidity and mortality. The need to refute imperialists, and to prove the CCP's own power, added a further urgency to involve the entire population in its heroic and historic project to transform China into a 'clean country' and to turn the 'Sick Man of Asia' into the new and healthy 'socialist man and woman'. To teach the Chinese peasants to live a clean life was to 'change the prevailing social customs' (移风易俗). Bio-medicine would serve as the pillar in this cultural revolution. The help would come from public health campaigners and members of the medical community.

中华人民共和国成立之初，中共最高领导层起草了他们新国家的临时宪法，题为《中国人民政治协商会议共同纲领》。由新任总理周恩来提出，并经中国人民政治协商会议一致通过的《共同纲领》将儿童和孕产妇健康列为其六十条条款之一。⁵⁴ 在这个新中国，孩子们不仅被视为强大中国的基石，正如严复庆曾经说过的那样，而且他们已经成为“中国的未来”、“小主人”和“革命的接班人”。因此，保护他们的健康就是保护革命的果实，建设未来的乌托邦国家。

由于寻求将生活模式向集体化方向转变是共产主义实践的一部分，因此，在中国，抗击疾病以及其他公共卫生举措必然要涉及全体人民。但是，新的政治领导层如何激励“群众”参与到这种“革命性”的卫生事业中来呢？这个难题将是对中国共产党及其卫生官员的真正考验。正如苏格兰寄生虫学家帕特里克·曼森（Patrick Manson）在本世纪初所观察到的那样，他被广泛认为是“热带医学”作为一门学科的创始人之一，并且在远东生活了近二十五年，“给中国人水龙头、陷阱、排水沟、通风机……[那些]不理解它们的用途并且不了解它们的使用方法的人……就像给猴子一把小提琴一样；他们不理解它们的用途，而且会把它弄坏。”换句话说，这些“中国人”永远不会改变他们不卫生的习惯。⁵⁵ 在接下来的五十年里，尽管西方传教士、国际卫生局以及中国的改革者们做出了共同努力，但在改变“无知”和“迟钝”的“中国人”的“不洁习惯”方面，几乎没有看到任何改善。广大的农村地区仍然疾病蔓延，人们认为中国农民不洁的习惯导致了高发病率和死亡率。为了驳斥帝国主义者，并证明中国共产党自身的力量，将全体人民纳入其将中国转变为“清洁国家”，并将“东亚病夫”转变为新的、健康的“社会主义男女”的英勇和历史性项目中，就显得更加紧迫。教导中国农民过上清洁的生活，就是“改变盛行的社会习俗”（移风易俗）。生物医学将成为这场文化革命的支柱。公共卫生运动者和医学界人士将提供帮助。

CONTROLLING SCHISTOSOMIASIS AND GUARDING CHINA'S NATIONAL SECURITY

On 19 December 1949, under the leadership of Cui Yitian, a team of thirty-three medical/health professionals from the Shanghai Medical School, the only state-run medical college in China until 1949, went to Zhejiang province's Jiading County to continue treating army casualties of schistosomiasis. For the previous decade, as part of their anti-Japanese and post-war reconstruction efforts, these health professionals had immersed themselves in treating ill and wounded soldiers and undertaking other medical humanitarian and public health work among the civilian population. Witnesses at the time stated how they saw treating the sick Communist soldiers as their irrefutable duty.⁵⁶

In the meantime, in December 1949, Mao, the leader of the new China, went on a state mission to Moscow. Stalin showed reluctance to accede to Mao's request to 'send volunteer pilots or secret military detachments to speed up the conquest of Formosa [Taiwan]' and, rather, advised Mao to organize an uprising in Taiwan using propaganda and not to attack the island with military force.⁵⁷ In January 1950, the Central Committee of the Communist Party of China (CPCC) took the decision to launch military assaults on Taiwan and other Nationalist-controlled islands between Taiwan and Zhejiang province. In order to ensure success, General Su Yu proposed that soldiers resume intensive swimming lessons, which had been terminated in the aftermath of a schistosomiasis outbreak amongst thousands of PLA soldiers a few months earlier. The Central Committee approved Su's proposal, fully aware that there was a great risk of even more soldiers being infected. To mitigate casualties, schistosomiasis treatment and prevention work became a top priority on the PLA's health and medical work agenda. On 1 February 1950, the Military Schistosomiasis Prevention and Treatment Committee was set up to ensure correct leadership and strict discipline. Schistosomiasis control was geared to military action. Geng Xichen, a veteran revolutionary from Shandong who had built up extensive experience in managing the PLA medical and health work, took charge.⁵⁸

On the ground, experts continued to provide technical assistance, but their number would increase dramatically and over a short period of time. By linking schistosomiasis control to the PLA's

控制血吸虫病，捍卫中国国家安全

1949年12月19日，在崔义田的领导下，来自上海医学院（1949年之前中国唯一的公立医学院）的33名医疗/卫生专业人员前往浙江省嘉定县，继续治疗军队中患血吸虫病的伤员。在过去的十年里，作为他们抗日和战后重建工作的一部分，这些卫生专业人员一直致力于治疗生病和受伤的士兵，并在平民中开展其他医疗人道主义和公共卫生工作。当时的目击者表示，他们认为治疗生病的共产主义士兵是他们不可推卸的责任。⁵⁶

与此同时，1949年12月，新中国领导人毛泽东对莫斯科进行了国事访问。斯大林不愿同意毛泽东提出的“派遣志愿飞行员或秘密军事分队加速征服福摩萨[台湾]”的要求，而是建议毛泽东利用宣传在台湾组织起义，不要用武力攻击该岛。⁵⁷ 1950年1月，中国共产党中央委员会（中共中央）决定对台湾以及台湾和浙江省之间的其他国民党控制的岛屿发动军事攻击。为了确保成功，粟裕将军建议士兵们恢复高强度的游泳训练，而几个月前，由于数千名解放军战士爆发血吸虫病而被终止。中央委员会批准了粟裕的提议，充分意识到有更多士兵被感染的巨大风险。为了减少伤亡，血吸虫病的治疗和预防工作成为解放军卫生和医疗工作议程中的首要任务。1950年2月1日，成立了军队血吸虫病防治委员会，以确保正确的领导和严格的纪律。血吸虫病的控制与军事行动紧密相关。耿希臣是一位来自山东的老革命家，在管理解放军医疗卫生工作方面积累了丰富的经验，他负责了这项工作。⁵⁸

在实地，专家们继续提供技术援助，但他们的数量将在短时间内大幅增加。通过将血吸虫病控制与pla联系起来

mission to liberate Taiwan and rid the country of Nationalist remnants, the political and military importance of schistosomiasis was ever increasing. More than 2,000 experts – technical staff as well as students from major medical universities in Shanghai, Nanjing, and nearby Hangzhou – were recruited to train the PLA health service personnel and assist them in carrying out extensive check-ups and treatment. They were also simultaneously to undertake thorough epidemiological investigations of the disease. To win over medical and health professionals, including medical students, in April 1950 the newly merged Shanghai Medical Workers' Union, a frontline organization assisting the CCP's takeover of Shanghai's existing medical and public health institutions, honoured 181 health and medical professionals for their 'heroic action' in supporting the PLA to guard the national security of the PRC by treating sick PLA soldiers and carrying out preventive work 'day and night', despite 'the spartan conditions and the lack of equipment'.⁵⁹ In Zhejiang province, the army officials praised students for showing 'unprecedented levels of political consciousness'. Jiang Minghua, a student nurse from the former Zhejiang Midwifery School (which merged into Zhejiang Medical University), and Wang Xiangyun, a medical student from Shanghai, were chosen as 'model workers' for demonstrating deep commitment to the work and for their caring attitudes toward these patients. As model workers, they set the example for others to follow, but more importantly they had become the exemplary 'masters of the new China'. Excited to take part in creating a new China through medical and health work, they and their colleagues were seen to be full of enthusiasm.⁶⁰ Many of the 'model workers' were given further training and went on to become frontline workers in public health and in women's work over the next decades. Whilst delivering health care and prevention, they were also the party's messengers and the bridge that linked the CCP to the people. Many more medical and public health students volunteered to serve in the Korean War to showcase the importance of medical and health work in guarding the new China's national security. The technical experts, too, were given special recognition. Mao Shoubai, for instance, who was internationally known for his research on the distribution of schistosomiasis in China's Yangtze region, was given the military award, 'second class', in addition to being honoured as Nanjing municipal's 'model worker' for supporting the PLA's schistosomiasis control work. From there he went on to lead the PRC's

为了解放台湾，清除国民党残余势力，血吸虫病在政治和军事上的重要性日益增加。上海、南京和附近杭州的主要医科大学的 2000 多名专家（技术人员和学生）被招募来培训军队的卫生服务人员，并协助他们进行广泛的检查和治疗。他们还同时对该疾病进行彻底的流行病学调查。为了赢得包括医学生在内的医疗卫生专业人员的支持，1950 年 4 月，新合并的上海医务工会（一个协助中国共产党接管上海现有医疗和公共卫生机构的前线组织）表彰了 181 名卫生和医疗专业人员，因为他们支持中国人民解放军通过治疗生病的解放军士兵和“日夜”开展预防工作来捍卫中华人民共和国的国家安全，他们的“英雄行动”，尽管“条件简陋，设备缺乏”。⁵⁹ 在浙江省，军队官员赞扬学生们表现出“前所未有的政治觉悟”。来自原浙江助产学校（合并到浙江医科大学）的护士学生蒋明华和来自上海的医学生王向云被选为“模范工作者”，因为他们对工作表现出深刻的承诺，并且对这些患者表现出关怀的态度。作为模范工作者，他们为他人树立了榜样，但更重要的是，他们已经成为模范的“新中国主人”。他们和他们的同事们很高兴通过医疗卫生工作参与创建新中国，他们被认为充满了热情。⁶⁰ 许多“模范工作者”接受了进一步的培训，并在接下来的几十年里成为公共卫生和妇女工作的前线工作人员。在提供医疗保健和预防的同时，他们也是党的信使和连接中国共产党与人民的桥梁。更多的医疗和公共卫生学生自愿参加朝鲜战争，以展示医疗卫生工作在捍卫新中国国家安全方面的重要性。技术专家也得到了特别的认可。例如，毛寿白因其在中国长江地区血吸虫病分布的研究而闻名国际，除了被授予南京市“模范工作者”称号外，还获得了军队“二等功”的奖励，以表彰他对解放军血吸虫病防治工作的支持。从那里，他继续领导中华人民共和国的

nationwide schistosomiasis control. He would also represent the new China on the international stage of parasitology. In the early years of the PRC, like many experts, Mao was relatively free of interference from the party. Thereafter, however, in 1957, with the advent of the Anti-Rightist Movement, he joined the CCP, persuaded to do so by both professional and political considerations.⁶¹

CIRCUMNAVIGATING SCIENCE

Having succeeded in co-opting young students as well as established experts into the system, disagreement amongst the experts on what approaches to take presented the Communist authorities with a serious challenge. Managing science was just as difficult as managing human resources, if not more so. One of the major obstacles in schistosomiasis control work was the unreliability of diagnosis results. Throughout the world, detection of the ova is the traditional method of schistosomiasis diagnosis. Before the two Japanese parasitologists Kan Kato and Momoshige Miura introduced the cellophane thick smear technique to the world in 1954, the standard method used for diagnosing schistosomiasis was the direct smear procedure. This was complicated and expensive and could be highly inaccurate, since it required a minute amount of faecal sample and used recyclable ordinary glass instead of disposable cellophane. The Kato method simplified such procedures. It was thought to be more accurate than the former methods, since it required a larger amount of the sample. The technique was refined in 1968, and from then on it became known as the Kato-Katz thick smear method. It is still the most widely used diagnosis technique for detecting both Asian and African schistosomiasis (*Schistosomiasis japonica* and *Schistosomiasis mansoni*). However, because of the everyday variations in the level of faecal egg excretion in individual patients, as well as its uneven distribution in excreta, both the direct smear technique, which used ordinary cover glass, and the Kato-Katz method thereafter lacked a high degree of accuracy and sensitivity.⁶² In 1950 in China, the unreliability of diagnosis results led to expert disagreement on treatment methods. Su Delong, who had gained experience in treating schistosomiasis victims during the eight years of the Sino-Japanese war, favoured a mass therapy intervention and advocated setting up massive treatment centres. This was the method introduced by John Brian Christopherson, the British physician employed by the

全国范围内的血吸虫病防治。他还将在寄生虫学国际舞台上代表新中国。与许多专家一样，在中华人民共和国成立初期，毛泽东相对不受党的干预。然而，此后，在1957年，随着反右运动的到来，他在专业和政治因素的双重影响下加入了中国共产党。⁶¹

绕行科学

在成功地将年轻学生和老牌专家纳入体制后，专家们在采取何种方法上存在分歧，这给共产党当局带来了严峻的挑战。管理科学就像管理人力资源一样困难，甚至更加困难。血吸虫病防治工作中的主要障碍之一是诊断结果的不可靠。在世界范围内，检测虫卵是血吸虫病诊断的传统方法。在两位日本寄生虫学家加藤和（Kan Kato）和三浦桃重（Momoshige Miura）于1954年向世界推出玻璃纸厚涂片技术之前，用于诊断血吸虫病的标准方法是直接涂片法。这种方法既复杂又昂贵，而且可能非常不准确，因为它需要极少量的粪便样本，并且使用可回收的普通玻璃而不是一次性玻璃纸。加藤法简化了这些程序。人们认为它比以前的方法更准确，因为它需要更大的样本量。这项技术在1968年得到了改进，从那时起，它就被称为加藤-卡茨厚涂片法。它仍然是检测亚洲和非洲血吸虫病（日本血吸虫病和曼氏血吸虫病）最广泛使用的诊断技术。然而，由于个体患者粪便排卵水平的日常变化，以及其在排泄物中的不均匀分布，使用普通盖玻片的直接涂片法以及随后的加藤-卡茨法都缺乏高度的准确性和灵敏度。⁶²1950年，在中国，诊断结果的不可靠导致专家在治疗方法上存在分歧。苏德隆在八年抗日战争期间积累了治疗血吸虫病患者的经验，他赞成大规模治疗干预，并主张建立大规模治疗中心。这是约翰·布莱恩·克里斯托弗森（John Brian Christopherson）介绍的方法，他是英国医生，受雇于

colonial office during World War I. Its success at the time led to him to conclude that 'It is mass treatment in schools and villages where the hope of eradicating the disease lies.' It was this method that was implemented by the Egyptian public health department.⁶³ Zhang Changshao, China's leading pharmacologist and a graduate of the School of Pharmacy at the University of London, had been involved in treating malaria victims at a cotton mill in southern China with a traditional local fever remedy made of hydrangea (*Dichroa febrifuga*; in Chinese 常山 Changshan).⁶⁴ Zhang vociferously argued against mass therapy, reasoning that it was both wasteful and impractical. His view was that unlike tuberculosis, it was not always possible to determine whether a person had been infected with schistosomiasis and that treating every villager in the affected regions was an enormous waste of limited resources on potentially healthy individuals.⁶⁵

To accommodate the experts' disagreement would delay the treatment work, but to alienate them would jeopardize the party's goal of winning its war against the disease. In addition to the disagreement amongst the experts, the wide range of localities and conditions on the ground also meant that mass therapy was impractical. For instance, the simple lack of beds at local treatment centres (some were former missionary hospitals, others temporary centres converted from old temples) meant that most of them could only take a very limited number of patients. Furthermore, mass therapy needed enormous quantities of treatment drugs. At the time, *fouadin* or *stibophen*, first manufactured by Bayer in the late 1920s, had to be imported, but it was not an affordable commodity in the amounts required for the new post-war nation with very limited resources. An injection of sodium antimony tartrate (at half a grain per day for twenty-nine days) was the therapy of choice for mass treatment throughout much of the world until the 1980s.* In China at the time, the therapy with sodium antimony tartrate was reported to have produced fantastic results when used to treat sick PLA soldiers.⁶⁶ However, there were serious reservations about its wide use, since the drug is known to be highly toxic and can therefore be dangerous

* Praziquantel (PZQ) is now the drug of choice for schistosomiasis treatment throughout the world. The drug was first discovered in Germany in the 1970s, but it only became available to the international market in the 1980s through a public and private partnership of the WHO and Bayer. Before PZQ became commercially available in 1978, China began to introduce it for clinic use. Soon after that, China successfully obtained the technology of synthesizing PZQ.

第一次世界大战期间的殖民地办公室。当时它的成功使他得出结论,认为“根除这种疾病的希望在于学校和乡村的大规模治疗”。埃及公共卫生部门实施的就是这种方法。⁶³ 中国首席药理学家、伦敦大学药学院毕业生张昌绍,曾在华南一家棉纺厂用一种当地传统的由绣球花制成的退烧药(*Dichroa febrifuga*; 中文名常山)治疗疟疾患者。⁶⁴ 张强烈反对大规模治疗,认为这既浪费又不切实际。他的观点是,与肺结核不同,并不总是能够确定一个人是否感染了血吸虫病,并且治疗受影响地区的所有村民是对有限资源在潜在健康个体上的巨大浪费。⁶⁵

为了顾及专家们的意见分歧,治疗工作将会延误;但若疏远他们,则会危及该党赢得抗击疾病战争的目标。除了专家们之间的分歧外,各地范围广泛,地面条件各异,也意味着大规模治疗是不切实际的。例如,地方治疗中心(有些是以前的教会医院,另一些是从旧寺庙改建的临时中心)简单地缺少床位,意味着它们中的大多数只能接收非常有限的病人。此外,大规模治疗需要大量的治疗药物。当时,*fouadin* 或 *stibophen* (拜耳公司于 20 世纪 20 年代后期首次生产)必须进口,但对于这个资源非常有限的新战后国家来说,它并不是一种能够负担得起的大宗商品。酒石酸锑钠注射液(每天半格令,持续 29 天)是 20 世纪 80 年代之前世界大部分地区大规模治疗的首选疗法。* 当时在中国,据报道,酒石酸锑钠疗法在治疗生病的解放军战士时取得了惊人的效果。⁶⁶ 然而,人们对其广泛使用存在严重的保留意见,因为已知该药物具有剧毒,因此可能很危险。

* 吡喹酮 (PZQ) 现在是全世界治疗血吸虫病的首选药物。该药物于 20 世纪 70 年代在德国首次被发现,但直到 20 世纪 80 年代才通过世界卫生组织和拜耳公司的公私合作关系进入国际市场。在 PZQ 于 1978 年上市之前,中国就开始将其引入临床使用。此后不久,中国成功获得了合成 PZQ 的技术。

if applied without the supervision of trained staff. Since the complete course of treatment took twenty-nine days, patients needed to be hospitalized for that length of time. Not only was there a shortage of beds and bedding, but most villagers refused to leave their homes for that length of time for complex cultural, social, and economic reasons (as will be discussed in chapters 2 and 5). Furthermore, the drug is not effective for patients in the advanced stages of schistosomiasis. Building on their previous experiences of controlling kala-azar, a parasitic disease prevalent in the Communist-controlled regions during the war, some army doctors experimented with using neostibosan (sodium stibogluconate), a comparatively non-toxic drug used to treat this sand fly-borne parasitic disease (visceral leishmaniasis). Because of high demand, China had been manufacturing its own neostibosan, so it would make good economic sense if it could also be used to treat schistosomiasis. However, experts in the south, such as Chen Fangzhi, felt less comfortable with this newer version of the tartar emetic. Unsure about neostibosan's efficacy, they argued that it should only be used as a temporary measure, not as a long-term solution for schistosomiasis.⁶⁷

One month prior to the outbreak of schistosomiasis among the PLA soldiers, Chen had deemed that existing drug therapies were largely ineffective and that they were, in any case, too costly for a war-torn country. To control the disease more effectively, he advocated a more comprehensive rescue plan. Having devoted many years to epidemiological investigations in Zhejiang and the nearby regions, Chen argued strongly for a nationwide epidemiological survey. His reasoning was that more data would allow the government to allocate limited resources efficiently. He also favoured building a small number of flagship hospitals in cities and towns with more resources and easier access by modern transport (at the time, many villages were not accessible except by narrow, unpaved paths). If all resources were directed into these central hospitals, they would act as nodal points for research as well as being responsible for treatment and health education plus providing prevention training on the ground. This rescue plan also emphasized the importance of implementing grassroots prevention programs. Such programs ranged from sanitizing villages to vector control and introducing 'hygiene-disciplines' to the impacted population. For Chen, the integral part of public health education was the literacy campaign, so those 'retarded and ignorant' villagers, as he called them, would

如果在没有受过训练的工作人员监督下使用。由于完整的疗程需要二十九天，患者需要住院这么长时间。不仅床位和床上用品短缺，而且大多数村民由于复杂的文化、社会和经济原因（将在第2章和第5章中讨论）而拒绝离开家这么长时间。此外，该药物对血吸虫病晚期患者无效。一些军队医生借鉴了他们之前控制黑热病的经验，黑热病是一种在战争期间共产党控制的地区流行的寄生虫病，他们试验使用新斯锑波散（葡萄糖酸锑钠），这是一种相对无毒的药物，用于治疗这种沙蝇传播的寄生虫病（内脏利什曼病）。由于需求量大，中国一直在生产自己的新斯锑波散，因此如果它也可以用于治疗血吸虫病，那将在经济上很有意义。然而，南方专家，如陈方志，对这种较新版本的吐酒石显得不太放心。由于不确定新斯锑波散的疗效，他们认为它只应作为一种临时措施，而不是治疗血吸虫病的长期解决方案。⁶⁷

在解放军士兵中爆发血吸虫病的一个月前，陈认为现有的药物治疗在很大程度上是无效的，而且无论如何，对于一个饱受战争蹂躏的国家来说，这些疗法过于昂贵。为了更有效地控制这种疾病，他主张制定一个更全面的救援计划。陈多年来致力于浙江及周边地区的流行病学调查，他强烈主张在全国范围内进行流行病学调查。他的理由是，更多的数据将使政府能够有效地分配有限的资源。他还赞成在资源更丰富、更容易通过现代交通工具到达的城镇建立少量旗舰医院（当时，许多村庄只能通过狭窄的、未铺砌的小路到达）。如果所有资源都投入到这些中心医院，它们将成为研究的节点，并负责治疗和健康教育，以及提供实地预防培训。这项救援计划还强调了实施基层预防计划的重要性。这些计划包括对村庄进行消毒、控制病媒以及向受影响的人群介绍“卫生纪律”。对于陈来说，公共卫生教育不可或缺的一部分是扫盲运动，这样，正如他所说的，那些“迟钝而无知”的村民们就会

be equipped to comprehend the new bio-medical knowledge.⁶⁸ The Financial and Economic Committee of the CCP's East China Bureau, responsible for managing the economic matters of eastern regions of China, favoured Chen's rescue plan, since it resonated well with the CCP's goal of introducing a cultural revolution to transform rural China. Faced with the reality of a shortage of funding, drugs, and medical equipment as well as trained personnel, the small-scale local program, together with public exhortations to change people's 'unclean' practices, was something, perhaps the only thing, they could really deliver. This was in no way unique to the PRC, however, since public health attempts aimed at changing behaviour were usually more cost-effective than other public health interventions.⁶⁹

Two months later, in April 1950, in consultation with leading experts, including Chen Fangzhi and Mao Shoubei, the newly formed PRC's Ministry of Health, an entity separate from but parallel to the Military Health Unit and which would oversee the nation's health work, issued a call for a nationwide schistosomiasis control program. While this program echoed Chen's rescue plan, there were also significant changes.⁷⁰ Instead of prioritizing epidemiological survey and research, the Ministry of Health's national schistosomiasis control program emphasized health education and the training of grassroots health workers, using them to spread health propaganda and introduce sanitation measures. This was not surprising. The PRC's new minister of health, Madame Li Dequan, was a former social activist and feminist who in the 1920s had sought refuge in the Soviet Union with her warlord husband Feng Yuxiang. Nicknamed the 'Christian General', Feng was an advocate for mass education, women's liberation, and the eradication of 'superstitions' and 'unclean' habits. He was also sympathetic to union activities and was said to have helped the CCP in developing its networks amongst students and workers. Li Dequan herself had been a strong advocate in promoting maternal and children's health. She also actively supported refugee children's relief work during the wartime, although she was not then directly involved in this or in any other aspects of public health work. The very existence of disease and ill health was a political question for her: they had resulted from 'unclean' habits that were directly linked to the 'feudal' way of life. Health education that promoted 'clean' habits, along with hygienic discipline, was therefore integral to disease prevention and the improvement of the population's health.⁷¹ Li was not alone in this. The belief that clean

能够理解新的生物医学知识。⁶⁸ 中共华东局的财经委员会负责管理中国东部地区的经济事务，他们赞成陈的救援计划，因为它与中共发动文化革命以改造中国农村的目标产生了共鸣。面对资金、药品和医疗设备以及训练有素的人员短缺的现实，小规模在当地项目，加上公开劝告人们改变“不洁”的习惯，也许是他们真正能够提供的唯一的東西。然而，这并非中国独有，因为旨在改变行为的公共卫生尝试通常比其他公共卫生干预措施更具成本效益。⁶⁹

两个月后，即1950年4月，在与陈方芝和毛守白等主要专家协商后，新成立的中华人民共和国卫生部（一个独立于军事卫生单位但与之平行的实体，负责监督国家卫生工作）呼吁在全国范围内开展血吸虫病防治计划。虽然该计划与陈的救援计划相呼应，但也存在重大变化。⁷⁰ 卫生部的全国血吸虫病防治计划没有优先考虑流行病学调查和研究，而是强调健康教育和基层卫生工作者的培训，利用他们来传播健康宣传和介绍卫生措施。这并不奇怪。中华人民共和国新任卫生部长李德全夫人是一位前社会活动家和女权主义者，她在20世纪20年代曾与军阀丈夫冯玉祥一起在苏联避难。冯玉祥被昵称为“基督将军”，是大众教育、妇女解放和消除“迷信”和“不洁”习惯的倡导者。他还同情工会活动，据说曾帮助中共发展其在学生和工人中的网络。李德全本人一直是促进妇幼健康的积极倡导者。她还在战时积极支持难民儿童的救济工作，尽管当时她没有直接参与这项工作或任何其他公共卫生工作。对她来说，疾病和不健康的存在是一个政治问题：它们是由与“封建”生活方式直接相关的“不洁”习惯造成的。因此，促进“清洁”习惯的健康教育，以及卫生纪律，是疾病预防和改善人口健康不可或缺的一部分。⁷¹ 李德全并非孤身一人。这种认为清洁

living conditions and healthy behaviours, as well as ordered lives, would produce healthy bodies and a politically enlightened, productive, and happy population was widely shared by contemporary revolutionaries and Marxist cultural theorists in China. For them, disease control was not merely about science but a core part of the utopian project, although science would have an important role to play in hastening the coming of the socialist utopia. In this utopian future, they envisioned a virile and productive population. In order to create the utopian body of the future, Madame Li and the new Ministry of Health worked alongside medical and hygiene professionals in Shanghai and other large urban areas to introduce measures to change habits of body and life.⁷² Schistosomiasis control thus presented a real opportunity to carry out their revolutionary experiments. It had to be framed into the PRC's overall public health agenda: to live a socialist life was to live a healthy life. Living such a healthy life would in turn bring forth the socialist utopia – or, to use the words of Lu Dingyi, the man in charge of the PRC's national political cultural work who would become the first minister of propaganda, schistosomiasis control work was a cultural revolution against 'old customs', 'superstitions', and 'unclean habits'.⁷³ Just as healthy behaviour was viewed as a signifier of politically correct behaviour, from its outset the PRC's crusade against schistosomiasis was a political project in which scientific expertise would play a vital but subordinate role.

As we have seen to this point, at the very moment that the PRC was established in 1949, there had been specific connections between health and politics as part of the foundational definition of the goals of the new state. The problem of schistosomiasis quickly became the poster child for public health in the new China because the disease was seen as deterring the progress of the revolution. This was true on both the level of medical professionals and the organizational level of the state. Pragmatically, the struggle against the disease reflected both internal divisions and Cold War politics but also the simple availability of current resources. Symbolically, it also reflected the function of such a public health undertaking as the fulcrum for the goals of a new, utopian society. The following chapters will show how early themes of disease intractability, the political desire for success, competing demands, and organizational and professional conflict play out in ways unsuspected by those involved at the outset of the building of a new communist society.

当时中国的革命家和马克思主义文化理论家普遍认为, 改善生活条件和培养健康行为, 以及有条理的生活, 将产生健康的身体和政治开明、生产力高且幸福的人民。对他们来说, 疾病控制不仅仅是科学问题, 而是乌托邦计划的核心组成部分, 尽管科学将在加速社会主义乌托邦的到来方面发挥重要作用。在这个乌托邦式的未来, 他们设想了一个充满活力和生产力的人群。为了创造未来的乌托邦身体, 李夫人和新的卫生部与上海和其他大型城市地区的医疗和卫生专业人员一起, 采取措施改变身体和生活习惯。⁷² 因此, 血吸虫病控制为开展革命实验提供了一个真正的机会。它必须被纳入中华人民共和国的整体公共卫生议程: 过社会主义生活就是过健康的生活。过上这样健康的生活反过来会带来社会主义乌托邦 —— 或者, 用主管中华人民共和国国家政治文化工作的陆定一的话来说, 他将成为第一任宣传部长, 血吸虫病控制工作是一场反对 “旧习俗”、“迷信” 和 “不洁习惯” 的文化革命。⁷³ 正如健康的行为被视为政治正确行为的象征一样, 从一开始, 中华人民共和国反对血吸虫病的运动就是一个政治项目, 其中科学专业知识将发挥重要但从属的作用。

正如我们目前所见, 早在 1949 年中华人民共和国成立之时, 健康与政治之间就存在着特定的联系, 将其作为新国家目标的基础定义的一部分。血吸虫病迅速成为新中国公共卫生的典型代表, 因为该疾病被视为阻碍革命的进程。这在医学专业人员和国家组织层面都是如此。务实地看, 与这种疾病作斗争既反映了内部的分歧和冷战政治, 也反映了当前资源的简单可用性。象征性地, 它也反映了这种公共卫生事业的功能, 即作为实现新的乌托邦社会目标的支点。接下来的章节将展示疾病的棘手性、对成功的政治渴望、相互竞争的需求以及组织和专业冲突等早期主题, 如何以那些参与建立新的共产主义社会之初的人们所未曾预料到的方式发挥作用。

An Unfolding Drama

TO TAKE OVER SCHISTOSOMIASIS CONTROL IS TO FIGHT 'FEUDAL SUPERSTITIONS'

On 30 May 1950, just a month after the Ministry of Health had issued the directive of a nationwide schistosomiasis control program, the Political Department of the People's Liberation Army (PLA)'s Twentieth Army announced that the mass treatment work amongst the infected PLA soldiers had been successfully completed. A large number of soldiers (13,294) were treated, had recovered, and had returned to their units, and the first battle was declared won.¹ Schistosomiasis control work soon shifted to civilian society. It would become the centrepiece for public health in the People's Republic of China (PRC).

For the new Communist state, schistosomiasis control provided a convenient model emphasizing the need for overall systematic state intervention. China's medical community and its many scientific-minded reformers also favoured state intervention. For them, the prevalence of schistosomiasis was but one example of the Nationalist government's failure to introduce a systematic disease control program. This view was also held by international public health advocates and parasitologists who sought to 'change' China through the benefit of concerted public health initiatives and a move to allopathic medicine.* In their 1924 landmark publication *Studies on Schistosomiasis Japonica*, Faust and Meleney drew the conclusion

* Modernizing elites in China usually also favoured allopathic medicine and advocated the need for making modern medicine widely available.

一场逐渐展开的戏剧

接管血吸虫病防治 是为了斗争“封建迷信”

1950年5月30日，在卫生部发布全国血吸虫病防治指示仅一个月后，中国人民解放军（PLA）第二十军政治部宣布，受感染的解放军士兵的群体治疗工作已成功完成。大量士兵（13,294人）接受了治疗、康复并返回了部队，并宣布赢得了第一场战斗。¹ 血吸虫病防治工作很快转移到民间社会。它将成为中华人民共和国（PRC）公共卫生的核心。

对于新的共产主义国家来说，血吸虫病防治提供了一个方便的模型，强调了全面系统的国家干预的必要性。中国的医学界及其许多具有科学头脑的改革者也赞成国家干预。对他们来说，血吸虫病的流行只是国民政府未能引入系统性疾病控制计划的一个例子。国际公共卫生倡导者和寄生虫学家也持有这种观点，他们试图通过协调一致的公共卫生倡议和转向对抗疗法来“改变”中国。^{*} 在他们1924年的里程碑式出版物《日本血吸虫病研究》中，Faust和Meleney得出了结论

* 中国的现代化精英通常也赞成对抗疗法，并主张广泛提供现代医学的必要性。

that schistosomiasis could not, under the present circumstances, be controlled in China, since 'neither the provincial nor the local authorities have any real interest in the welfare of the inhabitants' and 'it is not merely a public health problem; for within its scope are also included problems of economic and political life and general education.'² On another occasion, Meleney even went as far as to suggest it was the 'type of mind the Chinese have' that ruled out any possibility of controlling the disease.³ Therefore, the new Communist government's promise to improve the people's health by controlling the disease offered a startling contrast to the perceived reluctance of the previous regime. It rekindled a diminishing hope for 'reforming and improving' China and, if Meleney was correct, the Chinese themselves.⁴ With the Chinese Communist Party (CCP) and the state now backing them up, Chinese experts in parasitology (the majority of whom were at one point students of Faust and Meleney at the PUMC's Division of Parasitology), as well as social reformers, were determined to show the world that the Chinese were capable of rising to this public health challenge.

In its early stage, schistosomiasis control also gave the CCP legitimate reason to take over several existing medical and health institutions left by the Nationalists. After the establishment of the Shanghai Region Schistosomiasis Prevention and Treatment Committee, the Eastern China Military Committee gave the order to establish a parallel civilian schistosomiasis prevention committee in the former Nationalist capital Nanjing. Li Zhenxiang, a graduate of the Hunan-Yale Medical College, headed this new committee. Soon after the Japanese marched across the Yangtze, he joined the PLA to become an army doctor. Well trained and extremely efficient, Li successfully operated on thousands of wounded soldiers. He won himself the reputation of being the 'PLA's Hua Tuo'.^{*} From being an army surgeon, Li quickly rose to lead the PLA's public health work, working alongside Cui Yitian during the Chinese civil war in the 1940s. Other committee members, who were all trained in the West (a few were Faust and Meleney's students from the PUMC), were prominent figures of the Nanjing medical community during the Nationalist era. The committee in Nanjing was the first provincial

* Hua Tuo was a physician and surgeon who lived in the second and third centuries in the Han Empire, celebrated for his superior surgical skills and for using a 'herbal anaesthetic' made from hemp.

血吸虫病在中国目前的状况下无法得到控制, 因为 “省政府和地方政府对居民的福利没有任何真正的兴趣”, 而且 “这不仅仅是一个公共卫生问题; 它的范围还包括经济和政治生活以及普通教育等问题”。² 另一次, 梅莱尼甚至提出, 正是 “中国人的思维方式” 排除了控制这种疾病的任何可能性。³ 因此, 新的共产党政府承诺通过控制这种疾病来改善人民的健康, 这与之前政府被认为的不情愿形成了鲜明对比。它重新燃起了人们对 “改革和改善” 中国的希望, 如果梅莱尼是正确的, 也包括中国人自己。⁴ 有了中国共产党 (CCP) 和国家的支持, 中国的寄生虫学专家 (他们中的大多数人都曾在北京协和医学院的寄生虫学系师从福斯特和梅莱尼), 以及社会改革家, 决心向世界表明中国人有能力应对这一公共卫生挑战。

在早期阶段, 血吸虫病防治也为中国共产党接管国民党遗留下来的若干现有医疗卫生机构提供了合理的理由。在上海地区血吸虫病防治委员会成立后, 华东军区下令在前国民党首都南京成立一个平行的民用血吸虫病防治委员会。湖南-耶鲁医学院的毕业生李振祥担任这个新委员会的负责人。日军跨过长江后不久, 他就加入了人民解放军, 成为一名军医。李训练有素, 效率极高, 成功地成为成千上万的伤员做了手术。他因此赢得了 “解放军华佗” 的美誉。^{*} 从一名军医, 李迅速崛起, 领导解放军的公共卫生工作, 在 20 世纪 40 年代的中国内战期间与崔义田并肩工作。委员会的其他成员, 都曾在西方接受过培训 (其中一些是协和医学院福斯特和梅莱尼的学生), 是国民党时期南京医学界的知名人士。南京的委员会是第一个省级

* 华佗是汉朝二三世纪的一位医生和外科医生, 以其精湛的外科手术技巧和使用由大麻制成的 “草药麻醉剂” 而闻名。

authority other than the military to run a schistosomiasis control institution in the PRC. Its main function was not to carry out actual control work but to ensure the smooth integration of the existing Nationalist health and medical institutions into the new regime.⁵

In the next months, a huge number of local treatment centres and stations mushroomed in the endemic rural counties and towns around Nanjing and Shanghai, as well as in regions of Zhejiang province where the PLA's swimming training was taking place in preparation for invading Taiwan. Employees from the former Nationalist medical and health institutions were relocated to fill vacancies in these new centres. CCP military representatives took the leadership role at such centres to make sure they would not be simply replicas of former Nationalist institutions but would become socialist public health organizations for the new China. After carefully reviewing each employee's file, party officials at each of the centres would assign appropriate posts to them depending on their political reliability. Some of the posts were filled by recent graduates from the medical and nursing schools in the big cities. These newly minted physicians and nurses were among the most loyal supporters of the CCP and the new Communist state. The schistosomiasis control work imbued them with a specific sense of the inflection of the PRC's historical utopian mission: they were to use their bio-medical knowledge and skills to build a new and modern Chinese society that would lead to a world free of disease.⁶

Many new schistosomiasis treatment and control centres also served the function of co-opting existing local resources into the new state-run socialist health system. These centres held regular meetings for local private practitioners – some practised Chinese medicine – in the name of preparing them for the work of schistosomiasis control. Such meetings not only disseminated bio-medical knowledge but also doubled as political study meetings, spreading the message of the new state's commitment to an ideal future. Some of these practitioners welcomed the new authority's commitment to tackling this intrinsic public health problem. There were also those who had been unemployed or underemployed in the former private systems and who saw schistosomiasis control work as an opportunity for upward mobility. By taking part in this work, the latter (many of whom had been made redundant by their former employers) became state employees and thus earned a monthly salary. They also compensated for their lower-level skills by taking part in what

在中国大陆，除军队之外的权力机构负责管理血吸虫病防治机构。其主要职能不是开展实际的控制工作，而是确保将国民党原有的卫生和医疗机构顺利融入新政权。⁵

在接下来的几个月里，南京和上海周边的流行病农村县镇以及浙江省的解放军为入侵台湾而进行游泳训练的地区，大量的地方治疗中心和站点如雨后春笋般涌现。来自前国民党医疗卫生机构的员工被重新安置到这些新中心填补空缺。中共军队代表在这些中心担任领导职务，以确保它们不会仅仅是前国民党机构的翻版，而是成为新中国的社会主义公共卫生组织。在仔细审查每位员工的档案后，各中心的党委官员会根据他们的政治可靠性为他们分配适当的职位。一些职位由大城市医学院校的应届毕业生填补。这些新晋的医生和护士是中共和新共产主义国家最忠实的支持者之一。血吸虫病防治工作使他们对历史乌托邦使命的转变产生了一种特殊的认识：他们将利用他们的生物医学知识和技能来建设一个崭新的现代中国社会，从而引领世界摆脱疾病。⁶

许多新的血吸虫病治疗和控制中心也起到了将现有地方资源纳入新的国营社会主义卫生系统的作用。这些中心定期为当地私人医生举行会议——其中一些人 *практикует* 中医——名义上是为他们准备血吸虫病防治工作。这些会议不仅传播了生物医学知识，而且还兼作政治学习会议，传播了新国家致力于理想未来的信息。其中一些医生欢迎新当局致力于解决这一内在的公共卫生问题。也有一些人在以前的私人系统中失业或就业不足，他们认为血吸虫病防治工作是一个向上流动的机会。通过参与这项工作，后者（其中许多人被以前的雇主解雇）成为国家雇员，从而获得每月工资。他们还通过参与什么来弥补他们较低水平的技能

was labelled a vital project to create the new socialist state. Many were greatly motivated by having a sense of higher purpose, and they subsequently became local activists in public health work in the ensuing years.⁷ On the other hand, those who had come from a long tradition of family practice, who had been economically better off due to their professional skills and reputations, were being classified as 'landlords'. Their businesses were shut down, and their practices, along with their private possessions, were confiscated and redistributed as part of the Land Reform or forcibly converted to state ownership. Folk and spiritual practices, which had earlier played a central role in delivering health and well-being to rural villagers, were officially banned as 'feudal superstitions' and were eliminated from the new socialist health system. A few healers continued to practise in private, but in general, healing temples or shrines, including a number of those dedicated to Guandi or Lord Guan, and ancestral halls were converted to schistosomiasis treatment centres or local health stations.⁸

In Chinese popular belief, Lord Guan was the god of valour and loyalty. Throughout history, various social groups have assigned him different attributes. He was spontaneously the custodian of the empire, the protector of communities, and the provider of wealth and health for individuals. During the outbreak of bubonic plague in the late nineteenth century that ravaged southern China, the image of Lord Guan as a filial and righteous official in charge of the department of epidemics who intervened with Heaven on behalf of humans was deployed first by the Buddhist Society for the Performance of Good Deeds and then by the imperial authorities in order to mobilize the populace in efforts to control the crisis. In the process, unorthodox religious practices such as magical remedies, animal sacrifices, and the burning of paper-joss were disparaged. It was rumoured that in order to spare them misfortune, the voice of Lord Guan called the populace to repent of their misdeeds and to do good work. People were encouraged to attach an amulet with the ten characters of Lord Guan's name and title to their doors to ward off the infection. As important village institutions, Guandi temples would also provide free treatment as well as burial and crisis relief work in time of any major epidemics. In the Yangtze delta region, local popular beliefs often likened schistosomiasis to bad *fengshui* or misfortune. During the outbreak season, villagers regularly visited Guandi temples or other local village temples, praying to be spared the infection.⁹

被贴上标签，成为创建新的社会主义国家的重要项目。许多人因为拥有崇高的使命感而备受鼓舞，他们在随后的几年里成为了公共卫生工作的当地积极分子。⁷另一方面，那些拥有长期家庭行医传统、因其专业技能和声誉而在经济上较为富裕的人，被归类为“地主”。他们的生意被关闭，他们的诊所连同他们的私人财产一起被没收并重新分配，作为土地改革的一部分，或被强制转为国有。早些时候在为农村村民提供健康和福祉方面发挥了中心作用的民间和精神实践，被正式禁止，被认为是“封建迷信”，并从新的社会主义卫生系统中被清除。少数治疗师继续私下行医，但总的来说，包括一些供奉关帝或关老爷的寺庙和祠堂在内的治疗寺庙或神社，都被改造成血吸虫病治疗中心或当地卫生站。⁸

在中国民间信仰中，关公是勇猛和忠诚的象征。纵观历史，不同的社会群体赋予了他不同的属性。他自然而然地成为了帝国的守护者、社区的保护者，以及个人财富和健康的提供者。19 世纪末，在中国南方爆发鼠疫期间，关公的形象——一个孝顺和正直的官员，负责处理瘟疫事务，并代表人类向天堂求情——首先被佛教功德会所利用，然后被帝国当局利用，以动员民众控制危机。在此过程中，诸如魔法疗法、动物祭祀和焚烧纸钱等非正统宗教习俗受到了贬低。有传言说，为了避免不幸，关公的声音呼吁民众忏悔自己的罪行，多做好事。人们被鼓励将带有关公姓名和头衔的十个字的护身符贴在门上，以抵御感染。作为重要的乡村机构，关帝庙也会在任何重大疫情期间提供免费治疗以及埋葬和危机救济工作。在长江三角洲地区，当地的民间信仰经常将血吸虫病比作不好的风水或不幸。在爆发季节，村民们经常参观关帝庙或其他当地的村庙，祈祷能够免受感染。⁹

However, the new socialist China needed new cultural symbols. There was no place for the cult of Lord Guan or any of the other popular superstitions that had served 'feudal' ideologies. In the official culture for the new China, the war on disease was simultaneously a cultural war attacking 'feudal superstitions'. According to Article 41 of the Common Program of the Chinese People's Political Consultative Conference (CPPCC), to create a 'socialist and democratic culture' that was 'Chinese, scientific and popular' was 'to raise the cultural level of the people', to eradicate 'feudal, comprador and fascist ideology', as well as to 'develop the ideology of service to the people'.¹⁰ In the process of creating a new socialist democratic Chinese culture, schistosomiasis control became an arena in which to fight 'feudal superstitions' and at the same time to improve the people's health. Since the PRC's new socialist countryside would not be founded on ignorance and superstitions but rather on education and science, villagers were taught that the 'socialist bio-medical' interventions (socialist bio-medicine refers to Soviet medicine as opposed to the bio-medicine first introduced to China by Western 'imperialists') were the only means of relieving them of the sufferings caused by schistosomiasis, the 'disease of the feudal past'. They also learnt that Chairman Mao was more powerful than Lord Guan and any other healing gods.^{*11} To overcome the scepticism and distrust shown by villagers, some officials of the new Communist authorities promised free treatment as well as painless injections by the best doctors from the big city of Shanghai.¹² When villagers lined up outside local prevention and treatment centres demanding free treatment and expecting a miracle cure, many local authorities found they had no means of delivering the new regime's promise of health.¹³

* The CCP's cultural war on 'feudal superstitions' shared some similarities with the modernist campaign during the early Republican period attacking popular religion. Indeed, many of the voices that advocated the earlier campaign continued to serve as cultural theorists for the new Communist regime. However, the new Communist authorities were very conscious of marking its new cultural war as 'socialist', thus separating it from the earlier Nationalist campaign. For a study of the earlier Republican campaign, see Prasenjit Duara, 'Knowledge and Power in the Discourse of Modernity: The Campaigns against Popular Religion in Early Twentieth-Century China', *The Journal of Asian Studies* 50, no. 1 (1991): 67-83.

然而，新的社会主义中国需要新的文化符号。关老爷或其他流行的“封建”意识形态的迷信没有容身之地。在新中国的官方文化中，与疾病作斗争同时也是一场攻击“封建迷信”的文化战争。根据中国人民政治协商会议（cppcc）共同纲领第41条，创造“中国的、科学的、大众的”社会主义民主文化，就是要“提高人民的文化水平”，根除“封建的、买办的、法西斯意识形态”，以及“发展为人民服务的思想”。¹⁰在创造新的社会主义民主中国文化的过程中，血吸虫病防治成为了一个战场，既要打击“封建迷信”，又要改善人民的健康。由于中华人民共和国新的社会主义农村不会建立在无知和迷信的基础上，而是建立在教育和科学的基础上，村民们被告知，“社会主义生物医学”干预（社会主义生物医学指的是苏联医学，而不是西方“帝国主义者”首次引入中国的生物医学）是减轻血吸虫病带来的痛苦的唯一手段，血吸虫病是“封建过去的疾病”。他们还了解到毛主席比关老爷和其他任何治愈神灵都强大。^{*11}为了克服村民们表现出的怀疑和不信任，一些新的共产党政府官员承诺免费治疗，并由来自上海大城市的最好的医生进行无痛注射。¹²当村民们在当地防治中心外排队要求免费治疗并期待奇迹般的治愈时，许多地方当局发现他们没有办法兑现新政权对健康的承诺。¹³

* 中共对“封建迷信”的文化战争与民国初期攻击民间宗教的现代主义运动有一些相似之处。事实上，许多倡导早期运动的声音继续担任新共产主义政权的文化理论家。然而，新的共产主义当局非常清楚地将新的文化战争标记为“社会主义”，从而将其与早期的民族主义运动区分开来。关于早期共和运动的研究，参见 Prasenjit Duara, “现代性话语中的知识和权力：二十世纪初期中国反对民间宗教的运动”，*亚洲研究杂志* 50, no. 1 (1991): 67-83。

PREVENTION AND PUBLIC ENGAGEMENT
AS THE HEALTH BLUEPRINT

In the first two years following the Communist Liberation, shortages in funding, personnel, and goods were common problems faced by local authorities throughout the country. Even the large cities with more resources were not exempt. In newly liberated Nanjing and Shanghai, for instance, homeless refugees resulting from the revolution upheaval packed the streets, queuing for relief supplies. Food soon ran out. Malnutrition as well as unclean and crowded living conditions led to outbreaks of smallpox, tuberculosis, whooping cough, diphtheria, and measles. In the meantime, drought and floods wrecked large parts of the countryside. An influx of villagers had migrated to the cities for survival. Suffering severe malnutrition and exhaustion, many fell victim to the epidemics. This was exacerbated by the huge number of livestock that died of diseases that spread quickly.¹⁴ The economically defunct state had no resources to cope with the escalating public health devastation that accompanied the regime change. With only one trained health professional (including doctors, nurses, and paramedics) for every 1,000 people and a total of 2,600 hospitals, with only 80,000 hospital beds in all of China (the situation was much worse in rural China where there was only one hospital bed for every 20,000 people), the new regime's promise of free medical care for all was quickly suspended.¹⁵ It was replaced by the Labour Insurance Regulation in 1951 that guaranteed protection for a select few, namely workers in the larger state-owned factories as well as other government employees. At the time, China's population was 500 million, but only 12 million were industrial workers. The introduction of this Labour Insurance Regulation meant that unemployed urban residents, most of whom were poor, as well as rural villagers (the latter counted for more than 80 per cent of the total population) were left without health and medical care.¹⁶ Following the new insurance regulation, some regional authorities, such as the Nanjing municipal authority, introduced a three-tiered referral system that aimed to distribute the restricted resources more efficiently to the poor and the needy. In practice, however, instead of delivering resources to the poor and those most in need, the referral system encouraged often unfair competition for the limited resources. This meant that only those with good connections and political power had access to quality medical and health care, and so

预防和公众参与作为健康蓝图

在共产党解放后的头两年里，资金、人员和物资短缺是全国各地地方当局面临的常见问题。即使是资源较多的大城市也不能幸免。例如，在新解放的南京和上海，革命动荡造成的无家可归的难民挤满了街道，排队领取救济物资。食物很快就耗尽了。营养不良以及不洁和拥挤的生活条件导致了天花、肺结核、百日咳、白喉和麻疹的爆发。与此同时，干旱和洪水摧毁了农村的大部分地区。大量村民为了生存而迁移到城市。许多人因严重营养不良和疲惫而成为流行病的受害者。大量牲畜因疾病迅速蔓延而死亡，这加剧了这种情况。¹⁴ 经济上已经瘫痪的国家没有资源来应对伴随政权更迭而来的不断升级的公共卫生灾难。中国每 1000 人中只有一名训练有素的卫生专业人员（包括医生、护士和护理人员），总共有 2600 家医院，全国只有 8 万张病床（农村地区的这一情况更糟，每 2 万人才有一张病床），新政权承诺的为所有人提供免费医疗的承诺很快就被搁置了。¹⁵ 1951 年颁布的《劳动保险条例》取而代之，该条例保证为少数人提供保护，即大型国有工厂的工人以及其他政府雇员。当时，中国人口为 5 亿，但只有 1200 万产业工人。《劳动保险条例》的出台意味着失业的城市居民（其中大多数是穷人）以及农村村民（后者占总人口的 80% 以上）没有医疗保健。¹⁶ 在新保险条例颁布后，一些地区当局，如南京市当局，推出了一项三级转诊制度，旨在更有效地将有限的资源分配给穷人和有需要的人。然而，在实践中，转诊系统并没有将资源传递给穷人和最需要的人，反而鼓励了对有限资源的往往不公平的竞争。这意味着只有那些有良好关系和政治权力的人才能获得优质的医疗保健，因此

cronyism prevailed. The situation was worse in regions where there were fewer resources. Often, the most vulnerable not only had very little or zero access to medical care but were also denied the basic minimum for their health and well-being. In the name of helping them to become 'self-reliant', different districts in Nanjing municipality were assigned quotas to take in the disabled and vagrants. Having no resources, districts pushed the responsibility for looking after these people back on their families and communities. Homeless vagrants who had no family or community to return to were detained and forced to do hard labour at the detention centres. Later, these detention centres were configured into 're-education through labour' camps. Between 1950 and 1952, more than 10,510 homeless vagrants in Nanjing municipality, from orphans to petty criminals, including the disabled and elderly, were sent to such camps. More than 97 per cent of those incarcerated suffered from lung diseases, gastric diseases, and other infectious diseases. Having little or no access to medicine and health care, some 200 people died in these camps; 456 were lucky enough to escape.¹⁷

In the meantime, to the dismay of the authorities, the 'old feudal' healing practices that the new regime had tried hard to eradicate continued to attract many followers who either had no access to or did not trust a new health system that was based on science and rationality as well as favouritism. In rural Zhejiang, for instance, villagers were reported lining up under a 100-year-old ginkgo tree, waiting for the spirit to be revealed at night and deliver them the healing water that would protect them from ill health.¹⁸ In Sichuan's Luojiang County, historically a strategic crossroad linking the Chinese empire with the Himalayas, some 60,000 to 70,000 people of mixed cultural and ethnic backgrounds from the other side of Ngawa Prefecture on the Tibetan Plateau, as well as from Sichuan province, made annual pilgrimages here to pray to Guanyin (the Goddess of Mercy) for protection, wellness, and health. The pilgrims would bring with them a small empty container to draw the water from the river beneath the Guanyin rock, water that was said to possess healing power. Failing to understand the world view of the villagers who saw disease and ill health as a result of fate and divine retribution, the authorities viewed the claims of healing made by local cults as threatening to their newly gained political power. Presenting the pilgrims as seriously jeopardizing the necessary implementation of rural public health work, the local public

裙带关系盛行。在资源较少的地区，情况更糟。通常，最脆弱的人不仅几乎或根本无法获得医疗保健，而且还被剥夺了维持健康和福祉的基本最低限度的条件。为了帮助他们变得“自力更生”，南京市的不同地区被分配了接收残疾人和流浪者的名额。由于没有资源，各区将照顾这些人的责任推回给他们的家庭和社区。无家可归的流浪者，如果无家可归或没有社区可以返回，则会被拘留并被迫在拘留中心从事繁重的劳动。后来，这些拘留中心被改造成“劳动教养”营地。1950年至1952年间，南京市有一万零五百一十多名无家可归的流浪者，从孤儿到小罪犯，包括残疾人和老年人，被送往此类营地。超过97%的被监禁者患有肺部疾病、胃病和其他传染病。由于很少或根本无法获得药物和医疗保健，约有200人在这些营地中死亡；456人很幸运地逃脱了¹⁷。

与此同时，令当局沮丧的是，新政权努力取缔的“旧封建”治疗方法继续吸引了许多信徒，他们要么无法获得新的医疗系统，要么不信任这种基于科学和理性以及裙带关系的新医疗系统。例如，在浙江农村，据报道，村民们排队站在一棵百年银杏树下，等待夜晚显灵，为他们送来能保护他们免受疾病侵害的治疗之水。¹⁸在四川省罗江县，历史上这里是连接中华帝国和喜马拉雅山的战略要道，来自阿坝州藏族高原另一侧以及四川省的约6万至7万不同文化和民族背景的人们每年都会到这里朝圣，祈求观音菩萨（观世音菩萨）保佑他们平安、健康。朝圣者会带着一个小空容器，从观音石下的河中取水，据说这些水具有治疗功效。由于不了解村民们将疾病和不健康视为命运和神罚的结果的世界观，当局认为当地邪教的治疗主张对其新获得的政治权力构成威胁。当地政府将朝圣者描述为严重危害了农村公共卫生工作的必要实施。

security (police) blocked the road to the rock. In the meantime, local health officials tried to persuade these villagers to seek treatment for their ailments at local state-run treatment centres. Despite the repeated efforts of the authorities, however, more than 10,000 people continued to make pilgrimages to seek Guanyin's blessing and to draw the healing water.¹⁹ Throughout the countryside, various local sects also navigated their way under socialism to find new social meanings for their continuing existence. In the agricultural heartland of Henan province, a certain local sect that practised healing with 'divine water' appropriated the new regime's propagandist messages and methods for its own use. In the Land Reform, they maintained that 'the heavenly god sent the divine water to save the poor. But if landlords drunk the water, they would die.' A few months later in 1951, the context of Anti-Germ Warfare Campaign and the Patriotic Health Campaign during the Korean War allowed the sect to make the claim that the 'divine water' would kill germs and prevent epidemics.²⁰

Anxious that diseases were as much a destabilizing factor to its rule as 'the drought, flood, and locust' (the three natural phenomena traditionally seen as signs that predicted dynastic changes), the Central People's Government's State Administration Council (this would be replaced by the State Council in 1954) worked with its subsidiary, the Ministry of Health, as well as the PLA's Health Unit, to find solutions for the country's health crisis. The new state could barely provide the promised medicines and health care in major urban centres, let alone the impoverished countryside where 80 per cent of the population still lived. Prophylaxis, or preventive medicine, appeared the only affordable option.²¹ This matter was discussed at the first National Health Conference in August 1950. One month later, the PRC's new minister of health, Madame Li Dequan, reported back to the State Administration Council. Her report highlighted the fact that 'feudal superstitions' continued to have a stranglehold on rural villages. According to her, these superstitions were a major obstacle to disseminating bio-medical knowledge and to delivering socialist medicine and health care in the Chinese countryside. Yearning for China to overcome its historical backwardness through science and mass education, Madame Li believed, as did many of her contemporary modernizers, that the entire Chinese nation must be taught the difference between superstition and modern science and that it was necessary for the state to introduce the benefits of

保安（警察）封锁了通往岩石的道路。与此同时，当地卫生官员试图说服这些村民到当地国营治疗中心寻求疾病治疗。然而，尽管当局一再努力，仍有超过 10,000 人继续朝圣，祈求观音的祝福并汲取治愈之水。¹⁹ 在全国各地，各个地方教派也在社会主义制度下找到了新的社会意义，以维持其持续存在。在河南省的农业中心地带，某个用“神水”治病的地方教派挪用了新政权的宣传信息和方法供自己使用。在土地改革中，他们坚称“天神派神水来拯救穷人。但如果地主喝了水，他们就会死。”几个月后的 1951 年，在朝鲜战争期间，反细菌战运动和爱国卫生运动的背景下，该教派声称“神水”可以杀死细菌并预防流行病。²⁰

由于担心疾病像“旱灾、水灾和蝗虫”（传统上被视为预示王朝更迭的三种自然现象）一样，会成为动摇其统治的因素，中央人民政府政务院（1954 年由国务院取代）与其下属的卫生部以及解放军卫生部门合作，为国家的健康危机寻找解决方案。新成立的国家几乎无法在主要城市中心提供承诺的药品和医疗保健，更不用说仍然居住着 80% 人口的贫困农村地区了。预防，或预防医学，似乎是唯一负担得起的选择。²¹ 这个问题在 1950 年 8 月的第一届全国卫生会议上进行了讨论。一个月后，中华人民共和国新任卫生部长李德全部长向政务院汇报了情况。她的报告强调，“封建迷信”继续扼杀着农村。她认为，这些迷信是传播生物医学知识、在乡村提供社会主义医疗保健的主要障碍。李德全部长渴望中国通过科学和大众教育克服其历史性的落后，她和她同时代的许多现代化人士一样，认为必须教导全体中国人民区分迷信和现代科学，并且国家有必要引进带来的好处

bio-medical science in order to prevent the occurrence of disease and thus improve the people's health. She argued forcefully: 'Medicine is a scientific battle between the human and the disease ... Relying solely on health professionals to win such a battle would only cause alienation between them and the people ... Health professionals mustn't wait for people to get sick and turn up asking for treatment. Our job is to teach people how to fight diseases.'²² Preventive medicine became the blueprint of a new national program to improve the people's health. Since the socialist state considered the people one of its most valuable assets – perhaps the only tangible asset in the case of the PRC in the 1950s – Madame Li and her supporters called for a public engagement in the field of disease prevention. A few months later, at the second National Health Conference in 1952, it was officially announced that socialist medicine was to serve the people and also involve the people. Public health work would become the work of the masses.²³

In the PRC's socialist war against disease, the most celebrated showcase was schistosomiasis control. As discussed earlier, the crusade against schistosomiasis was the epitome of the new Communist state's political commitment to transform the backward (read: diseased) countryside through mass participation in disease prevention and science. Although it is classic communist praxis to move the focus from the individual to the collective, the PRC's view of bringing about advancement or modern transformation through community participation in the struggle to improve the people's health was, however, not new to the Chinese experience. For the first half of the twentieth century, Christian medical missionaries had been at the forefront of engaging local communities to 'improve' people's lives through changing attitudes and behaviours. Improving the health of 'the Sick Man of Asia' was central to the goal of reform of Western medical missionaries and the early twentieth-century political modernizers in China alike. It was backed by the financial commitment of the Rockefeller Foundation's International Health Board (IHB). The China Medical Board (CMB) was the second and perhaps largest-ever program of the IHB. The program included building a world-class medical school and hospital and facilitating the introduction of bio-medicine into post-Imperial China, Asia's first modern republic. In a CMB publication celebrating its 100th anniversary, the reader is informed that 'the knowledge generated through Peking Union Medical College (PUMC) helped usher in a

生物医学科学，以预防疾病的发生，从而改善人民的健康。她有力地辩称：“医学是人类与疾病之间的一场科学斗争……仅仅依靠卫生专业人员赢得这场斗争只会导致他们与人民之间的疏远……卫生专业人员不能等着人们生病后才来寻求治疗。我们的工作是要教人们如何与疾病作斗争。”²² 预防医学成为一项改善人民健康的新国家计划的蓝图。由于社会主义国家将人民视为其最宝贵的资产之一——在 20 世纪 50 年代的中国，这可能是唯一的有形资产——李夫人和她的支持者呼吁公众参与疾病预防领域。几个月后，在 1952 年第二次全国卫生会议上，正式宣布社会主义医学要为人民服务，也要让人民参与。公共卫生工作将成为群众的工作。²³

在中国（prc）的社会主义疾病防治运动中，血吸虫病防治是最受赞扬的典范。正如前面所讨论的，这场对血吸虫病的讨伐体现了新共产主义国家通过群众参与疾病预防和科学来改造落后（读作：患病）农村的政治承诺。虽然将重点从个人转移到集体是经典的共产主义实践，但是中华人民共和国通过社区参与改善人民健康的斗争来实现进步或现代转型的观点，对中国经验来说并非新鲜事。在二十世纪上半叶，基督教医疗传教士一直走在前列，通过改变态度和行为来“改善”当地社区人民的生活。“改善亚洲病夫”的健康状况是西方医疗传教士和二十世纪初中国政治改革者的共同目标。它得到了洛克菲勒基金会国际健康委员会（ihb）的财政支持。中华医学基金会（cmb）是国际健康委员会的第二个，也许也是有史以来最大的项目。该项目包括建立世界一流的医学院和医院，并促进生物医学引入后帝国时代的中国，亚洲第一个现代共和国。在中华医学基金会庆祝其成立 100 周年的出版物中，读者得知“通过北京协和医学院（pumc）产生的知识帮助开启了一个新的时代”，

revolution in the understanding and practice of medicine in China, which catalysed dramatic health progress over the course of a century ... It is an unprecedented human achievement.²⁴

Claude Barlow was a former Baptist medical missionary who first went to China in 1908. Famously known as a ‘martyr to medicine’, Barlow identified an internal parasite *fasciolopsis buski*, which claimed countless victims in Zhejiang province’s Shaoxing County in eastern China. With a fellowship from the CMB, Barlow conducted field research on this human intestinal fluke. Toward the end of his study, in 1922, eager to save the suffering Chinese, he swallowed thirty-two flukes taken from the body of his Chinese patient so that he could bring them back to the United States for laboratory study. His act literally bore fruit. His subsequent study on the life cycle of *fasciolopsis buski* in humans established him as the world’s leading authority on the topic. The study demonstrated that the main contributing factors for the prevalence of the disease amongst the local population in rural Zhejiang province was their practice of using night-soil for fertilizer, as well as their habit of eating land snails raw, just ‘as we do oysters’. From this finding, he concluded that the disease was easily controllable. One preventive measure was to ‘disinfect the night-soil in the jars and ripening tanks’. ‘Addition of lime will accomplish this,’ he assured. Another was killing the immediate snail hosts. As a medical missionary, Barlow was also a firm believer in community engagement through health education: ‘All of these [preventive] measures require the voluntary support of the community and to gain this there needs to be a widespread propaganda of enlightenment.’²⁵ Although he did not have the privilege of trying out his plan for action in his beloved China, it did make him an ideal person to lead the IHB’s sanitary campaign in Egypt to eradicate hookworm and schistosomiasis between 1929 and 1940.²⁶

Barlow’s initial finding in China would lead to ever more sophisticated public health interventions in the country. Undoubtedly, the increasing popularity of the vector control approach following the discovery of Paris Green in 1921 (see chapter 1) had an impact on Barlow’s belief in the efficacy of killing snails in controlling *fasciolopsis buski* in humans as well as in controlling the snail vectors for schistosomiasis, although a large-scale public health intervention by controlling the vector was only made possible in China with the advent of the Communist Liberation after 1949 (as will be discussed in chapter 3).

中国在理解 and 实践医学方面的革命，在一个世纪的过程中促进了显著的健康进步……这是一项前所未有的人类成就。’²⁴

克劳德·巴洛曾是一位浸信会医疗传教士，于1908年首次来到中国。巴洛因被誉为“医学殉道者”而闻名，他发现了一种名为姜片虫的体内寄生虫，该寄生虫在浙江省绍兴县造成了无数受害者。在中华医学基金会的资助下，巴洛对这种人体肠道吸虫进行了实地研究。在1922年研究即将结束时，为了拯救受苦受难的中国人，他吞下了从中国病人身上取出的32条姜片虫，以便将其带回美国进行实验室研究。他的行为确实结出了果实。他随后对人类姜片虫生活周期的研究使他成为该领域的世界权威。该研究表明，浙江省农村地区当地居民中该疾病流行的主要原因是他们使用粪便作为肥料的习惯，以及他们生吃陆地蜗牛的习惯，就像“我们吃牡蛎一样”。从这一发现中，他得出结论，这种疾病很容易控制。一种预防措施是“对罐子和成熟池中的粪便进行消毒”。他保证说，“添加石灰就可以做到这一点”。另一种方法是杀死直接的蜗牛宿主。作为一名医疗传教士，巴洛也坚信通过健康教育进行社区参与：“所有这些[预防]措施都需要社区的自愿支持，为了获得这种支持，需要广泛宣传启蒙运动。”²⁵ 尽管他没有机会在他热爱的中国尝试他的行动计划，但这确实使他成为领导国际卫生局1929年至1940年在埃及开展的根除钩虫病和血吸虫病卫生运动的理想人选。²⁶

巴洛在中国的最初发现将导致该国出现越来越复杂的公共卫生干预措施。毫无疑问，继1921年发现巴黎绿后，媒介控制方法越来越受欢迎（见第1章），这影响了巴洛对杀死钉螺来控制人体的布氏姜片吸虫病以及控制血吸虫病钉螺媒介的有效性的信念，尽管通过控制媒介进行的大规模公共卫生干预只有在1949年共产党解放后才成为可能（将在第3章中讨论）。

For the first half of the twentieth century, the control of contaminated faeces, the elimination of reservoir hosts, and access to clean water supplies were the most common interventions for schistosomiasis and hookworm prevention. The IHB Egyptian campaign aimed at controlling schistosomiasis began with sanitizing targeted Egyptian villages by building latrines, but it ended with killing snails. The former proved to have no measurable effect on morbidity or any impact on the parasite; the latter, however, was too labour- and time-intensive. In the end, the IHB concluded that ridding the communities of schistosomiasis by clearing snails in Egyptian canals was too lengthy an undertaking, and, losing patience, they passed the responsibility to the Egyptian government. Barlow stayed in Egypt for another three years on a contract with the Egyptian government to head an independent Bilharzia Destruction Section. Yet by 1949, the Egyptians had lost any hope of eradicating bilharzia, 'the most important disease of Egypt'. A year later, Barlow returned to the United States.²⁷ Back home, he reminisced about his earlier time in disease eradication in China: 'it was futile, since it saved lives and gave relief, but it did nothing about the basic control of disease. At the age of 54 with a wife and four daughters I gave it all up, went back to college, got my doctorate in science and have been working ever since on Preventive Medicine.'²⁸

At this exact moment, the Communist leadership in China waged their 'socialist' crusade against the 'oriental' schistosomiasis. Having drawn the conclusion that mass therapy was simply not affordable, following the advice of experts they turned to prevention through mass participation. This was not actually a surprising turn. As mentioned earlier, the majority of Chinese experts in public health were trained in the PUMC or other Rockefeller-funded institutions in the West, such as the Johns Hopkins School of Hygiene and Public Health. Many of them had received their modern education as children in missionary schools. Like Barlow, they, too, advocated engaging the public through a widespread 'propaganda of enlightenment'. The strategies employed for the new China's 'socialist' campaign resembled, in turn, those of the IHB's earlier sanitary campaign in Egypt, but this time around the CCP was determined that the outcome of its 'socialist' campaign would be different. It would show the world that what the Western 'imperialists' had failed to achieve in Egypt would succeed in the new China under its direction.

在二十世纪上半叶, 控制受污染的粪便、清除宿主以及获得清洁的供水是血吸虫病和钩虫病预防最常见的干预措施。ihb 的埃及运动旨在通过建造厕所来净化目标埃及村庄, 从而控制血吸虫病, 但最终因杀死蜗牛而告终。前者被证明对发病率没有可衡量的影响, 对寄生虫也没有任何影响; 然而, 后者太费时费力了。最终, i
hb 得出结论, 通过清除埃及运河中的蜗牛来清除社区的血吸虫病是一项太长的任务, 并且失去了耐心, 他们将责任移交给了埃及政府。巴洛与埃及政府签订合同, 留在埃及三年, 负责独立的血吸虫破坏科。然而, 到 1949 年, 埃及人已经失去了根除血吸虫病 (

“埃及最重要的疾病”) 的希望。一年后, 巴洛返回美国。²⁷ 回到家, 他回忆起他早些时候在中国根除疾病的经历: “这是徒劳的, 因为它挽救了生命并提供了缓解, 但它对疾病的基本控制没有任何作用。54 岁时, 我有了妻子和四个女儿, 我放弃了一切, 回到大学, 获得了科学博士学位, 并且从那时起一直从事预防医学工作。²⁸

就在此时, 中国共产党领导人发起了针对 “东方” 血吸虫病的 “社会主义” 运动。他们得出结论, 大规模治疗根本负担不起, 因此听取了专家的建议, 转而通过群众参与来进行预防。这实际上并不令人惊讶。正如前面提到的, 大多数中国公共卫生专家都曾在协和医学院或其他由洛克菲勒基金会资助的西方机构接受过培训, 例如约翰·霍普金斯大学公共卫生学院。他们中的许多人在传教士学校接受了现代教育。和巴洛一样, 他们也主张通过广泛的 “启蒙宣传” 来动员公众。新中国 “社会主义” 运动所采用的策略反过来又类似于国际卫生局早期在埃及开展的卫生运动, 但这一次, 中国共产党决心其 “社会主义” 运动的结果会有所不同。它将向世界展示, 西方 “帝国主义者” 在埃及未能实现的目标, 将在新中国的领导下获得成功。

In the received historiography, the CCP has been credited for its success in mobilizing the Chinese peasantry during the Chinese revolution. Such a claim also resonates in the official publications about the PRC's public health campaigns during the Mao era. Amongst these campaigns, the mass campaign against schistosomiasis was a shining example. Internationally, the campaign was also received positively at the time. In the Long 1970s, too, vast numbers of international medical and public health experts agreed that the PRC was the most successful example globally in terms of combatting the disease. The CCP's purported heroic struggle against schistosomiasis through mass participation was widely cited as evidence of the PRC's superior health system by those in favour of a horizontal approach to health care delivery. In contrast, the Egyptians' long and failed battle against the disease was used to critique a colonial approach to public health that neglected the participation of the community.²⁹ However, evidence I have unearthed from archives and oral interviews shows that as soon as the PRC campaign was set in motion, the authorities quickly learned that they really had little control over it. People responded to the campaign in vastly different ways, ranging from enthusiastic participation and passive conformity, to supplication, manipulation, and stealthy resistance, or even to active opposition. Confronted with unforeseen contingencies as well as political uncertainties, the authorities constantly revised their strategies. This added further obstacles to implementation. Contrary to the aim of bringing 'cultural revolution' to modernize the countryside, thus raising the cultural level of the 'ignorant' peasantry by involving the masses in the campaign to fight the disease through 'ruralizing' medicine and science, the day-to-day reality was very different and much more varied. To an extent, the more nuanced process of 'acculturation' allowed parallel systems, including multitudes of folk practices, to exist alongside the state system, hence paving the way for the decentralization of health care delivery in the late 1960s and early 1970s. While the commonplace idea that increased centralization was a hallmark of the PRC's health care system during the Mao era – at least in the propaganda from Beijing and the international reception of such views – the reality on the ground remained a complex mix of local, regional, and national solutions to the ongoing public health dilemmas of the new state, some of which were more effective than others.

在现有的史学研究中，中共因其在中国革命期间成功动员中国农民而备受赞誉。这种说法也与有关毛泽东时代中华人民共和国公共卫生运动的官方出版物相呼应。在这些运动中，大规模的血吸虫病防治运动就是一个光辉的例子。在国际上，这场运动当时也受到了积极的评价。在 20 世纪 70 年代初，大量的国际医疗和公共卫生专家也一致认为，中华人民共和国在防治该疾病方面是全球最成功的典范。中共通过群众参与与血吸虫病作斗争的英勇事迹被广泛引用，作为那些赞成采取水平方法提供医疗保健的人所认为的中华人民共和国优越的卫生系统的证据。相比之下，埃及人长期而失败的与该疾病作斗争的经历被用来批评一种忽视社区参与的殖民主义公共卫生方法。²⁹ 然而，我从档案馆和口头采访中发现的证据表明，中华人民共和国的运动一开始，当局很快就意识到他们对这场运动几乎没有控制权。人们对这场运动的反应方式千差万别，从热情参与和被动顺从，到恳求、操纵和隐秘的抵抗，甚至积极的反对。面对不可预见的突发事件以及政治上的不确定性，当局不断修改他们的策略。这给实施带来了更多的障碍。与通过“农村化”医药和科学，通过让群众参与与疾病作斗争的运动中来，将“文化革命”带到农村，从而提高“愚昧”农民的文化水平的目标相反，每天现实非常不同，而且更加多样化。在某种程度上，更加微妙的“文化适应”过程允许包括大量民间习俗在内的并行系统与国家系统并存，从而为 20 世纪 60 年代末和 70 年代初的医疗保健服务分散化铺平了道路。尽管普遍认为，至少在北京的宣传和国际上对这些观点的接受中，加强中央集权是毛泽东时代中华人民共和国医疗保健体系的一个标志，但实际情况仍然是地方、区域和国家对新国家持续存在的公共卫生困境的复杂混合体，其中一些比其他一些更有效。

SCHISTOSOMIASIS CONTROL CADRES

To engage and educate the masses demanded many more frontline grassroots workers. In 1951, more than 400 young people were recruited for the twenty-two local schistosomiasis control institutions in the three provinces of Zhejiang, Jiangsu, and Anhui where the disease was endemic. In the meantime, thousands of medical and nursing students from surrounding large cities were mobilized to go to the countryside during their school vacations to assist local schistosomiasis prevention and treatment work, as well as helping to conduct epidemiological surveys and gathering data.³⁰ Some stayed on to lead local disease prevention work in the following years. These new recruits were officially named ‘schistosomiasis control cadres’ (血防干部). This was part of the nationwide drive to train ‘personnel for national construction work, the eradicating of feudal, comprador and fascist ideology and the developing of the ideology of service to the people’, as stated in the Common Program.³¹ In 1950, the Ministry of Health and the Ministry of Education jointly issued an edict to train health cadres and health personnel at various levels for the ‘urgent need’ of building a health system for the new China.³² In some areas, however, recruitment of schistosomiasis cadres did not go as smoothly as hoped. The Zhejiang provincial authority had planned to build up to 200 grassroots schistosomiasis prevention and treatment teams. This would have required a large number of trained personnel, including 200 doctors, 200 paramedics, 400 nurses, and 200 lab technicians. Failing to meet the recruitment target, the authority turned to local private practitioners who had lost their livelihood as a result of the revolutionary upheaval and to young people just out of school who were drifting about with no prospect of employment. The latter were recruited according to their class background. It was believed that consciousness or ‘socialist attitudes’ depended on class (family) background. Hence, those from poorer peasant and working-class families had a greater chance of being accepted into the new system. The irony was that the greater the level of education (and the higher the economic status), the less likelihood there was of being employed in this new undertaking. The selected youngsters were enrolled in short crash courses on schistosomiasis prevention, without any general training in public health goals.³³ In these crash courses, they learned how to give injections, take blood pressure

血吸虫病防治干部

为了动员和教育群众，需要更多的一线基层工作人员。1951年，浙江、江苏、安徽三省的22个地方血吸虫病防治机构招募了400多名年轻人，这些省份都是血吸虫病流行区。与此同时，来自周边大城市的数千名医科和护理专业的学生在学校假期被动员到农村，协助当地的血吸虫病防治工作，并帮助进行流行病学调查和数据收集。³⁰一些人留下来，在随后的几年里领导当地的疾病预防工作。这些新招募的人员被正式命名为“血防干部”（血防干部）。这是全国范围内培训“为国家建设工作服务的人员，消除封建、买办和法西斯意识形态，以及发展为人民服务的意识形态”的一部分，正如《共同纲领》中所述。³¹1950年，卫生部和教育部联合发布了一项法令，为新中国建立卫生体系的“迫切需要”培训各级卫生干部和卫生人员。³²然而，在一些地区，血防干部的招募工作并不像预期的那样顺利。浙江省政府计划建立多达200个基层血吸虫病防治队。这将需要大量的训练有素的人员，包括200名医生、200名护理人员、400名护士和200名实验室技术员。由于未能达到招聘目标，政府转向了因革命动荡而失去生计的当地私人医生，以及刚刚离开学校，没有就业前景的年轻人。后者是根据他们的阶级背景招募的。人们认为，意识或“社会主义态度”取决于阶级（家庭）背景。因此，来自贫困农民和工人阶级家庭的人更有可能被新系统接受。具有讽刺意味的是，教育程度越高（以及经济地位越高），在这种新事业中获得雇用的可能性就越小。被选中的年轻人参加了血吸虫病预防的短期速成班，而没有接受任何公共卫生目标的综合培训。³³在这些速成班中，他们学会了如何注射、量血压

and temperature, and collect faecal samples. Ren Jixian was only eighteen years old. He had already joined a mass organization, the Communist Youth League. After two months of training, he was seconded to villages to lead schistosomiasis control work on the ground. It did not matter that he had little knowledge of treatment and prevention. The key was that he had the 'correct socialist attitude': he did not mind that there was no bed to sleep on at night, nor did he complain that his salary was only a meagre 30 *yuan* (less than \$10) per month. He was motivated by the idea of taking part in a greater historical mission that was for the good of his country, and he took pride in his ability to convey the message of the party concerning health and civic improvement to rural peasants. On the other hand, schistosomiasis control work, as part of the CCP's project of socialist transformation, offered young activists such as Jixian an opportunity for upward mobility. Three years on, the local authority awarded Jixian the position of deputy director of the Pinghu County epidemic prevention centre. He was to lead the county's schistosomiasis control work for the next three decades. Pinghu County in Zhejiang province was one of the places where Faust and Meleney conducted their field research back in the 1920s. In 1952, when Jixian first arrived at Pinghu, it was said that snails were so plentiful that one could easily grab a handful. As a model worker, Jixian was good with people as well as in organizing activities. He went out of his way to persuade villagers to hand over night-soil for disinfection, not to wash their night stool bucket in the canal, and to join him in burying snails. He tirelessly told stories of how schistosomiasis had claimed countless lives in the past and how in the new China, under the leadership of the CCP, victims of the disease were being restored to health.³⁴ After nearly thirty years of battling schistosomiasis, in 1978 the *People's Daily* declared that Pinghu County had succeeded in eradicating the disease. In 2014, at the advanced age of eighty-one, Jixian went to the local party archive to hand over his papers and his photos of local schistosomiasis control over the previous sixty years. When asked why, he answered that he wanted the history of schistosomiasis control in Pinghu to be remembered by future generations.³⁵ At Pinghu, the success story of schistosomiasis control did not arise automatically from the central policy, nor did it rely solely on the political commitment of the CCP. If China's schistosomiasis control

和测量体温,并收集粪便样本。任纪贤当时只有十八岁。他已经加入了一个群众组织——共产主义青年团。经过两个月的培训后,他被调到各个村庄,领导当地的血吸虫病防治工作。他对治疗和预防知之甚少,这并不重要。关键是他有“正确的社会主义态度”:他不介意晚上没有床睡觉,也不抱怨他的工资只有微薄的30元

(不到10美元)一个月。他的动力来自于参与一项为国家利益服务的更伟大的历史使命,他以自己能够向农村农民传达党关于健康和公民进步的信息而自豪。另一方面,血吸虫病防治工作,作为中国共产党社会主义改造项目的一部分,为像纪贤这样的年轻积极分子提供了向上流动的机会。三年后,当地政府授予纪贤平湖县防疫站副站长的职位。在接下来的三十年里,他将领导该县的血吸虫病防治工作。浙江省平湖县是福士特和梅勒尼在20世纪20年代进行实地调查的地方之一。1952年,当纪贤第一次来到平湖时,据说那里的钉螺非常多,人们可以很容易地抓到一把。作为一名模范工作者,纪贤不仅善于与人打交道,而且善于组织活动。他竭尽全力说服村民交出粪便进行消毒,不要在运河里洗马桶,并和他一起埋葬钉螺。他不厌其烦地讲述血吸虫病过去是如何夺走无数生命的,以及在新中国,在中国共产党的领导下,疾病的受害者是如何恢复健康的。³⁴ 经过近三十年的血吸虫病防治,1978年《人民日报》宣布平湖县已成功消灭该疾病。2014年,在81岁高龄时,纪贤去当地党史档案馆移交了他的文件和他过去六十年当地血吸虫病防治的照片。当被问及原因时,他回答说,他希望平湖血吸虫病防治的历史能够被后代记住。³⁵ 在平湖,血吸虫病防治的成功故事并非自动源于中央政策,也不仅仅依赖于中国共产党的政治承诺。如果中国的血吸虫病防治

was locally successful, it was because of the individual commitment of people such as Ren Jixian who participated in it and who, through the propaganda of the CCP, became emblematic of overall rather than local success.

Not everyone shared the same level of commitment and enthusiasm or indeed of success. Zhu Zhiwen was a near contemporary of Ren Jixian. She also enrolled in a short course on schistosomiasis prevention in 1952 and subsequently became a schistosomiasis control cadre in nearby Jiaying County in Zhejiang province. Zhiwen, however, had a very different story to tell. She remembered how difficult it was to collect night-soil from the villagers. In China, chemical fertilizer only replaced bio-fertilizers in the 1980s. Until the 1970s, human and animal night-soil was the most important bio-fertilizer for farming. To villagers, night-soil was worth its weight in gold. To hand over night-soil simply meant to have less food on the table. Thus, for this reason alone villagers were hostile to schistosomiasis control cadres and sanitation work. Villagers mocked them as 'Mr Shit' (大便先生) or 'Shit Collector' (讨粪先生), comparing them to the despised tax collectors from the former Nationalist regime.³⁶ On one occasion, on Zhiwen's mission to collect night-soil, the villagers set their vicious guard dog to chase her away. She was so frightened that she began to cry. Zhiwen also remembered that many of her fellow grassroots health workers frequently vomited over the awful smell of the faeces. In the early days, her team consisted of only four to five people. Each day they had to examine between 100 and 200 packages of faeces. They were not supplied with rubber gloves, so their hands were in direct contact with the samples. After spending an entire day handling faeces, they could not abide their own smell. What troubled her even more was watching villagers die of the disease and being unable to help. On one occasion, a severely ill male patient went to her team begging for treatment after his local centre had refused him. Moved by his pleading, Zhiwen's team took him in, and after twenty courses of sodium antimony tartrate injections the patient was on his way to recovery. But three days after the treatment ended, he suffered withdrawal and died. Fearing the guard dog would devour the man's dead body, Zhiwen and another team member stayed up all night guarding it. She was so traumatized by this event that decades later she could still viscerally remember how cold and frightened she felt that night.³⁷

之所以在当地取得成功，是因为像任继贤这样的人的个人付出，并通过中国共产党的宣传，使他成为整体而非地方成功的象征。

并非每个人都拥有同等程度的投入和热情，当然也并非每个人都获得成功。朱志文与任继贤是同一时代的人。她也在1952年参加了血吸虫病防治短期培训班，后来成为浙江省嘉兴县的血吸虫病防治干部。然而，朱志文却讲述了一个非常不同的故事。她记得从村民那里收集粪便有多么困难。在中国，化肥直到20世纪80年代才取代生物肥料。直到20世纪70年代，人粪尿和动物粪尿还是农业最重要的生物肥料。对村民来说，粪便的价值等同于黄金。交出粪便就意味着餐桌上的食物会减少。因此，仅凭这一点，村民们就对血吸虫病防治干部和卫生工作抱有敌意。村民们嘲笑他们是“大便先生”或“讨粪先生”，将他们比作前国民党政权中令人鄙视的收税人。³⁶有一次，在朱志文去收集粪便的任务中，村民们放出凶猛的看门狗来追赶她。她吓得哭了起来。朱志文还记得，她的许多基层卫生工作者经常因为粪便的恶臭而呕吐。在早期，她的团队只有四到五个人。他们每天必须检查100到200包粪便。他们没有配备橡胶手套，所以他们的手直接接触样品。在花了一整天的时间处理粪便后，他们无法忍受自己身上的气味。更让她困扰的是，眼睁睁地看着村民死于这种疾病，却无能为力。有一次，一位病情严重的男病人来到她的团队，恳求治疗，因为当地的中心拒绝了他。被他的恳求所感动，朱志文的团队收留了他，经过20个疗程的酒石酸锑钠注射后，病人正在康复。但在治疗结束后三天，他出现了戒断反应并死亡。由于害怕看门狗会吞噬死者的尸体，朱志文和另一名队员熬夜守护着它。这件事给她留下了极大的创伤，几十年后她仍然能清楚地记得那天晚上她感到多么寒冷和害怕。³⁷

CHAOS ON THE GROUND

In 1953, as the Land Reform was drawing to a close in many parts of rural China, the CCP's rural work entered its next phase: to consolidate state control over the management and distribution of agricultural resources. The party introduced an increasing number of such campaigns across the countryside, ranging from grain procurement (state monopoly over grain distribution) to developing agricultural co-operatives* and improving agricultural productivity. Grain procurement and the collectivization of agricultural production became the party's core rural policy. Over time, such policies trumped any public health catastrophes occasioned by agricultural expansion. The rural administrative machinery was constantly overburdened by these new programs. Because of the shortage of trained personnel, grassroots cadres often had to at least make attempts at multi-tasking. Many schistosomiasis control cadres were also assigned to implementing other public health programs. They had to enforce 'new-style' medicalized child-birth, engage villagers in the Patriotic Health Campaign (1952), help with mass immunization and combatting seasonal infectious diseases, as well as support army recruitment work and enforce the New Marriage law. In addition, they were dragged into implementing the party's core rural policy of supporting agricultural collectivization and grain procurement work. Ren Jixian remembered that after spending an entire day examining the faeces of schoolchildren, as well as treating those who tested positive, his team stayed up all night to help procure grain for the state.³⁸ Juggling their multiple tasks, besides having little experience in disease control and a limited amount of time and resources, the majority of grassroots cadres focused on completing each task quickly rather than implementing the policy systematically. Tasks were often measured in quantitative terms, with numbered targets set by their superiors, and in some cases when cadres resented what they were asked to do they came up with makeshift solutions. Planning was thrown out the window, and responsibility was pushed around as if in a ball game. In some villages, for instance, with no time to mobilize the villagers to catch snails, local cadres spent the government-allocated schistosomiasis prevention funds purchasing enough snails to meet the snail elimination quota.³⁹

* In the early stage, collectivization of agricultural production took place in the form of co-operatives.

CHAOS ON THE GROUND

1953年，随着土地改革在中国农村许多地区接近尾声，中国共产党的农村工作进入了下一个阶段：巩固国家对农业资源管理和分配的控制。党在农村地区推行了越来越多的此类运动，从粮食采购（国家对粮食分配的垄断）到发展农业合作社*和提高农业生产力。粮食采购和农业生产集体化成为党的核心农村政策。随着时间的推移，这些政策胜过了农业扩张造成的任何公共卫生灾难。农村行政机构不断被这些新项目压得喘不过气来。由于训练有素的人员短缺，基层干部往往至少要尝试多任务处理。许多血吸虫病防治干部也被分配到执行其他公共卫生项目。他们必须强制执行“新式”医疗化分娩，动员村民参与爱国卫生运动（1952年），帮助进行大规模免疫接种和防治季节性传染病，以及支持军队征兵工作和执行新婚姻法。此外，他们还被拖入执行党的核心农村政策，即支持农业集体化和粮食采购工作。任继贤回忆说，在花了一整天时间检查小学生的粪便并治疗检测呈阳性的人后，他的团队熬夜帮助国家采购粮食。³⁸除了在疾病控制方面经验不足，时间和资源有限之外，基层干部还要兼顾多项任务，他们中的大多数人专注于快速完成每项任务，而不是系统地执行政策。任务通常以定量方式衡量，上级设定了编号的目标，在某些情况下，当干部们对被要求做的事情感到不满时，他们会想出临时的解决方案。计划被抛到九霄云外，责任就像球赛一样被推来推去。例如，在一些村庄，由于没有时间动员村民抓蜗牛，当地干部便花费政府分配的血吸虫病预防资金购买足够的蜗牛，以达到消灭蜗牛的配额。³⁹

* 在早期阶段，农业生产的集体化以合作社的形式进行。

A lack of planning, the rush to meet quantitative goals, and eagerness to outdo each other, in some cases with over-enthusiasm and an overt emphasis on mass involvement, led to carelessness that, in turn, caused the enormous waste of already scarce resources. An official evaluation in 1951-52 of disease prevention work in the countryside near Shanghai showed there was no system of managing and distributing the precariously scarce equipment and drugs. Damage to equipment was common. Fearing failure to meet the designated treatment targets, and without sufficient time to obtain the necessary data on patient numbers, some treatment centres overstocked massive amounts of drugs.⁴⁰ Many local cadres also resented or had no patience for the time-consuming sanitation work. Ignoring the government's policy of putting prevention first, they focused on treatment, since it was quicker to obtain quantifiable results that way. In terms of mass mobilization, it was also more visibly persuasive to use the testimonies of those being cured. Thus, public engagement became engagement in therapy only. In Zhejiang province's Shaoxing region, local authorities spent the entire prevention fund on mass therapy alone, and those cadres who were trained to carry out sanitation work were sent to do propaganda work instead. In nearly two years, no progress at all was made in prevention.⁴¹ Further west, in Qu County, one township was reported to have spent more than 128 million *yuan* treating 128 sick patients. Most of that money was wasted because in a rush to reach the treatment target set by the higher-ups, the local authority had engaged a huge number of volunteers to give injections, but since the volunteers were not properly trained and lacked the necessary experience, they often botched the shots. Such incidents so angered villagers that they drove the schistosomiasis prevention team out of the village and occupied their office for other, more pragmatic uses.⁴² To meet the assigned prevention target quickly, local cadres spent a total of 18,686,200 *yuan* from the schistosomiasis prevention fund to build a brand new communal village lavatory. Built in haste, and badly, the lavatory was not rain-proof. The cadres had failed to consult or engage the villagers, and as a result no one would use it; in the end it had to be dismantled. In another village, after exhausting the schistosomiasis prevention fund in building a new village well, there was no money left for any new bricks. Instead, the construction team used old bricks from a ruined burial ground, but as a result the water became contaminated and the well was never used.⁴³ On the other hand, because of the lack

由于缺乏规划,急于完成定量目标,以及渴望超越彼此,在某些情况下表现出过度热情和过度强调群众参与,导致了粗心大意,进而造成了对本已稀缺的资源巨大浪费。1951-52 年对上海附近农村地区疾病预防工作的官方评估表明,没有管理和分配岌岌可危的稀缺设备和药物的系统。设备损坏的情况很常见。由于担心无法达到指定的治疗目标,并且没有足够的时间获取必要的患者数量数据,一些治疗中心过度储备了大量药物。⁴⁰ 许多地方干部也对耗时的卫生工作感到不满或缺乏耐心。他们无视政府将预防放在首位的政策,专注于治疗,因为这样可以更快地获得可量化的结果。就群众动员而言,使用那些正在被治愈的人的证词也更具有说服力。因此,公众参与变成了仅参与治疗。在浙江省绍兴地区,地方当局将全部预防资金都花在了大规模治疗上,而那些接受过卫生工作培训的干部则被派去做宣传工作。近两年,预防工作毫无进展。⁴¹ 再往西,在衢县,据报道,一个乡镇花费了超过 1.28 亿元治疗了 128 名病人。大部分钱都被浪费了,因为为了赶上上级设定的治疗目标,地方当局聘用了大量的志愿者进行注射,但由于志愿者没有经过适当的培训,缺乏必要的经验,他们经常搞砸注射。这些事件激怒了村民,他们将血吸虫病防治队赶出了村庄,并占领了他们的办公室,用于其他更实际的用途。⁴² 为了迅速达到分配的预防目标,地方干部共花费了 18,686,200 元的血吸虫病防治资金,建造了一个全新的公共村庄厕所。由于建造匆忙且质量低劣,厕所不防雨。干部们没有咨询或动员村民,结果没有人使用它;最终不得不将其拆除。在另一个村庄,在耗尽血吸虫病防治资金建造新村井后,没有剩余资金购买任何新砖。相反,施工队使用了来自废弃墓地的旧砖,但结果导致水受到污染,水井从未被使用。⁴³ 另一方面,由于缺乏

of advance planning, the southern Jiangsu regional health bureau simply failed to allocate the necessary funding in time for schistosomiasis control. The shortfall in the community jeopardized the entire preventive program for the region.⁴⁴

There were also rampant problems that undermined safety, a muddled reporting structure, and regular overcharging of treatment fees. In racing to meet their treatment targets, for instance, some local treatment centres shortened the sodium antimony tartrate treatment course from twenty days to fifteen days. However, this shortened treatment led to overdoses and subsequent deaths. After four patients died as result, the Eastern China Health authority gave the order to put safety first. Some local treatment centres then became overly cautious and reduced the treatment dosage. This weakened the effectiveness of the treatment, resulting in a high rate of re-infection and leading to the wastage of the limited number of drugs. Lacking enough personnel and equipment, many treatment centres also failed to carry out the necessary 'prep' work prior to the actual treatment. This often led to even greater waste. At one centre, for example, because of the lack of preparation, 6,000 faecal samples got muddled up. Mistakes such as this delayed treatment and caused unnecessary waste of human and material resources.⁴⁵ Because of the lack of funding and limited access to the necessary drugs to ensure the mandated rate of success, some centres refused to treat the newly infected or those who were severely ill. In some areas, only prime labourers received treatment, since agricultural production depended on them staying healthy, but villagers fought against such discrimination by raising their grievances with the local authorities.⁴⁶ In Kunshan County, outside of Shanghai, to make up for the shortfall in funding, the local authority overcharged patients for each treatment. The villagers rioted and demanded that the authority not only subsidize the treatment but reimburse them for the work time lost while they were receiving treatment. They also bargained for greater compensation for the cost of digging new wells.⁴⁷ The conflict between implementing schistosomiasis therapy and the villagers' desire to improve family income would be exacerbated with the introduction of the work point distribution system as of 1955 and the commune system as of 1958.

Health education, and propaganda too, failed to convey the desired message around health. In 1951, the official estimate is that 6,560 schistosomiasis prevention educational posters were distributed in

由于缺乏事先的计划, 苏南地区卫生局根本未能及时为血吸虫病防治工作分配必要的资金。社区的资金短缺危及了整个地区的预防计划。⁴⁴

还存在着普遍存在的问题, 这些问题破坏了安全性, 报告结构混乱, 并且经常过度收取治疗费用。例如, 为了赶上治疗目标, 一些当地治疗中心将酒石酸锑钠的治疗疗程从 20 天缩短至 15 天。但是, 这种缩短的治疗导致了过量用药和随后的死亡。在四名患者因此死亡后, 华东卫生局下令将安全放在首位。然后, 一些当地治疗中心变得过于谨慎, 并减少了治疗剂量。这削弱了治疗的有效性, 导致高复发率, 并导致有限数量的药物浪费。由于缺乏足够的人员和设备, 许多治疗中心也未能进行实际治疗之前必要的“准备”工作。这常常导致更大的浪费。例如, 在一个中心, 由于缺乏准备, 6,000 个粪便样本被搞混了。诸如此类的错误延误了治疗, 并导致不必要的人力和物力资源浪费。⁴⁵ 由于缺乏资金且无法获得必要的药物来确保规定的成功率, 因此一些中心拒绝治疗新感染者或重病患者。在某些地区, 只有主要劳动力才能接受治疗, 因为农业生产取决于他们的健康状况, 但是村民通过向地方当局提出不满来反对这种歧视。⁴⁶ 在上海郊外的昆山县, 为了弥补资金的不足, 地方当局向每位患者收取过高的治疗费用。村民们暴动了, 要求当局不仅要补贴治疗费用, 还要补偿他们在接受治疗时损失的工作时间。他们还讨价还价, 要求为挖掘新井的成本提供更高的补偿。⁴⁷ 随着 1955 年工分分配制度和 1958 年公社制度的引入, 实施血吸虫病治疗与村民改善家庭收入的愿望之间的冲突将加剧。

健康教育和宣传也未能传达围绕健康的预期信息。1951 年, 官方估计在以下地区分发了 6560 张血吸虫病防治教育海报:

eastern and western Zhejiang's endemic villages; there was also the sponsoring of 403 public lectures, fifty-seven slide shows, and nine public health exhibitions. Songs and operas were composed to teach people about the danger of disease as well as methods of prevention. There were even short stories written on the theme. Aiming to educate the masses in order to change their behaviour, those involved in health education and propaganda failed to appreciate that schistosomiasis was closely bound up with the way of life of the rural population. Centuries-old habits would not change overnight, especially when people's living conditions stayed the same. Villagers enjoyed the entertainment, but they rejected the message: 'Our livelihood depended on water and night-soil. Keeping away from the water and centrally managing night-soil prevents us getting on with our farming work. It's wasting our time.'⁴⁸ Some authorities sought to use new technologies such as slide shows or short films to engage the public. Villagers were excited about the new media but, again, were unimpressed by the content. In Zhejiang province's Changshan County, for example, 300 villagers were invited to a slide show on schistosomiasis prevention, but they quickly got bored. Only nineteen of them stayed to watch the show to the end.⁴⁹ With the aim of engaging the masses with science, rural villagers in some endemic regions were invited to look at *cercariae* (the free-swimming larval stage of the parasite as it passed from the snail to humans and other animals) under the microscope. Unimpressed by the science, they thought schistosomiasis control cadres were playing magic tricks. In 1953, during a local snail elimination campaign in Zhejiang province's Qu County, where Xu Xifan had been deeply distraught by the damage caused by a major schistosomiasis outbreak in 1948, one villager crushed a snail with his hands and threw the dead snail on a fellow villager's leg: 'Let's see you get infected,' he mocked.⁵⁰

MODELS OF SUCCESS

Official reports were biased toward the successes, however. Sanhuanfan district in Zhuji County was one of Zhejiang province's successful models for schistosomiasis control. In 1951, responding to the Ministry of Health's schistosomiasis control directive, as well as the policy of putting prevention first, the Zhejiang provincial authority chose Sanhuanfan as its first schistosomiasis prevention pilot district. Personnel, along with technical and financial resources,

浙江东部和西部的地方性村庄；还赞助了 403 场公共讲座、57 场幻灯片放映和 9 次公共健康展览。创作歌曲和歌剧来教育人们疾病的危害以及预防方法。甚至还有以该主题创作的短篇小说。那些参与健康教育和宣传的人，旨在通过教育大众来改变他们的行为，但他们未能意识到血吸虫病与农村人口的生活方式密切相关。几个世纪以来的习惯不会在一夜之间改变，尤其是在人们的生活条件保持不变的情况下。村民们喜欢这些娱乐活动，但他们拒绝接受这个信息：“我们的生计依赖于水和夜间粪便。远离水源和集中管理夜间粪便会妨碍我们进行农业工作。这是在浪费我们的时间。”⁴⁸ 一些当局试图使用幻灯片放映或短片等新技术来吸引公众。村民们对新媒体感到兴奋，但同样对内容不感兴趣。例如，在浙江省常山县，300 名村民被邀请观看血吸虫病预防的幻灯片，但他们很快就感到厌烦。只有 19 人坚持看完了节目。⁴⁹ 为了让大众参与科学，一些地方性地区的农村村民被邀请在显微镜下观察尾蚴（寄生虫从蜗牛传播到人类和其他动物的自由游泳幼虫阶段）。他们对科学不感兴趣，认为血吸虫病防治干部在玩魔术。1953 年，在浙江省衢县的一次当地灭螺运动中，徐锡凡曾对 1948 年血吸虫病大爆发造成的破坏感到非常苦恼，一位村民用手捏碎了一只蜗牛，然后把死蜗牛扔到一位村民的腿上：“让我们看看你是否会被感染，”他嘲讽道。⁵⁰

成功的模式

然而，官方报告倾向于报道成功案例。诸暨县的三环畈区是浙江省血吸虫病防治的成功模式之一。1951 年，为响应卫生部的血吸虫病防治指示，以及“预防为主”的政策，浙江省政府选择三环畈作为其首个血吸虫病防治试点区。人员以及技术和财政资源，

were poured in. To guarantee a successful result, grassroots cadres and activists – from the village party secretary to the heads of local mass organizations such as the peasant association and the women's association – filled positions on the Sanhuanfan Schistosomiasis Prevention Committee. They took charge of the mass mobilization work to free up those experts sent by the provincial authority so that the latter could focus on the mass therapy and the more technical aspects of preventive work. These grassroots committee members set an example by handing over their own families' night-soil and went around to the villagers' homes to persuade them to hand theirs over. They did not give lectures. Instead, they helped villagers to wash vegetables, to sew shoes, or to harvest grain. They also came up with the innovation of redistributing the collective night-soil to each family according to its size. Attracted by the economic benefit, villagers began to hand over their night-soil. Villager Jiang Huanfa had been re-infected with schistosomiasis three times and had lost faith in the efficacy of any therapy. From the day that he was admitted for further treatment, he had been planning to escape, but having sensed his reluctance to undergo schistosomiasis therapy, cadres sent activists from the local women's association to influence his mother. After repeated visits and conversations about how treatment plus prevention would bring her son back to health, Jiang's mother was won over. Persuaded by his mother, Jiang stayed in the treatment centre and successfully completed his treatment. After his health had been restored, he was invited to share his experience at a public meeting. Jiang became the living testimony for the virtue of the state-run schistosomiasis control.⁵¹ There were many other such public meetings where villagers learned about the horrific economic cost of the disease. They were told that schistosomiasis *and* the exploitative local landlords had driven them to poverty. Thus, in order to achieve economic betterment they had to fight the disease *as well as* the landlords. Such meetings often ended with party activists shouting slogans such as 'To fight against schistosomiasis was to fight against the enemy' and 'Just as we need every villager to fight against the landlords, we also need every villager to fight against schistosomiasis.' Enthusiasm soared, and with this repeated propaganda effort, as an archival report elucidates, the local schistosomiasis prevention work could be carried out effortlessly.⁵²

Why did this area produce success where others failed? In addition to successful grassroots mobilization work, the same report

都被倾注进去了。为了保证取得圆满成功，从村党支部书记到当地群众组织（如农民协会和妇女协会）的负责人等基层干部和积极分子，都在三环防治血吸虫病委员会中担任职务。他们负责群众动员工作，以便让省政府派来的专家能够专注于群众治疗和更具技术性的预防工作。这些基层委员会成员以身作则，交出自己家的粪便，并挨家挨户说服村民交出粪便。他们没有进行讲座，而是帮助村民洗菜、缝鞋或收割谷物。他们还创新性地根据每个家庭的规模重新分配集体粪便。受经济利益的吸引，村民们开始交出粪便。村民蒋焕发曾三次感染血吸虫病，对任何疗法的疗效都失去了信心。从他被收治进一步治疗的那天起，他就计划逃跑，但干部们感觉到他不愿接受血吸虫病治疗，便派当地妇联的积极分子去影响他的母亲。经过多次走访和关于治疗加预防将如何使她儿子恢复健康的谈话，蒋的母亲被说服了。在母亲的劝说下，蒋留在了治疗中心并成功完成了治疗。恢复健康后，他被邀请在一次公开会议上分享他的经验。蒋成为了国家血吸虫病控制美德的活生生的证明。⁵¹ 还有许多其他的公开会议，村民们在会上了解了这种疾病的可怕的经济代价。他们被告知血吸虫病和剥削性的当地地主使他们陷入贫困。因此，为了实现经济上的改善，他们必须与疾病以及地主作斗争。这样的会议通常以党的积极分子高呼诸如“与血吸虫病作斗争就是与敌人作斗争”和“正如我们需要每个村民与地主作斗争一样，我们也需要每个村民与血吸虫病作斗争”等口号结束。热情高涨，而且通过这种反复的宣传努力，正如一份档案报告所阐明的那样，当地的血吸虫病预防工作可以毫不费力地进行。⁵²

为什么这个地区取得了成功，而其他地区却失败了？除了成功的基层动员工作外，同一份报告

also indicates that ensuring the 100 per cent success rate demanded that no infected villager from outside the area be allowed to receive schistosomiasis therapy at Sanhuanfan pilot district.⁵³ By strictly limiting the patient population to the available resources, success could indeed be achieved. But such an approach only worked in the most tightly controlled and limited context and could not be ramped up for the broader population.

Buyung Township in Jiaxing County was Zhejiang province's other model of success. Here, the mass mobilization did not start well. Overburdened by different rural campaigns introduced by the CCP, local cadres had to make choices and weigh returns against risks. Some put all the township's resources on the CCP's core rural policy of agricultural collectivization and production. For them, schistosomiasis prevention was simply a waste of time. Faced with their initial failure to gain support from grassroots cadres and to engage the locals, the West Zhejiang Schistosomiasis Control Centre felt something had to change. Since they could not mobilize the masses, they decided to target only a small number of villages. Number Four village was chosen because the village cadre was supportive. It began with a snail-catching competition in which village cadres and activists took the lead. They were followed by children from the village school as well as elderly villagers who had lighter farming duties. The competition lasted a month, and nearly thirty kilograms of snails were caught. Winners were honoured as model workers and were rewarded with material goods according to the number of snails they caught and their practical needs. Some were given hoes, others axes and straw hats; children were rewarded with school bags, textbooks, and writing materials. Rewards were given at a public meeting to celebrate the village's achievement and to encourage other villages to follow their example. Sixty-year-old Chen Chuda was the champion snail-catcher, having caught 135 grams of snails, and was given first-class honours. He was invited to give a speech at the meeting: 'During the Nationalist era, I did not know anything about snails. After the [Communist] Liberation I began to see snails everywhere. Thank the Communist Party for leading us to catch snails.' Chen's speech was simple but sincere. Representatives of other villages were obliged to take up the vow: 'We had been too negligent. We must learn from Number Four Village and to carry out our [schistosomiasis control] work properly and well.' But the provincial health authority was not overly enthusiastic about either

也表明, 为了确保百分之百的成功率, 必须禁止来自该地区以外的受感染村民在三环畈试点区接受血吸虫病治疗。⁵³ 通过严格限制患者人数以适应现有资源, 确实可以取得成功。但这种方法只适用于最严格控制 and 有限的环境, 无法推广到更广泛的人群。

嘉兴县步云乡是浙江省的另一个成功典范。在这里, 群众动员一开始并不顺利。由于受到共产党推出的各种农村运动的重压, 当地干部不得不做出选择, 权衡回报与风险。有些人将全乡的资源都投入到共产党的核心农村政策——农业集体化和生产上。对他们来说, 血吸虫病防治简直是浪费时间。面对他们最初未能获得基层干部的支持以及未能动员当地人, 浙西血吸虫病防治中心感到必须做出改变。由于他们无法动员群众, 他们决定只针对少数几个村庄。第四村被选中, 因为该村的干部很支持。首先是捉螺比赛, 村干部和积极分子带头。紧随其后的是村小学的孩子以及务农较轻的老年村民。比赛持续了一个月, 共捉了近 30 公斤的钉螺。获胜者被授予劳动模范称号, 并根据捉螺的数量和实际需求获得物质奖励。有些人得到了锄头, 有些人得到了斧头和草帽; 孩子们则获得了书包、课本和文具。在一个公共会议上颁发奖励, 以庆祝该村取得的成就, 并鼓励其他村庄效仿。六十岁的陈楚达是捉螺冠军, 捉了 135 克钉螺, 并获得一等奖。他应邀在会上发言: “在国民党时代, 我对钉螺一无所知。[解放]后, 我开始到处看到钉螺。感谢共产党领导我们捉钉螺。” 陈的讲话简单而真诚。其他村庄的代表不得不立下誓言: “我们过去太疏忽了。我们必须向第四村学习, 认真做好我们的[血吸虫病防治]工作。” 但省卫生部门对这两种情况都不太热衷

Buyun Township's purported success or their method of snail eradication. In their reply to the West Zhejiang Schistosomiasis Control Centre's report on success, they warned: 'A snail-catching exercise is dangerous if those involved don't wear protection.' Despite the warning from their superiors, the West Zhejiang Schistosomiasis Control Centre took the decision that the local control program was to centre on the snail-catching competition, even though they were fully aware of the danger of infection as well as the improbability of the total elimination of snails. For them, snail-catching was the only mobilization work they had managed with some success so far, and it cost the local authority little money. Since there was no funding for implementing sanitation work such as building new wells or latrines, catching snails made better sense economically and was therefore more likely to gain support from grassroots cadres and villagers. Based on their experience in Number Four village, they calculated that on average one villager could catch fifteen kilograms of snails within three hours. Men had to work in the fields to meet the state quota for grain production, but women and children had little to do; they rather enjoyed the fun of catching snails. Local mass organizations such as the Women's Association, the Communist Youth League, and the Peasant Association had built up considerable experience during earlier mass campaigns, such as for the elimination of illiteracy as well as Land Reform. Subsequently, they played a key role in organizing snail eradication teams of women and children. In the first half of 1953, it was reported that 138,000 snails had been caught and killed in west Zhejiang.⁵⁴ Yet the ever-expanding number of snails killed seemed to have little or no effect on the actual spread of the illness, given that the true focus was the expansion of agricultural collectivization and the increase of yield. Increased agricultural activities also increased the chances of infection. In Jiaying region, for instance, 24,166 people tested positive. In 1953, the number of people testing positive grew to 30,030. The rate of infection was 23.38 per cent – 0.10 per cent higher than the previous year.⁵⁵

CONSOLIDATING

The goal of controlling the disease was nowhere near accomplished. In the drive to meet the quantitative target that prioritized therapeutic intervention, prevention was largely neglected. Treatment also failed to deliver satisfactory results. In parts of Zhejiang province, although

布云乡所谓的成功，或者说他们消灭钉螺的方法。在他们回复给浙西血吸虫病防治所关于成功的报告中，他们警告说：“如果不采取保护措施，捕捉钉螺的活动是很危险的。”尽管他们的上级提出了警告，但浙西血吸虫病防治所还是决定将当地的防治工作重点放在捉螺比赛上，尽管他们充分意识到感染的危险以及彻底清除钉螺的可能性不大。对他们来说，捉螺是他们迄今为止成功组织的唯一动员工作，而且地方政府为此花费的资金也很少。由于没有资金用于实施新建水井或厕所等卫生工作，因此捕杀钉螺在经济上更有意义，因此更有可能获得基层干部和村民的支持。根据他们在第四村的经验，他们计算出平均一个村民可以在三个小时内捕获十五公斤的钉螺。男人必须在田里工作以满足国家粮食生产的配额，但妇女和儿童无所事事；他们反而很享受捉螺的乐趣。当地群众组织，如妇联、共青团、农民协会等，在早期的群众运动中积累了相当的经验，如扫盲和土地改革。随后，他们在组织妇女和儿童的灭螺队方面发挥了关键作用。据报道，1953年上半年，浙西共捕杀钉螺13.8万只。⁵⁴然而，鉴于真正的重点是扩大农业集体化和提高产量，不断增加的钉螺捕杀数量似乎对疾病的实际传播几乎没有或根本没有影响。农业活动的增加也增加了感染的机会。例如，在嘉兴地区，有24166人检测呈阳性。1953年，检测呈阳性的人数增加到30030人。感染率为23.38%，比上一年高0.10%。⁵⁵

巩固

控制疾病的目标远未实现。在满足优先考虑治疗干预的量化目标的推动下，预防在很大程度上被忽视了。治疗也未能取得令人满意的结果。在浙江省的部分地区，虽然

the local authorities offered free treatment for those classified as poor peasants, many patients withdrew from their treatment halfway through the course because they could not afford to pay for the meals at the treatment centres. Even amongst those who completed their treatment, the rate of re-infection was high. Take Jiaying, for example, Zhejiang's flagship region for schistosomiasis control. Here, the rate of re-infection was as high as 42 per cent.⁵⁶ Villager Dong Liyin was one of those who were re-infected: 'The treatment was useless. One month after receiving the treatment, I became re-infected, even though I followed the prevention advice by keeping away from waterways or paddy fields,' he complained, with a hint of a whine in his voice. Faced with such failures and to ensure a consistent rate of success, some local authorities gave the order to treat only those in the early stage of disease who had less severe symptoms.⁵⁷ When sick villagers confronted the authorities for breaching the stated policy for access to treatment, in the context of the New 'Three Anti'-Campaign to struggle against growing 'bureaucratism, commandism, and violations of law and discipline' at the grassroots level, the Zhejiang provincial Party Committee charged grassroots schistosomiasis control organizations with growing too fast and getting too large. Their growing bureaucratic structure was said to have contributed to 'commandism', a result of 'going over the head' of local organizations to achieve their political objective. In turn, 'commandism' had resulted in the low quality of local control work, with a poor outcome as well as excess waste – at least according to the local officials who clearly understood how they could retain their power over those now wresting it away from them in the struggle for control of the public's health. 'Counter-revolutionaries', right or left opportunists, were also 'unmasked' for being the underlying cause of corruption. They were accused of violating the party's decrees and thus contributing to the overall failure of local schistosomiasis control work. As such, ninety-nine schistosomiasis control cadres in Qu County were found guilty of different political crimes. Yet it is often not clear why and how they acquired those labels. For instance, amongst the 117 'unmasked' as Nationalist spies, their 'criminal' activities did not involve spying but the fact that they were 'narcissistic'.⁵⁸ In addition, just as there were 'models of success', there were also 'models of failure'. The latter were named and shamed.

More severely, the Zhejiang Provincial Party Committee accused the grassroots schistosomiasis control work of jeopardizing agricultural production, thus undermining the CCP's core rural policy. As

地方当局为那些被归类为贫农的人提供免费治疗，但许多患者在疗程中途退出了治疗，因为他们无力支付治疗中心的膳食费用。即使是那些完成治疗的人，再感染率也很高。以浙江省血吸虫病防治的旗舰地区嘉兴为例。这里的再感染率高达 42%。⁵⁶ 村民董立银就是其中一位再感染者：“治疗没用。接受治疗一个月后，我又被感染了，即使我按照预防建议，远离水道或稻田，”他抱怨道，语气中带着一丝哀怨。面对这样的失败，为了确保稳定的成功率，一些地方当局下令只治疗那些病情早期、症状较轻的患者。⁵⁷ 当患病村民就违反既定治疗政策一事质问当局时，在新“三反”运动（反对日益严重的“官僚主义、命令主义和违反法律纪律行为”）的背景下，浙江省委指责基层血吸虫病防治机构发展过快、规模过大。据说他们日益官僚化的结构导致了“命令主义”，这是“越过”地方组织以实现其政治目标的结果。反过来，“命令主义”导致了地方控制工作质量低下，结果很差，浪费过多——至少根据那些清楚地了解他们如何能够保留对那些现在在争夺公众健康控制权中从他们手中夺走权力的人的权力的当地官员的说法。“反革命分子”、右倾或左倾机会主义者也被“揭露”为腐败的根本原因。他们被指控违反党的法令，从而导致当地血吸虫病防治工作的全面失败。因此，衢县有 99 名血吸虫病防治干部被判犯有不同的政治罪行。然而，通常不清楚他们为什么以及如何获得这些标签。例如，在被“揭露”为国民党间谍的 117 人中，他们的“犯罪”活动并不涉及间谍活动，而是因为他们“自恋”。⁵⁸ 此外，正如有“成功典范”一样，也有“失败典范”。后者被点名羞辱。

更严重的是，浙江省委指责基层血吸虫病防治工作危害农业生产，从而破坏了中国共产党的核心农村政策。因为

their 'rescue' plan to remedy past failures, the Zhejiang Provincial Party Committee closed many local schistosomiasis control centres and drastically cut back on the number of grassroots schistosomiasis cadres. According to the provincial deputy party secretary He Shilian, this was to 'clean up' the grassroots organizations and to avoid making errors of 'left deviationism' as well as 'right deviationism'.⁵⁹ As the leading province in schistosomiasis control work, Zhejiang boasted of having better resources, since it inherited many of the health and disease control institutions as well as experts and trained personnel from the former Nationalist government. During 1951-52, it had made impressive progress in building up a grassroots schistosomiasis control workforce. By 1953, Zhejiang's flagship schistosomiasis control region, Jiaxing, had successfully recruited and trained 763 full-time schistosomiasis control cadres. Many more were mobilized to take part in what they saw as a historical mission. In November 1953, all grassroots schistosomiasis control cadres in Zhejiang province were summoned to the beautiful provincial capital of Hangzhou for a 'rectification meeting'. Zhang Ziwei was the party representative initially assigned to the takeover of the former Nationalist health institutions in the areas around Jiaxing. He also took charge of building up the region's schistosomiasis control workforce. Many years later, in his memoir written in the early 1990s, he could still recall how his team were filled with great excitement and enthusiasm when they first arrived at Hangzhou.⁶⁰ They learned quickly, however, that their hard work over the past three years was condemned for undermining the party's core rural policy and for jeopardizing agricultural production. They were criticized for using styles of 'bureaucratism' and 'commandism' in schistosomiasis control work as well as for building too many wells, latrines, and septic tanks that led to enormous financial waste. Elsewhere, in Jiaxing region, 489 schistosomiasis control cadres, 64 per cent of the total number, were dismissed from duty. Some were presented with the option of pursuing further training in leading grassroots political work, others were 'mobilized' to go and work for the local railway and mail services, and a small number returned home to look for other livelihoods. Disheartened, they grumbled: 'We had been mobilized to take part in schistosomiasis control work, now we are mobilized to leave. What's the point?'⁶¹ In the meantime, the province's mass therapy program was dismantled. The number of people who could receive treatment was reduced from 50,000 to

他们的“拯救”计划是为了补救过去的失败，浙江省委关闭了许多地方血吸虫病防治中心，并大幅削减了基层血吸虫病防治干部的数量。据省委副书记何世莲说，这是为了“清理”基层组织，避免犯“左倾”和“右倾”错误。⁵⁹作为血吸虫病防治工作的领先省份，浙江拥有更好的资源，因为它继承了前国民政府的许多卫生和疾病控制机构以及专家和训练有素的人员。1951-52年间，浙江在建立基层血吸虫病防治队伍方面取得了令人瞩目的进展。到1953年，浙江的旗舰血吸虫病防治区嘉兴已成功招募和培训了763名专职血吸虫病防治干部。更多的人被动员起来参加他们认为具有历史意义的使命。1953年11月，浙江省所有基层血吸虫病防治干部被召集到美丽的省会杭州参加“整风会议”。张子伟是最初被派往接管嘉兴周边地区前国民党卫生机构的党委代表。他还负责建立该地区的血吸虫病防治队伍。多年后，在他20世纪90年代初写的memoir中，他仍然记得他的团队第一次到达杭州时是多么的兴奋和热情。⁶⁰然而，他们很快了解到，他们过去三年的努力被谴责为破坏党的农村核心政策，危害农业生产。他们被批评在血吸虫病防治工作中使用了“官僚主义”和“命令主义”的作风，以及建造了太多的水井、厕所和化粪池，导致了巨大的财政浪费。在嘉兴地区，489名血吸虫病防治干部，占总数的64%，被解除了职务。有些人被推荐去接受进一步的基层政治工作培训，另一些人被“动员”去当地的铁路和邮政部门工作，还有少数人回家寻找其他生计。他们灰心丧气地抱怨道：“我们被动员起来参加血吸虫病防治工作，现在我们又被动员离开了。这有什么意义呢？”⁶¹与此同时，该省的大规模治疗计划被拆除。可以接受治疗的人数从50000人减少到

15,000. As the CCP's priority changed, it took the entire country into the phase of 'political consolidation'. In its struggle against 'bureaucratism', 'commandism', and violations of law and discipline, schistosomiasis control as well other aspects of public health were also to be 'consolidated' and compromised. People's health was at stake. In Zhejiang province alone, an official report showed that some 35,000 villagers suffering from the disease were deprived of treatment and care as a greater number of schistosomiasis control organizations were consolidated.⁶²

From then on, grassroots schistosomiasis control work in Zhejiang province, as well as in other parts of the country, was compromised to a large extent. It was primarily made subordinate to agricultural production and the concomitant development of agricultural co-operatives. As the remaining schistosomiasis control cadres helped villagers to fight drought, plant rice, control pests, and harvest crops, they served as 'the thread that tied the people to the party' and thus played a key role on the ground in assisting the party to consolidate its rule in the endemic regions. In the meantime, managing night-soil and water became central to fostering agricultural collectivization. Snail elimination, too, became integral to land reclamation as well as water conservation – both were essential for China's agricultural development, as will be discussed in chapter 3. The PRC's schistosomiasis control entered the phase of 'two managing and one eradicating' (两管一灭), and more basic scientific research would be conducted to provide scientific backup for the program. In practice, integrated management meant an abandonment of local mass efforts and the amelioration of the anti-commandism campaign as it became evident that centralizing management structures always risked the charge of greater commandism.

Eliminating snails by engineering modifications to agricultural irrigation canals, such as clearing vegetation and cementing, was first developed in Japan. Construction of cement ditches was incorporated into the 1950 amendment to the National Parasite Prevention Act in Japan. A couple of years later, in China, Chinese researcher Wang Peixin began to pilot the use of cement-lined ditches in some endemic regions of Zhejiang province.⁶³ Back in 1947, as part of his master's degree study, Wang had spent a few months researching schistosomiasis control in mainland Japan and Taiwan. After returning to China in 1948, Wang joined the Nationalist Jiangsu Medical University and spent much of his time observing the movement of

15,000。随着中国共产党优先事项的改变，整个国家进入了“政治巩固”阶段。在反对“官僚主义”、“命令主义”和违反法律纪律的斗争中，血吸虫病防治以及公共卫生的其他方面也要被“巩固”和妥协。人民的健康危在旦夕。仅在浙江省，一份官方报告显示，由于更多的血吸虫病防治组织被合并，约有35,000名患有该病的村民被剥夺了治疗和护理。⁶²

从那时起，浙江省以及中国其他地区的基层血吸虫病防治工作在很大程度上受到了影响。它主要服从于农业生产和随之而来的农业合作社的发展。由于剩余的血吸虫病防治干部帮助村民抗旱、种植水稻、防治病虫害和收获作物，他们充当了“将人民与党联系在一起纽带”，因此在协助党巩固其在流行地区的统治方面发挥了关键作用。与此同时，管理粪便和水成为促进农业集体化的中心。消灭钉螺也成为开垦土地和水利保护的组成部分——两者对于中国农业发展至关重要，正如第3章将讨论的那样。中华人民共和国的血吸虫病防治进入了“两管一灭”阶段，并将进行更基础的科学研究，为该项目提供科学支持。在实践中，综合管理意味着放弃地方群众努力，并改善反命令主义运动，因为很明显，集中管理结构总是面临更大的命令主义指控。

通过对农业灌溉渠进行工程改造（如清除植被和水泥硬化）来消灭蜗牛的方法，最早在日本开发。水泥沟渠的建设被纳入1950年日本《国家寄生虫预防法》的修正案。几年后，在中国，中国研究员王培新开始在浙江省的一些地方病流行区试用水泥衬砌的沟渠。⁶³早在1947年，王培新作为硕士学位研究的一部分，曾花费几个月时间研究日本本土和台湾的血吸虫病防治。1948年回到中国后，王培新加入了国民党的江苏医学院，并将大部分时间用于观察

snails in the marshland along the Yangtze delta. After his institute was taken over by the Communist authority, Wang was transferred to the nearby Zhejiang province to lead schistosomiasis control research in west Zhejiang. The new co-operative agricultural system that began to emerge after 1952 made it easier to organize schistosomiasis control by burying the old snail-infested canals. Initially, a number of local authorities welcomed such undertakings because it allowed them to kill two birds with one stone: water conservation as well as schistosomiasis control, through mass participation. But at this early stage, the quality of such work was very poor. Villagers often complained that after the burial work, the surface of the earth became uneven and made walking between the ditches hazardous.⁶⁴

Around the same time, a team of researchers from Hunan in central China came up with another idea of eliminating snails through land reclamation. Hunan province, bordering the Yangtze, is famous for hot chilies and fiery food and for having produced many revolutionary leaders of modern China. It also has the longest recorded history of being a schistosomiasis endemic region. Archaeological evidence of *Schistosoma* eggs from an unearthed female corpse excavated in the Mawangdui Han burial site dates the disease back more than 2,000 years. Indeed, as mentioned in chapter 1, it was by the shore of Dongting Lake that Cao Cao's army suffered a massive defeat after succumbing to a strange disease. Scholars in China identified the disease as schistosomiasis. It was also in Hunan, in 1905, that the American missionary doctor O.T. Logan reported the first clinical case of *schistosoma japonicum* in China.⁶⁵

In June 1950, Chen Huxin, another graduate of Hunan-Yale College and a believer in saving his troubled motherland through the benefits of bio-medicine, was assigned the task of leading schistosomiasis control work in the Dongting Lake region. During their field research, Chen and his team came up with the hypothesis that land reclamation could help to eliminate snails by destroying their habitat. Their study was published in 1952, a few months before a catastrophic storm hit the southern regions around Dongting Lake. The storm flooded nearly 30,000 acres of farmland and claimed more than 2,000 lives. Around 90,000 people living along the lake were affected. That year's harvest was dealt a fatal hit, and agricultural development in the region suffered a heavy blow. There was therefore an urgent need to open up more land for cultivation.⁶⁶ The south Dongting Lake reconstruction project began in December

长江三角洲沿岸沼泽地中的钉螺。他的研究所被共产党当局接管后,王被调到附近的浙江省,领导浙西的血吸虫病防治研究。1952年后开始出现的新型农业合作体系,通过掩埋旧的、滋生钉螺的沟渠,使血吸虫病防治工作更容易组织。最初,一些地方当局欢迎这种做法,因为它让他们可以一石二鸟:通过群众参与,既可以进行水利建设,又可以控制血吸虫病。但在这个早期阶段,这项工作的质量非常差。村民们经常抱怨说,掩埋工作完成后,地表变得不平整,在沟渠间行走很危险。⁶⁴

大约在同一时间,来自华中湖南省的一个研究小组提出了通过土地复垦来消灭钉螺的另一个想法。湖南省与长江接壤,以其辛辣的辣椒和辛辣的食物以及涌现了许多中国现代革命领袖而闻名。它也是有记录以来血吸虫病流行时间最长的地区。从马王堆汉墓出土的女性尸体中挖掘出的血吸虫卵的考古证据表明,该疾病可以追溯到2000多年前。事实上,正如第一章所提到的,曹操的军队就是在洞庭湖畔遭受了一场大败,原因是他们屈服于一种奇怪的疾病。中国的学者们将这种疾病鉴定为血吸虫病。1905年,也是在湖南,美国传教士医生O.T. Logan报告了中国第一例日本血吸虫的临床病例。⁶⁵

1950年6月,另一位毕业于湖南雅礼学院的陈虎新被分配到洞庭湖地区领导血吸虫病防治工作,他坚信可以通过生物医药的益处来拯救他处境艰难的祖国。在实地调查中,陈虎新和他的团队提出了一个假设,即围垦可以通过破坏蜗牛的栖息地来帮助消灭蜗牛。他们的研究发表于1952年,几个月后,一场灾难性的风暴袭击了洞庭湖周围的南部地区。这场风暴淹没了近3万英亩的农田,夺去了2000多人的生命。大约有9万人居住在湖边,受到了影响。当年的收成遭受了致命的打击,该地区的农业发展遭受了沉重打击。因此,迫切需要开垦更多的土地用于耕种。⁶⁶南洞庭湖重建项目于12月开始

1952. The committee in charge of the project decided to put Chen's hypothesis of eliminating snails by reclaiming the marshes to the test. The marshland of Xiangyin County, infested with snails, was targeted for agricultural cultivation, and Chen and his team were invited to monitor snail numbers in the reclaimed land. Their survey data over the next two years showed a considerable reduction in the snail population. The reclamation project, however, involved more than 120,000 construction workers toiling in the snail-infested marshland and contaminated water, and as a result many (including those from non-endemic regions) were found to have contracted schistosomiasis.⁶⁷ Although Chen Huxin was concerned about this, what mattered more for the authority, however, was that the number of snails decreased significantly. The positive outcome of eliminating snails by land reclamation in Xiangyin County was timely and so was the experiment of linking water conservation with snail elimination in west Zhejiang. Such comprehensive approaches would grow ever more important and were adopted into the national program in the following decade as the CCP Central Committee marched the country into full-scale agricultural collectivization, matched by an even greater scale of the expanding nationwide schistosomiasis control program, as will be discussed in chapter 3.

In 1953, the First Five-Year Plan was put into effect and ran until 1957. Its goal was to rapidly industrialize China and transform the country into a modern industrial power, following the model of the five-year plans in the USSR. Because the largest amount of state investment would go to heavy industry and a much smaller amount would be made available for agricultural development, there was an urgent need to speed up the process of agricultural collectivization. Hence, to bring a full socialist transformation to the Chinese countryside would entail a radical restructuring of the national schistosomiasis control program and thus an improvement in the people's health.

1952年。负责该项目的委员会决定对陈氏关于通过开垦沼泽地来消灭钉螺的假设进行检验。湘阴县的沼泽地布满了钉螺，被选定用于农业耕作，陈和他的团队被邀请来监测开垦土地上的钉螺数量。他们在未来两年内的调查数据显示，钉螺数量大幅减少。然而，这项开垦项目涉及超过12万名建筑工人在布满钉螺的沼泽地和受污染的水域中劳作，结果许多人（包括来自非流行地区的）被发现感染了血吸虫病。⁶⁷ 尽管陈湖馨对此感到担忧，但对当局来说，更重要的是钉螺数量显著减少。湘阴县通过土地开垦消灭钉螺的积极成果非常及时，浙江西部将水利工程与消灭钉螺联系起来的实验也是如此。这种综合性的方法将变得越来越重要，并在随后的十年中被纳入国家计划，当时中共中央委员会带领全国进入全面农业集体化，与此同时，全国范围内的血吸虫病控制计划也在更大规模地扩展，正如第3章将要讨论的那样。

1953年，第一个五年计划开始实施，一直持续到1957年。其目标是迅速实现中国的工业化，并将中国转变为一个现代工业强国，效仿苏联的五年计划模式。由于国家投资的最大部分将用于重工业，而用于农业发展的资金则少得多，因此迫切需要加快农业集体化进程。因此，要给中国农村带来全面的社会主义改造，就需要对国家血吸虫病防治计划进行彻底的调整，从而改善人民的健康。

Eradicating Schistosomiasis as a Political Undertaking

THE STORM OF SOCIALISM

In the Chinese Agrarian Calendar, 1955 was the ‘Year of the Wooden Goat’, signifying productiveness and success. In the history of the People’s Republic of China (PRC), 1955 was a year of great significance: it marked the end of a period of makeshift years to a year of economic planning and the final adoption of the First Five-Year Plan. Although the First Five-Year Plan was actually inaugurated in 1953, work on the draft plan was only completed in February 1955 and was approved by the First National People’s Congress in July 1955. It was followed by corresponding plans at the provincial level. ‘We have won a new China, we must redouble our efforts in economic construction to protect and consolidate this new China. This construction is, in the main, a task of socialist industrialization,’ as Li Fuchun, the vice premier of the State Council of China and chairman of the State Planning Commission, announced with a level of optimism when delivering the lengthy *Report on the First Five-Year Plan for the Development of the National Economy of the People’s Republic of China in 1953–1957*.¹ At the same time, Li and his colleagues at the State Council and the State Planning Commission were aware that the new China they had won was faced with the problem of decreased agricultural production due to poor harvests in the previous two years as well as persistent grassroots resistance to agricultural co-operativization (合作化).^{*} With

^{*} Before 1958, collectivization of agricultural production took place in the form of co-operatives. The official term for this stage of collectivization was ‘co-operativization’. In 1958, agricultural co-operatives were merged into people’s communes.

将血吸虫病作为一项政治任务根除

社会主义的风暴

在中国农历中，1955年是“木羊年”，象征着生产力和成功。在中华人民共和国（prc）的历史上，1955年是意义重大的一年：它标志着权宜之计的结束，经济规划的开始，以及最终通过第一个五年计划。尽管第一个五年计划实际上是在1953年启动的，但计划草案的工作直到1955年2月才完成，并在1955年7月获得第一届全国人民代表大会的批准。随后，各省级层面也制定了相应的计划。正如时任中国国务院副总理兼国家计划委员会主任的李富春在发表冗长的《关于中华人民共和国发展国民经济的第一个五年计划（1953-1957）的报告》¹时，以乐观的态度宣布的那样：“我们已经赢得了一个新的中国，我们必须加倍努力进行经济建设，以保护和巩固这个新的中国。这项建设主要是一项社会主义工业化的任务。”与此同时，李富春及其在国务院和国家计划委员会的同事们意识到，他们赢得的新中国面临着农业生产下降的问题，这是由于过去两年收成不好以及基层对农业合作化（合作化）的持续抵制造成的。^{*}与

^{*} 1958年之前，农业生产的集体化以合作社的形式进行。集体化的官方术语是“合作化”。1958年，农业合作社合并为人民公社。

less food to export, there was not so much equipment to be imported from the USSR and other European countries for ‘socialist industrialization’. China had to rely on its own resources for the implementation of the First Five-Year Plan, and austerity was to continue. Since the state had invested huge amounts in heavy industry, there was little cash to invest in agricultural development. Thus, in addition to more efficient and rational management, there was an ever-greater need for agricultural co-operativization as well as for increased enthusiasm in encouraging production. ‘Under the present conditions, the agricultural producers’ co-operative is a means of increasing agricultural output that involves small investment and speedily yields good results. It is also a necessary step leading the peasants to socialism ... The enthusiasm which individual peasant households show for increasing production must be taken into consideration, and vigorous assistance and leadership should be given to them to develop all their resources to the full and raise per unit area yields,’ Li Fuchun’s report reasoned.² Yet on the ground, ‘vigorous assistance and leadership’ would pan out quite differently from what Li and the Chinese Communist Party (CCP) leadership envisaged.

Among the measures laid out in the draft Five-Year Plan to increase agricultural output, Li’s report prioritized land reclamation, water conservation, and intensive farming methods: ‘Since our population is large while the area under cultivation is still limited, we must take energetic measures to open up arable land in a planned way so that we can finally solve the problem of shortages in agricultural produce ... Water conservancy work can extend the irrigated area to increase agricultural output as well as preserve crops from the ravages of flood or drought. The increased agricultural output and other benefits brought about by the elimination or reduction of natural calamities through water conservancy works in many cases to pay back within a few years the funds invested ... Peasants should be encouraged to cultivate their land still more intensively, to put in more fertilizer and step by step adopt more advanced farming techniques ... The use of improved animal-drawn ploughs ... vigorous efforts to store and prepare manure ... are effective ways of raising farm output.’³ However, all of this was easier to propose than to actually realize.

The minimal target for the expansion of cultivated area as laid out in the First Five-Year Plan was 38,680,000 *mu* (ca. 6,372,024 acres): ‘Every effort must be made and means used to surpass it. One

用于出口的食物减少了，从苏联和其他欧洲国家进口用于“社会主义工业化”的设备也减少了。中国不得不依靠自身资源来实施第一个五年计划，并且紧缩政策将继续。由于国家在重工业上投入了巨额资金，因此几乎没有资金可以投入到农业发展中。因此，除了更有效和合理的管理之外，还需要更大程度的农业合作化以及鼓励生产的积极性。“在目前的条件下，农业生产者合作社是一种增加农业产量的方式，它涉及少量投资并能迅速产生良好的效果。这也是引导农民走向社会主义的必要步骤……必须考虑到个体农户在提高产量方面表现出的热情，并应大力帮助和领导他们充分开发所有资源，并提高单位面积产量，”李富春的报告推断道。²然而，在基层，“大力帮助和领导”的结果与李富春和中国共产党（CCP）领导层设想的截然不同。

在五年计划草案中，为了提高农业产量，李的报告优先考虑了土地开垦、水利建设和精耕细作的方法：“由于我国人口众多，而耕地面积仍然有限，我们必须采取有力的措施，有计划地开垦耕地，以便最终解决农产品短缺的问题……水利工程可以扩大灌溉面积，增加农业产量，并保护作物免受洪水或干旱的破坏。通过水利工程消除或减少自然灾害所带来的农业产量增加和其他好处，在许多情况下，可以在几年内偿还投资的资金……应鼓励农民更加精耕细作，投入更多的肥料，逐步采用更先进的耕作技术……使用改良的畜力犁……大力努力储存和准备肥料……是提高农场产量的有效方法。”³然而，所有这些都比实际实现更容易提出。

第一个五年计划中提出的耕地面积扩大的最低目标是 38,680,000 亩（约 6,372,024 英亩）：“必须尽一切努力并使用一切手段来超越它。一

method is for state farms to reclaim land. Where local conditions permit, all state farms should expand their cultivated area by a reasonable amount. At the same time, wherever there are large tracts of reclaimable wasteland, efforts should be made to reclaim them and systematically build up new state farms. Another method is to use machines and other means to reclaim wasteland and to organize cultivation by pioneer emigrants,' Li announced in his broad Hunan accent. A man with big eyes and bushy eyebrows, Li had skilfully handled the supply lines for the Communist Red Army during the Long March in the 1930s. He was known as a competent manager but a cautious planner. Having envisaged even larger-scale, nationwide land reclamation for the Second Five-Year Plan, he also anticipated the complexity and difficulties of such a grand undertaking. A gradual approach to agricultural development ('in a planned way', in his own words) was central to his report. While urging the state to work closely with local authorities to draw up a good overall plan, he also encouraged them to undertake smaller-scale reclamation projects in this interim period.⁴

On 31 July 1955, twenty-five days after Li Fuchun delivered his report, Chairman Mao Zedong of the CCP delivered a speech titled 'The Question of Agricultural Cooperation' at a meeting attended by provincial-level party secretaries. The chairman's speech scaled up several targets that had been set out in Li Fuchun's report. Mao, eight years senior to Li, had been friends with him since they were middle school students in their native province of Hunan. Later, they had both joined the CCP and journeyed together on the Long March. While Li skilfully managed the supply lines for the Red Army, Mao had led and won many difficult battles. In 1935, on the Long March, Mao rose to become the supreme leader of the CCP. As the leader of the new China, which he had fought for and won, Mao had little interest in the detailed planning and the day-to-day management of the country; it was too tedious for him. Aspiring to the 'great historical experience in building socialism', as modelled by the Soviet Union, for Mao building socialism was a way to continue the revolution. Having led his forces to win many battles, he was ambitious to win this one too. 'Fight no battle that is not well prepared, no battle whose outcome is uncertain,' he urged provincial leaders. Building gigantic industrialized state farms, such as those in the USSR, across the Chinese countryside in the next twelve years (toward the end of the Third Five-Year Plan) with 500 million *mu* of cultivated land

另一种方法是国营农场开垦土地。凡是当地条件允许的，所有国营农场都应在合理的范围内扩大耕地面积。同时，凡是有大片可开垦荒地的地方，都应努力开垦，有计划地建立新的国营农场。李用他那浓重的湖南口音宣布：“另一种方法是利用机器和其他手段开垦荒地，并组织拓荒移民进行耕种”。李身材高大，浓眉大眼，20 世纪 30 年代长征期间，他曾出色地负责共产党红军的补给线。他以精明能干著称，但同时也是一个谨慎的规划者。他预见到第二个五年计划中更大规模、全国范围的土地开垦，他也预料到如此宏大事业的复杂性和困难。循序渐进的农业发展（用他自己的话说，“有计划地”）是他报告的核心。在敦促国家与地方当局密切合作，制定一个良好的总体规划的同时，他也鼓励他们在过渡时期开展小规模の開垦项目。⁴

1955 年 7 月 31 日，在李富春发表报告二十五天后，中共中央主席毛泽东在省委书记参加的会议上发表了题为《关于农业合作化问题》的讲话。主席的讲话提高了李富春报告中提出的几个目标。毛比李年长八岁，自从他们在湖南省的家乡上中学时就已经是朋友。后来，他们都加入了中国共产党，一起走过了长征。当李巧妙地管理着红军的补给线时，毛领导并赢得了许多艰苦的战斗。1935 年，在长征途中，毛泽东升任中共最高领导人。作为他为之奋斗并赢得的新中国的领导人，毛对国家的详细规划和日常管理兴趣不大；这对他来说太乏味了。毛泽东渴望像苏联那样“在建设社会主义方面的伟大历史经验”，对毛泽东来说，建设社会主义是继续革命的一种方式。在带领他的部队赢得许多战斗后，他渴望赢得这场战斗。“不打无准备之仗，不打无把握之仗，”他敦促各省领导人。在未来的十二年里（到第三个五年计划结束时），在中国农村建立像苏联那样的巨大的工业化国营农场，拥有 5 亿亩耕地

(ten times the figure laid down in the First Five-Year Plan), as well as speeding up agricultural co-operativization, was for him a critical step toward the successful outcome of socialist construction.⁵

Mao was also aware that his speech went against the tide. The situation on the ground had been rather ominous. Following the ‘little upsurge’ of agricultural collectivization in the last quarter of 1954 during which more than 600,000 co-operatives were organized, the movement suffered serious setbacks. By the first quarter of 1955, it had reached a crisis point. A few months prior to Mao’s 31 July speech, the failure to organize agricultural co-operatives in Zhejiang province had raised serious alarm at the Central Committee’s Rural Work Department. Reports by government work teams had shown that many middle peasants in Zhejiang province had ganged up and left the co-operatives.⁶ Some killed their livestock to protest co-operativization, and many more retained their night-soil for private use. This action in particular had a serious impact on local schistosomiasis control. In the meantime, a poor harvest as well as the increased amount of grain being appropriated by the state left millions of poor peasants without food. A great number of them turned to local authorities to beg for aid. Responding to the emergency, the policy of ‘drastic compression’ was introduced by the Zhejiang provincial authorities. As a result, 15,000 agricultural co-operatives out of 53,000 were dissolved. In Jiaying region, where schistosomiasis was endemic, the dissolution rates reached a high of 40 per cent.⁷ Amid such confusion, some villages and counties simply reverted to private ownership. As villagers abandoned co-operatives, a good number of villages were left uninhabited. Rumours began to spread, and those peasants who had supported the co-operativization felt betrayed: ‘Chairman Mao no longer wants us to organize co-operatives,’ they cried.⁸

Some top leaders used the situation in Zhejiang as a warning about the speed of agricultural collectivization. Yet Mao was not someone who was easily cowed. He had acquired the habit of conducting investigations on local conditions back in the 1930s while he was a young CCP leader struggling to build up grassroots support. Once again, he took a trip south to study the local situation himself. More important, he wanted to take control of the chaos of ‘mass dissolution’ and put a stop to ‘rightist mistakes’. During Mao’s stay in Shanghai, Ke Qingshi, the first secretary of the Central Party Committee’s Eastern China Bureau, gave him unwelcome news. Because of the fierce opposition of peasants seeking to be ‘free’, up to 30 per cent of grassroots

(是第一个五年计划中规定的数字的十倍)，以及加快农业合作化，对他来说是社会主义建设成功结果的关键一步。⁵

毛也意识到他的讲话逆潮流而动。当时的形势相当不妙。继1954年最后一个季度的农业集体化“小高潮”中组织了60多万个合作社之后，这场运动遭受了严重的挫折。到1955年第一季度，它已经达到了一个危机点。在毛泽东7月31日讲话前的几个月，浙江省未能组织农业合作社引起了中央农村工作部的严重警惕。政府工作队的报告显示，浙江省许多中农联合起来退出了合作社。⁶一些人宰杀牲畜以抗议合作化，更多的人将粪便留作私用。特别是这一行动对当地的血吸虫病控制产生了严重影响。与此同时，歉收以及国家征用的大量粮食使数百万贫困农民没有食物。他们中的很多人转向地方当局乞求援助。为了应对紧急情况，浙江省当局出台了“大幅压缩”的政策。结果，53,000个农业合作社中解散了15,000个。在血吸虫病流行的嘉兴地区，解散率高达40%。⁷在这种混乱中，一些村庄和县干脆恢复了私有制。随着村民放弃合作社，许多村庄无人居住。谣言开始传播，那些支持合作化的农民感到被背叛：“毛主席不再希望我们组织合作社了，”他们哭喊道。⁸

一些高层领导人以浙江的情况为鉴，警告农业集体化速度过快。然而，毛泽东不是一个容易被吓倒的人。早在20世纪30年代，当他还是一个努力建立基层支持的年轻的中国共产党领导人时，他就养成了调查当地情况的习惯。这一次，他再次南下考察当地情况。更重要的是，他想控制“群众解散”的混乱局面，制止“右倾错误”。在毛泽东在上海期间，华东中央局第一书记柯庆施向他报告了一个不受欢迎的消息。由于农民强烈反对，要求获得“自由”，多达30%的基层

cadres in eastern China no longer wanted to practise socialism in the countryside.⁹ Ke's words set Mao on fire: the chairman was ready to strike and turn the tide around. A month later, in June, despite his busy schedule as the paramount leader of a colossal country, Mao returned to eastern China. While in Zhejiang, Mao learned from his bodyguards that schistosomiasis had damaged the health of many working-age male peasants in the region and prevented them from doing agricultural work. One desperately ill patient told Mao's bodyguards: 'If Chairman Mao could find a way to cure us from this deadly disease, he would be a living Buddha, our saviour.'¹⁰

The words 'living Buddha' and 'saviour' of the people went straight to Mao's heart. He already had some awareness of the damage inflicted by schistosomiasis in rural Zhejiang. Some fifteen months previously, Shen Junru, one of modern China's most respected political figures, who was the first president of the Supreme People's Court of the PRC, wrote a letter to Mao detailing the damage caused by a schistosomiasis outbreak he had witnessed during his trip to sweep his ancestral tomb in his native Zhejiang province's Jiaying County.* In the same letter, Shen pleaded with Chairman Mao and the people's government to introduce strict measures to control the disease. Mao passed the letter to Xi Zhongxun (the father of Xi Jinping, the future president of the PRC). Xi was then the minister of propaganda, in charge of cultural and public health work.¹¹ Fifteen months later, however, in the midst of fighting for socialism, Mao envisaged that the battle for the health of ten million Chinese peasants who had been injured by schistosomiasis – the disease was also said to be endangering 1 billion more people living in the countryside – was not merely a matter of political propaganda, it should be on the revolutionary agenda. According to Lenin, 'the fight for socialism is at the same time the fight for health.'¹² In China in 1955, as Mao was to bring forth a socialist storm (in an edited version, this become 'an upsurge in socialist transformation') to sweep across the entire Chinese countryside, he was determined to fight the war against schistosomiasis, whatever the price. For him, ridding the people of this disease that plagued rural China was a necessary step in bringing socialism to the countryside, and his crusade against the disease would be a revolutionary experience that would involve the masses.

* Shen also served as the vice chairman of the Chinese People's Consultative Conference as well as the chairman of the Standing Committee of the People's Congress.

华东地区的干部们不再想在农村实践社会主义。⁹柯庆施的话激怒了毛泽东：毛主席准备发起反击，扭转局面。一个月后，6月，尽管身为一个庞大国家的最高领导人，日程繁忙，毛泽东还是回到了华东地区。在浙江期间，毛泽东从警卫那里得知，血吸虫病损害了该地区许多适龄男性农民的健康，使他们无法从事农业劳动。一位病情危重的病人对毛泽东的警卫说：“如果毛主席能找到一种方法来治愈我们这种致命的疾病，他将是一位活佛，我们的救星。”¹⁰

“活佛”和人民的“救星”这些话直击毛泽东的心房。他已经意识到血吸虫病对浙江农村造成的损害。大约在十五个月前，现代中国最受尊敬的政治人物之一，也是中华人民共和国最高人民法院首任院长沈钧儒写信给毛泽东，详细描述了他在回浙江嘉兴老家扫墓时亲眼目睹的血吸虫病爆发造成的损害。^{*}在同一封信中，沈恳求毛主席和人民政府采取严格措施控制这种疾病。毛泽东将这封信转给了习仲勋（后来的中国国家主席习近平的父亲）。当时习仲勋是宣传部长，负责文化和公共卫生工作。¹¹然而，十五个月后，在为社会主义而战的过程中，毛泽东设想，为一千万被血吸虫病伤害的中国农民的健康而战——据说这种疾病也威胁着居住在农村的另外10亿人——不仅仅是一个政治宣传问题，它应该被纳入革命议程。列宁认为，“争取社会主义的斗争同时也是争取健康的斗争。”¹²1955年，当毛泽东掀起一场社会主义风暴（在修改后的版本中，这变成了“社会主义改造高潮”）席卷整个中国农村时，他决心不惜一切代价打赢这场与血吸虫病的战争。对他来说，摆脱这种困扰中国农村的疾病是把社会主义带到农村的必要一步，而他对这种疾病的讨伐将是一次涉及群众的革命体验。

* 沈还曾担任中国人民政治协商会议副主席和人民代表大会常务委员会主任。

In 1933, the year when his mass military warfare tactics came under heavy criticism from the CCP leadership, Mao, a relatively young local branch Communist leader, was removed from the Communist Party Central Committee's Revolutionary Military Council. Faced with this setback, Mao went to Jiangxi province's Changgang Township, the model township of the Central Soviet Area, for an investigation of local conditions. His detailed investigation showed that in one after another mass campaign, from agricultural production to eradication of illiteracy, army recruitment, and public health work, every villager in Changgang Township was, to one extent or another, involved in revolutionary activities. Mao concluded that Changgang's revolutionary success rested on their programs that represented the interests of the people as well as the leadership methods that involved as many people as possible in the implementation of those programs. 'The revolutionary war is a war of the masses; it can be waged only by mobilizing the masses and relying on them,' Mao declared in summarizing his position. To win the support of the masses, Mao reminded the CCP, cadres must attend to the everyday needs of the people. One such need was health: 'Many people suffer from boils and other ailments. What are we going to do about it? All such problems concerning the well-being of the masses should be placed on our agenda ... We should convince the masses that we represent their interests, that our lives are intimately bound up with theirs. We should help them to proceed from these things to an understanding of the higher tasks which we have put forward, the tasks of the revolutionary war, so that they will support the revolution and spread it throughout the country, respond to our political appeals and fight to the end for victory in the revolution.'¹³

In Zhejiang province in 1955, as Mao gathered information on the severity of schistosomiasis in the countryside, he ordered officials at the Ministry of Health to come to the south to discuss eradicating the disease once and for all. He also gave orders that those who were affected were to be given free treatment. Confident that he had turned the tide, Mao left Hangzhou and went to Changsha in his native province of Hunan. There, disregarding all warnings, he symbolically swam across the River Xiang against the current. While still a young student, Mao had loved the thrill of venturing into the middle of the River Xiang to strike against the rapids. In 1925, pondering the future of the Chinese revolution, he had written 'Remember still / *How* venturing midstream, we struck the waters /

1933年，毛泽东的群众军事战术受到中共领导层严厉批评，这位相对年轻的当地党支部领导人被撤销了共产党中央委员会革命军事委员会的职务。面对这一挫折，毛泽东前往江西省的中央苏区模范乡——长岗乡，调查当地情况。他的详细调查显示，在从农业生产到扫盲、征兵和公共卫生工作的各项群众运动中，长岗乡的每个村民都在不同程度上参与了革命活动。毛泽东总结说，长岗乡的革命成功归功于代表人民利益的各项计划，以及让尽可能多的人参与这些计划实施的领导方法。“革命战争是群众的战争；只有动员群众、依靠群众才能进行，”毛泽东在总结自己的立场时说道。毛泽东提醒中共，要赢得群众的支持，干部必须关心人民的日常需求。其中一个需求就是健康：“许多人患有疖子和其他疾病。我们该怎么办？所有这些关系到群众福祉的问题都应该摆上我们的议事日程……我们应该让群众相信我们代表他们的利益，我们的生活与他们的生活息息相关。我们应该帮助他们从这些事情出发，理解我们提出的更高任务，即革命战争的任务，以便他们支持革命并将其传播到全国，响应我们的政治号召，为革命的胜利奋斗到底。”

¹³

1955年，当毛泽东在浙江省收集关于农村血吸虫病严重程度的信息时，他命令卫生部的官员到南方来讨论彻底根除这种疾病。他还下令对那些受影响的人进行免费治疗。毛泽东确信自己已经扭转了局面，便离开了杭州，前往他家乡湖南省的长沙。在那里，他不顾一切警告，象征性地逆流游过了湘江。毛泽东年轻时，就喜欢冒险进入湘江中心，冲击激流。1925年，在思考中国革命的未来时，他写道：“还记得 / 如何冒险到中流，我们拍击水面 / ”

And waves stayed the speeding boats.' On 31 July 1955, after having furiously criticized colleagues who cautioned him to slow down the speed of agricultural collectivization, Mao delivered his speech on the 'Question of Agricultural Cooperation'. In addition to scaling up the targets set by the State Planning Commission in the First Five-Year Plan, Mao ended by announcing 'an upsurge in socialist transformation will soon come about all over the Chinese countryside.' He was prepared for an aggressive assault, and no one and nothing would stop him. Straight after the speech, he again went swimming at the Beidaihe summer resort, about 300 kilometres to the east of Beijing. Plunging in as a storm was arriving, Mao exclaimed, 'the storm of socialist transformation will soon come about all over China, we must go deep into the current!'¹⁴ When the Leader swims against the stream, the stream changes direction. On 11 October, the Central Committee of the CCP adopted a decision calling for the completion of elementary co-operatives by the spring of 1958.

ERADICATING SCHISTOSOMIASIS WITHIN SEVEN YEARS'

In the next few months, 'the storm of socialist transformation' in the shape of agricultural collectivization swept across the entire countryside. In November, Mao was back in his favourite city, Hangzhou, to meet with provincial leaders from southern China and reveal his draft Articles on Agricultural Work. This time, he put schistosomiasis control on the agenda for discussion and invited the deputy minister of health, Xu Yunbai, to join the discussion. In Hangzhou, Mao preached that the primary aspect of socialist transformation was to improve economic well-being: the living conditions as well as the physical and cultural health of the rural population. Because there had been a rapid advance in developing agricultural co-operatives, the eradication of diseases such as schistosomiasis that were harming the health of rural peasants needed to catch up with the speed of socialist transformation. He commanded that eradicating schistosomiasis would become a political undertaking to which every local party organization was to give 'all-round' leadership. Of equal importance, he told them that this would be a mass campaign that was to involve as many people as possible.¹⁵ Although not everyone at the meeting shared Mao's utopian vision, having lived under the constant strain of 'watching out for the wolf in front and the

“浪遏飞舟。”1955年7月31日，毛泽东严厉批评那些劝他放慢农业集体化速度的同事后，发表了关于《农业合作化问题》的讲话。除了扩大国家计划委员会在第一个五年计划中设定的目标外，毛泽东最后宣布“社会主义改造高潮很快就会在整个中国农村到来。”他已经准备好进行一场猛烈的攻击，没有人，也没有任何事物能够阻止他。讲话结束后，他又去了北京以东约300公里的北戴河避暑胜地游泳。当暴风雨来临时，他纵身跳入水中，高呼：“社会主义改造的暴风雨很快就会席卷中国，我们必须深入到潮流中去！”¹⁴当领导人逆流而上时，潮流就会改变方向。10月11日，中共中央通过一项决议，要求在1958年春天前完成初级合作社。

在七年内根除血吸虫病

在接下来的几个月里，以农业集体化形式出现的“社会主义改造风暴”席卷了整个乡村。11月，毛泽东回到他最喜欢的城市杭州，会见了华南地区的省级领导，并公布了他的《农业工作条例（草案）》。这一次，他将血吸虫病防治列入讨论议程，并邀请卫生部副部长徐运北参加讨论。在杭州，毛泽东宣称，社会主义改造的首要方面是改善经济福祉：农村人口的生活条件以及身心健康和文化健康。由于发展农业合作社取得了迅速进展，因此，根除血吸虫病等危害农村农民健康的疾病，需要赶上社会主义改造的速度。他命令根除血吸虫病将成为一项政治事业，每个地方党组织都要给予“全面”领导。同样重要的是，他告诉他们，这将是一场群众运动，要尽可能多地让人民参与。¹⁵虽然并非会议上的每个人都赞同毛泽东的乌托邦式愿景，但由于他们一直生活在“提防前狼”的持续压力下，

tiger behind' during the New 'Three Anti'-Campaign for the past two years, some had hoped that a 'storm of socialist transformation' would allow them to release all the nervous tension that had built up. Another focused campaign would also mean a chance for upward mobility if they managed to win the 'lottery' of meeting the eradication target. The decision was reached that schistosomiasis control, as well as the control of other 'local diseases' harmful to rural villagers and livestock – such as filariasis, bubonic plague, encephalitis, cattle plague, and hog cholera – would be included in the Seventeen Articles of Agricultural Work. This would be implemented across the entire country.¹⁶ As soon as Mao obtained the 'full commitment' from regional leaders, he wasted no time in sending Xu Yunbai to Shanghai that same evening to prepare for the first national meeting on schistosomiasis eradication.

Ke Qingshi, the party secretary of the CCP Central Committee Eastern China Bureau, the man who had sided with Mao a few months earlier to put a stop to the 'right conservatism' in the party, was appointed the party boss of the CCP Central Committee's Nine-Man Subcommittee on Schistosomiasis Control, even though he had no experience in medical or health work or in leading them. Bo Yibo,* then China's minister of finance and the chairman of the State Planning Commission, remarked that Ke Qingshi was someone who always knew the right thing to say to Chairman Mao. It could be said that in the storm of socialist transformation that swept across the Chinese countryside in the second half of 1955, there was no one Mao trusted more than Ke to push forward a nationwide mass crusade against schistosomiasis, that 'enemy of socialism' from the natural world.¹⁷ In Mao's heart, Ke was the person who could stand up to bureaucratism and answer only to the chairman himself. Suddenly, what had been a problem of public health to be combatted by trained experts in the field came to be seen as a clearly political undertaking: the people's health was and would be a focus for the utopian promises that were inherent in the new state's political ideology. A healthy people defined the perfect state. The Nine-Man Subcommittee was to take over the Ministry of Health's power over schistosomiasis control, since Mao had intended the eradication of schistosomiasis as a political undertaking to be led by party authorities, not technocrats. In this way, the Anti-Schistosomiasis Campaign

* The father of the disgraced politician Bo Xilai.

在过去两年里,新的“三反”运动中,有些人一直处于“老虎追赶”的状态,他们希望一场“社会主义改造风暴”能让他们释放所有积聚的紧张情绪。如果他们设法赢得实现根除目标的“彩票”,那么另一场重点运动也将意味着向上流动的机会。最终决定将血吸虫病防治以及其他对农村村民和牲畜有害的“地方病”——如丝虫病、鼠疫、脑炎、牛瘟和猪霍乱——纳入农业工作的十七条中。这将在全国范围内实施。¹⁶毛泽东一旦获得地方领导人的“完全承诺”,他便立即派徐运北当晚前往上海,准备召开第一次全国血吸虫病防治会议。

中共中央华东局书记柯庆施,几个月前曾与毛泽东站在一起,制止党内的“右倾保守主义”。尽管他没有医学或卫生工作经验,也没有领导它们的经验,但他还是被任命为中共中央防治血吸虫病九人小组的负责人。薄一波*,当时的中国财政部长兼国家计划委员会主任,评论说,柯庆施总能对毛主席说出正确的话。可以说,在1955年下半年席卷中国农村的社会主义改造浪潮中,在推动一场全国性的群众性运动,对抗来自自然界的“社会主义敌人”——血吸虫病方面,没有人比柯庆施更受毛泽东的信任了。¹⁷在毛泽东心中,柯是可以对抗官僚主义,只对主席本人负责的人。突然,原本是由该领域训练有素的专家来对抗的公共卫生问题,被视为一项明确的政治事业:人民的健康是并将成为新国家政治意识形态中固有的乌托邦承诺的焦点。一个健康的人民定义了一个完美的国家。九人小组将接管卫生部对血吸虫病防治的权力,因为毛泽东打算将血吸虫病的根除作为一项政治事业,由党的机关来领导,而不是由技术官僚来领导。通过这种方式,防治血吸虫病运动

* 那位声名狼藉的政治人物薄熙来的父亲。

would not to be hindered by ministerial bureaucracy; the expert had been demoted to a subordinate role in this political attempt to improve the people's health.

On 22 November 1955, although the chairman was not present at the first national meeting to kick-start the nationwide Anti-Schistosomiasis Campaign, Ke Qingshi spoke to the ninety delegates from all the affected regions of southern China and made sure that 'Chairman Mao' and 'the Party' were constantly invoked. 'We must eradicate the disease within seven years (at the completion of the Second Five-Year Plan). This is the task entrusted to us by Chairman Mao and the Party,' Ke proclaimed. To win his points, Ke relied on political rhetoric, not expertise. According to him, the Anti-Schistosomiasis Campaign was an integral part of the chairman's and the CCP's plan to realize a specific socialist way of life in the Chinese countryside by transforming rural villages into a disease-free 'socialist garden' overflowing with 'socialist culture'. Hence, 'in the seven years, in addition to eradicating diseases, we would eradicate illiteracy too. Each villager would learn to read 1,500–2,000 words,' he announced.¹⁸

For the first time, the term 'eradicating' replaced 'control'. Yet Ke, as well as the Nine-Man Subcommittee, were either unaware of or overlooked the magnitude of the challenge. Among the data from the thirteen provinces where the disease was endemic, they learned that there were 224 endemic counties and that around 10 million people were infected. The same data also suggested that the severity of the disease in eighty-four of these counties was questionable. More worrying, no data was collected from Shanghai County, one of the worst affected regions. Furthermore, local reports from different parts of the country showed that the majority of those testing positive showed few or no symptoms. Those affected did not see the point of treatment, never mind eradicating the disease. Even the incomplete data that was available to them did not seem to deter Ke. Because the chairman had depicted public health as an integral part of the revolutionary war of the masses, it could only be won by involving the masses. To win over the masses would involve propaganda: 'I used to hear people talking about big bellies, but I never saw one. When I saw photographs of villagers with big bellies, I got frightened. Let's make films about the harm of schistosomiasis to wake people up and to warn them that the enemy is very canny ... To eradicate the disease, we must involve the masses as well as

不会受到部门官僚主义的阻碍；在这场旨在改善人民健康的政治尝试中，专家已被降级为从属角色。

1955年11月22日，尽管主席没有出席首次启动全国血吸虫病防治运动的全国会议，但柯庆施向来自中国南方所有受影响地区的90名代表发表了讲话，并确保不断提及“毛主席”和“党”。“我们必须在七年内（在第二个五年计划完成时）根除这种疾病。这是毛主席和党委托给我们的任务，”柯庆施宣称。为了赢得他的观点，柯庆施依靠政治辞藻，而不是专业知识。他认为，血吸虫病防治运动是主席和中国共产党将农村村庄转变为没有疾病的“社会主义花园”，并充满“社会主义文化”，从而在中国农村实现一种特定的社会主义生活方式的计划的组成部分。因此，“在这七年内，除了根除疾病外，我们还要消除文盲。每个村民都要学会读1500-2000个字，”他宣布道。¹⁸

“根除”一词首次取代了“控制”。然而，柯庆施和九人小组委员会要么没有意识到，要么忽略了挑战的巨大规模。从该疾病流行的13个省份的数据中，他们了解到有224个流行县，约有1000万人受到感染。同样的数据也表明，其中84个县的疾病严重程度值得怀疑。更令人担忧的是，没有收集到来自受影响最严重的地区之一上海县的数据。此外，来自全国不同地区的当地报告显示，大多数检测呈阳性的人几乎没有或没有症状。受影响的人没有看到治疗的意义，更不用说根除这种疾病了。即使他们掌握的不完整数据似乎也没有阻止柯庆施。由于主席将公共卫生描述为群众革命战争不可分割的一部分，因此只有通过群众参与才能赢得这场战争。赢得群众的支持将涉及宣传：“我过去常听人们谈论大肚子，但我从未见过。当我看到村民们大肚子的照片时，我很害怕。让我们制作关于血吸虫病危害的电影，以唤醒人们，并警告他们敌人非常狡猾……要根除这种疾病，我们必须让群众参与进来，并

science. We must combine mass mobilization with mass science ... By next year, 80 per cent of peasants would join agricultural cooperatives. That would make it easier for us to mobilize the masses to join the battle [against schistosomiasis] ... Science is not to involve a few scientists. Science is to involve people to come up with innovations.' But how? And in what form? Ke Qingshi had envisioned the eradication of schistosomiasis as a propaganda war that could be won through exhortation. He gave provincial leaders three goals for its implementation: be optimistic; identify successful models; and make local plans. Following his boss, Wei Wenbo, the deputy party head of Shanghai municipal as well as of the Anti-Schistosomiasis Campaign, drew up an eradicating timetable of 'Two years of preparation; Four years of fighting; Two years of cleaning up'. In addition, the Nine-Man Subcommittee sent delegates away with a regular schedule of meetings (one each month at the county level and one every four months at the provincial and national levels) to discuss, review, and report on the progress of the campaign. To help the provincial authorities reach the goal of eradication, Wei Wenbo also urged them to utilize folk medical practices, or Traditional Chinese Medicine (TCM) as such practices had now become known.¹⁹ This was the first time the incorporation of TCM had been emphasized at a high level. The abandonment of the 'expert society' simultaneously meant an abandonment of allopathic medicine as the sole model for the diagnosis and treatment of illness.

TRADITIONAL CHINESE MEDICINE (TCM) AS A REMEDY BUT NOT A CURE

In reality, the practice of combatting schistosomiasis with folk remedies had already begun. Two years previously, in the context of the New 'Three Anti'-Campaign (as discussed in the previous chapter), the heightened risk of accidental death as well as the need to reduce costs had led the Changshan health centre in Zhejiang province to abandon treating patients with sodium antimony tartrate. Instead, they involved some local physicians who treated severely ill villagers with a folk remedy made of the herb *veronicastrum axillare*, which grows abundantly in southern China. This had fewer negative side effects than sodium antimony tartrate, and it did relieve symptoms in those severely ill of oedema, but it could not cure the disease.²⁰ Experts trained in allopathic medicine, such as Su Delong, did raise

科学。我们必须将群众动员与群众科学相结合……到明年，80%的农民将加入农业合作社。这将使我们更容易动员群众参加[对抗血吸虫病]的战斗……科学不是要让少数科学家参与，而是要让人民参与创新。’但如何参与？以什么形式参与？柯庆施设想将根除血吸虫病作为一场可以通过劝导赢得的宣传战。他为省级领导人制定了三个实施目标：保持乐观；确定成功的模式；制定当地计划。继他的老板、上海市委副书记兼血吸虫病防治运动负责人魏文伯之后，制定了一个根除时间表，即“两年准备；四年战斗；两年清理”。此外，九人小组委员会派代表参加定期会议（县级每月一次，省级和国家级每四个月一次），讨论、审查和报告运动的进展情况。为了帮助省级当局实现根除目标，魏文伯还敦促他们利用民间医疗实践，或传统中医（tcm），因为这些实践现在已经为人所知。¹⁹这是中医的融合首次在高层得到强调。“专家社会”的放弃同时意味着放弃对抗疗法作为诊断和治疗疾病的唯一模式。

作为一种补救措施而非治疗手段的传统中医（TCM）

实际上，用民间疗法对抗血吸虫病的做法已经开始了。两年前，在新“三反”运动的背景下（如前一章所述），意外死亡风险的增加以及降低成本的需要导致浙江省常山卫生院放弃用酒石酸锑钠治疗病人。相反，他们请来了一些当地医生，用一种用玄参草药制成的民间疗法治疗重病村民，这种草药在中国南方大量生长。与酒石酸锑钠相比，它的副作用更少，确实可以缓解那些患有水肿的重病患者的症状，但无法治愈这种疾病。²⁰像苏德隆这样接受过对抗疗法训练的专家确实提出了

the shortcomings, as did healers who used herbal medicine. One traditional practitioner in the Shanghai region, for instance, cautioned that ‘herbal remedies are only useful to give a temporary break’.²¹ More problematic was that some remedies also contained highly toxic herbal plants – such as the rhizomes of *Veratrum nigrum*, traditionally used to kill parasites. When widely promoted without careful monitoring of administration and dosage, herbal remedies, purportedly ‘safe’, could sometimes lead to fatalities. In the case of *Veratrum nigrum* roots, it could also cause permanent damage to the central nervous system.²²

Yet the concerns of the experts, as well as the possible health risks, were ignored in the increasing politicizing of the campaign. Prior to 1956, the Ministry of Health, the seat of the public health experts, had been in charge of controlling schistosomiasis. After 1956, the power was handed over to the purely political Nine-Man Subcommittee.²³ As a response to this politicization, the Ministry of Health encouraged the development or rediscovery of folk herbal remedies to combat schistosomiasis – as in Changshan: ‘Changshan’s innovation was of political importance,’ it was argued.²⁴ Abandoning its earlier attitude that labelled traditional healing practices as backward practices of the feudal past, herbal remedies and acupuncture were now advocated as a cultural heritage of the Chinese people. The process had begun a couple of years earlier. In 1954, the Military Commission’s Ministry of Health, the military rival of the Ministry of Health, launched critical attacks on the Ministry of Health for being overly ambitious, lacking leadership and clear goals, and for allowing too much waste, as well as for an overwhelming number of accidents. The Political Affairs Council’s Commission on Cultural and Education was assigned the task of ‘helping’ the Ministry of Health to re-evaluate its work. This allowed some eminent figures from the community of traditional healers, as well as those advocating a new medicine that was distinctively Chinese, to attack the Ministry of Health’s earlier policy to modernize Chinese medicine by synthesizing pluralistic traditional medical practices with bio-medicine (the slogan was to ‘westernize or scientize Chinese medicine and to make Western medicine Chinese’). They criticized such policies as having marginalized traditional medical practices – the national cultural heritage of the Chinese people.²⁵ The event coincided with an outbreak of Japanese encephalitis in the summer of 1954 in Shijiazhuang, 263 kilometres south of Beijing. After failed

草药疗法的使用者也看到了这些缺点。例如，上海地区的一位传统医生告诫说，“草药只能暂时缓解”。²¹更成问题的是，有些药物还含有剧毒的草本植物，比如传统上用于杀死寄生虫的藜芦根茎。如果在大力推广的同时，不对给药和剂量进行仔细的监控，那么据称“安全”的草药有时会导致死亡。以藜芦根为例，它还可能对中枢神经系统造成永久性损害。²²

然而，在运动日益政治化的过程中，专家们的担忧以及可能的健康风险都被忽视了。1956年以前，公共卫生专家所在的卫生部负责控制血吸虫病。1956年以后，权力移交给了纯粹政治性的九人小组。²³为了回应这种政治化，卫生部鼓励开发或重新发现民间草药疗法来对抗血吸虫病——就像在常山一样：“常山的创新具有重要的政治意义，”有人认为。²⁴卫生部放弃了此前将传统疗法视为封建时代落后做法的态度，现在提倡草药疗法和针灸是中国人民的文化遗产。这个过程在几年前就开始了。1954年，军委卫生部（卫生部在军事上的竞争对手）对卫生部发起了严厉的批评，指责卫生部野心勃勃、缺乏领导和明确的目标，浪费过多，以及事故过多。政务院文教委员会被指派“帮助”卫生部重新评估其工作。这使得一些来自传统医生界的知名人士，以及那些提倡一种具有鲜明中国特色的新医学的人，能够攻击卫生部早先的政策，即通过将多元化的传统医疗实践与生物医学相结合来使中国医学现代化（口号是“西化或科学化中国医学，使西方医学中国化”）。他们批评这些政策边缘化了传统医疗实践——中国人民的民族文化遗产。²⁵这一事件恰逢1954年夏天石家庄（北京以南263公里）爆发日本脑炎之后，在失败之后

attempts to control the outbreak using penicillin, a local herbal doctor suggested using White Tiger soup, a decoction made of gypsum *Fibrosus*, *Rhizoma anemarrhenae*, Radix Glycyrrhizae Preparata, and non-glutinous rice that had been used in China for more than 2,000 years to treat the early stage of acute infection with systemic inflammation. The decoction did help to reduce the number of casualties.²⁶ In response to both events and under the instruction of the Commission on Cultural and Education, the Ministry of Health was obliged to revise its earlier policy. Out of this debate arose a state-defined and certified program of systemized ‘folk medicine’ that was now under the label ‘Traditional Chinese Medicine’ (TCM).²⁷ In 1956, the Ministry of Health sent a team of experts from its new Research Academy of Traditional Chinese Medicine in Beijing, the first state-funded institution for TCM research, to Zhejiang province’s Jiaxing County to experiment with using acupuncture and herbal remedies to treat schistosomiasis. Although these experts had never encountered a single schistosomiasis case before, the experiment did bear some fruit, and it helped to popularize the use of acupuncture to cut down the adverse effect of sodium antimony tartrate.

The local authorities in Jiaxing welcomed the call to engage traditional practices in schistosomiasis control for practical reasons, as did the authorities in some other regions, such as the provinces of Jiangsu and Anhui. In this new and more radical phase of schistosomiasis control, developing or rediscovering existing local practices would help to reduce costs and win the support of traditional healers, especially since the State Council and the State Planning Commission made it clear that austerity was to continue. In Zhejiang province, for instance, after the number of schistosomiasis control cadres had been reduced in the New ‘Three Anti’-Campaign two years previously, there was a severe shortage of personnel to lead the mass campaign on the ground. This shortage was further exacerbated by the simultaneous launching of other public health campaigns, such as killing the ‘four pests’ (除四害) in 1958.* In December 1955, a few days

* The campaign aimed at removing disease vectors such as rats, flies, mosquitos, and sparrows, but it had devastating, unintended consequences because it upset the natural ecological balance. After sparrows were slaughtered, for example, locusts drove in and devoured the fields of grain, paving the way for the famine that claimed tens of millions of lives. For further readings, see Rebecca Kreston, ‘Paved with Good Intentions: Mao Tse-Tung’s “Four Pests” Disaster’, *Discover Magazine Blogs*, <http://blogs.discovermagazine.com/bodyhorrors/2014/02/26/mao-four-pests-china-disease/#.XKWxSKR7kaE>; Frank

在使用青霉素控制疫情的尝试中，一位当地的草药医生建议使用白虎汤，一种由石膏 *Fibrosus*、*Rhizoma anemarrhenae*、炙甘草和糯米制成的汤剂，这种汤剂在中国已经使用了 2000 多年，用于治疗伴有全身炎症的急性感染的早期阶段。这种汤剂确实有助于减少伤亡。²⁶ 针对这两起事件，并在文化教育委员会的指示下，卫生部不得不修改其先前的政策。在这场辩论中，产生了一个由国家定义和认证的系统化“民间医学”项目，现在被称为“中医药”(tcm)。²⁷ 1956 年，卫生部从北京新成立的中医研究院（第一个由国家资助的 tcm 研究机构）派遣了一个专家组到浙江省嘉兴县，试验用针灸和草药治疗血吸虫病。虽然这些专家以前从未遇到过一例血吸虫病病例，但这项实验确实取得了一些成果，并有助于推广使用针灸来减少酒石酸锑钠的副作用。

嘉兴地方当局以及江苏和安徽等其他地区的当局出于实际原因，欢迎呼吁将传统做法纳入血吸虫病防治工作中。在这个新的、更为激进的血吸虫病防治阶段，发展或重新发现现有的地方做法将有助于降低成本并赢得传统治疗师的支持，特别是因为国务院和国家计划委员会明确表示将继续实行紧缩政策。例如，在浙江省，在两年前的新“三反”运动中减少了血吸虫病防治干部人数之后，严重缺乏领导基层群众运动的人员。由于同时发起了其他公共卫生运动，例如 1958 年的“除四害”运动，这种短缺进一步加剧。^{*} 1955 年 12 月，几天

* 这场运动旨在清除老鼠、苍蝇、蚊子和麻雀等疾病媒介，但由于破坏了自然的生态平衡，造成了灾难性的、意想不到的后果。例如，在麻雀被屠杀后，蝗虫涌入并吞噬了田里的谷物，为夺去数千万人生命的饥荒铺平了道路。更多阅读材料，请参见 Rebecca Kreston 的《好心办坏事：毛泽东的“四害”灾难》，*Discover* 杂志博客，<http://blogs.discovermagazine.com/bodyhorrors/2014/02/26/mao-four-pests-china-disease/#.XKWxSKR7kaE> ; Frank

after the campaign to eradicate schistosomiasis began, the Ningpo regional health authority in Zhejiang province was bombarded by requests from counties and towns for more personnel: 'We need more schistosomiasis cadres.' Yet the local personnel department had no budget to increase the staff quota, and they had little alternative but to turn to traditional practitioners for help.²⁸ Herbal therapies did not require patients to take time off from working in the fields; thus, cadres in the agricultural co-operatives, as well as sick villagers, preferred them to hospital or clinic-based treatment. In regions where villagers had little contact with or no access to allopathic medicine, they were more willing to be treated by local healers, since they trusted them. As a result, and somehow unintended, traditional practices flourished, as did grassroots health workers. Although it quickly became clear that herbal remedies would do little to help achieve the goal of eradicating schistosomiasis, these health auxiliaries, some of whom were herbal medicine healers or who practised traditional techniques such as acupuncture, became indispensable in carrying out the campaign as well as further public health work on the ground.²⁹ They not only relieved local cadres of the burden of time-consuming public health work, but, more important, they were essential in delivering health care to the villagers, thus reducing the health care costs of the co-operatives, since the health workers were either paid directly or were reimbursed with work points. These grassroots health auxiliaries would eventually serve as the model for the 'Barefoot Doctor' program in the 1960s and 1970s.

The initial euphoria over folk remedies and acupuncture, however, quickly exhausted itself because they not only failed to deliver a cure but had almost zero impact on reaching the goal of eradication. In Zhejiang province, in the heavily infected villages of the coast, there remained some residual resistance to TCM among villagers who had gradually built up a belief in the power of allopathic medicine that was due, in part, to the efforts of the Western medical missionaries prior to Communist rule but also to the persistent public health propaganda in the PRC before 1954 that prejudiced traditional practices as 'feudal' or 'superstitious' practices of the past. With their ears full of testimony such as: 'In the old China we had no money to treat disease, so we prayed to Buddha and drank folk remedies. Now

Dikötter, *Mao's Great Famine: The History of China's Most Devastating Catastrophe*, 1958-62 (London: Bloomsbury 2010), 187-6.

在血吸虫病防治运动开始后, 浙江省宁波地区卫生局收到了来自各县镇要求增加人员的请求: “我们需要更多的血吸虫病干部。”然而, 当地人事部门没有增加人员编制的预算, 他们别无选择, 只能求助于传统从业者。²⁸ 草药疗法不需要患者请假离开田地工作; 因此, 农业合作社的干部以及患病的村民更喜欢草药疗法而不是医院或诊所的治疗。在村民很少接触或无法获得对抗疗法的地区, 他们更愿意接受当地治疗师的治疗, 因为他们信任他们。因此, 在某种程度上是无意的, 传统做法蓬勃发展, 基层卫生工作者也是如此。尽管很快就清楚地表明, 草药疗法对实现根除血吸虫病的目标几乎没有帮助, 但这些卫生辅助人员 (其中一些人是草药医生或 практикуют 针灸等传统技术) 在开展运动以及进一步的基层公共卫生工作中变得不可或缺。²⁹ 他们不仅减轻了当地干部耗时的公共卫生工作负担, 更重要的是, 他们对于向村民提供医疗保健至关重要, 从而降低了合作社的医疗保健成本, 因为卫生工作者要么直接获得报酬, 要么获得工分报销。这些基层卫生辅助人员最终将成为 20 世纪 60 年代和 70 年代的 “赤脚医生” 计划的典范。

然而, 对民间疗法和针灸的最初狂热很快就消退了, 因为它们不仅未能提供治愈方法, 而且对实现根除的目标几乎没有影响。在浙江省, 沿海重度感染的村庄中, 村民们对中医仍然存在一些抵触情绪, 他们逐渐建立了对西医力量的信念, 这部分归功于共产党统治之前西方医疗传教士的努力, 但也归功于 1954 年前中华人民共和国持续的公共卫生宣传, 这些宣传将传统做法视为过去的 “封建” 或 “迷信” 做法。他们耳边充满了这样的证词: “在旧中国, 我们没有钱治病, 所以我们祈求佛祖, 喝民间疗法。现在

Dikötter, 毛泽东的大饥荒: 1958-62 年中国最具破坏性的灾难史 (伦敦: Bloomsbury 2010), 187-6。

the party has sent doctors to cure us', these villagers, too, wanted the 'miraculous' cure of the magic allopathic bullet. Some of them refused folk remedies, seeing them as less effective.³⁰

SETBACKS

Further up the ladder, the authorities soon realized that herbal therapies and acupuncture would not help them in meeting their seven-year eradication target. In the meantime, the plan to educate the public on schistosomiasis prevention through the new medium of film was underway. Upon receiving the edict requesting their participation in the Anti-Schistosomiasis Campaign, the Shanghai Film Studio responded quickly. By January 1956, two short educational films on schistosomiasis prevention were produced and made ready for public viewing. Yet at the grassroots level, very few cadres were willing to organize any viewings, since most of them were pressed for time to lead agricultural production as well the task of reorganizing the newly merged, larger co-operatives. Some cadres sent a note to say they would be away, and others said they had too much work to do. A few used the cold weather and cloudless sky as excuses. Those cadres managing the mobile communities living on rivers and canals around Shanghai complained that the minimal fee of five *fen* (less than one penny) charged for the viewing was too expensive given the poverty of the boat people.³¹ Thus, the films were never shown to their intended audiences.

Although the implementation of mass sanitation work seemed to be made easier with the instituting of agricultural co-operatives, as Ke Qingshi had anticipated, and produced more immediate results, such results were often short-lived. In addition, under severe pressure to involve as many people as possible and get the job done quickly, there was neither the means nor the time to check on the quality of the work. In Zhejiang province's Jiaxing and Shaoxing counties, regions that had piloted the technically sound method of controlling the disease by reducing snail numbers with earth burial – a measure thought to have been carried out in Japan with some success – officials sent to evaluate the alleged success were horrified to find that the majority of the snails survived after these mass burial campaigns. It turned out that the snails were not buried deep enough, nor was the work carried out with the frequency it needed to be to be successful. In some villages, after the snail-infested mud

“党派医生来给我们治病”，这些村民也想要神奇的“特效”西医治疗。他们中的一些人拒绝民间疗法，认为效果较差。³⁰

挫折

更上一层楼，当局很快意识到，草药疗法和针灸无助于他们实现七年消灭目标。与此同时，通过电影这种新媒介对公众进行血吸虫病预防教育的计划也在进行中。收到要求他们参与抗血吸虫病运动的命令后，上海电影制片厂迅速做出回应。到1956年1月，两部关于血吸虫病预防的教育短片制作完成，准备公映。然而，在基层，很少有干部愿意组织任何放映活动，因为他们中的大多数人都忙于领导农业生产以及重组新合并的更大的合作社的任务。一些干部发纸条说他们要离开，另一些干部说他们太多的工作要做。少数人以寒冷的天气和万里无云的天空为借口。那些管理生活在上海周围河流和运河上的流动社区的干部抱怨说，每次观看收取的五分（不到一分钱）的最低费用对于船民的贫困来说太贵了。³¹因此，这些电影从未放映给预期的观众。

尽管如柯庆施所预期的那样，随着农业合作社的建立，大规模卫生工作的实施似乎变得更加容易，并且产生了更直接的效果，但这些效果往往是短暂的。此外，在让尽可能多的人参与并尽快完成工作的巨大压力下，既没有手段也没有时间检查工作质量。在浙江省的嘉兴和绍兴县，这些地区曾试点通过用土掩埋来减少钉螺数量从而控制疾病的可靠方法——据认为在日本已经取得了一些成功——派去评估所谓成功的官员们惊恐地发现，大多数钉螺在这些大规模掩埋行动后仍然存活。事实证明，钉螺埋得不够深，工作频率也不足以使其成功。在一些村庄，在布满钉螺的泥土之后

was cleaned out from old ditches, villagers mobilized to carry out the task were not given any instructions as to what to do with the mud. They dumped the mud randomly in fields as fertilizer or piled it up all over the ground, thus allowing the snails to simply migrate over even greater areas. Similarly, the collecting of night-soil seemed to hit the target, but it could not be used as fertilizer during the busy planting season because it had not been treated to destroy the schistosoma eggs. When used as fertilizer, it simply increased the risk of infection, causing wider outbreaks, but without this fertilizer the farming co-operatives concerned would face the penalty of not meeting the government's production quota – an even greater political risk for the grassroots cadres. In desperation, they purchased fertilizer from elsewhere, including other affected regions. They quickly learned that even this fertilizer was contaminated, but by then it was too late to prevent massive, acute outbreaks.³²

FURTHER COMPLICATIONS

The situation was looking more desperate farther inland in China's southwest, with more and more counties being added to the endemic list. One such county was Xichang and its surrounding villages. A fertile valley situated on the edge of the Himalayas, Xichang, as part of the former Xikang province, had been ruled by the Chinese warlord Liu Kang prior to the Communist Liberation in 1950. The surrounding hill villages were occupied by different Lolo tribes. Growing illegal opium crops for the black market had been the main source of revenue for the local Han Chinese, who had migrated there as early as the eighteenth century with the Manchu military expansion into this region. These Han Chinese rented land from the nomadic Lolos and introduced agriculture to the region. After opium was banned in China, its price shot up, and it became ever more profitable to grow and trade in opium. In the late 1940s, just before the Communist army conquered the region, one kilo of opium could be exchanged for thirty kilos of rice. Therefore, local villagers had little or no incentive to grow rice or to engage in other agriculture activities.³³ After the Communists took over the region, tackling the region's opium problems was the one of the first measures the authorities introduced to consolidate the CCP's power – that and ridding the area of bandits, many of whom were opium lords or traffickers. By the end of 1952, the region was declared opium-free.³⁴

清理旧沟渠时，动员起来执行任务的村民没有得到任何关于如何处理泥土的指示。他们随意地将泥土倾倒在田地里作为肥料，或者将其堆积在地面上，从而使钉螺可以迁移到更大的区域。同样，收集夜粪似乎击中了目标，但由于没有经过处理以破坏血吸虫卵，因此在繁忙的种植季节无法用作肥料。当用作肥料时，它只会增加感染的风险，导致更大范围的爆发，但是如果没有这种肥料，相关的农业合作社将面临无法达到政府生产配额的惩罚——这对基层干部来说是更大的政治风险。绝望之下，他们从其他地方购买肥料，包括其他受影响的地区。他们很快了解到，即使是这种肥料也被污染了，但那时已经太晚了，无法阻止大规模的急性爆发。³²

further complications

在中国西南内陆地区，情况看起来更加令人绝望，越来越多的县被列入地方性流行病名单。其中一个县是西昌及其周边村庄。西昌位于喜马拉雅山脉边缘的一个肥沃山谷中，作为前西康省的一部分，在1950年共产党解放前一直由中国军阀刘康统治。周围的山村居住着不同的彝族部落。为黑市种植非法鸦片作物一直是当地汉族的主要收入来源，早在18世纪，他们就随着满清军队向该地区的扩张而移居到那里。这些汉族人从游牧的彝族人那里租地，并将农业引入该地区。在中国禁止鸦片之后，它的价格飞涨，种植和交易鸦片变得更加有利可图。在20世纪40年代末，就在共产党军队征服该地区之前，一公斤鸦片可以换取30公斤大米。因此，当地村民几乎没有动力种植水稻或从事其他农业活动。³³ 共产党接管该地区后，解决该地区的鸦片问题是当局为巩固共产党权力而采取的首批措施之一——以及清除该地区的土匪，其中许多人是鸦片领主或贩运者。到1952年底，该地区被宣布为无鸦片区。³⁴

However, the price for being opium-free was a devastating famine. Having had their opium confiscated, the opium fields wiped out, and the market closed or pushed underground, local villagers were deprived of their livelihood. With no opium to exchange for food, they resorted to eating grass and earth to fill their empty stomachs.³⁵ The authorities seized the opportunity to launch Land Reform in the region, although this had already been completed in other parts of the country. As part of the government's famine relief, starving local villagers, most of whom were Han Chinese opium farmers who had previously rented plots of land in the hills, were given small loans and seed for growing rice so that they could move down to the valleys around Xichang to begin rice cultivation. In addition, the local authorities helped them to build small irrigation networks and convert the fertile valley into terraced rice paddies. By 1952, the cultivable land in the region had increased by 30 per cent,³⁶ but coupled with this agricultural expansion was the concomitant spread of schistosomiasis. An official epidemiological survey conducted in 1952 found not a single schistosomiasis patient. By 1955, however, Xichang was declared an endemic region, with more than 500 cases of acute infection reported by the local hospital that year.³⁷

On the other side of the hills, in Yunnan province's Dali and Lijiang prefectures, which were inhabited by different ethnic groups, hundreds of cases of *Schistosomiasis japonica* were also reported. Prior to that, there had been warnings about the dangers of the disease. In 1939, a series of malaria epidemics, aggravated by the construction of the Yunnan-Burma Highway, wiped out entire towns in Yunnan province in China's southwest.³⁸ Robert Cecil Robertson, professor of pathology at the University of Hong Kong as well as commissioner for the League of Nations Epidemic Unit operating in central China, was transferred to the region to oversee the matter. While surveying the situation of malaria in Xiaguan city on the highway, Robertson encountered some schistosomiasis patients from villages in the plain along the shore of Dali's Lake Erhai, where the altitude is about 6,900 feet above sea level. This alerted Robertson, since Xiaguan was the most important city for trade on the highway as well as the rice basket of the region. Equally important was that the British were planning to open several transport depots there, so he decided to investigate the matter further. Assisted by a Chinese technician from the South-Western Hospital in Xiaguan, they managed to collect hundreds of molluscs, similar to those found in Fuzhou in

然而, 摆脱鸦片的代价是一场毁灭性的饥荒。当地村民的鸦片被没收, 鸦片田被摧毁, 市场被关闭或转入地下, 他们失去了生计。由于没有鸦片来换取食物, 他们只能靠吃草和泥土来填饱空空的肚子。³⁵当局抓住机会在该地区发起土地改革, 尽管该国其他地区已经完成了这项改革。作为政府的饥荒救济措施的一部分, 饥饿的当地村民, 他们中的大多数是汉族鸦片种植者, 他们以前在山上租用土地, 得到了小额贷款和稻种, 以便他们可以搬到西昌周围的山谷开始种植水稻。此外, 地方当局还帮助他们建造小型灌溉网络, 并将肥沃的山谷改造成梯田。到 1952 年, 该地区的可耕地增加了 30 %, ³⁶但伴随着这种农业扩张的是血吸虫病的相应蔓延。1952 年进行的一项官方流行病学调查没有发现一例血吸虫病患者。然而, 到 1955 年, 西昌被宣布为地方病流行区, 当地医院当年报告了 500 多例急性感染病例。³⁷

在山的那一边, 在云南省的大理和丽江地区, 那里居住着不同的民族, 也报告了数百例日本血吸虫病病例。在此之前, 已经有关于该疾病危险性的警告。1939 年, 一系列疟疾流行病, 因滇缅公路的建设而加剧, 消灭了中国西南部云南省的整个城镇。³⁸香港大学病理学教授、国际联盟驻华中部流行病防治处专员罗伯特·塞西尔·罗伯逊 (Robert Cecil Robertson) 被调到该地区负责此事。在考察公路上的下关市疟疾情况时, 罗伯逊遇到了一些来自大理洱海沿岸平原村庄的血吸虫病患者, 那里的海拔约为海平面以上 6900 英尺。这引起了罗伯逊的警觉, 因为下关是公路上最重要的贸易城市, 也是该地区的粮仓。同样重要的是, 英国人计划在那里开设几个运输站, 所以他决定进一步调查此事。在下关西南医院的一位中国技术人员的协助下, 他们设法收集了数百个软体动物, 类似于在福州发现的那些。

southern China, on 'sloping banks of the irrigation ditches which led to the rice paddies'. Although the data he obtained on the prevalence of schistosomiasis in the region was 'very meagre', some of the Chinese doctors working in the region told him that the disease, often mistaken for malaria, was very common in the villages along Lake Erhai. Although Robertson and his Chinese assistant were unable to visit these villages, they found traces of schistosomiasis as far afield as the foothills, about twenty kilometres to the east of Xiaguan. As a public health commissioner, he was keen that 'the public should be warned about this matter and the employees of the transport companies who use ponds along the highway in this vicinity for washing, domestic purposes or cleaning down cars should realize that the waters of these ponds may be highly infected at certain seasons of the year.'³⁹ Perhaps Robertson's discovery was too much of a puzzle, or perhaps war interrupted his work, but there was no further study of schistosomiasis in the Dali region until 1952 when 246 cases were reported by the Dali People's Hospital, the former Gospel Hospital of the China Inland Mission. The majority of those infected were young people and children who had contracted schistosomiasis while swimming in the lake. After the incident, the local health department in Dali pleaded with the Yunnan provincial health authority for resources to investigate the matter further. A year later, in 1953, the CCP Central Committee South-Western Bureau's Epidemic Prevention Department sent a team to Dali to open the Yunnan Provincial Schistosomiasis Prevention Centre.⁴⁰ They simultaneously opened a Pandora's box, however.

Yunnan province, situated at the border of the Chinese empire, was also a public health periphery. This changed briefly during World War II when Yunnan temporarily became an important global centre for malaria research and control, partly because major medical institutions from the coastal regions of China retreated to the southwest to escape the Japanese and partly because of the efforts of some international health organizations in the wake of the building of the Yunnan-Burma Highway. But with the end of the war in the Pacific and the advent of the Communist takeover of the region, malaria control work in Yunnan ceased. Prior to Communist rule, Yunnan was also a centre for Christian missionary activities, since many of the ethnic groups marginalized by the dominant Chinese culture were more amenable and easier targets for conversion. Western medical missionaries were the main force, competing with

在中国南方，在“通往稻田的灌溉沟渠的倾斜河岸上”。虽然他获得的关于该地区血吸虫病流行的数据“非常少”，但该地区的一些中国医生告诉他，这种疾病常被误认为疟疾，在洱海沿岸的村庄中非常常见。虽然罗伯逊和他的中国助手无法访问这些村庄，但他们在西关以东约 20 公里的山麓地区发现了血吸虫病的踪迹。作为一名公共卫生专员，他渴望“公众应该对此事发出警告，并且使用这条公路沿线池塘进行洗涤、家庭用途或清洗汽车的运输公司的员工应该意识到，这些池塘的水在一年中的某些季节可能受到高度感染。”³⁹也许罗伯逊的发现太过令人困惑，或者战争中断了他的工作，但在 1952 年大理人民医院（前中国内地会福音医院）报告了 246 例病例之前，大理地区没有对血吸虫病进行进一步研究。大多数感染者是在湖中游泳时感染血吸虫病的年轻人和儿童。事件发生后，大理当地卫生部门恳请云南省卫生厅提供资源，以进一步调查此事。一年后，1953 年，中共中央西南局防疫处派出一个团队到大理开设云南省血吸虫病防治中心。⁴⁰然而，他们同时也打开了潘多拉的盒子。

云南省位于中国帝国的边界，也是公共卫生领域的边缘地区。二战期间，这种情况发生了短暂的改变，云南省一度成为全球疟疾研究和控制的重要中心，部分原因是来自中国沿海地区的主要医疗机构为了躲避日本人而撤退到西南地区，部分原因是滇缅公路修建后一些国际卫生组织的努力。但随着太平洋战争的结束和共产党对该地区的接管，云南的疟疾控制工作停止了。在共产党统治之前，云南也是基督教传教活动的中心，因为许多被主流汉文化边缘化的民族群体更容易接受和更容易成为皈依的目标。西方医疗传教士是与

Buddhist institutions and indigenous shamanic healers, in delivering health care to the local population. As the Communists moved to take final control of the area, Buddhist institutions were shut down, and shamanic healers were suppressed. Foreign missionaries as well as lay medical personnel were forced out. Missionary organizations stopped sending money to their mission hospitals, so without funding and personnel, these abandoned hospitals turned into a living hell. Many patients died of hunger, while those who were still alive cried out for succour. Officials sent by the new People's Government for the southwest region were so horrified by what they saw that they 'could not bear the sight' and wept in despair.⁴¹

For the provincial and local health authorities in Yunnan as well as in nearby Xichang, schistosomiasis was one more nightmare that was added to their public health quandary. In addition to the severe shortage of funding and personnel, they had no idea how to control schistosomiasis, a disease that had long been mistaken by the locals for malaria. The majority of cadres employed by the new Schistosomiasis Prevention Centre had only primary school education and did not even know how to fill in a form properly. With no clear instruction on how to collect epidemiological data, they simply made guesses as they went along. Some thought 'the more the better', so they made up the number of infected villagers as well as snail numbers, whereas others did not think it was necessary to examine all the villagers.⁴² Furthermore, the local terrain in both Dali and Xichang was mountainous; this made snail surveys challenging, since snails tended to congregate in small groups concealed in grass or under stones and big trees. Nor were the reservoir hosts for the disease limited to humans. In the mountainous regions of southwestern China, cattle, goats, dogs, and even voles were more important than humans in the spread of schistosomiasis. This meant that the methods used in the lower reaches of the Yangtze and the lake regions along the Yangtze to control the spread of the disease were largely irrelevant or ineffective here. The nomadic population's cultural preference for defecating and urinating in the wild, as well as the overwhelming poverty of the more settled communities, also made sanitation campaigns an impossible task. For instance, after villagers were banned from using fresh night-soil in rice paddies, they stored it in open spaces or in animal barns, since they had no money to buy storage pots or to build latrines. When it rained, the night-soil flooded all over the place, causing an even greater public

佛教机构和当地萨满巫医为当地居民提供医疗保健服务。随着共产党人逐步取得该地区的最终控制权，佛教机构被关闭，萨满巫医受到压制。外国传教士以及非专业的医疗人员被迫离开。传教组织停止向其教会医院提供资金，因此，在缺乏资金和人员的情况下，这些被遗弃的医院变成了人间地狱。许多病人死于饥饿，而那些仍然活着的人则呼吁救助。新成立的西南地区人民政府派来的官员对他们所看到的情景感到震惊，他们“不忍目睹”，绝望地哭泣。⁴¹

对于云南以及邻近西昌的省级和地方卫生部门来说，血吸虫病是他们公共卫生困境中又一个噩梦。除了严重的资金和人员短缺外，他们不知道如何控制血吸虫病，这种疾病长期以来被当地人误认为是疟疾。新成立的血吸虫病防治中心雇用的大多数干部只有小学文化水平，甚至不知道如何正确填写表格。由于没有明确的关于如何收集流行病学数据的指示，他们只是随意猜测。有些人认为“越多越好”，所以他们编造了感染村民的人数以及钉螺数量，而另一些人则认为没有必要检查所有村民。⁴²此外，大理和西昌的当地地形都是山地，这使得钉螺调查具有挑战性，因为钉螺倾向于聚集在隐藏在草丛中或石头和大树下的小群体中。而且，该疾病的储主宿主不仅限于人类。在中国西南部的山区，牛、羊、狗，甚至田鼠在血吸虫病的传播中比人类更重要。这意味着在长江下游和长江沿岸湖区控制该疾病传播的方法在这里基本上是无紧要或无效的。游牧民族在野外大小便的文化偏好，以及更为定居的社区的极度贫困，也使得卫生运动成为一项不可能完成的任务。例如，在禁止村民在稻田中使用新鲜粪便后，他们将其储存在露天场所或动物棚里，因为他们没有钱购买储藏罐或建造厕所。下雨时，粪便到处蔓延，造成更大的公共

health menace.⁴³ In Xikang, to save money on the pesticide '666', as well as having no trained personnel to carry out the spraying, villagers were mobilized to go into the rice paddies to catch snails using their bare hands. Without any protection, many were infected with schistosomiasis.⁴⁴

KILLING SNAILS

April 1956 was the time for the Nine-Man Subcommittee to hold its second national meeting to review the progress of the Anti-Schistosomiasis Campaign. Contrary to the optimistic image painted in the state media, reports put together by the provincial authorities showed a rather bleak picture on the ground. It became clear that relying purely on political will plus the party leadership and mass participation was no guarantee that the snails, the 'enemy of the natural world', would be eliminated. As with any public health program, to successfully control schistosomiasis would require an achievable goal with realistic targets, proper planning, coordination of relevant departments – from water conservation to agricultural and public health – and the financial commitment of the state, as well as the appropriate cultural and economic conditions on the ground. All these were missing, however, from the design of the Maoist schistosomiasis eradication campaign.

From the outset of the campaign in 1956, a number of experts questioned the feasibility of eradicating the disease in seven years. Drawing on the Japanese experience, Huang Mingxin, the vice chair of the Ministry of Health's new Experts Advisory Committee for schistosomiasis control, cast serious doubt on the goal of eradicating the disease: 'Japan's medical science is far more advanced. The affected region there is much smaller, and fewer people are impacted. Japanese scientists have been working on controlling the disease for more than fifty years. Still they have not managed to eradicate it ... Isn't the goal of eradication in seven to twelve years a bit too high for us?' 'How is it possible?' he asked.⁴⁵ Following his reading of the official statement concerning the eradication plan, Su Delong pencilled two words: 'wild boasting'.⁴⁶ He even confronted Chairman Mao on this. Eager to win the 'golden' prize of eradication, however, the Zhejiang Provincial Party Committee paid no attention to the experts' warning. Instead, it shortened the goal of eradication by two years. In other words, Zhejiang would take the lead in

健康威胁。⁴³ 在西康，为了节省农药“666”的费用，并且没有训练有素的人员进行喷洒，村民们被动员到稻田里用手抓蜗牛。在没有任何保护的情况下，许多人感染了血吸虫病。⁴⁴

消灭钉螺

1956年4月是九人小组召开第二次全国会议，审查防治血吸虫病运动进展情况的时候。与国家媒体描绘的乐观形象相反，各省当局汇总的报告显示，实际情况相当黯淡。很明显，仅仅依靠政治意愿加上党的领导和群众参与，并不能保证消除作为“自然界敌人”的钉螺。与任何公共卫生项目一样，要成功控制血吸虫病，需要一个可实现的目标和切合实际的指标，适当的规划，相关部门（从水利到农业和公共卫生）的协调，以及国家的财政投入，以及当地适当的文化和经济条件。然而，在毛泽东时代的血吸虫病根除运动的设计中，所有这些都缺失了。

从1956年运动一开始，许多专家就质疑在七年内根除该疾病的可行性。卫生部新成立的血吸虫病防治专家咨询委员会副主席黄明新借鉴了日本的经验，对根除该疾病的目标表示严重怀疑：“日本的医学比我们先进得多。那里的疫区要小得多，受影响的人也少。日本科学家已经研究控制这种疾病五十多年了。他们仍然没有设法根除它……七到十二年内根除的目标对我们来说是不是太高了？”“怎么可能？”他问道。⁴⁵ 苏德隆在阅读了关于根除计划的官方声明后，用铅笔写下了两个字：“胡吹乱谤”。⁴⁶ 他甚至就此事与毛主席对质。然而，浙江省委渴望赢得根除的“金”奖，对专家的警告置之不理。相反，它将根除的目标缩短了两年。换句话说，浙江将率先

eradicating schistosomiasis by 1960. Reporting on this, a party-inspired editorial in the *Zhejiang Daily News* attacked the experts' questioning as a 'bureaucratic attitude'.⁴⁷ Later, in 1958 during the Great Leap Forward, Su Delong would once again be heavily criticized for questioning the seven-year eradication goal and doubting the effectiveness of TCM remedies. In the PRC, when politics confronted scientific expertise, politics always won.

Yet parasites are apolitical. They defied the politics and thrived. By this time, because of a lack of personnel as well as hospital beds, local sanitation work, and mass therapy in the control model region, Jiaxing was in total disarray. The local department of disease prevention protested that they would never be able to meet the eradication goal.⁴⁸ In the meantime, Madame Li Dequan, on behalf of the Ministry of Health, which had lost the authority over schistosomiasis control work to the Nine-Man Subcommittee, waged a powerful attack on the Anti-Schistosomiasis Campaign for its design flaws. Speaking at the Chinese People's Political Consultative Conference, she stated that 'there have also been many defects in this work. In some areas, there was a lack of understanding of the stupendous and complicated nature of the task, and this gave rise to a feeling of hastiness and of an inclination to belittle the enemy.'⁴⁹

Watching the goal of eradication slipping away, yet holding no winning card against the unseen disease, the Nine-Man Subcommittee had little choice but to come up with proper plans. This time they turned to the experts for help. It was agreed that science had no national boundaries. 'We must take science seriously, be it the scientific achievement of the Soviet or the West ... Science is universal and impartial,' proclaimed Ke Qingshi on behalf of the Nine-Man Subcommittee.⁵⁰ Since the Soviets claimed little knowledge of schistosomiasis control, Soviet scientists could offer no help except for a few visits as a symbolic gesture. The best they could do was to encourage their Chinese colleagues to put more effort into utilizing TCM. In the end, help came from Japan, the only country in the world that had had some success in controlling the disease. Yet, as John Farley points out, Japan's ability to mitigate rather than eliminate the disease had little to do with elaborate control schemes; rather, it was a decrease in agricultural activities as well as improved standards of living and hygiene that had brought it about.⁵¹ In addition, Japanese epidemiological studies showed that natural environmental change also 'resulted in the annihilation of snails' in a number of

到 1960 年根除血吸虫病。为此,《浙江日报》上发表了一篇由党启发撰写的社论,抨击专家们的质疑是“官僚主义态度”。⁴⁷后来,在 1958 年大跃进期间,苏德隆再次因质疑七年根除目标和怀疑中药疗法的有效性而受到严厉批评。在中华人民共和国,当政治与科学专业知识发生冲突时,政治总是获胜。

然而,寄生虫是无政治性的。它们无视政治,蓬勃发展。此时,由于缺乏人员和医院床位,地方卫生工作以及控制模型地区的群众治疗,嘉兴陷入一片混乱。当地疾病预防部门抗议说,他们永远无法实现根除目标。⁴⁸与此同时,李德全女士代表卫生部(卫生部已将血吸虫病防治工作的权力移交给九人小组),对血吸虫病防治运动的设计缺陷发起了猛烈攻击。她在全国政协会议上发言时表示,“这项工作也存在许多缺陷。在一些地区,对这项任务的艰巨性和复杂性缺乏了解,从而产生了一种急躁情绪和一种轻视敌人的倾向。”⁴⁹

眼看消灭血吸虫的目标渐行渐远,却又没有战胜这种无形疾病的制胜法宝,“九人小组”别无选择,只能制定适当的计划。这一次,他们向专家寻求帮助。人们一致认为科学没有国界。“我们必须认真对待科学,无论是苏联还是西方的科学成就……科学是普遍的、公正的,”柯庆施代表“九人小组”宣称。⁵⁰由于苏联声称对血吸虫病的控制知之甚少,苏联科学家除了象征性地进行几次访问外,无法提供任何帮助。他们能做的最好的事情就是鼓励他们的中国同事更加努力地利用中医。最终,帮助来自日本,世界上唯一在控制这种疾病方面取得一些成功的国家。然而,正如约翰·法利指出的那样,日本减轻而非消除疾病的能力与精心设计的控制方案关系不大;相反,是农业活动的减少以及生活和卫生水平的提高促成了这一结果。⁵¹此外,日本的流行病学研究表明,自然环境的变化也导致了“在许多地区蜗牛的灭绝”。

regions in Japan.⁵² So only mitigation, not eradication, took place even under the best of circumstances.

It was Premier Zhou Enlai, the chief administrator of the PRC and the architect of the country's earlier foreign policy, who first approached the Japanese for help. It has been argued that Zhou Enlai was the 'midwife' responsible for turning Chairman Mao's radical visions (the Great Leap Forward, for instance) into concrete policies and delivering on them.⁵³ Zhou was anxious about the disease spreading northward and causing greater menace in further water conservation projects, as well as bearing in mind the agricultural expansion that aimed to increase agriculture output. On 7 October 1955, just over a month before Chairman Mao decreed the eradication of schistosomiasis, Zhou confided to Amrit Kaur, independent India's first minister of health, that schistosomiasis control would be a major public health undertaking in the PRC: 'Schistosomiasis is the most harmful disease in China. It's widespread along the Yangtze River. We are planning to open up more paddy fields in the north [in the Second Five-Year Plan period]. It is possible snails would migrate northwards to harm the population in the north. This is a serious matter. [Eradicating schistosomiasis] must be a major public health undertaking for the Ministry of Health,' he told Kaur.⁵⁴ Zhou Enlai already knew several things: that agricultural work had to be expanded (even though it is not clear that he even believed this) and that it involved risks to the people's health. But he also understood that the government must be seen to be avoiding these risks by every possible means. For Zhou, as for the State Council that he represented, the eradication of schistosomiasis as a major public health undertaking was integral to China's agricultural development, even though he also understood that the latter meant the people's health was at risk. Despite some experts' warning that under the economic and social conditions at the time, the eradication of the snail host was impossible and was even fraught with danger, a few weeks after his meeting with the Indians, at an official meeting to welcome the Japanese medical delegation, Zhou, with the false expectation of eradicating the disease, asked Japanese medical experts to help China eliminate the snails.⁵⁵

On 16 September 1956, at the Eighth Party Congress, Zhou Enlai, in the Report on the Proposal for the Second Five-Year Plan for Developing of the National Economy, announced a planned further increase of agricultural output by 35 per cent in the period between

日本的各个地区。⁵² 因此,即使在最好的情况下,也只能进行缓解,而不能根除。

是中华人民共和国的总理周恩来,也是中国早期外交政策的架构师,他首先向日本人寻求帮助。有人认为,周恩来是负责将毛主席的激进设想(例如大跃进)转化为具体政策并加以实现的“助产士”。⁵³ 周恩来担心这种疾病会向北蔓延,并在进一步的水利工程中造成更大的威胁,并且考虑到旨在增加农业产出的农业扩张。1955年10月7日,就在毛主席颁布根除血吸虫病法令的一个多月前,周恩来向独立的印度第一任卫生部长阿姆里特·考尔透露,控制血吸虫病将是中华人民共和国的一项重大公共卫生事业:“血吸虫病是中国危害最大的疾病。它沿长江广泛分布。我们计划在第二个五年计划期间]在北方开辟更多的稻田[。蜗牛有可能向北迁移,危害北方的人口。这是一件严肃的事情。[根除血吸虫病]必须是卫生部的一项重大公共卫生事业,”他告诉考尔。⁵⁴ 周恩来已经知道了几件事:农业工作必须扩大(即使他不清楚他是否相信这一点),而且这会给人民的健康带来风险。但他也明白,政府必须被视为通过一切可能的手段来避免这些风险。对于周恩来来说,就像对于他所代表的国务院一样,根除血吸虫病作为一项重大的公共卫生事业,是中国农业发展的组成部分,即使他也明白后者意味着人民的健康面临风险。尽管一些专家警告说,在当时的经济和社会条件下,根除钉螺宿主是不可能的,甚至充满了危险,但在与印度人会面几周后,在欢迎日本医疗代表团的官方会议上,周恩来怀着根除疾病的虚假期望,要求日本医疗专家帮助中国消灭钉螺。⁵⁵

1956年9月16日,在党的第八次代表大会上,周恩来在《关于发展国民经济的第二个五年计划的建议的报告》中宣布,计划在以下期间进一步增加35%的农业产量

1958 and 1962. This would entail increasing the extent of water conservation work and land reclamation. In the same report, he also urged authorities to ‘actively popularize our experience in combating schistosomiasis, and eliminate, by periods, by districts, and in a planned way, the most harmful local diseases’.⁵⁶ A few days after the meeting, the Japanese Delegation for the Prevention and Eradication of *Schistosoma Japonicum*, led by Komiya Yoshitaka, the head of the Department of Parasitology at the National Institute of Preventive Hygiene in Japan and an advocate of social medicine (and also a member of the Japanese Communist Party), arrived in the PRC. After spending nearly three months surveying in Jiangsu province in the east and Sichuan province in the southwest, the Japanese experts warned their Chinese hosts about the challenge of controlling the disease in China, let alone eradicating it. The differences between the two countries were massive. In Japan, the principal habitat of vector snails was limited to only five districts with a population of 400,000, whereas in China the endemic *foci* were distributed over twelve provinces with a population of 100 million. In addition to China being vast, the habitat of vector snails in the country was also rich in variety. This meant that the method of controlling vector snails in Japan, where the snails were only found on the margins of small irrigation ditches (except in the marshy areas along rivers) might not be applicable to many parts of China, where snails were also found on the leaves and stems of reeds or on the margins of small streams between mountains. Furthermore, the Japanese observed, the method of night-soil control, only partly successful in Japan, was much more difficult and complicated in China where control was closely related to protection of the water supply sources. In Japan, night-soil control only involved killing parasitic ova or preventing contact with snails, whereas in China, almost all the inhabitants in the endemic regions washed their night-soil containers, provided by local authorities, in the same water that was used for drinking as well as agriculture. Thus, the Japanese concluded, the long-term policy for schistosomiasis control in China should give priority to environmental intervention over physico-chemical methods to control the *oncomelania* snails. Environmental interventions suggested by the Japanese consisted of burying snails in soil, blocking the water margin of creeks with stone, cementing irrigation ditches, and reclaiming and developing marshland. Warning the Chinese that such work would entail serious financial commitments

1958 年和 1962 年。这将需要增加水利工程和土地开垦的范围。在同一份报告中，他还敦促当局“积极推广我们防治血吸虫病的经验，有计划地、分期分批地消除最有害的地方病”。⁵⁶ 会议结束后几天，由日本国立预防卫生研究所寄生虫学部部长小宫义孝率领的日本血吸虫病防治代表团抵达中国。小宫义孝是日本的社会医学倡导者（也是日本共产党党员）。他们在江苏省东部和四川省西南部进行了近三个月的调查后，日本专家警告中国接待方，在中国控制这种疾病是一项挑战，更不用说根除它了。两国之间的差异是巨大的。在日本，钉螺的主要栖息地仅限于五个区，人口为 40 万，而在中国，地方性疫源地分布在十二个省份，人口为 1 亿。除了中国幅员辽阔之外，该国钉螺的栖息地也种类繁多。这意味着在日本控制钉螺的方法可能不适用于中国的大部分地区，在日本，钉螺仅在小型灌溉沟渠的边缘发现（除了沿河的沼泽地区），而在中国，钉螺也在芦苇的叶子和茎上或山间小溪的边缘发现。此外，日本人观察到，夜间粪便控制的方法在日本只取得了部分成功，但在中国要困难和复杂得多，因为控制与保护水源密切相关。在日本，夜间粪便控制只涉及杀死寄生虫卵或防止与钉螺接触，而在中国，地方当局提供的几乎所有地方性居民都用同一水源清洗夜间粪便容器，这些水既用于饮用，也用于农业。因此，日本人得出结论，中国血吸虫病控制的长期政策应优先考虑环境干预，而不是物理化学方法来控制钉螺。日本人建议的环境干预措施包括将钉螺埋在土壤中，用石头堵住小溪的水边，用水泥砌筑灌溉沟渠，以及开垦和开发沼泽地。他们警告中国人，这些工作将需要大量的资金投入

from the state, Komiya concluded that in the long run it would 'pay off when the control work is combined with agriculture, irrigation, and fishery development'.⁵⁷

By claiming that environmental control of schistosomiasis was integral to agricultural development and water conservancy, the timing of this Japanese recommendation was almost perfect. The year 1957 began with the PRC getting ready to launch the Second Five-Year Plan. If China could succeed in combining the elimination of *oncomelania* snails with the development of agriculture, irrigation, and fishery production, as put forward by the Japanese experts, it would mean more land for agricultural cultivation, better irrigation systems, and increased fishery production, thus reaching the goals set in the Second Five-Year Plan. It would be seen to achieve multiple goals with limited resources. In April, on behalf of the State Council, Zhou Enlai decreed that all local authorities in the endemic regions had to include schistosomiasis control in their yearly plan for agricultural and water conservation for 1957. In addition, they should work at promoting research on environmental control of snails, pesticides, and the examination of treatments that included Western medicine as well as TCM remedies. Yet there was no mention of state funding to help local authorities to achieve these goals.⁵⁸ Furthermore, as 1957 moved on, it became clear that there would be no Second Five-Year Plan. Mao Zedong was to march his country into a disastrous Great Leap Forward.

In the Chinese calendar, 1957 was the Year of the Fire Rooster, the year of big tasks and great ambition as well as a year of disturbances. The year began with the Hundred Flower Campaign, which encouraged intellectuals to participate in policy-making. Mao's words 'Letting a hundred flowers bloom' in the arts and 'Hundred schools of thoughts contend' in science brought millions of people out onto the streets. Like fire roosters, they spoke out. They ferociously debated party policies and China's future. But as with the human body (according to ancient Chinese medical theory), excessive fire (heat) causes inflammation that results in pain. The optimism of liberalization was quickly crushed. With the arrival of the blazing summer, the CCP launched the Anti-Rightist Campaign to attack those who spoke out during the Hundred Flower Campaign. Like viruses, it was said, they had crept into the party and multiplied. To defend itself, the CCP used the Anti-Rightist Campaign to 'boot out' those purported 'viruses' that were thought to be threatening

小官得出结论，从长远来看，将防治工作与农业、灌溉和渔业发展相结合将“得到回报”。⁵⁷

通过声称血吸虫病的环境控制是农业发展和水利建设不可分割的一部分，日本的这一建议的时机几乎是完美的。1957年初，中华人民共和国准备启动第二个五年计划。如果中国能够像日本专家提出的那样，将消灭钉螺与农业、灌溉和渔业生产的发展相结合，那就意味着有更多的土地用于农业耕作，更好的灌溉系统和增加的渔业产量，从而达到第二个五年计划中设定的目标。这将被人视为以有限的资源实现多重目标。4月，周恩来代表国务院下令，地方病流行地区的所有地方当局都必须将血吸虫病防治纳入1957年的农业和水利年度计划中。此外，他们应该努力促进对蜗牛环境控制、农药以及包括西药和中药疗法在内的治疗方法的研究。然而，没有提到国家为帮助地方当局实现这些目标提供资金支持。⁵⁸此外，随着1957年的推进，情况变得很明显，不会有第二个五年计划。毛泽东将带领他的国家进入灾难性的大跃进。

在中国农历中，1957年是火鸡年，是承担重大任务和拥有雄心壮志的一年，也是动荡不安的一年。这一年以百花运动开始，该运动鼓励知识分子参与政策制定。毛泽东的“百花齐放”在艺术领域和“百家争鸣”在科学领域的号召，吸引了数百万人走上街头。像火鸡一样，他们大胆发言，激烈辩论党的政策和中国的未来。但就像人体一样（根据中国古代医学理论），过度的火（热）会导致炎症，从而引起疼痛。自由化的乐观情绪很快被粉碎。随着炎炎夏日的到来，中国共产党发起了反右运动，打击那些在百花运动期间敢于直言的人。据说，他们像病毒一样，悄悄潜入党内并迅速繁殖。为了自卫，中国共产党利用反右运动“清除”那些被认为具有威胁的所谓的“病毒”。

the party's existence. The country was turned upside down, with millions ending up in labour camps. The year ended with Chairman Mao Zedong executing the Great Leap Forward, marching his country into catastrophe.*

THE GREAT LEAP INTO SCHISTOSOMIASIS ERADICATION

At the end of 1957, in a drive to transform China into an industrial power, Mao launched the Great Leap Forward. A year earlier, in 1956, severe flooding, typhoons, and then an intense drought had already damaged agricultural output. The Great Leap Forward began with a massive drive to divert the water from the south to irrigate the arid yellow earth in the north as well as to control floods in the south. The colossal Great Leap water conservation project also aimed to power China's rapid industrialization. By January 1958, the official estimate was that more than 100 million people in China had been mobilized to take part in the Great Leap water conservation campaign. Not only were the masses the only tangible resource the party could call on, equally important for the CCP was the promise that to transform the countryside into the socialist garden was a political project that had to involve the entire population. The Great Leap Forward in agriculture would entail a great leap of schistosomiasis control. As a political undertaking, the Anti-Schistosomiasis Campaign would require the collective strength of the masses of peasants. Mass mobilization of the undereducated peasant population, however, required skilled grassroots leadership. Experts and doctors from major medical institutions were dispatched to different parts of the country to carry out snail surveys, treatment work, and the grassroots training of health personnel for the mass campaign. With great haste, 1,400 hospitals and research institutes, as well as control stations, were opened throughout the country, with more than 13,000 schistosomiasis control cadres. In the meantime, 84,000 grassroots health workers and 25,000 local cadres were recruited to lead the campaign on the ground.⁵⁹

More did not mean better, however. It entailed cumbersome administrative structures that often hampered the actual control

* Mao was born in December 1893, the Year of the Snake. According to the Chinese Zodiac, he was a black water snake, which becomes dangerous when disturbed by fire.

党的存在。这个国家被搞得天翻地覆，数百万人最终进了劳动营。年底，毛泽东主席发动了大跃进，带领他的国家走向灾难。*

大跃进 SCHISTOSOMIASIS ERADICATION

1957 年底，为了将中国转变为工业强国，毛泽东发动了大跃进。一年前的 1956 年，严重的洪水、台风，以及随之而来的严重干旱，已经破坏了农业生产。大跃进开始于一项大规模的行动，将水从南方引到北方干旱的黄土地区进行灌溉，并控制南方的洪水。这项规模巨大的大跃进水利工程也旨在为中国快速的工业化提供动力。到 1958 年 1 月，官方估计中国有超过 1 亿人被动员起来参加大跃进水利建设运动。群众不仅是党可以依靠的唯一有形资源，同样重要的是，对于中国共产党来说，将农村改造为社会主义花园的承诺是一项必须全体人民参与的政治项目。农业方面的大跃进将带来血吸虫病防治工作的大跃进。作为一项政治事业，血吸虫病防治运动需要广大农民群众的集体力量。然而，大规模动员受教育程度低的农民人口需要熟练的基层领导。来自主要医疗机构的专家和医生被派往全国各地，进行蜗牛调查、治疗工作，以及为群众运动进行基层卫生人员培训。各地迅速开设了 1400 家医院和研究所，以及防治站，拥有超过 13000 名血吸虫病防治干部。与此同时，招募了 84000 名基层卫生工作者和 25000 名地方干部，领导当地的运动。

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然而，更多并不意味着更好。它需要繁琐的行政结构，这往往会妨碍实际的控制。

* 毛泽东生于 1893 年 12 月，蛇年。根据中国十二生肖，他是一条黑水蛇，受到火的干扰时会变得危险。

work. For instance, by failing to understand that schistosomiasis was a seasonal disease, as treatment should also be, the newly opened schistosomiasis control hospital in Jiaying overestimated the number of staff and beds needed. Since hardly any patients turned up in the winter, the more than 1,000 hospital staff members had little to do. Slacking became commonplace. The hospital administration reacted by reducing the number of staff and beds. In the meantime, the Great Leap Forward in agriculture led to increased agricultural activities, which, in turn, dramatically increased the prevalence and morbidity of schistosomiasis. When the spring arrived, the season of outbreaks, the number of villagers infected by the disease grew precipitously, and suddenly the number of patients far exceeded hospital capacity. As a result, all useable rooms in the villages, including villagers' homes, were used as temporary treatment spaces. Without beds, patients were asked to bring their own quilts and to lie on the grass. The remaining medical staff often worked more than ten hours a day kneeling on the ground to provide treatment to sick villagers. Short of medical equipment, they often had to come up with rudimentary alternatives and cut corners by compromising treatment or simply sending patients away.⁶⁰

The pressure from above to hurry the Great Leap in eradicating the disease was felt throughout the country. In Yunnan province's Dali and Fengyi region and in Sichuan province's Xichang, for example, the newly opened schistosomiasis control stations quickly filled their vacancies with enthusiastic local youngsters who had just completed primary or middle school education and with women, who had fewer farming duties. While they were useful, often talented, and played a key role in mobilizing the masses for the snail elimination work, they lacked the necessary technical skills. Most of them took only a week-long schistosomiasis control crash course. Despite their commitment (they often worked long shifts and skipped meals), the quality of the work was generally poor. In Dali and Fengyi, for instance, after announcing that they had practically eradicated schistosomiasis with repeated 'assaults' on snails, teams of experts who went to inspect their 'success' reported that an equal number of snails remained alive as had been killed.⁶¹

The Great Leap marching forward at an ever-accelerating speed put even greater strain on skilled personnel. Nationwide construction projects (e.g., transport and telecommunication), water conservancies, iron and steel works, and agricultural production constantly

工作。例如，嘉兴新成立的血吸虫病防治医院未能理解血吸虫病是一种季节性疾病，因此治疗也应该是季节性的，它高估了所需的工作人员和床位数。由于冬天几乎没有病人，1000多名医院工作人员几乎无事可做。偷懒变得司空见惯。医院管理部门通过减少工作人员和床位数来应对。与此同时，农业上的大跃进导致农业活动增加，进而大大增加了血吸虫病的流行和发病率。当春天到来，爆发季节来临时，感染该疾病的村民人数急剧增加，突然病人人数远远超过了医院的容量。结果，村里所有可用的房间，包括村民的家，都被用作临时治疗场所。由于没有床位，病人被要求自带被褥，躺在草地上。剩下的医务人员经常每天工作超过十个小时，跪在地上为生病的村民提供治疗。由于医疗设备短缺，他们经常不得不提出简陋的替代方案，并通过损害治疗或简单地遣送病人来偷工减料。

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来自上级的压力，要求加快大跃进中根除这种疾病的步伐，这在全国各地都能感受到。例如，在云南省的大理和凤仪地区以及四川省的西昌，新开设的血吸虫病防治站迅速招满了热情的当地年轻人，他们刚刚完成小学或中学教育，还有承担较少农活的妇女。虽然他们很有用，通常也很有才华，并且在动员群众进行灭螺工作中发挥了关键作用，但他们缺乏必要的技术技能。他们中的大多数人只参加了一周的血吸虫病防治速成班。尽管他们很投入（他们经常长时间轮班工作，甚至不吃饭），但工作质量普遍较差。例如，在大理和凤仪，在宣布他们通过反复“袭击”钉螺几乎根除了血吸虫病之后，前往检查他们“成功”的专家组报告说，仍然存活的钉螺数量与被杀死的数量一样多。⁶¹

大跃进以不断加快的速度向前迈进，给技术人员带来了更大的压力。全国性的建设项目（如交通运输和电信）、水利工程、钢铁厂和农业生产不断

competed for trained personnel to lead their efforts and to manage the masses. All state organs, public institutions, and agricultural co-operatives were advised to practise strict economy to make full use of manpower, materials, and money. Local authorities had to curtail unnecessary expenditures and personnel. They constantly weighed their immediate interests against the long-term interests of the collectives. ‘Questions of Improving the Health of People’ was one of the last items on the State Council’s agenda. Thus, on-the-ground health work was seen as the least important of their multitude of tasks.⁶² Health care delivery was debated at the very moment when large numbers of grassroots public health personnel were being transferred to support the other ‘more important’ projects intended to transform the country into an industrial powerhouse. In Sichuan province’s Meishan County, one of the worst endemic regions in the southwest, for instance, after the nationwide steel and iron production campaign began in September 1958, many local schistosomiasis control cadres were transferred to support the expanding steel and iron production. In the meantime, pandemics such as measles, whooping cough, diphtheria, and other infectious diseases became rampant at irrigation construction sites, steel and iron production camps, and the people’s communes (collective farms). As a result, the public health priorities turned to fighting these epidemics and their prevention. There were no spare personnel to lead the mass campaign against schistosomiasis. Schistosomiasis control work became next year’s task or was simply removed from the list of local public health work completely.⁶³ In Hunan province’s endemic Yueyang region, ten schistosomiasis control cadres were transferred to other departments. The director of the local hospital multitasked in order to lead the local eradication campaign, but overburdened with too much work, he was unable to focus. The absent leadership resulted in diminished activities in the local schistosomiasis control.⁶⁴

On the ground, grassroots cadres not only were burdened by multiple tasks, they also had unobtainable goals. As result, choices constantly had to be made regarding what to do first, depending on what the priorities were at any given moment. Since public health work such as killing snails or treating sick villagers required much time and effort, local cadres often saw them as a bother or hindrance to their winning medals in the race for the Great Leap Forward. In Jiaxing, for instance, Zhejiang province’s schistosomiasis control flagship county, at a meeting to pledge mass effort to eradicate

竞相争夺训练有素的人员来领导他们的工作并管理群众。建议所有国家机关、公共机构和农业合作社厉行节约，充分利用人力、物力和财力。地方当局必须削减不必要的开支和人员。他们不断权衡眼前利益与集体长远利益。“改善人民健康问题”是国务院议程上的最后一项。因此，基层卫生工作被认为是他们众多任务中最不重要的。⁶² 卫生保健服务的讨论正值大量基层公共卫生人员被调走，以支持其他旨在将国家转变为工业强国的“更重要的”项目。例如，在四川省西南部最严重的流行病区之一眉山县，在1958年9月全国钢铁生产运动开始后，许多地方血吸虫病防治干部被调去支持不断扩大的钢铁生产。与此同时，麻疹、百日咳、白喉和其他传染病等流行病在灌溉工地、钢铁生产营地和人民公社（集体农场）中蔓延。结果，公共卫生的重点转向对抗这些流行病及其预防。没有多余的人员来领导反对血吸虫病的大规模运动。血吸虫病防治工作变成了明年的任务，或者干脆从当地公共卫生工作中完全移除。⁶³ 在湖南省的血吸虫病流行区岳阳地区，有十名血吸虫病防治干部被调到其他部门。当地医院的院长身兼数职，以领导当地的根除运动，但由于工作负担过重，他无法集中精力。领导的缺席导致当地血吸虫病防治活动减少。⁶⁴

在基层，基层干部不仅负担多重任务，而且目标遥不可及。因此，他们必须不断做出选择，决定首先做什么，这取决于当时的首要任务是什么。由于诸如消灭钉螺或治疗生病村民等公共卫生工作需要大量时间和精力，地方干部通常认为这些工作是他们在“大跃进”竞赛中赢得奖牌的麻烦或障碍。例如，在浙江省血吸虫病防治的旗舰县嘉兴，在一次动员大会上，人们承诺要大规模努力根除

the disease, a grassroots party secretary stated: 'Eliminating snails is a waste of money and manpower. It decreases productivity. I strongly oppose it,' he shouted loudly.⁶⁵ In Guangdong province's Qujiang County, due to the enormous pressure to collect fertilizer to increase the agricultural yields, the plan to eliminate snails was simply withdrawn. Instead of incorporating snail elimination into local irrigation projects, as advised by the government policy, the two campaigns constantly competed for local resources. Fighting for labour became the most common scenario throughout the country. In Guangdong province's Hua County, all labourers were engaged in irrigation construction or steel and iron work, leaving the schistosomiasis control cadres alone to eliminate snails. Unable to meet their projected target, they abandoned the work altogether. As soon as the people retreated, however, the snails returned. Similar situations were also reported in Jiangsu and Zhejiang provinces.⁶⁶

SCHISTOSOMIASIS: THE NEW PLAGUE OF THE GREAT LEAP IN AGRICULTURE

In the lake regions along the Yangtze River, propaganda images of bumper harvests and healthy, happy-looking workers gathering crops were replaced by the reality of acute infection. The environmental control method by land reclamation and water conservancy work exposed hundreds, indeed thousands, of healthy labourers to the disease when such projects were carried out with little or no protection for those who participated in them. This was the case during the PRC's great rush to Leap Forward. Earlier experts had already warned about the potential health risks. Chen Huxin, the expert who had led the pilot study of controlling schistosomiasis by reclaiming the marshes for agricultural cultivation during the South Dongting Lake Reconstruction project of 1953-56 (as discussed in chapter 2), testified that among the 250,000 labourers who had participated in that reconstruction project, more than half toiled in the contaminated water. Many previously fit and healthy labourers, imported from non-endemic regions, became infected with schistosomiasis as a result. Against such massive risk, Chen warned that it was necessary to wait until all the snails had been destroyed before the reclaimed land could be cultivated. He also urged all local authorities to regard the protection of labourers against infection as

这种疾病，一位基层党支部书记说：‘消灭钉螺是浪费金钱和人力。这会降低生产力。我强烈反对，’他大声喊道。⁶⁵ 在广东省曲江县，由于收集肥料以提高农业产量的巨大压力，消灭钉螺的计划被直接撤回。当地的灌溉工程没有按照政府政策的建议将消灭钉螺纳入其中，而是这两个运动不断争夺当地资源。争夺劳动力成为全国最常见的景象。在广东省花县，所有劳动力都从事灌溉建设或钢铁工作，只留下血吸虫病防治干部来消灭钉螺。由于无法达到预计的目标，他们完全放弃了这项工作。然而，一旦人们撤退，钉螺就又回来了。江苏省和浙江省也报告了类似的情况。⁶⁶

血吸虫病：农业大跃进的新瘟疫

在长江沿岸的湖区，宣传画中丰收的景象和健康快乐的工人们收割庄稼的画面，被急性感染的现实所取代。通过土地开垦和水利工程进行环境控制的方法，使成百上千的健康劳动者暴露于这种疾病之中，因为这些项目在实施时，几乎没有或根本没有为参与者提供保护。在中华人民共和国大跃进时期，情况就是如此。早期的专家已经警告过潜在的健康风险。陈 Huxin 是 1953-56 年洞庭湖南部湖区重建项目中，通过开垦沼泽进行农业耕作来控制血吸虫病试点研究的负责人（如第 2 章所述），他证明，在参与该重建项目的 25 万劳动者中，有一半以上的人在受污染的水中劳作。许多以前健康强壮的劳动者，从非流行地区引进，结果感染了血吸虫病。面对如此巨大的风险，陈警告说，有必要等到所有钉螺都被消灭后，才能耕种开垦后的土地。他还敦促所有地方当局将保护劳动者免受感染视为

imperative when planning for land reclamation and water conservation projects as well as for agricultural work.⁶⁷

But the Great Leap Forward would wait for no one and nothing. On 15 November 1958, addressing a question asked by a Polish delegation visiting the PRC regarding the huge number of industrial accidents, Marshall Chen Yi, the vice premier and foreign minister of China at the time, put it crudely: ‘these accidents will not prevent us advancing forward. Such is the price worth paying. No need to get hysterical over this. So many lives were lost during the war. A few accidents and sickness are nothing.’⁶⁸ The masses may have been the CCP’s most tangible resource, but human life was also cheap in the PRC. For local authorities with their eyes fixed on winning the prize, making sure labourers and farmers were thoroughly protected against infection was the last thing on their mind. Furthermore, because of the severe shortage of cotton (all cotton produced was procured by the state, mainly for export), local authorities could not supply enough of the fabric needed to make protective socks and leg wrappings for all the workers labouring in the contaminated water or paddy fields.⁶⁹ The worst season was from March to August. This was also the busiest farming season, and the increased agricultural activity multiplied the chances of infection. In the meantime, the warmer and humid climate allowed the parasite to breed more quickly. The warmer and humid weather also added further difficulties to treatment work: ‘The more we treat people, the higher the morbidity,’ a health official from Hunan lamented at the fourth national schistosomiasis control conference.⁷⁰

An increasing number of people were infected as more land was reclaimed and more irrigation canals constructed. Between 1958 and 1962, along Hunan province’s Dongting Lake and Jiangxi province’s Poyang Lake, two of the worst endemic regions along the Yangtze, more than 18,121 acres (110,000 *mu*) of marshland were reclaimed. Thirty-three state farms were built on this reclaimed land, and more than 30,000 workers, many of them from non-endemic regions, were drafted to work on these new, makeshift farms. Schistosomiasis was the new plague for these farm employees and their families. Official statistics show that from 1960 to 1961, more than 4,000 workers from twenty different state farms suffered acute infection in that year alone. Among them were more than 170 workers from state farms in Hunan who died of schistosomiasis.⁷¹

在规划土地开垦和水利工程以及农业工作时，这是当务之急。⁶⁷

但大跃进不会等待任何人或事。1958年11月15日，时任中国副总理兼外交部长的陈毅元帅在回答来访中国的波兰代表团提出的关于工业事故数量庞大的问题时，粗暴地说：“这些事故不会阻止我们前进。这是值得付出的代价。没有必要为此感到歇斯底里。战争期间失去了如此多的生命。一些事故和疾病算不了什么。”⁶⁸ 群众可能是中国共产党最实在的资源，但在中国，人命也很廉价。对于那些目光集中在赢得奖励上的地方当局来说，确保劳动者和农民得到充分保护免受感染是他们最不关心的事情。此外，由于棉花严重短缺（所有棉花都由国家采购，主要用于出口），地方当局无法提供足够的布料来为所有在受污染的水域或稻田中劳作的工人制作防护袜和护腿。⁶⁹ 最糟糕的季节是从三月到八月。这也是最繁忙的农忙季节，农业活动的增加增加了感染的机会。与此同时，温暖潮湿的气候使寄生虫繁殖得更快。温暖潮湿的天气也给治疗工作增加了更多的困难：“我们治疗的人越多，发病率就越高，”湖南省的一位卫生官员在第四次全国血吸虫病防治会议上感叹道。⁷⁰

随着越来越多的土地被开垦和更多的灌溉渠被修建，越来越多的人受到感染。1958年至1962年间，在湖南省洞庭湖和江西省鄱阳湖沿岸（长江流域最严重的两个地方病流行区），超过18121英亩（11万亩）的沼泽地被开垦。在这片开垦的土地上，建立了33个国营农场，3万多名工人（其中许多人来自非疫区）被征召到这些新建的临时农场工作。血吸虫病成为这些农场雇员及其家人的新瘟疫。官方统计数据显示，从1960年到1961年，仅当年就有来自20个不同国营农场的4000多名工人患上急性感染。其中，湖南省国营农场的170多名工人死于血吸虫病。⁷¹

Yueyang Prison was one of the first prison labour camps in the PRC. Euphemistically called the 'Jianxin (Constructing New) National Farm', it was on the shore of Dongting Lake and was jointly run by the Hunan Provincial Bureau of Public Security and the Yueyang County authority. It was the 'home' of the PRC's major political prisoners, including a huge number of Rightists. When Liu Fengxiang, the former editor-in-chief of the *Hunan Daily* and a prominent Rightist, first arrived at the farm, he told his fellow prisoner Yang Xiaokai (Yang was twice nominated for the Nobel Prize in Economics): 'Mao Zedong has sent us here to die.'⁷² This so-called farm was built on a reclaimed marsh surrounded by water. Prisoners lived in shoddy pop-up tents built out of reeds, which were also an ideal habitat for snails. During spring and summer, large parts of the farm were flooded. Prisoners had to endure living and working in the contaminated water unprotected during the day, wearing their torn clothes. At night they shared their primitive living spaces with infected snails. It is not surprising that many became ill with schistosomiasis, and indeed many died as a result. According to an official estimate, during the busiest planting season in 1958, 613 prisoners suffered acute infections within a few days, increasing the infection rates from 23.7 per cent in the previous year to 27.3 per cent. The situation worsened as the speed of the Great Leap accelerated. Between 1960 and 1963, more than 9,400 prisoners suffered acute infections, and the disease claimed eighty-five lives. Although unintended, schistosomiasis had indeed become a means of getting rid of political dissidents.⁷³ If human life was cheap in the PRC, political prisoners such as Rightists were treated as the scum of earth and died as a result.

Along the east bank of Dongting Lake is the Miluo River, a minor tributary to the Yangtze.* In October 1958, under the Great Leap

* During the war against the Japanese, the Miluo was highlighted by the patriotic historian Wen Yiduo, who tied the river to the legend of Qu Yuan as well as to the Dragon Boat festival. Qu Yuan (340–278 BCE) was a court official and poet of the Chu state (nowadays Hunan province) who lived in the Warring State period. He committed suicide by drowning himself in the Miluo after being driven in disgrace from office. Throughout history, the suicide of Qu Yuan has represented many things to different people at different times. During the Sino-Japanese War, Qu Yuan was turned into a patriotic martyr. At the same time, the Dragon Boat festival, a lunar festival celebrated by communities living on the water in southern China long before the time of Qu Yuan, became a commemoration of Qu Yuan's death. The Miluo River was turned into the birthplace of the festival.

岳阳监狱是中华人民共和国最早的监狱劳改营之一。它委婉地被称为“建新农场”，位于洞庭湖畔，由湖南省公安厅和岳阳县政府联合管理。这里是中华人民共和国主要政治犯的“家”，其中包括大量的右派分子。原《湖南日报》总编辑、著名右派分子刘凤翔刚到农场时，对狱友杨小凯（杨曾两次获得诺贝尔经济学奖提名）说：“毛泽东派我们在这里等死。”⁷² 这个所谓的农场建在被水包围的开垦沼泽地上。囚犯们住在用芦苇搭建的简陋的临时帐篷里，芦苇也是钉螺的理想栖息地。春夏期间，农场的大部分地区都被洪水淹没。囚犯们不得不忍受白天在受污染的水中无保护地生活和工作，穿着破烂的衣服。晚上，他们与受感染的钉螺共享简陋的居住空间。毫不奇怪，许多人因此患上了血吸虫病，事实上，许多人因此丧生。据官方估计，1958年最繁忙的种植季节，几天内就有613名囚犯遭受急性感染，感染率从上一年的23.7%上升到27.3%。随着大跃进速度的加快，情况变得更加糟糕。1960年至1963年间，9400多名囚犯遭受急性感染，该疾病夺去了85人的生命。尽管并非有意，但血吸虫病确实已成为摆脱政治异议人士的一种手段。⁷³ 如果说人命在中国很廉价，那么像右派这样的政治犯就被视为人渣，最终死于非命。

沿洞庭湖东岸是汨罗江，它是长江的一条小支流。* 1958年10月，在大跃进时期

* 在抗日战争期间，爱国历史学家闻一多突出强调了汨罗江，他将这条河流与屈原的传说以及端午节联系起来。屈原（公元前340–278年）是楚国（今湖南省）的一位朝廷官员和诗人，生活在战国时期。他因被免职而耻辱地离开官位后，投汨罗江自尽。纵观历史，屈原的自杀对不同时期不同的人来说代表着许多事情。在抗日战争期间，屈原被塑造成一位爱国烈士。与此同时，端午节——一个早在屈原时代之前，居住在中国南方水域的社区庆祝的农历节日——变成了对屈原逝世的纪念。汨罗江被变成了这个节日的诞生地。

banner of ‘Going all out, aiming high, and achieving more and faster economic results’, Hunan province’s Xiangyin County authority embarked on the Miluo River Water Conservation and Reclamation project. This was one of the many ambitious projects of the Great Leap Forward aimed at reclaiming the marsh for agricultural cultivation and diverting the water from the Milou into Dongting Lake for flood control. More than 14,000 labourers were thrown into the project to excavate canals and fill the marshes. They toiled around the clock with no sleep and little to eat so that they could complete the project before the arrival of the monsoon season. But the monsoon arrived a few days earlier than forecast. On 4 June, the river’s water level rose so high that the dyke was about to burst. The newly reclaimed land, which had already been planted with crops, as well as the half-built irrigation canals, were in danger. In an attempt to prevent the dyke from bursting, the authority sent 550 labourers, including 470 political prisoners (most of them Rightists) down to the flooded river carrying twenty logs. Despite their herculean efforts, the dyke burst in the end, and the deluge washed over 1,977 acres (12,000 *mu*) of newly reclaimed and cultivated land. Half-grown crops as well as more than 200 dead bodies of workers floated away with the floods. One survivor named himself ‘Iron Torrent’ to commemorate the incident. At the age of eighty-one, he could still recall the horror of one fellow worker’s body being swallowed up by a giant fish.* As soon as the water receded, snails returned, carrying schistosoma eggs. The prisoners and peasant workers who survived the flood were ordered to return to the construction site. Without any protection, many became infected with the disease.⁷⁴

None of this mattered, however. The news of the total ‘eradication’ of schistosomiasis resounding from Yujiang, a small and unremarkable county along the Yangtze in central China, was to save the failing campaign, as well as inspiring Chairman Mao Zedong to write his often-cited poem ‘Sending off the Plague God’ that would immortalize the campaign. Both the claim and the poem were the fictions needed at the time to confront the brutal realities of the disease and its impact on the people’s health. In Mao’s China, the exigencies of national politics often trumped the provision of the public’s health.

* This man was recently arrested by public security and imprisoned in southwestern China for speaking out about this tragic event that took place nearly sixty years ago.

在“鼓足干劲、力争上游、多快好省地建设社会主义”的旗帜下，湖南省湘阴县政府启动了汨罗江水利建设和垦殖工程。这是大跃进时期旨在开垦沼泽地进行农业耕作，并将汨罗江水引入洞庭湖以控制洪水的众多雄心勃勃的项目之一。14000多名劳工被投入该项目，挖掘运河和填埋沼泽。他们夜以继日地工作，没有睡眠，也没有充足的食物，以便在季风季节到来之前完成项目。但季风比预报提前几天到来。6月4日，河流水位上涨，堤坝即将决堤。新开垦的土地，以及已经种植的农作物，以及半建成的灌溉渠，都处于危险之中。为了防止堤坝决堤，当局派出了550名劳工，其中包括470名政治犯（其中大部分是右派），带着20根木头下到被洪水淹没的河流中。尽管他们竭尽全力，但堤坝最终还是决堤了，洪水冲走了1977英亩（12000亩）新开垦和耕种的土地。半大的庄稼以及200多具工人的尸体随着洪水漂走了。一位幸存者自称“铁流”，以纪念这一事件。81岁高龄的他，仍然能回忆起一位工友的尸体被一条巨鱼吞噬的恐怖情景。^{*}水一退去，钉螺就带着血吸虫卵回来了。幸免于洪水的囚犯和农民工人奉命返回建筑工地。在任何保护的情况下，许多人感染了这种疾病。⁷⁴

然而，这些都不重要。来自中国中部长江沿岸一个不起眼的小县——余江县，关于血吸虫病被完全“根除”的消息，拯救了这场失败的运动，并激励了毛泽东主席写下了他经常被引用的诗篇《送瘟神》，这首诗将使这场运动不朽。这一说法和这首诗都是当时应对这种疾病及其对人民健康影响的残酷现实所需要的虚构故事。在毛泽东时代的中国，国家政治的需要往往胜过提供公众健康。

* 最近，此人因公开谈论近六十年前发生的这场悲剧事件而被公安逮捕并监禁在中国西南地区。

The Propaganda War

YUJIANG MAKES THE NEWS

Situated in eastern Jiangxi province, just south of the Yangtze River, Yujiang County is a hilly area. The River Xin links it to Poyang Lake, the largest freshwater lake in China, which is also one of the worst endemic schistosomiasis regions along the Yangtze. In the 1800s, as part of the Qing imperial government's ambitious water conservancy initiative to develop agriculture – to end the chronic problem of famine in the region – a network of ditches was built to irrigate the surrounding countryside with water from the lake as well as the river.¹ These irrigation schemes, alongside increased agricultural activities, brought prosperity to Yujiang and the surrounding regions. At the same time, they increased the prevalence and morbidity of schistosomiasis, since irrigated farmland is the ideal environment for snails. The spread of schistosomiasis meant that the region's prosperity was short-lived. It weakened the manpower economy and eventually destroyed the once thriving agriculture. A local record suggests that between 1919 and 1949, schistosomiasis claimed more than 29,000 lives here. Hardly anyone was left to plant and tend crops, and more than 20,000 *mu* of fertile farm fields were soon abandoned and covered in weeds; in the meantime, snails multiplied in them. Villagers remembered that it was impossible to avoid stepping on snails in these fields. Calling them 'killing fields', those who were lucky enough not to be infected by the disease ran away from the area as if running from the plague. As a result, more than forty-two villages were abandoned.²

In 1951, Yujiang was declared a schistosomiasis endemic county. The highest number of casualties was reported at the construction

宣传战

YUJIANG MAKES THE NEWS

余江县位于江西省东部，长江以南，是一个丘陵地区。信江将其与中国最大的淡水湖鄱阳湖相连，鄱阳湖也是长江流域血吸虫病最严重的流行区之一。19 世纪，作为清朝政府雄心勃勃的水利工程计划的一部分，旨在发展农业——结束该地区长期存在的饥荒问题——修建了一个沟渠网络，用来自湖泊和河流的水灌溉周围的乡村。¹ 这些灌溉计划以及农业活动的增加，给余江及周边地区带来了繁荣。与此同时，它们也增加了血吸虫病的流行率和发病率，因为灌溉农田是钉螺的理想环境。血吸虫病的蔓延意味着该地区的繁荣是短暂的。它削弱了劳动力经济，最终摧毁了曾经繁荣的农业。当地的一份记录显示，1919 年至 1949 年间，血吸虫病夺去了这里 29000 多人的生命。几乎没有人留下种植和照料庄稼，超过 2 万亩的肥沃农田很快被废弃，长满了杂草；与此同时，钉螺在其中大量繁殖。村民们回忆说，在这些田地里，根本无法避免踩到钉螺。那些幸运地没有感染这种疾病的人称这些田地“杀戮场”，他们像逃离瘟疫一样逃离了该地区。结果，超过 42 个村庄被废弃。²

1951 年，余江县被宣布为血吸虫病流行县。在建设过程中，伤亡人数达到最高峰。

site of the White Tower irrigation project in Dengjiapu, the new Communist authority's scheme to revitalize local agriculture. In 1946, as part of the post-war reconstruction, the Nationalist government had opened an experimental farm there. This new farm, a modernist development project, pioneered the use of machinery to replace traditional farming methods in order to increase agricultural yields. In November 1949, the newly established Jiangxi provincial Chinese Communist Party (CCP) authority sent army representatives to take over the farm and turn it into a model farm for teaching modern farming methods. Soon after, the White Tower irrigation project began. The White Tower project set out to build a major irrigation canal to bring water from the River Xin to the farm and to the surrounding agricultural fields. But the shock of a schistosomiasis outbreak meant the project had to be suspended.³ In the mid-1950s, however, the draft of the First Five-Year Plan demanded that all state farms expand their cultivable areas. This was followed by the CCP Central Committee's proposal to raise the grain output for the Second Five-Plan Year period to a total of about 2,200,000 million catties and the cotton output to a total of about 210 million catties.⁴ The farm in Dengjiapu was converted to a national experimental centre for breeding improved varieties of rice and other agricultural crops. In the meantime, another state farm named Genxin (which literally translates as 'Improved' or 'Renewal') was opened next to it. The provincial water conservancy bureau was pressured to resume the suspended White Tower irrigation project to bring about the projected high yield, despite the danger of creating further outbreaks of schistosomiasis.

In 1955, Fang Zhichun, the former People's Liberation Army (PLA) commissioner who oversaw the takeover of Dengjiapu state farm and the deputy party secretary of Jiangxi province, was named the head of the Jiangxi Provincial Five-Man Anti-Schistosomiasis Subcommittee, the parallel provincial organization to the CCP Central Committee's Nine-Man Subcommittee. In other words, Fang was the party boss appointed to lead the eradication campaign in Jiangxi province. Fang was also a distant cousin of the former CCP military leader and revolutionary martyr Fang Zhimin, who was a native of Jiangxi province as well as a veteran revolutionary experienced in party political and propaganda work. 'A new office applies strict measures, and a newly appointed official works hard to show his efficiency' is an old Chinese saying. As soon as Fang Zhichun

邓家埠白塔灌溉工程所在地，这是新成立的共产党当局为振兴当地农业而制定的计划。1946年，作为战后重建的一部分，国民党政府在那里开设了一个实验农场。这个新的农场是一个现代化的发展项目，率先使用机械代替传统的耕作方法，以提高农业产量。

1949年11月，新成立的江西省中国共产党（CCP）当局派遣军队代表接管该农场，并将其改造成一个教授现代农业技术的模范农场。不久之后，白塔灌溉工程开始。白塔工程计划修建一条主要的灌溉渠，将信江的水引到农场和周围的农田。但是血吸虫病的爆发意味着该项目不得不暂停。³然而，在20世纪50年代中期，第一个五年计划的草案要求所有国营农场扩大其可耕种面积。紧随其后的是中共中央委员会提出的第二个五年计划期间粮食总产量提高到约2,200,000百万斤，棉花总产量提高到约2.1亿斤的建议。⁴邓家埠的农场被改造成国家级的改良水稻和其他农作物育种实验中心。与此同时，另一个名为“革新”（字面意思是“改进”或“更新”）的国营农场在其旁边开放。尽管存在引发进一步血吸虫病爆发的危险，但省水利局仍被迫恢复已暂停的白塔灌溉工程，以实现预计的高产量。

1955年，曾负责接管邓家埠农场的原中国人民解放军（PLA）政委、江西省委副书记方志纯被任命为江西省防治血吸虫病五人小组组长，该小组是与中共中央九人小组平行的省级机构。换句话说，方是受命领导江西省消灭血吸虫病运动的党内负责人。方还是原中共军事领导人兼革命烈士方志敏的远房堂兄，方志敏是江西人，也是一位经验丰富的党内政治和宣传工作的革命老兵。“新官上任三把火”是一句中国老话。方志纯一上任，

received the mandate from the First National Schistosomiasis Eradication Conference, while still at Shanghai he consulted experts on how to effectively eliminate snails and thus control the disease. He was told to use sodium pentachlorophenate (NaPCP), the molluscicide widely recommended by international experts in favour of the chemical control approach. In China, the 'old folk' in Jiangxi are famed for being shrewd and calculating. As one of them, Fang did not simply follow the experts' recommendation; he did a quick calculation and decided that molluscicides were not affordable. At the time, China had not developed the technology to manufacture NaPCP; it would have to be imported and would cost around 500 *yuan* (ca US\$90) per 0.165 acre of land. In total, there were 494,211 acres of farming land in Jiangxi – which would mean a cost of 2.5 billion *yuan*. Fang could not get his head around such an enormous figure. Subsequently, the provincial authority did not budget for molluscicides at all. Instead, they opted for buying umbrellas and shoes for local schistosomiasis cadres whom they sent to the countryside to study local conditions so that they could come up with cost-effective innovations. However, nothing came about, despite the umbrellas and shoes. Less costly methods such as using toxic herbs to poison snails or burning down the reeds with gas fires produced temporary but not lasting results. The method of converting paddy fields into dry fields reduced the number of snails, but it also killed the crops. As a result, agricultural productivity plummeted, exacerbating the food shortage that had already been underway.⁵

In neighbouring Anhui province, the local authority had promoted the method of draining the paddy fields, but acres and acres of rice crops had withered after the paddy fields were drained.⁶ Anhui experienced much worse food shortages than Jiangxi, and the famine there killed more villagers than schistosomiasis. The villagers in Jiangxi were luckier in comparison. After a short period of eliminating snails by draining the paddy fields, the local authority quickly abandoned the method as they realized that it was hampering local grain production.⁷ For the same reason, the method was also rejected by authorities in the rice-growing provinces in the southwest. In the mountainous regions of Sichuan and Yunnan, for instance, the increased amount of terraced fields in previous years had boosted rice production significantly. At the same time, however, the moist environment created the ideal habitat for *oncomelania* snails, thus helping to spread the disease. Weighing up schistosomiasis control

方接到了第一次全国血吸虫病防治会议的任务，在上海时，他咨询了专家，了解如何有效消灭钉螺，从而控制疾病。他被告知使用五氯酚钠 (NaPCP)，这是一种由国际专家广泛推荐的杀螺剂，支持化学控制方法。在中国，江西的“老表”以精明能干著称。作为其中一员，方没有简单地听从从专家的建议；他快速计算了一下，认为杀螺剂负担不起。当时，中国还没有开发出生产五氯酚钠的技术；它必须进口，每 0.165 英亩土地的成本约为 500 元（约合 90 美元）。江西省共有 494,211 英亩耕地——这意味着成本为 25 亿元。方无法接受如此巨大的数字。随后，省政府根本没有为杀螺剂拨款。相反，他们选择为当地血吸虫病干部购买雨伞和鞋子，并将他们派往农村研究当地情况，以便他们能够提出具有成本效益的创新方法。然而，尽管有雨伞和鞋子，但没有任何结果。成本较低的方法，如使用有毒草药毒杀钉螺或用瓦斯放火烧毁芦苇，产生了暂时的但不是持久的效果。将稻田改造成旱地的方法减少了钉螺的数量，但也杀死了庄稼。结果，农业生产直线下降，加剧了已经开始的粮食短缺。⁵

在邻近的安徽省，地方政府曾推广过排干稻田的方法，但排干稻田后，成片的稻田枯萎了。⁶ 安徽省的粮食短缺情况比江西省严重得多，那里的饥荒造成的村民死亡人数超过了血吸虫病。相比之下，江西的村民比较幸运。在通过排干稻田消灭蜗牛的短暂时期后，地方政府很快放弃了这种方法，因为他们意识到这正在阻碍当地的粮食生产。⁷ 出于同样的原因，西南水稻产区的政府也拒绝了这种方法。例如，在四川和云南的山区，前几年梯田数量的增加大大提高了水稻产量。然而，与此同时，潮湿的环境为钉螺创造了理想的栖息地，从而有助于疾病的传播。权衡血吸虫病的防治

against agricultural productivity, the local authorities in those areas rejected the drainage method from the outset.⁸ In addition, epidemiological studies conducted after the drainage program took place showed that morbidity had increased in both Gaoan County in Jiangxi province and in parts of Anhui province, despite the decrease in the number of snails. It turned out that to carry out the drainage project, villagers and labourers had to work in the contaminated water for an extended period, and furthermore the increased number of local water conservancy projects had also increased the chance of infection. As a result, local data gathered in March 1957 showed that the number of people infected had doubled compared to the previous year.⁹

Faced with these shortcomings, Fang spotted an opportunity. Part of the White Tower irrigation scheme was to build networks of new ditches and reservoirs while blocking off the old, decaying ditches and muddy ponds that had been built in imperial times. During the 1955 annual snail survey, which took place in the winter, the team carrying it out reported that the number of snails had decreased significantly in places where old ditches were blocked off, whereas no snails were found in the newly built ditches. This was not really news, however. As epidemiological studies of the landscape had shown, although there had been some success in reducing snail numbers by the earth burial method in regions on the plain dotted with waterway networks, as in parts of Jiangsu and Zhejiang provinces, the method was not applicable to those endemic regions in the lake basins along the Yangtze River. In this low-lying, wet, and waterlogged land, characterized by the local popular saying ‘Land in winter, water in summer’, the low temperature in the colder and dryer winter months caused the snails to become dormant. The dry and cold weather is suitable for snails to lay their eggs and for hatching, but during the warmer, wet months the land becomes submerged under water. This wet and warmer weather is ideal for the growth of young snails; thus, the water quickly becomes contaminated with schistosomiasis.¹⁰ Even in Jiangsu and Zhejiang provinces, work reports in the 1950s had shown that, despite some local success, in general the quality of snail burial work was not satisfactory.¹¹ But Fang Zhichun had neither the time nor the patience for probabilities or uncertainties in science. He had his eyes fixed on the decreasing number of snails. He had been given the task of leading the provincial eradication campaign in addition to a target of producing

由于该方法不利于农业生产，这些地区的当地政府从一开始就拒绝采用排水法。⁸此外，排水计划实施后的流行病学研究表明，尽管血吸虫数量有所减少，但江西省高安县和安徽省部分地区的发病率都有所上升。结果表明，为了实施排水项目，村民和劳工不得不在受污染的水中长时间工作，而且当地水利工程数量的增加也增加了感染的几率。因此，1957年3月收集的当地数据显示，与上一年相比，感染人数增加了一倍。⁹

面对这些缺点，方志纯发现了一个机会。白塔灌溉计划的一部分是在封堵帝国时期建造的旧的、腐烂的沟渠和泥泞池塘的同时，建造新的沟渠和水库网络。在1955年冬季进行的年度血吸虫调查中，调查小组报告说，在旧沟渠被封堵的地方，血吸虫的数量显著减少，而在新建的沟渠中没有发现血吸虫。然而，这并不是什么新闻。正如对景观的流行病学研究表明，虽然在江苏省和浙江省等布满水道网络的平原地区，通过土埋法减少血吸虫数量取得了一些成功，但该方法不适用于长江沿岸湖盆的那些流行地区。在这种地势低洼、潮湿、涝渍的土地上，当地流传着一句俗语“冬天是土地，夏天是水”，寒冷干燥的冬季导致血吸虫进入休眠状态。干燥寒冷的天气适合血吸虫产卵和孵化，但在温暖潮湿的月份，土地被水淹没。这种温暖潮湿的天气非常适合幼虫的生长；因此，水很快就会被血吸虫病污染。¹⁰即使在江苏省和浙江省，20世纪50年代的工作报告也表明，尽管取得了一些局部成功，但总的来说，埋螺工作的质量并不令人满意。¹¹但方志纯既没有时间也没有耐心来考虑科学上的可能性或不确定性。他的目光一直集中在血吸虫数量的减少上。除了完成生产目标外，他还被赋予了领导全省灭螺运动的任务。

high-yield crops. Yujiang was going to be his model of success, and he was determined to make it happen.

In December 1955, a snail burial 'shock work' was carried out in Yujiang County's Magang Township. The quality of the work was so poor at first that it failed to meet the projected target. Yet this initial failure did not discourage Fang Zhichun. With a long career in party propaganda work, he knew how to mobilize the masses. In January, he gave orders to repeat the 'shock work'. Four thousand snail burial 'stormtroopers' camped onsite. They worked against bitterly cold wind and snow and often skipped meals and sleep. To safeguard their morale, women and children were also mobilized to bring hot tea to their hard-working husbands and fathers. After a three-day stormy 'shock work', two days ahead of schedule, Magang Township authority announced that their 'stormtroopers' had heroically completed the task. The shells of the dead snails were presented as evidence of this first victory. Smiling at the white shells, Fang wrote a jingle titled 'Song of Eradicating Snails'. With years of experience in party propaganda work, Fang was a firm believer in winning the masses through propaganda: he had the jingle published in the provincial daily newspaper.¹² According to Deng Tuo, the first editor-in-chief of the central party newspaper, *People's Daily*, news media such as newspapers were the party's most potent weapon 'to educate, and to lead the broad masses of people in the revolutionary struggle and the construction of the new life'.¹³

Picturing Magang as the 'blossoming flower' of the local eradication campaign, Fang Zhichun envisioned turning Yujiang into a 'blooming' county. He instructed every township in the county to send a team to Magang to be inspired by its 'successful' revolutionary experience. Fang also urged Yujiang's schistosomiasis control cadres to prepare for even bigger battles. 'He followed the snail burial campaign very closely, gave us full support if we encountered any obstacles. Whenever the campaign suffered setbacks, he called or wrote to Yujiang county authority immediately. Sometimes he even went to the campaign site personally to supervise the campaign, thus to hasten the success,' recalled a local cadre who was involved in the campaign.¹⁴ With Fang pushing at his back, Li Junjiu, Yujiang County's party secretary and the executive manager of the Yujiang eradication campaign, camped at Magang Township to prevent the 'blossoming flower' from withering. Li, a former railway worker from Manchuria who joined the PLA during the Chinese civil war,

高产作物。余江将成为他成功的典范，他决心实现这一目标。

1955年12月，余江县马岗乡开展了灭螺“突击工作”。起初，这项工作的质量非常差，未能达到预定目标。然而，最初的失败并没有使方志纯气馁。凭借在党的宣传工作方面的长期经验，他知道如何动员群众。一月份，他下令重复“突击工作”。四千名灭螺“突击队员”在现场扎营。他们顶着严寒的寒风和白雪工作，经常不吃饭、不睡觉。为了维护他们的士气，妇女和儿童也被动员起来，给辛勤工作的丈夫和父亲送去热茶。经过三天暴风雨般的“突击工作”，比原计划提前了两天，马岗乡政府宣布他们的“突击队员”已经英勇地完成了任务。死亡的蜗牛壳被作为第一次胜利的证据展示出来。方志纯对着白色的贝壳微笑，写了一首题为《灭螺歌》的打油诗。凭借多年的党宣传工作经验，方坚信通过宣传赢得群众：他将这首打油诗发表在省级日报上。¹² 据中央党报《人民日报》的第一任总编辑邓拓说，报纸等新闻媒体是党“教育和领导广大人民群众进行革命斗争和建设新生活”的最有力武器。¹³

方志纯将马冈描绘成本地血吸虫病根除运动的“盛开之花”，设想将余江变成一个“开花”的县。他指示县里每个乡镇都派一个团队到马冈，从其“成功”的革命经验中汲取灵感。方志纯还敦促余江的血吸虫病防治干部为更大的战斗做好准备。“他非常密切地关注灭螺运动，如果我们遇到任何障碍，他都会给予我们全力支持。每当运动遭受挫折时，他都会立即致电或写信给余江县委。有时他甚至亲自到运动现场监督运动，从而加速成功，”一位参与该运动的当地干部回忆道。¹⁴ 在方志纯的支持下，余江县委书记兼余江根除运动的执行经理李俊久驻扎在马冈乡，以防止“盛开之花”枯萎。李是来自满洲里的一名前铁路工人，在中国内战期间加入了人民解放军。

was often seen taking part in the snail burial work, holding a shovel in his hands.¹⁵

In the meantime, Fang invited experts from Shanghai to put a stamp on Magang Township's purported 'success'. Su Delong, Wu Guang, and Huang Minxin, three of the People's Republic of China's (PRC) most revered experts in schistosomiasis control, laboriously arrived at Magang by taking a small boat and then walking halfway across the river. After being taken to see how snail burial was done in some villages, Su turned around and asked the following questions: 'How many ditches have been filled? How many cubic metres of earth have been dug up? How many acres of farm land have been increased? Has the agricultural yield increased, and how much per acre? How many labourers have you involved in the work? What is their cost? How many of them are volunteers?' The cadres in charge of the snail elimination work mumbled in response, unable to come up with any precise answers. Staring at their puzzled and embarrassed faces, Su sighed: 'Maybe you could try killing snails in this muddy pond with molluscicides. If it doesn't kill all the snails the first time, try spraying more the second time, and see what happens? I think this method is more cost effective than engaging so many labourers in burying snails.' Considering the fact that the environmental control measure was simply too costly given China's economic conditions, Su was in favour of using molluscicides to kill snails to control the disease. Yet, as mentioned above, according to their calculations, Fang Zhichun and the Jiangxi provincial authority had concluded that using molluscicides to kill the snails was simply not affordable considering the local circumstances in Jiangxi province. They had opted for the earth burial method because they counted on their ability to organize the local masses of cheap labour in order to get the work done. Yet the technical disagreement between the local authority in Jiangxi and the experts from Shanghai on the precise method to use to control the disease was immediately politicized, given that the control of schistosomiasis was an important political undertaking. Some months later, during the Great Leap Forward's political campaign of '*Pull down the White Flag*', Su was criticized for 'blowing the cold wind' by undermining the power of the masses in eradicating the disease. Wu and Huang managed to escape being purged by making public statements acknowledging Yujiang's success in destroying snails through earth burial.¹⁶ In recent years, there have also been attempts to reinterpret Su Delong's words to resurrect the 'successful

经常可以看到他参与蜗牛埋葬工作，手里拿着一把铲子。¹⁵

与此同时，方邀请了上海的专家来为马冈乡所谓的“成功”盖章。苏德隆、吴光、黄敏心这三位中华人民共和国在血吸虫病防治方面最受尊敬的专家，费力地乘坐小船，然后步行穿过半条河，才抵达马冈。在被带去参观了一些村庄的蜗牛埋葬工作后，苏转过身来问了以下问题：“填了多少条沟渠？挖了多少立方米的土？增加了多少亩农田？农业产量增加了吗，每亩增加了多少？你们有多少劳动力参与了这项工作？他们的成本是多少？他们中有多少是志愿者？”负责消灭蜗牛工作的干部们含糊其辞地回应，无法给出任何准确的答案。苏看着他们困惑和尴尬的表情，叹了口气：

“也许你们可以用杀软体动物剂来杀死这个泥塘里的蜗牛。如果第一次没有杀死所有的蜗牛，那就尝试第二次喷洒更多，看看会发生什么？我认为这种方法比雇用这么多劳动力来埋葬蜗牛更具成本效益。”考虑到环境控制措施在中国当时的经济条件下成本太高，苏赞成使用杀软体动物剂来杀死蜗牛以控制疾病。然而，如上所述，根据他们的计算，方志纯和江西省政府已经得出结论，考虑到江西省当地的情况，使用杀软体动物剂杀死蜗牛是根本无法承受的。他们选择了土葬法，因为他们指望自己有能力组织当地大量的廉价劳动力来完成这项工作。然而，鉴于血吸虫病的控制是一项重要的政治任务，江西地方当局和上海专家之间在控制疾病的具体方法上的技术分歧立即被政治化。几个月后，在大跃进的“拔白旗”政治运动中，苏因“吹冷风”破坏群众根除疾病的力量而受到批评。吴和黄通过公开声明承认余江通过土葬消灭蜗牛的成功，从而避免了被清洗。¹⁶近年来，也有人试图重新解释苏德隆的话，以重塑“成功”。

story' of Yujiang's eradication and hence the triumph of the PRC's Anti-Schistosomiasis Campaign.¹⁷

With the 'white flag' pulled down, the 'red flag' was hoisted. To raise the 'red flag' in Jiangxi was a symbolic ritual. Jiangxi is often called the cradle of the new China. In the 1920s, it was in Jiangxi that the then-young CCP leader Mao Zedong and his close ally Zhu De, the military strategist, built up their Soviet base among the peasants and waged guerrilla warfare against the Nationalist army and the local powerholders. It was during his time in Jiangxi, as discussed earlier, that Mao came to understand that public health work was an integral part of the revolutionary war of the masses. According to Bing Xin, one of modern China's most celebrated female writers, the earth of Jiangxi was tinted red. To keep up with Jiangxi's revolutionary tradition, Yujiang was to raise the first 'red flag' in the PRC's Anti-Schistosomiasis Campaign. To win the battle was to involve the masses. In order to call up the masses, a red poster was put up outside the Yujiang County authority office. Li Junjiu, the man assigned to lead the battle, remembered that he could not sleep for many nights. 'When I put up the poster, I had tears in my eyes. After the Land Reform, the campaign to eradicate schistosomiasis was to bring villagers the second LIBERATION.' Su Xiangmao, the head of Magang Township, also recalled that 'The campaign was carried out with the great power of a thunderbolt and the speed of lightning. Within four days, we had completed the planning, and kick started the campaign.' Between 1956 and 1957, more than 20,000 labourers were involved in the Yujiang campaign to eliminate snails. Villagers shouted slogans such as 'If we don't destroy snails, we will never close our eyes and will never rest in peace.'¹⁸ Each township organized its own 'stormtroops' for snail elimination. Altogether, there were fifteen teams, each carrying their own red banners. The competition was fierce. The slogans included 'We are not men if we don't eliminate snails.' Each team had their own work song to synchronize their physical movements, and one person led the singing. Some villagers who participated in the campaign remembered that during one 'shock work' session, which lasted three days and nights, the earth in Yujiang was shaken by their loud singing and shouting.¹⁹ Those who were not engaged in the 'shock work', such as the elderly and women and children, were also out on the streets, picking up snails with pairs of chopsticks. Inspired by Fang's jingle, school-children made up their own snail eliminating song: 'I am the Little

余江消灭血吸虫病的“故事”，以及因此而来的中华人民共和国血吸虫病防治运动的胜利。¹⁷

随着“白旗”的降下，“红旗”升起来了。在江西升起“红旗”是一种象征性的仪式。江西常被称为新中国的摇篮。20世纪20年代，当时年轻的中共领导人毛泽东及其亲密盟友、军事战略家朱德就是在江西的农民中建立了他们的苏维埃根据地，并对国民党军队和地方势力发动了游击战。正如前面讨论的那样，正是在江西期间，毛泽东才认识到公共卫生工作是群众革命战争不可分割的一部分。据中国现代最著名的女作家之一冰心说，江西的土地被染成了红色。为了跟上江西的革命传统，余江要在新中国血吸虫病防治运动中升起第一面“红旗”。赢得这场战斗就是要动员群众。为了号召群众，一张红色的海报贴在了余江县政府办公室外。被指派领导这场战斗的李俊久回忆说，他有很多个晚上都睡不着。

“当我贴出海报时，我热泪盈眶。继土地改革之后，消灭血吸虫病的运动将给村民带来第二次解放。”马岗乡乡长苏祥茂也回忆说：“这场运动以迅雷不及掩耳之势进行。四天之内，我们就完成了规划，并启动了这场运动。”1956年至1957年间，有2万多名劳动力参与了余江的灭螺运动。村民们高呼“不灭螺，死不瞑目，永不罢休”等口号。¹⁸各乡镇都组建了自己的“灭螺突击队”。总共有15个团队，每个团队都扛着自己的红旗。竞争非常激烈。口号包括“不灭螺，非好汉”。每个团队都有自己的劳动号子，以协调他们的身体动作，并由一个人领唱。一些参加过这场运动的村民回忆说，在一次持续了三天三夜的“突击工作”中，余江的土地被他们响亮的歌声和喊叫声震动了。¹⁹那些没有参与“突击工作”的人，如老人、妇女和儿童，也走上街头，用筷子捡拾蜗牛。受方之椿歌谣的启发，学生们创作了自己的灭螺歌：

“我是小

Guards of the Anti-Schistosomiasis Crusade'.²⁰ Within a matter of days, eliminating snails became the most important political event in Yujiang.*

After failing to impress the experts such as Su Delong, Fang approached Zheng Gang, the deputy head of the Office of the Nine-Man Subcommittee responsible for running the Anti-Schistosomiasis Campaign. The Office of the Nine-Man Subcommittee, situated in Shanghai, was the administrative agency set up by the CCP Central Committee's Nine-Man Subcommittee in January 1956 to jointly manage the campaign affairs with the Schistosomiasis Prevention Bureau, a subsidiary of the Ministry of Health. A younger revolutionary than Fang, Zheng Gang had joined the New Fourth Army in 1938 after China entered into war with Japan. In military ranking, he was Fang's junior; he addressed Fang as 'Commissioner Fang' and was respectful of everything Fang had to say. After having toured Magang Township's snail burial sites with Fang, Zheng and his delegation – which consisted mainly of bureaucrats administering the national eradication campaign from their office in Shanghai – were impressed by what they saw. They turned to Li Junjiu and local schistosomiasis cadres and urged them to carry on with their 'good work'. Both Zheng and Fang also gave speeches at a public meeting to inspire villagers to continue fighting the battle against the snails.²¹ Yujiang was established as a national model for potential public health success in the PRC. The politics of eradication came to be coterminous with a range of claims about the efficacy of the new system. The underlying claim was that the health officials and experts at the centre, whether in the Central Committee or in the health establishment, knew what was inherently best for the people. Thus, the claims about the people's health and its improvement were an index of the success of the centralized planned economy.²²

In the winter of 1957–58, when more than 100 million people across China were reported to have been mobilized to take part in the Great Leap Forward's water conservation projects, thousands of villagers in Yujiang were filling up old muddy ditches and ponds. By January 1958, the last few old ditches and ponds had been filled up. In the meantime, snail numbers had decreased substantially. Again, the snail survey was carried out during the winter months

* Today, local school children even play a snail elimination computer game to commemorate the Yujiang campaign.

血吸虫病防治队的卫兵”。²⁰几天之内，消灭钉螺成为余江最重要的政治事件。*

方未能给苏德隆等专家留下深刻印象，之后他找到了负责血吸虫病防治运动的九人小组办公室副主任郑刚。九人小组办公室位于上海，是中共中央九人小组于1956年1月设立的行政机构，与卫生部下属的血吸虫病防治局共同管理防治工作。郑刚比方更年轻，是一位革命者，在中国与日本开战后于1938年加入新四军。在军衔上，他是方的下级；他称方为“方专员”，并尊重方所说的一切。郑刚和他的代表团（主要由在上海办公室管理国家根除运动的官员组成）与方一起参观了马岗乡的埋螺场后，他们对所见景象印象深刻。他们转向李俊久和当地血吸虫病干部，敦促他们继续“做好工作”。郑刚和方还在一次公开会议上发表讲话，激励村民继续与钉螺作斗争。²¹余江被确立为中华人民共和国潜在公共卫生成功的国家典范。根除运动的政治与一系列关于新系统有效性的主张相吻合。其根本主张是，无论是中央委员会还是卫生机构的卫生官员和专家，都知道什么对人民来说是最好的。因此，关于人民健康及其改善的主张是中央计划经济成功的一个指标。²²

在1957-58年冬季，据报道，中国各地有超过1亿人被动员起来参与大跃进的水利工程，余江县的数千名村民正在填埋旧的泥泞沟渠和池塘。到1958年1月，最后剩下的几条旧沟渠和池塘都被填满了。与此同时，钉螺的数量也大幅减少。此外，钉螺调查也是在冬季进行的。

* 如今，当地的小学生们甚至玩一种消除钉螺的电脑游戏，以纪念余江的这场运动。

when the low temperatures slowed down the snails' development. The news of the retreating snails excited Fang Zhichun in Jiangxi as well as Zheng Gang in Shanghai. Zheng immediately sent a team to report on Yujiang's 'success'. Together with local schistosomiasis control cadres, they drafted the 'Investigative Report on Yujiang's Near Eradication of Schistosomiasis'. For the draft to be finalized, however, and for Yujiang to become the Anti-Schistosomiasis Campaign's national model, it needed the official stamp of approval. To make sure that Yujiang passed the final test, Fang instigated an even more ferocious battle against snails. This time another 7,000 villagers were called up for duty, and snail elimination 'shock work' lasted for nearly three months. Every village wrote their pledges and posted their challenge letter to the contest, as well as their responses to the challenge. It was only during this final phase that the mass treatment work was carried out in Yujiang. A makeshift treatment centre was set up in the former Daoist temple, although it was too small because there were so many patients. In the end, villagers were moved out of their homes to make space for the treatment work. In May 1958, the team sent by Fang Zhichun to carry out the final check reported that only forty-three live snails were found. The same report also showed that amongst the 159 victims who received treatment, 95.6 per cent had been cured.²³

Concurrently, however, the bi-weekly work reports published by the Nine-Man Subcommittee – which were based on the investigations of government work teams that regularly went around the countryside to survey the local work – painted a rather gloomy picture of the Anti-Schistosomiasis Campaign throughout Jiangxi province. The 1958 January report had shown that overall, the Jiangxi province's schistosomiasis control work was shoddily planned and haphazardly carried out. Many local cadres resisted the Anti-Schistosomiasis Campaign by complaining that 'there are too many snails to kill' and 'the disease is so widespread that it's impossible to control it. It would take forever.' The February report stated that the situation was worsening: 'Jiangxi province's schistosomiasis control work has stagnated. Local authorities there claim the pressure of water conservancy and agriculture work is too great. Villagers want to earn more work points, so they often resist joining snail burial work or receiving treatment. On the other hand, local cadres did not want to bear the blame of jeopardizing agricultural productivity and water conservancy. They dragged sick villagers out

当低温减缓了钉螺的生长速度时，江西的方志纯和上海的郑刚都对钉螺的退缩感到兴奋。郑刚立即派出一个团队去报道余江的“成功”。他们与当地的血吸虫病防治干部一起起草了《关于余江县基本消灭血吸虫病的调查报告》。然而，为了最终确定草案，并使余江成为抗血吸虫病运动的全国模范，它需要得到官方的批准。为了确保余江通过最终的考验，方志纯发动了一场更加猛烈的灭螺战役。这一次，又有 7000 名村民被征召，灭螺“突击工作”持续了近三个月。每个村庄都写了保证书，张贴了挑战书，以及对挑战的回应。只有在最后的这个阶段，余江才进行了大规模的治疗工作。在原来的道观里设立了一个临时治疗中心，但由于病人太多，地方太小。最后，村民们搬出自己的家，为治疗工作腾出空间。1958 年 5 月，方志纯派出的进行最终检查的团队报告说，只发现了 43 只活着的钉螺。同一份报告还显示，在接受治疗的 159 名患者中，有 95.6% 的人已经痊愈。²³

然而，与此同时，九人小组每两周发布的工作报告——这些报告基于政府工作队的调查，这些工作队定期到农村调查当地工作——描绘了一幅关于江西省血吸虫病防治运动的相当黯淡的画面。1958 年 1 月的报告显示，总体而言，江西省的血吸虫病防治工作计划粗糙，执行随意。许多地方干部抵制血吸虫病防治运动，抱怨说“要杀的钉螺太多了”和“这种疾病如此普遍，以至于无法控制。这将是永恒的。2 月份的报告称，情况正在恶化：“江西省的血吸虫病防治工作已经停滞。当地政府声称水利和农业工作的压力太大。村民们想挣更多的工分，所以他们经常抵制参加埋螺工作或接受治疗。另一方面，地方干部不想承担危及农业生产力和水利工程的责任。他们把生病的村民拖出来

of treatment centres and hospitals and forced them to work in the paddy fields or to dig canals.’ By 17 April, the work report from Jiangxi province was equally disappointing: ‘A number of regions in Jiangxi province produced only slogans but no actions. The thunder is very loud there but we see no rain. A few counties and municipalities haven’t produced any plans for schistosomiasis control. They kept on postponing control work from one month to another, and from this year to the next. We have not seen any progress being made at all.’²⁴ Yet all of this was quickly erased and forgotten as soon as Yujiang County in Jiangxi province raised the first ‘red flag’ of the PRC’s Anti-Schistosomiasis Campaign.

THE PROPAGANDA OF HEALTH IMPROVEMENT

On 10 May 1958, an article entitled ‘How Did We Eradicate Schistosomiasis’ appeared in the *Jiangxi Daily News*. Thirty-three years later in 1991, the author of the article, Li Junjiu, the former Yujiang County’s party boss who led the Yujiang’s eradication campaign in 1958, conveyed to Zou Yihua (a native of Yujiang who had a distinguished career in local media and journalism) that Fang Zhichun had cowed him into writing the article: ‘Yujiang was the model set up by Commissioner Fang. He constantly reminded me that good leadership is knowing how to evaluate the success. He impelled me not to fight a battle behind the closed door but to write articles to evaluate its success and to publicize it to inspire others.’²⁵ For Fang Zhichun, the man experienced in party propaganda work, the Yujiang campaign was propaganda from the outset. In the following months and years, the propaganda was to be presented as news and broadcast across China and globally.

In 1955, at a party politburo meeting, Chairman Mao Zedong of the CCP proposed that the Xinhua News Agency, the mouthpiece of the CCP and the new China, expand its activities around the globe: ‘To let the world hear our voice,’ Mao proclaimed.²⁶ Three years later, in the midst of the Great Leap Forward, the world was to hear the exciting news of how the new China, under the leadership of the CCP, had successfully eradicated schistosomiasis – acknowledged by the 1949 Second World Health Assembly and the international experts as the most widespread debilitating disease, after malaria, in many parts of the world, as well as a global social, economic, and health problem.

治疗中心和医院，并强迫他们到稻田工作或挖掘运河。’到4月17日，江西省的工作报告同样令人失望：“江西省一些地区只有口号，没有行动。那里的雷声很大，但我们看不到雨。一些县市还没有制定血吸虫病控制计划。他们不断地将控制工作从一个月推迟到另一个月，从今年推迟到明年。我们根本没有看到任何进展。’²⁴然而，这一切很快就被抹去和遗忘了，因为江西省余江县升起了中国防治血吸虫病运动的第一面“红旗”。

改善健康的宣传

1958年5月10日，《江西日报》发表了一篇题为《我们是如何消灭血吸虫病的》的文章。33年后的1991年，这篇文章的作者，即1958年领导余江县消灭血吸虫病运动的前余江县委书记李俊九向邹义华（余江本地人，在当地媒体和新闻界有杰出成就）透露，方志纯曾胁迫他写这篇文章：“余江是方专员树立的榜样。他不断提醒我，好的领导就是要懂得如何评估成功。他促使我不要闭门造车，而是要写文章来评估它的成功，并将其宣传出去，以激励他人。”²⁵对于方志纯这位在党的宣传工作方面经验丰富的人来说，余江的这场运动从一开始就是宣传。在随后的几个月和几年里，这些宣传将被作为新闻呈现，并在中国乃至全球范围内传播。

1955年，在一次政治局会议上，中共中央主席毛泽东提出，新华通讯社，即中共及新中国的喉舌，应将其活动扩展到全球：

“要让世界听到我们的声音，”毛泽东宣称。²⁶三年后，在大跃进时期，世界将听到令人兴奋的消息，即新中国在中国共产党的领导下，已成功消灭了血吸虫病——1949年第二届世界卫生大会和国际专家承认，血吸虫病是世界上许多地区继疟疾之后最广泛的衰弱性疾病，同时也是一个全球性的社会、经济和健康问题。

The spring of 1958 was unusually busy for the Chinese villagers and their cadres. In April, as soon as the Soviet Union launched Sputnik II into orbit, Chayashan on the north China plain – the reputed cradle of Chinese civilization – gave birth to the PRC's first People's Commune and launched the first agricultural 'sputnik' of the Great Leap Forward. Provincial party leaders throughout the country were challenged by Chairman Mao to join in a race to launch their own 'sputnik fields'.^{*27} Being a 'metal snake' himself, Fang Zhichun was in no hurry to join the 'dog race', however.[†] In his mind, Yujiang had won a smashing 'victory' by hoisting the first red flag of the Anti-Schistosomiasis Campaign. Their 'sputnik' had been launched already, and it was time for jubilation and publicity. Fang planned a three-day celebration and called Xinhua News Agency's Jiangxi branch to cover the event. A young and eager Liu Guanghui was given the assignment. New to journalism and to Jiangxi, Liu was a native of Hunan province; he had been one of the talented high school graduates recruited in 1956 by the ever-expanding Xinhua News Agency. After a short induction, he was dispatched to work at the Xinhua's Jiangxi branch. As the cradle of new China, Jiangxi province was also the birthplace of the Xinhua News Agency itself. The Yujiang celebration was Liu's first major assignment, and he was to travel there with Commissioner Fang, Jiangxi province's party boss and the chief engineer of the Yujiang 'victory'. 'Little Liu', as he was called by everyone then, was excited and proud to have been chosen. On the way, he learned from Fang how Yujiang, the land once rampaged by schistosomiasis, had eradicated the disease after three years' hard battle under the party leadership. A quick learner and a rising star at Xinhua's Jiangxi branch, Liu understood that Yujiang's story of fighting disease was a story of fighting for socialism. This battle against the threats of the natural world was also a heroic struggle of the Chinese peasantry, under the leadership of the CCP, against the old 'feudal' system, the Nationalist past with its corruption, and imperialist/capitalist medicine that relied on experts and denied the masses access and participation. In the context of the Great Leap Forward, Liu's responsibility as a news reporter for

* Inflated high-yield experimental farming fields.

† According to the Chinese zodiac, 'metal snakes' are confident, quick at spotting opportunities, and always determined to accomplish their goals; 1958 was the Year of the Earth Dog in the Chinese calendar.

1958年春，对于中国村民和他们的干部来说，是非常忙碌的。4月，苏联将人造卫星二号送入轨道后不久，位于华北平原的茶叶山——据说是中华文明的发祥地——诞生了中华人民共和国第一个人民公社，并发射了“大跃进”时期的第一个农业“卫星”。全国各地的省委领导都被毛主席挑战，加入到一场发射他们自己的“卫星田”的竞赛中。^{*27} 方志纯本人是一条“金属蛇”，但他并不急于加入这场“狗赛”。[†] 在他看来，余江通过升起第一个血吸虫病防治运动的红旗，已经赢得了一场巨大的“胜利”。他们的“卫星”已经发射了，现在是庆祝和宣传的时候了。方计划了一个为期三天的庆祝活动，并打电话给新华通讯社江西分社来报道这次活动。一位年轻而热情的刘光辉被分配到了这项任务。刘是湖南省人，对新闻和江西都不熟悉；他是1956年新华通讯社不断扩招的有才华的高中毕业生之一。经过短暂的入职培训后，他被派往新华社江西分社工作。作为新中国的摇篮，江西省也是新华社本身的发源地。余江的庆祝活动是刘的第一个主要任务，他将与江西省的党魁、余江“胜利”的总工程师方专员一起前往那里。当时大家都称他为“小刘”，他很高兴也很自豪自己被选中了。在路上，他从方那里了解到，余江这片曾经饱受血吸虫病肆虐的土地，在党的领导下，经过三年的艰苦战斗，已经根除了这种疾病。刘是新华社江西分社的一位快速学习者和冉冉升起的新星，他明白余江的抗击疾病的故事是为社会主义而战的故事。这场对抗自然界威胁的战斗，也是中国农民在中国共产党领导下，反对旧的“封建”制度、民族主义的腐败过去以及依赖专家、拒绝群众获得和参与的帝国主义/资本主义医学的英勇斗争。在大跃进的背景下，刘作为一名新闻记者的责任是

* 虚报的高产实验农田。

† 根据中国生肖，“金蛇”自信、善于发现机会，并且总是决心完成他们的目标；1958年是中国农历的土狗年。

the state news agency, the mouthpiece for the party, was to report praiseworthy examples of implementing party policies as well as individual models who would inspire his readers regarding socialist reconstruction. For him, Yujiang's 'heroic battle' against schistosomiasis would make a perfect example. This was the victory narrative of the eradication in Yujiang that would appear in the *People's Daily* on 30 June 1958, and it was to become the epitome of the ever-victorious Anti-Schistosomiasis Campaign in the PRC as well as the model for the victory of the socialist road to public health.²⁸

At first glance, this more than 5,000-word special report, entitled 'The First Red Flag: Jiangxi Province's Yujiang County's Experience in Eradicating Schistosomiasis', co-authored by Liu Guanghui with Chen Bingyan, a local journalist from the *Jiangxi Daily News*, was written as a typical, formulaic editorial of its time, which, according to Deng Tuo, the editor-in-chief of the *People's Daily*, always began 'with a discourse on current conditions, followed by a presentation of good examples and a criticism of a few bad examples. And then, the subjective causes of each. Toss in a few lessons from experience, and repeat a few generalities on advancing our work, which everyone already knows anyway. Finish up with a few sentences on how under the leadership of the Party this task will be completely achieved.' In 1955, lecturing editors and journalists working for the *People's Daily*, Deng Tuo had remarked that such a formulaic editorial was so dull that it made people want to vomit.²⁹ But in 1958, Deng was being pushed out of the role, even though he publicly praised and supported the Great Leap Forward. One of his faults was that he had opposed the Hundred Flowers Campaign in 1957. As the editor-in-chief, he had deterred many employees at the *People's Daily* from publicly speaking out, although perhaps more important was that he had sided with technocrats in the party, casting doubt on Mao's effort to speed up socialist transformation in the Chinese countryside.

Liu and Chen's special report, 'The First Red Flag', began with a sentence stating that the red flag of Yujiang was a new chapter and historic victory for the PRC's achievements in science. The report ended with a paragraph highlighting that it was the CCP's General Line for socialist construction that had led to the great victories in cultural and health work, and thus the eradication of schistosomiasis was an integral part of the Great Leap Forward. The positive message was also told through the mouths of a wide range of

国家通讯社, 党的喉舌, 要报告实施党的政策的值得称赞的例子, 以及能够激励读者进行社会主义重建的个人模范。对他来说, 余江的“抗血吸虫病英勇斗争”将是一个完美的例子。这就是1958年6月30日发表在《人民日报》上的余江根除血吸虫病的胜利叙事, 它将成为中国人民解放军常胜的抗血吸虫病运动的缩影, 也是社会主义道路走向公共卫生胜利的典范。²⁸

乍一看, 这篇五千多字的特稿, 题为《第一面红旗: 江西省余江县消灭血吸虫病的经验》, 由刘光辉与《江西日报》的当地记者陈秉炎合著, 写成了一篇典型的、公式化的社论, 用《人民日报》总编辑邓拓的话说, 总是以“论述当前形势开始, 然后介绍一些好的例子, 批评一些坏的例子。然后, 是各自的主观原因。加入一些经验教训, 重复一些关于推进我们工作的泛泛之谈, 反正每个人都已经知道了。最后用几句话说明, 在党的领导下, 这项任务将彻底完成。”1955年, 邓拓在《人民日报》对编辑和记者们讲课时曾说, 这种公式化的社论太枯燥了, 让人想呕吐。²⁹但在1958年, 邓拓逐渐被边缘化, 尽管他公开赞扬和支持大跃进。他的一个错误是, 他在1957年反对百花运动。作为总编辑, 他阻止了《人民日报》的许多员工公开讲话, 但或许更重要的是, 他站在了党内的技术官僚一边, 对毛泽东加快中国农村社会主义改造的努力表示怀疑。

刘和陈的特别报道《第一面红旗》以一句话开头, 指出余江的红旗是中国在科学成就方面的新篇章和历史性胜利。该报告以一段文字结尾, 强调正是中国共产党社会主义建设总路线导致了文化和卫生工作的伟大胜利, 因此消灭血吸虫病是“大跃进”不可分割的一部分。积极的信息也通过各种各样的

individuals. They included Chen Chongyi, a Jiangxi province schistosomiasis control committee member as well as the director of Jiangxi Medical College, who stated: 'For sixteen years I worked for the medical establishment in Old China. I had not heard or seen anywhere about schistosomiasis being eradicated. I have not seen any papers from the capitalist countries saying it's possible to eradicate schistosomiasis. But today, I have not only heard it's possible, I have seen it with my own eyes.' (Chen was a renowned medical educator who advocated saving China through modern medicine and education, and Jiangxi Medical College was one of the most prestigious state medical colleges during the Nationalist period.) The article also quoted nameless villagers from Yujiang county rejoicing that 'The Party and Chairman Mao saved our lives ... In the future, if Chairman Mao and the Party give us any more directives, as long as our cadres build the ladders, we are willing to climb up to the sky.'³⁰

Yet the report on Yujiang was far from a dull formulaic editorial. It had a narrative that was told in a typical Great Leap Forward literary style that combined 'revolutionary realism' and 'revolutionary romanticism'. In the spring of 1958, at the CCP Central Committee's Chengdu conference, which lasted three weeks and was attended by leaders of different CCP Central Committee departments as well as eighteen provincial party bosses, Mao succeeded in further energizing the Great Leap Forward. In addition to the plan for greater speed in economic construction, he also pushed through agendas launching technological and cultural revolutions, continuing criticism of right revisionism, and 'smashing the blind faith in experts' (professors and bourgeois intellectuals) and Soviet practices. According to Mao, Soviet-style literary social realism did not reflect the reality in China. It was therefore inadequate for portraying the Great Leap Forward and the new China's progress toward a glorious communist future. He proposed a new Chinese formulation that combined revolutionary realism and romanticism.³¹ For the next months or years, the PRC's leading cultural theorists and literary figures, such as Zhou Yang and Guo Morou, debated over and expounded on the fundamental meaning of combining realism and romanticism. This entailed arts and literature putting an emphasis on the 'romantic' projection of ideals in ordinary life, or, as Zhou Yang put it, to 'describe the wide world of the people, depict the great struggles of the labouring masses, reflect the rise and prosperity of the new socialist world, the birth and development of the new men and

个人。他们包括江西省血吸虫病防治委员会委员兼江西医学院院长陈崇义，他说道：“在旧中国，我为医疗机构工作了16年。我从来没有听说或看到过任何地方根除血吸虫病。我没有看到任何资本主义国家的文件说有可能根除血吸虫病。但今天，我不仅听说有可能，而且亲眼看到了。”（陈是一位著名的医学教育家，他主张通过现代医学和教育来拯救中国，江西医学院是国民党时期最负盛名的国立医学院之一。）文章还引用了余江县不具名的村民欢欣鼓舞地说：“党和毛主席救了我们的命……今后，毛主席和党再给我们什么指示，只要我们的干部搭梯子，我们都愿意爬到天上去。”

³⁰

然而，有关余江的报告远非枯燥乏味的社论。它采用了一种典型的“大跃进”文学风格，融合了“革命现实主义”和“革命浪漫主义”。1958年春，在中共中央成都会议上，毛泽东成功地进一步激发了大跃进的热情。这次会议持续了三个星期，中共中央各部门的领导以及18位省委书记参加了会议。除了加快经济建设的计划外，他还推动了发起技术和文化革命、继续批判右倾修正主义以及“打破对专家（教授和资产阶级知识分子）和苏联实践的盲目迷信”的议程。毛泽东认为，苏式文学的社会主义现实主义不能反映中国的现实。因此，它不足以描绘大跃进和新中国走向光辉的共产主义未来的进程。他提出了一种新的中国式提法，将革命现实主义和浪漫主义结合起来。³¹在接下来的几个月或几年里，中国的主要文化理论家和文学人物，如周扬和郭沫若，就现实主义和浪漫主义相结合的根本意义进行了辩论和阐述。这需要艺术和文学强调普通生活中理想的“浪漫”投射，或者像周扬所说的那样，去“描写人民的广阔世界，描绘劳动群众的伟大斗争，反映新社会主义世界的崛起和繁荣，以及新人的诞生和发展”，

women of communism'.³² The life story of Deng Rumei, the heroine of Yujian's eradication narrative, as featured in Liu Guanghui and Chen Bingyan's special report, turned out to be a textbook example of this interpretation of the romantic projection of ideals in ordinary life. Deng's real name was not Rumei, however. While researching for this book, I learned that the error in her name was the result of a missing Chinese character on the authors' typewriter. This did not matter to the authors, however, since she was to represent the collective rather than any real individual.

The story of Deng was supposed to permit the reader a close-up view of an 'individual's experience of the disease that was said to have resulted from China's feudal past as well as Nationalist corruption'. As a woman, Deng had twice been a 'victim' of the old China. After marrying a man in her village, she became infected with schistosomiasis: 'She lost her spirit as well as so much weight, and her skin turned sallow. The disease left her with a distended abdomen, and for some years she was unable to conceive,' the authors informed the reader. As result of her infertility, Deng's marriage fell apart, and even her own mother turned against her. But the reader quickly learned that in 1956 the socialist upsurge in the Chinese countryside led by Chairman Mao Zedong brought profound transformation to the life of Deng as well as to her family. She was cured of the disease and returned to health. She even got her husband back, and a year later became pregnant and gave birth to a baby. 'This thirty-year-old woman has a naturally tanned face with a shade of red colour. The first glance tells us that she was healthy and strong. She was holding a baby in her arms when being interviewed ... With the arrival of the baby, their family life has turned ever more happy and harmonious.' And 'now the whole family work hard in the field,' the authors exclaimed. The story of Deng's pregnancy was not a mere detail; it was important to the PRC's claim to the singular triumph of its socialist road to improve the people's health. In the post-war world, one reason that schistosomiasis rose to become a major public health concern was that it caused infertility and dwarfism – both seen to be detrimental to the manpower economy of many newly decolonized countries in the developing world, from Egypt to countries in Southeast Asia and Latin America. The story ended with Deng telling the authors: 'The party and the Chairman are our saviour.' By linking social progress to individual benefit, and more specifically linking personal health with economic and social

共产主义妇女”。³² 玉鉴根除叙事中的女英雄邓汝梅的故事，正如刘光辉和陈炳炎的特别报道中所描述的那样，最终成为了对普通生活中理想的浪漫投射的典型例证。然而，邓的真名并不是汝梅。在为本书研究时，我了解到她名字的错误是由于作者的打字机上缺少一个汉字造成的。然而，这对作者来说并不重要，因为她代表的是集体，而不是任何真实的个人。

邓的故事本应让读者近距离观察一个“个人对疾病的体验”，据说这种疾病源于“中国的封建过去以及国民党的腐败”。作为一个女人，邓两次成为旧中国的“受害者”。在与村里的一个男人结婚后，她感染了血吸虫病：“她失去了精神，体重也减轻了很多，皮肤也变得蜡黄。这种疾病导致她腹部膨胀，多年无法怀孕，”作者告诉读者。由于不孕，邓的婚姻破裂，甚至她自己的母亲也反对她。但读者很快了解到，1956年，毛泽东主席领导下的中国农村的社会主义高潮给邓和她的家庭带来了深刻的转变。她的疾病被治愈，恢复了健康。她甚至找回了丈夫，一年后怀孕并生下了一个孩子。“这位三十岁的妇女有着自然黝黑的脸庞，带着一丝红色。第一眼就告诉我们，她很健康也很强壮。她接受采访时抱着一个婴儿……随着婴儿的到来，他们的家庭生活变得越来越幸福和谐。”作者惊叹道：“现在全家人都在田里努力工作。”邓怀孕的故事不仅仅是一个细节；它对于中华人民共和国声称其改善人民健康的社会主义道路取得了独特的胜利至关重要。在战后世界，血吸虫病之所以成为一个主要的公共卫生问题，其中一个原因是它会导致不孕不育和侏儒症——这两种情况都被认为不利于从埃及到东南亚和拉丁美洲等许多新近非殖民化发展中国家的人力经济。故事的结尾是邓告诉作者：“党和主席是我们的救星。”通过将社会进步与个人利益联系起来，更具体地将个人健康与经济和社会联系起来。

justice, the authors brought out one key political message of the Great Leap Forward: 'This transformative experience of the Deng family is an epitome of the transformation experienced by hundreds and thousands of families in the endemic Yujiang County.' To make the narrative newsworthy, this individual life story was paired with 'sputnik' statistics: after more than 4,000 victims were cured of the disease, more than 247 acres of improved agricultural land were reclaimed, and after eradicating schistosomiasis, Magang Township, which in previous years had relied on government subsidy, handed over an extra 15,000 kilograms of rice to the government, while the annual income of individual villagers had nearly doubled from 38 RMB per year in 1953 to 74 RMB (in previous years, 40 per cent of local villagers lived on government loans).³³

THE CHAIRMAN WRITES A POEM

The newspaper report on the success of the eradication campaign caught the eye of Chairman Mao Zedong. Jiangxi had occupied a special place in his heart. Nearly thirty years earlier, it was in Jiangxi that Mao, a young Communist leader, had succeeded in commanding a revolutionary war of the masses that would lead to the creation of the new China. The seed of revolutionary spirit that he had planted then seemed to have borne fruit again in the Great Leap Forward. Mao called the victory narrative of Yujiang's eradication of schistosomiasis the 'heroic display' of the Chinese people's revolutionary struggle in bringing about the Great Leap Forward, and he wanted this to be the narrative for the whole of China. Not long before – after reading the report from Henan province's Fengqiu County in north China boasting about how under the agricultural co-operative system local villagers in the Yingqu co-operative had eradicated poverty, freeing themselves of annual floods by building extensive water conservancy networks with their bare hands and thus bringing dry land under irrigation and converting alkaline land into paddy fields – Mao had written an essay in praise of agricultural collectivization. The essay was published together with the report from Fengqiu County in the inaugural edition of the *Red Flag* magazine, a theoretical journal of the CCP. Having just succeeded in driving the Great Leap further forward, in his essay the high-spirited Mao trumpeted that more and more Chinese people had joined his socialist revolution in transforming their 'poor and blank land' into

公正起见，作者提出了大跃进的一个关键政治信息：“邓家的这种变革性经历是地方病流行的余江县成千上万家庭经历的变革的缩影。”为了使叙述具有新闻价值，这个个人的生活故事与“人造卫星”统计数据配对：在4000多名患者被治愈后，247多英亩的改良农田被开垦，在消灭血吸虫病后，马岗乡在过去几年里一直依赖政府补贴，向政府额外上缴了15000公斤大米，而村民的年收入几乎翻了一番，从1953年的每年38元人民币增加到74元人民币（过去几年里，当地40%的村民靠政府贷款生活）。³³

主席写了一首诗

The newspaper report on the success of the eradication campaign caught the eye of Chairman Mao Zedong. Jiangxi had occupied a special place in his heart. Nearly thirty years earlier, it was in Jiangxi that Mao, a young Communist leader, had succeeded in commanding a revolutionary war of the masses that would lead to the creation of the new China. The seed of revolutionary spirit that he had planted then seemed to have borne fruit again in the Great Leap Forward. Mao called the victory narrative of Yujiang's eradication of schistosomiasis the 'heroic display' of the Chinese people's revolutionary struggle in bringing about the Great Leap Forward, and he wanted this to be the narrative for the whole of China. Not long before – after reading the report from Henan province's Fengqiu County in north China boasting about how under the agricultural co-operative system local villagers in the Yingqu co-operative had eradicated poverty, freeing themselves of annual floods by building extensive water conservancy networks with their bare hands and thus bringing dry land under irrigation and converting alkaline land into paddy fields – Mao had written an essay in praise of agricultural collectivization. The essay was published together with the report from Fengqiu County in the inaugural edition of the *Red Flag* magazine, a theoretical journal of the CCP. Having just succeeded in driving the Great Leap further forward, in his essay the high-spirited Mao trumpeted that more and more Chinese people had joined his socialist revolution in transforming their 'poor and blank land' into

a pest-free 'communist paradise': 'Never before I have seen so many people so high in spirit, so strong in morale and so firm in determination,' he exclaimed.³⁴ To him, it was with the same high spirit, strong morale, and firm determination that the people of Yujiang had 'broken down superstition, and let [their] initiative and creativity explode'. They had not only achieved the impossible by '[making] the high mountain bow its head and the river yield the way', they had also eradicated the disease where the imperialist West had failed.³⁵ For Mao, Yujiang's claim to success was his answer to those who doubted his Great Leap Forward policy, including the technocrats in the party who criticized the 'rushed advance' as well as medical experts such as Su Delong who doubted the feasibility of eradication: 'Until now our modern Hua Tuos [doctors and experts] had been rather reluctant. They lacked faith. Under the good party leadership, the situation is slowly improving. The masses have been mobilized to fight this battle. Now we need to get the scientists to join the battle too. By working together, they can send the Plague God back to heaven,' Mao proclaimed.³⁶ A month earlier, in a Politburo meeting, he had also said that 'it won't take us long to catch up with the United States and Great Britain.'³⁷ As his thoughts drifted back and forth, Mao could not sleep. On the spur of the moment, he grabbed a pen and jotted down a few verses in his favourite classic, eight-syllable quatrain style and with romantic passion. These verses would become his placard to 'propagate Yujiang's success, and to give [the Anti-Schistosomiasis Campaign as well as the Great Leap Forward] a push'.³⁸

The poem mirrored one of the most influential late Qing reformist thinkers Gong Zizhen's cycle poem about the role of human agency in bringing cosmic change. On the eve of the First Anglo-Chinese War – commonly known as the First Opium War – Gong quit his office for good. On his way home to Hangzhou, he stepped into a temple. Watching worshippers bowing to different gods, Gong felt alone. In anguish and wrath, he burst out, envisaging the cosmos being brought to a standstill, followed by the power of human agency in bringing about radical changes: 'From Gale and Thunder comes the Chinese Nation's Vital Force; How pitiful then, that thousands of horses all struck dumb; I urge [you], the Lord of Heaven, to shake us up again, and grant us human talents not bound to a single kind.'³⁹ In 1958, set to promote radical collectivization or the People's Commune against all opposition, Mao cited these words of Gong's several times to invoke

一个没有害虫的“共产主义天堂”：“我从未见过如此精神焕发、士气高昂、决心坚定的人们，”他惊叹道。³⁴对他来说，正是以同样高昂的精神、高昂的士气和坚定的决心，余江人民“打破了迷信，让[他们的]主动性和创造力爆发”。他们不仅实现了“[让]高山低头，河流让路”的不可能，还消灭了帝国主义西方未能消除的疾病。³⁵对于毛泽东来说，余江宣称的成功是他对那些怀疑他大跃进政策的人们的回应，包括党内批评“冒进”的技术官僚以及像苏德隆这样怀疑根除可行性的医学专家：“直到现在，我们的现代华佗[医生和专家]一直相当不情愿。他们缺乏信心。在党的正确领导下，情况正在慢慢改善。群众已经被动员起来参加这场战斗。现在我们需要让科学家也加入这场战斗。通过共同努力，他们可以将瘟神送回天堂，”毛泽东宣称。³⁶一个月前，在一次政治局会议上，他还说“我们不需要很长时间就能赶上美国和英国”。³⁷思绪飘忽不定，毛泽东无法入睡。一时兴起，他拿起笔，用他最喜欢的经典八句绝句风格和浪漫的激情写下了几句诗。这些诗句将成为他“宣传余江的成功，并推动[血吸虫病防治运动和大跃进]”的标语牌。³⁸

这首诗反映了晚清最具影响力的改革思想家龚自珍关于人的能动性在带来宇宙变化中的作用的组诗。在第一次英中战争——通常被称为鸦片战争——前夕，龚辞官归隐。在回杭州的路上，他走进了一座寺庙。看着信徒们向不同的神灵叩拜，龚感到孤独。在痛苦和愤怒中，他爆发了，设想宇宙停滞不前，随后是人的能动性带来根本性变革的力量：“九州生气恃风雷，万马齐喑究可哀。我劝[天公]，重抖擞，不拘一格降人才。”³⁹1958年，为了反对一切阻力，促进激进的集体化或人民公社，毛泽东多次引用龚自珍的这些话来唤起

a burned paper boat.⁴² Here, Mao was recalling how, since the seventeenth century, communities in southern China had celebrated the Plague God Festival every spring, the peak season for the outbreak of diseases. The festival was a prophylactic measure against epidemics. It always ended with a large, collective procession to expel all remaining plague demons and send the Plague God back to heaven. This was done by placing the Plague God on a massive boat made of paper that was then burned or floated away on the waters.⁴³

Despite his revolutionary romanticism, Mao was aware that the ‘demon’ schistosomiasis was not so easily evicted. To mobilize the masses and to have them committed to the eradication campaign, hard-core propaganda was needed in place of the traditional exorcist offering rites. On 1 July 1958, in a note to his secretary Hu Qiaomu, who was also the director of the Press Administration as well as the deputy minister of propaganda, Mao admitted that ‘eradicating schistosomiasis is going to be a hard battle’. He urged Hu to polish up what he had written and to have it published in the *People’s Daily* in a day or two so as ‘not to let people’s high spirit run cold’.⁴⁴ Hu, however, waited for another three months until the right moment to let Mao’s poem appear in print and aptly titled it ‘Send off the Plague God’. In the meantime, on 29 August, the CCP Central Committee had passed the resolution approving the People’s Commune as the new and only form of organization for rural China. The resolution claimed that this would ‘accelerate the speed of socialist construction ... thus prepare actively for the transition to communism. It seems that the attainment of communism in China is no longer a remote future event.’ The transition to communism would, however, involve ‘raising the entire people’s communist consciousness and morality to a higher degree’ and destroying those ‘legacies of the old society ... and remnants of bourgeois inequality’.⁴⁵ ‘Send off the Plague God’ hence became a perfect metaphor for a new China and its people, prepared to move toward the future utopian communist society.

‘SPRING COMES TO A WITHERED TREE’

With the publication of Mao’s ‘Send off the Plague God’ poem, those in charge of the implementation were compelled to revamp the rather troublesome Anti-Schistosomiasis Campaign. There was a far more enthusiastic atmosphere in 1958 than in 1955 when the campaign

一只烧毁的纸船。”⁴² 毛泽东在这里回忆说，自 17 世纪以来，中国南方各地的社区每年春天都会庆祝瘟神节，这是疾病爆发的高峰期。这个节日是一种预防流行病的措施。它总是以一个大型的集体游行结束，以驱逐所有剩余的瘟疫恶魔，并将瘟神送回天堂。这样做的方法是将瘟神放在一个巨大的纸船上，然后将其烧毁或漂浮在水面上。⁴³

尽管毛泽东具有革命的浪漫主义情怀，但他意识到“血吸虫”这种“恶魔”并非如此容易驱逐。为了动员群众并使他们致力于根除运动，需要用强硬的宣传来代替传统的驱魔仪式。1958 年 7 月 1 日，毛泽东在给他的秘书胡乔木（他同时也是新闻出版署署长和宣传部副部长）的便条中承认，“消灭血吸虫病将是一场艰苦的战斗”。他敦促胡润色他所写的内容，并在几天之内在《人民日报》上发表，以便“不让人们的高昂情绪冷却下来”。⁴⁴ 然而，胡又等了三个月，直到合适的时机才让毛泽东的诗歌发表，并恰如其分地将其命名为《送瘟神》。与此同时，8 月 29 日，中共中央委员会通过了决议，批准人民公社作为中国农村新的也是唯一的组织形式。该决议声称，这将“加速社会主义建设的速度……从而积极准备向共产主义过渡。在中国实现共产主义似乎不再是遥远的未来事件。”然而，向共产主义的过渡将涉及“将全体人民的共产主义意识和道德提升到更高的程度”，并摧毁那些“旧社会的遗产……以及资产阶级不平等的残余”。⁴⁵ 因此，“送瘟神”成为了一个完美的隐喻，象征着准备走向未来乌托邦共产主义社会的新中国及其人民。

枯木逢春

随着毛泽东《送瘟神》诗歌的发表，负责实施的人员不得不修改颇为棘手的血吸虫病防治运动。1958 年的气氛比 1955 年该运动开始时更热情得多。

had first been inaugurated. At the height of the Great Leap, with Chairman Mao marching the people of China toward his vision of a future communist society, the Nine-Man Subcommittee took on the challenge of eradicating not just schistosomiasis but also malaria, filariasis, ancylostomiasis, and kala azar. All were viewed as diseases of rural poverty. In November 1958, a month after the publication of the chairman's poem, the All China Conference on Parasitic Disease, under the direction of Wei Wenbo, resolved that China would achieve the basic eradication of all five parasitic diseases within one year, in time for the tenth anniversary of the founding of the PRC, their socialist Motherland. To reach this goal would entail a superhuman breakthrough in disease control. This meant 'doing away with the mysterious notions about science' and 'combining scientific research with technological revolution and the mass campaign'. In the meantime, experts such as Su Delong were publicly denounced for 'right conservatism' and being restrained by 'bourgeois conventions'. Medical and Health Technological Revolution conferences and exhibitions were held in major Chinese cities. In the autumn of 1958, the first National Health Technological Revolution Exhibition in Beijing showcased the PRC's Anti-Schistosomiasis Campaign and Yujiang County's putative victory in eradicating the disease, as well as local medical and health innovations from all over the country. As with agricultural 'sputnik fields', provinces competed to win the trophy in medical and health innovations bringing about disease eradication. Having done away with right conservatism, the masses were invited to participate in this technological revolution.⁴⁶ A campaign to collect folk remedies spread across the country. Many previously abandoned and excluded traditional practices, as well as long-lost home remedies, were rediscovered as innovations or national treasures and given the 'Traditional Chinese Medicine' (TCM) label. Within a few months, Sichuan province alone had collected and published 666,000 folk remedies, including a herbal mixture that contained *Achizoma Atractylodis*, *Angelica Dahurica*, *Szechwan Lovage Rhizome*, and *Cassia Twig*, traditionally used to treat headache and chills. A local practitioner had been using it since the 1930s to relieve malaria symptoms with some success. The local health authority, eager to add their mark to the Great Leap in disease eradication, publicized it as a malaria cure, and it was soon promoted in the rest of the country.⁴⁷ Yet not all the herbs were widely available, and there were no spare workers to collect

首次启动。在大跃进的高峰期，毛主席带领中国人民朝着他所设想的未来共产主义社会前进，九人小组承担了不仅要根除血吸虫病，还要根除疟疾、丝虫病、钩虫病和黑热病的挑战。所有这些都视为农村贫困的疾病。1958年11月，在主席发表诗歌一个月后，由魏文伯领导的全国寄生虫病会议决定，中国将在一年内，即在中华人民共和国成立十周年之际，基本消灭所有五种寄生虫病，为了实现这一目标，需要在疾病控制方面取得超人的突破。这意味着

“消除关于科学的神秘概念”，以及“将科学研究与技术革命和群众运动结合起来”。与此同时，苏德隆等专家因“右倾保守主义”和受到“资产阶级惯例”的束缚而受到公开谴责。中国主要城市举办了医疗卫生技术革命会议和展览。1958年秋，在北京举行的首届全国卫生技术革命展览展示了中华人民共和国的血吸虫病防治运动和余江县在根除该疾病方面的假定胜利，以及来自全国各地的当地医疗卫生创新。与农业“卫星田”一样，各省竞相赢得医疗卫生创新带来疾病根除的奖杯。在消除了右倾保守主义之后，群众被邀请参加这场技术革命。⁴⁶ 一场收集民间疗法的运动在全国蔓延开来。许多以前被抛弃和排除的传统做法，以及长期失传的家庭疗法，被重新发现为创新或国宝，并被贴上“中医药”（tcm）的标签。短短几个月内，仅四川省就收集并出版了666,000种民间疗法，其中包括一种含有苍术、白芷、川芎和桂枝的草药混合物，传统上用于治疗头痛和发冷。一位当地医生自20世纪30年代以来一直使用它来缓解疟疾症状，并取得了一些成功。当地卫生部门渴望在根除疾病的大跃进中留下自己的印记，便将其宣传为治疗疟疾的方法，并很快在全国推广。⁴⁷ 然而，并非所有的草药都能广泛获得，而且也没有多余的工人去收集。

them; efficacy also varied from province to province because of the lack of quality control. In 1959, based on their laboratory studies, researchers at the Chinese Academy of Medical Science's Parasitic Institute announced that this malaria cure had no effect in treating malaria. To achieve the Great Leap in eradicating malaria, hospitals and parasitic disease treatment centres relied on Cycloguanil – the most widely used anti-malaria drug before China obtained the technology to domestically produce chloroquine, primaquine, and primethamine in the 1960s.⁴⁸ Similarly, to reach the target of eradicating schistosomiasis, many local health authorities also opted for using NaPCP to kill snails, even though it was more expensive and harmful than plant-based TCM pesticides made of *Camellia oleosa* or *Croton tiglium* (this will be discussed further in the next chapter).

Promoting technological revolution and mobilizing the broad masses would involve further propaganda. In an article titled 'The Labouring Mass Should Become the Master of Culture', published in the inaugural edition of the *Red Flag* magazine, Ke Qingshi, the party boss in charge of the Anti-Schistosomiasis Campaign once again stressed that public health work was an integral part of the cultural revolution to transform the dirty, backward, and diseased Chinese countryside into pest-free socialist towns and cities with 'new style housing filled with sunshine and fresh clean air, boulevards and sports facilities'. 'There will be willow branches dangling outside each home like curtains as well as running water in front of every house,' Ke proclaimed. According to him, villagers would learn socialist and communist ideologies as well as how to live a healthy, clean, and cultured socialist way of life. For Ke, this would entail rural communities (which by then had been transformed into production brigades) forming their own drama groups and watching revolutionary-themed movies. At the same time, villagers would adopt the habits of hygiene as everyday norms, from regularly washing their hands with soap to bathing each day.⁴⁹ To deliver this cultural revolution would require involving the masses, with the masses, of course, functioning under the direction of the centralized state authority.

Wei Wenbo, by then actively running the campaign, was experienced in party propaganda work amongst the rural masses. A keen calligrapher and poet himself, he adapted Mao's romantic poem into a popular jingle so that the undereducated villagers would learn to recite it in their literacy class. This paid off, and 'Send off the Plague

它们；由于缺乏质量控制，疗效也因省而异。1959年，中国医学科学院寄生虫病研究所的研究人员根据他们的实验室研究宣布，这种治疗疟疾的方法在治疗疟疾方面没有任何效果。为了实现根除疟疾的大跃进，医院和寄生虫病治疗中心依赖于环胍乙胺——中国在20世纪60年代获得国内生产氯喹、伯氨喹和乙胺嘧啶技术之前使用最广泛的抗疟疾药物。⁴⁸同样，为了达到根除血吸虫病的目标，许多地方卫生部门也选择使用五氯酚钠来杀死蜗牛，尽管它比用山茶油或巴豆制成的植物性中药杀虫剂更昂贵、更有害（这将在下一章中进一步讨论）。

为了推动技术革命和动员广大群众，需要进一步宣传。柯庆施在《红旗》杂志创刊号上发表了一篇题为《劳动群众应该成为文化的主人》的文章，再次强调公共卫生工作是文化革命不可分割的一部分，旨在将肮脏、落后和多病的中国农村转变为没有害虫的社会主义城镇，拥有“充满阳光和新鲜空气的新式住房，林荫大道和体育设施”。柯庆施宣称：“每家每户的外面都会有柳枝像窗帘一样垂下来，每家每户的门前都会有自来水。”他认为，村民们将学习社会主义和共产主义意识形态，以及如何过上健康、清洁和有文化的社会主义生活。对于柯庆施来说，这将需要农村社区（当时已转变为生产队）组建自己的剧团并观看革命题材的电影。与此同时，村民们会将卫生习惯作为日常规范，从经常用肥皂洗手到每天洗澡。⁴⁹为了实现这场文化革命，需要动员群众，当然，群众要在中央集权的国家权力指导下发挥作用。

当时积极开展运动的魏文伯，在农村群众的党内宣传工作方面经验丰富。他本人是一位热心的书法家和诗人，他将毛泽东的浪漫诗改编成一首流行的顺口溜，以便让那些受教育程度不高的村民在扫盲班上学习背诵。这得到了回报，‘送瘟神’

God' soon became an everyday buzz-phrase in many endemic regions as the Anti-Schistosomiasis Campaign became the new Plague God Festival. Ironically, in recent years the exorcist rites of 'Send off the Plague God' are back in fashion in some parts of rural China, although this has little to do with the resurgence of schistosomiasis – villagers now regularly hold 'Send off the Plague God' processions to protest the wide-scale corruption that plagues China.

In her study on 'Emotion Work in the Chinese Revolution', Elizabeth Perry argues that, for the CCP, theatre was 'a critical means of eliciting an emotional reaction that was used intentionally to solidify popular commitment'.⁵⁰ With his long experience in party propaganda work, Wei Wenbo understood how to harness emotional excitement to further the revolutionary cause. Prior to turning 'Send of the Plague God' into a popular jingle, he gave instructions to adapt Mao's romantic poem and Yujiang's victory narrative into a popular spoken drama so that the masses would be moved to throw themselves with renewed vigour into the chairman-led Anti-Schistosomiasis Campaign. The reason Wei utilized spoken drama to move the villagers to act was that traditionally in rural China, public theatre and spectacle had played a central role in promoting social cohesion. Wei named the play *Spring Comes to the Withered Tree*, partly in the hope that it would help to revive the dying campaign, and scheduled it to be performed before a mass audience during the PRC's tenth anniversary celebration. Wang Lian was the young playwright chosen for the task, and for several years Wang was assigned to the PLA art troupe with the task of supporting the Land Reform and the Communist takeover of the Lower Yangtze regions by producing and directing revolutionary dramas as well as rice-planting dance musicals (*Yangge*). The latter was a popular performance form with its roots in rural northwest China. It was first adopted by the CCP in Yan'an in the 1940s to disseminate the message of revolution and socialist transformation. After the Communist Liberation in 1949, *Yangge*, as well as revolutionary spoken drama, was introduced to southern China by the new Communist authority as a vehicle for propaganda.⁵¹ They did, however, encounter competition from local operas. Prior to the Communist takeover, a vibrant operatic theatre culture had existed in southern China, with each region having its own local operatic traditions. Since most of them had told local narratives that conflicted with the CCP's narrative of revolution and the new Communist nationhood, they had had to undergo

在许多地方病流行地区,“瘟神”很快成为日常用语,因为血吸虫病防治运动变成了新的“瘟神节”。具有讽刺意味的是,近年来,在中国一些农村地区,“送瘟神”的祭祀仪式又开始流行起来,尽管这与血吸虫病的复发关系不大——村民现在经常举行“送瘟神”游行,以抗议困扰中国的广泛腐败现象。

伊丽莎白·佩里在她关于“中国革命中的情感劳动”的研究中认为,对于中国共产党来说,戏剧是“引发情感反应的关键手段,这种情感反应被有意识地用来巩固民众的承诺”。⁵⁰ 魏文伯长期从事党的宣传工作,深知如何利用情感激发革命事业。在将“送瘟神”变成一首流行的顺口溜之前,他指示将毛泽东的浪漫诗歌和余江的胜利故事改编成通俗的活报剧,以便群众受到感动,以新的活力投身于主席领导的血吸虫病防治运动。魏文伯之所以利用活报剧来感动村民,是因为在中国农村,公共戏剧和表演传统上在促进社会凝聚力方面发挥着核心作用。魏文伯将这部戏命名为枯木逢春,部分原因是希望它能帮助振兴垂死的运动,并计划在中华人民共和国成立十周年庆典期间向大众演出。王莲是为这项任务选定的年轻剧作家,王莲多年来被分配到该剧团,负责通过制作和导演革命戏剧以及插秧歌舞(秧歌)来支持土地改革和共产党对长江下游地区的接管。后者是一种流行的表演形式,其根源在于中国西北农村。它最初在1940年代被中国共产党在延安采用,以传播革命和社会主义改造的信息。1949年共产党解放后,秧歌和革命话剧一起被新的共产党政权引入中国南方,作为宣传的工具。⁵¹ 然而,他们确实遇到了来自地方戏曲的竞争。在共产党接管之前,中国南方存在着充满活力的戏曲文化,每个地区都有自己的地方戏曲传统。由于他们中的大多数讲述的地方叙事与中国共产党关于革命和新共产主义民族国家的叙事相冲突,因此他们不得不经历

reform or reconfiguration.⁵² Wang Lian and his colleagues in the art troupe participated in this process.

In addition to his duty to mobilize the masses through theatre work, Wang Lian could draw on his own personal experience, since he himself had been infected by schistosomiasis and subsequently recovered from it. The experience gave him the motivation to tell the party narrative as a human drama experienced by individuals and families. A native of Shandong province near the Korean peninsula, Wang was one of the thousands of southbound cadres who crossed the Yangtze to bring the revolution to the south. Upon arriving at the ancient canal town Suzhou, Wang Lian was diagnosed with schistosomiasis. While recovering in hospital, Wang had befriended another sick soldier. Too ill to write, this soldier had asked Wang to draft a love letter on his behalf to his wife back home in northern China, telling her how much he missed her but that for the greater cause of bringing the revolution to southern China, he had to stay away from her and the family until the work was done. After surviving the disease, for years Wang could not get the sick soldier and his wife out of his mind. He imagined his play, about how the new China had achieved the Great Leap in eradicating schistosomiasis under CCP leadership, as a family drama of separation and love but with a happy ending. Furthermore, the love story was a genre familiar to the mass audience in the Lower Yangtze region, since it had been an enduringly popular genre for local operas in all the endemic regions. It would therefore make the play appealing to them.⁵³

Five years prior to Wang taking on the task of writing the play, the Shanghai Film Studio had turned a famous local Yue opera (which originated from regions around Zhejiang and Shanghai), *The Butterfly Lovers*, into a technicolour movie. It was the first colour feature ever made in the PRC. Subsequently, on an official visit to Geneva, China's premier Zhou Enlai entertained his Western guests, including the British prime minister Sir Anthony Eden, by showing them the film. Zhou gave the film its English name: *The Chinese Romeo and Juliet*. The Western guests were said to have been deeply moved by its tragic love story as well as mesmerized by the outlandish performance and exotic scenery. The movie was subsequently shown in French cinemas with great success. We are told that George Sadoul was moved by the long line waiting outside a Paris theatre and that even Charlie Chaplin praised the film as extraordinary. It also won awards in international film festivals and was a box

改革或重塑。⁵² 王连和艺术团的同事们参与了这一过程。

除了通过戏剧工作动员群众的职责外，王连还可以借鉴自己的亲身经历，因为他自己也曾感染血吸虫病，后来又康复了。这段经历激励他以个人和家庭经历的人间戏剧来讲述党的叙事。王是山东省朝鲜半岛附近人，是成千上万南下干部中的一员，他们横渡长江，将革命带到南方。王连抵达古运河城镇苏州后，被诊断出患有血吸虫病。在医院康复期间，王结识了另一名生病的士兵。这名士兵病得太重，无法写字，便请王代他给家乡中国北方的妻子写一封情书，告诉她他多么想念她，但为了将革命带到中国南方的更伟大的事业，他不得不远离她和家人，直到工作完成。在战胜疾病后，多年来，王一直无法将这名生病的士兵和他的妻子从脑海中抹去。他设想他的剧本，讲述了新中国如何在 {ccp} 的领导下实现了消灭血吸虫病的大跃进，讲述了一个关于分离和爱的家庭戏剧，但结局是美好的。此外，爱情故事是长江下游地区大众观众熟悉的类型，因为它是所有流行地区的当地歌剧中经久不衰的流行类型。因此，这将使这部剧对他们具有吸引力。⁵³

在王先生承担创作这部剧本任务的五年前，上海电影制片厂已将当地著名的越剧（起源于浙江和上海周边地区）《梁山伯与祝英台》改编成一部彩色电影。这是中华人民共和国第一部彩色故事片。随后，在中国总理周恩来对日内瓦进行正式访问时，他通过放映这部电影来招待他的西方客人，其中包括英国首相安东尼·艾登爵士。周恩来给这部电影起了英文名：《中国的罗密欧与朱丽叶》。据说西方客人被其悲惨的爱情故事深深感动，并被奇异的表演和异国情调的风景所吸引。这部电影随后在法国电影院上映并取得了巨大的成功。我们被告知，乔治·萨杜尔被巴黎一家剧院外排起的长队所感动，甚至查理·卓别林也称赞这部电影非常出色。它还在国际电影节上获奖，并且票房

office hit in Hong Kong.⁵⁴ After this international success, *Yue* opera became even more popular throughout China. For culture cadres or playwrights like Wang Lian, there was an ever-greater desire to produce artwork that was revolutionary but distinctively Chinese,^{*} while at the same time appealing to both international and Chinese mass audiences. On the one hand, the success of *The Chinese Romeo and Juliet* had shown that romantic love was a genre that cut across cultures and politics; on the other, by giving human tragic love a happy ending, it also became the story of revolution and thus of the new China. Indeed, for intellectuals and culture cadres like Wang Lian, the Chinese revolution itself was a love story about themselves and their troubled Motherland.

After its successful première in Shanghai, *Spring Comes to a Withered Tree*, Wang Lian's play about the successful eradication of schistosomiasis that he framed as a love story, was chosen to go on a national tour. In March 1960, when it was staged at the Capital Theatre by the Beijing People's Theatre, it caught the eye of Premier Zhou Enlai. He proposed to turn the play into a film. As he relished the international success of *The Chinese Romeo and Juliet*, Zhou was keen for film to be utilized as a propaganda medium to mobilize the masses as well as to promote the image of China to a world audience, winning over sympathisers who might not have subscribed to the revolutionary cause of the PRC – in this instance, the Great Leap Forward. With the Sino-Soviet split following Stalin's death and subsequent de-Stalinization of the USSR and with Sino-Western relationships in serious disarray, Zhou and Lu Dingyi, the PRC's minister of propaganda at the time, encouraged Chinese playwrights and filmmakers to create plays and films that bore no resemblance to 'Khrushchev's revisionist' as well as imperialist films and plays. In a later address to the Ninth National Conference on Schistosomiasis Control (1963), Lu stated unequivocally that the film of *Spring Comes to a Withered Tree* was a piece of political propaganda for the international and the domestic audience:

* Since the early twentieth century, it has been an enduring theme for Chinese artists and writers to create an art and literature that was modern or revolutionary as well as Chinese. In the PRC, this became a state-sponsored enterprise, since the arts and literature are an important propaganda tool. During and after the Great Leap Forward, there has also been a great push to produce 'folk' and 'Chinese' arts and literature that was of and for the masses.

办事处在香港大受欢迎。⁵⁴ 在这次国际成功之后, 越剧在中国各地变得更加流行。对于像王炼这样的文化干部或剧作家来说, 他们越来越渴望创作出具有革命性但又具有鲜明中国特色的艺术作品,^{*} 同时又能吸引国际和中国的广大观众。一方面, 《中国版的罗密欧与朱丽叶》的成功表明, 浪漫爱情是一种跨越文化和政治的题材; 另一方面, 通过赋予人类悲剧爱情一个圆满的结局, 它也成了革命的故事, 从而也成了新中国的故事。的确, 对于像王炼这样的知识分子和文化干部来说, 中国革命本身就是一个关于他们自己和他们饱经磨难的祖国之间的爱情故事。

在上海成功首演后, 王炼的关于成功根除血吸虫病的剧作《枯木逢春》被选为全国巡演。1960年3月, 该剧在北京人民剧场演出时, 引起了周恩来总理的注意。他提议将这部剧改编成电影。由于他很欣赏《中国版的罗密欧与朱丽叶》在国际上取得的成功, 周恩来热衷于将电影作为一种宣传媒介, 以动员群众, 并向世界观众宣传中国的形象, 争取那些可能没有赞同中华人民共和国革命事业(在本例中为大跃进)的同情者。随着斯大林去世后中苏分裂以及苏联的去斯大林化, 以及中西方关系严重失调, 周恩来和当时的中华人民共和国宣传部长陆定一鼓励中国剧作家和电影制作人创作与“赫鲁晓夫修正主义”以及帝国主义电影和戏剧毫无相似之处的戏剧和电影。在后来对第九届全国血吸虫病防治会议(1963年)的讲话中, 陆毫不含糊地表示, 《枯木逢春》的电影是为国际和国内观众准备的政治宣传片:

* 自20世纪初以来, 对于中国艺术家和作家来说, 创作具有现代性或革命性以及中国特色的艺术和文学作品一直是一个持久的主题。在新中国, 这成为一项国家赞助的事业, 因为艺术和文学是重要的宣传工具。在大跃进期间和之后, 人们也大力推动创作属于大众、服务于大众的“民间”和“中国”艺术和文学。

We are the world leader in schistosomiasis control ... We have produced a film and a play about [the Anti-Schistosomiasis Campaign]. Recently, some Japanese experts marvelled over our success: 'You have achieved much more [in schistosomiasis control] than we could in Japan. This is because China is a socialist country,' they praised. We wanted the imperialists and revisionists to know that we are more successful in public health work than them ... Some people have failed to understand that the arts are powerful tools in building socialism. They debated for many years over the role of the arts, and ended up with revisionists like Tito and Khrushchev ... Under Khrushchev, the Soviet Union has produced many films, plays, and literature that are anti-revolution, anti-revolutionary war, and anti-Stalin ... We need to be prepared, and to fight against imperialist and revisionist arts.* To do so, we need to produce revolutionary plays and films that reflect the spirit of our time, and at the same time are anti-feudal, anti-imperialist, and anti-revisionist. This is an integral part of our cultural revolution.⁵⁵

As an integral part of the cultural revolution, Lu urged that arts such as the cinema and drama must use their power to mobilize the masses at home.

There was an important political reason for this outburst of propagandistic exuberance. It is worth noting that by 1960, famine had spread across the entire country and had already led to the death of millions in the countryside. With Great Leap euphoria withering, the Anti-Schistosomiasis Campaign had literally ground to a halt. Struggling to fulfil the government's grain procurement target, as well as cope with the escalating public health emergency caused by mass malnutrition, local cadres and health officials openly refused to implement the campaign: 'Our task now is to provide food and clothing for villagers, schistosomiasis control has to wait for a couple of years.' 'Eradicating schistosomiasis is like trying to reach the stars in the sky, we will never be able achieve it ... Our hair will turn white, and our teeth will fall out, but we will never get rid

* Strictly speaking, in 1963 there was no revisionist art in the Soviet Union and eastern Europe. Revisionism was first crystallized in literature only in 1967. But for the Chinese, 'apparatchik' here refers to films that depart from the hard Stalinist line as well as the Socialist Realist mold. Even more important, Lu Dingyi was less interested in Socialist Realism. He was asserting the Chinese determination to carve its own path to socialism, independent of other existing experiences.

我们是血吸虫病防治的世界领导者 …… 我们制作了一部关于 [血吸虫病防治运动] 的电影和戏剧。最近, 一些日本专家对我们的成功赞叹不已: “你们在 [血吸虫病防治] 方面取得的成就远比我们在日本所能取得的成就大得多。这是因为中国是一个社会主义国家。” 他们称赞道。我们想让帝国主义者和修正主义者知道, 我们在公共卫生工作方面比他们更成功 …… 有些人未能理解艺术是建设社会主义的有力工具。他们就艺术的作用争论多年, 最终落得像铁托和赫鲁晓夫这样的修正主义者 …… 在赫鲁晓夫的带领下, 苏联制作了许多反革命、反革命战争和反斯大林的电影、戏剧和文学作品 …… 我们需要做好准备, 与帝国主义和修正主义艺术作斗争。* 为此, 我们需要制作反映我们时代精神的革命戏剧和电影, 同时又要反封建、反帝国主义和反修正主义。这是我们文化革命的一个组成部分。⁵⁵

作为文化大革命的组成部分, 鲁迅敦促电影和戏剧等艺术形式必须利用其力量来动员国内民众。

这种宣传狂热的爆发有一个重要的政治原因。值得注意的是, 到 1960 年, 饥荒已经蔓延到全国, 并已导致农村数百万人死亡。随着大跃进的狂热消退, 血吸虫病防治运动实际上已经停滞不前。为了完成政府的粮食采购目标, 并应对大规模营养不良造成的日益严重的公共卫生紧急情况, 地方干部和卫生官员公开拒绝实施该运动: “我们现在的任务是为村民提供食物和衣物, 血吸虫病防治工作必须等待几年。” “根除血吸虫病就像试图摘取天上的星星, 我们永远无法实现它 …… 我们的头发会变白, 牙齿会掉光, 但我们永远无法摆脱它”

* 严格来说, 1963 年苏联和东欧还没有修正主义艺术。修正主义最初只是在 1967 年的文学作品中结晶。但对中国人来说, “官僚” 指的是那些背离强硬的斯大林主义路线以及社会主义现实主义模式的电影。更重要的是, 陆定一不太关注社会主义现实主义。他是在强调中国决心开辟自己通往社会主义的道路, 独立于其他现有经验。

of schistosomiasis,' they lamented.⁵⁶ On the other hand, following Yujiang's 'successful' model in declaring eradication, a huge number of counties made a political gesture in carrying out 'shock work' eliminating snails and providing mass treatment for the disease. Having gone through the motion a couple of times, they, too, declared near-eradication and buried the campaign together with the snails. With villagers and their cadres losing the 'high spirit' for the Anti-Schistosomiasis Campaign – or more generally, for the Great Leap Forward – there was an urgent need to use a mass medium such as film to win back the masses. Echoing Ke Qingshi's earlier speech, Lu Dingyi once again stressed that arts and health workers should work hand-in-hand to bring about a cultural revolution aimed at changing prevailing social customs. According to him, while public health work would enable the masses to transform their everyday life and live in a socialist way, cinema and drama, as well as other forms of visual arts, were powerful tools for not only spreading the message of health and hygiene but also for changing the way people thought: 'that's not the same as injecting medicine into their body.' And 'we need to teach our five hundred million peasants new socialist morals and ideologies, and transform them into builders for communism, in particular our youth who had never seen a landlord or an imperialist.'⁵⁷

Although the policy of austerity was to continue, with millions starving to death in the countryside (it was rumoured that Chairman Mao Zedong had even given up eating his favourite fatty pork, while Premier Zhou Enlai had switched from drinking tea to plain boiled water), the leadership at the Shanghai Film Studio backed the *Spring Comes to a Withered Tree* film with full financial support as well as manpower. They understood it was to be an important piece of political propaganda. Zheng Junli, modern China's most celebrated film director, was named director. Xu Sangchu, the PRC's top film cadre and producer, who was the producer for this film, had hoped it would become another classic, on a par with Zheng Junli's immensely popular wartime epic *A Spring River Flows East* (1947). Initially, Zheng hesitated to take on the film project because he had been criticized for glorifying bourgeois urban intellectuals and misrepresenting the masses in his earlier film *Husband and Wife* (1951) and was cautious about taking on another film project about the masses. In addition, he had just finished making *Lin Zexu* – an epic film about the First Opium War

他们哀叹道：“血吸虫病”。⁵⁶另一方面，在余江宣布根除血吸虫病的“成功”模式之后，大量县份开展了一项政治姿态，即开展“突击工作”，消灭钉螺并为该疾病提供大规模治疗。经过几次这样的形式主义之后，他们也宣布接近根除，并将这场运动与钉螺一起埋葬。由于村民和他们的干部失去了对“防治血吸虫病运动”的热情——或者更普遍地说，对“大跃进”的热情——迫切需要使用电影等大众媒体来赢回群众。与柯庆施之前的讲话相呼应，陆定一再次强调，文艺和卫生工作者应携手合作，发起一场旨在改变社会习俗的文化革命。他认为，公共卫生工作将使群众能够改变他们的日常生活并以社会主义的方式生活，而电影和戏剧以及其他形式的视觉艺术，不仅是传播健康和卫生信息的有力工具，也是改变人们思想方式的有力工具：“这与将药物注射到他们体内不同。”并且“我们需要教导我们的五亿农民新的社会主义道德和意识形态，并将他们转变为共产主义的建设者，特别是我们那些从未见过地主或帝国主义者青年。”⁵⁷

尽管紧缩政策仍在继续，农村数百万人饿死（有传言说毛泽东主席甚至放弃了他最喜欢的肥猪肉，而周恩来总理也从喝茶改成了喝白开水），但上海电影制片厂的领导层全力支持电影《枯木逢春》，给予了充分的财政和人力支持。他们明白这将是一部重要的政治宣传片。中国现代最著名的电影导演郑君里被任命为导演。许桑楚是中华人民共和国的顶级电影干部和制片人，也是这部电影的制片人，他希望这部电影能成为又一部经典之作，与郑君里广受欢迎的战争史诗《一江春水向东流》（1947年）相提并论。起初，郑君里犹豫是否要接手这部电影项目，因为他之前的电影《夫妇》（1951年）因美化资产阶级城市知识分子和歪曲群众而受到批评，因此他对拍摄另一部关于群众的电影项目持谨慎态度。此外，他刚刚完成了《林则徐》的制作——这是一部关于第一次鸦片战争的史诗电影

— that was chosen as the visual centrepiece of the tenth anniversary celebration of the PRC. Zheng was at the high point of his career and had his heart set on making a landmark film about modern China's cultural icon, Lu Xun, because the eightieth anniversary of Lu Xun's birth was coming up.⁵⁸ But Zheng could not refuse to take on the film project *Spring Comes to a Withered Tree* for several reasons. He was told that the film was Premier Zhou's idea; he also learned from Xu Sangchu, who was a native of Zhejiang province, that Xu's father as well as the children of two of Xu's relatives had died of schistosomiasis. Perhaps equally important, Zheng was energized by the Great Leap claim that his Motherland would soon surpass the imperialists in the West in all fields, from public health to the arts.

To prepare for the film, Zheng retreated to the countryside to be 'closer' to the masses, hoping to avoid being accused again of being 'detached from the masses'. The rural environment, as well as the euphoria over breaking down bourgeois conventions, encouraged him to experiment with a relatively new cinematic style that employed folk arts (thought to be spontaneously 'mass culture') as well as an ink and wash technique evocative of Chinese landscape painting. This allowed him to justify that the film was *about* the masses and yet also *of* the masses while at the same time being distinctively Chinese. To capitalize on the success of *The Chinese Romeo and Juliet*, Zheng invited one of the immensely popular local opera singers, Yu Hongxian, to sing Mao's poem 'Send off the Plague God' to an enchanting *Pingtán* melody.* Zheng also gave priority to the love story of Bitter Sister and Winter Brother, the heroine and hero of the film (modelled on the story of Deng Rumei in Yujiang, as had first appeared in the *People's Daily* special report). It is also worth mentioning that both Zheng Junli and his crew became bored with shooting scenes of village cadres and health professionals arguing about the relative importance of agricultural productivity and schistosomiasis control. They found such debates tedious to film. So they spent more and more time building up a melodramatic plot between Bitter Sister and Winter Brother and the latter's mother (played by one of modern China's most popular actors, Shangguan Yunzhu).⁵⁹ The crew also shot a great deal of Orientalist background 'wallpaper' scenes

* *Pingtán* is a popular local storytelling opera from Jiangsu and Zhejiang provinces.

——这被选为中华人民共和国成立十周年庆典的视觉中心。郑正处于他事业的巅峰，并决心制作一部关于现代中国文化偶像鲁迅的具有里程碑意义的电影，因为鲁迅诞辰八十周年即将到来。⁵⁸ 但郑有几个原因无法拒绝接受电影项目枯木逢春。他被告知这部电影是周总理的想法；他还从浙江省人徐桑楚那里得知，徐的父亲以及徐的两个亲戚的孩子都死于血吸虫病。也许同样重要的是，郑因为“大跃进”的说法而精神焕发，该说法称他的祖国很快将在所有领域超越西方的帝国主义者，从公共卫生到艺术。

为了准备这部电影，郑君里退居乡下，以便“更贴近”群众，希望避免再次被指责“脱离群众”。乡村环境，以及打破资产阶级传统的狂热，鼓励他尝试一种相对较新的电影风格，这种风格运用了民间艺术（被认为是自发的“大众文化”）以及让人联想到中国山水画的笔墨技巧。这使他能够证明这部电影是关于群众的，而且也是属于群众的，同时又具有鲜明的中国特色。为了利用 *The Chinese Romeo and Juliet*（中国的罗密欧与朱丽叶）的成功，郑君里邀请了广受欢迎的当地戏曲演员俞红仙，用迷人的评弹曲调演唱毛泽东的诗《送瘟神》。⁵⁹ 郑君里还优先考虑了苦妹和冬哥的爱情故事，他们是电影中的男女主角（以余江的邓汝梅的故事为原型，该故事最初出现在人民日报的特别报道中）。值得一提的是，郑君里和他的剧组都厌倦了拍摄村干部和卫生专业人员争论农业生产力与血吸虫病控制相对重要性的场景。他们觉得拍摄这些辩论很乏味。因此，他们花了越来越多的时间在苦妹和冬哥以及后者的母亲（由现代中国最受欢迎的演员之一上官云珠饰演）之间构建一个情节剧。⁵⁹ 剧组还拍摄了大量东方主义背景的“墙纸”场景

* 评弹是流行于江苏和浙江一带的民间说书曲艺。

of beautiful canal towns, stone bridges, traditional Jiangnan style courtyards, flowing rivers and mountains, and dangling branches of willow trees. Fully aware that this film was a piece of political propaganda, the crew filmed many scenes of bumper harvests and the masses happily working in the fields or digging canals. Despite Zheng's ambiguity, *Spring Comes to a Withered Tree* managed to use the cinema to reinforce the narrative of Yujiang's heroic battle against schistosomiasis that was first reported in the *People's Daily* in 1958. In the context of the increasingly severe famine, plus the Sino-Soviet split, the authorities were scrambling around for resources. Since eradication was nowhere in sight, there was an ever greater push for a technological revolution. To this, the film added the theme of Bitter Sister being cured by the TCM doctors after the Western bourgeois doctor denied her treatment. This was a theme beloved by authors and artists in the Soviet Union, since medicine and health care were tangible programs that made socialism, either Stalinist or Maoist, appealing locally as well as abroad. Furthermore, the tension between medicine for the masses and Western bourgeois medicine would become an important feature of public health in the PRC and abroad in the following decade. This was exemplified by the decentralization of health care delivery in the PRC, later marked by the Barefoot Doctor Campaign, another public health centrepiece in the PRC, which still lay some years ahead.

In the meantime, with the goal of eradication slipping further away and the failure to deliver the promised health and well-being, the film *Spring Comes to a Withered Tree*, together with cheap public health posters, were used by authorities to bridge the gap between the increasingly harsh daily reality and the Maoist utopian future as well as to inspire villagers to participate in the official Anti-Schistosomiasis Campaign.

Screenings were organized for villagers in the endemic regions to watch the film in the open air. In 1964, an official estimate showed that the film had been screened more than 1,100 times in schistosomiasis endemic regions, with 440,000 viewers.⁶⁰ In rural Jiangsu province, mobile projection teams screened the film 57 times, with a reported more than 58,000 viewers.⁶¹ According to Guan Wenwei, Jiangsu province's deputy head at the time who was also a PLA

of beautiful canal towns, stone bridges, traditional Jiangnan style courtyards, flowing rivers and mountains, and dangling branches of willow trees. Fully aware that this film was a piece of political propaganda, the crew filmed many scenes of bumper harvests and the masses happily working in the fields or digging canals. Despite Zheng's ambiguity, *Spring Comes to a Withered Tree* managed to use the cinema to reinforce the narrative of Yujiang's heroic battle against schistosomiasis that was first reported in the *People's Daily* in 1958. In the context of the increasingly severe famine, plus the Sino-Soviet split, the authorities were scrambling around for resources. Since eradication was nowhere in sight, there was an ever greater push for a technological revolution. To this, the film added the theme of Bitter Sister being cured by the TCM doctors after the Western bourgeois doctor denied her treatment. This was a theme beloved by authors and artists in the Soviet Union, since medicine and health care were tangible programs that made socialism, either Stalinist or Maoist, appealing locally as well as abroad. Furthermore, the tension between medicine for the masses and Western bourgeois medicine would become an important feature of public health in the PRC and abroad in the following decade. This was exemplified by the decentralization of health care delivery in the PRC, later marked by the Barefoot Doctor Campaign, another public health centrepiece in the PRC, which still lay some years ahead.

与此同时，由于根除血吸虫病的目标渐行渐远，且未能实现承诺的健康和福祉，当局利用电影枯木逢春以及廉价的公共卫生海报来弥合日益严酷的日常生活与毛泽东时代的乌托邦未来之间的差距，并激励村民参与官方的反血吸虫病运动。

放映 were 组织 for 村民 in the 地方性的 地区 to 在露天观看这部电影。1964 年，⁶⁰ 一项官方估计显示，该电影在血吸虫病流行地区放映了 1100 多次，观众达 44 万人。⁶¹ 在江苏省农村，流动放映队放映了 57 次，据报道有超过 58000 名观众。⁶¹ 据当时江苏省副省长关文蔚说，他也是一位



Figure 4.1 'Involve the Masses. Adopt Integrated Approach. Constantly Fight [the Disease]. To Eradicating Schistosomiasis Is to Greet the New High Tide of Agricultural Production'. (Poster produced by Zhejiang Province Disease Prevention Centre, 1964.)



图 4.1 “发动群众, 全面规划, 经常战斗[, 消灭血吸虫病], 迎接农业生产新高潮”。(浙江省疾病预防控制中心制作的海报, 1964 年。)



Figure 4.2 'Earth Burial Is an Excellent Way to Eliminate Snails'.
(Poster produced by Zhejiang Province Disease Prevention Centre, 1964.)



图 4.2 “土埋是消灭钉螺的好办法”。（浙江省疾病预防控制中心制作的海报，1964 年。）



Figure 4.3 'It Is Also Necessary to Apply Chemical Control to Eliminate Snails'. (Poster produced by Zhejiang Province Disease Prevention Centre, 1964.)

general who had led the revolution in Jiangsu province,* the film inspired many villagers to participate in the local control work.⁶² In urban Shanghai, however, when the film was first released in the spring of 1964, cinemas across the city reported having real difficulty selling tickets because urban residents did not want to watch another propaganda film about rural China. To attract an audience, urban dwellers were handed free cinema tickets, and government work units made it compulsory for employees to watch the film. After watching the film, the audience response suggested

* Guan was a native of Jiangsu province and had extensive experience in peasant-mobilizing work in different parts of Jiangsu. After the PLA liberated Jiangsu, as the new Communist authority's party representative who led the rural work, for some years Guan oversaw the local schistosomiasis control. As of the late 1980s, his daughter Guan Xiaohong was an academic researcher who participated in the Anti-Schistosomiasis Campaign. I interviewed her informally some years ago. Some of the research questions for this book have grown out of that interview.



图 4.3 “也有必要使用化学防治来消灭蜗牛”。（浙江省疾病预防控制中心制作的海报，1964 年。）

一位曾领导江苏省革命的将军,* 这部电影激励了许多村民参与当地的控制工作。⁶² 然而, 在上海, 当这部电影于 1964 年春天首次上映时, 全市的电影院都表示很难售出电影票, 因为城市居民不想再看一部关于中国农村的宣传片。为了吸引观众, 城市居民被免费赠送电影票, 政府工作单位强制要求员工观看这部电影。看完电影后, 观众的反应表明

* 关是江苏省人, 在江苏省不同地区的农民动员工作方面有丰富的经验。解放军解放江苏后, 作为新共产党当局领导农村工作的党代表, 关多年来负责当地的血吸虫病防治工作。截至 20 世纪 80 年代末, 他的女儿关小红是一名学术研究人员, 曾参与抗血吸虫病运动。多年前, 我曾非正式地采访过她。本书的一些研究问题就是从那次采访中产生的。



Figure 4.4 Poster for film *Spring Comes to the Withered Tree* (1961).



图 4.4 电影枯木逢春 (1961) 的海报。

that they were deeply moved by the love story as well as impressed by the beautiful scenery and the *Pingtian* melody but much less so by the official victory narrative of the Anti-Schistosomiasis Campaign.⁶³ During the Great Proletarian Revolution, the film was temporarily banned after Wei Wenbo was officially condemned as a ‘counter-revolutionary’ in 1967 and subsequently ‘disappeared’. (Later, it turned out that he was incarcerated in a political prison.) Zheng Junli, too, was put under house arrest and died.

After the death of Mao, Wei Wenbo was rehabilitated, as was the film. It again played to mass audiences in the endemic regions to ‘educate and promote awareness about schistosomiasis’. Without adequate data, it is difficult to assess the film’s impact on the PRC’s Anti-Schistosomiasis Campaign. There is little doubt, however, that it inspired some individuals to identify with the heroine and the hero of the film. Recently, while visiting local museums dedicated to the Anti-Schistosomiasis Campaign in Jiangsu province’s Qingpu County, Zhejiang province’s Jiaying County, and Jiangxi province’s Yujiang County, I came across similar texts in each museum claiming that the film was based on their own ‘heroic’ experience of ‘Sending off the Plague God’ and that Bitter Sister or Winter Brother had been ‘real’ villagers from their counties. After speaking to some participants in the campaign, it dawned on me that for villagers who had endured extremely cold weather burying snails with half-empty stomachs, it was important that they remember the time as worth celebrating rather than as time wasted. The film *Spring Comes to a Withered Tree* allowed them to see themselves as ‘heroes’ who had taken part in a ‘heroic battle’ against a disease that had claimed the lives of many of the people they knew: in the lower reaches of the Yangtze River, almost every family had some connection to the disease. The film thus gave them a sense of self-worth. By sharing the happiness of Bitter Sister and Winter Brother, they could, at least for a moment, forget the darker side of the campaign, despite the bitter experience of the famine and the resulting fear of an uncertain future. As Gerard Lemos’s illuminating study, *The End of the Chinese Dream: Why Chinese People Fear the Future*, shows, the majority of people in China remain worried about health problems.⁶⁴ Their anxieties over poor health are exacerbated because of the deficiencies in the Chinese health care system. This anxiety has its roots, to no little extent, in the memories of the famine and the mass death and illnesses that resulted.

他们深受爱情故事的感动，也对美丽的景色和平潭旋律印象深刻，但对血吸虫病防治运动的官方胜利叙事却不那么感兴趣。⁶³ 在无产阶级文化大革命期间，该影片在魏文伯于 1967 年被正式谴责为

“反革命分子”并随后“失踪”后被暂时禁止。（后来，事实证明他被关在政治监狱里。）郑君里也被软禁并去世。

After the death of Mao, Wei Wenbo was rehabilitated, as was the film. It again played to mass audiences in the endemic regions to ‘educate and promote awareness about schistosomiasis’. Without adequate data, it is difficult to assess the film’s impact on the PRC’s Anti-Schistosomiasis Campaign. There is little doubt, however, that it inspired some individuals to identify with the heroine and the hero of the film. Recently, while visiting local museums dedicated to the Anti-Schistosomiasis Campaign in Jiangsu province’s Qingpu County, Zhejiang province’s Jiaying County, and Jiangxi province’s Yujiang County, I came across similar texts in each museum claiming that the film was based on their own ‘heroic’ experience of ‘Sending off the Plague God’ and that Bitter Sister or Winter Brother had been ‘real’ villagers from their counties. After speaking to some participants in the campaign, it dawned on me that for villagers who had endured extremely cold weather burying snails with half-empty stomachs, it was important that they remember the time as worth celebrating rather than as time wasted. The film *Spring Comes to a Withered Tree* allowed them to see themselves as ‘heroes’ who had taken part in a ‘heroic battle’ against a disease that had claimed the lives of many of the people they knew: in the lower reaches of the Yangtze River, almost every family had some connection to the disease. The film thus gave them a sense of self-worth. By sharing the happiness of Bitter Sister and Winter Brother, they could, at least for a moment, forget the darker side of the campaign, despite the bitter experience of the famine and the resulting fear of an uncertain future. As Gerard Lemos’s illuminating study, *The End of the Chinese Dream: Why Chinese People Fear the Future*, shows, the majority of people in China remain worried about health problems.⁶⁴ Their anxieties over poor health are exacerbated because of the deficiencies in the Chinese health care system. This anxiety has its roots, to no little extent, in the memories of the famine and the mass death and illnesses that resulted.

For the post-Mao PRC government, *Spring Comes to a Withered Tree* continued to serve as China's 'cultural ambassador' and was screened to audiences in some African and Latin American countries as well as in Japan. The Anti-Schistosomiasis Campaign had become an ever more important piece of cultural propaganda for the PRC in winning over fellow travellers in Latin America, Africa, and Southeast Asia. Not lived experience but the propagandistic representation of the successes of the PRC in improving people's health, made accessible to all in the form of narratives and films, shaped not only the internal debates about health care but also their international resonance. The reality at the time was, however, radically different from such propagandistic representation. As the next chapter shows, the Maoist utopian project of transforming the Chinese countryside into a disease-free 'communist paradise' turned out to be a total disaster. While millions died of starvation, schistosomiasis morbidity was also on the rise.

对于后毛泽东时代的中华人民共和国政府,《枯木逢春》继续作为中国的“文化大使”,并在一些非洲和拉丁美洲国家以及日本向观众放映。血吸虫病防治运动已成为中国在赢得拉丁美洲、非洲和东南亚的同路人方面,越来越重要的文化宣传手段。并非亲身经历,而是中华人民共和国在改善人民健康方面的成功之处的宣传性再现,以叙事和电影的形式向所有人开放,这不仅塑造了关于医疗保健的内部辩论,也塑造了其国际共鸣。然而,当时的现实与这种宣传性的再现截然不同。正如下一章所示,将中国农村改造为无疾病的“共产主义天堂”的毛主义乌托邦计划最终彻底失败。数百万人死于饥饿,血吸虫病的发病率也在上升。

The Great Leap Backward

THE WAR ON NATURE: BATTLING SNAILS

Nineteen-fifty-nine was the Year of the Pig in the Chinese calendar. For China's agricultural communities, the pig is associated with fertility and virility, but 1959 turned out to be a harsh and barren year. The Great Leap Forward demanded constant increases in production from the peasants. The State Council increased the target for agricultural output to 40 per cent higher than the previous year.¹ It compelled many local authorities, from provincial to county, commune and production brigade, to inflate their yield by tens and even hundreds of times the actual output. This came to be known as the 'Wind of Exaggeration'. As this 'Wind of Exaggeration' swept across the Chinese countryside, the famine that had already haunted parts of the country worsened and quickly spread. Oedema caused by malnutrition became the new plague, claiming more and more lives. Death and ill health put further stress on the ongoing labour shortage caused by the expanding demands for workers for the Great Leap Forward's industrial construction and water conservation initiatives. With less and less labour available for agricultural work, many half-cultivated agricultural fields – some of which were newly reclaimed fields – turned into wasteland overgrown with weeds. As human activity receded, the unheroic yet resilient snails, which had previously gone into hiding, slowly crawled out, and to borrow the words of the Nobel Prize-winning novelist Günter Grass: 'it seldom wins and then by the skin of its teeth. It crawls, it goes into hiding but keeps on, putting down its quickly drying track on the historical landscape.'²

大跃退

与自然开战：大战蜗牛

1959 年是中国农历的猪年。对于中国的农业社区来说，猪象征着丰产和活力，但 1959 年却是一个严酷而贫瘠的年份。“大跃进”要求农民不断提高产量。国务院将农业产出目标提高到比上一年高出 40%。¹ 这迫使许多地方政府，从省到县、公社和生产队，将产量夸大到实际产量的数十倍甚至数百倍。这被称为“浮夸风”。随着这股“浮夸风”席卷中国农村，已经困扰该国部分地区的饥荒进一步恶化并迅速蔓延。营养不良引起的水肿成为新的瘟疫，夺走了越来越多的生命。死亡和疾病给因“大跃进”工业建设和水利工程不断扩大的劳动力需求而造成的持续劳动力短缺带来了更大的压力。由于可用于农业工作的劳动力越来越少，许多半耕种的农田——其中一些是新开垦的土地——变成了杂草丛生的荒地。随着人类活动的减少，原本躲藏起来的不起眼但坚韧的蜗牛慢慢地爬了出来，借用诺贝尔奖得主小说家君特·格拉斯的话说：“它很少赢，而且总是险胜。它爬行，它躲藏，但它会继续，在历史的景观上留下它迅速干燥的痕迹。”²

The situation worsened as the warmer spring weather brought the first monsoons. In the lake regions along the Yangtze River, water quickly submerged the land, making snail burial work impossible. As a result, in the regions surrounding Poyang Lake, literally next to the 'victorious' Yujiang County, many local authorities abandoned the practice of schistosomiasis control by burying snails. There, schistosomiasis control work retreated to treatment only, conflicting with the government policy that had previously put prevention first.³ Elsewhere, heavy rains also prevented the land reclamation needed to control the vector snail. In Guangdong province's Dawang state farm, for instance, 20,000 labourers were brought in to reclaim the snail-infested marshes and turn them into agricultural land. Heavy rain, however, halted the work, and as a result these workers had nothing to do except play cards in their makeshift tents. In some regions, such as Yunnan province's Tunxi County in the southwest, local authorities assumed that the snails would automatically vanish after new irrigation canals and ditches were built, so they did not even bother to plan for snail control. During the annual snail survey, 'routs' of young snails were found crawling cheerfully in the newly built irrigation ditches. Even in the eastern provinces of Zhejiang, Jiangsu, and Shanghai, where they had boasted for years of their experience in snail control work and where they enjoyed more resources and technical support, it was reported that buckets of snails were being harvested in many counties just a few months after their near-elimination had been declared. In the suburbs of Shanghai in Jinshan and Nanhui counties, it was reported that merely 30 per cent of snails were being killed as result of local snail burial work – nowhere near the projected target of elimination.⁴

Later, in 1961, in Shanghai's Songjiang County, the head of one commune health centre admitted: 'we declared near elimination twice in the previous two years [1959 and 1960], but this year we not only found a few snails, we actually found *lots* of snails.' His excuse for the failure to reduce snail numbers was the poor quality of the snail control work on the ground. According to him, this was partly exacerbated by the labour shortfall, but his colleague in Shanghai County's epidemic prevention centre disagreed. This colleague argued that 'the problem is not due to the poor quality of the work. I led the snail control work myself, and two or three of my colleagues went along to supervise the work and verify the result. We made sure there were no snails left before calling off the work.'

随着温暖的春季天气带来了第一场季风，情况变得更加糟糕。在长江沿岸的湖区，洪水迅速淹没了土地，使得掩埋钉螺的工作无法进行。结果，在鄱阳湖周围的地区，就在“胜利”的余江县旁边，许多地方当局放弃了通过掩埋钉螺来控制血吸虫病的做法。在那里，血吸虫病防治工作退回到仅治疗，这与政府此前将预防放在首位的政策相冲突。³ 在其他地方，暴雨也阻碍了控制传播媒介钉螺所需的土地复垦。例如，在广东省大旺农场，有2万名劳工被调来开垦受钉螺侵扰的沼泽地，将其变成农业用地。然而，暴雨阻止了这项工作，结果这些工人除了在他们临时搭建的帐篷里打牌之外，无事可做。在一些地区，如云南省西南部的屯溪县，地方当局认为，新的灌溉渠和沟渠建成后，钉螺会自动消失，因此他们甚至懒得计划控制钉螺。在每年的钉螺调查中，人们发现成群的幼螺在新挖的灌溉沟渠中欢快地爬行。即使在浙江、江苏和上海等东部省份，这些省份多年来一直吹嘘其在钉螺控制方面拥有经验，并享有更多的资源和技术支持，据报道，在许多县，就在宣布几乎消灭钉螺几个月后，人们又收获了一桶桶的钉螺。在上海郊区的金山县和南汇县，据报道，由于当地的钉螺掩埋工作，仅杀死了30%的钉螺——远未达到预计的消除目标。⁴

后来，在1961年，上海松江县的一个公社卫生院院长承认：

“在前两年，我们两次宣布接近消灭[1959和1960]，但是今年我们不仅发现了一些钉螺，实际上发现了很多钉螺。”他对未能减少钉螺数量的解释是地面上的灭螺工作质量差。他认为，这部分是由于劳动力短缺造成的，但他在上海县防疫站的同事不同意这种说法。这位同事认为：“问题不是由于工作质量差造成的。我亲自领导了灭螺工作，我的两三个同事也一起去监督工作并核实结果。我们确保在结束工作之前没有留下钉螺。”

But a few months later, snails repopulated those scoured canals and ditches. How do you explain that? My head aches whenever I think about snails. Some younger colleagues who recently joined the control work grumbled: "I fear snail control work more than I fear death."⁵ He concluded that to eliminate snails was simply not feasible: 'No one has any idea when and how we can eliminate snails.' Drawing on years of experience, one schistosomiasis control cadre from Zhejiang province's Jiading County supported this view: 'Snails are difficult to spot because they are very small, and there are too many of them. Not only that, they can survive in both water and on land. Even if we don't see them today, they will pop out tomorrow. After more than six years of effort to eliminate the snail, none of us think it is possible to get rid of them completely.'⁵

Some experts, such as Su Delong, acknowledged that there were far too many challenges in controlling the snail vector by the environmental method. He saw that China's economic condition at the time meant that the environmental control measures such as cementing ditches, which had been applied in small areas of Japan with some success in mitigating schistosomiasis, was not feasible in China where the endemic area was so vast and the terrain and the soil composition so complex. They therefore sought to control the snail vector with chemical control measures, despite the Japanese recommendation in 1957 that had warned against it because of the toxicity to other species. In the long run, molluscicides such as sodium pentachlorophenate (NaPCP) 'may increase the resistance of vector snails,' noted the Japanese experts. The drug is also soluble in water; the creeks of the Yangtze delta region could be easily contaminated by it. 'In those areas the inhabitants often culture fish in creeks and sodium pentachlorophenate was proved to be poisonous for fish in a concentration of one ppm [1 ppm = 1 mg per litre]. There exists the danger against human beings because the inhabitants in these areas usually drink creek water.'⁶ Chemical control measures were also rejected by the local authority in Jiangxi province for economic rather than health reasons.

Knowing the danger of pollution did not preclude the use of this easier method because immediate successes were vital for the political status of the leadership of the People's Republic of China (PRC). The late 1950s were also the moment when public health authorities around the world came increasingly to favour chemical control, seeing it as offering a rapid means of bringing the disease under

但几个月后，蜗牛又重新占据了那些被清理过的沟渠。你如何解释这一点？每当我想到蜗牛，我就头疼。一些最近加入防治工作的年轻同事抱怨说：“我害怕防治蜗牛胜过害怕死亡。”他得出结论，消灭蜗牛根本不可行：“没有人知道我们何时以及如何才能消灭蜗牛。”浙江省嘉定县一位血吸虫病防治干部根据多年的经验，支持这一观点：“蜗牛很难发现，因为它们非常小，而且数量太多。不仅如此，它们还能在水里和陆地上生存。即使我们今天看不到它们，它们明天也会冒出来。经过六年多的努力来消灭蜗牛，我们中没有人认为完全摆脱它们是可能的。”⁵

一些专家，如苏德隆，承认通过环境方法控制钉螺媒介面临着太多的挑战。他认为，中国当时的经济状况意味着，像水泥沟渠这样的环境控制措施，在日本小范围内应用，在减轻血吸虫病方面取得了一些成功，但在中国是不可行的，因为中国的流行区域非常广阔，地形和土壤构成非常复杂。因此，尽管日本在1957年就警告说，由于对其他物种的毒性，不建议使用化学控制措施，但他们还是寻求用化学控制措施来控制钉螺媒介。日本专家指出，从长远来看，五氯酚钠（NaPCP）等杀螺剂“可能会增加媒介钉螺的抵抗力”。这种药物也溶于水；长江三角洲的河流很容易受到污染。

“在这些地区，居民经常在小溪中养鱼，五氯酚钠被证明对鱼类有毒，浓度为百万分之一 [1 ppm = 1 毫克 / 升]。存在对人类的危险，因为这些地区的居民通常饮用河水。”⁶江西省地方当局也出于经济而非健康原因拒绝了化学控制措施。

尽管知道污染的危险，但使用这种更简便的方法仍在所不免，因为眼前的成功对于中华人民共和国（PRC）领导层的政治地位至关重要。20世纪50年代末，世界各地的公共卫生部门也越来越倾向于化学防治，认为这是一种迅速控制疾病的手段。

control.⁷ Eyeing the crown of being the first in the world to eradicate schistosomiasis, it was no wonder that many Chinese experts sought out molluscicides as the ‘magic bullet’ for getting the job done. Since copper sulphate was too expensive for China at the time, the molluscicides that were widely used were calcium arsenate and the synthetic sodium pentachlorophenate. Although spraying molluscicides could effectively kill the intermediate snail host, experts both inside and outside China were also aware that these chemicals were highly toxic to fish, crops, and humans, as well as damaging to the entire ecology. Furthermore, molluscicides such as calcium arsenate are highly soluble in water. The toxicity remains for many months and is extremely harmful to humans and animals well after the initial application.⁸ On many occasions, Su Delong stressed the importance of training villagers to take special precautions when spraying.^{*} Aware that synthetics became ineffective in cold weather, Su and other experts recommended carrying out the spraying in spring or summer for effective control of the snails because in warmer weather the toxicity of the chemical was enhanced in warm water and the snail was hungry after its dormant period over the cold weather. Equally important, they emphasized that the spraying should be repeated at least once a month.⁹

Against their advice, however, in many affected regions the chemical control work was carried out in winter when there was little agricultural work and excess labour could be used for this purpose. Generally, the severe labour shortage caused by the interminable Great Leap Forward construction work and the ongoing campaign to increase agricultural yields, as well as the subsequent famine, meant that the spraying was often carried out in an intense burst of activity, in a similar way to the snail burial ‘shock attacks’ in Yujiang County (as described in chapter 4). It is not surprising, therefore, that the result was disappointing. Reports from western Sichuan showed that in many locations where there was a high density of snails, spraying NaPCP only managed to kill a mere 3.9 per cent of the snails. While spraying calcium arsenate was reported to have a higher success rate of killing 25 per cent of the snails, it was,

* For instance, after spraying villagers should avoid using the water from the nearby creek or river for at least three days, as well as stopping animals from drinking water from ponds or any other still-water source for as long as two to three months. Also, they had to avoid spraying on grasses, or, if they did, the contaminated grass was to be removed completely, since animals might die of the poison from grazing.

控制。⁷ 为了争夺世界上第一个根除血吸虫病的桂冠，许多中国专家寻求杀螺剂作为完成这项工作的“灵丹妙药”也就不足为奇了。由于当时硫酸铜对中国来说太贵了，因此广泛使用的杀螺剂是砷酸钙和合成的五氯酚钠。虽然喷洒杀螺剂可以有效地杀死中间宿主蜗牛，但中国国内外的专家也意识到这些化学物质对鱼类、农作物和人类具有剧毒，并且对整个生态系统造成损害。此外，砷酸钙等杀螺剂极易溶于水。毒性会持续数月，并且在最初使用后很长一段时间内对人类和动物都极其有害。⁸ 在许多场合，苏德隆都强调了培训村民在喷洒时采取特殊预防措施的重要性。^{*} 苏德隆和其他专家意识到合成物质在寒冷天气中会失效，因此建议在春季或夏季进行喷洒，以有效控制蜗牛，因为在温暖的天气中，化学物质的毒性会在温水中增强，并且蜗牛在度过寒冷天气的休眠期后会感到饥饿。同样重要的是，他们强调喷洒应至少每月重复一次。⁹

然而，与他们的建议相反，在许多受影响的地区，化学防治工作是在冬季进行的，那时农业工作很少，多余的劳动力可以用于此目的。一般来说，“大跃进”时期无休止的建设工作和持续的提高农业产量运动，以及随后的饥荒，导致了严重的劳动力短缺，这意味着喷洒工作通常是在紧张的突击活动中进行的，这与玉江县的蜗牛掩埋“冲击战”（如第4章所述）类似。因此，结果令人失望也就不足为奇了。来自川西的报告显示，在许多蜗牛密度高的地区，喷洒五氯酚钠（NaPCP）仅能杀死3.9%的蜗牛。据报道，喷洒砷酸钙对杀死蜗牛的成功率更高，达到25%，但这是

* 例如，喷洒后，村民应避免使用附近小溪或河流中的水至少三天，并阻止动物饮用池塘或任何其他静止水源中的水长达两到三个月。此外，他们必须避免在草地上喷洒，或者，如果他们这样做了，必须完全清除受污染的草，因为动物可能会因放牧而中毒死亡。

however, ineffective in killing their eggs. In addition, because of a shortfall in trained personnel, the spraying was often carried out in a haphazard fashion without any proper planning. This reduced the limited efficacy of molluscicides and led to enormous waste. Since there was little time and labour for the spraying, some grass-roots cadres deliberately avoided taking a snail survey prior to the application of the chemicals. In these communes, they only sprayed major ditches and neglected smaller ones, assuming 'the bigger the ditch, the more snails'. When public health officials came to verify the result, they were stunned to learn that snail density there was as high as one per every five square metres.¹⁰

In the desperate rush to reach the goal of eliminating snails, the very real concerns over human and animal health were regularly compromised because of the severe shortage of trained personnel to carry out or supervise the spraying. In the suburbs of Shanghai, the local authority of the schistosomiasis control model county Qingpu, for instance, gave an order to spray everywhere, irrespective of the warning to avoid spraying near residential areas and animal barns.¹¹ A similar 'spray everywhere' program was carried out in nearby Jiangsu province, where sixteen schistosomiasis control cadres were reported to have suffered severe toxic poisoning after continuously handling NaPCP for several days without any protective measures. Ingesting fish poisoned by the chemical made their condition even worse, and one person subsequently died. Eating poisoned fish was also widespread among villagers due to the increasing starvation caused by the famine. In Zhejiang province's model county of schistosomiasis control, Jiaxing, some communes planned to solve the labour shortage problem by carrying out spraying during the rainy season when farming work would be suspended. Because of the rise in water level, however, snails crawled out from the canals where NaPCP had been applied. At the same time, floods transported dissolved chemicals downstream into sections of the river used for breeding fish, and a huge number of fish were poisoned as a result.¹² Further inland, in Sichuan province's Mianyang County in the southwest, many female sprayers were asked to carry out spraying barefoot in the paddy fields over an extended period. Some subsequently vomited repeatedly, while others fainted after inhaling too much of the toxin.¹³ Despite frequent reports of NaPCP poisoning, its application continued into the 1980s. As late as 1985, cases of death by NaPCP poison relating to the local schistosomiasis

然而,在杀死钉螺卵方面效果不佳。此外,由于训练有素的人员短缺,喷洒工作通常是在没有任何适当计划的情况下随意进行的。这降低了灭螺剂的有限功效,并导致了巨大的浪费。由于喷洒的时间和劳动力很少,一些基层干部故意在施用化学品之前避免进行钉螺调查。在这些公社,他们只喷洒主要沟渠,而忽略了较小的沟渠,认为“沟渠越大,钉螺越多”。当公共卫生官员来核实结果时,他们震惊地发现那里的钉螺密度高达每五平方米一只。¹⁰

在急于达到消灭钉螺目标的过程中,由于严重缺乏进行或监督喷洒的训练有素的人员,对人类和动物健康的真正担忧经常受到损害。例如,在上海血吸虫病防治模范县青浦郊区,地方当局下令到处喷洒,而不顾避免在居民区和动物棚附近喷洒的警告。¹¹在附近的江苏省也开展了类似的“到处喷洒”计划,据报道,有16名血吸虫病防治干部在连续几天处理五氯酚钠而没有任何防护措施后,遭受了严重的毒性中毒。食用被化学品污染的鱼使他们的情况更加恶化,随后一人死亡。由于饥荒造成的日益严重的饥饿,村民中也普遍存在食用有毒鱼的情况。在浙江省的血吸虫病防治模范县嘉兴,一些公社计划通过在农活暂停的雨季进行喷洒来解决劳动力短缺的问题。然而,由于水位上升,钉螺从施用了五氯酚钠的运河中爬了出来。与此同时,洪水将溶解的化学物质向下游输送到用于养鱼的河段,导致大量鱼类中毒。¹²更内陆的四川省绵阳县西南部,许多女喷洒员被要求长期赤脚在稻田中进行喷洒。一些人随后反复呕吐,另一些人在吸入过多的毒素后昏倒。¹³尽管经常有五氯酚钠中毒的报告,但其应用一直持续到20世纪80年代。迟至1985年,仍有与当地血吸虫病相关的五氯酚钠中毒死亡案例

control work were reported in Yunnan Province's Dali region.¹⁴ Dermabrasion was also a common ailment among the sprayers. In Mianyang County's Mawei Commune, seeing their skin peeling off, the villagers were extremely frightened. In shock, they began to scream and threw away the spraying guns as they ran out of the village and paddy fields. Here, the villagers complained that molluscicides killed more crops and grass than snails. Without any grass to eat, the cows and pigs also died of starvation. The locals began to view snail control work as directly contributing to the local famine, and they called the schistosomiasis cadres 'life snatchers'.¹⁵

In addition to the danger of molluscicides getting into the food chain and harming human and animal health, there were also concerns that the unrestricted use of chemicals over a long period of time would damage the quality of the soil. Studies show that copper salts and other molluscicides that kill snails and fish also destroy nitrogen-fixing algae (BGA). The latter is recognized as one of the bio-fertilizers necessary to keep the rice paddies fertile. After the application of copper salts and other molluscicides repeatedly over time, the BGA was destroyed, and thus the quality of the soil of the rice paddies deteriorated.¹⁶ In light of the ongoing famine as well as a severe labour shortage, an increasing number of local authorities began to resist the method of controlling snails with molluscicides. Some rejected it outright on the grounds that it would damage the quality of the soil and thus decrease the agricultural yield. In Fengxian County on the south side of Shanghai, for instance, local authorities gave the order not to carry out any spraying in or near the rice paddies. It also forbade any spraying near the village for fear that insecticides would harm livestock and fisheries as well as the inhabitants.¹⁷ This drove the snails to migrate to the untreated (unsprayed) areas to hibernate or lay their eggs, but after the spraying stopped and as spring approached, the snails would quickly repopulate the previously treated area. Here, control through molluscicides was not only ineffective but also wasteful of already scarce local resources. In addition, with little or no food to eat, those with the task of spraying complained about the lack of adequate diet that left them with insufficient energy to do the job effectively. Some made a calculation that to apply a whole bucket of liquid chemical mixture would involve 700 spraying movements. In the summer, when the daylight was long, their workload would increase. 'To carry out the spraying, we need sufficient calories. With the ration of 400g rice

云南省大理地区报告了防治工作中的问题。¹⁴ 磨皮也是喷洒人员中常见的疾病。在绵阳市的马尾公社，村民们看到自己的皮肤脱落，非常害怕。在震惊之下，他们开始尖叫，扔掉喷枪，跑出村庄和稻田。在这里，村民们抱怨说，杀软体动物剂杀死的庄稼和草比蜗牛还多。由于没有草吃，牛和猪也饿死了。当地人开始认为控制蜗牛的工作直接导致了当地的饥荒，他们称血吸虫病干部为“夺命者”。

¹⁵

除了杀软体动物剂进入食物链并危害人类和动物健康的危险外，人们还担心长期无限制地使用化学品会损害土壤质量。研究表明，硫酸铜和其他杀死蜗牛和鱼类的杀软体动物剂也会破坏固氮藻类（bga）。后者被认为是保持稻田肥沃所必需的生物肥料之一。随着时间的推移，反复施用硫酸铜和其他杀软体动物剂后，bga 被破坏，因此稻田的土壤质量恶化。¹⁶ 鉴于持续的饥荒和严重的劳动力短缺，越来越多的地方当局开始抵制使用杀软体动物剂控制蜗牛的方法。有些人以会损害土壤质量并因此降低农业产量为由，断然拒绝了这种方法。例如，在上海南侧的奉贤县，地方当局下令不在稻田内或附近进行任何喷洒。它还禁止在村庄附近进行任何喷洒，因为担心杀虫剂会伤害牲畜和渔业以及居民。¹⁷ 这驱使蜗牛迁移到未经处理（未喷洒）的区域冬眠或产卵，但在停止喷洒并随着春天的临近，蜗牛会迅速重新占领先前处理过的区域。在这里，通过杀软体动物剂进行控制不仅无效，而且浪费了本已稀缺的当地资源。此外，由于几乎没有或没有食物可吃，那些负责喷洒的人抱怨说，食物不足，使他们没有足够的精力有效地完成工作。有人计算出，要喷洒一整桶液体化学混合物，需要进行 700 次喷洒动作。在夏天，白天很长，他们的工作量会增加。“要进行喷洒，我们需要足够的卡路里。每天的口粮是 400 克大米”

per day, where can we get the energy to do it?’ they groaned. Some even composed a mocking bit of doggerel: ‘We get up every morning to try to eliminate snails. All day long we only get some spoonful of gruel to eat and two bowls of salty water to drink. When is this going to end?’¹⁸

The work of controlling snails with molluscicides was further hampered by the supply shortage that was a result of the collapse of the overall nationwide distribution system during the famine, as well as poor management at the local level.¹⁹ Goods perished or were degraded at transport depots because there were not enough trains, trucks, boats, or workers to move them. An increasing number of train robberies, triggered by the famine crisis, also prevented molluscicides from being delivered to their designated places in time for the control work.²⁰ The other national problem was simply the lack of available containers to store and transport the chemicals. For instance, the Nanjing pharmaceutical factory complained that they were seriously short of cylinders and acid-resistant tubs. Similar problems were also reported elsewhere. As a result, the demand for molluscicides far exceeded the supply.²¹

With molluscicides in short supply, local authorities were encouraged to practise ‘self-reliance’. This meant coming up with local solutions to eliminate snails that required either no or little financial input from the state. From its conception, the PRC’s Anti-Schistosomiasis Campaign was a crusade that would involve the masses. In the context of the Great Leap Forward, communities, including grassroots schistosomiasis institutions in affected regions, were further encouraged to come up with technical solutions or local innovations to eliminate snails. Bio-pesticides were widely promoted, such as an oil emulsion extracted from croton seeds and a paste made of *Camellia oleosa* because they were claimed to have produced some success in reducing snail populations. To further the dissemination of such methods, experts from research institutions were involved in testing their efficacy. *Camellia oleosa* is a plant found throughout southern China. Traditionally, fishermen and villagers in parts of China used it as a poison bait to stun fish and shellfish. Prior to the introduction of commercial soap, rural villagers also used it as a sanitizer to wash their hands and hair. In 1956, at the start of the Anti-Schistosomiasis Campaign, researchers, with the help of grassroots health auxiliaries in Zhejiang province, experimented with using it to kill snails. The initial experiment conducted in Jiangshan County showed that the

每天这样干，我们从哪里获得能量？’他们抱怨道。有些人甚至编了一段嘲讽的打油诗：‘我们每天早上起床，努力消灭蜗牛。整天下来，我们只能吃几勺粥，喝两碗咸水。这什么时候才能结束？’

¹⁸

由于饥荒期间全国范围内的整体分配系统崩溃以及地方层面的管理不善，使用杀螺剂控制蜗牛的工作进一步受到供应短缺的阻碍。¹⁹ 由于没有足够的火车、卡车、船只或工人来搬运货物，货物在运输站腐烂或变质。越来越多的火车抢劫事件，也是由饥荒危机引发的，也阻碍了杀螺剂及时运送到指定地点进行控制工作。²⁰ 另一个全国性的问题是缺乏可用的容器来储存和运输化学品。例如，南京制药厂抱怨说，他们严重缺乏气瓶和耐酸桶。其他地方也报告了类似的问题。因此，对杀螺剂的需求远远超过了供应量。²¹

由于灭螺剂供应短缺，地方当局被鼓励实行“自力更生”。这意味着要拿出当地的解决方案来消灭蜗牛，这些解决方案要么不需要国家投入资金，要么只需要很少的资金投入。从一开始，中国血吸虫病防治运动就是一场需要群众参与的运动。在“大跃进”的背景下，包括受影响地区的基层血吸虫病防治机构在内的各个社区，被进一步鼓励提出技术解决方案或当地创新措施来消灭蜗牛。生物农药得到了广泛的推广，例如从巴豆籽中提取的油乳剂和用油茶制成的糊状物，因为据称它们在减少蜗牛数量方面取得了一些成功。为了进一步推广这些方法，研究机构的专家也参与了测试它们的有效性。油茶是一种遍布华南地区的植物。传统上，中国部分地区的渔民和村民用它作为毒饵来麻醉鱼类和贝类。在商业肥皂引入之前，农村村民也用它作为消毒剂来洗手和头发。1956年，在血吸虫病防治运动开始时，研究人员在浙江省基层卫生员的帮助下，试验用它来杀死蜗牛。在江山县进行的初步实验表明，

efficacy of *camellia oleosa* in reducing snails was uneven: at a warm temperature ($+32^{\circ}\text{C}$), the snail reduction rate could reach as high as 97.7 per cent, but as the temperature dropped, its efficacy also diminished. Furthermore, its impact dissipated quickly. A longer-lasting result would involve constantly applying it to affected paddies and ditches, which was impossible to undertake. Given the impact of other interventions on human health, Su Delong and his team of researchers at Shanghai Medical University (today's Fudan University Medical College) developed prevention techniques to protect villagers from infection. This required applying 30 grams of dust of freshly collected *Camellia oleosa* to a cloth and then using the cloth to wrap around villagers' legs prior to them stepping into contaminated water or muddy fields. The field study in Qingpu County, outside of Shanghai, showed that this method could be effective for up to eight hours. In light of an ongoing famine, however, using *Camellia oleosa* to reduce the snail population or protect villagers from infection proved impractical for a number of reasons: first of all, both methods were time-consuming and labour-intensive; second, no cotton cloth or any other fabric was available;^{22*} and third, the overuse of *Camellia oleosa* paste (to kill snails rather than using it as a poison bait at lower doses) caused a huge quantity of fish, shellfish, and water plants to die. Since many villagers relied on fish and shellfish for food and water plants were an important source of fertilizer, it is understandable that the method was unpopular. In laboratory tests, the oil emulsion of croton (at $4\text{--}8\text{ g/m}^2$) achieved a similar success in killing snails as NaPCP (1 g/m^2). In the field, however, its efficacy was much less. It was also even more harmful to human and animal health than *camellia oleosa* paste. A few grams of it could cause severe diarrhoea and even fatalities. Traditionally, it was an abortifacient, well known for causing miscarriage. At the time of an escalating famine crisis, with malnutrition and infertility being two of the most widespread health risks, many rumours arose that croton oil was a cause of the problem. There were also reports of villagers suffering from skin ailments after handling it. Thus, instead of viewing it as a beneficial intervention, villagers regarded it as just one more health risk. Furthermore, it too was constantly in short supply, since it was expensive and labour-intensive to produce by hand.²³

* To make up the ever-increasing cotton procumbent target set by the government, villagers were forced to hand over their cloths and quilts.

山茶油在减少钉螺方面的效果并不均衡：在温暖的温度（ $+32^{\circ}\text{C}$ ）下，灭螺率可能高达 97.7%，但随着温度下降，其功效也会降低。此外，其影响消散得很快。要获得更持久的效果，需要不断地将其施用于受影响的稻田和沟渠，这是不可能实现的。鉴于其他干预措施对人类健康的影响，苏德隆和他在上海医科大学（今天的复旦大学医学院）的研究团队开发了预防技术，以保护村民免受感染。这需要将 30 克新鲜采集的山茶油粉尘涂在一块布上，然后用这块布包裹在村民的腿上，然后再让他们进入受污染的水域或泥泞的田地。在上海市郊青浦县的田野调查显示，这种方法可以有效长达八个小时。然而，鉴于持续的饥荒，使用山茶油来减少钉螺数量或保护村民免受感染被证明是不切实际的，原因如下：首先，这两种方法都非常耗时和耗费劳力；其次，没有棉布或任何其他织物可用；^{22*} 第三，过度使用山茶油糊（杀死钉螺而不是以较低剂量将其用作毒饵）导致大量鱼类、贝类和水生植物死亡。由于许多村民依靠鱼类和贝类获取食物，而水生植物是重要的肥料来源，因此可以理解这种方法不受欢迎。在实验室测试中，巴豆油乳剂（ $4\text{--}8\text{ 克/平方米}$ ）在杀死钉螺方面取得了与氯硝柳胺（NaPCP， 1 克/平方米 ）相似的成功。然而，在田野中，其功效远不如氯硝柳胺。而且，它比山茶油糊对人类和动物健康的危害更大。几克巴豆油就可能导致严重腹泻甚至死亡。传统上，它是一种堕胎药，以引起流产而闻名。在饥荒危机日益严重，营养不良和不孕症是两种最普遍的健康风险时，许多谣言传开，说巴豆油是造成问题的原因。还有报道称，村民在处理巴豆油后患有皮肤病。因此，村民没有将其视为有益的干预措施，而是将其视为又一种健康风险。此外，巴豆油也经常供不应求，因为它价格昂贵且手工生产耗费劳力。²³

* 为了完成政府不断提高的棉花征购目标，村民们被迫交出衣服和被子。

Still, the goal of eradication could not be questioned. In the meantime, local authorities, with the help of experts, were compelled to come up with new cost-effective innovations. They used toxic industrial chemical waste – such as a residual of calcium carbamide and a sodium salt of coal tar acid and phenol – as a means of getting rid of snails by poisoning the habitat. In addition to reducing snail populations, the method helped to recycle industrial waste as well as saving local authorities the cost of purchasing molluscicides. Transforming industrial waste into a ‘useful treasure’ was part of a nationwide drive to mitigate the ever-increasing problem of industrial waste and pollution as China embarked on industrialization at an unprecedented speed. From the outset of the Great Leap Forward in the late 1950s until the 1970s, there had been frequent complaints of factories dumping large quantities of coal tar residue and other toxic waste into the rivers, poisoning fisheries, damaging human health, and killing crops. The situation was at its worst in the industrial zones of the northeast and the northwest of China. According to an official government report, in the first half of 1959 nearly 13,000 tons of toxic industrial waste was dumped into the upper stretches of the Yellow River along the banks of Gansu province’s capital city Langzhou. In China’s northeast, the centre of heavy industry, another government report in 1960 showed that the phenol level in these rivers had reached between 2 and 24 milligrams per litre as a result of the huge quantities of industrial waste dumped into local rivers. In the Nonni River (also known as Non lua), local fishermen trawled more than 600,000 kilograms of dead or half-dead fish. Further south in Liaoning province’s Hun River, locally referred to as the ‘Mother River’ of the Manchu rulers’ former spiritual home Mukden, because of the large quantity of toxic industrial waste in the water, fish (including sturgeon) had become extinct.²⁴ It was also reported that more than 10,000 *mu* (ca 16,473.7 acres) of rice crops in Liaoning were killed by toxins in the waste water discharged from local factories. Because of the water shortages in large parts of China, during the Great Leap Forward many local authorities diverted waste water from factories to irrigate agricultural fields. From the late 1950s to the 1960s, food poisoning resulting from ingesting fish poisoned by toxic industrial waste was commonplace along the Yangtze. An official estimate in 1963 indicated that more than 10,000 tons of industrial waste was dumped upstream each day into the Yangtze. As a result, there was a sharp rise in gastritis

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among the population who lived by the river.²⁵ Yet the practice of using waste water from factories to irrigate agricultural fields continued well into the 1970s.

Despite claims that people's health and well-being were integral parts of socialist social welfare, as promised by the Communist state, the state also saw that the rapid industrialization of China was ever more necessary in order to achieve the 'higher' goal of quickly attaining the communist utopia. Weighing the future against the present, and caught between development and backwardness, the public good and individual lives, Zhou Enlai, the premier and manager of the PRC representing the State Council, boldly announced that China would achieve the impossible: 'transform the waste into treasure that would bring benefit to the greater good of the people'. In 1959, while visiting a coke plant 270 kilometres south of the PRC's capital city Beijing, he pointed to the thick black smoke: 'This smoke is a real treasure, we must recycle it. This way we can also reduce pollution,' he stated in a voice that was simultaneously firm and determined.²⁶ Responding to the challenge set by the premier, the State Scientific and Technological Commission, the Ministry of Health, and the Ministry of Agriculture jointly held a national conference to address the matter. At the conference, it was agreed that to recycle toxic industrial waste for the good of the country and the people would be a political undertaking just as vital as the eradication of schistosomiasis. It was regarded as 'science with a higher purpose'. To achieve such a scientific breakthrough – another great leap – all provincial party authorities were to make the recycling of industrial waste a priority. On 19 March 1960, amidst an ongoing famine, the Chinese Communist Party (CCP) Central Committee handed down a directive requesting all provincial-level authorities to urgently propose solutions to deal with the problem of toxic industrial waste.²⁷ Because of the severe labour shortage, however, reality soon showed that the campaign to transform industrial waste into 'treasure' meant going through the motions with no proper planning. In the case of schistosomiasis control, since the goal was to eliminate snails with industrial waste, such as the residual products of calcium carbamide or the sodium salts resulting from coal tar acid, little attention was paid to what quantity would be safe for the purpose or even how they should be applied. As a result, the actual amount used, and the application procedure, varied from location to location. In general, to eliminate snails required at least

在居住于河边的人口中。²⁵ 然而，使用工厂废水灌溉农田的做法一直持续到 20 世纪 70 年代。

尽管声称人民的健康和福祉是社会主义社会福利不可或缺的一部分，正如共产党国家所承诺的那样，但国家也认为，为了实现迅速达到共产主义乌托邦的“更高”目标，中国的快速工业化变得越来越必要。在未来与现在之间权衡，并在发展与落后、公共利益与个人生活之间左右为难的情况下，作为国务院总理兼管理者的周恩来大胆宣布，中国将实现不可能的任务：“将废物转化为宝藏，为人民的更大福祉带来利益”。1959 年，在参观距离中国首都北京 270 公里的一个焦化厂时，他指着浓浓的黑烟说：“这种烟雾是真正的宝藏，我们必须回收它。这样我们也可以减少污染，”他用既坚定又果断的声音说道。²⁶ 为了响应总理提出的挑战，国家科学技术委员会、卫生部和农业部联合举行了一次全国会议来解决这个问题。会议一致认为，为了国家和人民的利益而回收有毒工业废物将是一项与根除血吸虫病一样重要的政治任务。这被认为是“具有更高目的的科学”。为了实现这一科学突破——又一次大跃进——所有省级党委都要将工业废物的回收利用作为优先事项。1960 年 3 月 19 日，在持续的饥荒中，中国共产党（ccp）中央委员会下达了一项指示，要求所有省级当局紧急提出解决有毒工业废物问题的方案。²⁷ 然而，由于严重的劳动力短缺，现实很快表明，将工业废物转化为“宝藏”的运动意味着走过场，没有适当的计划。以血吸虫病控制为例，由于目标是用工业废物（如碳酸钙的残留产物或煤焦油酸产生的钠盐）来消灭钉螺，因此很少有人关注用于此目的的安全剂量，甚至应该如何使用。因此，实际用量和使用程序因地制宜。一般来说，要消除钉螺至少需要

one kilogram of such a chemical mix for every square metre area. But in many affected regions, in the haste to eliminate snails, the actual amount used far exceeded what was recommended. Writing in the 1980s, Mao Shoubai, the PRC's foremost authority on schistosomiasis control, discouraged such measures because 'they harm human and animal health, damage the quality of soil, and pollute water. There are more drawbacks than benefits. Such measures cannot be recommended.'²⁸

Because such undertakings were always focused on short-term gains, either because of political pressure from above or because of the resultant social and economic gains among those undertaking the eradication, little or no attention was given to the longer-term consequences. Not only did the chemical control method cause harm to the environment as well as to human and animal health, but the two widely applied environmental control measures employed by the Chinese in the late 1950s and 1960s – burning grass to roast the snails and destroy their habitat as well as draining swamps and marshes to reclaim low and flooded land – also had long-term consequences on the environment. Burning the grass that grew on the banks of small creeks and ditches, as well as decimating the reeds in the lake regions, was initially promoted by experts and the relevant political authorities as a simple and cost-effective way of reducing the snail population. While the method was reported to have achieved some success, it turned out to be more time-consuming and labour-intensive than killing snails with bio-pesticides made from plants. More important, its efficacy was unreliable because it depended on the season and topography. Furthermore, it destroyed reeds, bushes, grass, and other vegetation, all of which were necessary to a healthy lake ecosystem. Reeds were also a source of vital economic revenue for villagers living in the lake regions. Traditionally, reeds were used as building materials and as the raw materials to make weapons, tools, and musical instruments, as well as fashioned into paper. During the Great Leap Forward and the subsequent famine, trees, bushes, and grass were also in high demand because they were essential to keeping starving animals alive, as fuel for local iron and steel production, and in everyday use as fertilizer. One interviewee remembered that in 1958, after a campaign to kill snails by burning grass, the two brigades involved lost 44,000 kilograms of grass fuel as result. It is not surprising, therefore, that many villagers in these regions resisted applying such methods.²⁹ While I was researching for

每平方米面积使用一公斤这种化学混合物。但在许多受影响的地区, 由于急于消灭蜗牛, 实际用量远远超过了建议量。中国血吸虫病防治工作的权威人士茅寿白在 20 世纪 80 年代撰文指出, 不应提倡这种做法, 因为 “它们危害人类和动物的健康, 破坏土壤质量, 污染水源。弊大于利。这种措施不可取。”²⁸

由于这些事业总是着眼于短期利益, 要么是因为来自上级的政治压力, 要么是因为消灭行动中由此产生的社会和经济利益, 很少或根本没有关注长期的后果。化学防治方法不仅对环境以及人类和动物的健康造成危害, 而且中国在 20 世纪 50 年代末和 60 年代广泛采用的两种环境控制措施——焚烧草地以烤死蜗牛并摧毁其栖息地, 以及排干沼泽和湿地以开垦低洼和水淹土地——也对环境造成了长期影响。专家和相关政治部门最初提倡焚烧小溪和沟渠岸边生长的草, 以及消灭湖区芦苇, 作为一种简单且具有成本效益的减少蜗牛数量的方法。虽然据报道该方法取得了一些成功, 但事实证明它比用植物制成的生物农药杀死蜗牛更费时费力。更重要的是, 它的功效不可靠, 因为它取决于季节和地形。此外, 它还破坏了芦苇、灌木、草和其他植被, 所有这些对于健康的湖泊生态系统都是必不可少的。芦苇也是湖区村民重要的经济收入来源。传统上, 芦苇被用作建筑材料, 并作为制造武器、工具和乐器的原材料, 以及制成纸张。在大跃进和随后的饥荒期间, 树木、灌木和草地也供不应求, 因为它们对于维持饥饿动物的生命、为当地钢铁生产提供燃料以及在日常生活中用作肥料至关重要。一位受访者回忆说, 1958 年, 在开展焚烧草地灭蜗牛运动后, 参与的两个生产队损失了 44,000 公斤草燃料。因此, 这些地区的许多村民抵制使用这种方法也就不足为奇了。²⁹ 当我为

this book, I was often told by former schistosomiasis control cadres that this method of burning grass and reeds was less harmful to people and the environment than the chemical control method. A 2016 study by a team of researchers from the PRC and Australia, however, argued the opposite. According to this study, continuous open-field biomass burning, sometimes over several days – as was done to reduce the snail population – caused severe air pollution with the attendant risks to public health and a potentially adverse impact on the climate.³⁰

As was discussed in chapter 3, the environmental intervention to control schistosomiasis by reclaiming and developing marshland was highly recommended by the team of Japanese experts at the end of their 1956 mission to the PRC. Subsequently, between 1957 and 1962, with the CCP frog-marching China and its people into the Great Leap Forward, more than 18,121 acres of marshes along Dongting Lake in Hunan province and Poyang Lake in Jiangxi province were reclaimed. As a result, the cultivable land in these regions doubled, a boon, it would seem, both in the struggle against the disease vector snails and in the expansion of agricultural acres and the concomitant food production.³¹ Yet the continuous large-scale land reclamation initiatives, which first began in the 1950s, were thought to be a contributing factor to the worst flood of the twentieth century on the Yangtze that devastated the regions along Poyang Lake and Dongting Lake in the summer of 1999.³² Large-scale land reclamation not only proved extremely costly and wasteful, it also ran into conflict with the systems for water conservation and as a result contributed to severe flooding. Research showed that land reclamation caused siltation and reduced the size of the lakes, which in turn decreased the lakes' flood storage capacity. Land reclamation and siltation, as well as the construction of levees, also restricted flood discharge capacity, causing flood levels to rise even higher³³ – a truly vicious circle. Furthermore, unplanned, excessive reclamation as was undertaken during the Great Leap Forward and during the Anti-Schistosomiasis Campaign also had a serious impact on the lakes' ecosystem. While it reduced the number of snails, it also reduced the number of other aquatic species living in these lakes. As a case study in 1993 by local researchers Li Jingbo and Deng Luo Jin shows, land reclamation activities on Dongting Lake after the 1950s, carried out partly as a measure to control schistosomiasis, seriously damaged the lake's eco-environment. As a result, the number of fish (and therefore food stocks) decreased by more than half in less than

在写这本书时，我经常听到以前的血吸虫病防治干部说，这种焚烧草和芦苇的方法对人和环境的危害比化学防治方法小。然而，prc 和澳大利亚的一个研究小组在 2016 年进行的一项研究却提出了相反的观点。根据这项研究，持续的露天生物质燃烧，有时持续几天——就像减少蜗牛数量一样——会造成严重的空气污染，从而带来公共健康风险，并可能对气候产生不利影响。³⁰

正如第 3 章所讨论的，日本专家组在 1956 年结束对 prc 的访问后，强烈建议通过开垦和开发沼泽地来进行环境干预，以控制血吸虫病。随后，在 1957 年至 1962 年间，随着 ccp 强行推动中国及其人民进入大跃进，湖南省洞庭湖和江西省鄱阳湖沿岸超过 18,121 英亩的沼泽地被开垦。结果，这些地区的耕地面积增加了一倍，这似乎对与疾病传播媒介蜗牛作斗争以及扩大农业用地和随之而来的粮食生产都有利。³¹ 然而，始于 20 世纪 50 年代的持续大规模土地开垦计划被认为是 1999 年夏季袭击鄱阳湖和洞庭湖沿岸地区的那场 20 世纪最严重洪灾的促成因素。³² 大规模的土地开垦不仅被证明成本高昂且浪费严重，而且还与水利系统发生冲突，从而导致了严重的洪水。研究表明，土地开垦导致淤积，减少了湖泊的面积，进而降低了湖泊的蓄洪能力。土地开垦和淤积，以及堤坝的建设，也限制了泄洪能力，导致洪水水位上升³³——这真是一个恶性循环。此外，在大跃进和血防运动期间进行的未经规划的过度开垦也对湖泊的生态系统产生了严重影响。虽然它减少了蜗牛的数量，但也减少了生活在这些湖泊中的其他水生物种的数量。正如当地研究人员李景波和邓罗金在 1993 年进行的一项案例研究表明，20 世纪 50 年代后在洞庭湖进行的土地开垦活动，部分是为了控制血吸虫病，严重破坏了湖泊的生态环境。结果，鱼类（以及食物储备）的数量在不到

thirty years. In addition, the expansion of agricultural activities led to an increase in water pollution through the use of fertilizer and other forms of polluting run-off.³⁴ By the early 1970s, due to strong opposition from water conservancy and agriculture departments, such methods of controlling snails by land reclamation were gradually abandoned.³⁵

INCREASED AGRICULTURAL ACTIVITIES AND RISING MORBIDITY

Land reclamation may have reduced the snail numbers, but it failed to mitigate morbidity. With the increased cultivable land, agricultural activity as well as livestock populations also increased. Accompanied by the expansion of human settlement, such shifts provided new hosts for the schistosome and its parasites, increasing the risk of disease transmission. Between 1957 and 1961, at the height of the Great Leap Forward, thirty-three state-owned collective farms were built on the reclaimed land along the shores of the Dongting and Poyang lakes. More than 30,000 people, along with livestock from different parts of the country, were forcibly relocated there to work on these farms. Since most of them had no resistance to schistosomiasis, they quickly became infected. Between 1958 and 1962, seasonal outbreaks were reported annually on these state-owned farms, causing many casualties. In 1960, at the height of the famine, some 1,300 cases of acute infection were reported on twelve farms. The number doubled in the following two years. Simultaneously, large numbers of livestock were also infected.³⁶

At the same time, the push to increase agricultural yields in order to meet the authorities' ever-increasing procumbent quota demanded that these farms use additional fertilizer. Traditionally, local villagers used grasses, tree branches, and water plants to make compost, but as a preventive measure the public health authorities in the impacted regions had banned state-farm workers and villagers from harvesting the floating water plants that grew on the lake. As a result, sod houses were pulled down, and their rammed-earth walls were soaked in ponds to make compost. The snails quickly migrated to these ponds. Because the local villagers, as well as the employees of the state-owned farms, relied on the water from the ponds for drinking and daily activities, all were infected with schistosomiasis. Failing to meet their fertilizer quota, many farm workers

三十年。此外，农业活动的扩大导致通过使用肥料和其他形式的污染径流造成的水污染增加。³⁴ 到 1970 年代初期，由于水利和农业部门的强烈反对，逐渐放弃了这种通过土地开垦来控制蜗牛的方法。³⁵

农业活动增加和发病率上升

土地开垦可能减少了蜗牛的数量，但未能减轻发病率。随着可耕地增加，农业活动和牲畜数量也随之增加。伴随着人类住区的扩张，这种转变为血吸虫及其寄生虫提供了新的宿主，增加了疾病传播的风险。1957 年至 1961 年大跃进高峰时期，在洞庭湖和鄱阳湖沿岸的开垦土地上建造了 33 个国有集体农场。来自全国不同地区的 30,000 多人以及牲畜被强行迁移到那里在这些农场工作。由于他们中的大多数人对血吸虫病没有抵抗力，他们很快就被感染了。1958 年至 1962 年间，这些国有农场每年都有季节性爆发的报告，造成了许多伤亡。1960 年，在饥荒高峰时期，十二个农场报告了约 1,300 例急性感染病例。在接下来的两年里，这个数字翻了一番。与此同时，大量牲畜也被感染了。³⁶

与此同时，为了满足当局不断增加的征购配额，提高农业产量的努力要求这些农场使用更多的化肥。传统上，当地村民使用草、树枝和水生植物制作堆肥，但作为一项预防措施，受影响地区的公共卫生部门禁止国营农场的工人和村民收割湖上生长的漂浮水生植物。结果，土屋被拆毁，它们的夯土墙被浸泡在池塘中以制作堆肥。蜗牛迅速迁移到这些池塘中。由于当地村民以及国有农场的雇员都依赖池塘里的水来饮用和日常活动，因此所有人都感染了血吸虫病。许多农场工人未能完成化肥配额，

and villagers ventured farther afield to areas of the lake that had not been reclaimed or sprayed with molluscicides so that they could harvest the floating plants to make compost. Most of them suffered acute infection. Their boats carrying the water plants also brought the snails back to their communities.³⁷ Intense farming activities, including spending long periods of time working collectively in water during the harvest and planting seasons, further increased the chances of an outbreak.³⁸

Contrary to the claim of the CCP leadership, having more land for agricultural production did not bring economic betterment for the rural villagers. The campaign in Mao's China, directed at reclaiming pestilential lands for production through improved housing and farming practices (technically called *integral bonification*),* proved disastrous. The planned economy, large farming collectives, and the state grain procurement policy denied individual farmers the ability to make profits from market participation. This meant that a great majority of villagers continued to live in destitution. Agricultural intensification, when coupled with land overuse (i.e., deforestation, land reclamation, and overgrazing), damaged soil quality by causing alkalization and increased drought as well as waterlogging and flooding, which in turn damaged agricultural productivity. The attempt at a quick leap into the communist utopia – a superior form of modernity – resulted in the worst famine in human history, a famine that lasted four years between 1958 and 1962 and claimed an estimated forty-five million lives in the Chinese countryside. Its adverse effects on the environment can still be seen today.³⁹ The effort expended by the rural population to simply survive, as well as to recover from the famine, further increased the risk of disease transmission. In the schistosomiasis endemic regions along Dongting Lake and Poyang Lake, some of the worst regions hit by the famine, in order to secure food a huge number of farm workers and villagers waded into the lakes to pull up lotus roots and fished in the same water for basic sustenance. Many were infected with schistosomiasis. While researching for this book, I met several famine survivors who had been infected

* The term *bonification* was first used during the malaria campaign in Fascist Italy under Mussolini. In the late 1920s, the integral *bonification* was recommended by the League of Nations' Health Organization (LNHO)'s Malaria Commission as a radical measure for its global malaria control program. See 'Conclusions of the Malaria Commission, Health Section, League of Nations, at the Conference in Geneva, June 25–29, 1928', in *Public Health Reports* (1896–1970), vol. 43, no. 45 (9 Nov. 1928): 2957–61.

并且, 村民们冒险进入更远的湖区, 那里没有被开垦或喷洒过杀软体动物剂, 以便收获漂浮植物来制作堆肥。他们中的大多数人患有急性感染。他们运送水生植物的船只也将钉螺带回了他们的社区。

³⁷ 密集的农业活动, 包括在收割和种植季节长时间集体在水中工作, 进一步增加了爆发的机会。³⁸

与中国共产党领导人的说法相反, 拥有更多的土地用于农业生产并没有给农村村民带来经济上的改善。毛泽东时代的中国开展的运动, 旨在通过改善住房和耕作方式(技术上称为整体改良)*来开垦有害土地用于生产, 结果证明是灾难性的。计划经济、大型农业集体以及国家粮食采购政策剥夺了个体农民从市场参与中获利的能力。这意味着绝大多数村民继续生活在贫困之中。农业集约化, 加上土地过度使用(即森林砍伐、土地开垦和过度放牧), 通过引起碱化和增加干旱以及涝渍和洪水来破坏土壤质量, 进而损害农业生产力。试图快速跃入共产主义乌托邦——一种更高级的现代性形式——导致了人类历史上最严重的饥荒, 这场饥荒从1958年持续到1962年, 造成中国农村地区约4500万人死亡。其对环境的不利影响至今仍可见到。³⁹ 农村人口为生存以及从饥荒中恢复所付出的努力进一步增加了疾病传播的风险。在洞庭湖和鄱阳湖沿岸的血吸虫病流行地区(这些地区是受饥荒影响最严重的地区之一), 为了获得食物, 大量农民和村民涉入湖中采摘莲藕, 并在同一水域捕鱼以维持基本生计。许多人感染了血吸虫病。在为本书进行研究时, 我遇到了一些曾感染过饥荒的幸存者。

* 改良一词最初是在墨索里尼领导下的法西斯意大利疟疾运动期间使用的。在20世纪20年代末, 国际联盟卫生组织(LNHO)疟疾委员会建议将整体改良作为其全球疟疾控制计划的根本措施。参见《国际联盟卫生科疟疾委员会于1928年6月25日至29日在日内瓦举行的会议结论》, 载于公共卫生报告(1896-1970), 第43卷, 第45期(1928年11月9日): 2957-61。

with schistosomiasis in this way. For the next fifty years, they continued to live with the disease because the state-sponsored therapeutic interventions remained largely ineffective or inadequate.⁴⁰

The official estimate in 1962 showed that the schistosomiasis morbidity in that year was approximately 8,000–9,000 cases in the regions along Dongting Lake and approximately 6,000–7,000 along Poyang Lake.⁴¹ The latter included the PRC's eradication model county, Yujiang. A year later, at the Ninth National Conference on Schistosomiasis Control, when presented with the high morbidity rate and the increase in the snail-populated areas, Su Yongjiu, Yujiang County's new party boss, regretted that impetuosity and inaccuracy had been two major problems in the local schistosomiasis control work. To calm his nerves while attributing this catastrophe to himself, the skinny, thirty-something-year-old former PLA soldier lit a cigarette and inhaled deeply while stating that 'cadres on the ground misunderstood, believing that the disease could be controlled with one big shock attack. After Yujiang declared eradication in 1958, they stopped the control effort completely. No wonder we continue to find patients every year, even patients in the advanced stage of the disease. In 1961, the snail-populated area was as high as 2,198 square metres. Inaccuracy was a common problem in the local schistosomiasis work.'⁴² It was not merely inaccurate reporting but conscious, repeated falsification that caused these ever-greater numbers of infections through the sense that the mission had been accomplished as promised.

Reports from other endemic regions also showed the increasing numbers of areas now infested by snails. In Sichuan province's Qionglai County, for example, in 1957 the snail-infested area was 20,500 square metres. Leaping into eradication, as inspired by Yujiang's 'triumph' in destroying the snails, many communes here declared the near-elimination of the snails. Yet the snail survey in June 1961 showed that the snail-infested area had actually increased to 320,000 square metres.⁴³ In parts of Jiangsu province in eastern China, too, the snail-infested areas were reported to have increased by more than fourteen times in the three years following 1960.⁴⁴ Clearly, trying to control the vector, by means of either environmental or chemical control, was not helping the PRC to reach its eradication goal, and the seven-year time frame set by the chairman was running out fast. It seemed that the PRC's Anti-Schistosomiasis Campaign was quickly turning from initially being a win/win situation into a lose/lose disaster. The political

以这种方式患上血吸虫病。在接下来的五十年里，他们继续与这种疾病作斗争，因为国家支持的治疗干预措施在很大程度上仍然无效或不足。⁴⁰

1962年的官方估计显示，当年洞庭湖沿岸地区的血吸虫病发病率约为8,000-9,000例，鄱阳湖沿岸地区约为6,000-7,000例。⁴¹ 后者包括中华人民共和国的血吸虫病根除模范县，余江县。一年后，在第九次全国血吸虫病防治会议上，当得知高发病率和钉螺孳生面积增加时，余江县新任党委书记苏永久遗憾地表示，急躁和不准确是当地血吸虫病防治工作中的两个主要问题。为了平复心情，同时将这场灾难归咎于自己，这位瘦骨嶙峋、三十多岁的退伍军人点燃了一支烟，深深地吸了一口气，同时表示，“基层干部误解了，认为只要一次大冲击就能控制住这种疾病。余江县在1958年宣布根除后，他们完全停止了控制工作。难怪我们每年都能发现病人，甚至是一些晚期病人。1961年，钉螺孳生面积高达2198平方米。不准确是当地血吸虫病工作中的一个普遍问题。”⁴² 不仅仅是不准确的报告，而是有意识的、反复的伪造，通过已经完成任务的错觉，导致了越来越多的感染病例。

来自其他流行地区的报告也显示，现在受蜗牛侵扰的地区数量不断增加。例如，在四川省邛崃县，1957年受蜗牛侵扰的面积为20500平方米。在玉江消灭蜗牛的“胜利”的鼓舞下，许多公社纷纷宣布几乎消灭了蜗牛，并跳跃式地开展了根除工作。然而，1961年6月的蜗牛调查显示，受蜗牛侵扰的面积实际上增加到了32万平方米。⁴³ 在中国东部江苏省的部分地区，据报道，在1960年之后的三年里，受蜗牛侵扰的面积增加了14倍以上。⁴⁴ 显然，试图通过环境或化学控制来控制病媒，并没有帮助中国实现其根除目标，而且主席设定的七年时间框架也快要到期了。看起来，中国的血吸虫病防治运动正迅速从最初的双赢局面转变为双输的灾难。政治

claim that had fuelled the demand for the eradication of the vector and the interventions into the newly expanded agricultural land led to long-term disasters that swiftly followed the short-term claims of success. Given the context of this undertaking, the abandonment of expert advice and the coupling of the public health goals with the political ideology of the Great Leap Forward meant that the only claims that could be made were for the purposes of propaganda. Such claims were evidently seen as false, even corrupt, by those on the ground whose lives and livelihoods were impacted by the attempts at eradication, and this dissatisfaction soon became apparent at the very pinnacle of leadership.

ADDRESSING THE FAILURE

In 1962, the CCP leadership, led by the president of the PRC, Liu Shaoqi, acknowledged that the famine was a man-made disaster, and the Great Leap Forward policy was subsequently withdrawn. A few months afterwards, the Nine-man Subcommittee on Schistosomiasis Control, with its vice chair Wei Wenbo, reporting at a national schistosomiasis control conference, collectively acknowledged that the earlier goal of eradicating the disease within seven years was unrealistic. They also admitted that many of the widely publicized claims of eradication were simply false: 'Some places immediately claimed they had eradicated the disease after a few days of shock work, whereas those responsible for verifying the result were too sloppy. As such the estimates were often too high. Under pressure, 167 counties and municipalities hurried to declare near-eradication. In many places, the grassroots schistosomiasis control work has run into a vicious circle.' Careful not to discredit the Chairman Mao-led Anti-Schistosomiasis Campaign or to criticize the model function played by Yujiang County (the PRC's first 'red flag' for this campaign), the report laid the responsibility for the campaign's overall failure to the 'error of misjudgement' at the level of the Nine-man Subcommittee and on poor leadership on the ground.⁴⁵ This was in line with the approach that the CCP leadership and its subsidiary departments took in addressing the Great Leap debacle. Toward the latter period of the famine, faced with millions of people dying of starvation and widespread violence on the ground, the CCP leadership could no longer deny that this was a man-made disaster. It was impelled to come up with an explanation for such a major catastrophe. Instead of questioning the overall

声称已经推动了对根除病媒的需求，并且对新扩展的农业用地的干预导致了长期的灾难，这些灾难迅速地跟随在短期的成功声明之后。鉴于这项事业的背景，放弃专家建议以及将公共卫生目标与大跃进的政治意识形态相结合，意味着唯一可以提出的主张是为了宣传目的。这些说法显然被那些生活和生计受到根除尝试影响的人视为虚假甚至腐败，这种不满很快在领导层的最高层显现出来。

解决失败问题

1962年，由中国国家主席刘少奇领导的中国共产党领导层承认，饥荒是一场人为的灾难，随后大跃进政策被撤回。几个月后，以副主席魏文伯为首的血吸虫病防治九人小组在全国血吸虫病防治会议上报告，集体承认了七年内根除该疾病的早期目标是不切实际的。他们还承认，许多广为宣传的根除声明根本是虚假的：“有些地方在经过几天的突击工作后立即声称他们已经根除了该疾病，而负责核实结果的人员却过于马虎。因此，估计往往过高。在压力下，167个县市赶忙宣布接近根除。在许多地方，基层血吸虫病防治工作陷入了恶性循环。”该报告小心翼翼地避免诋毁毛主席领导的血吸虫病防治运动或批评余江县（中国首个该运动的“红旗”）发挥的模范作用，而是将这场运动的总体失败归咎于九人小组层面的“判断失误”以及基层领导不力。⁴⁵这与中国共产党领导层及其附属部门在处理大跃进惨败时采取的方法一致。在饥荒的后期，面对数百万人死于饥饿和地面上普遍存在的暴力，中国共产党领导层再也无法否认这是一场人为的灾难。它被迫对如此重大的灾难做出解释。与其质疑整体

utopian goal of making a perfect communist society, however, they attributed the failure of the Great Leap Forward to simple ‘errors of judgement’. To avoid being held accountable, which would have threatened the party’s power, they turned against the grassroots leadership and waged yet another campaign, this time attacking the ‘Five Winds’: exaggeration, communism/egalitarian, blind directive/leadership, commandism, and bureaucratism.* In other words, it was not the directives from above but the ideological subversion from below that was the cause of this man-made disaster: it was the ‘corrupt’ cadres on the ground who were at fault because of their intentional incompetence, not the leadership and its goals.

In the historiography of the Great Leap Forward, the ‘Five Winds’ explanation is regularly used by scholars both within and outside China to account for what happened. Yet, in his recent reflection on the campaign attacking the ‘Five Winds’, Zhang Guangyou, the former editor-in-chief of the CCP-owned newspaper, the *Peasants’ Daily* as well as an eyewitness of the campaign, was more thoughtful about the actual experience of those grassroots cadres and their practice on the ground: ‘Those cadres with the task of implementing [party policies] carried heavy burdens on their shoulders. It’s like being in a big play, they each had a part to play ... they had to deal with countless problems and variables on the ground. That’s very different from writing a news report. No journalists would be held accountable for what happened on the ground, but the cadres were.’⁴⁶ By shifting the burden from the decision-makers to those required to carry out their policies, the leadership looked for and found the perfect scapegoat, the purposeful incompetence of those underlings whose goal was to undermine the striving for the perfect, utopian society.

Having come up with their explanation for the past failures, the Nine-Man Subcommittee needed to introduce interventions to rectify the damage caused by the false claims and authoritarian methods on the ground as well as to deal with the urgency of ever-increasing morbidity. The vector control had not only failed to achieve its earlier goal of eradication, but it was also too costly for a country struggling to survive a horrendous famine. There were simply no

* With Mao’s permission, the campaign attacking the Five Winds began in the winter of 1960, more than a year before Liu Shaoqi officially announced that the famine was a man-made disaster in January 1962 at the enlarged party cadres conference, commonly known as the Seven Thousand Cadres Conference.

为了实现建立一个完美的共产主义社会的乌托邦式的目标，他们将大跃进的失败归咎于简单的“判断失误”。为了避免承担责任

（这会威胁到党的权力），他们转而反对基层领导，并发起了另一场运动，这次是攻击“五风”：浮夸风、共产风/平均主义、瞎指挥/领导、命令主义和官僚主义。^{*}换句话说，造成这场人为灾难的不是来自上面的指示，而是来自下面的意识形态颠覆：是基层“腐败”的干部因其故意的无能而犯了错误，而不是领导层及其目标。

在大跃进的历史研究中，“五风”的解释经常被中国国内外的学者用来解释所发生的事情。然而，在最近他对攻击“五风”运动的反思中，中共报纸《农民日报》的前总编辑、这场运动的目击者张广友对那些基层干部及其在基层的实践的实际经历进行了更深入的思考：“那些肩负着执行[党的政策]任务的干部肩负着沉重的负担。就像在一场大戏里，他们每个人都扮演着一个角色……他们必须处理无数的问题和基层的变数。这与写一篇新闻报道非常不同。记者不会为基层发生的事情负责，但干部会。⁴⁶”通过将负担从决策者转移到那些需要执行他们政策的人身上，领导层寻找并找到了完美的替罪羊，那些下属的故意无能，他们的目标是破坏对完美乌托邦社会的追求。

在提出了他们对过去失败的解释之后，“九人小组委员会”需要采取干预措施，以纠正虚假声明和专制方法在当地造成的损害，并应对日益增长的发病率的紧迫性。病媒控制不仅未能实现其早期的根除目标，而且对于一个在可怕的饥荒中挣扎求生的国家来说，成本也太高了。根本没有

* 经毛泽东的允许，攻击“五风”的运动始于1960年冬天，比刘少奇在1962年1月扩大的党干部会议（通常称为“七千人干部大会”）上正式宣布饥荒是人为灾难早了一年多。

resources to carry it out, let alone for it to succeed. In a time of national emergency, therapeutic interventions regained prominence. This was further catalyzed by the excitement generated when, in 1961, China successfully synthesized the non-antimonial antibacterial drug furapromidum and other nitrofurans derivatives. Initial experiments in using them to treat schistosomiasis were successful, but the early enthusiasm was short-lived. Experts quickly agreed that these drugs were less efficacious and also toxic, and as a result they were withdrawn from clinical use and mass production.⁴⁷

CONTROLLING SCHISTOSOMIASIS BY MASS THERAPY

As with any disease, an accurate diagnosis is of the utmost importance in determining the cause of any patient's symptoms. Yet this remained a major problem with schistosomiasis. In addition to the Kato-Katz smear, the hatching test was another common diagnosis test for schistosomiasis used in the PRC. This test determined whether the person was infected or not based on the observation of miracidia, the second stage of development of the parasitic eggs that were hatched from the patient's faeces. The test involves diluting faeces in water and allowing it to sediment. To achieve the highest possible accuracy, it also requires repeating the observation of any emergence of miracidia every three or four hours – in other words, several times a day. Like the smear test, the hatching test is normally inexpensive and requires minimal technical training. To perform the test, however, requires a certain length of time. Also, the sensitivity of both tests decreases with decreasing schistosome intensity. In the early 1960s, during the post-famine mass therapy work in the PRC, the sheer number of patients, combined with the severe shortage of both trained and untrained personnel and of equipment, put further stress on both forms of testing and thus undermined their accuracy. To meet the quantitative target often meant compromising or neglecting quality. Thus, the test procedure was frequently performed incorrectly. For instance, returning from their field inspection, experts from Sichuan Medical University and the provincial schistosomiasis control office were both appalled by how haphazardly the grassroots diagnostic work was being carried out.* They noted that there was no standard of diagnostic work across different rural counties. Some coun-

* Sichuan Medical University was formerly the Medical and Dental Faculty of the West China Union University, a joint venture of some Canadian missionary groups. It is now the Sichuan University Medical School.

资源来执行，更不用说成功了。在国家紧急时期，治疗性干预再次受到重视。1961年，中国成功合成了非锑抗菌药物呋喃妥因和其他硝基呋喃衍生物，由此产生的兴奋感进一步推动了这一进程。最初用它们治疗血吸虫病的实验是成功的，但早期的热情是短暂的。专家们很快一致认为，这些药物的疗效较差，而且有毒性，因此停止了临床使用和大规模生产。⁴⁷

通过大规模疗法控制血吸虫病

与任何疾病一样，准确的诊断对于确定任何患者症状的原因至关重要。然而，这仍然是血吸虫病的一个主要问题。除了加藤-加茨涂片外，孵化试验是中华人民共和国使用的另一种常见的血吸虫病诊断试验。该试验根据对毛蚴的观察来确定此人是否被感染，毛蚴是寄生虫卵发育的第二阶段，从患者的粪便中孵化出来。该试验包括将粪便在水中稀释并使其沉淀。为了达到尽可能高的准确性，还需要每三到四个小时重复观察一次毛蚴的出现——换句话说，一天几次。与涂片试验一样，孵化试验通常成本低廉，并且只需要最少的技术培训。但是，要执行该测试，需要一定的时间。而且，两种测试的敏感性都随着血吸虫强度的降低而降低。在1960年代初期，在中华人民共和国遭受饥荒后的大规模治疗工作中，大量患者，再加上训练有素和未经训练的人员以及设备的严重短缺，进一步加剧了两种测试的压力，从而损害了它们的准确性。为了满足定量目标，通常意味着要妥协或忽略质量。因此，测试程序经常执行不正确。例如，从野外检查返回后，四川医科大学和省血吸虫病防治办公室的专家都对基层诊断工作的随意性感到震惊。^{*} 他们指出，不同农村县的诊断工作没有标准。一些县

* 四川医科大学前身为华西协和大学的医学和牙科学院，由一些加拿大传教士团体合资创办。现为四川大学医学院。

ties only performed the hatching test, others only the smear test. This meant that it was impossible to do a comparative study of the results. Because of their lack of experience in detecting parasite eggs presented in faeces, grassroots health workers regularly confused the rather non-descript, round shape of the eggs with air bubbles or hookworm eggs, etc. In addition, many failed to understand that fluctuating temperatures would affect the speed of hatching, so they sometimes wrongly evaluated unhatched stool samples as negative and disposed of them without disinfecting. These false negative findings caused the disease to spread widely. Equally important, carrying out the Kato-Katz smear test requires the bulk storage of specimens as well as freezing facilities and transportation, but this was simply impossible due to the lack of resources and equipment and the general collapse of the transportation network. Because of the shortfall in equipment, grassroots health workers were encouraged to practise 'self-reliance'. They often had to make do by using unsterilized and not always leak-proof earthen bowls and tubs as stool containers. As result, the overall diagnosis results were unreliable; hence the poor treatment outcomes.⁴⁸ Similar problems were reported in other affected regions, including some schistosomiasis control 'model counties' such as Songjiang and Qingpu in the suburbs of Shanghai. There, when the actual treatment outcome failed to meet the estimated target, grassroots health workers would simply fabricate the figures. The cadres managing such work cared only that the assigned job was being done, and therefore they did not bother to verify the figures.⁴⁹

On the ground, eradicating or even controlling the disease by mass therapy was easier said than done, nor was it any less costly. From the outset, the PRC's health blueprint that put prevention first was sought partly as a cost-effective solution to address the existing health crisis and also to meet the financial shortfall. But the state was simply unable to deliver the promised free health and medical care. The advent of the People's Commune in 1958 was a hoped-for solution, and communes became the newest and highest form of administrative, economic, and political organization in rural China.* Each commune was divided into large brigades (most of which contained several small villages), and each large brigade was in turn divided into smaller production brigades. Households were organized into production brigades. On the ground, production brigades

* These People's Communes were gigantic farming collectives; some were as big as a township.

有的地方只做孵化试验, 有的地方只做涂片试验。这意味着无法对结果进行比较研究。由于他们缺乏检测粪便中寄生虫卵的经验, 基层卫生工作者经常将这些并不明显的圆形虫卵与气泡或钩虫卵等混淆。此外, 许多人没有理解温度波动会影响孵化速度, 因此他们有时错误地将未孵化的粪便样本评估为阴性, 并在未消毒的情况下处理掉。这些假阴性结果导致疾病广泛传播。同样重要的是, 进行加藤-卡茨涂片检查需要大量储存样本以及冷冻设施和运输, 但这由于缺乏资源和设备以及运输网络的普遍崩溃而根本不可能实现。由于设备短缺, 基层卫生工作者被鼓励实行“自力更生”。他们经常不得不使用未经消毒且并非总是防漏的土碗和盆作为粪便容器。因此, 总体诊断结果不可靠; 因此治疗效果不佳。⁴⁸ 在其他受影响地区也报告了类似问题, 包括上海郊区的松江和青浦等一些血吸虫病防治“模范县”。在那里, 当实际治疗效果未能达到预估目标时, 基层卫生工作者会简单地捏造数据。管理这项工作的干部只关心分配的工作是否完成, 因此他们懒得核实数据。⁴⁹

在实际操作中, 通过大规模治疗来根除甚至控制这种疾病说起来容易做起来难, 而且费用也不低。从一开始, 中华人民共和国将预防放在首位的卫生蓝图, 部分原因是为了找到一种经济有效的解决方案, 以应对现有的卫生危机, 并弥补资金缺口。但是, 国家根本无法兑现承诺的免费医疗保健。1958年人民公社的出现, 是人们希望的解决方案, 公社成为中国农村最新的、最高形式的行政、经济和政治组织。* 每个公社都划分为大队(其中大多数包含几个小村庄), 每个大队又划分为较小的生产队。家庭被组织到生产队中。在实际操作中, 生产队

* 这些人民公社是巨大的农业集体; 有些大到相当于一个乡镇。

were the units of accounting and farming production within the large farming collectives. This meant that the production brigade was responsible for financing health and medical care, although overall it was the commune that managed health care delivery and disease prevention. Mao envisioned that with the People's Commune, there would be a surplus of grain and an abundance of other commodities. This way, the production brigade would generate enough income to defray villagers' care and medical costs. However, the amount of funds available varied from brigade to brigade, depending on their productivity. Brigades with greater numbers of children, the sick, and the elderly had poor productivity and thus were unable to generate enough income to pay for their greater need for medical care. In the early days of the People's Commune, some wealthier communes subsidized poorer brigades or directly defrayed the villagers' medical and health care costs from the commune's centralized fund. This sometimes led to cadres, as well as villagers who had a good relationship with the cadres, abusing and overusing the 'free' care.⁵⁰ Toward the last months of 1960, in the campaign attacking the 'Five Winds', this practice came under fire as 'egalitarian' (一平二调) and was subsequently stopped and replaced by a collective medical system.^{*} This collective medical system required villagers to contribute a small amount to a collective welfare fund and also to pay a registration fee if hospital treatment was needed. In return, these villagers would benefit from 'free' medical and health care. Although the concept looked rational and workable, in practice, as soon as it encountered the realities on the ground, it proved to have some important flaws. First, the cost of care was never free. A standard seven-day treatment would require the schistosomiasis patient to stay in the hospital/clinic/treatment centre for the entire period. This meant that he would lose seven days' worth of work points – a large chunk of income for the family, especially if the patient was the primary breadwinner. Under the commune distribution system,

* Some researchers, including Fang Xiaoping, confused the collective medical system with the earlier co-operative medical system. While the collective medical system under the People's Commune shared some features with the co-operative medical system that began in the early 1950s and was reintroduced after 1965, there were fundamental differences between the two systems. The co-operative system was financed by voluntary contributions, whereas the collective system was financed by the collective welfare fund, which was not voluntary. To build up the collective welfare fund, the commune or the large production brigade automatically deducted villagers' work points without their consent.

生产队是大型农业集体内的会计和农业生产单位。这意味着生产队负责资助医疗保健，尽管总体上是公社管理医疗保健服务和疾病预防。毛泽东设想，随着人民公社的建立，粮食会有盈余，其他商品也会非常丰富。这样，生产队就能产生足够的收入来支付村民的医疗费用。然而，可用资金的数额因生产队的生产力而异。儿童、病人和老年人较多的生产队生产力低下，因此无法产生足够的收入来支付他们对医疗保健的更大需求。在人民公社的早期，一些较富裕的公社补贴较贫困的生产队，或直接从公社的集中资金中支付村民的医疗保健费用。有时，这导致干部以及与干部关系良好的村民滥用和过度使用“免费”医疗保健⁵⁰。在1960年最后几个月，在攻击“五风”的运动中，这种做法被指责为“平均主义”（一平二调），随后被停止并被集体医疗体系所取代^{*}。这种集体医疗体系要求村民向集体福利基金捐款少量资金，如果需要住院治疗，还需支付挂号费。作为回报，这些村民将从“免费”医疗保健中受益。虽然这个概念看起来合理可行，但在实践中，一旦遇到现实情况，就被证明存在一些重要的缺陷。首先，医疗费用从来都不是免费的。一个标准的七天疗程需要血吸虫病患者在医院/诊所/治疗中心住满整个疗程。这意味着他将损失七天的工作积分——这对家庭来说是一大笔收入，特别是如果病人是主要的养家糊口者。在公社分配制度下，

* 一些研究人员，包括方小平，将集体医疗制度与早期的合作医疗制度混淆。虽然人民公社下的集体医疗制度与20世纪50年代初开始并在1965年后重新引入的合作医疗制度有一些共同的特征，但两种制度之间存在根本的区别。合作医疗制度由自愿捐款资助，而集体医疗制度由集体福利基金资助，后者并非自愿。为了建立集体福利基金，公社或大型生产队在未经村民同意的情况下自动扣除村民的工分。

since the amount of food each family would receive depended on the amount of work points earned, the fewer the work points, the less food there would be. Even though there was some greater access to health care, its real costs, even under this system, resulted in many villagers who were carrying the disease refusing to seek treatment. For the severely ill and for the elderly as well as children, the collective medical system made them worse off. Since they were unable to work, or worked less, they earned fewer or even zero work points. To participate in the collective medical system in order to receive the needed medical treatment, they thus ended up in perpetual debt. In Hunan province's Yueyang region, the family of schistosomiasis patient Xie, from Binghu Commune, did not earn enough work points to exchange for sufficient food to feed all of the family's six members. They therefore had no extra work points to put into the collective medical pot. Xie was thus denied the much-needed treatment for his schistosomiasis.⁵¹ Further north in Sichuan province's Langzhou County, the Feng family, who lived in a poor mountainous village, had five children and a sick mother as well as two elderly members who were in poor health. Since only one family member could work to earn work points, the family did not generate enough work points to contribute to the collective fund. So that the sick mother could receive the medical treatment she needed, the family ended up in debt. With insufficient work points to repay the debts, the family had to endure living in squalid conditions, with neither adequate food to eat nor clean clothes to wear. In 1960, because of the mother's ill health and the lack of food, the family watched their newborn baby die of malnutrition. Their poverty was only ameliorated after one of the children joined the army and was able to send money back home to pay for his mother's medical expenses. In 2006, when I interviewed Mr Feng, he told me the family was still paying back the debt they owed to the commune, even though the commune system had been abolished in 1983.⁵² The Xie and Feng families' experiences were by no means unique. While researching for this book, as well as for my social history of the Great Famine, several villagers I interviewed told similar stories.⁵³

Second, a further complication was that with the advent of the People's Commune, commune clinics (the larger ones were called hospitals) took on the overall responsibility of delivering the mass treatment that was previously carried out by specialized hospitals or schistosomiasis control centres/stations. Many commune clinics

由于每个家庭获得的食物量取决于挣得的工分，工分越少，食物就越少。即使医疗保健的可及性有所提高，但即使在这种制度下，其实际成本也导致许多患有血吸虫病的村民拒绝就医。对于重病患者、老年人以及儿童来说，集体医疗制度使他们的境况更加糟糕。由于他们无法工作或工作较少，他们挣得的工分更少甚至为零。为了参与集体医疗制度以获得所需的医疗，他们最终陷入了永久的债务之中。在湖南省岳阳地区，来自冰湖公社的血吸虫病患者谢某一家，挣得的工分不足以换取足够的食物来养活全家六口人。因此，他们没有额外的工分投入集体医疗基金。谢某因此被剥夺了急需的血吸虫病治疗。⁵¹ 在更北的四川省阆中县，冯家住在一个贫困的山村，有五个孩子和一个生病的母亲，还有两个健康状况不佳的老人。由于只有一个家庭成员可以工作挣工分，因此该家庭没有产生足够的工分来贡献给集体基金。为了让生病的母亲能够接受她需要的医疗，这个家庭最终负债累累。由于没有足够的工分来偿还债务，这个家庭不得不忍受住在肮脏的环境中，既没有足够的食物吃，也没有干净的衣服穿。1960年，由于母亲的健康状况不佳和缺乏食物，这个家庭眼睁睁地看着他们的新生婴儿死于营养不良。他们的贫困状况在其中一个孩子参军并能够寄钱回家支付母亲的医疗费用后才得到缓解。2006年，当我采访冯先生时，他告诉我，即使公社制度在1983年已被废除，他的家人仍在偿还欠公社的债务。⁵² 谢家和冯家的经历绝非个例。在为本书以及我的大饥荒社会史进行研究时，我采访的几位村民讲述了类似的故事。⁵³

其次，另一个复杂的问题是，随着人民公社的出现，公社诊所（规模较大的称为医院）承担了全面负责提供大规模治疗的责任，而这些治疗 раньше 由专业医院或血吸虫病防治中心 / 站负责。许多公社诊所

were staffed by traditional healers who had come from diverse medical traditions. Most of them were unfamiliar with or had doubts about bio-medical interventions such as treating infected patients with sodium antimony tartrate. Understandably, some were simply reluctant to administer it. A few even contested how schistosomiasis and malaria were transmitted, given their understanding of patterns of illness as explained by different medical systems. Some viewed these diseases as caused by *qi* deficiency, and they doubted the efficacy of sodium antimony tartrate (for schistosomiasis) or cycloguanil (for malaria) as therapeutic drugs. In addition, because of the enormous size of some communes, commune hospital/clinics often had to involve younger villagers in the delivery of this mass therapy.* Most of these younger people had some education, and indeed some were even enthusiastic about relieving sick villagers of their suffering, but they lacked the necessary training, knowledge, and experience to carry out the sort of mass treatment required, which was always demanded in a great hurry. For instance, in eastern Sichuan's Wan County, when some villagers challenged grassroots health workers about the efficacy of the drug, the health workers could not come up with an adequate answer except for saying: 'I have no idea whether they will cure your problem. The authority from higher-ups told us to use it. That's all I know.'⁵⁴

Local authorities were confronted with the ongoing problem of labour shortages as a result of the Great Leap drive for more construction projects as well as the increased demands for higher agricultural yields. Therefore, they were constantly weighing the value of medical, political, and social risks and considering what work would bring them the maximum gain as individuals – for instance, their advancement in the party. Labour would be allocated according to such calculations. In Hunan province's Yiyang County, the local authority estimated that a male patient receiving a standard seven-day treatment would cost each production brigade the equivalent of moving five less cubic metres of earth.[†] With so many villagers in Yiyang in need of treatment, this would mean that they would fail the water conservancy target as set by the provincial authority. In addition to serious financial loss, it would also amount to a greater

* On average, each commune consisted of 5,000 households.

† Yiyang County in Hunan province was, and still is, one of the worst endemic counties along the Yangtze River.

由来自不同医疗传统的传统治疗师组成。他们中的大多数人对生物医学干预措施（如用酒石酸锑钠治疗感染患者）不熟悉或有所怀疑。可以理解的是，有些人只是不愿意使用它。考虑到他们对不同医疗系统解释的疾病模式的理解，少数人甚至对血吸虫病和疟疾的传播方式提出异议。有些人认为这些疾病是由气虚引起的，他们怀疑酒石酸锑钠（用于血吸虫病）或环胍（用于疟疾）作为治疗药物的疗效。此外，由于一些公社规模庞大，公社医院/诊所通常不得不让年轻的村民参与到这种大规模治疗的实施中。* 这些年轻人大多受过一些教育，事实上，有些人甚至热衷于减轻患病村民的痛苦，但他们缺乏必要的培训、知识和经验来开展所需的那种大规模治疗，而这种治疗总是被要求以非常快的速度进行。例如，在四川东部的万县，当一些村民质疑基层卫生工作者药物的疗效时，卫生工作者除了说：“我不知道它们是否能治好你的问题。上级部门告诉我们使用它。我只知道这些。”⁵⁴

由于大跃进运动推动了更多的建设项目，以及对更高农业产量的日益增长的需求，地方当局面临着持续的劳动力短缺问题。因此，他们不断权衡医疗、政治和社会风险的价值，并考虑什么工作能为他们个人带来最大收益——例如，他们在党内的晋升。劳动力将根据这些计算进行分配。在湖南省益阳县，地方当局估计，一名男性患者接受标准七天治疗将使每个生产队付出相当于少移动五立方米土的代价。[†] 由于益阳有如此多的村民需要治疗，这意味着他们将无法达到省政府设定的水利目标。除了严重的经济损失外，这还将导致更大的

* 平均而言，每个公社由 5000 户家庭组成。

† 湖南省益阳县过去是，现在仍然是长江沿岸最严重的流行病县之一。

political risk for them. Thus, the Yiyang County authority took the drastic decision not to treat any male patients at all. To make up for the treatment target handed down from the higher authority for the Anti-Schistosomiasis Campaign, they sent only women and children for the mass treatment. Since the untreated male patients continued to work on crowded construction sites that often involved standing in water or mud for a long period of time, the disease spread quickly. So it is no wonder that the morbidity rate continued to increase in spite of the claims made on paper for the large number of people treated. Similar situations were also reported in Zhejiang, Jiangsu, and Fujian provinces.⁵⁵

At the height of the famine, faced with the sharply rising numbers of schistosomiasis morbidity as well as decreased financial resources and manpower, the production brigade (responsible for financing the mass treatment), as well as the commune (responsible for organizing and delivering the treatment), came up with ever-more erratic solutions. The quality of treatment, and patient safety, were regularly compromised in desperate attempts to meet treatment targets at great speed. Despite the caution from experts and health authorities to put patient safety first after earlier incidents of patients dying of overdoses of the drug sodium antimony tartrate, speeding up treatment by increasing the drug dosage and shortening the length of treatment was a common problem.⁵⁶ The situation worsened as the famine took hold. To meet the even greater labour shortfall (due to the mass deaths and widespread illness resulting from famine), many grassroots authorities gave orders to further increase the drug dosage and reduce the length of the course of treatment. Although experts advising the local mass therapy, such as Dr Zhou Xuezhong in Jiaying, a renowned expert in mass therapy, were concerned about the potential risk, in the end they bowed to political pressure.⁵⁷ Over and over again, in the struggle between politics and science in the PRC, it was politics that always took precedence.

According to official data obtained by the American Schistosomiasis Delegation to the PRC, between 1958 and 1962 a record of five million patients throughout the country received the three-day intensive treatment.⁵⁸ Because of the high dosage of sodium antimony tartrate involved, sometimes administered by inexperienced grassroots health auxiliaries or local doctors who had little or no understanding of standard procedure in such bio-medical intervention, the treatment that aimed to cure often turned into a serious risk. In severe

这对他们来说是政治风险。因此，弋阳县政府做出了不治疗任何男性患者的极端决定。为了弥补上级下达的血吸虫病防治运动的治疗目标，他们只派妇女和儿童进行大规模治疗。由于未接受治疗的男性患者继续在拥挤的建筑工地工作，这些工地通常需要长时间站在水或泥里，因此疾病迅速蔓延。因此，尽管纸面上声称治疗了大量人群，但发病率持续上升也就不足为奇了。浙江、江苏、福建等地也报告了类似情况。⁵⁵

在饥荒高峰期，面对血吸虫病发病率急剧上升以及财政资源和人力减少的情况，生产队（负责资助大规模治疗）以及公社（负责组织和提供治疗）提出了越来越不稳定的解决方案。为了以极快的速度达到治疗目标，治疗质量和患者安全经常受到损害。尽管专家和卫生部门在早些时候发生患者因酒石酸锑钠过量而死亡的事件后，告诫要将患者安全放在首位，但通过增加药物剂量和缩短治疗时间来加速治疗是一个普遍存在的问题。⁵⁶ 随着饥荒的蔓延，情况变得更加糟糕。为了弥补更大的劳动力短缺（由于饥荒导致的大规模死亡和普遍疾病），许多基层政府下令进一步增加药物剂量并缩短疗程。尽管像嘉兴的周学章博士这样，为当地大规模治疗提供建议的专家（周学章是著名的大规模治疗专家）对潜在的风险感到担忧，但最终他们还是屈服于政治压力。⁵⁷ 在中华人民共和国政治与科学的斗争中，政治一次又一次地占据了主导地位。

根据美国血吸虫病代表团从中国获得的数据，1958年至1962年间，全国共有500万患者接受了为期三天的强化治疗。⁵⁸ 由于涉及酒石酸锑钠的高剂量，有时由缺乏经验的基层卫生辅助人员或对这种生物医学干预的标准程序知之甚少的当地医生管理，旨在治愈的治疗往往变成了严重的风险。在严重

cases, it sent patients to the grave even more quickly. One commune hospital in Hunan province's Lilin County, for instance, treated nine patients with the dosage meant for twenty-six people. It was no wonder, therefore, that the outcome was 'seriously severe'.⁵⁹ In Zhejiang province's Jiaxing region, approximately 80,000 patients were treated within one month. Many were given two days of intensive treatment. Because of the high dosage of drug applied, eighty patients died during treatment. One victim's family, whose relative died after the two-day treatment, appealed to the local authority. Instead of reviewing the underlying problem and stopping the intensive treatment, the authorities simply arrested the doctor who had overseen the treatment, Dr Xu. Xu was a much-loved local doctor of Chinese medicine and one of the many local doctors co-opted by the authority to support the mass therapy work because of the shortfall of trained personnel. The authorities gave him a three-year prison sentence for causing a death for which he was not responsible. Deeply distressed by the event, Dr Xu died not long after he was released from prison.⁶⁰

As a consequence of the famine, an increasing number of communes had to cut back on their salaried staff, including doctors and other health personnel. With fewer or no doctors, commune hospitals/clinics were dissolved, and the mass treatment work was handed over to the semi-state-owned united clinics.* The united clinics had been introduced in the early 1950s to co-opt private practitioners into the state system as well as to reduce the state expenditure on medical and health care for rural villagers, who accounted then for 80 per cent of the population. Until the introduction of the People's Commune in 1958, united clinics were the main source of medical and health care for rural villagers. Although these united clinics were part of the state medical system, they were, at least in theory, voluntary and run by a collective of practitioners. To partly finance themselves, they charged the patients fees. These clinics were also financed by the co-operative medical fund, which was based on voluntary donations. Each practitioner and member of staff received a salary according to their contribution. To be sustainable, these clinics needed to show positive outcomes. Since many of them were quite small (some consisted of only one or two doctors) and lacked preparation for and experience in mass therapy work, they opted

* 'United clinics' is also translated as 'union clinics'.

在这种情况下，它甚至更快地将病人送入坟墓。例如，湖南省醴陵县的一家公社医院用 26 个人的剂量治疗了 9 名病人。因此，结果“非常严重”也就不足为奇了。⁵⁹在浙江省嘉兴地区，大约有 8 万名患者在一个半月内接受了治疗。许多人接受了两天的强化治疗。由于使用了高剂量的药物，80 名患者在治疗期间死亡。一名受害者的家属，他们的亲属在两天的治疗后死亡，向地方当局提出上诉。当局没有审查根本问题并停止强化治疗，而是简单地逮捕了负责治疗的徐医生。徐医生是一位深受爱戴的当地中医，也是众多被当局征用以支持大规模治疗工作的当地医生之一，因为训练有素的人员短缺。当局判处他三年徒刑，理由是他造成了一起他没有责任的死亡。徐医生对这件事深感痛心，出狱后不久就去世了。⁶⁰

由于饥荒，越来越多的公社不得不削减他们的受薪员工，包括医生和其他卫生人员。由于医生减少或没有医生，公社医院/诊所解散，大规模治疗工作移交给了半国有的联合诊所。* 联合诊所于 20 世纪 50 年代初引入，目的是将私人从业者纳入国家体系，并减少国家在农村村民医疗保健方面的支出，当时农村村民占人口的 80%。在 1958 年人民公社成立之前，联合诊所是农村村民医疗保健的主要来源。虽然这些联合诊所是国家医疗系统的一部分，但至少在理论上，它们是自愿的，由从业者集体运营。为了部分资助自己，他们向病人收取费用。这些诊所也由合作医疗基金资助，该基金基于自愿捐款。每位从业者和工作人员都根据他们的贡献获得工资。为了可持续发展，这些诊所需要显示出积极的结果。由于它们中的许多规模都很小（有些只由一两名医生组成），并且缺乏大规模治疗工作的准备和经验，他们选择了

* “联合诊所”也可翻译为“联盟诊所”。

to treat only patients with minor symptoms. Inevitably, this meant that severely ill patients desperately in need of treatment were often deprived of medical care.⁶¹

EVER-RISING MORBIDITY IN THE AFTERMATH OF THE FAMINE

In the aftermath of the famine, the morbidity of schistosomiasis had shifted from mainly rural inhabitants to an increasing number of urban casualties. The flight of rural populations to urban centres for employment and food meant that the disease migrated with the population. In addition, the morbidity continued to rise at an alarming rate across all affected regions – partly because of the worsening of the famine as it spread to towns and cities. For instance, in metropolitan areas such as Shanghai and Hangzhou, the desperately hungry urban residents regularly stepped into nearby rivers to fish for food, and as a result many suffered acute infection.⁶² Their severe malnutrition also made them more susceptible to the disease. More important, as during the Great Leap Forward period, the increase in morbidity was linked to the expanded agricultural activities and to the growing number of people now participating in such activities. As part of the state relief effort, the authorities often exerted overt pressure on many government employees, factory workers, and PLA soldiers, as well as university and high school students, to go to the countryside to support agricultural work. With zero resistance to schistosomiasis and no protection from exposure to the vector, many quickly became infected. A 1962 telegram from the Nine-Man Subcommittee showed a huge number of acute infections reported in more than fifty counties in Anhui, Hubei, Jiangxi, Hunan, Jiangsu, and Shanghai. In Hubei province's Hanyang, Xinzhou, and Yangxin counties, the attempt to save rice crops from flooded paddies backfired, with more than 10,000 male villagers being infected with schistosomiasis. Among them were 300 cases of acute infection. The high morbidity left little or no labour to carry on the agriculture work. A few months later, in 1963, another emergency report from Jiangsu province stated that in the forty-two endemic counties, the morbidity rate had increased by more than 40 per cent compared to the previous year. Among the 6,031 cases of acute infection, most were group infections. In some cases, entire production brigades were infected at the same time. Since 80 per cent of victims were

只治疗有轻微症状的患者。不可避免地，这意味着急需治疗的重病患者经常被剥夺医疗服务。⁶¹

EVER-RISING MORBIDITY IN THE AFTERMATH OF THE FAMINE

在饥荒之后，血吸虫病的发病率已从主要农村居民转移到越来越多的城市伤亡者。农村人口涌入城市中心寻找工作和食物，这意味着疾病随人口迁移。此外，所有受影响地区的疾病发病率继续以惊人的速度上升——部分原因是饥荒蔓延到城镇和城市而恶化。例如，在上海和杭州等大都市地区，极度饥饿的城市居民经常下到附近的河流捕鱼，结果许多人遭受了急性感染。⁶² 他们的严重营养不良也使他们更容易感染这种疾病。更重要的是，与大跃进时期一样，发病率的增加与农业活动的扩大以及现在参与此类活动的人数不断增加有关。作为国家救济工作的一部分，当局经常公开向许多政府雇员、工厂工人、解放军士兵以及大学和高中生施加压力，让他们下乡支援农业工作。由于对血吸虫病零抵抗力，并且没有采取任何保护措施来避免接触媒介，许多人很快就被感染了。1962年九人小组委员会的一份电报显示，安徽、湖北、江西、湖南、江苏和上海等50多个县报告了大量急性感染病例。在湖北省的汉阳、新洲和阳新县，从被洪水淹没的稻田中抢救稻谷的尝试适得其反，导致10,000多名男性村民感染血吸虫病。其中有300例急性感染。高发病率导致几乎没有劳动力从事农业工作。几个月后，在1963年，江苏省的另一份紧急报告称，在42个地方病县，发病率比上一年增加了40%以上。在6,031例急性感染病例中，大多数是群体感染。在某些情况下，整个生产队同时被感染。由于80%的受害者是

prime labourers, no one remained to transport the harvested grain and fight against drought. In Anhui province's Guichi County, too, in an emergency attempt to save the grain from flooded paddies, cadres and students from thirty-six government offices and schools worked in the contaminated water with no protection whatsoever. The effort resulted in 772 people suffering acute infection, twenty-three of whom subsequently died. Among those who died, one production brigade lost more than half of its prime workers. In addition to the rise in human casualties, an increasing number of cattle also died of infection. In Jiangxi province, all of the 135 cattle on one state farm were infected with schistosomiasis; fifty-two of them died of it.⁶³ Fearing becoming infected, more and more people refused to work in the rice paddies. In 1963, a team of public security soldiers were sent to guard the local agricultural work in Sichuan province's Lushan County. Since their duty involved walking into rice paddies, 96 per cent became infected with schistosomiasis. After some soldiers died of the disease, the remaining soldiers refused to enter the rice paddies again. 'The leadership from higher up only cares about production, not human life,' they complained. Some even protested that they would rather be shot than step back into rice paddies.⁶⁴

RESISTANCE BY THE LOCAL CADRES

At the grassroots level, too, an increasing number of cadres contested the Anti-Schistosomiasis Campaign, seeing it as a financial loss or even a direct cause of the famine. In Sichuan province's Mianzhu County, some commune cadres and villagers blamed the effort to eradicate schistosomiasis as of 1956 for directly causing the famine: 'We had spent the past [six] years trying to eradicate schistosomiasis. We not only exhausted many labourers, we also wasted lots of money. To eradicate schistosomiasis is such hard work, which pays no dividend. Please spare us from it,' they pleaded. Some put it more bluntly: 'If you want to eradicate schistosomiasis, you send a team, but we will have nothing to do with it.' Cadres and public health workers assigned to implement the control work compared themselves to 'rats trapped in a barrel, being bullied from both ends': 'cadres don't support us, and villagers don't like us, how can we carry out the work?' they grumbled.⁶⁵ Aside from contesting schistosomiasis control, grassroots cadres throughout the country, as they weighed the urgent need to feed starving villagers and animals – and

主要劳动力, 没有人留下运输收获的粮食和对抗干旱。在安徽省的贵池县, 为了紧急抢救被洪水淹没稻田里的粮食, 来自三十六个政府机关和学校的干部和学生在受到污染的水中工作, 没有任何保护措施。这一努力导致 772 人患上急性感染, 其中 23 人随后死亡。在死者中, 一个生产队损失了一半以上的主要劳动力。除了人员伤亡增加外, 越来越多的牛也因感染而死亡。在江西省, 一个国营农场的 135 头牛全部感染了血吸虫病, 其中 52 头因此死亡。⁶³ 越来越多的人害怕被感染, 拒绝在稻田里工作。1963 年, 一支公安战士被派往四川省芦山县保卫当地的农业生产。由于他们的职责涉及到进入稻田, 96% 的人感染了血吸虫病。在一些士兵死于该疾病后, 剩下的士兵拒绝再次进入稻田。 “上面的领导只关心生产, 不关心人命, ” 他们抱怨道。有些人甚至抗议说, 他们宁愿被枪毙也不愿再踏入稻田。⁶⁴

地方基层干部的抵制

在基层, 越来越多的干部也对血吸虫病防治运动提出质疑, 认为这是一项财政损失, 甚至是造成饥荒的直接原因。四川省绵竹县的一些公社干部和村民将 1956 年开始的血吸虫病根除工作直接归咎于饥荒: “我们过去 [六] 年都在努力根除血吸虫病。我们不仅耗尽了大量劳动力, 还浪费了大量资金。根除血吸虫病是如此艰苦的工作, 没有任何回报。请饶了我们吧, ” 他们恳求道。有些人则更直截了当: “如果你想根除血吸虫病, 你可以派一个团队来, 但我们不会参与其中。” 被指派执行控制工作的干部和公共卫生工作者将自己比作 “困在桶里的老鼠, 两头受气”: “干部不支持我们, 村民不喜欢我们, 我们怎么能开展工作?” 他们抱怨道。⁶⁵ 除了质疑血吸虫病防治之外, 全国各地的基层干部在权衡喂饱饥饿的村民和牲畜的迫切需要时 ——

fulfil the state's grain procumbent target at the same time – saw health work in general as a bother, and it was handed down to the women activists in the commune to undertake. Despite being portrayed as 'half of the sky' by the state media propaganda, on the ground women were counted as 'half labour' in the agricultural fields. In Hunan province's Henyang County, the party secretary at Chashi commune passed the responsibility for disease prevention and health delivery to the heads of the production brigades. As in a basketball match, one brigade head turned around and passed it to the section head of the local Women's Federation. But the ball quickly bounced back: 'Health work was a lose/lose last year, and there is no way I am going to take it on again,' the head of the Women's Federation contended. Little or no support from the local leadership left many grassroots health workers feeling stranded. Frustrated, they began to slack off.⁶⁶ This was a pattern that repeated itself constantly: rather than confronting even the local authorities, their key to survival was trying to pass accountability to other groups on the ground.

The lack of cooperation from the grassroots cadres also contributed to the unreliability of the statistics. Besides slacking, as mentioned earlier some grassroots health workers, along with schistosomiasis control cadres, would sometimes simply invent figures: 'For many years, we had no resources or incentive to implement the control work. All we did was fill in the forms and hand them back to the high-ups,' explained a retired schistosomiasis control cadre in Hubei province's Jingzhou region, still one of the worst endemic regions on the Yangtze River. In an interview with her in 2003, this retired cadre admitted: 'We simply made up the figures. Officials from higher-up knew it all along. To avoid getting into trouble or being sacked from office, they needed us to inflate or forge figures.' She then pointed out to me some old dusty papers covered with spider webs and said: 'It's an open secret.'⁶⁷ In another interview, Professor Yin, a retired expert from a state-owned schistosomiasis control research institute in a nearby province, agreed: 'Every year, we went the countryside to collect data. Where to start? We simply went to see the grassroots health worker [who later came to be called the "Barefoot Doctor"]'. We would write down whatever she or he told us. We did not verify their figures. How could we? We did not even know how to get from one end of the village to the other. They were our best and only source. Besides, we did not want to stay long in the village. We just wanted to collect the data and leave. We did not question the data.'⁶⁸

在完成国家粮食征购目标的同时，普遍认为卫生工作是一种麻烦，并将其下放给公社的女积极分子承担。尽管国家媒体宣传将她们描绘成“半边天”，但在基层，妇女在农田中被视为“半劳力”。在湖南省衡阳县，茶石公社的党委书记将疾病预防和卫生服务的责任转交给生产大队的负责人。就像篮球比赛一样，一个大队负责人转身将球传给当地妇联的主任。但球很快又反弹回来：“去年的卫生工作是吃力不讨好，我再也不会接手了，”妇联主任争辩道。地方领导的几乎没有或根本没有支持，使许多基层卫生工作者感到孤立无援。他们感到沮丧，开始懈怠。⁶⁶这是一种不断重复的模式：他们生存的关键不是对抗地方当局，而是试图将责任推给其他基层群体。

基层干部的缺乏合作也导致了统计数据的不可靠。除了偷懒之外，正如前面提到的，一些基层卫生工作者，以及血吸虫病防治干部，有时会直接编造数字：“多年来，我们没有资源或动力来实施防治工作。我们所做的只是填写表格并将其交还给上级，”湖北省荆州地区一位退休的血吸虫病防治干部解释说，那里仍然是长江流域最严重的流行病地区之一。在2003年对她的采访中，这位退休干部承认：“我们只是编造了数字。上级官员一直都知道。为了避免惹麻烦或被撤职，他们需要我们夸大或伪造数字。”然后她指着一些布满蜘蛛网的旧文件对我说：“这是一个公开的秘密。”⁶⁷在另一次采访中，附近省份一家国有血吸虫病防治研究所的退休专家尹教授也同意：“每年，我们都去农村收集数据。从哪里开始呢？我们只是去拜访基层卫生工作者[后来被称为“赤脚医生”]。我们会记下他或她告诉我们的任何信息。我们没有核实他们的数字。我们怎么能呢？我们甚至不知道如何从村庄的一端走到另一端。他们是我们最好也是唯一的来源。此外，我们不想在村里呆太久。我们只想收集数据然后离开。我们没有质疑这些数据。”

This is the key to unravelling the conundrum of the repeated propaganda claims about the successful eradication of schistosomiasis followed by the public lodging of blame against specific groups, or even individuals, for the re-emergence of the disease. In Jingzhou, as elsewhere in China, the 'schistosomiasis plague' did not 're-emerge' in recent years as we had been led to believe.⁶⁹ It had never been controlled, since many of the 'near-eradication' claims were invented in the first place or were radically overestimated, as was later publicly acknowledged. The ever-shifting politics of public health trumped the reality of poor or mismanaged health care delivery and eradication. Success had to be the goal because the state had placed the overall improvement of the people's health at the centre of its social compact. Engage the masses, and the people's health would improve – and improve it had to, whether supported by the experiences of the people or not.

这是解开以下难题的关键：反复宣传成功消灭了血吸虫病，随后公众将责任归咎于特定群体，甚至个人，认为他们导致了该疾病的再次出现。在荆州，以及中国其他地方，近年来“血吸虫病瘟疫”并没有像我们被告知的那样“再次出现”。⁶⁹它从未得到控制，因为许多“接近根除”的说法最初是虚构的，或者被严重高估了，正如后来公开承认的那样。公共卫生领域不断变化的政治超越了糟糕或管理不善的医疗保健服务和根除的现实。成功必须是目标，因为国家已将全面改善人民健康置于其社会契约的中心。动员群众，人民健康就会改善——而且必须改善，无论是否得到人民的经验支持。

The People's Health in Crisis

SCHISTOSOMIASIS AS YESTERDAY'S PUBLIC HEALTH PROBLEM

The health consequences of the nationwide famine were horrific. As of 1961, famine-related oedema, gynaecological problems, and child malnutrition – referred to at the time as the ‘Three Diseases’ (三病) – ravaged the countryside as well as the cities. This meant that there were even fewer resources and less manpower for health authorities in affected regions to implement schistosomiasis control through mass therapy. In Hunan province's Xiangtan region, in 1962 more than 5 per cent of the local population suffered the ‘Three Diseases’, and the number was reported to be on the increase each month.¹ Similarly, in Sichuan province, one of the largest agricultural provinces badly hit by the famine, in the first half of 1962 a huge number of people died of oedema and other famine-related illnesses. Child malnutrition and rickets were also prevalent there. With 50 per cent of commune hospitals/clinics in Sichuan preoccupied with treating oedema, and with a decreased health budget, there was neither money nor personnel left to implement schistosomiasis control through mass therapy.²

In addition to the famine-related ‘Three Diseases’, in 1962 many provinces along or south of the Yangtze River also experienced some of the worst epidemics of malaria in recent history, as well as incidents of ‘El Tor’ (a particular strain of the bacterium *Vibrio cholerae*, also called paracholera and named after the site in Egypt where it was discovered in 1905 by Felix Gotschlich). The malaria outbreak was partly linked to the Great Leap drive to irrigate the ever-expanding

人民的健康危机

SCHISTOSOMIASIS AS YESTERDAY'S 公共卫生问题

全国性饥荒对健康造成的后果是可怕的。截至 1961 年，与饥荒相关的浮肿、妇科问题和儿童营养不良——当时被称为“三病”

（三病）——肆虐于乡村和城市。这意味着受影响地区的卫生部门用于通过大规模治疗来控制血吸虫病的资源和人力更少。在湖南省湘潭地区，1962 年当地人口中有超过 5% 的人患有“三病”，并且据报道每月都在增加。¹ 同样，在受饥荒严重打击的最大农业省份之一四川省，1962 年上半年，大量人口死于浮肿和其他与饥荒相关的疾病。儿童营养不良和佝偻病也很普遍。四川省 50% 的公社医院 / 诊所都在忙于治疗浮肿，加上卫生预算减少，既没有资金也没有人员来通过大规模治疗来控制血吸虫病。²

除了与饥荒相关的“三种疾病”外，1962 年长江沿线或以南的许多省份也经历了近代史上最严重的疟疾疫情，以及“埃尔托”（霍乱弧菌的一种特殊菌株，也称为副霍乱，以 1905 年 Felix Gotschlich 在埃及发现它的地点命名）的事件。疟疾爆发部分与大跃进运动有关，该运动旨在灌溉不断扩大的

agricultural land as well as power the rapid industrialization by building more dams. More agricultural land and more construction also entailed unrestricted deforestation. Intensive agricultural practice, irrigation, and dam-building, as well as deforestation, not only caused long-term damage to the environment through alkalisation and waterlogging of the soil but also increased malaria transmission through the explosion in the numbers of mosquitos.³ The official estimate of malaria morbidity in 1962 surpassed 10 million people, with the highest number of casualties in Jiangsu, Sichuan, and Zhejiang provinces. All three provinces were also the worst endemic schistosomiasis regions.

In the meantime, an El Tor pandemic had spread across large parts of Southeast Asia. As with the outbreaks of SARS in 2003 and avian influenza in 2013, Guangdong province in southern China was identified by the World Health Organization (WHO) as the location transmitting the disease to the nearby international port cities of Hong Kong and Macau. Worried that the disease would spread to the West, the WHO simply reclassified El Tor as cholera so that its member states could impose draconian quarantines and other control regulations. The official health propaganda and media reports in the People's Republic of China (PRC), however, suggested that the disease was brought into China from overseas, not dissimilar to the PRC's media coverage of the AIDS pandemic in the late 1980s. Suddenly, El Tor was of international and political importance to the PRC authorities. Regional health authorities in southern China were instructed to divert resources allocated for schistosomiasis control to El Tor research and control. For a while, schistosomiasis became yesterday's public health problem. Despite the official figure showing that more than 20,000 people suffered acute infection of schistosomiasis, the highest since 1957, and with more than 200 people dying as result, Su Delong and his team of researchers at Shanghai Medical School were asked to suspend their schistosomiasis work and to support El Tor control.* In Shanghai and the nearby regions, public health propaganda also prioritized El Tor prevention.⁴ The PRC authorities were much more concerned with the international image of China as the source of a new global epidemic than with

* The El Tor incident almost led Su Delong into further political trouble, since his research showed that the El Tor outbreaks in southern China were not transmitted from abroad; they were related to the high level of sodium chloride in the ground water.

农业用地以及通过修建更多水坝来推动快速工业化。更多的农业用地和更多的建设也意味着不受限制的森林砍伐。集约化的农业实践、灌溉和水坝建设,以及森林砍伐,不仅通过土壤碱化和涝渍对环境造成长期损害,还通过蚊子数量的爆炸式增长增加了疟疾的传播。³据官方估计,1962年疟疾发病率超过1000万人,其中江苏、四川和浙江的伤亡人数最多。这三个省份也是血吸虫病最严重的流行地区。

与此同时,埃尔托霍乱疫情已蔓延至东南亚大部分地区。与2003年的sars疫情和2013年的禽流感疫情一样,世界卫生组织(who)认定中国南方的广东省是将该疾病传播到邻近的香港和澳门国际港口城市的地点。由于担心该疾病会蔓延到西方,世卫组织干脆将埃尔托霍乱重新归类为霍乱,以便其成员国可以实施严厉的隔离和其他控制规定。然而,中华人民共和国(prc)的官方卫生宣传和媒体报道表明,该疾病是从海外传入中国的,这与中国媒体在20世纪80年代末对艾滋病大流行的报道非常相似。突然之间,埃尔托霍乱对中国当局具有国际和政治意义。华南地区的卫生部门接到指示,将分配用于血吸虫病控制的资源转移到埃尔托霍乱研究和控制上。有一段时间,血吸虫病成了昨天的公共卫生问题。尽管官方数据显示,有超过20,000人患有急性血吸虫病感染,这是自1957年以来最高的,并且有200多人因此死亡,但上海医学院的苏德隆及其研究团队被要求暂停他们的血吸虫病工作,并支持埃尔托霍乱的控制。*在上海和附近地区,公共卫生宣传也优先关注埃尔托霍乱的预防。⁴中国当局更关心中国作为新的全球流行病来源的国际形象,而不是

* El Tor事件几乎使苏德隆陷入进一步的政治麻烦,因为他的研究表明,中国南方的El Tor疫情并非来自国外,而是与地下水中高含量的氯化钠有关。

continuing, no matter how feebly, to try and deal with what had come to be seen as 'local' problems.

THE BREAKDOWN OF THE STATE HEALTH SYSTEM AND THE EMERGENCE OF A PRIVATE MEDICAL MARKET

At the same time, the lack of funding, as well as the severe food shortages spreading to cities and towns, led to a cutting back on even salaried government employees. As a result, a huge number of state-owned anti-schistosomiasis units were dissolved. According to an official estimate, schistosomiasis control work groups, often consisting of experts responsible for advising on technical aspects of control work in the fields as well as for quality control, had been reduced by half since 1957. In Sichuan province, one of the largest agricultural provinces ravaged by the famine and the resultant 'Three Diseases' as well as schistosomiasis and malaria, the annual funding in 1962 for parasitic disease prevention and therapy was 500,000 *yuan* (ca US\$100,000). The shortfall in funding led to cutting back on personnel and the acquisition of drugs, hence leading to a reduction in treatment. Because of the shortage of staff and drugs, many treatments already begun could not be completed. In addition to these staff and drug shortages, there was also an urgent lack of equipment. It was reported that 80 per cent of the necessary equipment, initially bought with the state's schistosomiasis control funding, had been damaged or destroyed because of lack of care. This added further burdens to the already stressed local health fund.⁵

Further down the ladder, as mentioned earlier, an increasing number of commune hospitals/clinics were shut down. In Hunan province's Henyang County, out of 660 commune hospitals/clinics, nearly half had neither money nor food to compensate their staff. With no food to feed their families, doctors and other health workers lost all interest in delivering any health care. The lack of shoes, umbrellas, and torches for grassroots health workers also prevented them from implementing local public health work.⁶ In some places, such as Yunnan province in the southwest, commune doctors were sent back to their original production brigades to work in the agricultural fields so that the commune did not have to feed them. Although in theory they could practise medicine in addition to doing agricultural work, in reality doing medical work would mean that

继续（无论多么微弱）尝试处理那些被视为“地方”问题的事情。

国家卫生体系的崩溃和私人医疗市场的出现

与此同时，资金的缺乏以及蔓延到城镇的严重粮食短缺，导致即使是领取薪水的政府雇员也受到削减。结果，大量国有血吸虫病防治单位被解散。根据官方估计，自 1957 年以来，血吸虫病控制工作组（通常由负责为田间控制工作的技术方面提供建议以及进行质量控制的专家组成）已经减少了一半。在四川省，这个受饥荒和由此产生的“三种疾病”以及血吸虫病和疟疾肆虐的最大农业省份之一，1962 年用于寄生虫病防治的年度资金为 50 万元（约合 10 万美元）。资金短缺导致人员削减和药品采购减少，从而导致治疗减少。由于人员和药物短缺，许多已经开始的治疗无法完成。除了这些人员和药物短缺外，还迫切需要设备。据报道，最初用国家血吸虫病控制资金购买的必要设备中，有 80% 因为缺乏维护而损坏或毁坏。这给本已紧张的地方卫生资金增加了进一步的负担。⁵

如前所述，随着层级的下降，越来越多的公社医院 / 诊所被关闭。在湖南省衡阳县，660 家公社医院 / 诊所中，近一半既没有钱也没有粮食来补偿其员工。由于没有食物养家糊口，医生和其他卫生工作者对提供任何医疗保健失去了兴趣。基层卫生工作者缺乏鞋子、雨伞和火把，也妨碍了他们开展当地的公共卫生工作。⁶ 在一些地方，如西南部的云南省，公社医生被送回原来的生产队，从事农业劳动，这样公社就不必养活他们了。虽然理论上他们可以在从事农业工作的同时行医，但实际上做医疗工作意味着

they would lose work points and thus access to food. As result, a number of them gave up practising medicine, whether Western or herbal medicine.⁷

Not only was the commune health care system collapsing, a growing number of state-owned county or township hospitals, which were the next level up in the pyramid of the PRC's centralized health system, were also faced with serious financial difficulties. As of late 1960, with less food, the state reduced the amount of the food ration. It also introduced a policy to drastically cut back on the number of government employees, similar to the policy of consolidation in 1953 during the 'Three Anti'-Campaign. Health departments throughout the country were some of the worst-hit sectors, since health work was seen as costing more money than it generated. Many doctors and health personnel became unemployed. Those who had originally come from the countryside were forced to return to their former production brigades where they could at least feed themselves and their families by doing agricultural work.⁸ Those with good skills and reputations, as well as social contacts or connections (*guanxi* 关系), managed to remain in towns and cities. They opened private practices where they charged high prices. A private medical market emerged to compete with the state-owned and the collective medical system. With fewer hospitals and more patients, access became a privilege, and those with power to grant such privileges, such as officials at state-owned hospitals, often abused their power. In addition to introducing fees for treatment like their private and semi-private competitors, many health officials as well as some high-ranking doctors used their position to extract more food or other goods that were in short supply. Some also bartered drugs and medical equipment for personal favours or money.⁹ In Sichuan province, for example, several state-owned county and city hospitals turned into narcotic black markets where medical personnel regularly prescribed morphine and other opiates for each other. The drugs were subsequently sold at a high price to newly opened private practices or were used as bribes.¹⁰ Dissatisfied with their work conditions and low pay and faced with uncertainty and increasing pressure, some of the health workers in government units gave up on the Chinese Communist Party's promise of the communist utopia and returned to older practices and their equally amorphous promises of divine help from the Buddha, spirits, or diviners.¹¹

他们会失去工作分，从而失去获得食物的途径。因此，他们中的许多人放弃了行医，无论是西医还是草药。

不仅是公社医疗体系崩溃，越来越多的国有县或乡镇医院（它们是中华人民共和国集中式医疗体系金字塔中的上一层）也面临着严重的财政困难。截至1960年末，由于食物减少，国家减少了食物配给量。它还推出了一项大幅削减政府雇员人数的政策，类似于1953年“三反”运动期间的合并政策。全国各地的卫生部门是受打击最严重的部门之一，因为卫生工作被认为比它产生的收益花费更多的钱。许多医生和卫生人员失业了。那些原本来自农村的人被迫返回他们以前的生产队，在那里他们至少可以通过从事农业工作来养活自己和家人。⁸那些拥有良好技能和声誉的人，以及社会关系或人脉 (*guanxi* 关系) 设法留在城镇。他们开设私人诊所，收取高价。一个私人医疗市场出现，与国有和集体医疗系统竞争。由于医院减少而患者增多，获得医疗服务成为一种特权，而那些有权授予这种特权的人，例如国有医院的官员，经常滥用他们的权力。除了像他们的私人和半私人竞争对手一样引入治疗费用外，许多卫生官员以及一些高级医生还利用他们的职位来榨取更多的食物或其他短缺的商品。有些人还用毒品和医疗设备来换取个人好处或金钱。⁹例如，在四川省，几家国有县市医院变成了麻醉品黑市，医疗人员定期相互开具吗啡和其他鸦片类药物。这些药物随后以高价出售给新开设的私人诊所或用作贿赂。¹⁰政府部门的一些卫生工作者对他们的工作条件和低工资感到不满，并面临着不确定性和日益增长的压力，他们放弃了中国共产党对共产主义乌托邦的承诺，转而回到旧的做法，以及他们同样模糊的来自佛陀、精神或占卜者的神圣帮助的承诺。¹¹

SEEKING HELP FROM SPIRITS

Not only did medical and health personnel lose their morale, but the lack and low standard of medical and health care throughout the countryside led many villagers to question the professional competence of health professionals, the reliability of the collective medical system, the state health care delivery, and the socialist welfare system in general. As this section shows, many so-called old 'feudal' and 'backward' practices that the Communist authorities and the modernizers (including many medical and health professionals) had worked hard to eliminate re-emerged and gained an ever-increasing number of adherents.

High up in the mountains and clinging to the bank of the Yangtze River, eastern Sichuan province's Wanxian region (nowadays Wanzhou in Chongqing municipality), for example, was one place where state welfare provisions, including health and medical care were (and still are) appallingly poor. Although on paper there existed a collective medical system with several commune clinics, the reality was that only one doctor of traditional medicine, assisted by his apprentice, attended the medical and health needs of villagers for this whole region consisting of several communes. Commune hospitals as well as disease prevention centres, which were few and far between, were all situated in townships. Since travel in and out of villages was a real hazard for most villagers, they had little or no access to hospital care. Likewise, health workers working at disease prevention centres rarely travelled to villages to implement the government's disease prevention programs.¹² During the Great Leap famine, nearly half of the local population died of starvation or of diseases resulting from food deprivation.* To survive, some villagers even ate the flesh of their dead children.¹³ Villagers lost their trust in the socialist state's welfare system. During the malaria outbreak in the summer of 1962, local villagers refused to go to the town hospitals or to take the Western drugs distributed by the state. Instead, they turned to local healing deities and performed spirit-healing rituals. Those who were healthy enough simply ran away to avoid

* In times of scarcity, food was a more powerful weapon than torture. Food deprivation was a common method used by local cadres to force villagers to work harder or to produce more food. Cf. Zhou, Xun, 'Violence during the Great Leap Forward', in *Cambridge World History of Violence*, vol. 4 (forthcoming).

向神灵寻求帮助

不仅医务人员士气低落，而且整个农村医疗保健的缺乏和低标准导致许多村民质疑卫生专业人员的专业能力、集体医疗体系的可靠性、国家医疗保健服务以及整个社会主义福利体系。正如本节所示，许多所谓的旧“封建”和“落后”的做法，共产党当局和现代化人士（包括许多医疗卫生专业人员）曾努力消除，但现在又重新出现，并获得了越来越多的追随者。

例如，位于四川省东部山区、紧靠长江沿岸的万县地区（现在是重庆市的万州区），是国家福利（包括医疗保健）极度匮乏的地方（现在仍然如此）。虽然名义上存在一个拥有多个公社诊所的集体医疗体系，但现实情况是，只有一个中医大夫在他的学徒的协助下，满足了这个由几个公社组成的整个地区村民的医疗和健康需求。公社医院以及疾病预防控制中心，数量很少且相隔遥远，都位于乡镇。由于对大多数村民来说，进出村庄的交通都非常危险，他们几乎无法获得住院治疗。同样，在疾病预防控制中心工作的卫生工作者也很少到村庄实施政府的疾病预防计划。¹² 在大跃进饥荒期间，近一半的当地人口死于饥饿或因食物匮乏导致的疾病。* 为了生存，一些村民甚至吃了他们死去的孩子的肉。¹³ 村民们失去了对社会主义国家福利体系的信任。1962年夏季的疟疾爆发期间，当地村民拒绝去城镇医院或服用国家分发的西药。相反，他们转向当地的治疗神灵，并举行精神治疗仪式。那些足够健康的人干脆逃跑以避免

* 在匮乏时期，食物比酷刑更有力。剥夺食物是地方干部强迫村民更努力工作或生产更多粮食的常用方法。参见周迅，“大跃进时期的暴力”，载于《剑桥世界暴力史》，第4卷（即将出版）。

being infected, just as they would have done prior to the Communist Liberation.¹⁴

The situation was thought to be worse in regions where the population consisted of a mixture of social and ethnic groups, who were treated as subalterns or officially classified as ‘minorities’. A report from the Sichuan Provincial Bureau of Health and Hygiene indicated that since free medical and health care had been withdrawn as of 1961, the implementation of medical and public health work among ‘minority’ groups in rural Sichuan became increasingly difficult. On the eve of the Communist Liberation in 1950, public health work and free medical care that centred on allopathic medicine had been a key strategy employed by the Chinese Communist Party (CCP) to gain support from the lower and middle strata of society in many of these so-called minority regions. State provision of free health and medical care had become a crucial thread that bound the local population to Mao and the CCP. For example, in Angla (nowadays in Qinghai province), after the CCP medical team cured her illness, a sixty-year-old woman was convinced that Chairman Mao was more powerful than the Sangji Lama. Another Tibetan woman recited sutras while holding Mao’s portrait over her head after being cured by the People’s Liberation Army (PLA) doctors.¹⁵ For these villagers, Mao had come to represent a new power at the top of the religious hierarchy of their world. The initial success of dispensing free allopathic medicine to cure illness, as well as the repeated state public health campaigns to engage the local population, had built up an expectation among many villagers for adequate state health provisions. They began to rely on the state to protect them against illnesses as well as to provide medical care in the event of them falling sick or in the occurrence of a disease outbreak.

After 1961, however, because of the lack of financial resources, free medical care was replaced by a pay-for-treatment system. In this new system, the patient had to pay either the total costs or part of them. Because most of the people living in these regions were economically worse off than populations in the other parts of China, they simply could not afford to go to the hospital for check-ups or treatment. Poverty also hindered public health work in these regions because many villagers could not afford to buy soap to wash their ‘dirty’ hands or to feed themselves an ‘adequate’ and ‘healthy’ diet as they had been taught to do by the state public health campaign.¹⁶ The withdrawal of the promised free care, the low standard

像他们在共产主义解放前一样被感染。¹⁴

人们认为，在人口由社会和民族群体混合组成的地区，情况会更糟，这些人被视为下属或被正式归类为“少数民族”。四川省卫生厅的一份报告指出，自从1961年取消免费医疗保健以来，在四川农村“少数民族”群体中开展医疗和公共卫生工作变得越来越困难。在1950年共产主义解放前夕，以西医为主的公共卫生工作和免费医疗是中共（CCP）争取许多所谓少数民族地区社会中下层阶层支持的关键战略。国家提供的免费医疗保健已成为将当地居民与毛泽东和中共联系起来的关键纽带。例如，在昂拉（现在青海省），在中共医疗队治好她的病后，一位六十岁的老妇人相信毛主席比桑吉喇嘛更强大。另一位藏族妇女在人民解放军（PLA）医生治好她的病后，手持毛泽东的画像，口中念诵经文。¹⁵对于这些村民来说，毛泽东已经成为他们世界宗教等级制度顶端的一种新力量的代表。免费提供西医治疗疾病的最初成功，以及国家反复开展的旨在吸引当地居民参与的公共卫生运动，已经在许多村民中建立了一种对国家充分提供卫生保健的期望。他们开始依赖国家来保护他们免受疾病侵害，并在他们生病或发生疾病爆发时提供医疗服务。

然而，在1961年之后，由于缺乏资金，免费医疗被有偿治疗制度所取代。在这个新制度下，病人必须支付全部或部分费用。由于居住在这些地区的大多数人在经济上不如中国其他地区的人口，他们根本无力去医院检查或治疗。贫困也阻碍了这些地区的公共卫生工作，因为许多村民买不起肥皂来清洗他们“肮脏”的手，或者按照国家公共卫生运动的教导，给自己提供“充足”和“健康”的饮食。¹⁶ 承诺的免费医疗的撤回，以及低标准

of care available, plus the state public health campaigns' persistent obliviousness to the cultural/social sensitivities and to the economic condition of the local population were perceived by these villagers as a breach of their trust in the CCP as the new ruler. As a result, not only did the number of outpatients as well as inpatients in state-run hospitals begin to decrease, but many villagers refused to even be treated by the government mobile medical teams. In contrast, lamas and shamans in many temples, which had been replaced by the socialist health care system in delivering health after the Communist Liberation, were again in high demand.¹⁷

Reports from the Yunnan provincial authority also sounded the alarm as a vernacular collective culture, including everyday healing rituals, strove to compete with socialism. This collective culture proved more capable than the latter in facilitating a convergence of programs and visions in this time of crisis.¹⁸ In the schistosomiasis endemic region of Chuxiong, collective rituals such as exorcism or sorcery involving entire villages became a regular feature of community life. Nestled among high mountains on all sides in the Yunnan-Guizhou high plateau on the edge of Himalayas, Chuxiong had always been considered a backwater of Chinese civilization with its mixed population and multiple cultural traditions. Different ethnic groups – some of them with their own subgroups – had their own popular gods and supernatural rituals. Being far away from the centre of the 'high' Chinese culture, it had always maintained a greater degree of cultural fluidity. In times of extreme calamity such as epidemics, existing cultural and religious boundaries would often collapse. Villagers from different cultural backgrounds who otherwise worshipped their own gods and goddesses would turn up at a public healing session. There they would bow to someone else's god or goddess, who, it was claimed, possessed healing power over a certain group of diseases, and perform a customary sacrificial ritual. They would also collectively recite an appropriate healing text. With the advent of the Communist Liberation in the 1950s, for a while, as Erik Mueggler's ethnographic study of one village in Chuxiong illustrates, the socialist state penetrated communities here by 'installing itself within, at the centre of production and social reproduction'. With the radical collectivization and the subsequent famine, however, this centre quickly turned hollow and literally collapsed. For these villagers, the Communist authority or socialist state had been transformed into an avaricious 'wild ghost' with an empty mouth

可获得的护理不足，加上国家公共卫生运动一直无视当地居民的文化 / 社会敏感性和经济状况，这些村民认为这是对他们信任中国共产党作为新统治者的背叛。因此，不仅国立医院的门诊和住院人数开始减少，而且许多村民甚至拒绝接受政府流动医疗队的治疗。与此相反，许多寺庙中的喇嘛和萨满巫师在共产主义解放后曾被社会主义医疗保健系统取代，再次受到高度需求。¹⁷

来自云南省政府的报告也发出了警报，因为包括日常治疗仪式在内的本土集体文化正努力与社会主义竞争。在危机时期，这种集体文化被证明比后者更能促进项目和愿景的融合。¹⁸ 在楚雄血吸虫病流行地区，包括整个村庄的驱魔或巫术等集体仪式成为社区生活的常态。楚雄四周高山环绕，坐落在喜马拉雅山边缘的云贵高原上，一直被认为是中华文明的落后地区，人口混杂，文化传统多样。不同的民族——其中一些民族还有自己的亚群体——都有自己的民间神灵和超自然仪式。由于远离“高级”中国文化的中心，它一直保持着更大的文化流动性。在流行病等极端灾难时期，现有的文化和宗教界限往往会崩溃。来自不同文化背景的村民，原本崇拜自己的神灵，也会出现在公共治疗场所。在那里，他们会向别人的神灵鞠躬，据说这些神灵拥有治疗某些疾病的力量，并举行传统的祭祀仪式。他们还会集体背诵一段适当的治疗文本。正如埃里克·穆格勒对楚雄一个村庄的民族志研究所示，随着 20 世纪 50 年代共产主义解放的到来，社会主义国家一度通过“将自身安置在生产和社会再生产的中心”渗透到这里的社区。然而，随着激进的集体化和随之而来的饥荒，这个中心迅速变得空洞并真正崩溃。对于这些村民来说，共产主义政权或社会主义国家已经变成了一个贪婪的“野鬼”，张着空洞的嘴巴。

who impotently demanded ever more obeisance and acknowledgement.¹⁹ While some resisted this 'ghost' by raiding the commune granaries and farming fields, others cursed or physically attacked local cadres who were seen as the 'ghost's' officials. Like elsewhere in the country, slacking became a more common way of resisting the Maoist collectivization program.²⁰

As the power of the CCP was displaced, there was a resurgence of traditional practices. Villagers interpreted the famine, as well as subsequent outbreaks of disease and their poor health in general, as the pain inflicted by the 'ghost' and the 'ghost's' officials. As Mueggler's study and a number of sources that have emerged from the party archives show, in the aftermath of the famine, ghost exorcism was revived as a popular practice. According to an official report, between 1961 and 1964 villagers from one commune in Chuxiong performed at least 227 exorcisms. This was coupled with a growing number of 'superstitious' practitioners. According to the official estimate, in 1964 there were at least 117 such practitioners across eight different communes who made a living by practising 'superstition'. Among them, many practised supernatural healings. Not only villagers turned to these supernatural healers for help; even cadres and party members became increasingly weary of the poor quality and inadequacy of the state system in addressing their health and medical needs, and they, too, sought the help of supernatural healers. After falling ill, the wife of a brigade cadre at Mahuangjing Commune, for example, refused to seek hospital treatment. Instead, she sent for a diviner to read her fortune. In Mahuangjing Commune, it was also reported that between 1961 and 1964 an entire village spent a total of 1,762 *yuan* on vernacular healing rituals. Even more alarming to the authorities was that these vernacular practices were attracting a growing number of disciples. These disciples had taken up apprenticeships in these traditional practices, and the majority of villagers took these 'superstitions' very seriously.²¹

Closer to the PRC's political and cultural centre in Beijing, an official report estimated that there were more than 50,000 practising wizards, witches, and fortune-tellers in Hebei province. In the aftermath of the famine and with the weakening of local party structures given the failures and famines, many party members and cadres here also took a lead in 'superstitious' activities.²² The extraordinary burden on the centrally organized state structures, many of which were only superficially supported by the overall population, meant that

他们无力地要求越来越多的服从和承认。¹⁹虽然有些人通过袭击公社粮仓和耕种田地来抵抗这个“鬼”，但另一些人则诅咒或人身攻击被视为“鬼”的官员的当地干部。与该国其他地方一样，怠工成为抵制毛主义集体化计划的一种更常见的方式。²⁰

随着中国共产党权力的转移，传统习俗复苏。村民们将饥荒以及随之而来的疾病爆发和普遍的健康状况不佳，解释为“鬼”和“鬼”的官员造成的痛苦。正如穆格勒的研究以及从党的档案中涌现出的一些资料表明，在饥荒之后，驱鬼作为一种流行的做法而复兴。据一份官方报告称，1961年至1964年间，楚雄一个公社的村民至少进行了227次驱鬼活动。与此同时，“迷信”从业者的人数也在不断增加。据官方估计，1964年，至少有117名此类从业者遍布八个不同的公社，他们以从事“迷信”为生。其中，许多人 практикуют 超自然疗法。不仅村民向这些超自然治疗师寻求帮助；甚至干部和党员也越来越厌倦国家系统在解决他们的健康和医疗需求方面的质量差和不足，他们也向超自然治疗师寻求帮助。例如，马皇井公社的一名大队干部妻子生病后，拒绝去医院治疗。相反，她派人请算命先生为她算命。据报道，在马皇井公社，1961年至1964年间，整个村庄在地方治疗仪式上总共花费了1,762元。更令当局震惊的是，这些地方习俗正在吸引越来越多的信徒。这些信徒已经接受了这些传统习俗的学徒训练，大多数村民都非常认真地对待这些“迷信”。²¹

河北省更靠近中国政治文化中心北京，一份官方报告估计，该省有超过5万名巫师、女巫和算命先生。在大饥荒之后，鉴于失败和饥荒，地方党组织结构有所削弱，许多党员和干部也带头参与“迷信”活动。²²中央组织的国家结构负担异常沉重，其中许多结构只是表面上得到全体人民的支持，这意味着

the repressed older social networks of belief, local in their manifestation and flexible in their appropriation of a wide range of response, resurfaced. The local beliefs and structures trumped the centrally planned economy because they maintained the trust of communities who were never quite sure of the consistency and efficacy of centralized responses.

What troubled the CCP leadership even more was an increasing number of sectarian insurgencies. Many of them had a strong apocalyptic streak: a major famine that was followed by outbreaks of diseases, as well as abuses in hospitals, came to be heavenly signs that foretold the world was coming to an end. In China's agricultural heartland, official estimates show that between 1961 and 1962 there were more than 8,000 incidents of sectarian activities in Hebei province and the surrounding regions.²³ Farther away in the mountainous southwest, in Guizhou province's Zunyi County, more than 1,000 villagers were reported to have taken part in the five-day Hungry Ghost Festival in the autumn of 1962. At the festival, villagers recounted the 'bitterness' they had suffered under the Communist rule and chanted verses such as: 'Oh, the dead, what a terrible life you suffered, and how miserable was your death. In 1959 there was no food, so you starved to death, just like that.' After quoting the Buddhist teachings on suffering and salvation, some declared that 'the real salvation is coming soon from heaven' and 'Chairman Mao's regime will last only another two and a half years.' This was deeply disturbing for the Communist Party Central Committee because Zunyi was a key location in the official history of the CCP. In 1935, during the Communist Long March, it was at the conference in Zunyi that Mao first rose to power and where the CCP and its Red Army (the PLA) succeeded in building its power structure. (Nowadays, Zunyi is a 'Red tourism' destination for visitors wishing to evoke the revolutionary past.) But with more than 5,000 villagers dead of starvation and illness within a single month in the winter of 1960-61 and with 50 per cent of the local population suffering from diseases of malnutrition and the absence of clean water, villagers in Zunyi began to challenge the official memory of the Communist revolution and its promise of Liberation. As the news of suffering in Zunyi spread, the Guanyin Laomu Dao sect, which the CCP had tried hard to suppress since the early 1950s, became active again in the nearby border regions across Guizhou, Hunan, and Sichuan. Beginning in 1962, this sect

那些受到压抑的、更为古老的信仰社会网络重新浮出水面，它们在表现形式上具有地域性，在对各种回应的运用上具有灵活性。地方信仰和结构胜过了中央计划经济，因为它们维持了社区的信任，而这些社区从未完全确信中央集权式回应的一致性和有效性。

让中国共产党领导层更加不安的是宗派叛乱事件日益增多。其中许多都带有强烈的末世色彩：一场大饥荒之后爆发了疾病，以及医院里的虐待行为，这些都被视为预示世界末日即将来临的天象。在中国农业腹地，官方估计显示，1961年至1962年间，河北省及周边地区发生了8000多起宗派活动。²³在更远处的西南山区，贵州省遵义县，据报道1962年秋天有1000多名村民参加了为期五天的饿鬼节。在节日上，村民们讲述了他们在共产党统治下所遭受的“苦难”，并吟唱着这样的诗句：“哦，死去的人啊，你们遭受了多么可怕的生活，你们的死亡是多么悲惨。1959年没有食物，所以你们就这样饿死了。”在引用了佛教关于苦难和拯救的教义后，一些人宣称“真正的拯救即将从天而降”，并且“毛主席的政权只能再维持两年半的时间。”这对中共中央委员会来说是极度不安的，因为遵义是中国共产党官方历史上的一个关键地点。1935年，在共产党长征期间，正是在遵义会议上，毛泽东首次崛起，中共及其红军（解放军）成功地建立了它的权力结构。（如今，遵义是希望唤起革命过去的游客的“红色旅游”目的地。）但在1960-61年冬天的短短一个月內，有5000多名村民死于饥饿和疾病，并且当地人口的50%患有营养不良和缺乏清洁水引起的疾病，遵义的村民开始挑战共产党革命的官方记忆及其解放的承诺。随着遵义苦难消息的传播，自20世纪50年代初以来一直被中共努力压制的观音老母道教派，再次活跃在贵州、湖南和四川附近的边境地区。从1962年开始，这个教派

had successfully recruited more than 1,200 new followers and built up their own power order by appointing their 'emperor' and 'marshals'. They had also built their own 'Central Altar' for the worship of Heaven and Earth to challenge the central power stemming from Beijing. The 'Central Altar' was surrounded by seven 'Branch Altars' for the worship of different gods, including the medicine god (药王). In the meantime, villagers seized a huge number of cattle owned by the People's Commune and killed them for food or as offerings to Heaven. Several party activists and cadres who had violently abused villagers for resisting their implementation of the Land Reform and the Maoist collectivization were assaulted. Violence was first undertaken by those suffering extreme hardship and then furthered by those enthused by apocalyptic visions that depicted a life and death battle between the Communist cadres and the starving villagers: 'We have inconceivable hatred towards the Communist Party'; 'To follow the words of Communist cadres is to die a hungry ghost. To live in human dignity, we must seek help from spirits.' Villagers shouted as the 'spirit' revealed to them: 'To live a good life is to work for yourself.'²⁴ Neither collectivization nor the utopian goals of the CCP mattered any more.

DECENTRALIZATION

In the meantime, in different parts of the countryside, more and more grassroots cadres and villagers began to push for agricultural de-collectivization.²⁵ In early 1961, for instance, at a Zunyi region cadres' meeting, grassroots cadres from different counties appealed to the regional authority to allow individual villagers to keep private plots and to grant the production brigade the power to manage grain collection as well as its local distribution. Since the regional authority was reluctant to reach any agreement on the matter, some local cadres, supported by villagers, took the matter into their own hands. In a number of communes, the collective canteens were dissolved. As brigade cadres went up the mountains to pick ferns for private consumption or to sell in an informal local market, villagers chopped down trees in the publicly owned forests and brought the wood home for private use. Opium, the main source of local revenue prior to the Communist Liberation in 1950, was 'openly' bartered in the illicit local market. Dozens of dens that had been closed down by the Communist authorities after the Liberation re-opened their

已成功招募了 1200 多名新追随者，并通过任命他们的“皇帝”和“元帅”来建立自己的权力秩序。他们还建造了自己的“中央祭坛”来祭拜天地，以挑战来自北京的中央权力。“中央祭坛”周围有七个“分坛”，用于祭拜不同的神灵，包括药神（药王）。与此同时，村民们夺取了人民公社拥有的大量牛，并宰杀它们作为食物或献给上天。一些因暴力虐待村民以抵抗他们实施的土地改革和毛泽东集体化而对村民施暴的党员积极分子和干部受到了袭击。暴力首先由那些遭受极端困苦的人采取，然后由那些对末日景象充满热情的人进一步推动，这些景象描绘了共产主义干部与饥饿的村民之间的一场生死之战：“我们对共产党怀有难以想象的仇恨”；“听从共产主义干部的话就是饿死鬼。为了活得有尊严，我们必须寻求神灵的帮助。”村民们喊道，因为“神灵”向他们揭示：“过上好日子就是为自己工作。”²⁴集体化和中国共产党乌托邦式的目标都不再重要。

权力下放

与此同时，在乡村的不同地区，越来越多的基层干部和村民开始推动农业的非集体化。²⁵例如，1961年初，在遵义地区干部会议上，来自不同县的基层干部呼吁地区当局允许村民保留自留地，并授予生产队管理粮食征收及其地方分配的权力。由于地区当局不愿就此事达成任何协议，一些地方干部在村民的支持下，开始自行其是。在一些公社，集体食堂被解散。当大队干部上山采摘蕨菜供个人食用或在非正规的当地市场出售时，村民们砍伐了公有森林中的树木，把木材带回家供个人使用。鸦片是1950年共产党解放前当地的主要收入来源，在非法的当地市场“公开”进行易货贸易。共产党当局解放后关闭的数十家烟馆重新开张。

doors despite the official restrictions. As ever-greater numbers of villagers died of starvation and famine-related illnesses, the survivors sought help from opium-smoking. Even cadres visited opium dens as a means of releasing their stress.²⁶

The situation in Zunyi was by no means an isolated case. From elsewhere in the countryside, top CCP leaders also learned that the peasants no longer trusted the party's policy or socialism in general. The widespread famine and the subsequent outbreaks of diseases had led many to question the party's political legitimacy. In Sichuan province's Hongya County, after the authorities suspended the promised state food subsidy for oedema patients, a female villager from Huaxi Commune was said to have been miraculously 'reincarnated' as the 'superior immortal'. She was accompanied by two other 'minor immortals'. All three had been classified as 'poor peasants' during the Land Reform, and they had initially been among the most loyal supporters of the Communist Party. But the famine propelled villagers such as these three individuals to turn away from the CCP and to take their survival into their own hands. They took the lead in spreading rumours such as 'the world is turning into a chaos; patients are dying in hospitals; taking medicine by injection or oral administration is killing people.' Under their influence, according to a commune hospital report, more and more sick villagers refused to seek medical treatment at the hospital. No longer trusting the socialist welfare system, they turned to these 'immortals' for guidance and help and began to collect donations to build a temple in their honour. With the temple becoming the centre of village life, villagers handed over any food they harvested to the temple, leaving nothing for the state granaries. And in turn, the temple redistributed the food to those in need.²⁷

Around the same time that this was happening in Hongya, Mao's nephew Mao Huachu went back to their home village, Dong Guantang in Shaoshan County, central Hunan province, to investigate local conditions. There, an elderly villager confronted him: 'Does your uncle [Chairman Mao] have any idea what has been going on in the past two years? You must let him know the situation here.' Here, in Mao's home county, according to an official estimate obtained by Mao's secretary Hu Qiaomu, the three years of famine had claimed more than 30,000 lives. By the end of 1960, the mortality and morbidity rates had reached a high of 70,000, and the situation was worsening at the time of Mao Huachu's visit. During

尽管有官方限制，但他们还是秘密打开了救济粮仓库的大门。由于越来越多的村民死于饥饿和与饥荒相关的疾病，幸存者开始寻求鸦片的帮助。甚至干部也光顾鸦片馆，以此来释放他们的压力。²⁶

遵义的情况绝非孤例。来自农村其他地区的中共高层领导人也了解到，农民不再信任党的政策或一般的社会主义。广泛的饥荒和随之而来的疾病爆发使许多人开始质疑党的政治合法性。在四川省洪雅县，当局暂停承诺的对水肿病人的国家粮食补贴后，据说华溪公社的一位女村民奇迹般地“转世”为“上仙”。她还带着另外两位“小仙”。这三人在土改期间都被归类为“贫农”，他们最初也是共产党最忠实的支持者。但是，饥荒迫使像这三个人这样的村民背离了共产党，并将他们的生存掌握在自己手中。他们带头散布谣言，例如“世界正在变成混乱；病人死在医院里；通过注射或口服药物正在杀死人。”根据公社医院的报告，在他们的影响下，越来越多的患病村民拒绝去医院就医。他们不再信任社会主义福利制度，而是向这些“神仙”寻求指导和帮助，并开始募捐以纪念他们建造一座寺庙。随着寺庙成为村庄生活的中心，村民们将收获的任何食物都交给寺庙，而没有给国家粮仓留下任何东西。反过来，寺庙将食物重新分配给那些需要帮助的人。²⁷

大约在洪雅发生这些事情的同一时间，毛的侄子毛华初回到湖南省中部韶山县东官塘老家，调查当地情况。在那里，一位年长的村民质问他：“你叔叔[毛主席]知道过去两年发生了什么吗？你一定要让他知道这里的情况。”据毛的秘书胡乔木获得的一份官方估计，在毛的家乡，三年的饥荒夺去了3万多人的生命。到1960年底，死亡率和发病率已高达7万人，在毛华初访问期间，情况还在恶化。在

a consultation meeting with Mao Huachu, some villagers cried out: 'Collectivization will never work ... If this [the collectivization] is not stopped, the problem will become even worse.'²⁸

'MAKE THEM TRUST US AGAIN'

Mao was well aware of the problem. By the winter of 1960-61, the famine crisis in the countryside had reached a tipping point. For the CCP to hold on to power, it was necessary to win back the support of the peasants, the bulwark of the CCP's power in the countryside. The CCP Central Committee began to introduce a gradual decentralization. In November 1960, to curb the damage caused by radical collectivization, the CCP Central Committee approved the 'Urgent Directives on Rural Work' (commonly known as the 'Twelve Articles'), drafted by Zhou Enlai. This directive officially removed the commune's power to make decisions. It also relieved it of the burden of managing the labour shortage. In the following months, unofficial practices converged with official policy to bring about change. Some production brigades took advantage of this directive and began to allow villagers to work for themselves and to sell their output or barter it for other goods in informal markets. Quickly, a shadow barter economy emerged in parallel to the socialist economic system. Hearing what happened on the ground, President Liu Shaoqi responded: 'To work for themselves is better than not to work at all.'²⁹ In a meeting with some provincial leaders from Zhejiang province in February 1961, Mao also agreed that it was necessary to make concession by allowing villagers to keep private plots: 'We have to let villagers keep their private plots ... To make them trust us again, we need to make this loud and clear to them.'³⁰ The meeting led to a draft version of the 'Work Regulations on Rural People's Commune' (commonly known as the 'Sixty Articles'), which formally granted the production brigade the autonomy to manage its own production and labour, while members of the production brigade were allowed to keep their own private plots. After several months of trial implementation as of the spring of 1961, a revised version of the 'Sixty Articles' was launched throughout China that autumn.

With the implementation of the 'Sixty Articles', the task of organizing snail elimination as part of schistosomiasis control was given to the production brigades. This had several advantages: 1. the production brigade could plan the snail control work in the less busy

在与毛华初的一次协商会议上, 一些村民哭喊道: “集体化永远行不通……如果[集体化]再不停止, 问题会变得更糟。”²⁸

'让Ta们再次相信我们'

毛非常清楚这个问题。到1960-61年冬天, 农村的饥荒危机已经达到了临界点。为了使中国共产党能够继续执政, 必须重新赢得农民的支持, 农民是中国共产党在农村的堡垒。中共中央开始推行逐步的权力下放。1960年11月, 为了遏制激进集体化造成的损害, 中共中央批准了由周恩来起草的《关于农村工作的紧急指示》(俗称“十二条”)。该指示正式取消了公社的决策权, 也减轻了其管理劳动力短缺的负担。在接下来的几个月里, 非官方的做法与官方政策相结合, 带来了改变。一些生产队利用这项指示, 开始允许村民为自己劳动, 并在非正规市场上出售或交换其产品。很快, 一个影子易货经济与社会主义经济体系并行出现。听到基层发生的事情后, 刘少奇主席回应说: “为自己劳动总比不劳动好。”²⁹ 在1961年2月与浙江省一些省级领导人的会晤中, 毛也同意有必要做出让步, 允许村民保留自留地: “我们必须让村民保留他们的自留地……为了让他们再次信任我们, 我们需要向他们大声明确地说明这一点。”³⁰ 这次会议促成了《农村人民公社工作条例》(俗称“六十条”)草案的形成, 该条例正式授予生产队管理其自身生产和劳动力的自主权, 同时允许生产队成员保留自己的自留地。经过1961年春季几个月的试行后, 修订后的“六十条”于当年秋季在中国各地推出。

随着“六十条”的实施, 将作为血吸虫病防治一部分的灭螺任务交给了生产队。这有几个优点: 1. 生产队可以在不那么繁忙的时候计划灭螺工作

season when the workload was lighter; and 2. the snail control work was carried out on a much smaller scale and gradually, thus requiring less labour each time. More important, under the 'Sixty Articles', the production brigade, as well as individual households, could generate extra income through side-line productions (副业) such as furniture-making or growing medicinal herbs. With this extra income, the production brigade could afford the labour for small-scale snail control work and even award villagers work points for their time. In Chuansha County near Shanghai (the county would later become famous for its village health worker or 'Barefoot Doctor' scheme), most production brigades organized middle school graduates and other unemployed youth to do the snail control work. This way, not only did the production brigade not dissipate its agricultural workforce, it also provided a livelihood for these youngsters. By awarding their time with work points, it gave them incentives to carry out the snail control work to a reasonably high standard.³¹ In Chuansha and in nearby counties, some brigades applied a similar principle to the sanitary control work as well as other public health activities as part of disease prevention. In addition to rewarding villagers for the time they spent on collecting animal and human night-soil for fertilizer, these brigades introduced a coupon system, which drew inspiration from an earlier schistosomiasis prevention experiment first begun in 1952 at Zhejiang province's schistosomiasis control model district at Sanhuanfan.³² To overcome the problem of villagers hiding night-soil for their private plots, each household would receive fertilizer coupons based on the number of people in the household. With fertilizer coupons, each household could buy back sterilized fertilizer free of schistosomiasis infection from the brigade to use on their private plots. In addition to the fertilizer coupons, these brigades also awarded grain coupons to each household based on the weight and quality of the night-soil they handed over to the brigade. At the end of the year, the household would use these coupons to buy food from the brigade.³³

Granting the brigade the autonomy to manage snail and sanitary control work meant that the outcome was often uneven, depending largely on local agency and resources. Dividing up management responsibility meant that it was never clear who would be responsible for the ditches between different production brigades. Also, there was no standard monetary value for the snail and sanitary work across the brigades. Some brigades paid more and some less. This

工作量较轻的季节; 2. 灭螺工作规模小得多, 而且是逐步进行的, 因此每次需要的劳动力也较少。更重要的是, 在“六十条”下, 生产队和个人家庭可以通过副业(副业)获得额外收入, 例如家具制作或种植药材。有了这笔额外收入, 生产队就可以负担小规模灭螺工作的劳动力, 甚至可以奖励村民工分。在上海附近的川沙县

(该县后来以其乡村卫生工作者或“赤脚医生”计划而闻名), 大多数生产队组织中学毕业生和其他失业青年从事灭螺工作。这样, 生产队不仅没有分散农业劳动力, 还为这些年轻人提供了生计。通过以工分奖励他们的时间, 这激励他们以相当高的标准进行灭螺工作。³¹ 在川沙及附近各县, 一些生产队也将类似的原则应用于卫生控制工作以及作为疾病预防一部分的其他公共卫生活动。除了奖励村民花费时间收集人畜粪便作为肥料外, 这些生产队还引入了一种票券制度, 该制度的灵感来自1952年在浙江省三环矾血吸虫病防治模范区首次开始的血吸虫病防治实验。³² 为了解决村民为私人土地隐藏粪便的问题, 每个家庭将根据家庭人口数量获得肥料票券。凭借肥料票券, 每个家庭可以从生产队免费购回经过消毒、不含血吸虫病感染的肥料, 用于他们的私人土地。除了肥料票券外, 这些生产队还根据家庭上交生产队的粪便重量和质量, 向每个家庭奖励粮票。年底, 家庭将使用这些票券从生产队购买食物。³³

授予生产队管理血吸虫和卫生控制工作的自主权意味着结果往往参差不齐, 很大程度上取决于当地机构和资源。分担管理责任意味着永远不清楚谁将负责不同生产队之间的沟渠。此外, 各生产队之间的血吸虫和卫生工作的标准货币价值也不一样。有些生产队付的更多, 有些则更少。这

caused resentment among those villagers who received fewer work points for the time they spent on schistosomiasis control or any other public health work; this would, in turn, impact the quality of such work. Partly to address these shortcomings, after consultation with impacted counties, in its draft five-year plan for schistosomiasis control, the Shanghai Region Disease Control Committee proposed that, according to local conditions, the production brigade should train one or two village health workers (auxiliaries) who, in addition to carrying out their normal agricultural work, would lead the local schistosomiasis control as well as other public health activities. The time they spent on such work would be counted as doing voluntary service, and they would be awarded work points accordingly.* Economically better-off brigades could also pay these health workers from the brigade welfare fund. The exact amount would be based on the villagers' evaluation of their work.³⁴

Like the coupon system, such a proposal for training village health workers was not entirely new. It drew inspiration in part from some earlier village health auxiliary schemes in counties around Shanghai, as well as in the nearby Jiangsu and Zhejiang provinces, prior to the Communist Liberation. In parts of Zhejiang province, such a scheme had been introduced in 1952 in the effort to control schistosomiasis. This scheme had suffered setbacks, however, after the introduction of the work points distribution system as of 1955, and with the advent of the People's Commune following 1958, the scheme had been abandoned.³⁵ Many brigades now welcomed it, for they saw it as an efficient way of utilizing manpower resources. In the aftermath of the famine, many former government or commune employees had been forced to return to the brigades to work for food. Many of these returnees were doctors or commune public health workers who were less capable of doing agricultural work. Their leadership in schistosomiasis control work would free other quality labourers to do agricultural work, and this would also spare brigade cadres from the burden of managing public health tasks. At the same time, given the rising mortality and morbidity, the impacted brigades saw the long-term economic benefit of schistosomiasis control, since it

* In the agricultural collective, non-agricultural work was called 'voluntary service'. Villagers would still receive work points for doing such work but much less than for doing agricultural work. Since a shortfall of labour was a widespread problem in many communes or brigades, these voluntary services were assigned to 'class enemies' or villagers who were physically unfit for agricultural work.

这引起了村民的不满，因为他们花在血吸虫病控制或任何其他公共卫生工作上的时间获得的工分较少；反过来，这会影响此类工作的质量。部分为了解决这些缺点，在与受影响县协商后，上海地区疾病控制委员会在其血吸虫病控制五年计划草案中建议，根据当地情况，生产队应培训一到两名乡村卫生员（辅助人员），他们除了从事正常的农业工作外，还将领导当地的血吸虫病控制以及其他公共卫生活动。他们花在这些工作上的时间将被视为做志愿服务，并将相应地奖励他们工分。* 经济条件较好的生产队也可以从生产队福利基金中支付这些卫生员的工资。具体金额将根据村民对他们工作的评价而定。³⁴

与票证制度一样，这种培训乡村卫生员的提议并非完全是新的。它部分借鉴了上海周边各县以及附近的江苏和浙江省在共产主义解放前的一些早期乡村卫生辅助计划。在浙江省的一些地区，1952年就引入了这种计划，以控制血吸虫病。然而，自从1955年实行工分制以来，以及1958年人民公社成立之后，该计划遭受了挫折，并被放弃了。³⁵ 现在许多生产队欢迎它，因为他们认为这是利用人力资源的有效方法。饥荒过后，许多前政府或公社雇员被迫返回生产队干活以获取食物。这些返回者中，许多是医生或公社公共卫生工作者，他们不太擅长农业劳动。他们领导血吸虫病防治工作将解放其他优质劳动力从事农业劳动，这也将减轻生产队干部管理公共卫生任务的负担。与此同时，鉴于死亡率和发病率的上升，受影响的生产队看到了血吸虫病控制的长期经济效益，因为它

* 在农业集体中，非农业工作被称为“义务劳动”。村民做这些工作仍然会获得工分，但比从事农业工作获得的工分要少得多。由于劳动力短缺是许多公社或生产队普遍存在的问题，这些义务劳动通常分配给“阶级敌人”或身体不适合农业劳动的村民。

would help to prevent a further drain on labour. More labour would mean higher productivity and thus better income for the brigade and for the villagers.

In the next couple of years, the number of village health auxiliaries grew steadily in the counties around Shanghai, Zhejiang, and Jiangsu province. Since they normally worked in the paddy fields barefoot like other villagers, local villagers nicknamed them 'Barefoot Doctors'. For instance, Zhejiang province's Jiaxing County trained 440 health auxiliaries to support local schistosomiasis control work.³⁶ The number was to explode over the next decade with the official nationwide launching of the 'Barefoot Doctor' program, as we will see in the next chapter. Yet the initial idea of 'Barefoot Doctors' did not begin as a policy from above; rather, it began as one of many local innovations in response to the collapse of the state-run rural health system. With the quiet acquiescence, if not active cooperation, of local cadres, these initially quiet health revolutions in villages accelerated the post-famine decentralization of rural health care delivery in the PRC.

The overall status of health care remained confused and conflicted after the peak of the famine. While sensible structures seemed to be emerging to support the work of disease control, they were often badly implemented or faced active resistance. Their success was never guaranteed. The successes were often due to local, small-scale implementation rather than to the mass efforts heralded in the state's propaganda. On the ground, it was individuals who made things work; if they saw some opening for local success, or if they decided they had had enough of state bungling and resisted, they guaranteed the failure of any enterprise.

将有助于防止劳动力进一步流失。更多的劳动力将意味着更高的生产力，从而为生产队和村民带来更好的收入。

在接下来的几年里，上海、浙江和江苏省周边各县的乡村卫生员人数稳步增长。由于他们通常像其他村民一样赤脚在稻田里劳作，当地村民戏称他们为“赤脚医生”。例如，浙江省嘉兴县培训了440名卫生员，以支持当地的血吸虫病防治工作。³⁶在接下来的十年里，随着全国范围内的“赤脚医生”计划的正式启动，这一数字将会爆炸式增长，我们将在下一章中看到这一点。然而，“赤脚医生”最初的想法并非源于自上而下的政策，而是源于对国家运营的农村卫生系统崩溃的众多地方创新之一。在地方干部的默许（如果不是积极合作）下，这些最初在乡村进行的悄无声息的卫生革命加速了中华人民共和国农村卫生保健服务在后饥荒时期的权力下放。

在饥荒高峰期过后，医疗保健的总体状况仍然混乱和冲突。虽然看起来正在出现一些合理的结构来支持疾病控制工作，但它们往往执行得很糟糕，或者面临积极的抵制。它们的成功从未得到保证。这些成功往往归功于地方性的小规模实施，而不是国家宣传中所宣扬的大规模努力。在基层，是个人使事情得以运作；如果他们看到当地取得成功的机会，或者如果他们认为已经受够了国家的拙劣表现并进行抵制，他们就会保证任何企业的失败。

The Health Worker as ‘True Revolutionary’

THE SOCIALIST EDUCATION CAMPAIGN

In 1962, as agricultural production in parts of rural China began to recover, the rural produce market, largely illicit, was thriving. Chen Yun, the vice chair of the Chinese Communist Party (CCP) Central Committee overseeing famine recovery, encouraged the limited free market as a measure for economic recovery. A variety of goods, including many food items such as pork and vegetables, as well as medicinal herbs that had disappeared for quite some time, reemerged for sale. Because of the collapse of the state health system in the rural countryside, herbal remedies were greatly in demand and were thus a valuable cash crop for villagers. In Zhejiang province's Haining County, prior to the collectivization medicinal herbs had been a major source of income for villagers at the Zhoujia market district. In 1962, almost every household there switched from food crops back to growing medicinal herbs.¹ In comparison, rice and cotton crops, as well as tobacco and oil seed rape, became less attractive for villagers because they would only be procured by the state. In some places, villagers opted to grow sweet potatoes or corn, since the state would not acquire such produce. As such, the state continued to face difficulties in procuring the projected quota of agricultural products. This meant that China would have to import 4 million tons of rice in 1962 and possibly 1.5 million tons in 1963, as well as 10,000 tons of cotton. The 5.5 million tons of rice would cost the state US\$400,000,000, and 10,000 tons of cotton would cost US\$8,000,000. After that, the state would have little or no resources left to purchase raw materials and machines for industrial development or to import drugs and ‘white goods’ such

作为 “真正的革命者” 的卫生工作者

the socialist education campaign

1962 年，随着中国农村部分地区农业生产开始复苏，主要为非法的农村农产品市场蓬勃发展。时任中共中央委员会副主席，负责饥荒恢复工作的陈云，鼓励实行有限的自由市场，以此作为经济复苏的措施。包括猪肉和蔬菜等多种食品以及已经消失了一段时间的药材等各种商品再次上市销售。由于农村地区国家卫生体系的崩溃，草药的需求量很大，因此成为村民们宝贵的经济作物。在浙江省海宁县，在集体化之前，药材一直是周家集市地区村民的主要收入来源。1962 年，几乎家家户户都从粮食作物转回种植药材。¹ 相比之下，水稻、棉花作物以及烟草和油菜籽对村民的吸引力降低，因为它们只能由国家采购。在一些地方，村民选择种植红薯或玉米，因为国家不会收购这些产品。因此，国家在采购预计的农产品配额方面仍然面临困难。这意味着中国将在 1962 年进口 400 万吨大米，可能在 1963 年进口 150 万吨大米，以及 1 万吨棉花。这 550 万吨大米将耗费国家 4 亿美元，而 1 万吨棉花将耗费 800 万美元。此后，国家几乎没有剩余资源来购买工业发展所需的原材料和机器，或进口药品和 “白色家电” 等。

as refrigerators and bicycles. To save the state 4 million 'lovely US dollars', Li Xiannian, the vice premier of the People's Republic (PRC) and also the minister of finance, handed out procurement quotas to provincial leaders.² Sichuan province, one of the largest agricultural provinces, again won the trophy for fulfilling the state quota. The cost for winning it was that the famine in Sichuan continued for another year into 1962, with oedema and other famine-related diseases claiming further casualties.³

In the meantime, a free labour market was also thriving. Many villagers (mostly the strong or skilled males) sold their labour to private bidders from across the country. They relied on a fragmented private medical market that had emerged after the collapse of the state system in the aftermath of the famine. Millions were on the move from one part of the country to the other for employment in a growing private sector: from farming to manufacturing, construction, handicrafts, and healing. Huge numbers of people were crossing the border to escape the famine but also socialism. In Yunnan province in the southwest, entire villages embarked on an exodus. Between April and May 1962, nearly 40,000 people, including many doctors, crossed the border into Hong Kong to seek a better life and freedom.⁴ The flight of health care professionals further undermined any reliance on the state system that centred on medical expertise. The authorities were anxious, since population control (in all senses of the phrase) was not only integral to central planning, it was also seen as essential to national security and the stability of the country. In May, the news of another 60,000 villagers crossing the border at Yili, a Kazakh autonomous prefecture in northwest China, and fleeing into the adjacent Soviet republics alarmed Mao and the CCP Central Committee. Mao was convinced that this was a conspiracy by the Soviet 'Revisionists' to undermine the PRC.⁵

Mao also became increasingly concerned about the growing 'revisionism' inside China. One cause for his anxiety was the ever-growing number of household farming plots and villagers' evident enthusiasm for individual farming. In parts of central Hunan, close to Mao's native home, according to Marshal Peng Dehuai's field research in late 1961,^{*} almost 80 per cent of the land was,

* Peng Dehuai was one of the leaders to criticize the Great Leap Forward policy at the Lushan Conference in August 1959. It led to Peng's downfall; he was disgraced as the leader of an 'anti-party clique'. In 1961, Peng was permitted to return to his home region in Hunan province to conduct a field investigation of the local conditions.

比如冰箱和自行车。为了给国家节省 400 万“可爱的美元”，中华人民共和国（prc）副总理兼财政部部长李先念向各省领导下达了采购配额。²四川省是最大的农业省份之一，再次赢得了完成国家配额的奖杯。赢得奖杯的代价是，四川的饥荒一直持续到 1962 年，水肿和其他与饥荒有关的疾病夺去了更多的生命。³

与此同时，自由劳动力市场也在蓬勃发展。许多村民（主要是强壮或熟练的男性）将他们的劳动力卖给来自全国各地的私人竞标者。他们依赖于饥荒过后国家体系崩溃后出现的零散的私人医疗市场。数百万人从该国的一个地方转移到另一个地方，在不断增长的私营部门寻找就业机会：从农业到制造业、建筑业、手工艺品和医疗。大量的人越过边境，逃离饥荒和社会主义。在西南部的云南省，整个村庄都开始了逃亡。1962 年 4 月至 5 月间，包括许多医生在内的近 4 万人越过边境进入香港，寻求更好的生活和自由。⁴ 医疗保健专业人员的流失进一步削弱了对以医疗专业知识为中心的国家体系的依赖。当局感到焦虑，因为人口控制（在所有意义上）不仅是中央计划的组成部分，而且也被视为对国家安全和国家稳定的重要组成部分。5 月，又有 6 万村民在伊犁（中国西北部的一个哈萨克自治州）越过边境，逃往邻近的苏维埃共和国的消息震惊了毛泽东和中共中央委员会。毛泽东坚信这是苏联“修正主义者”破坏 prc 的阴谋。⁵

毛泽东也越来越关注中国国内日益增长的“修正主义”。他焦虑的一个原因是越来越多的家庭农场和村民们对个体农业的明显热情。根据彭德怀元帅 1961 年末在湖南中部（靠近毛泽东的家乡）的实地调查，^{*} 几乎 80% 的土地是

* 彭德怀是 1959 年 8 月庐山会议上批评大跃进政策的领导人之一。这导致了彭的倒台；他被贬为“反党集团”的头目。1961 年，彭被允许返回湖南家乡，对当地情况进行实地调查。

because of local cadres' resourcefulness and inventiveness, privatized.⁶ In north Zhejiang too, by allowing individual households to plough around collective farming fields, more and larger household plots were developed. In some villages, the collective fields had virtually vanished. The popular term for this was 'dug into the collective ground' (挖边). It would later be reinterpreted as 'undermining socialism' (挖社会主义的墙脚) when the authorities tried to put a stop to the expansion of these household plots. Describing their plots as a 'golden rice bowl', 'treasure basket', or 'money trees', villagers would come up with tactics to circumvent or violate the official restrictions. Some entire villages were involved in the growth and production of medical herbals as a cash crop. In regions where local authorities took a more hard-line approach to stopping villagers taking part in such illicit activities, the actions of the authorities often led to violent clashes. With the increasing sectarian activities, there was an increased danger of these grassroots-level disputes escalating into a civil war between the state and the peasants.⁷

Although earlier on Mao had shown he was willing to make concessions by allowing peasants to keep some household plots, he saw it only as a short-term solution to what he considered a 'temporary' food crisis. 'The household plot (自留地) is a mere leftover of private ownership. It is not dangerous [to the socialist economical system].' He was full of confidence about their transitory nature.⁸ Between the spring and early summer of 1962, while touring around the country, Mao became increasingly obsessed about the idea that 'revisionists' were seizing the countryside. He saw villagers' enthusiasm for private ownership and markets as evidence of 'revisionist restoration': 'The revisionists are turning five billion peasants into petit bourgeois. Giving authority to the petit bourgeois is to embrace petit bourgeois dictatorship. It has never happened in history.'⁹ On 7 July, Mao warned against Chen Yun and Tian Jiaying, who were in favour of a 'household contract' system as a measure for famine recovery, arguing that such a system would destroy the rural collective economy and ruin the People's Commune system: 'It is Chinese-style revisionism.' And Mao was determined to fight revisionism; for him, to fight against revisionism was to safeguard the dictatorship of the proletariat and hence the world communist movement. At the Tenth Plenum of the Central Committee's Eight Party Conference in the autumn of 1962, he launched a fierce attack on those party leaders who sought to restore the Chinese economy by

由于地方干部的机智和创造性，私有化了。⁶在浙江北部也是如此，通过允许个体户在集体农田周围耕作，开发了更多更大的农户地块。在一些村庄，集体土地实际上已经消失了。对此流行的说法是

“挖边”。当当局试图阻止这些家庭地块的扩张时，它后来被重新解释为“挖社会主义墙脚”。村民们将他们的地块描述为“金饭碗”、“聚宝盆”或“摇钱树”，并会想出各种策略来规避或违反官方的限制。一些村庄甚至参与了药用草本植物的种植和生产，将其作为经济作物。在地方当局采取更强硬的手段阻止村民参与此类非法活动的地区，当局的行为往往导致激烈的冲突。随着宗派活动的增加，这些基层纠纷有升级为国家与农民之间的内战的危险。⁷

尽管毛泽东早些时候曾表示愿意做出让步，允许农民保留一些自留地，但他认为这只是解决他所谓的“暂时”粮食危机的短期方案。“自留地不过是私有制的一点残余。它对社会主义经济制度[并不危险]。”他对它们的过渡性充满信心。⁸1962年春夏之交，毛泽东在全国巡视期间，越来越痴迷于“修正主义分子”正在夺取农村的观点。他认为村民对私有制和市场的热情是“修正主义复辟”的证据：“修正主义分子正在把五十亿农民变成小资产阶级。把权力交给小资产阶级就是拥抱小资产阶级专政。历史上从未发生过这样的事。”⁹7月7日，毛泽东警告陈云和田家英，他们赞成以“家庭承包”制作为饥荒恢复的措施，认为这种制度会破坏农村集体经济，摧毁人民公社制度：“这是中国式的修正主义。”毛泽东决心与修正主义作斗争；对他来说，与修正主义作斗争就是捍卫无产阶级专政，从而捍卫世界共产主义运动。在1962年秋天举行的中共八届十中全会上，他对那些试图通过以下方式恢复中国经济的党的领导人发起了猛烈的攻击

allowing privatization. According to him, to uphold the socialist collective economy and fight against capitalism was a matter of 'class struggle'. 'Never forget class struggle,' the chairman exclaimed.¹⁰

Mao had always believed that private farming practice, as well as the 'backwardness' of the Chinese peasantry, was a root cause of rural poverty. For him, to attain and consolidate socialism, these 'undereducated' and 'self-centred' individuals needed to be 'socialized' into collectives. This meant educating and persuading the Chinese peasantry to embrace agricultural collectivization.¹¹ It was with this belief, when responding to peasants' resistance to collectivization in 1957, that Mao had pushed for the CCP Central Committee to launch the 'Socialist Education Campaign' in the Chinese countryside.¹² Four years later, in 1961, with the Great Leap falling apart and peasants demanding privatization, he found Tan Zhenlin on his side. In November 1961, at the Central Committee Politburo meeting discussing the difficulty of procuring projected agricultural products, Tan argued against the idea of permitting the rural marketplace:

It's not a matter of buying and selling goods. This is a political matter ... We need to introduce socialist education ... Our problem now is not about the one or two catties more cotton we would be able to procure. Our problem is we don't even know how much has been produced in the countryside ... We can't allow this. This is not socialist economy. This is not planned economy ... to procure grains, cottons, as well as agricultural products of commercial values such as oil, tobacco, and tea was to fight against revisionism and uphold Marxism ... We need to explain to peasants what socialism and socialist construction is. We need to help them to understand the relationship between the state, the collective, and the individual. We must remind them of our fine revolutionary tradition.¹³

Deng Xiaoping, the man who would march China to embrace neo-liberalism some thirty years later, also added his voice and insisted on the need to change peasants' 'state of mind' so that they would be persuaded to grow cotton: 'Our party must enforce that. This is an important matter.'¹⁴

Gradually, the recharged Socialist Education Campaign was dragged out in different parts of rural China. Officials as well as

允许私有化。他认为，维护社会主义集体经济和反对资本主义是“阶级斗争”的问题。“永远不要忘记阶级斗争，”主席疾呼道。¹⁰

毛泽东一直认为，个体农业经营以及中国农民的“落后”是农村贫困的根本原因。对他来说，要实现和巩固社会主义，就需要将这些“未受教育”和“以自我为中心”的个人“社会化”到集体中。这意味着教育和说服中国农民接受农业集体化。¹¹正是基于这种信念，在1957年回应农民对集体化的抵制时，毛泽东推动中共中央委员会在中国农村发起“社会主义教育运动”。¹²四年后，1961年，随着大跃进的瓦解和农民要求私有化，他找到了谭震林的支持。1961年11月，在中央政治局会议讨论采购预期农产品困难时，谭震林反对允许农村市场的想法：

这不是买卖商品的问题。这是一个政治问题……我们需要引进社会主义教育……我们当前的问题不是我们能够获得多一两斤棉花。我们的问题是我们甚至不知道农村生产了多少……我们不能允许这样。这不是社会主义经济。这不是计划经济……采购粮食、棉花以及具有商业价值的农产品，如石油、烟草和茶叶，是为了反对修正主义和坚持马克思主义……我们需要向农民解释什么是社会主义和社会主义建设。我们需要帮助他们理解国家、集体和个人之间的关系。我们必须提醒他们我们优秀的革命传统。

¹³

邓小平，这位在三十年后将带领中国走向新自由主义的人，也发表了自己的看法，并坚持认为需要改变农民的“思想状态”，以便说服他们种植棉花：“我们党必须强制执行。这是一件重要的事情。”¹⁴

逐渐地，重新启动的社会主义教育运动在中国农村的不同地区被拖延。官员以及

other government employees were encouraged to go to the countryside to 'unite with the peasantry' so that together they would 'pull out the roots of revisionism'. Despite the constant pressure from their immediate party boss, Ke Qingshi, who was also in charge of the official Anti-Schistosomiasis Campaign, provincial leaders in Jiangsu were reluctant to implement the Socialist Education Campaign. They argued that there had been too many campaigns, including those aimed at improving people's health: 'Grassroots cadres have suffered enough stress. The productivity has only just begun to recover. They simply have no strength to carry out another campaign.'¹⁵ In the meantime, the practice of sideline production, with illicit markets and household plots, including the production and sale of medicinal herbs, continued in Jiangsu. On the other side of the country, the feedback from Hunan and Hebei province was cheering to Mao. There, the Socialist Education Campaign had indeed led to class struggle against 'counter-revolutionary substage', local 'capitalists and feudal powerholders'.¹⁶ Mao decided to disseminate their 'successful' experiences and to push forward the Socialist Education Campaign throughout rural China. In May 1963, Mao approved Song Renqong's proposal to raise ordinary people's socialist consciousness by engaging them in a revolutionary oral history project. 'To educate ordinary people, in particular young people, by talking to them about their village history, family history, community history and factory history' should, according to Mao, be widely promoted.¹⁷

In the countryside, the revolutionary public/oral history campaign often turned into mass political meetings where tales of illness and disease suffered in the 'old' China were contrasted to the CCP's care for the people in the 'new' China. In the schistosomiasis endemic regions, the schistosomiasis control in 'new' China was the topic that was most evoked. Speaking at the Ninth National Conference on Schistosomiasis Control, Wei Wenbo and Xu Yunbei (Xu was the deputy minister of health and had had a long career in party propaganda work) put it explicitly that the harmful impact of schistosomiasis and its control under the CCP leadership would 'arouse people's affection for the party' and raise their political consciousness, thus inspiring them to fight the disease - 'the enemy of the natural world'.¹⁸ In 1964, an official estimate claimed that the film *Spring Comes to the Withered Tree*

其他政府雇员被鼓励去农村“与农民结合”，以便他们一起“拔除修正主义的根源”。尽管来自他们的直接党老板柯庆施（他同时负责官方的血吸虫病防治运动）的不断压力，江苏的省级领导人不愿意实施社会主义教育运动。他们认为运动太多了，包括那些旨在改善人民健康的运动：“基层干部已经承受了足够的压力。生产力才刚刚开始恢复。他们根本没有力量开展另一场运动。”¹⁵与此同时，包括药材生产和销售在内的副业生产、非法市场和自留地在江苏继续存在。在中国的另一边，来自湖南和河北省的反馈让毛泽东感到高兴。在那里，社会主义教育运动确实导致了反对“反革命次级阶段”、当地“资本家和封建权力持有者”的阶级斗争。¹⁶毛泽东决定传播他们的“成功”经验，并在中国农村全面推进社会主义教育运动。1963年5月，毛泽东批准了宋任穷的建议，即通过让他们参与革命口述历史项目来提高普通民众的社会主义意识。“通过与普通民众，特别是年轻人，谈论他们的村庄历史、家庭历史、社区历史和工厂历史来教育他们”，应该按照毛泽东的说法，广泛推广。¹⁷

在农村，革命性的公众/口述历史运动经常演变成大规模的政治会议，在会上，人们讲述了在“旧”中国所遭受的疾病和痛苦，并与中国共产党在“新”中国对人民的关怀形成对比。在血吸虫病流行地区，“新”中国的血吸虫病防治工作是被提及最多的主题。在第九次全国血吸虫病防治工作会议上，魏文伯和徐运北（徐是卫生部副部长，长期从事党的宣传工作）明确指出，在中国共产党领导下，血吸虫病的危害及其防治将“唤起人民对党的感情”，提高他们的政治觉悟，从而激励他们与疾病作斗争——“自然界的敌人”。¹⁸1964年，一项官方估计声称电影《枯木逢春》

(see chapter 4) had been screened more than 1,100 times in these regions, with 440,000 viewers. More than 180,000 posters of the popularized version of Mao's poem 'Send off the Plague God' were distributed to villagers' homes and schools.¹⁹ Yu Xiang was at the time a grassroots schistosomiasis control cadre in a remote mountainous region in Sichuan province. In a 2014 interview, he recalled that, throughout the Long 1970s, political meetings where tales of how schistosomiasis had been controlled under CCP leadership became a core part of schistosomiasis control on the ground.²⁰ Public health education was simultaneously political propaganda, whether true or not.

Equally important, as Xu Yunbei proclaimed, 'fighting schistosomiasis is a class struggle'. 'In the endemic region, class enemies are particularly active. [Outbreaks of diseases] provide them with opportunities to spread feudal superstitious ideologies, and to attempt to restore old orders. To fight disease is thus to struggle against class enemies.' Because the PRC's Anti-Schistosomiasis Campaign was an integral part of the socialist economy, the management of human and animal night-soil (to be used as fertilizer) and water was intrinsic to the sustainability of a rural socialist collective economy. But with an increasing number of villagers fighting to keep their private plots, they also refused to hand over domestic night-soil to the farming collective so that they could keep it for their private use. The continued success of schistosomiasis control, according to Xu, was thus 'crucial to uphold the socialist collective economy and improve agricultural productivities'.²¹

While agreeing with his colleagues at the Ministry of Health, Liao Luyan, the minister for agriculture, also highlighted the challenge ahead: 'We cannot just give orders to 500 million peasants, it won't work. We must convince them.'²² To convince 500 million peasants, according to Ke Qingshi, the party boss in charge of the campaign, was to 'straighten out their thinking' (思想通). This would be achieved by educating the peasants: 'If we don't educate [peasants], how can we practise socialism?' 'We won't succeed by relying on a few [scientists]. Scientists' task is research. That's a different matter ... We must rely on the masses. We need to straighten out their thinking ... So they would fight the disease themselves,' Ke reminded the delegates at the Ninth National Conference on Schistosomiasis Control.²³ Echoing Ke, Xu Yunbai went on to

(见第4章)在这些地区放映了1100多次,观众达44万人。超过18万张普及版的毛泽东诗词《送瘟神》海报被分发到村民的家中和学校。¹⁹于祥当时是四川省偏远山区的一名基层血吸虫病防治干部。在2014年的一次采访中,他回忆说,在整个20世纪70年代漫长的时期里,讲述在中国共产党领导下如何控制血吸虫病的故事的政治会议,成为基层血吸虫病防治工作的核心组成部分。²⁰公共卫生教育同时也是政治宣传,无论真假。

同样重要的是,正如徐运北所宣称的那样,“防治血吸虫病是一场阶级斗争”。“在流行地区,阶级敌人特别活跃。[疾病的爆发]为他们提供了传播封建迷信思想和企图恢复旧秩序的机会。因此,与疾病作斗争就是与阶级敌人作斗争。”由于中华人民共和国的血吸虫病防治运动是社会主义经济不可分割的一部分,因此人畜粪便(用作肥料)和水的管理对于农村社会主义集体经济的可持续性至关重要。但是,随着越来越多的村民为保留自己的自留地而斗争,他们也拒绝将家庭粪便交给农业集体,以便他们可以将其用于私人用途。徐认为,血吸虫病防治的持续成功因此“对于维护社会主义集体经济和提高农业生产力至关重要”。²¹

农业部长廖鲁言虽然同意卫生部的同事们的观点,但也强调了未来的挑战:“我们不能仅仅向5亿农民下命令,这是行不通的。我们必须说服他们。”²²在负责这场运动的党内领导人柯庆施看来,说服5亿农民就是要“理顺他们的思想”(思想通)。这将通过教育农民来实现:“如果我们不教育[农民],我们怎么能实践社会主义呢?”“仅仅依靠少数[科学家],我们不会成功的。科学家的任务是研究。那是另一回事……我们必须依靠群众。我们需要理顺他们的思想……这样他们就会自己与疾病作斗争,”柯在第九届全国血吸虫病防治会议上提醒代表们。²³徐运北呼应了柯的观点,并接着说

stress that public health work was to 'change prevailing social customs': a 'class struggle at a cultural front'. It could be won only by following the 'mass line':*

From managing night-soil to eliminating snails, washing hands and receiving the right kind of therapy, these have to do with changing peasants' mentality and their unclean habits. It is a behaviour issue. Health work is to change prevailing social customs. It is cultural work. We can't just rely on government policy or the effort of a few health professionals. We must educate and mobilize the masses to fight unclean habits and diseases.²⁴

The Nine-Man Subcommittee set up Sichuan province's Zundao Commune in Mianyang County as the model for others to follow. Accordingly, by upholding the collective economy, by struggling against 'privatization' and 'class enemies', as well as by contracting night-soil collection and snail elimination tasks to politically 'reliable' villagers, from village cadres to party members and model workers – and with the spirit of the Foolish Old Man who 'moved high mountains', as advocated by Chairman Mao (see the next chapter) – Zundao Commune boasted that it had moved the 'high mountain' and successfully managed night-soil collection and fulfilled their snail elimination target. Three weeks after the Ninth National Conference on Schistosomiasis Control, on 24 January 1964, the *People's Daily's* editorial entitled 'Advance Further Following the Victory, and Fight the War to Eradicate Schistosomiasis' called for 'All to follow the example of Zundao to control schistosomiasis by following the mass line'. 'To improve productivity, to reinforce the socialist economy and strengthen the People's Commune, to speed up building the socialist new countryside, we must put schistosomiasis prevention and eradication high on our agenda.'²⁵ The Anti-Schistosomiasis Campaign was once more at the forefront of the PRC's public health work.

* The 'mass line' is a leadership and organizational method first developed by Mao during the Chinese civil war. According to Mao, all revolutionary war was a war of the masses, and it could only be won by mobilizing the masses. Since the communist praxis was to change their manner of living, moving toward the modernization of the collective, to reach such goals, including those of public health, would have to involve the whole population.

强调公共卫生工作是为了“改变流行的社会习俗”：“文化战线上的一场阶级斗争”。只有遵循“群众路线”才能赢得胜利：*

从管理夜粪到消灭钉螺，洗手和接受正确的治疗，这些都与改变农民的观念和他们不卫生的习惯有关。这是一个行为问题。卫生工作是为了改变流行的社会习俗。这是一项文化工作。我们不能仅仅依靠政府的政策或少数卫生专业人员的努力。我们必须教育和动员群众与不卫生的习惯和疾病作斗争。²⁴

九人小组将四川省绵阳市的遵道公社树立为其他地方学习的榜样。因此，通过坚持集体经济，通过反对“私有化”和“阶级敌人”，以及将夜粪收集和消灭钉螺的任务承包给政治上“可靠”的村民，从村干部到党员和模范工人——并以毛主席倡导的“愚公移山”的精神（见下一章）——遵道公社自豪地宣称，他们已经移走了“高山”，成功地管理了夜粪的收集，并完成了他们的消灭钉螺目标。在第九届全国血吸虫病防治会议三周后，1964年1月24日，《人民日报》发表题为《乘胜前进，打一场消灭血吸虫病的战争》的社论，呼吁“大家学习遵道的榜样，通过群众路线控制血吸虫病”。“为了提高生产力，加强社会主义经济，巩固人民公社，加快建设社会主义新农村，我们必须把血吸虫病防治工作提到重要议事日程。”²⁵ 防治血吸虫病运动再次成为中华人民共和国公共卫生工作的前沿。

* “群众路线”是毛泽东在中国内战期间首创的一种领导和组织方法。毛泽东认为，一切革命战争都是群众的战争，只有动员群众才能赢得胜利。由于共产主义的实践是要改变人们的生活方式，朝着集体现代化迈进，要达到这些目标，包括公共卫生，就必须让全体人民参与进来。

THE HEALTH WORKER AS MORAL CRUSADER:
FROM LEI FENG TO NORMAN BETHUNE

Up to this point, the Anti-Schistosomiasis Campaign as the centerpiece of the PRC's public health work had been a top-down campaign led by the party, delivered by experts as well as health cadres, and aimed at disease eradication. With the collapse of rural health care delivery in the aftermath of the famine, the orientation of health work shifted to prophylactic medicine and the provision of basic, if often poor-quality, care. The burden of health care delivery also shifted from experts and hospitals to individuals, in this case grassroots health auxiliaries. This shift from top-down to bottom-up in health care delivery also changed the goal from disease eradication to an emphasis on the political rhetoric of 'serving the people wholeheartedly'. As Xu Yunbai stated, in this renewed war against schistosomiasis it was the health worker who was the 'pioneer' and the 'true revolutionary'.²⁶ Being the 'true revolutionary' was to be 'selfless' like Lei Feng and Dr Norman Bethune,* who had devotedly and selflessly served the revolutionary cause, the party, and the people.

Today, people in China still debate whether Lei Feng ever really existed. Whether he was a real person or a political myth, what matters is that Lei Feng left behind nine volumes of his diary and reading notes as well as more than 300 photographs. Generations of Chinese youth have read his words, and many were moved by Lei Feng's 'selfless spirit'. Song Renqiong, the man who proposed using oral history to raise ordinary people's socialist consciousness, was also the architect behind the creation of this 'selfless' revolutionary hero. A native of Hunan province, Song was also a New Cultural youth who joined the Chinese Communist Party in the 1920s and had since devoted his life to the revolutionary cause of 'saving' China and transforming it into a 'new, independent, and prosperous' society. Experienced in party political work, Song was the People's Liberation Army (PLA) commissary who took charge of the CCP's political consolidation of Yunnan province in the early 1950s. Bordering Burma, Vietnam, and Laos, Yunnan was, and still is, one of the most ethnically diverse regions in China, where the Chinese population was

* Norman Bethune was the Canadian physician and a noted communist who served as a medical volunteer for the CCP's Eighth Route Army in the Shanxi-Hebei border region of China between 1937 and 1939 during the Sino-Japanese War.

THE HEALTH WORKER AS MORAL CRUSADER:
FROM LEI FENG TO NORMAN BETHUNE

到目前为止,作为中国公共卫生工作核心的反血吸虫病运动,是由党领导的自上而下的运动,由专家和卫生干部执行,旨在根除疾病。在饥荒之后农村卫生保健服务体系崩溃的情况下,卫生工作的方向转变为预防医学和提供基本的(尽管通常质量较差)护理。卫生保健服务的负担也从专家和医院转移到个人,在本例中是基层卫生辅助人员。这种卫生保健服务从自上而下到自下而上的转变,也改变了目标,从根除疾病转变为强调“全心全意为人民服务”的政治辞藻。正如徐运北所说,在这场新的抗击血吸虫病战争中,卫生工作者是“先锋”和“真正的革命者”。²⁶成为“真正的革命者”就像雷锋和诺尔曼·白求恩医生一样“无私”,*他们忠诚而无私地为革命事业、党和人民服务。

今天,中国人民仍在争论雷锋是否真的存在过。无论他是一个真实的人物还是一个政治神话,重要的是雷锋留下了九卷日记和读书笔记,以及300多张照片。一代又一代的中国青年阅读了他的文字,许多人被雷锋的“无私精神”所感动。宋任穷是提出用口述历史来提高普通民众社会主义觉悟的人,他也是创造这个“无私”革命英雄的幕后推手。宋任穷是湖南省人,也是一位新文化青年,他在20世纪20年代加入了中国共产党,此后一生致力于“拯救”中国并将其转变为“新的、独立的、繁荣的”社会的革命事业。宋任穷在党的政治工作方面经验丰富,他是中国人民解放军(PLA)的政委,在20世纪50年代初负责中共对云南省的政治巩固。云南与缅甸、越南和老挝接壤,过去是,现在仍然是中国民族最多样化的地区之一,中国的汉族人口在那里是

* 诺尔曼·白求恩是加拿大医生,也是一位著名的共产主义者,曾在1937年至1939年中国抗日战争期间,作为医疗志愿者为中共的八路军在晋察冀边区服务。

only a small minority. To allow a relatively 'peaceful' takeover of the region, Song focused on cultural work, including introducing farming technologies as well as providing free medical care based on allopathic medicine. To make the local population feel part of the socialist 'motherland', Song also organized many mass meetings where the Communist soldiers and cadres made 'friends' with the locals by singing revolutionary songs and dancing together (instead of giving them interminable public lectures). By being 'friends' with the local people, the CCP cadres also taught villagers the 'clean' and 'socialist way of life' – from washing their hands with soap to emptying their bowels in the modern toilets built by the new Communist authorities. To bring the socialist cultural revolution to the local population, under Song's leadership the CCP authorities also commissioned linguistic experts to help in the creation of 'indigenous' written languages and systematized cultures for these people. By learning to read and write and by singing revolutionary songs that had their roots in the genre of village ballads, these villagers could thus participate in the 'new socialist-ethnic' culture.²⁷ Yet in 1955, Song Renqiong did not hesitate to hand down killing quotas when the local population resisted joining the agricultural collectives as well as the subsequent policy of handing over their harvested food to the state.²⁸

Following his successful tenure in the consolidation of the southwest, Song went on to lead the upgrading of the PLA and to direct the political work in the PRC's emerging nuclear industry. In 1960, Song was appointed the party boss of the northeast region (formerly known as Manchuria), the PRC's heavy industry zone bordering the far eastern region of the Soviet Union. Song was also the commissary of the Shenyang Military Region for the PLA. It was during this period that Song popularized, or indeed created, Lei Feng and resurrected the 'fine revolutionary tradition of the Eighth Route Army', the latter including the 'spirit of Dr Norman Bethune'. Health policy, whether real or invented, remained a central part of the vocabulary of the new national identity across all reaches of the PRC.

In 1961, as the state granary stocks were running low, the food crisis began to hit the large cities such as Beijing and Shanghai as well as the industrial centres in the northeast. In the previous year, responding to Song's appeal for help, the State Council signed off on 1 billion kilograms of food aid to the northeast. But in December 1961, at the State Planning meeting, Premier Zhou Enlai made it

只有少数人。为了实现该地区相对“和平”的接管，宋重视文化工作，包括引进农业技术以及提供基于对抗疗法的免费医疗服务。为了让当地居民感受到自己是社会主义“祖国”的一部分，宋还组织了许多群众大会，共产党员和干部通过唱革命歌曲和一起跳舞（而不是没完没了地给他们上公共课）与当地人“交朋友”。通过与当地人民“交朋友”，共产党干部还教村民“干净”和“社会主义的生活方式”——从用肥皂洗手到在新共产主义当局建造的现代厕所里排便。为了将社会主义文化革命带给当地人民，在宋的领导下，共产党当局还委托语言专家帮助为这些人创造“本土”书面语言和系统化的文化。通过学习阅读和写作，以及演唱植根于乡村民谣的革命歌曲，这些村民可以参与“新的社会主义民族”文化。²⁷然而，在1955年，当当地人民抵制加入农业集体以及随后的将收获的粮食上交国家的政策时，宋任穷毫不犹豫地发布了杀人指标。²⁸

在成功巩固西南地区之后，宋平继续领导军队的升级，并指导中国新兴核工业的政治工作。1960年，宋平被任命为东北地区（原名满洲里）的党委书记，该地区是中国与苏联远东地区接壤的重工业区。宋平还是中国人民解放军沈阳军区的政委。正是在这一时期，宋平推广，或者说是创造了雷锋，并复兴了“八路军的优良革命传统”，后者包括“诺曼·白求恩博士的精神”。无论真实与否，卫生政策仍然是中国各地新的民族认同词汇的核心组成部分。

1961年，随着国家粮仓库存的减少，粮食危机开始冲击北京和上海等大城市以及东北地区的工业中心。在前一年，为了响应宋平的求助，国务院批准向东北地区提供10亿公斤的粮食援助。但在1961年12月的国家计划会议上，周恩来总理

clear that there would be no more food aid from the state. Each province and region had to fulfil their state grain and cotton procurement quotas: 'If any region failed to fulfil the procurement quota, the local authorities must take the full responsibility,' the premier said adamantly. When Yang Yichen, the director of finance and commerce of Helongjiang province (the food-growing region of the northeast), responded with 'Tough!' Zhou Enlai turned around and asked: 'What does that mean? "Tough"? Why can't you just answer "Yes" or "No"?' When Yang explained to the premier that Helongjiang province was unable to 'climb up the hill' (meaning to fulfil the state procurement quota), Zhou insisted: '[Helongjiang] has to climb up the hill.'²⁹ In addition to the difficulties being faced in procuring the projected amount of agricultural goods, in 1961 coal and steel production throughout China had plummeted. If China failed to produce the projected steel output target of 18.6 million tons in the following year, it was thought that the whole world would 'spit on China'. 'This was a political matter,' Song Renqiong reminisced.³⁰ Since the northeast was the backbone of the PRC's heavy industry, Song was charged with the daunting task of seeing that the region was producing enough coal to power the entire country's increased industrialization. With the USSR having withdrawn its aid, being 'self-reliant' and 'walking on two legs' were more than just ideological statements; they were also necessary. The desperate situation reminded Song of the time when he led the Communist army's famine relief work in southern Hebei province in 1943 during the Sino-Japanese war. By uniting and relying on the local people, the Communist army successfully completed the task of transporting 325 million kilograms of grain across the enemy line to the famine-stricken villages. 'We could only do it with the help of villagers ... From our past experiences, we learn that by uniting with the people, we could conquer any difficulties. We must continue with such fine revolutionary traditions.'³¹ This may have been true in wartime with an external enemy, but the reality on the ground at this moment was very different.

In the early 1960s, however, with corruption running rampant in the countryside, very few villagers trusted their local cadres. To 'reunite the people to the party', Song understood that a propaganda campaign was needed to 'humanize' it in order to succeed. Thus, Lei Feng, the hero of the new China, was manufactured. Most important, Lei Feng did not represent top-down authority. The official version of his

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然而，在1960年代初期，由于乡村腐败猖獗，很少有村民信任当地干部。宋明白，为了“使人民重新团结在党周围”，需要一场宣传运动来“人性化”党，才能取得成功。因此，新中国的英雄雷锋应运而生。最重要的是，雷锋并不代表自上而下的权威。官方版本的雷锋

story tells us that he was an ordinary soldier, born into a poor peasant family in Hunan province. He was a fellow countryman of some of the top CCP leaders, including Chairman Mao, President Liu, and Commissary Song. Like them, he was of the people. More important, he was the 'successor of the revolution'. Being a new soldier and a new party member, he was said to have inherited the 'fine revolutionary traditions' of the CCP and its army, the PLA. He donated his savings to famine relief; he used his pocket money to buy a train ticket for a stranger who had lost hers; he helped an old lady crossing the road. Equally important, he was not an ordinary individual. When strangers he had helped asked for his name, he would always answer: 'I am a PLA soldier, and my home is China.' According to Song Renqiong, one of the people responsible for 'discovering' this hero, Lei Feng was the embodiment of new China: 'Lei Feng is the hero of our time ... he is the model of our time. He is the pride of new China'.³²

This hero of new China died at a timely moment on 15 August 1962 at the very young age of twenty-two, and, of course, he died while on duty, which made him a martyr. Until then, very few people in China had ever heard of him, even though we are told that he had become a local hero in Shenyang and had received numerous honours from the PLA Shenyang Military Region for his good deeds. A month later, after Chairman Mao reminded the party and the people of China 'Never to forget class struggle', the story of Lei Feng began to spread from Shenyang to Beijing. On 5 March 1963, the *People's Daily* published Mao's motto 'Learn from Comrade Lei Feng!' which officially launched the nationwide 'Learning from Lei Feng' campaign. To learn from Lei Feng, every young person in China, including schoolchildren, read and recited *Lei Feng's Diaries*. The success of the Lei Feng campaign was that this exemplary worker replaced the state-directed admonitions to increase production. By personifying the utopian goal in and through this perfect citizen of the new state, the authorities encouraged the individual's involvement in the goals of the state rather than in any type of mass action.

A month prior to the *People's Daily's* publication of Chairman Mao's 'Learn from Comrade Lei Feng', on 7 February 1963 the newspaper had published a co-authored feature article entitled 'Chairman Mao's Good Soldier Lei Feng', paired with an editorial 'A Great yet Ordinary Soldier'. The paper also ran an entire page of excerpts from Lei Feng's diaries as well as photographs of Lei Feng doing good deeds and reading Chairman Mao's works. Through

这个故事告诉我们，他是一位普通的士兵，出生于湖南省一个贫困的农民家庭。他是包括毛主席、刘主席和宋专员在内的一些中共最高领导人的同乡。像他们一样，他也是人民的一份子。更重要的是，他是“革命的接班人”。据说，作为一名新兵和一名新党员，他继承了中国共产党及其军队——中国人民解放军的“优良革命传统”。他将自己的积蓄捐给了赈灾；他用自己的零用钱为一位丢了火车票的陌生人买了一张票；他帮助一位老太太过马路。同样重要的是，他不是普通人。当他帮助过的陌生人问他的名字时，他总是回答：“我是一名解放军战士，我的家在中国。”据负责“发现”这位英雄的人之一宋任穷说，雷锋是新中国的化身：“雷锋是我们时代的英雄……他是我们时代的榜样。他是新中国的骄傲”。³²

这位新中国的英雄在1962年8月15日及时地去世了，年仅22岁，当然，他是因公殉职，这使他成为了一名烈士。在那之前，中国很少有人听说过他，尽管我们被告知他已成为沈阳的当地英雄，并因其善行而受到中国人民解放军沈阳军区颁发的无数荣誉。一个月后，毛主席提醒党和全国人民“永远不要忘记阶级斗争”，雷锋的故事开始从沈阳传到北京。1963年3月5日，《人民日报》发表了毛主席的题词“向雷锋同志学习！”，正式启动了全国性的“学习雷锋”运动。为了向雷锋学习，中国每一个年轻人，包括学童，都阅读并背诵《雷锋日记》。雷锋运动的成功之处在于，这位模范工人取代了国家指导的增加生产的训诫。通过在新国家的这个完美公民身上和通过他来实现乌托邦式的目标，当局鼓励个人参与国家的目标，而不是任何形式的群众运动。

在《人民日报》发表毛主席“学习雷锋同志”的指示前一个月，即1963年2月7日，该报发表了一篇合作署名的特写文章，题为《毛主席的好战士雷锋》，并配发了社论《一个伟大而平凡的战士》。该报还用一整版刊登了雷锋日记的摘录以及雷锋做好事和阅读毛主席著作的照片。通过

both the editorial and the diary excerpts, the reader also encountered yet another revolutionary hero, 'Dr Bethune': 'The embodiment of true spirit of internationalism and communism'. The editorial in the *People's Daily* explained to the reader that it was through reading Chairman Mao's essay 'In Memory of Norman Bethune' (1939) and by learning to emulate 'Comrade Bethune' that Lei Feng had become 'a great revolutionary soldier'. In the words of Lei Feng, the reader was reminded that 'To live is to be like Comrade Bethune, who devoted all his energy and gave his life to the revolutionary cause and the cause of building communism.'³³

As with Lei Feng, until then very few people in China had heard of Dr Norman Bethune. In November 1939, while serving as a medical volunteer with the CCP's Eighth Route Army, Norman Bethune had died of septicaemia after he accidentally lanced his finger while operating on a wounded soldier.³⁴ In other words, he died on duty as a revolutionary martyr. Twenty-three years later, Lei Feng also died as a revolutionary martyr while on duty. In December 1939, a month after Bethune's death, Mao apotheosized the dead doctor. He wrote the essay 'In Memory of Norman Bethune' and transformed Bethune into a living revolutionary symbol. It was Chairman Mao's memorial to Bethune, we are told, that inspired Lei Feng to devote his life to the revolutionary cause and communism. In his memorial, Mao told his readers that although he did not know Bethune well in person because they had only met once, he wanted every Chinese communist to learn from the 'Spirit of Comrade Bethune' – namely, 'the true spirit of internationalism and Communism'.

Shaped after Sergey Nechayev's eponymous 'Revolutionary' – the 'doomed man' – Mao's Comrade Bethune had no personal interests but was wholly absorbed by a single focus, his passion for revolution; he had neither friendships nor attachments but was devoted solely to his comrades – the collective – who, like him, were dedicated to the revolution: '[Bethune's] utter devotion to others without

* In 1930, Mao's young wife Yang Kaihui, who was a fellow revolutionary (a comrade) of Mao's, was imprisoned by the Nationalists and was subsequently executed. After Yang's death, their three sons were put into orphanages. They were found many years later and were reunited with their father. The oldest one, Anying, was killed in the Korean War in 1953. In 1957, in a poem, titled 'Butterfly Courting Flowers', in memory of Yang Kaihui and another revolutionary martyr, Liu Zhixu (Liu was their friend and comrade), Mao turned them both into the revolutionary symbol of 'righteous and loyal spirit' who had sacrificed their lives in the revolution.

无论是社论还是日记摘录,读者还会遇到另一位革命英雄“白求恩医生”:“国际主义和共产主义真正精神的化身”。《人民日报》的社论向读者解释说,雷锋正是通过阅读毛主席的文章《纪念白求恩》(1939年)以及学习效仿“白求恩同志”才成为“伟大的革命战士”的。用雷锋的话说,读者被提醒说:“活着就要像白求恩同志那样,把他全部的精力都贡献出来,把生命献给革命事业和共产主义事业。”³³

与雷锋一样,在那之前,中国很少有人听说过诺曼·白求恩医生。1939年11月,诺曼·白求恩作为一名医疗志愿者在中国共产党八路军服役时,在为一名受伤的士兵做手术时不慎刺破手指,死于败血症。³⁴换句话说,他作为一名革命烈士殉职了。二十三年后,雷锋也在执勤时作为革命烈士牺牲。1939年12月,白求恩去世一个月后,毛泽东将这位已故医生神化。他写了《纪念白求恩》一文,将白求恩变成了活生生的革命象征。据说,正是毛主席对白求恩的纪念,激励了雷锋将毕生精力奉献给革命事业和共产主义。毛泽东在纪念文章中告诉读者,虽然他本人并不十分了解白求恩,因为他们只见过一次面,但他希望每个中国共产党党员都能学习“白求恩同志的精神”——即“真正的国际主义和共产主义精神”。

毛泽东的白求恩同志是以谢尔盖·涅恰耶夫的同名“革命者”——“注定之人”为蓝本塑造的,他没有任何个人利益,完全沉浸于一个焦点,即他对革命的热情;他既没有友谊也没有感情,而是完全奉献给他的同志们——集体——他们像他一样,致力于革命:“[白求恩]对他人完全的奉献,没有

* 1930年,毛泽东的年轻妻子杨开慧是毛泽东的革命同志,被国民党监禁并被处决。杨开慧死后,他们的三个儿子被送进孤儿院。多年后他们被找到,并与他们的父亲团聚。他们的大儿子岸英在1953年的朝鲜战争中丧生。1957年,在一首题为《蝶恋花》的诗中,为了纪念杨开慧和另一位革命烈士柳直荀(柳是他们的朋友和同志),毛泽东将他们都变成了“忠魂”的革命象征,他们为革命献出了生命。

any thought of self was shown in his great sense of responsibility in his work and his great warm-heartedness toward all comrades and the people.' Only with this spirit of absolute selflessness, proclaimed Mao, can a man be of value to the people. Mao's Dr Bethune thus stood as the antithesis of those false or faltering communists

who are irresponsible in their work, preferring the light and shirking the heavy, passing the burdensome tasks on to others and choosing the easy ones for themselves. At every turn they think of themselves before others. When they make some small contribution, they swell with pride and brag about it for fear that others will not know. They feel no warmth toward comrades and the people but are cold, indifferent, and apathetic. In truth such people are not Communists, or at least cannot be counted as devoted Communists.

Toward the end of the memorial, Mao also wrote:

Comrade Bethune was a doctor, the art of healing was his profession and he was constantly perfecting his skill, which stood very high in the Eighth Route Army's medical service. His example is an excellent lesson for those people who wish to change their work the moment they see something different and for those who despise technical work as of no consequence or as promising no future.³⁵

In 1939, Mao was certainly not writing about how to be a good doctor or commenting on medical ethics. His argument was that only a 'true' 'revolutionary' and 'communist' like Bethune could attain the fine skills needed to excel in any profession. Equally important was that as a person Norman Bethune, in analogy to Jesus Christ, was a healer. But in death, his 'spirit of true communism and internationalism' was immortalized by Mao. For Mao, only communism, not Christianity or any other religion, could bring the ultimate healing to his country, 'the Sick Man of Asia', and through the 'spirit of internationalism', communist revolution would bring about the salvation of all of mankind.*

* Norman Bethune was born a Christian. In 1935 he travelled to the USSR where he became a convinced communist, and upon returning to Canada he joined the Canadian Communist Party. He was an advocate of socialized medicine.

任何自私的想法都体现在他对工作的强烈责任感以及对所有同志和人民的热爱之中。'毛泽东宣称,只有具备这种绝对无私的精神,一个人才能对人民有价值。因此,毛泽东的白求恩医生是那些虚伪或动摇的共产党员的反面。

他们在工作中不负责任,喜欢轻松的工作而逃避繁重的工作,把繁重的任务推给别人,而自己选择轻松的工作。他们总是先想到自己。当他们做出一些小贡献时,他们就会沾沾自喜,并吹嘘自己,生怕别人不知道。他们对同志和人民没有热情,而是冷漠、麻木和漠不关心。事实上,这些人不是共产党员,或者至少不能算作忠诚的共产党员。

在悼念的结尾,毛泽东还写道:

白求恩同志是一位医生,医术是他的职业,他不断完善自己的医术,这在八路军的医疗服务中非常突出。他的榜样对于那些一看到不同的东西就想改变工作的人,以及那些鄙视技术工作没有意义或没有前途的人来说,是一个极好的教训。³⁵

1939年,毛当然不是在写如何成为一名好医生,也不是在评论医学伦理。他的论点是,只有像白求恩这样“真正”“革命”和“共产主义者”的人,才能获得在任何职业中脱颖而出所需的精湛技能。同样重要的是,诺曼·白求恩这个人,类似于耶稣基督,是一位治疗者。但在他去世后,他的“真正的共产主义和国际主义精神”被毛泽东永垂不朽。对毛泽东来说,只有共产主义,而不是基督教或任何其他宗教,才能给他的国家——“东亚病夫”——带来最终的治愈,并且通过“国际主义精神”,共产主义革命将带来全人类的救赎。*

* 诺曼·白求恩出生于基督教家庭。1935年,他前往苏联,在那里他成为一名坚定的共产主义者,回到加拿大后,他加入了加拿大共产党。他是社会化医疗的倡导者。

Transformed into a revolutionary symbol, Norman Bethune would be evoked differently at different times by different people. Thirteen years after Mao wrote his memorial essay, at the anniversary commemoration of Bethune's death, Dr Nelson Fu (Fu Lianzhang in Chinese), the president of the Chinese Medical Association (CMA) as well as the deputy minister of health, delivered a lecture titled 'What We Should Learn from Dr Bethune's Revolutionary Humanitarianism' (1952). Like Bethune, Fu was born into a Christian family. After graduating from a missionary medical school, he worked as the doctor for a local branch of the Red Cross as well as the director of a local Christian Gospel hospital. Fu later joined the Communist revolution and was instrumental in building a military health system for the Communist army during the Sino-Japanese and the Chinese civil wars. Fu was not a revolutionary like Mao; he was a communist sympathizer and a humanitarian who participated in the revolution in order to save individual lives. Fu only became a party member in 1938. In that same year, he worked alongside another new convert to communism, none other than the Canadian physician Norman Bethune, a veteran of the International Brigade in Spain, at the Eighth Route Army hospital. In his 1952 lecture, Fu recalled how impressed he was by Bethune's devotion to medical work and his 'unravelling skill, boundless sympathy for his patients, sense of responsibility and zeal in teaching others'.³⁶ In 1952, as the frontrunner of the PRC's medical and public health work, Fu was trying to provide a meaning and purpose for medical and health work in the new China. Dr Bethune, the revolutionary symbol first evoked by Mao, became his 'sterling example'. With his background in Christian medical work, Fu focused his exegesis of Chairman Mao's memorial on the 'spirit of selflessness', one of the fundamentals of being a Christian according to Christ's 'Sermon on the Mount'. Having been baptized a second time as a communist, Fu rebranded this old Christian teaching as 'revolutionary humanitarianism'.³⁷ Fu's Dr Bethune was a humanitarian like himself who joined the revolution to save the lives of individuals. This was markedly different from Mao's Norman Bethune as the revolutionary and the collectivist anti-humanist.*

* Like many other supporters of the Communist revolution, Fu was purged during the Great Proletarian Cultural Revolution. As a humanist, he pleaded with Mao, asking Mao to save his life on the grounds that he had once saved Mao's life. But Mao did not intervene; Mao was a revolutionary who had denounced the 'I' for the 'we'.

诺尔曼·白求恩被塑造成一个革命的象征，不同的人在不同的时期会以不同的方式来唤起人们对他的记忆。在毛泽东撰写纪念文章十三年后，在白求恩逝世周年纪念会上，中国医学会（CMA）主席兼卫生部副部长傅连璋博士发表了题为《我们应该学习白求恩博士的革命人道主义精神》（1952年）的演讲。与白求恩一样，傅连璋也出生于一个基督教家庭。从教会医学院毕业后，他曾担任当地红十字会的分会医生以及当地基督教福音医院的院长。后来，傅连璋加入了共产主义革命，并在抗日战争和中国内战期间为共产党军队建立军事卫生系统发挥了重要作用。傅连璋不像毛泽东那样是一位革命家；他是一位共产主义同情者和人道主义者，他为了拯救个人的生命而参与了革命。傅连璋直到1938年才成为一名党员。同年，他与另一位新加入共产主义的人并肩工作，此人不是别人，正是加拿大医生诺尔曼·白求恩，一位参加过西班牙国际纵队的退伍军人，当时他们在八路军医院工作。在1952年的演讲中，傅连璋回忆起他对白求恩的医疗工作奉献精神以及他的“精湛技艺、对病人无限的同情心、责任感和教导他人的热情”印象深刻。³⁶ 1952年，作为中国医疗卫生工作的领头人，傅连璋试图为新中国的医疗卫生工作提供意义和目标。毛泽东首次提出的革命象征白求恩博士成为了他的“杰出典范”。凭借他在基督教医疗工作方面的背景，傅连璋将他对毛主席纪念文章的解读重点放在“无私奉献的精神”上，这是根据基督的“登山宝训”成为基督徒的根本要素之一。在作为一名共产党员接受第二次洗礼后，傅连璋将这种古老的基督教教义重新包装为“革命人道主义”。³⁷ 傅连璋笔下的白求恩博士是一位像他自己一样的人道主义者，他加入了革命以拯救个人的生命。这与毛泽东笔下的诺尔曼·白求恩作为革命者和集体主义反人道主义者截然不同。*

* 像许多其他支持共产主义革命的人一样，傅在文化大革命期间遭到清洗。作为一个有人道主义思想的人，他恳求毛泽东，要求毛泽东看在他曾经救过毛泽东一命的份上饶他一命。但毛泽东没有干预；毛泽东是一位革命者，他谴责“我”而支持“我们”。

'REVOLUTIONIZE' HOSPITAL SERVICES

Twenty-five years later, in the post-famine Socialist Education Campaign, humanitarian values, such as individuals volunteering to do medical and public health work, would nevertheless be utilized as a tool to reunite the party with the people. Aggravated by the famine crisis, the PRC's already flimsy health system had collapsed. Most people in the countryside had very little or no access to even the most basic health care, whilst in cities and factories the medical treatment provided by hospitals was a far cry from the goal propagated by the party. Poor-quality hospital care was one of the greatest popular complaints. In addition, corruption in hospitals and government health departments was rampant, creating a gulf between the haves and the have-nots and causing further public resentment. The Ministry of Health, held accountable for the low standards of care as well as the widespread corruption in medical and health care establishments, was obliged to respond and to introduce measures to combat corruption and improve services. Even in government departments, the emphasis of the campaign was on anti-corruption. At the national conference on hospital governance in January 1964, Qian Xinzong, the deputy minister of health, urged the attendees to carry forward the 'spirit of revolutionary humanitarianism': '[during the war years] we won a lot of praise from the people ... they used to call military medical workers "the Old Eighth Route" and our military hospital "the hospital that saves lives" ... Today we need to forward our fine revolutionary traditions by promoting good governance and improve services as well as to train health cadres [who would be equipped for such a task].'³⁸ 'Revolutionary humanitarianism', inspired by Norman Bethune, became the new catchword for a focus on individual-based health reform as opposed to the state-focused institutions that were seen by the public as not only corrupt but also incompetent.

In the following months, to reform hospitals into 'truly socialist hospitals', armies of urban medical and health workers throughout the country were sent to the countryside to support the 'Three Revolutionary' campaigns – namely, 'Class Struggle', 'Struggle for Production', and 'Scientific Experiment' – as well as deliver medical care to peasants in the fields, workers in the mines, and the sick in their homes.³⁹ To ensure that they would 'rectify' their 'attitudes' toward services so that they would be able to solve the question

“彻底变革” 医院服务

25年后,在后饥荒时期的社会主义教育运动中,人道主义价值观,例如个人自愿从事医疗和公共卫生工作,仍被用作党与人民重新团结的工具。由于饥荒危机的加剧,中华人民共和国本就脆弱的医疗体系已经崩溃。农村地区的大多数人几乎无法获得最基本的医疗保健,而在城市和工厂,医院提供的医疗服务与党宣传的目标相去甚远。劣质的医院护理是民众最大的抱怨之一。此外,医院和政府卫生部门的腐败现象十分猖獗,造成了贫富差距,进一步加剧了公众的不满。卫生部对低标准的护理以及医疗卫生机构中普遍存在的腐败现象负责,因此有义务做出回应并采取措施打击腐败并改善服务。即使在政府部门中,这场运动的重点也是反腐败。在1964年1月举行的全国医院管理会议上,卫生部副部长钱信忠敦促与会者发扬“革命人道主义精神”:“[在战争年代]我们赢得了人民的许多赞扬……他们过去称军队医务人员为“老八路”,称我们的军队医院为“救死扶伤的医院”……今天,我们需要通过促进善治和改善服务,以及培训[能够胜任这项任务]的卫生干部来发扬我们优良的革命传统。”³⁸ 受诺曼·白求恩启发,“革命人道主义”成为关注以个人为基础的医疗改革的新口号,而不是公众认为不仅腐败而且无能的以国家为中心的机构。

在接下来的几个月里,为了将医院改革成“真正的社会主义医院”,全国各地的城市医疗卫生工作者大军被派往农村,以支持“三大革命”运动——即“阶级斗争”、“生产斗争”和“科学实验”——并向田间的农民、矿山的工人和家中的病人提供医疗服务。³⁹ 为了确保他们能够“纠正”他们对服务的“态度”,从而能够解决问题

of 'service for whom' and 'how to serve', medical and health professionals read Chairman Mao's works, including 'In Memory of Norman Bethune'. Many, we are told, were inspired by the 'spirit of revolutionary humanitarianism' and had become 'one with [the people] in thought and feeling' as well as 'remoulded their ideology'. In addition to 'tirelessly' delivering medical and health care to the workers and peasants, they also 'cooked food and boiled water for [the people]' as well as helping with agricultural work.⁴⁰

A few outstanding examples were named and honoured as 'Five Good' health/medical workers so that others would emulate them. The story of a young surgeon, Li Gong from Gansu province, who used skin grafts from his own legs and thighs to treat the burns on a Tibetan villager's body and donated his blood to save the lives of ten Tibetan cholera patients, was adapted into a narrative poem and two operas with songs in the Tibetan genre. Tibetan villagers would remember him as the 'Red Doctor', the 'Manba (Tibetan Doctor) sent by Chairman Mao', while medical and health workers throughout China would aspire to become like the 'Red Doctor' sent by Chairman Mao and the party. Labelled as another martyr who died while on duty, Li Gong was the Ministry of Health's own Lei Feng and the new China's Dr Bethune. Interestingly, in 2011, to celebrate the sixtieth anniversary of the CCP's 'Liberation' of Tibet, Li Gong's story was re-invoked in a Peking opera titled *The Manba of Grassland* to demonstrate the 'deep affection' between the Communist Party and the Tibetan people. It also served as an exemplary instance of fine moral professional conduct for the new generation of medical and health professionals in China to imitate.⁴¹

In 1964, in order to build a truly socialist hospital and health service that would 'contribute to the country's socialist revolution and construction' by protecting the health of the 'broad masses of working people', 25,000 new medical graduates, according to the Ministry of Health's statistics, were 'channelled into county hospitals and mining enterprises'.⁴² The so-called 'minority regions', including Tibet, were given priority, partly because medical humanitarian work had proved a successful strategy in winning the support of local populations in the early 1950s. By the end of 1964, according to official statistics, medical personnel in these regions had increased by 30 per cent, and hospital beds had increased by 20 per cent compared to 1949.⁴³ The shift from an expert-centred, top-down health care to a focus on health care disseminated across the widest reaches

为了解决“为谁服务”和“如何服务”的问题，医疗卫生专业人员阅读了毛主席的著作，包括《纪念白求恩》。据说，许多人受到了“革命人道主义精神”的鼓舞，并在思想和感情上与[人民]“融为一体”，并“改造了他们的思想”。除了“不知疲倦地”为工人和农民提供医疗保健服务外，他们还“为[人民]做饭和烧水”，并帮助从事农业劳动。⁴⁰

一些杰出的例子被命名为“五好”卫生/医务工作者并受到表彰，以便其他人能够效仿他们。来自甘肃省的年轻外科医生李宫的故事，他用自己腿部和大腿的皮肤移植来治疗一位藏族村民身上的烧伤，并献血挽救了十名藏族霍乱患者的生命，被改编成叙事诗和两部藏族风格的歌剧。藏族村民会记住他是“红医生”、“毛主席派来的曼巴（藏族医生）”，而中国各地的医务工作者都会渴望成为像毛主席和党派来的“红医生”一样的人。李宫被贴上又一位因公殉职的烈士的标签，是卫生部自己的雷锋和新中国的白求恩。有趣的是，2011年，为了庆祝中国共产党“解放”西藏六十周年，李宫的故事在京剧中再次被提及，剧名是草原上的曼巴，以展示共产党和藏族人民之间的“深厚感情”。它也成为了中国新一代医疗卫生专业人员效仿的良好道德职业行为的典范。⁴¹

1964年，为了建设一个真正社会主义的医院和卫生服务体系，通过保护“广大劳动人民”的健康，为“国家的社会主义革命和建设做出贡献”，根据卫生部的统计，有25000名新毕业的医学生“被输送到县医院和矿业企业”。⁴²所谓的“少数民族地区”，包括西藏，被给予优先考虑，部分原因是医疗人道主义工作已被证明是赢得20世纪50年代初当地居民支持的成功策略。到1964年底，根据官方统计，这些地区的医疗人员增加了30%，与1949年相比，医院床位增加了20%。⁴³这种转变是从以专家为中心、自上而下的医疗保健转变为关注在最广泛范围内传播的医疗保健。

of the state through the activities of individuals had begun in earnest. Ironically, in northern Shaanxi, however, as a result of this intense campaign, it had helped to 'immortalize' the 'spirit of Norman Bethune' but in a very unintended manner. As Adam Chau learned from his local informants, in the mid-1960s villagers there turned Norman Bethune into a local deity or 'the god Bethune' who travelled around northern Shaanxi to cure people's illnesses. Since there was no temple or shrine dedicated to 'Dr Bethune', villagers uttered Bethune's name in prayer and lit what was thought to be Bethune's favourite cigarettes as incense to obtain his divine assistance.⁴⁴

'THE THIRD FRONT': CHINA PREPARING FOR A WAR

In 1964, the CCP Central Committee decided to embark on the 'Third Front' construction project in order to build a comprehensive system of defence, industry, energy, and transportation in the mountainous regions of the southwest in preparation for the possibility of an American invasion in the wake of the Vietnam War as well as an attack from Moscow. It became imperative that there be a concomitant effort to construct an adequate health infrastructure in these remote regions.⁴⁵ With schistosomiasis identified as one of the major health risks for the military and civilian populations in wide swatches of the 'Third Front' area along the upper stretches of the Yangtze River administered by Sichuan and Yunnan provincial authorities, the Sichuan provincial health authority allocated 120,000 *yuan* to build a brand new schistosomiasis treatment hospital with fifty hospital beds in Xichang City.⁴⁶ This was a huge commitment, considering Sichuan province's health budget had been in deficit over the previous three years.⁴⁷

On the south side of Xichang is Panzhihua, the ancient Cycas forest at the intersection of the Jinsha and Yalu rivers, once the home of giant pandas. It was chosen as the location for a gigantic steel and iron mill complex because it was rich in natural mineral resources, in particular iron ore. Panzhihua's drawback was that it was bleak and desolated. The existing inhabitants were said to be rather 'uncivilized', since most of them were illiterate. It was also

* The 'Third Front' area was centred in Sichuan province; it also stretched into parts of Yunnan, Guizhou, Hubei, and Shaanxi.

† Xichang was to become the PRC's satellite launching base.

通过个人活动来实现国家目标的热潮已经真正开始了。具有讽刺意味的是，在陕北，这场运动却以一种意想不到的方式帮助“永垂不朽”了“诺尔曼·白求恩的精神”。正如周亚当从当地线人那里了解到的，在20世纪60年代中期，那里的村民将诺尔曼·白求恩变成了一个当地的神，或者说是“白求恩神”，他在陕北各地巡游，治愈人们的疾病。由于没有专门为“白求恩医生”设立的寺庙或神龛，村民们在祈祷时念诵白求恩的名字，并点燃他们认为是白求恩最喜欢的香烟作为香，以获得他的神圣帮助。⁴⁴

“三线”：中国备战

1964年，中共中央决定启动“三线”建设工程，以便在西南山区建设一个综合性的国防、工业、能源和交通运输体系，为应对越南战争后美国入侵以及莫斯科袭击的可能性做准备。因此，在这些偏远地区建设完善的卫生基础设施势在必行。⁴⁵ 血吸虫病被认为是“三线”地区长江上游沿岸（由四川和云南省当局管辖）军事和民用人口的主要健康风险之一，四川省卫生厅拨款12万元在西昌市新建一家拥有50张病床的血吸虫病治疗医院。⁴⁶ 考虑到四川省的卫生预算在过去三年里一直处于赤字状态，这是一项巨大的投入。⁴⁷

在西昌的南面是攀枝花，那里有金沙江和雅砻江交汇处的古老苏铁林，曾经是大熊猫的家园。之所以选择这里作为大型钢铁厂的所在地，是因为它拥有丰富的自然矿产资源，特别是铁矿石。攀枝花的缺点是荒凉。据说，现有的居民相当“不文明”，因为他们大多数都是文盲。而且，

* “三线”地区以四川省为中心；它还延伸到云南、贵州、湖北和陕西的部分地区。

† 西昌将成为中国（prc）的卫星发射基地。

a region of endemic leprosy. There were rumours that even hens and chicks became infected with the disease after encountering lepers and that the chickens then passed the disease to other humans through the food chain. To eliminate such fears, the Ministry of Health was impelled to issue a statement affirming that leprosy was under control in the PRC and that the future workforce would not be at risk from the disease.⁴⁸ In addition to schistosomiasis and leprosy, researchers also identified large sections of the subtropical 'Third Front' industrial development zone as the natural *foci* of a number of infectious diseases, including Japanese B encephalitis, leptospirosis, orientia tsutsugamushi (also known as scrub typhus), and plague – thus, a hostile environment for human habitation.⁴⁹

Although no major outbreaks of these diseases were reported, official records showed that the risk of morbidity from meningitis, malaria, gastritis, and anthrax did increase dramatically. In 1965, the meningitis morbidity in the border regions between Sichuan, Yunnan, Guizhou, and Hubei rose nearly 30 per cent compared to the previous year. The main cause of such a sharp rise, as suggested by the official analysis, was simply that greater numbers of workers had been brought into these regions to work on this colossal national defence project.⁵⁰ The crowded and unsanitary living conditions turned the 'Third Front' industrial development zone (including road and railway construction sites, mines, factories, steel mills, and power plants) into a hothouse for the spread of a wide range of infectious diseases. Malaria, gastritis, and influenzas, in particular, plagued such places. In Sichuan province, the centre of the 'Third Front' project, government health reports consistently showed that many railway construction sites became covered with puddles of filthy water when it rained, while piles of rubbish were to be seen everywhere. Workers often had to share cups, wash bowls, and towels. There was no hot water for showers and limited amounts of clean and boiled water to drink. As a result, one tenth of the workforce fell ill across all factories and construction sites in the development zone. At some construction sites, the sickness rate was as high as one-third, and in addition, many workers were infested with fleas and lice.⁵¹ At the Yingxiu hydraulic dam construction site in western Sichuan, bordering Tibet, there were only six toilets for 6,000 employees and workers.* With no space to store night-soil or any

* Yingxiu was the epicentre of the 2008 Sichuan earthquake.

一个地方性麻风病流行的地区。有传言说，即使是母鸡和小鸡在接触到麻风病人后也会感染这种疾病，然后鸡通过食物链将疾病传染给其他人。为了消除这种担忧，卫生部不得不发表声明，申明麻风病在中国已得到控制，未来的劳动力不会受到这种疾病的威胁。⁴⁸除了血吸虫病和麻风病，研究人员还发现，亚热带“三线”工业开发区的很大一部分地区是许多传染病的自然疫源地，包括日本乙型脑炎、钩端螺旋体病、恙虫病（也称为丛林斑疹伤寒）和鼠疫——因此，对人类居住来说是充满敌意的环境。⁴⁹

虽然没有报告这些疾病的大规模爆发，但官方记录显示，脑膜炎、疟疾、胃炎和炭疽的发病风险确实急剧增加。1965年，四川、云南、贵州和湖北交界地区的脑膜炎发病率比上一年上升了近30%。官方分析表明，这种急剧上升的主要原因是大量工人被带到这些地区从事这项巨大的国防工程。⁵⁰拥挤和不卫生的生活条件使

“三线”工业开发区（包括道路和铁路建设工地、矿山、工厂、钢铁厂和发电厂）成为各种传染病传播的温床。特别是疟疾、胃炎和流感困扰着这些地方。在“三线”项目的中心四川省，政府卫生报告始终表明，许多铁路建设工地在下雨时变成了肮脏的积水坑，到处可见成堆的垃圾。工人们经常不得不共用杯子、洗碗和毛巾。没有热水淋浴，饮用的清洁和开水也有限。结果，十分之一的劳动力在开发区的所有工厂和建筑工地都生病了。在一些建筑工地，患病率高达三分之一，此外，许多工人身上都长满了跳蚤和虱子。⁵¹在与西藏接壤的四川西部映秀水利大坝建筑工地，6000名雇员和工人只有六个厕所。* 没有地方储存粪便或任何

* 映秀是2008年四川地震的震中。

transportation to remove it from the construction site, it was simply dumped into the Ming River, a major tributary of the Yangtze. In the meantime, swarms of flies invaded the factory and construction site canteens.⁵² Mass political meetings in the countryside, as well as public entertainment for employees of the factories and mines, also helped to spread disease.⁵³

Despite the state's claims that the health care provision in these 'backward' regions had improved dramatically since the Communist Liberation in the early 1950s, there were not nearly enough hospitals, let alone sufficient physicians. The problem was exacerbated by the Central Committee's directive to drastically cut back the number of state employees and to reduce food rations because of the severe food shortages in the aftermath of the famine. Because health was seen as a 'wasteful' rather than a 'productive' sector of the economy, a huge number of county- and commune-level hospitals throughout the country were closed down. In many remote hinterland regions, such as the southwest, with fewer resources and no proper roads, many smaller counties and towns as well as communes had no hospitals at all. In eastern Sichuan's Fengjie County, along the bank of the Yangtze next to what is today the Three Gorges Dam, there was only one medical worker per 2,000 people. Next to Fengjie County is Wushan County, where nearly two-thirds of the medical workers were dismissed. Although on paper there were fifty-six union clinics in Wushan County, in reality many of them had no medical staff.⁵⁴ For the whole of Sichuan province, official statistics showed there were on average only 6.1 hospital beds for every 10,000 people in 1964. This was more than one-third lower than the national average of ten beds per 10,000 people. The situation was even more desperate for rural Sichuan where there was an average of only 3.7 beds per 10,000 people – again, one-third lower than the national average of 5.2 beds per 10,000 rural residents. Although nearly all railway and road construction sites, power plants, factories, and mines were supplied with doctors and other health personnel – and arms factories, as well as steel mills, often came with attached hospitals – in every case, the demand far exceeded the number of doctors and beds. For the 'Third Front' industrial development zone, there were on average only 1.46 health workers (including doctors and nurses) for every 1,000 employees and their families.⁵⁵

Wu Junmin, the novelist, was born in the 1960s when the 'Third Front' project first began. Both her parents were drafted to work

没有用交通工具将其从建筑工地运走，而是直接倾倒入长江的主要支流岷江中。与此同时，成群的苍蝇侵入了工厂和建筑工地的食堂。⁵² 农村的大型政治集会，以及为工厂和矿山职工提供的公共娱乐活动，也有助于疾病的传播。⁵³

尽管国家声称，自 20 世纪 50 年代初共产主义解放以来，这些“落后”地区的医疗保健服务已得到显著改善，但医院数量远远不够，更不用说足够的医生了。由于中央委员会指示大幅削减国家雇员人数，并因饥荒后严重的粮食短缺而减少口粮，问题进一步恶化。由于卫生被视为经济中“浪费性”而非“生产性”的部门，全国各地大量县级和公社级医院被关闭。在许多偏远的内地地区，如西南地区，资源匮乏，没有合适的道路，许多较小的县镇以及公社根本没有医院。在四川东部的奉节县，沿长江岸边，紧邻今天的长江三峡大坝，每 2000 人中只有一名医务人员。紧邻奉节县的是巫山县，那里近三分之二的医务人员被解雇。虽然名义上巫山县有 56 个工会诊所，但实际上许多诊所都没有医务人员。⁵⁴ 四川省的官方统计数据显示，1964 年平均每万人只有 6.1 张病床。这比全国平均每万人 10 张床位低三分之一以上。四川农村的情况更加糟糕，平均每万人只有 3.7 张床位——同样，比全国平均每万农村居民 5.2 张床位低三分之一。虽然几乎所有铁路和公路建设工地、发电厂、工厂和矿山都配备了医生和其他卫生人员——军工厂以及钢铁厂通常都附设医院——但在每一种情况下，需求都远远超过了医生和床位的数量。对于“三线”工业开发区，平均每 1000 名员工及其家属只有 1.46 名卫生工作者（包括医生和护士）。

⁵⁵

小说家吴俊敏出生于 20 世纪 60 年代，“三线”工程最初开始时。她的父母都被征调去工作

on the project. She recalled that many factories had only one health clinic with limited amounts of drugs and very few doctors. If a person fell seriously ill, they had to be sent to the nearest provincial-level hospital, which was often a long way away.⁵⁶ Furthermore, because of the shortage of medical staff and hospital beds, workers who relocated or were recruited to work at the 'Third Front' industrial development zone consistently competed with the local population for what were already scarce local health resources. For example, in Dechang County (locally known as *dep cha xiep*), the ancient trading centre between Panzhihua and Xichang that linked the Himalayas with the Chinese empire, the county hospital had forty beds, sixty beds fewer than the actual demand; the average number of outpatients had been 100. With the 'Third Front' project, the number of outpatients tripled. Unable to cope with the huge increase, the hospital sent more serious cases – those who needed X-rays or specialized care, for instance – to the Xichang regional hospital. The Xichang regional hospital quickly ran out beds, drugs, and equipment, and as a result, more than 200 patients there were denied medical care in June 1964.⁵⁷ Health care, while remaining a staple of state propaganda showing the efficacy of the new state system, became ever less important on the ground during and in the aftermath of the famine. The gap between the lived experience of those needing health care, figures that mushroomed during and after the famine, and the claims of the state that it was providing health care became ever more evident.

The shortage of doctors, beds, drugs, and equipment meant that only cadres and engineers or their friends had access to them. Since many of the temporary construction workers were local villagers or from elsewhere in the country, as soon as they left construction work, often due to ill health or injury, they ended up with no access to any medical or health care. Still, they counted themselves lucky if they came out alive, since pneumoconiosis and industrial accidents, two major health risks during the Great Leap Forward period, were made worse with the 'Third Front' project and claimed tens of thousands of lives. In many arms and chemical factories, one accident would often kill hundreds of people. The high level of coal dust in the atmosphere meant that pneumoconiosis casualties were not limited to factory employees or mine workers. Inhabitants of the nearby villages, particularly schoolchildren, were also at risk from the disease.⁵⁸ One other major re-occurring public health menace to

关于这个项目。她回忆说，许多工厂只有一个医疗诊所，药品数量有限，医生也很少。如果有人病得很重，他们必须被送到最近的省级医院，这通常是很远的路。⁵⁶此外，由于医务人员和医院病床的短缺，搬迁或被招募到“三线”工业开发区工作的工人，一直在与当地居民争夺本已稀缺的当地卫生资源。例如，在德昌县（当地称为 *dep cha xiep*），这个连接攀枝花和西昌的古代贸易中心，连接着喜马拉雅山脉和中华帝国，县医院有四十张病床，比实际需求少六十张病床；平均门诊病人人数为 100 人。随着“三线”项目的开展，门诊病人人数增加了两倍。由于无法应付这一巨大的增长，医院将更严重的病例——例如那些需要 X 光或专门护理的病例——送到西昌地区医院。西昌地区医院很快就耗尽了床位、药品和设备，结果，1964 年 6 月有 200 多名患者在那里被拒绝医疗。⁵⁷ 卫生保健，虽然仍然是国家宣传新国家制度有效性的主要内容，但在饥荒期间和之后，在实地却变得越来越不重要。那些需要医疗保健的人的实际经历（在饥荒期间和之后，这个数字急剧增加）与国家声称提供医疗保健之间的差距变得越来越明显。

医生、床位、药品和设备的短缺意味着只有干部、工程师或他们的朋友才能获得这些资源。由于许多临时建筑工人是当地村民或来自全国各地，一旦他们离开建筑工作，通常是由于健康不佳或受伤，他们最终无法获得任何医疗保健。尽管如此，如果他们能活着出来，他们就觉得自己很幸运，因为在“大跃进”时期，尘肺病和工业事故是两大健康风险，而“三线”项目使情况变得更糟，夺走了成千上万人的生命。在许多军工和化工厂，一次事故往往会造成数百人死亡。大气中高浓度的煤尘意味着尘肺病的伤亡不仅仅局限于工厂员工或矿工。附近村庄的居民，特别是学童，也面临着患这种疾病的风险。⁵⁸ 另一个反复出现的重大公共卫生威胁是

which all were exposed was the regular discharging of waste water by many of the factories into nearby rivers, releasing toxic gas into the environment that was harmful to the health of the local population as well as to food crops. Infectious diseases that first began at construction sites, mines, and in the new industrial towns and cities would often spread to nearby villages and towns through contaminated waterways or to the villages when the construction or mine workers returned home at the end of their jobs or during holidays (similar to the spread of the SARS epidemic in 2003). All of this added further stressors to the already inadequate health provision in the countryside.⁵⁹

As poor health undermined productivity, and in a desperate attempt to battle the ever more evident public health emergencies, the relevant authorities were advised to organize mass clean-ups or other public health activities. In the Luzhou Gas Field in southeastern Sichuan, bordering Guizhou and Yunnan, employees and their families resisted such activities, seeing it as a waste of time and effort because employees' children regularly defecated on the newly cleaned ground. 'What is the point?' they complained. Others protested that their accommodation was overcrowded and poorly built (some lived in makeshift straw huts) and were thus impossible to keep clean. Instead of introducing measures to improve the quality of life, the authorities handed out brand new, coloured public health posters to 'brighten up' the workers' otherwise shabby accommodations. They also organized mass study meetings for workers and their families to read the diaries of Lei Feng and Chairman Mao's essay 'In Memory of Norman Bethune', replacing open-air cinema or opera – the most common forms of mass entertainment for factory workers. After these study sessions, workers and their families would clean up the communal kitchen as well as visit nearby villages to help villagers repair the pig barns. Anyone who showed any reluctance would run the risk of experiencing the 'class struggle' first-hand.⁶⁰

In the context of the Socialist Education Campaign, public health activities that centred on prevention served the double function of attempting to mitigate health risks and mobilizing the masses in the struggle against the revisionists who doubted the efficacy of socialist health care. With prevention now re-elevated as the core strategy to protect people's health, as well as a political tool in the struggle against revisionism, preventive services expanded at a far greater pace than hospital services – in mining and industrial areas as well as

所有暴露出来的问题是，许多工厂经常将废水排放到附近的河流中，向环境中释放有毒气体，这些气体对当地居民的健康以及粮食作物有害。最初在建筑工地、矿山以及新的工业城镇和城市爆发的传染病，通常会通过受污染的水道传播到附近的村庄和城镇，或者在建筑或矿工完成工作或放假回家时传播到村庄（类似于 2003 年非典疫情的传播）。所有这些都进一步加剧了农村地区本已不足的医疗保健的压力。⁵⁹

由于糟糕的健康状况损害了生产力，为了孤注一掷地应对日益明显的公共卫生紧急事件，有关部门被建议组织大规模的清理或其他公共卫生活动。在四川省东南部、与贵州和云南接壤的泸州气田，员工及其家人抵制这些活动，认为这浪费时间和精力，因为员工的孩子经常在新清理的地面上排便。他们抱怨说：“这有什么意义？”其他人抗议说，他们的住宿拥挤且建造得很差（有些人住在临时草棚里），因此不可能保持清洁。当局没有采取措施来改善生活质量，而是分发了全新的彩色公共卫生海报，以“美化”工人们原本破旧的住所。他们还为工人和他们的家人组织了大规模的学习会议，阅读雷锋日记和毛主席的文章《纪念白求恩》，取代了露天电影或歌剧——工厂工人最常见的群众娱乐形式。在这些学习会议之后，工人和他们的家人会清理公共厨房，并访问附近的村庄，帮助村民修理猪圈。任何表现出任何不情愿的人都有可能亲身经历“阶级斗争”。⁶⁰

在社会主义教育运动的背景下，以预防为中心的公共卫生活动具有双重功能：一方面试图降低健康风险，另一方面动员群众与那些怀疑社会主义医疗保健有效性的修正主义者作斗争。随着预防重新被提升为保护人民健康的核心战略，以及反对修正主义的政治工具，预防性服务的发展速度远远超过了医院服务，无论是在矿区、工业区，还是其他地方。

in the countryside. Furthermore, as China prepared for war, mobile medical units, rather than big urban hospitals, were seen not only as cost-effective but also as strategically important. At the same time, the struggle against schistosomiasis continued to be seen as a factor in increasing military preparedness and efficacy and became one of the models for this more expansive model of health care.

Mao, who had had extensive experience in fighting guerrilla warfare during the revolution, often reminisced about the 'good old' military tradition of the revolutionary army engaging with the masses to fight disease as well as their enemies. In June 1964, in a conversation with a team of Vietnamese guerrilla fighters, styled as a true revolutionary leader as opposed to the Soviet revisionists, he remarked that health care in the PRC had been modelled after the Soviet system and had 'too many specialized doctors who can't treat a range of diseases. That's not good. It needs to change.'⁶¹ When these words of Chairman Mao spread to the Ministry of Health, they triggered panic. There was awareness that centralized, expert care had already been undercut by the inefficient, unequal, and corrupt pattern of its availability across the nation. A month later, the ministry's Party Committee drafted a proposal to cut back on the health care services provided to high-ranking officials. The proposal began by apologizing that the health care system in the PRC had been oriented toward high-ranking officials and had failed to deliver services to the broad masses, thus encouraging selective privileges. But the proposal gave Mao one more excuse to pass the blame for the crisis in the countryside onto the 'privileged few' or 'revisionists': 'There are too many doctors in hospitals, they far outnumber patients. Our hospitals have become the hospital of old lords. It must open up.' For Mao, to 'open up' did not necessarily mean to provide more services for the greater population; rather, it suggested changing the doctor's 'false worldview'. 'The doctor views the world as a microorganism. That is a false worldview,' he had remarked earlier at the height of the Great Leap Forward. In his canvas of a perfect future world, there was little space for bourgeois doctors whose view of the world was different from that of the masses: 'There is no need for too many doctors. They should be sent to lead public health work such as killing flies and cleaning streets. We could keep a few top doctors, but they don't need to see too many patients. They could use their spare time to sing opera. Health workers should do health work. Spending all day with patients undermines their morale.'⁶² Mao's

在农村。此外，在中国备战之际，流动医疗队，而不是大型城市医院，不仅被认为是具有成本效益的，而且具有战略重要性。与此同时，与血吸虫病的斗争继续被视为提高军事战备和效能的一个因素，并成为这种更广泛的医疗保健模式的典范。

毛泽东在革命期间曾有丰富的游击战经验，他经常回忆起革命军队与群众一起对抗疾病和敌人的“美好”军事传统。1964年6月，在与一支越南游击队会面时，他把自己塑造成一个真正的革命领袖，与苏联修正主义者形成对比，他评论说，中华人民共和国的医疗保健是仿照苏联模式建立的，并且“有太多的专科医生，他们无法治疗多种疾病。这不好。需要改变。”⁶¹当毛主席的这些话传到卫生部时，他们引发了恐慌。人们意识到，集中式的专家护理已经被全国范围内效率低下、不平等和腐败的模式所削弱。一个月后，卫生部党组起草了一份提案，削减向高级官员提供的医疗保健服务。该提案首先道歉，称中华人民共和国的医疗保健系统一直面向高级官员，未能向广大群众提供服务，从而助长了选择性特权。但该提案给了毛泽东又一个借口，将农村的危机归咎于“特权阶层”或“修正主义者”：“医院里的医生太多了，他们远远超过了病人。我们的医院已经变成了旧领主的医院。它必须开放。”对于毛泽东来说，“开放”并不一定意味着为更多的人口提供更多的服务；相反，它暗示着改变医生的“错误世界观”。“医生把世界看作是一种微生物。这是一种错误的世界观，”他早些时候在大跃进的高潮时评论道。在他对完美未来世界的设想中，资产阶级医生几乎没有空间，因为他们对世界的看法与群众不同：“不需要太多的医生。他们应该被派去领导公共卫生工作，比如消灭苍蝇和清洁街道。我们可以保留一些顶尖的医生，但他们不需要看太多的病人。他们可以利用他们的空闲时间唱戏。卫生工作者应该做卫生工作。整天和病人在一起会削弱他们的士气。”⁶²毛泽东的

irony was often misread as comments on practice rather than on the core nature of health care in the promised communist utopia.

The Ministry of Health neither closed down the Bureau of Health Care for Senior Officials nor moved toward closing the gap between urban and rural health care. But as of January 1965, an ever-greater number of hospital medical staff, as well medical students from universities and researchers from medical research institutes, were being regularly sent to the industrial and mining areas and into the countryside to 'revolutionize' hospital services by bringing bio-medical knowledge to remote corners of the country. In the spring of 1965, Huang Jiasi, the president of the Chinese Academy of Medical Science and one of the PRC's top surgeons, led a medical mission of top doctors from the Peking Union Medical College (PUMC) and the Chinese Academy of Medical Science to Xiangyin County in Hunan province. Reporting on the trip, Huang commented: 'the best way to convince the peasants of the significance of prevention was to do propaganda through treatment.'⁶³ The ambiguity of this statement underlines its inherent contradictions. It actually means the opposite of what was intended by 'putting prevention first' – a widely circulated slogan of the time. It was not prophylactic public health interventions with long-term effects but the immediate treatment of the existing ills of the population that provided the greatest propaganda benefit to the state.

On the ground, however, the effort to draw on urban resources to provide health care to the rural areas encountered many problems. One evident problem was the chronic shortage of urban medical personnel. In Sichuan province, the centre of the 'Third Front' project, where most new medical graduates had already been sent to support medical and health work in the newly built 'Third Front' factories, mines, and power plants, the provincial hospital authority resisted sending any more medical and health personnel to the countryside: 'How are we going to run hospitals if we send everyone away?' Existing personnel shortages were regularly used as an excuse for not carrying out preventive programs at all or to do them superficially and thus inadequately: 'Since we are short of personnel, we can only carry out the work poorly. If we have more staff, we will do it better.' Contrary to the official propaganda that many of the health care workers enthusiastically embraced the Ministry of Health's call, a great number of those who were drafted to serve the countryside and the remote 'Third Front' industrial development

具有讽刺意味的是，人们常常误读这些评论，认为它们是对实践的评论，而不是对承诺的共产主义乌托邦中医疗保健核心本质的评论。

卫生部既没有关闭高级官员保健局，也没有采取措施缩小城乡医疗保健之间的差距。但截至 1965 年 1 月，越来越多的医院医务人员，以及大学的医学生和医学研究机构的研究人员，被定期派往工矿区和农村地区，通过将生物医学知识带到该国偏远地区来

“革新”医院服务。1965 年春天，中国医学科学院院长、中国外科界顶级专家之一的黄家驷率领由北京协和医学院 (pumc) 和中国医学科学院的顶级医生组成的医疗队前往湖南省湘阴县。黄在汇报这次考察时评论说：“说服农民认识预防重要性的最好方法是通过治疗进行宣传。”⁶³ 这句话的模糊性强调了其内在的矛盾。它实际上意味着与“预防为主”的初衷截然相反——这是当时广为流传的口号。为国家带来最大宣传效益的不是具有长期效果的预防性公共卫生干预措施，而是对现有民众疾病的立即治疗。

然而，在实际操作中，利用城市资源为农村地区提供医疗保健的努力遇到了许多问题。一个明显的问题是城市医疗人员长期短缺。在“三线”建设的中心四川省，大多数新毕业的医科学生已经被派往支持新建的“三线”工厂、矿山和发电厂的医疗卫生工作，省医院管理局抵制派遣更多的医疗卫生人员到农村地区：“如果我们把所有的人都派走，我们怎么经营医院？”现有人员短缺经常被用作根本不开展预防性计划的借口，或者只是敷衍了事，因此无法充分开展：“既然我们人手不足，我们只能把工作做得不好。如果我们有更多的人员，我们会做得更好。”与官方宣传的许多医护人员热情拥护卫生部的号召相反，许多被征召到农村和偏远的“三线”工业发展区服务的人员

zone complained that such schemes split up families, damaged their health and well-being, and undermined their professional development. Many more worried about losing their existing benefits. One nurse in the Sichuan Traditional Medical Hospital openly announced: 'If I am called to go to the countryside, I will resign.'⁶⁴ For the villagers and their cadres in the countryside, doctors and health cadres from the cities often proved more of a burden than a help. In Sichuan province, nearly 1,500 schistosomiasis control cadres were sent to the countryside to support grassroots control work as well as the rural Socialist Education Campaign. They took with them the food rations they were assigned while living in the city, but the amount of food often turned out to be inadequate. To fill their stomachs, they would consume the villagers' food, placing an extra burden on the already impoverished rural peasants.⁶⁵

Furthermore, as Chen Zhiqian (commonly known in the West as C.C. Chen), the doyen of public health in modern China, observed, the 'Mobile [health] units could go only where the roads took them, however, and so proved to be of limited use.' More important, they were 'stunned with the multitude of needs they encountered in rural market towns and villages, especially by the incidences of infectious diseases'.⁶⁶ For brigade cadres and communities on the ground, however, sending mobile medical units to do public health work conflicted with the ideal of inspiring community participation. It undermined local agency and autonomy and gave cadres the perfect excuse not to organize any public health activities for their brigades on the local level. Communities often left urban medical and health teams to carry out sanitation work or snail elimination on their own.⁶⁷

Although authorities attempted to overcome such shortcomings by introducing measures of prevention, their ideas about prophylactic medicine were often, as Chen pointed out, vague.⁶⁸ To save state expenditure on public health and grassroots health personnel, the authorities in charge tended to put stress on the locals to practise self-reliance, but this did little to help in building a sustainable rural health system. In January 1964, at the Ninth National Conference on Schistosomiasis Control, Ke Qingshi, the party boss of the Anti-Schistosomiasis Campaign, urged all local authorities to follow the mass line and utilize local resources:

The state only has so much money. It's impossible to completely rely on the state. We won't be able to achieve our goal if we

该地区的居民抱怨说，这样的计划会分裂家庭，损害他们的健康和福祉，并破坏他们的职业发展。更多的人担心失去他们现有的福利。四川省中医院的一位护士公开宣称：“如果我被要求下乡，我就辞职。”⁶⁴ 对于乡村的村民和他们的干部来说，来自城市的医生和卫生干部往往弊大于利。在四川省，近 1500 名血吸虫病防治干部被派往农村，支持基层防治工作和农村社会主义教育运动。他们带着他们在城市生活时分配的口粮，但食物的数量往往不足。为了填饱肚子，他们会消耗村民的食物，给本已贫困的农村农民带来额外的负担。⁶⁵

此外，正如中国现代公共卫生界的元老陈志潜（西方通常称其为 C.C. Chen）所观察到的，“流动[医疗]队只能沿着道路到达的地方，因此用途有限。”更重要的是，他们“被在农村集镇和乡村遇到的众多需求震惊，尤其是传染病的发病率”。⁶⁶ 然而，对于基层的生产队干部和社区来说，派遣流动医疗队进行公共卫生工作与激发社区参与的理想相冲突。它削弱了地方机构和自主权，并为干部们提供了一个完美的借口，让他们不必在地方一级为他们的生产队组织任何公共卫生活动。社区经常让城市医疗卫生队独自进行卫生工作或消灭钉螺。⁶⁷

尽管当局试图通过引入预防措施来克服这些缺点，但正如陈所指出的那样，他们关于预防医学的想法往往很模糊。⁶⁸ 为了节省国家在公共卫生和基层卫生人员方面的支出，负责的当局倾向于强调当地人实行自力更生，但这对于建立可持续的农村卫生系统几乎没有帮助。1964 年 1 月，在第九次全国血吸虫病防治工作会议上，血吸虫病防治运动的负责人柯庆施敦促所有地方当局遵循群众路线，利用当地资源：

国家只有这么多钱。完全依赖国家是不可能的。如果我们

don't rely on the masses. We probably have the highest number of state employees in the world ... We can't just rely on these people to do the work. We need to rely on the masses ... We have not utilized our resources efficiently. For instance, in Shanghai, during the school holiday seasons, you see students all over the streets, playing with slingshots, gang fighting, climbing trees. Why can't we organize them to do some street cleaning or to kill flies? The same goes for children in the countryside. We could teach them how to prevent infection. We could ask them to dig ditches. Dig a little each day. This is something we can do. It's important that we rely on the masses.⁶⁹

Echoing his boss, in 1965 Zheng Gang, on behalf of the Nine-Man Subcommittee, instructed Sichuan province's health authorities 'to uphold the spirit of self-reliance. To fight the battle to prevent the disease by relying on grassroots resources, from grassroots party leadership to cadres, villagers and health workers. To eradicate schistosomiasis with or without little financial and personnel aid from the state.'⁷⁰

Such projects seemed to cause logistical nightmares and could only be answered by ignoring them and evoking the mantra of local self-reliance, a euphemism for the simple inability to appropriately distribute scarce resources from the urban to the rural arena. Yet, as Chen Zhiqian rightly argued: 'intermittent visits by mobile health teams simply cannot do the job. Without systematic organization, the tendency is toward hit-or-miss measures of prevention.'⁷¹ Before the preventive program could be carried out, a true rural health system had to evolve.

‘MORE PART-TIME GRASSROOTS HEALTH WORKERS
FOR THE COUNTRYSIDE’: THE BEGINNING OF
THE BAREFOOT DOCTORS

In Chen's native Sichuan province, because of a huge deficit in health and medical care as well as a shortage of resources and increased morbidity, the provincial health authority had proceeded to franchise health care delivery, partly by prompting the training of greater numbers of grassroots, part-time health workers (health auxiliaries) to use traditional therapeutic medicine and to undertake public health work. This was well before any policy change emanating

不要依靠群众。我们可能拥有世界上最多的国家雇员……我们不能仅仅依靠这些人来完成工作。我们需要依靠群众……我们没有有效地利用我们的资源。例如，在上海，在学校假期期间，你看到学生们遍布街头，玩弹弓，团伙斗殴，爬树。为什么我们不能组织他们进行街道清洁或杀死苍蝇？乡村的儿童也是如此。我们可以教他们如何预防感染。我们可以要求他们挖沟。每天挖一点。这是我们可以做的事情。重要的是我们要依靠群众。⁶⁹

1965年，郑刚代表九人小组委员会呼应他的上司，指示四川省卫生部门“坚持自力更生的精神。依靠基层资源，从基层党的领导到干部、村民和卫生工作者，打好预防疾病的战役。无论国家是否提供少量资金和人员援助，都要根除血吸虫病。”⁷⁰

这类项目似乎会造成后勤方面的噩梦，而唯一的解决办法就是忽视它们，并唤起地方自力更生的信条，这委婉地表达了简单地无法将稀缺资源从城市合理分配到农村地区。然而，正如陈志潜正确地指出：“流动医疗队的间歇性访问根本无法完成这项工作。如果没有系统的组织，预防措施往往会时好时坏。”⁷¹在预防计划能够实施之前，必须发展出一个真正的农村卫生系统。

‘MORE PART-TIME GRASSROOTS HEALTH WORKERS
FOR THE COUNTRYSIDE’: THE BEGINNING OF
THE BAREFOOT DOCTORS

在陈志潜的家乡四川省，由于卫生和医疗保健方面存在巨大赤字，资源短缺以及发病率增加，省级卫生部门已开始特许经营医疗保健服务，部分是通过促使培训更多基层兼职卫生工作者（卫生助理员）来使用传统治疗药物并开展公共卫生工作。这远远早于任何政策变化。

from Beijing. In January 1965, at a meeting for regional health officials, the provincial leadership instructed local authorities to engage in training part-time health workers for the countryside and the mines. The training would focus on traditional medicine and would take place in the less busy agricultural seasons; those enrolled in the course would work in the fields or the mines for five months of the year while attending training during the other seven months. In the context of the Socialist Education Campaign, to deliver health was also to deliver class struggle. Thus, not surprisingly, the training curriculum also consisted of studying the works of Chairman Mao as well as politics. These auxiliaries were nominated by the communes and the brigades as well as the mines, with an emphasis placed on their political reliability. They were, for instance, from middle (class) or poor peasant families. Unemployed educated youth from the cities could also enrol in the training program, provided they were from the correct 'class' with the 'right consciousness'.⁷²

In the countryside, a number of communes also opened after-hours health training courses for villagers. In January 1965, within a few months, some sixty villagers in Gaoliang commune in Neijiang County had attended the commune's part-time, health training school where they learned to carry out first aid and preventive medicine. The latter included recognizing most common parasites and diseases and giving vaccinations, as well as managing night-soil and water.⁷³ By the autumn of 1965, according to an official estimate, the number of grassroots, part-time health workers (including midwives) in rural Sichuan grew to 25,000. Because Sichuan was the centre of the 'Third Front' area, the majority of village-level part-time health workers also took part in militia training. This included military paramedics in preparation for the feared war. In addition to carrying out public health work like schistosomiasis control and administering vaccinations, many would also treat minor illnesses and deliver babies.

The shift to local urgent care lifted health burdens down the line. With the growing number of grassroots, part-time health workers in parts of rural Sichuan, as the data gathered by provincial health authority suggests, the demand for hospital services as well as the services of mobile medical teams decreased by almost half in 1965. In Mianyang County's Zundao Commune, after becoming the model for successfully controlling schistosomiasis by following the mass line, as mentioned above, the commune authority invested in

来自北京。1965年1月，在一次区域卫生官员会议上，省级领导指示地方当局为农村和矿山地区培训兼职卫生工作者。培训将侧重于传统医学，并在农业不繁忙的季节进行；参加课程的人员一年中有五个月在田间或矿山工作，其余七个月参加培训。在社会主义教育运动的背景下，提供医疗卫生也是进行阶级斗争。因此，毫不奇怪的是，培训课程还包括学习毛主席的著作以及政治。这些辅助人员由公社、大队和矿山提名，重点考察其政治可靠性。例如，他们来自中农（阶级）或贫农家庭。来自城市的失业受过教育的青年也可以参加培训计划，前提是他们来自正确的“阶级”，具有“正确的意识”。⁷²

在农村，许多公社也为村民开设了业余健康培训课程。1965年1月，几个月内，内江县高粱公社约有60名村民参加了公社的业余健康培训学校，学习开展急救和预防医学。后者包括识别最常见的寄生虫和疾病、接种疫苗以及管理粪便和水。⁷³据官方估计，到1965年秋，四川农村的基层业余卫生工作者（包括助产士）人数增加到25000人。由于四川是“三线”地区的中心，大多数村级业余卫生工作者也参加了民兵训练。这包括为应对可能发生的战争而进行的军事医护人员训练。除了开展血吸虫病控制和接种疫苗等公共卫生工作外，许多人还会治疗小病和接生。

转向当地的紧急护理减轻了后续的健康负担。正如四川省卫生部门收集的数据显示，随着四川农村地区基层、兼职卫生工作者数量的不断增加，1965年对医院服务和流动医疗队服务的需求减少了近一半。在绵阳市的遵道公社，在通过群众路线成功控制血吸虫病成为典范后，如上所述，公社当局投资于

training more than 100 village-level part-time workers in 1965. In the autumn of that year, these health auxiliaries carried out small-pox vaccination for the entire commune within fifteen days. In addition, they treated 6,700 villagers for minor illnesses and injuries and thus saved villagers more than 690 *yuan* in hospital registration fees. 'It's great to have health workers at our village. We don't need to go all the way to the town to find a doctor. They saved us the hassle as well as money. Great idea,' villagers noted.⁷⁴ In northern Sichuan's Langzhong County, an official investigative report dated 1965 showed that the plan to build more rural hospitals and united clinics was not economically viable, nor was it what the rural population desired: Villagers want what's cheap and what works ... They prefer herbal medicine and they always decide on which doctor to see.' As such, Baishan Commune joined with a united clinic run by local doctors to train sixty village-level part-time health workers, including eight midwives. As part of the training, experienced local traditional healers were invited to share the medical knowledge that had been handed down in their families. After a seven-day training, these health auxiliaries returned to their villages. In addition to health work, they were also assigned the responsibility of engaging their communities to take part in public health activities as well as to care for themselves. Village festivals and market days turned into public health information events, and villagers were even invited to produce public health ballads in the form of the local opera.⁷⁵

In addition to training rural, grassroots, part-time health workers and encouraging communities to become involved in health planning, the authorities in Sichuan province also introduced the policy of allowing all medical institutions, from hospital to clinic to pharmacy, to procure medicinal herbs directly from brigades or individual villagers. By cutting out the 'middle-man' (the state), this reduced the cost of herbal remedies, thus enabling each commune or town to become 'self-sufficient' with its own pharmacies. For most villagers, this meant that, once again, traditional herbal remedies became affordable and accessible.⁷⁶ It also meant, however, that it was difficult for the state to monitor the quality and distribution of herbal medicine, paving the way for an illicit and often questionable medical market to emerge in the following years, as will be discussed in the next chapter. This contributed to the increasing stigma around herbal medicine as less efficacious, while simultaneously reinforcing the idea that some type of local

1965 年培训了 100 多名村级兼职人员。当年秋天, 这些卫生助理在十五天内为整个公社进行了天花疫苗接种。此外, 他们还还为 6700 名村民治疗了小病小伤, 从而为村民节省了 690 多元的医院挂号费。村民们说: “我们村里有卫生员真是太好了。我们不需要跑到镇上去找医生。他们为我们省去了麻烦, 也省了钱。真是个好主意。”⁷⁴ 四川北部阆中县 1965 年的一份官方调查报告显示, 建造更多乡村医院和联合诊所的计划在经济上不可行, 也不是农村人口所希望的: 村民想要的是便宜的和有效的……他们更喜欢草药, 而且总是自己决定看哪个医生。因此, 白山公社与当地医生经营的联合诊所合作, 培训了 60 名村级兼职卫生员, 其中包括 8 名助产士。作为培训的一部分, 经验丰富的当地传统治疗师被邀请分享他们家族世代相传的医学知识。经过七天的培训后, 这些卫生助理回到了各自的村庄。除了卫生工作外, 他们还被分配了责任, 让他们的社区参与公共卫生活动, 并照顾好自己。乡村节日和集市变成了公共卫生宣传活动, 村民们甚至被邀请以当地歌剧的形式创作公共卫生歌谣。⁷⁵

除了培训农村基层兼职卫生工作者和鼓励社区参与健康规划外, 四川省当局还推出了一项政策, 允许所有医疗机构, 从医院到诊所再到药房, 直接从生产队或村民个人那里采购药材。通过取消

“中间人” (国家), 这降低了草药的成本, 从而使每个公社或城镇都能通过自己的药房实现“自给自足”。对于大多数村民来说, 这意味着传统草药再次变得负担得起且容易获得。⁷⁶ 然而, 这也意味着国家很难监控草药的质量和分销, 为未来几年出现非法且常常成问题的医疗市场铺平了道路, 这将在下一章中讨论。这导致了人们对草药疗效越来越低的负面看法, 同时也强化了某种本地

control or production in the arena of health care was at least possible, if not completely desirable.

Elsewhere, as in rural Henan province, the number of grassroots, part-time health workers was also increasing at a steady pace. Like Sichuan, Henan, China's agricultural heartland, had been hit by radical collectivization and the resultant famine. In the aftermath of the famine, most commune hospitals and clinics had been closed down because of staff cuts. After the latter half of 1963, as agriculture began to recover in Henan, a growing number of communes began to offer training courses off-season to villagers on how to treat minor illnesses and provide elementary acupuncture and some knowledge of common diseases and their prevention. Each brigade would choose one villager to attend such a course. Upon completing the course, he or she would bear the responsibility of providing basic health care to the villagers; this included treating minor illnesses and injuries as well as organizing public health activities. By the end of 1964, according to official data, there were more than 300,000 such grassroots, part-time health workers in Henan province, some at the village level, some at the commune level.⁷⁷

In the meantime, some of the technical officials within the Ministry of Health began to see training more grassroots, part-time health workers as a way forward toward solving the rural health crisis and building an affordable and more sustainable rural health system. Reporting on the PUMC and the Chinese Academy of Medical Science medical mission to rural Hunan, the president of the Chinese Academy of Medical Science, Huang Jiashi, affirmed that 'medical teams from the city cannot completely change the picture of rural health or thoroughly solve the problem of medical care in the countryside. It is necessary to train local personnel.'⁷⁸ Even the experts had begun to see the double value in the decentralization of health care as an answer to what had seemed intractable problems of supply and demand.

FRANCHISING RURAL HEALTH CARE DELIVERY

In April 1965, the Party Committee of the Ministry of Health sent a consultation plan to expand the rural, grassroots, part-time health worker and midwife training program to all provinces as well as medical colleges. It foresaw each village having at least one health worker as well as a midwife within the next three to five years.⁷⁹ To

至少在医疗保健领域，控制或生产是可能的，即使不是完全可取的。

在其他地方，如河南省农村地区，基层兼职卫生工作者的人数也在稳步增加。和四川一样，河南这个中国的农业中心，也受到了激进的集体化和由此造成的饥荒的打击。饥荒过后，由于人员削减，大多数公社医院和诊所都被关闭了。1963年下半年之后，随着河南农业开始复苏，越来越多的公社开始在农闲时为村民提供培训课程，教他们如何治疗小病、进行基本针灸以及了解常见疾病及其预防。每个生产队都会选出一名村民参加这样的课程。完成课程后，他或她将承担为村民提供基本医疗保健的责任；这包括治疗小病和外伤以及组织公共卫生活动。根据官方数据，到1964年底，河南省有超过30万这样的基层兼职卫生工作者，有些在村一级，有些在公社一级。⁷⁷

与此同时，卫生部的一些技术官员开始认为，培训更多基层、兼职的卫生工作者是解决农村卫生危机、建立负担得起且更可持续的农村卫生体系的一种前进方向。中国医学科学院院长黄家驊在汇报北京协和医学院和中国医学科学院赴湖南农村的医疗任务时，肯定了“城市医疗队不可能完全改变农村卫生的面貌，也不可能彻底解决农村的医疗问题。必须培养当地人才”。⁷⁸甚至专家们也开始看到医疗保健权力下放的双重价值，以此作为解决似乎难以解决的供需问题的方法。

农村医疗保健服务特许经营

1965年4月，卫生部党委向各省和医学院校发出了一项咨询计划，旨在扩大农村基层兼职卫生员和助产士培训计划。该计划预计在未来三到五年内，每个村庄至少有一名卫生员和一名助产士。⁷⁹为

a degree, this consultation plan mirrored a number of rural, grass-roots, part-time health worker programs that had already been set in motion in Sichuan, Henan, and in Zhejiang and Shanghai, as discussed earlier. The process of franchising rural health delivery was further catalyzed by the escalating political tension between Mao and Liu Shaoqi that had begun in 1962, partly triggered by the debacle of the Great Leap Forward. Liu, by publicly acknowledging that the famine was man-made, had undermined Mao's leadership.⁸⁰

On 26 June 1965, in a conversation with Mao, Dr Li Zhishui, Mao's personal physician, remarked that published medical textbooks intended for popular readership neglected the fact that the majority of the population in China lived in the countryside. The occasion allowed Mao to attack his political opponent Liu Shaoqi, making him 'the enemy of the people' responsible for the appalling reality of the countryside. The Ministry of Health served as a more than convenient scapegoat. According to Mao, it had come under the control of the technocrats and bureaucrats who followed Liu Shaoqi's revisionist line: '[The Ministry of Health] is providing health care to only 15 per cent of the people in this nation. Of these 15 per cent, it is those lords in the national and local governments who receive the best care ... but the vast percentage of people in the countryside have no health care at all ... The Ministry of Health is not serving the people. It is not the people's ministry.' Medical education oriented toward training students to provide a high-tech, urban-based medical service was another target for Mao's attack. Quoting Mencius that 'believing everything in books is worse than having no books at all,' he urged the Ministry of Health to reform medical education by shortening the length of training and to promote practice-based learning. 'Medical students don't need to read so many books ... Medical schools don't have to enrol high school graduates. A primary school certificate should be sufficient to begin studying medicine. Medical skill is learned through practice. The type of doctor we need in the villages does not have to be so talented; they would still be better than witch doctors. Besides, this is the only type of doctor the villages can afford.' Having created scapegoats in the 'scandalous' Ministry of Health and 'useless' medical education, Mao once more established himself as the 'saviour of the people', hence the sole authority about health (and everything else) in the socialist state. He concluded that 'we should devote a major portion

一定程度上, 该咨询计划反映了四川、河南以及浙江和上海等地已经启动的许多农村、基层、兼职卫生员项目, 如前文所述。农村卫生保健特许经营进程进一步受到毛泽东和刘少奇之间日益升级的政治紧张局势的催化, 这场紧张局势始于1962年, 部分原因是由于大跃进的惨败。刘公开承认饥荒是人为造成的, 从而削弱了毛的领导地位。⁸⁰

1965年6月26日, 在与毛泽东的一次谈话中, 毛泽东的私人医生李志绥指出, 为大众读者出版的医学教科书忽略了一个事实, 即中国的大部分人口居住在农村。毛借此机会攻击他的政治对手刘少奇, 称他是“人民的敌人”, 对农村的可怕现实负有责任。卫生部成为了一个非常方便的替罪羊。毛认为, 它已受到追随刘少奇修正主义路线的技术官僚和官僚的控制: “[卫生部] 只为这个国家15%的人提供医疗保健。在这15%的人中, 是那些在国家和地方政府中的老爷们得到了最好的照顾……但农村的绝大多数人根本没有医疗保健……卫生部不是为人民服务的。它不是人民的部门。”面向培养学生提供高科技、城市医疗服务的医学教育是毛泽东攻击的另一个目标。他引用孟子的话说“尽信书不如无书”, 敦促卫生部通过缩短培训时间来改革医学教育, 并提倡基于实践的学习。“医学生不需要读那么多书……医学院不必招收高中毕业生。小学毕业证书应该足以开始学习医学。医术是通过实践学到的。我们在农村需要的医生类型不必那么有才华; 他们仍然比巫医好。此外, 这是农村唯一能负担得起的医生类型。”毛泽东在“可耻的”卫生部和“无用的”医学教育中创造了替罪羊, 再次将自己确立为“人民的救星”, 因此是社会主义国家中关于健康(以及其他一切)的唯一权威。他总结说“我们应该投入主要部分”

of our resources to what the masses need most ... Our future medical profession should concentrate its future work in the villages.”⁸¹ After this conversation, Dr Li drew up a memo and sent it to the Ministry of Health and Peng Zhen, the mayor of Beijing and also a member of the Politburo. According to the doctor, writing in his memoir, he had no idea that this memo would become the famous ‘June 26 Directive’, the basis for decentralizing aspects of health care, which led to the launching of the nationwide Barefoot Doctor campaign three years later at the height of the Great Proletarian Cultural Revolution.⁸²

Although this conversation between Mao and his doctor was not made public at the time, it prompted the Ministry of Health to switch on a green light to allow the franchising of health care delivery, a process that had already begun in parts of rural China, from Zhejiang and Jiangsu as well as in the suburbs of Shanghai in the east, to Sichuan in the remote southwest, and to Henan in the heartland. In the next couple of months, Qian Xinzong met with top CCP leaders, including Zhou Enlai and Liu Shaoqi as well as Mao on different occasions, to finalize the decentralization of health care delivery in the countryside. Qian was rather anxious because he had just been promoted to minister of health. More important, as we have seen, the process of decentralization on the ground had already begun without approval from the top, and it seemed imperative to earn the endorsement of top leaders before he, and the Ministry of Health, could be penalized for allowing such grassroots health care development to flourish without official authorization from the centre.

While Premier Zhou and President Liu endorsed a partially centralized structure that relied on regularly sending urban medical teams to the countryside as well as promoting preventive medicine, the chairman showed greater interest in training village-level part-time health workers. For many months, Mao had been hearing news of peasants in the countryside becoming weary of the Socialist Education Campaign. One complaint concerned the harsh methods of the work teams sent by the authorities to ‘assist’ the campaign in the countryside. Mao was not pleased and began to contemplate replacing the Socialist Education Campaign with a new program that would rally the masses around him to promote and protect socialism in the countryside. In May 1965, a month prior to his conversation with Dr Li, Mao, at the age of seventy-one, climbed the

把我们的资源用于群众最需要的地方……我们未来的医疗行业应该把未来的工作重心放在农村。⁸¹在这次谈话之后，李医生起草了一份备忘录，并将其发送给卫生部以及北京市市长、同时也是政治局委员的彭真。根据这位医生在其回忆录中的记载，他并不知道这份备忘录会成为著名的“6·26指示”，这是卫生保健方面权力下放的基础，并在三年后大张旗鼓的无产阶级文化大革命高潮时期，促成了全国性的赤脚医生运动的开展。⁸²

尽管毛泽东和他的医生之间的这段对话当时并未公开，但它促使卫生部亮起了绿灯，允许以特许经营的方式提供医疗服务。这一过程实际上已经在中国农村的部分地区展开，从东部的浙江和江苏以及上海郊区，到偏远西南地区的四川，再到中部的河南。在接下来的几个月里，钱信忠会见了包括周恩来、刘少奇和毛泽东在内的中共高层领导人，最终确定了在农村地区下放医疗保健服务的权力。钱信忠相当焦虑，因为他刚刚晋升为卫生部长。更重要的是，正如我们所见，权力下放的过程实际上已经在基层展开，但没有得到高层的批准，因此，在他和卫生部因为允许这种基层医疗保健发展在没有中央官方授权的情况下蓬勃发展而受到惩罚之前，争取高层领导人的认可似乎势在必行。

当周总理和刘主席赞成一种部分集中的结构，这种结构依赖于定期派遣城市医疗队到农村以及推广预防医学时，毛主席对培训村级兼职卫生工作者表现出更大的兴趣。在过去的几个月里，毛泽东一直听到农村的农民对社会主义教育运动感到厌倦的消息。一个抱怨是当局派出的工作队以“协助”农村运动的严厉方法。毛泽东对此并不满意，并开始考虑用一项新的计划取代社会主义教育运动，该计划将团结群众在他周围，以促进和保护农村的社会主义。1965年5月，在与李医生谈话前一个月，毛泽东以71岁的高龄攀登了

Jinggang Mountain in Jiangxi province, where he had led the revolutionary war of the masses some thirty years before. The trip cheered him up. He was pleased to see that the 'seed of revolution' he had planted had borne fruit, and he was determined to guard the 'fruit of the revolution' by once more mobilizing the masses on his side. At a planning meeting for the Third Five-Year Plan two weeks later, he warned against fuelling industrialization by increasing the state procurement quota for agricultural products: 'If we procure too much, people will rebel ... we mustn't push them too hard. The masses are the most important to us. We mustn't lose the heart of the people ... We will lose our way if we alienate the masses,' he stressed.⁸³

Yet how to win back the masses – here, the peasants – who were individuals with little or no political ideology but self-interest? Mao, well versed in Chinese history, understood the propaganda power of promises of healing and health. Since the Yellow Turban Rebellion in the late second century, epidemic, war, and natural disasters, including famine, had been seen as signs leading to the downfall of many dynasties. In the post-famine PRC, with war potentially looming in the background, the collapse of rural health care could be the final mishap that might undermine the revolutionary project and bring down the political power of the Communist Party. Yet Mao also knew well that the Chinese understood that disease and illness were part of the human condition that demanded amelioration.⁸⁴ Beginning with the Zhang brothers, the leaders of the Yellow Turban Rebellion, almost all major sectarian insurgencies in Chinese history had involved some form of faith-healing. According to the received view of Chinese history, in the late Han period (25–220 CE), as a result of epidemics and natural disasters, the demand for medical treatment grew. Many people had complained that the cost of medicine was too high, especially considering the economic damage caused by various natural disasters, and while looking for cheaper alternatives they found faith-healers such as Zhang Jue, Zhang Bao, and Zhang Liang, the leaders of the Yellow Turban Rebellion. Those who were cured became 'believers' and joined the rebellion in different parts of the country.⁸⁵

In 1934, when Mao's revolutionary army was under siege by the Nationalist army, in order to win the support of the masses, he had urged his comrades to 'be concerned with the wellbeing of the masses'. In his concluding speech that year at the Second National Congress of Workers and Peasants, he famously proclaimed that by becoming the 'true organizer of their well-being', from food to shelter,

江西井冈山，大约三十年前，他曾在那儿领导群众进行革命战争。这次旅行让他精神振奋。他很高兴看到他播下的“革命的种子”已经结出了果实，他决心再次动员群众站在他这边，以保卫“革命的果实”。两周后，在第三个五年计划的规划会议上，他警告说，不要通过提高国家对农产品的采购配额来推动工业化：“如果我们采购过多，人民会反抗……我们不能对他们施加太大的压力。群众对我们来说是最重要的。我们不能失去民心……如果我们疏远群众，我们就会迷失方向，”他强调说。⁸³

然而，如何赢回群众——这里指的是农民——他们是几乎没有政治意识形态，但有自身利益的个体？毛泽东精通中国历史，深知承诺治愈和健康的宣传力量。自公元2世纪末的黄巾起义以来，瘟疫、战争和自然灾害，包括饥荒，都被视为导致许多王朝覆灭的迹象。在后饥荒时代的prc，潜在的战争阴影若隐若现，农村医疗保健的崩溃可能是最终的不幸，可能会破坏革命事业并推翻共产党的政治权力。然而，毛泽东也清楚地知道，中国人明白疾病是人类状况的一部分，需要改善。⁸⁴从黄巾起义的领袖张氏兄弟开始，中国历史上几乎所有主要的宗派叛乱都涉及某种形式的信仰疗法。根据中国历史的传统观点，在东汉时期（公元25-220年），由于瘟疫和自然灾害，对医疗的需求增长。许多人抱怨药价过高，特别是考虑到各种自然灾害造成的经济损失，在寻找更便宜的替代品时，他们找到了张角、张宝和张梁等信仰治疗师，他们是黄巾起义的领袖。那些被治愈的人成为“信徒”，并在全国各地加入了叛乱。

⁸⁵

1934年，当毛泽东的革命军队被国民党军队围困时，为了赢得群众的支持，他曾敦促他的同志们“关心群众的福祉”。在那一年于第二次全国工农代表大会上的闭幕词中，他著名地宣称，通过成为“他们福祉的真正组织者”，从食物到住所，

clothing, sickness and health, and marriage, 'the masses would rally around us and give us their warm support.'⁸⁶ Some twenty years later, in 1955, to rally the masses around him to support his socialist revolution in the countryside, Mao had called for the eradication of schistosomiasis. In 1965, in the aftermath of the Great Leap famine, in order to prove the infallibility of his socialist revolution project, he would attempt once again turn the masses into 'believers' with the promise of health and healing. Much to the Ministry of Health's surprise, during one of his conversations with Qian Xinzong, the minister of health, and Zhang Kai from the Ministry of Health's Party Committee, Mao used the political history of 'magic medicine' to illustrate his point of the centrality of health for the political project: 'Magic medicine [the Yellow Turban leader Zhang Jue's healing techniques included using charm water as well as other forms of magic] has three advantages: firstly, it's safe, they won't poison anyone; secondly, it's cheap, it only costs a few coppers; thirdly, it's comforting; it makes people feel better,' he reasoned.⁸⁷

But who would be trusted to be the 'apostle' to deliver the 'gospel' of healing and health? Certainly not the 'corrupt' health cadres and the 'bourgeois' doctors, who, according to Mao, were responsible for the inequality as well as the misery of the masses. His answer was the training of village-level, part-time health workers; they would spring from the masses. Standing at the opposite pole from the 'bourgeois' and 'impractical', therefore 'useless', expert, as well as the 'revisionist' officials, these health workers would form a new class in the countryside as the 'successors of the revolution' who would carry the spirit of the revolution forward. Mao went on at great length to discuss the criteria for choosing grassroots health workers: 'Middle school graduates would do. The important thing is they must be politically reliable. This is very important,' he reminded the Ministry of Health. With the looming war as another major fear, Mao also stressed the need to promote primary care as opposed to specialized medicine as well as to build an adequate health network that would cover all remote corners of the country: 'if the war breaks out, it won't be possible to attend hospitals in Beijing. [Planning] health work should be preparing for war ... When the war breaks out, doctors should be able to treat all kinds of illnesses as well as nurse the wounded. Specialized medicine is not good [for such a purpose].'⁸⁸ The new health workers would thus help to solve a wide range of existing and potential difficulties in post-famine China.

服装、疾病与健康以及婚姻，“群众就会团结在我们周围，给予我们热烈的支持。”⁸⁶大约二十年后，在1955年，为了团结群众支持他在农村的社会主义革命，毛泽东呼吁根除血吸虫病。1965年，在大跃进饥荒之后，为了证明他的社会主义革命项目的绝对正确，他将再次尝试用健康和治愈的承诺将群众变成“信徒”。令卫生部惊讶的是，在与卫生部部长钱信忠和卫生部党委会的张凯的一次谈话中，毛泽东用“巫医”的政治史来说明健康对于政治项目的重要性：“巫医[黄巾军领袖张角的治疗技术包括使用符水和其他形式的魔法]有三个优点：第一，它很安全，他们不会毒害任何人；第二，它很便宜，只需要几个铜板；第三，它很安慰人；它让人感觉更好，”他推理道。⁸⁷

但是，谁会被信任为传递治疗和健康的“福音”的“使徒”？当然不是那些“腐败”的卫生干部和“资产阶级”医生，根据毛的说法，他们应对群众的不平等和痛苦负责。他的答案是培训村级兼职卫生工作者；他们将来自群众。这些卫生工作者与“资产阶级”和“不切实际”，因此“无用”的专家，以及“修正主义”官员完全相反，他们将在农村形成一个新阶级，成为“革命的接班人”，他们将把革命精神发扬光大。毛泽东用了大量篇幅讨论选择基层卫生工作者的标准：“中学毕业生就可以了。重要的是他们必须在政治上可靠。这非常重要，”他提醒卫生部。由于迫在眉睫的战争是另一个主要的担忧，毛还强调需要促进初级保健而不是专科医学，并建立一个覆盖全国所有偏远角落的充足的卫生网络：“如果战争爆发，就不可能去北京的医院就诊。[规划]卫生工作应该为战争做准备……战争爆发时，医生应该能够治疗各种疾病并护理伤员。专科医学对于[这样的目的]不好。’⁸⁸因此，新的卫生工作者将有助于解决中国后饥荒时期现有的和潜在的各种困难。

In September 1965, the CCP Central Committee approved the decentralization of rural health care delivery, and the nationwide rural, grassroots, part-time health worker training program leapt into top gear.⁸⁹ On the ground, however, there was confusion. Ever since the Great Leap Forward, the development of grassroots health organizations below the county level had been organized from the bottom up, first by the commune and then by the brigade. The latter came about with the introduction of the Sixty Articles to give production brigades the autonomy to manage their own production and labour more efficiently. It allowed local authorities to develop a grassroots health organization tailored for local conditions, such as those in Sichuan and Henan, as discussed earlier in this chapter, as well as in the regions around Zhejiang and Shanghai (see the previous chapter). Transforming it into a national program administered from the top down impeded local autonomy, thus actually undermining the initiatives. Decentralization turned out to be a centralized system with the grassroots health workers at the bottom of the health care pyramid, supervised by district or town hospitals and health centres. The provincial health departments responsible for implementing the Ministry of Health's policy coordinated and planned the grassroots health work, from training health auxiliaries to preventive activities. This often caused struggles on the ground. For instance, in many parts of the country the training schedule was decided by provincial and regional health departments, not by the production brigades, so it did not always parallel the agricultural seasons. When the training took place during the planting or harvesting seasons, it cost the brigades precious labour time, and so those brigades that were affected refused to send health workers to attend. Given that the same problem had previously occurred in the attempts at schistosomiasis control and similar public health work, it seemed that the centralized health authorities could not even learn from their own prior mistakes.

Uneven, often poor-quality training was another major problem. In a great rush to cover the entire countryside with health auxiliaries, in 1965 the Ministry of Health set the goal that over the next five to ten years, each production brigade would have at least one health worker equipped to attend to villagers' minor illnesses and be able to administer elementary acupuncture. On the next level up, each large brigade would have at least one part-time doctor who

1965年9月,中共中央批准了农村卫生保健服务下放,全国性的农村基层兼职卫生工作者培训计划迅速启动。⁸⁹然而,实际情况却令人困惑。自从“大跃进”以来,县级以下基层卫生组织的发展都是自下而上组织的,首先是公社,然后是生产大队。后者是随着“六十条”的引入而产生的,旨在赋予生产大队自主管理生产和劳动的权力,从而提高效率。它允许地方当局根据当地情况发展基层卫生组织,例如本章前面讨论的四川和河南的情况,以及浙江和上海周边地区(参见前一章)。将其转变为由上而下管理的全国性计划,阻碍了地方自主权,实际上破坏了这些举措。事实证明,权力下放是一个中央集权系统,基层卫生工作者位于医疗保健金字塔的底部,由区或镇医院和卫生中心监督。负责执行卫生部政策的省级卫生部门协调和规划基层卫生工作,从培训卫生辅助人员到预防活动。这经常在基层引起冲突。例如,在该国许多地区,培训时间表由省级和区域卫生部门决定,而不是由生产大队决定,因此它并不总是与农业季节同步。当培训发生在播种或收获季节时,它会消耗大队宝贵的劳动时间,因此那些受影响的大队拒绝派卫生工作者参加。鉴于以前在血吸虫病控制和类似的公共卫生工作中也出现过同样的问题,看来中央卫生部门甚至无法从自己以前的错误中吸取教训。

培训不均衡,质量通常较差,是另一个主要问题。1965年,为了尽快在整个农村地区普及卫生员,卫生部制定了一个目标,即在未来五到十年内,每个生产队至少配备一名卫生员,负责处理村民的小病小痛,并能够进行基本的针灸治疗。上一级的大队至少要有一名兼职医生,

would be responsible for treating and diagnosing common diseases as well as leading public health work. In addition, each large brigade would train one or two midwives, whereas each commune would also have four to five well-trained doctors. Paradoxically, as Chen Zhiqian, who played a noteworthy role in developing public health in modern China, remarked, there was no guideline for developing a carefully structured system that linked these grassroots health workers to 'a hierarchy of more fully trained medical personnel at higher rural levels or to advanced scientific personnel in the cities'.⁹⁰ In Chen's native Sichuan province, there were in total 544,669 production brigades and 71,122 large brigades. This meant that across the entire province, if the Ministry of Health's policy were to be carried out successfully, there would be at least 540,000 village-level health auxiliaries and 70,000 midwives as well as 71,000 part-time doctors. This was a giant leap up from the 25,000 grassroots, part-time health workers/midwives the local authorities had already trained. Meeting the Ministry of Health's target meant an explosion of crash training courses, with many of them overlapping each other. With an emphasis on quantity, most of these courses compromised quality. The same problems that had emerged with the political goal of 'eradicating schistosomiasis within seven years' reoccurred. The inadequate training affected the efficiency of these health workers, and even after their training many did not have the basic skills to even diagnose diseases or administer drugs.⁹¹

Aside from uneven, often poor-quality training and by putting an emphasis on quantity, the program was further hampered by the cultural sensibilities of local populations in different parts of the country. Further south in Yunnan province's Ruili County, across the border from Myanmar, Zhimai, a fifteen-year-old Jingpo girl from an ethnic group who lived in the hills between Myanmar and Yunnan, was sent by her brigade to attend the health auxiliary/midwife training course at a local health school. Because her father was a devoted Christian convert, Zhimai was the only girl in the village who had completed primary school education.* For this reason, she was chosen to be the brigade's part-time health worker and the new-style midwife so that the brigade could fulfil

* Christian missionaries working amongst subaltern groups such as the Jingpo promoted education for girls.

将负责治疗和诊断常见疾病,以及领导公共卫生工作。此外,每个大队将培训一到两名助产士,而每个公社也将有四到五名训练有素的医生。矛盾的是,正如在现代中国公共卫生发展中发挥了显著作用的陈志潜所说,没有指导方针来发展一个精心构建的系统,将这些基层卫生工作者与“农村较高层次的训练有素的医疗人员或城市中的高级科研人员的等级体系”联系起来。⁹⁰在陈志潜的家乡四川省,共有544669个生产队和71122个大队。这意味着,如果在全省范围内成功执行卫生部的政策,将至少有54万名村级卫生助理、7万名助产士以及7.1万名兼职医生。这与地方当局已经培训的25000名基层兼职卫生工作者/助产士相比,是一个巨大的飞跃。要达到卫生部的目标,意味着速成培训班的激增,其中许多课程相互重叠。由于强调数量,大多数课程都牺牲了质量。在“七年内消灭血吸虫病”的政治目标中出现的同样问题再次出现。培训不足影响了这些卫生工作者的效率,即使在培训之后,他们的许多人仍然没有诊断疾病或管理药物的基本技能。⁹¹

除了不均衡、通常质量低劣的培训以及对数量的强调之外,该项目还受到该国不同地区当地居民文化敏感性的阻碍。在云南省瑞丽县更南端,与缅甸接壤,来自景颇族的十五岁女孩志迈被她的生产队送到当地卫生学校参加卫生员/助产士培训课程。志迈来自一个居住在缅甸和云南之间山区的少数民族。因为她的父亲是一位虔诚的基督教皈依者,所以志迈是村里唯一完成小学教育的女孩。*因此,她被选为生产队的兼职卫生员和新式助产士,以便生产队能够完成

* 在景颇等下层群体中工作的基督教传教士提倡女孩接受教育。

the national requirement of at least one brigade/one health worker plus one new-style midwife. But Zhimai did not want to become a health worker or midwife; she wanted to learn how to weave cloth, not how to give injections because she feared them. Furthermore, in her village there were special childbirth rituals, and many of the villagers considered the new-style medicalized childbirth promoted by the PRC's public health authorities as taboo. In the end, the brigade head bullied her into attending the course. During the training, she could not face examining the different parts of the vagina; she felt ashamed, and during her first practical training session she was so scared that her body could not stop shaking. She sweated so much that someone in her class had to help her by wiping her face. In addition, because too many people were enrolled in the training, there were barely any chances to practise what she had learned. Yet, after only two training sessions on delivering babies, she was sent back to her village to serve as the front-line health worker.⁹²

Despite many initial drawbacks, with the advent of the Great Proletarian Cultural Revolution, which began in the summer of 1966, the political importance of the rural village health worker program grew exponentially. In 1974, Wang Guizhen would become the PRC's 'model Barefoot Doctor' representing her country and speaking to the Twenty-Seventh World Health Assembly on China's Barefoot Doctor program. She had been in her twenties when the 'Learn from Lei Feng' campaign first began in 1963. Inspired by this hero, as her hagiography relates, she aspired to 'serve the people'. She was awarded the accolade of model worker for being a 'reliable successor of the revolution' and for that reason was chosen to attend the part-time village health worker training. After four months of training, she returned to her village in Chuansha County, Jiangsu province. In addition to attending to the villagers' health needs and leading local public health work, Wang worked barefoot in the paddy field like other villagers. Villagers nicknamed her 'Barefoot Doctor'. In the beginning, the brigade cadre admonished the villagers: 'Guizhen is our Lei Feng style model worker. Don't nickname her Barefoot Doctor!' But he was wrong. Three years later, at the height of the Great Proletarian Cultural Revolution, Wang Guizhen would become a national hero just like Lei Feng, and Chairman Mao would turn the nickname 'Barefoot Doctor' into a revolutionary brand.

国家要求至少配备一个医疗队 / 一名卫生员和一名新式助产士。但志麦不想成为一名卫生员或助产士; 她想学习如何织布, 而不是如何打针, 因为她害怕打针。此外, 在她的村庄里, 有特殊的生育仪式, 许多村民认为中华人民共和国公共卫生部门提倡的新式医疗分娩是禁忌。最后, 大队领导强迫她参加了培训课程。在培训期间, 她无法面对检查阴道的不同部位; 她感到羞愧, 在第一次实践培训时, 她非常害怕, 身体不停地颤抖。她汗流浹背, 以至于班上有人不得不帮她擦脸。此外, 由于参加培训的人太多, 她几乎没有机会练习所学的东西。然而, 在只接受了两次分娩培训后, 她就被派回村里担任一线卫生员。⁹²

尽管最初存在许多缺点, 但随着 1966 年夏季开始的文化大革命的到来, 农村乡村卫生员计划的政治重要性呈指数级增长。1974 年, 王桂珍成为中华人民共和国的 “模范赤脚医生”, 代表她的国家在第二十七届世界卫生大会上就中国的赤脚医生计划发表演讲。1963 年, 当 “学习雷锋” 运动首次开始时, 她正值二十多岁。正如她的圣徒传记所说, 她受到这位英雄的启发, 渴望 “为人民服务”。她因成为 “革命的可靠接班人” 而被授予模范工作者的荣誉, 因此被选去参加兼职乡村卫生员培训。经过四个月的培训, 她回到了江苏省川沙县的村庄。除了满足村民的健康需求和领导当地的公共卫生工作外, 王桂珍还像其他村民一样赤脚在稻田里劳作。村民们给她起了个绰号 “赤脚医生”。起初, 大队干部告诫村民: “桂珍是我们雷锋式的模范工作者。不要给她起绰号叫赤脚医生!” 但他错了。三年后, 在文化大革命的高潮时期, 王桂珍将像雷锋一样成为全国英雄, 毛主席将把 “赤脚医生” 这个绰号变成一个革命品牌。

THE BAREFOOT DOCTOR AS A
'NEWLY EMERGED REVOLUTIONARY THING'

In the final months of 1964, approaching the end of the Year of Dragon – the Dragon being the Year of Upheaval in the Chinese calendar – Mao had become increasingly dissatisfied with the results of the Socialist Education Campaign. He had also become weary of how Liu Shaoqi was using the campaign to overshadow his leadership. In his last attempt to prove to the socialist world that he alone was at the vanguard of the proletarian revolution fighting revisionism, in the summer of 1966 the aged Mao went for another symbolic swim in the Yangtze River and then launched the Great Proletarian Revolution. He began by delegating the task of purging those he considered undesirable elements in the higher level of power to young radicals – namely, university and high and middle school students, who became known as the Red Guards. In the next months, unprecedented chaos erupted across the country. China was on the verge of a civil war.⁹³ In the summer of 1968, Mao lost his patience and blamed the Red Guards for personally letting him down by destroying his revolution.

The problem with the Red Guards, according to Mao, was that the schools and universities continued to be dominated by the 'bourgeois intellectuals'. To liquidate the intellectuals and end their domination at schools and colleges, he brought in workers with the support of the army. He mandated sole power to them to bring about an educational revolution and transform the intellectuals' world view. They would build an 'army of proletarian intellectuals'. He also urged schools and colleges to be filled with students 'selected from workers and peasants with practical experience. After their study at school for several years they should return to practical production.'⁹⁴ In the countryside, Mao delegated the task of carrying out his educational revolution to the poor and middle (class) peasants – 'the steadfast ally of the working class'. On the basis of two investigative reports from his chief polemicist Yao Wenyuan, one of which was about the Barefoot Doctors in Chuansha County's Jiangzhen Commune, just outside of Shanghai, he launched the 'education revolution' in the countryside: 'The majority of the students trained in the old schools and colleges can integrate themselves with workers, peasants, and soldiers, and some have made changes and innovations; they must, however, under the guidance of the correct line,

“赤脚医生”作为一种“新生事物”

在1964年的最后几个月，临近中国历法中象征着动荡之年的龙年年末，毛泽东对社会主义教育运动的成果越来越不满意。他还厌倦了刘少奇利用这场运动来掩盖他的领导地位。1966年夏天，为了向社会主义世界证明他独自站在反对修正主义的无产阶级革命的最前沿，年迈的毛泽东再次象征性地畅游长江，并发起了伟大的无产阶级文化大革命。他首先将清除他认为高层权力中不良分子的任务委托给年轻的激进分子——即大学、高中和中学的学生，他们后来被称为红卫兵。在接下来的几个月里，前所未有的混乱席卷了全国。中国正处于内战的边缘。⁹³ 1968年夏天，毛泽东失去了耐心，指责红卫兵摧毁了他的革命，让他们个人让他失望。

毛认为，红卫队的问题在于，学校和大学仍然由“资产阶级知识分子”主导。为了清除知识分子并结束他们在学校和学院的统治，他在军队的支持下引进了工人。他授权他们拥有唯一的权力，以带来一场教育革命，并改变知识分子的世界观。他们将建立一支“无产阶级知识分子大军”。他还敦促学校和学院招收“从有实践经验的工人和农民中选拔的学生。他们在学校学习几年后，应该回到实际生产中。”⁹⁴ 在农村，毛将开展教育革命的任务委托给了贫农和中农——“工人阶级坚定的盟友”。根据他的主要论战家姚文元所做的两份调查报告，其中一份是关于上海郊外川沙县江镇公社的赤脚医生的，他在农村发动了“教育革命”：“在旧学校和学院培养的大部分学生都能与工农兵相结合，有些人已经做出了改变和创新；但是，他们必须在正确路线的指导下，

be re-educated by workers, peasants, and soldiers and thoroughly change their old ideology. Such intellectuals will be welcomed by the workers, peasants, and soldiers. Is there anyone who doesn't believe me? Look at the case of the Barefoot Doctor at Chuansha County's Jiangzhen Commune,' Mao proclaimed. Despite the title of the report, 'The Orientation of the Revolution in Medical Education as Seen in the Growth of "Barefoot Doctors"', Mao made clear that the true goal was the revolutionary transformation of the entire education system. Following the chairman's instruction, Yao published the report, accompanied by the editorial 'On the Re-education of Intellectuals', in the 10 September (1968) edition of the Party's *Red Flag* magazine.⁹⁵

When Yao Wenyuan brought the report about the Barefoot Doctors to the chairman, Yao was also seeking to eliminate Wei Wenbo's influences in the health departments in Shanghai and eastern China. Wei was one of the chief opponents of the Cultural Revolutionary Group in Shanghai. After Ke Qingshi died a premature death in 1965, Wei succeeded him as the party boss of the eastern China region with additional responsibility to oversee the region's public health work, including the Anti-Schistosomiasis Campaign. Wei had been Ke's deputy for the campaign. From the onset of the Great Proletarian Cultural Revolution, Wei had not seen eye-to-eye with Zhang Chunqiao, the former head of propaganda for eastern China, and Yao Wenyuan. Both Zhang and Yao would become powerful leading figures of the Cultural Revolution Group. In autumn 1966, Wei intervened to prevent the Red Guards from Beijing going to hospitals in Shanghai to cause chaos, but his efforts would eventually fail. By the end of 1966, with the support of Zhang Chunqiao and Yao Wenyuan, Shanghai's Cultural Revolution rebels, led by Wang Hongweng, who would also become a member of the Cultural Revolution Group, ransacked Shanghai and seized power. Wei Wenbo was removed from office and publicly denounced for being a 'traitor' and the 'agent of China's Khrushchev Liu Shaoqi'.^{*} But the purge did not stop there. 'Wei is a traitor. The urgent task is to clean the ranks,' Zhang urged. For the next months and years, the ongoing campaign to expose Wei's crimes continued to allow the rooting out of the 'traitor's' influences, as well as those of 'secret

* In August 1966, Liu Shaoqi was denounced as the number one revisionist in the Communist Party. He was put under house arrest.

接受工人、农民、士兵的再教育，彻底改造旧思想。这样的知识分子会受到工人、农民、士兵的欢迎。有人不相信我吗？看看川沙县江镇公社的赤脚医生吧，”毛泽东宣称。尽管报告的标题是《从“赤脚医生”的成长看医学教育革命的方向》，但毛泽东明确表示，真正的目标是整个教育体系的革命性改造。根据主席的指示，姚文元在9月10日（1968年）的党的红旗杂志上发表了这份报告，并附有社论《论知识分子的再教育》。⁹⁵

姚文元将关于赤脚医生的报告带给主席时，姚文元也在寻求消除魏文伯在上海和华东地区卫生部门的影响。魏是上海文化革命小组的主要反对者之一。柯庆施于1965年过早去世后，魏接替他担任华东地区党的负责人，并额外负责监督该地区的公共卫生工作，包括血吸虫病防治运动。魏一直是柯的运动副手。从无产阶级文化大革命开始，魏文伯就与华东地区的原宣传部长张春桥和姚文元意见不合。张和姚都将成为文化革命小组的强大领导人物。1966年秋，魏出面阻止来自北京的红卫兵前往上海的医院制造混乱，但他的努力最终失败了。到1966年底，在张春桥和姚文元的支持下，由王洪文领导的上海文化革命造反派（王洪文也将成为文化革命小组的成员）洗劫了上海并夺取了政权。魏文伯被撤职，并因是“叛徒”和“中国赫鲁晓夫刘少奇的代理人”而受到公开谴责。但清洗并没有就此停止。“魏是叛徒。当务之急是清理队伍，”张敦促道。在接下来的几个月和几年里，持续进行的揭露魏罪行的运动继续允许清除“叛徒”的影响，以及“秘密

* 1966年8月，刘少奇被谴责为党内头号修正主义分子，并被软禁。

agents' in different government departments in Shanghai and eastern China – from art to education and health. By rooting out the revisionist influences and their agents in these government institutions, the Cultural Revolution Group would also fill them with 'fresh new blood'. The report about the Barefoot Doctors in Jiangzhen Commune thus opened by condemning the 'agent of China's Khrushchev in Shanghai's public health' who 'unscrupulously pushing a counter-revolutionary revisionist line in medical and public health work, made a malicious report viciously slandering the health workers in the production brigades and ordering them to drop their medical work. The new-type public health force created in the period of the Great Leap Forward in the rural areas was thus destroyed; the number of health workers, originally over 11,900, was slashed to just over 300.' On other occasions, Wei and Liu Shaoqi were both accused of pushing their revisionist line in health work, and it was claimed that their 'revisionist line' was responsible for the failure of the Anti-Schistosomiasis Campaign, even though Liu had had little direct involvement in the PRC's public health work. To fight against the 'revisionist line in medical and public health work', the report quoted revised excerpts from the memo of Mao's conversation with Dr Li Zhishui: 'On 26 June 1965, Chairman Mao issued his brilliant instruction: "In medical and health work, put the stress on the rural areas."' Mao was credited with giving birth to the Barefoot Doctors. With the growing number of Barefoot Doctors, the report trumpeted that the 'health protection network was thus developed all around'. The report went further by proclaiming that 'with the Great Proletarian Cultural Revolution, many more Barefoot Doctors have emerged. Tempered in the storms of class struggle and the struggle between the two lines, this force has expanded and been consolidated and made much progress.'⁹⁶ The Cultural Revolution subsumed the Barefoot Doctor program, but in the meantime, because a huge number of experts and public health officials had been purged following Wei Wenbo's downfall, there was no one to lead the schistosomiasis control work in eastern China. In Chuansha, the Barefoot Doctor model county, all grassroots schistosomiasis stations were closed.

* In the beginning of the Cultural Revolution, this memo was seized by the Revolutionary Committee for All Medical and Health Institutions in Beijing (北京医药卫生界大联合革命委员会). The committee subsequently published it in one of the Great Proletarian Cultural Revolution pamphlets, *Red Medical Battle Bulletin* (红医战报), in their attack on the Ministry of Health.

在上海和华东地区的不同政府部门（从艺术到教育和卫生）中清除“代理人”。通过清除这些政府机构中的修正主义影响及其代理人，文化革命小组还将用“新鲜血液”填补他们。因此，关于江镇公社赤脚医生的报告首先谴责了“中国赫鲁晓夫在上海公共卫生领域的代理人”，他“肆无忌惮地在医疗和公共卫生工作中推行反革命修正主义路线，恶意诽谤生产队中的卫生工作者，并命令他们放弃医疗工作。大跃进时期在农村地区建立的新型公共卫生力量因此被摧毁；卫生工作者的人数最初超过11,900人，被削减到300多人。”在其他场合，魏文伯和刘少奇都被指责在卫生工作中推行修正主义路线，并声称他们的“修正主义路线”应对血吸虫病防治运动的失败负责，尽管刘文伯几乎没有直接参与中华人民共和国的公共卫生工作。为了反对“医疗和公共卫生工作中的修正主义路线”，该报告引用了毛泽东与李志绥医生谈话备忘录的修订摘录：“1965年6月26日，毛主席发表了光辉指示：‘在医疗卫生工作中，要把重点放在农村。’”^{*}毛泽东被认为是赤脚医生的缔造者。随着赤脚医生人数的增加，该报告宣称“卫生保护网络因此得以全面发展”。该报告进一步宣称，“随着伟大的无产阶级文化大革命，涌现出更多的赤脚医生。这支队伍在阶级斗争和两条路线斗争的风暴中锻炼，不断壮大、巩固并取得了很大进展。”⁹⁶文化大革命将赤脚医生计划纳入其中，但与此同时，由于魏文伯垮台后大量专家和公共卫生官员遭到清洗，没有人领导华东地区的血吸虫病控制工作。在赤脚医生模范县川沙，所有基层血吸虫病防治站都被关闭。

* 在文化大革命初期，这份备忘录被北京医药卫生界大联合革命委员会查抄。该委员会随后在其攻击卫生部的《无产阶级文化大革命》小册子之一《红医战报》中发表了这份备忘录。

The Barefoot Doctors had replaced the bourgeois experts in delivering the people's health.⁹⁷

Four days after its first appearance in the *Red Flag* magazine on 10 September 1968, the same report about the Barefoot Doctors of the Jiangzhen Commune appeared on the front page of the *People's Daily* and then a week later in the English-language magazine *Peking Review*, the PRC's official propaganda tool targeting the outside world.⁹⁸ In October, at the Eighth Central Committee's Twelfth Plenary Session, Liu Shaoqi was publicly condemned as a 'renegade, traitor, and scab' and expelled from the party. At the same meeting, it was also proposed that the middle-level 'revolutionary intellectuals' such as Barefoot Doctors and school teachers would be nominated to participate in the National People's Congress, replacing bourgeois intellectuals. Having been reconfigured as a 'newly emerged thing (新生事物) that grew out of class struggle' as well as the struggle between revolutionary and revisionist lines, the rural, grassroots, part-time health worker training program took on a new political life. From then on it became universally known as the 'Barefoot Doctor' program: Mao's 'revolutionary approach to health'.

Health care had been a political keystone in the rhetoric of the founding of the PRC. By the 1960s, the state had moved to a greater if somewhat uneven ability to achieve actual health improvements over the primarily propagandistic claims of previous periods. In spite of the collapse of the centralized health system as a result of the famine, the Great Proletarian Cultural Revolution meant that the CCP under Mao's leadership had a more secure position, and economic development was beginning to become more consistent. The absence of any political alternatives to the state, especially after the official break with the USSR after 1962, meant that conflicts and inconstancies were seen as problems to be resolved rather than calls for the alteration or abandonment of the Maoist utopian goals. Health care remained in all cases a central concern of individuals, party, and state.

赤脚医生已经取代了资产阶级专家，负责人民的健康。⁹⁷

1968年9月10日，在《红旗》杂志首次刊登四天后，同一篇关于江镇公社赤脚医生的报道出现在《人民日报》的头版，一周后又出现在英文杂志《北京周报》上，这是中国面向外部世界的官方宣传工具。⁹⁸ 10月，在第八届中央委员会第十二次全体会议上，刘少奇被公开谴责为“叛徒、内奸、工贼”，并被开除出党。在同一次会议上，还提议提名赤脚医生和学校教师等中层“革命知识分子”参加全国人民代表大会，取代资产阶级知识分子。农村基层兼职卫生工作者培训计划被重新定义为“阶级斗争中涌现出的新生事物（新生事物）”，以及革命路线和修正主义路线之间的斗争，从而获得了新的政治生命。从那时起，它就被普遍称为“赤脚医生”计划：毛泽东“革命的卫生方法”。

医疗保健一直是中华人民共和国成立之初政治宣传中的一块基石。到了20世纪60年代，国家在实现实际的健康改善方面，即使是不平衡的，也比以往主要是宣传性的说法有了更大的能力。尽管由于饥荒导致了中央卫生系统的崩溃，但无产阶级文化大革命意味着在毛泽东领导下的中国共产党拥有更稳固的地位，经济发展也开始变得更加稳定。在1962年与苏联正式决裂后，由于没有任何政治选择可以替代国家，这意味着冲突和不一致被视为需要解决的问题，而不是要求改变或放弃毛泽东的乌托邦目标。在任何情况下，医疗保健仍然是个人、党和国家的核心关切。

Delivering Rural Health: From the State to Communities

RE-COLLECTIVIZATION AND THE CO-OPERATIVE MEDICAL SERVICE

Despite the disastrous result of the Great Leap Forward, Mao continued to hold on to the view that substituting labour for capital was the key to achieving rapid ‘socialist transformation’ in China.¹ In the aftermath of the famine resulting from the Great Leap and of the ‘Big Brother’ Soviet Union cancelling its aid and withdrawing from major development projects, Mao evoked the spirit of the ‘Foolish Old Man Who Removed the [Taihang and Wangwu] Mountains’ at the Dazhai Commune and enshrined the ‘Spirit of Dazhai’. Located at the foothills of the Taihang Mountain in the impoverished corner of Shanxi province, villagers at the Dazhai People’s Commune had supposedly transformed arid earth into fertile agricultural fields without any aid from the state. Setting Dazhai as a model, Mao once more called upon the people’s power, according to him China’s most valuable asset and the ‘only motivating force in history’.² In December 1964, the entire

* ‘The Foolish Old Man Who Removed the Mountains’ was originally an ancient fable from the body of Daoist texts known as *Liezi*. Mao first evoked the ‘Spirit of the Foolish Old Man Who Removed the Mountains’ in 1945, during the Chinese Civil War, in his concluding speech at the Seventh National Congress of the Communist Party of China (CCP). For Mao and the CCP, ‘imperialism’ and ‘feudalism’ were the two major obstacles – the two mountains – they needed to remove in order to achieve their revolutionary goal. He urged the CCP to unite with the masses of the Chinese people – the ‘God’ in this case – to fight against imperialism and feudalism. In 1958, at height of the Great Leap Forward, he again advocated ‘the high spirit, strong morale and firm determination’ that, according to him, had made ‘the high mountain bow its head and the river yield the way’.

提供农村医疗卫生服务：从 国家到社区

重新集体化和 合作医疗服务

尽管大跃进造成了灾难性的后果，但毛仍然坚持认为，以劳动力替代资本是中国实现快速“社会主义改造”的关键。¹ 在大跃进造成的饥荒之后，以及“老大哥”苏联取消援助并撤出重大发展项目之后，毛泽东唤起了大寨公社“愚公移[太行、王屋]二山”的精神，并将“大寨精神”奉为圭臬。大寨人民公社位于山西省贫困地区的太行山脚下，据说那里的村民在没有任何国家援助的情况下，将贫瘠的土地变成了肥沃的农田。毛泽东以大寨为榜样，再次呼吁人民的力量，他认为这是中国最宝贵的财富和“历史上唯一的动力”。² 1964年12月，全体

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population of the countryside was called to 'learn from Dazhai' by practising 'self-reliance' in order to transform their lives with their bare hands. This simultaneously led to a process of devolution in rural health care delivery as villagers across China were left to their own devices to come up with the means to pay for the state-guaranteed social welfare, from education to health care.

Partially catalyzed by the official 'To Learn from Dazhai' campaign, but, even more importantly, driven by villagers and many grassroots cadres' desire to improve daily life, across different parts of rural China – from Hubei province on the bank of Yangtze River, to Shanxi province in the arid north, Sichuan province in the mountainous southwest, and the suburbs of Beijing – communes and brigades came up with similar but not identical schemes to help finance the costs of villagers' medical and health care. In northern Sichuan's Langzhong County, for instance, after the collapse of the commune health system some doctors from the former Baishan commune hospital used the social capital of the community to service villagers. With money donated by villagers, they purchased basic drugs and rented a small room where they opened the Baishan United Clinic. Partly to reduce the cost of drugs but equally in order to comply with the villagers' preference for folk herbal remedies, these doctors also sought out traditional formulas. To utilize local resources as well as to engage the community, the doctors and the commune jointly ran training courses to empower individual villagers to become caregivers for their own village. Villagers welcomed this initiative: it saved them the hassle of having to travel to town hospitals or clinics as well as money on drugs. Equally important, if not more so, they were more comfortable with traditional remedies as well as caregivers in their own communities who were familiar to them and whom they trusted. Many villagers also volunteered in their spare time to help with the refurbishing of the clinic room as well as constructing beds and other furniture for the clinic.² Similarly, in Xier Hedao Commune in the suburb of Beijing and in Xingao Commune in Shanxi province's Dai County, doctors from commune clinics – calling on their own knowledge of folk medicine as well as their desire to reduce the cost of treatment – regularly went into the hills to collect locally grown medicinal herbs.³ Yet it was the Leyuan (which literally translates as 'Happy Garden') Commune's Co-operative Medical service model that fit best with the overall 'revolutionary' agenda of the time, and it was this model that was subsequently rolled out as a national program.

农村人口被号召“学习大寨”，通过实行“自力更生”，用自己的双手改变生活。与此同时，农村医疗保健服务的权力下放进程也开始了，中国各地的村民不得不靠自己的力量来筹集资金，以支付国家保障的社会福利，从教育到医疗保健。

在官方“学习大寨”运动的部分推动下，但更重要的是，在村民和许多基层干部的改善日常生活的愿望驱动下，中国农村的不同地区——从长江沿岸的湖北省，到干旱北方的山西省，再到西南山区的四川省，以及北京郊区——公社和大队提出了类似但不完全相同的计划，以帮助支付村民的医疗保健费用。例如，在川北阆中县，在公社卫生系统崩溃后，原白山公社医院的一些医生利用社区的社会资本为村民提供服务。他们用村民捐赠的钱购买基本药物，并租了一个小房间，在那里开设了白山联合诊所。部分是为了降低药品成本，但也同样为了满足村民对民间草药疗法的偏好，这些医生还寻找传统配方。为了利用当地资源并让社区参与进来，医生和公社联合举办培训课程，以授权村民成为自己村庄的护理人员。村民们欢迎这项举措：它为他们省去了去城镇医院或诊所的麻烦，也节省了药费。同样重要的是，如果不是更重要的话，他们更习惯于传统疗法以及自己社区中他们熟悉和信任的护理人员。许多村民还利用业余时间自愿帮助翻新诊所房间，以及为诊所建造床和其他家具。² 同样，在北京郊区的西二合道公社和山西省代县的兴高公社，公社诊所的医生——凭借他们自己的民间医学知识以及降低治疗成本的愿望——经常进山采摘当地生长的药草。³ 然而，乐园（字面意思是“快乐花园”）公社的合作医疗服务模式最符合当时的总体“革命”议程，随后该模式作为一项国家计划推广开来。

Already, in October 1959 at the height of the Great Leap Forward, in a conversation with a Polish delegation, Mao boasted about the advantages of the collective economy, more specifically the People's Commune in the People's Republic of China (PRC): 'With the People's Commune, we have achieved things which were unattainable [under the private economy], from commune-run reservoirs to factories, schools, and nurseries.'⁴ As of the summer of 1968, in the midst of the Great Proletarian Cultural Revolution and assisted by revolutionary committees in the countryside, Mao waged a 'revolutionary' war to revert the rural economy back to collectivization. To re-collectivize the rural economy entailed abolishing private plots as well as removing from individual villages – the production brigade, in this case – the autonomy to manage their own production and labour. Accordingly, the aim was to foster villagers' collective spirit to 'gradually bring about a complete break with the old concept of private ownership', thus 'realizing the goal of communism'. This included building or reinstating the collective welfare system, from health to education.⁵ Under such conditions, during the last quarter of 1968 and the first quarter of 1969 Mao's chief polemicist Yao Wenyuan orchestrated the appearance in the *People's Daily* of a series of articles praising the collective economy. Among them was the report (published in the newspaper on 5 December 1968) about how the Leyuan Commune had achieved free health and medical care for all the villagers by organizing a Co-operative Medical service. According to the official version of the Leyuan story, in 1966, as part of the Great Proletarian Revolution fighting against the revisionist power over health and medicine, villagers at the Leyuan Commune in Hubei province's Changyang County organized a Co-operative Medical service to provide their own inexpensive and effective medical care. To join the service, each villager contributed one *yuan* (RMB) each year toward the collective medical fund. For each contributing villager, their production brigade also added 10 per cent toward the fund. By joining the Co-operative Medical service, villagers would enjoy free medical care, ranging from therapy to drugs. As with the village health worker scheme, this initially unique, grass-roots innovation was reconfigured as another 'newly emerged thing' that the Great Proletarian Revolution brought to health work.⁶ In the next months, the Rural Co-operative Medical System (合作医疗 RCMS) was implemented throughout rural China. It became the only approved system for rural health care delivery between 1968

早在 1959 年 10 月，在大跃进的高潮中，毛泽东在与波兰代表团的一次谈话中，就吹嘘集体经济，更具体地说是中华人民共和国的人民公社的优势：“有了人民公社，我们取得了在私有经济下无法实现的成就[]，从公社经营的水库到工厂、学校和托儿所。”⁴截至 1968 年夏天，在大无产阶级文化大革命期间，在农村革命委员会的协助下，毛泽东发动了一场“革命性”战争，将农村经济恢复到集体化。农村经济的再集体化需要废除自留地，并取消各个村庄（在本例中为生产队）管理自身生产和劳动的自主权。因此，目标是培养村民的集体精神，以“逐步彻底打破旧的私有观念”，从而“实现共产主义的目标”。这包括建立或恢复从卫生到教育的集体福利制度。⁵在这种情况下，在 1968 年第四季度和 1969 年第一季度，毛泽东的主要论战家姚文元策划了一系列文章在《人民日报》上发表，赞扬集体经济。其中一篇报道（1968 年 12 月 5 日在报纸上发表）讲述了乐园公社如何通过组织合作医疗服务，为所有村民实现免费医疗。根据乐园故事的官方版本，1966 年，作为反对卫生和医药领域修正主义势力的大无产阶级革命的一部分，湖北省长阳县乐园公社的村民组织了一项合作医疗服务，以提供他们自己廉价而有效的医疗服务。为了加入该服务，每位村民每年向集体医疗基金捐款一元（人民币）。对于每位捐款的村民，他们的生产队也向该基金额外捐款 10%。通过加入合作医疗服务，村民将享受从治疗到药物的免费医疗。与乡村卫生员计划一样，这种最初独特的基层创新被重新配置为大无产阶级革命给卫生工作带来的另一个“新事物”。⁶在接下来的几个月里，农村合作医疗制度（合作医疗 rcms）在整个中国农村实施。它成为 1968 年至 1968 年间农村医疗保健服务的唯一批准系统

and 1983. Villagers in rural China, many living in extreme poverty, were forced to help finance the Co-operative Medical service as well as the official Barefoot Doctor program.⁷ Anyone who resisted or doubted the efficacy of Co-operative Medicine and the Barefoot Doctors would be charged with the political offence of 'not following Chairman Mao's revolutionary line' or sabotage. And sabotage brought draconian punishment.

The official Co-operative Medical service and the Barefoot Doctor program, analogous to the Anti-Schistosomiasis Campaign, were enacted at the centre of political power. In the process of being implemented, they frequently evoked a variety of responses from below that were often at odds with one another, given the sheer size and vast diversity of the country. When these conflicts were significant enough, they could lead to the non-implementation or the complete alteration of the official program. Although official accounts depicted the Barefoot Doctors and Co-operative Medical service in a highly favourable light, I have uncovered evidence from previously unseen archival documents from nine different provinces, as well as through conducting oral interviews with former Barefoot Doctors, villagers, local cadres, and health officials and experts, that suggest that the reality on the ground was often radically different from the official account.^{*} Indeed, any positive or negative outcome of the Co-operative Medical service, as well as the perseverance (or lack of it) of the Barefoot Doctors, had much more to do with multifaceted factors on the ground than on the government policy superimposed from above. Often, locally based health workers and their cadre, as well as the villagers, were unaffected by the revolutionary vision of Mao and the Communist Party. While many were quick at discerning the advantages or disadvantages of the newly introduced health services, they did so on their own terms. In the process of implementation, these local actors regularly circumvented the official program to accommodate for local conditions and their own needs or to fit official

* These interviews were conducted by myself and others at the Institute of Health at the Kunming Medical University and Yunnan Health and Development Research Association (YHDRA) between 2000 and 2016. The interviews conducted by the team at the YHDRA are available in Zhang Kaining, Wen Yiqun, and Liang Ping, eds, *From Barefoot Doctors to Village Doctors* (Kunming: Yunnan Renmin chubanshe 2002). The English translations are by me.

和 1983 年。中国农村的村民，许多人生活在极端贫困中，被迫帮助资助合作医疗服务以及官方的赤脚医生计划。⁷ 任何抵制或怀疑合作医疗和赤脚医生效力的人，都会被指控犯有“不遵循毛主席革命路线”或破坏的政治罪行。而破坏行为会受到严厉的惩罚。

官方的合作医疗服务和赤脚医生计划，类似于血吸虫病防治运动，是在政治权力的中心颁布的。在实施过程中，鉴于这个国家幅员辽阔、差异巨大，它们经常引起来自下级的各种各样的反应，这些反应往往彼此不一致。当这些冲突足够显著时，它们可能导致官方计划无法实施或完全改变。虽然官方的说法将赤脚医生和合作医疗服务描绘得非常有利，但我从来自九个不同省份的以前未公开的档案文件中，以及通过对以前的赤脚医生、村民、当地干部和卫生官员和专家的口头采访中，发现了一些证据，表明实际情况往往与官方的说法截然不同。^{*} 事实上，合作医疗服务的任何积极或消极的结果，以及赤脚医生的坚持（或缺乏坚持），更多地与当地的多种因素有关，而不是与政府从上而下强加的政策有关。通常，当地的卫生工作者及其干部，以及村民，并不受毛泽东和共产党的革命愿景的影响。虽然许多人很快就能辨别出新引进的卫生服务的优点或缺点，但他们是按照自己的方式做的。在实施过程中，这些当地参与者经常规避官方计划，以适应当地的情况和他们自己的需要，或适应官方的需求。

* 这些访谈由我和昆明医科大学卫生研究所及云南省健康与发展研究会（yhdra）的其他人员在 2000 年至 2016 年间进行。云南省健康与发展研究会团队进行的访谈可在张开宁、温一 qun 和梁平编著的《从赤脚医生到乡村医生》（昆明：云南人民出版社，2002 年）中找到。英文翻译由我完成。

policy into existing local schemes. As one former Barefoot Doctor from Shanxi province stated: ‘Indeed, there was official policy regarding the [Barefoot Doctors]. When being implemented, many aspects of the policy were ignored. What has been implemented on the ground resembled nothing of what is said in the official documents.’⁸ Although many of the previously existing grassroots health programs were rebranded as the Barefoot Doctor program and were then financed by what became universally known as the Co-operative Medical service, they remained specific to their locality and were noticeably different from those in other parts of China. In addition, folk healers ranging from witch doctors to exorcists and other ritual healers (most of them operating on their own) coexisted alongside the Barefoot Doctors in supporting sick villagers and their families through prayer and exorcism as well as in community and domestic healing rituals.

Social scientists have examined in detail the strengths and weaknesses of the policy design of the Maoist official health care.⁹ However, we know very little about how both the Co-operative Medical service and the Barefoot Doctor program were carried out in practice and how they were experienced on the ground. By shifting our focus from the policy at the top to the diverse aspects of support, accommodation, and resistance on the ground during the process of implementation, we can get much closer to the pulse of everyday life. In doing so, we can better understand complex and often strikingly dissimilar experiences of a whole range of individuals struggling to cope with illness and disease when the state system failed to deliver the promise of ‘health care for all’, since such a system – with state support in addition via the county hospital – was delivering on the promise, with the state as the enabler if not the source of finance. By exploring how state health ideologies were communicated, circumvented, or sometimes simply ignored on the ground, we can see how the world views of not only the villagers but also of those grassroots cadres in charge of policy implementation and the caregivers – the Barefoot Doctors in this case – who were officially responsible for health delivery affected the actual outcome of the health of individuals and communities. This grassroots data is essential, for it provides an insight much more effectively than repeating the claims formulated in the central health policy as promulgated by the state.

将政策融入现有的地方计划中。正如山西省一位前赤脚医生所说：

“的确，有关于[赤脚医生]的官方政策。但在实施过程中，政策的许多方面都被忽视了。在基层实施的措施与官方文件中所说的毫无相似之处。”⁸ 尽管许多先前存在的基层卫生项目被重新命名为赤脚医生项目，并由后来普遍被称为合作医疗服务的机构资助，但它们仍然特定于其所在地，并且与其他地区的项目明显不同。此外，从巫医到驱魔师和其他仪式治疗师（他们中的大多数人独自运营）的民间医生与赤脚医生并存，通过祈祷和驱魔以及社区和家庭治疗仪式来支持生病的村民及其家人。

社会科学家已经详细研究了毛泽东时代官方医疗保健政策设计的优势和劣势。⁹ 然而，我们对合作医疗服务和赤脚医生计划在实践中是如何进行的，以及它们在基层是如何被体验的知之甚少。通过将我们的重点从顶层的政策转移到实施过程中基层支持、适应和抵制的各个方面，我们可以更接近日常生活的脉搏。通过这样做，我们可以更好地理解各种各样的人在国家系统未能兑现“人人享有医疗保健”的承诺时，努力应对疾病的复杂且通常截然不同的经历，因为这样一个系统——在国家的支持下，通过县医院——正在兑现这一承诺，国家是推动者，即使不是资金的来源。通过探索国家卫生意识形态是如何被传达、规避，或者有时只是在基层被忽视的，我们可以看到不仅村民的世界观，而且负责政策实施的基层干部和护理人员——在本例中为赤脚医生——他们对卫生服务的实际结果产生了影响，影响了个体和社区的健康。这种基层数据至关重要，因为它比重复国家颁布的中央卫生政策中提出的主张，更能有效地提供深入的见解。

THE 'DAWDLING' CO-OPERATIVE MEDICAL SERVICE
AND INEQUALITIES IN THE PEOPLE'S COMMUNE

After agricultural collectivization was relaunched in the Chinese countryside in the late 1960s during the Great Proletarian Cultural Revolution, villagers once more lost their enthusiasm for agricultural work. 'Sweating in the private plots, while dawdling in the collective fields' was a popular saying in Shanxi province at the time. Here, in the arid land where the 'Spirit of Dazhai' was born, more villagers were seen spending their time idling when working in the collective fields.¹⁰ In S's village at the foothills of the Great Wall, the widespread problem of slacking, taken together with the chronic drought, meant the village was too poor to generate sufficient income to finance the Co-operative Medical service. Nevertheless, the brigade authority was determined to transform itself into a Co-operative Medicine model: 'To turn our village into a model, the whole village were involved. The big brigade sent us lots of medical equipment to make it look good,' S recalled. This Co-operative Medicine 'model' turned out to be a 'Potemkin's village': for many years, the village health centre remained an empty, dark room with no drugs, staffed by S, the Barefoot Doctor who had no basic medical training. It was only after 1978, when villagers were given the sole responsibility for making a profit or loss, that they were able to generate sufficient income to finance the Co-operative Medical service. Only then was the village health centre able to stock up on drugs, and it was only then that S received his basic training in medicine and care. To earn a livelihood for himself so that he could continue caring for the villagers, S admitted he had to introduce various charges for drugs and services because otherwise 'Co-operative Medical service would never work'. He was not ambiguous about this at all.¹¹

Further south in Shanxi province's agricultural heartland, the Co-operative Medical service at Qin County's Koutou village ended after only a single year. It was economically unviable, and the villagers did not want it. At the time, Koutou village had about 1,000 inhabitants. According to the policy, to join the service each villager would contribute one *yuan* per year. Together, the village Co-operative Medicine fund generated just over 2,000 *yuan* annually to cover the villagers' medical costs. But the village cadres and

人民公社中“慢吞吞”的合作医疗服务与不平等现象

在 20 世纪 60 年代末大无产阶级文化大革命期间，中国农村重新启动农业集体化后，村民们再次失去了对农业工作的热情。“在自留地上挥汗如雨，在集体地里磨蹭”是当时山西省的一句流行语。在这里，在“大寨精神”诞生的干旱土地上，更多的村民被看到在集体土地上工作时无所事事。¹⁰在 S 村，位于长城脚下，普遍存在的偷懒问题，加上长期干旱，意味着该村太穷，无法产生足够的收入来资助合作医疗服务。尽管如此，大队当局决心把自己改造成合作医疗模式：“为了把我们的村庄变成一个模范，全村都参与了。大队给我们送了很多医疗设备，让它看起来很好，”S 回忆说。这个合作医疗“模范”结果变成了一个“波将金村庄”：多年来，村卫生所仍然是一个空荡荡的黑暗房间，没有药物，由 S 负责，他是没有基本医疗培训的赤脚医生。直到 1978 年之后，村民们被赋予了盈亏的唯一责任，他们才能够产生足够的收入来资助合作医疗服务。只有到那时，村卫生所才能够储备药品，也只有到那时，S 才接受了他的基本医学和护理培训。为了自己谋生，以便他能够继续照顾村民，S 承认他不得不对药物和服务收取各种费用，否则“合作医疗服务永远无法运作”。他对这一点毫不含糊。

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在山西省农业中心地带的更南端，沁县口头村的合作医疗服务仅一年后就结束了。它在经济上不可行，村民也不想要它。当时，口头村大约有 1000 名居民。根据政策，每位村民每年需缴纳一元才能加入该服务。这样，村里的合作医疗基金每年产生略多于 2000 元，以支付村民的医疗费用。但是村干部和

the village accountant worked out very quickly that the 2,000 *yuan* would not go very far if even one or two villagers needed hospital care. With no other funds, and being unwilling or unable to subsidize the Co-operative Medical service, they modified the government policy by slashing the amount of reimbursement for hospital care in half. This meant that seriously ill villagers were denied any necessary treatment, since they could not afford the other half of the hospital fee. Between 1969 and 1970, Koutou village's Co-operative Medical service only funded the cost of village Barefoot Doctors and some vaccinations. After one year, most of the villagers refused to enrol in the Co-operative Medical service, since they saw no personal benefit in doing so. More important, as in the case of both S's village and Koutou village, under the People's Commune system, because individual villagers were denied the opportunity to make a profit by participating in the marketplace, most of them were too poor to afford even the one *yuan* health care premium. 'When I was the village cadre [in the mid-1970s], most villagers were very poor. As such I was unable to collect any money for the Co-operative Medical Fund,' recalled L, the village's former Barefoot Doctor and cadre.¹² Clearly, it was not so much health disparity and the state's lack of resources but rather the collective economic system that disincentivized villagers to engage in the service. How can they be blamed for not looking after their well-being when they had no actual access to basic health and care?

Equally, the political and social hierarchy in the People's Commune gave the village cadre and the Barefoot Doctor enormous power to decide how the Co-operative Medical fund was spent, whether and how to absorb the cost when there were shortages in the fund, and how to determine who had access to the service. The overt abuse of power and misuse of the services were regular features that rendered the universally promised Co-operative Medical service an empty political declaration. In such cases, the health disparity reflected political and social inequality in the People's Commune. On the one hand, even the Rural Co-operative Medical System (the RCMS), as enforced by the welfare state, was unable to protect the most vulnerable villagers from the structural inequality because they lacked social capital, in this case a good relationship with cadres and Barefoot Doctors. On the other hand, the radical re-engineering of the rural health delivery system, even if it seemed a positive

村里的会计很快就算出来, 如果一两个村民需要住院治疗, 2000元根本不够用。由于没有其他资金, 而且不愿意或无法补贴合作医疗服务, 他们修改了政府政策, 将住院费的报销额度削减了一半。这意味着重病村民无法获得任何必要的治疗, 因为他们负担不起另一半的住院费用。1969年至1970年间, 口头村的合作医疗服务只资助了村里的赤脚医生和一些疫苗接种费用。一年后, 大多数村民拒绝加入合作医疗服务, 因为他们认为这样做没有任何个人好处。更重要的是, 正如S村和口头村的情况一样, 在人民公社制度下, 由于村民个人被剥夺了通过参与市场来获利的机会, 他们中的大多数太穷了, 甚至负担不起1元的医疗保险费。“20世纪70年代中期, 我担任村干部[时], 大多数村民都很穷。因此, 我无法为合作医疗基金筹集到任何资金,” 该村前赤脚医生兼干部L回忆说。¹²显然, 这不仅仅是健康差距和国家资源的匮乏, 而是集体经济体制阻碍了村民参与这项服务。当他们无法真正获得基本医疗保健时, 又怎么能责怪他们不关心自己的健康呢?

同样, 人民公社的政治和社会等级制度赋予村干部和赤脚医生巨大的权力, 可以决定合作医疗基金的支出方式、在基金短缺时是否以及如何承担成本, 以及如何决定谁有权获得这项服务。公开滥用权力和滥用服务是常见的现象, 这使得普遍承诺的合作医疗服务成为空洞的政治宣言。在这种情况下, 健康差距反映了人民公社的政治和社会不平等。一方面, 即使是福利国家强制实施的农村合作医疗制度(rcms)也无法保护最脆弱的村民免受结构性不平等的影响, 因为他们缺乏社会资本, 在这种情况下, 他们与干部和赤脚医生没有良好的关系。另一方面, 即使对农村卫生服务体系进行彻底的改造看起来是积极的

innovation at the time, often created social stress as the insecurity led to ever-increasing intolerance.* As a result, instead of sharing resources as the system was designed for them to do, villagers and cadres regularly, and often viciously, competed for resources. At the shore of the Korean peninsula in eastern China, in the Daotian big brigade in Shandong province's Shouguang County (nowadays Shouguang Municipal), a former local cadre recalled that under the RCMS, because the power of managing the Co-operative Medical fund was solely controlled by the cadres of the big brigade, only those who had a good relationship with brigade cadres or Barefoot Doctors had any access to the promised service. Although in theory the RCMS entitled all villagers to an equal amount of reimbursement if they needed hospital care, in practice the amount of reimbursement depended not on the actual cost of hospital care but on whether the villager had a good relationship with the brigade party secretary. Only those who had a rapport with the brigade party secretary could get him to sign the piece of paper that entitled them to claim the cost of their hospital care. 'Ordinary villagers who did not have a good relationship with the party secretary did not even bother to try making a claim. They knew it was a waste of their time. They had no chance of claiming back the medical cost,' this former village cadre explained. With limited amounts of drugs as well as funds, 'ordinary villagers could only get painkillers for free. If they wanted more expensive drugs such as antibiotics, they would have to pay for it out of their own pocket.'¹³ On the other side of the Korean peninsula, in Laoshao County outside of Qingdao, one former village cadre there also admitted that unfairness and abuse of power was fairly common: 'There was a shortage of drugs. Only those who had face [a good relationship with the cadres] had access to good and expensive drugs ... Villagers regularly complained about having no access to medicine, particularly good medicine.' Here, as elsewhere in China, the Barefoot Doctors were nominated by the local cadres. In most cases, they were chosen to be the Barefoot Doctor because they had good relationships with the local cadres and were seen as politically reliable. This had an effect on their service; in most cases, they were

* For centuries, villagers in China had sought medical counsel and obtained medication from fellow villagers who had greater knowledge than they did – from family practitioners, scholars, Daoist or Buddhist priests, village witches, exorcists, and other ritual healers.

当时的创新常常会造成社会压力, 因为这种不安全感会导致越来越严重的不容忍。^{*} 因此, 村民和干部经常 (而且往往是恶毒地) 争夺资源, 而不是像系统设计的那样分享资源。在中国东部朝鲜半岛的海岸, 在山东省寿光县 (现在的寿光市) 的道田大队, 一位前当地干部回忆说, 在合作医疗制度下, 由于管理合作医疗基金的权力完全由大队干部控制, 只有那些与大队干部或赤脚医生关系良好的人才能获得承诺的服务。虽然理论上合作医疗制度赋予所有村民同等金额的医疗费用报销权, 但实际上报销金额并不取决于实际的医疗费用, 而是取决于村民与大队党支部书记的关系是否良好。只有那些与大队党支部书记关系融洽的人才能让他签署那张让他们有权报销医疗费用的纸。这位前村干部解释说: “与党支部书记关系不好的普通村民甚至懒得尝试报销。他们知道这是在浪费时间。他们没有机会报销医疗费用。” 由于药品和资金有限, “普通村民只能免费获得止痛药。如果他们想要更昂贵的药物, 如抗生素, 他们必须自掏腰包。”¹³ 在朝鲜半岛的另一边, 在青岛郊外的崂山县, 一位前村干部也承认, 不公平和滥用权力的情况相当普遍: “药品短缺。只有那些有面子 [与干部关系良好] 的人才能获得好药和贵药……村民经常抱怨无法获得药物, 尤其是好药。” 在中国其他地方, 赤脚医生由当地干部提名。在大多数情况下, 他们被选为赤脚医生是因为他们与当地干部关系良好, 并且被认为是政治上可靠的。这对他们的服务产生了影响; 在大多数情况下, 他们是

* 几个世纪以来, 中国的村民一直向比他们更有知识的村民寻求医疗建议和药物, 这些人包括家庭医生、学者、道教或佛教牧师、乡村女巫、驱魔者和其他仪式治疗师。

more eager to serve the local cadres who appointed them and less inclined to help ordinary villagers. 'At the time, villagers were at the mercy of the Barefoot Doctor. They could not simply request services from the Barefoot Doctors,' the same cadre reflected.¹⁴

Furthermore, the lack of experience and resources of the grass-roots cadres, and the conflicting interests, often meant that village cadres were incompetent in managing the Co-operative Medical service and the Barefoot Doctors program. They were often focused only on immediate gain. In this way, they mirrored the shift in the schistosomiasis control programs from the long range of the prevention projects to the more immediate models of treatment. Their incompetence would increase health inequality, even if this was unintended. As one retired village cadre from Shanxi province's Shiling County, responsible for managing the village health work in the 1970s, reflected: 'Most of us village cadres had little education, and we had no experience in health work. We were simply fumbling in the dark. We had not anticipated there would be so many issues. We only tried to solve them when they caused real problems. Most of them were badly dealt with. There were a lot of problems regarding how to manage money, equipment, and drugs as well as the quality control of Barefoot Doctors. Maybe that's why the Co-operative Medical service collapsed.'¹⁵

In Henan Province's Qingyang County, a government investigative report in 1973 showed that many brigade cadres failed to see the advantage of the Co-operative Medical service. Their view was that 'The Co-operative Medical service would not make any difference. Those villagers who could afford medical costs don't need any help in the first place. For many poor villagers, the Co-operative Medical service would not be much help as it can't [generate enough money to] pay for their total medical cost. It would be better to leave villagers alone to pay for their own medical cost. If they could not afford it, they could then apply for help from the big brigade.'¹⁶ Up until 1975, because of the cadres and the villagers' lack of interest as well as funds, about 20 per cent of village-run Co-operative Medical services in Qingyang County failed. Even among the other 80 per cent, there were many complaints that the Co-operative Medical service had disadvantaged those most in need of help. The problem was said to be so widespread throughout the entire province that in 1975 Henan provincial authority waged a campaign to rectify 'the false attitudes and bad management' on the ground.¹⁷ Rectification,

更热衷于为任命他们的当地干部服务，而不愿帮助普通村民。

‘当时，村民们任由赤脚医生摆布。他们不能简单地要求赤脚医生提供服务，’同一位干部反映道。¹⁴

此外，基层干部缺乏经验和资源，以及利益冲突，往往意味着村干部在管理合作医疗服务和赤脚医生计划方面能力不足。他们通常只关注眼前的利益。таким образом, 他们反映了血吸虫病控制计划从长远的预防项目向更直接的治疗模式的转变。他们的无能会加剧医疗不平等，即使这是无意的。正如山西省石岭县一位负责 1970 年代村卫生工作的退休村干部所反映的那样：‘我们大多数村干部文化水平不高，也没有卫生工作经验。我们简直是在黑暗中摸索。我们没有预料到会有这么多的问题。我们只是在它们造成实际问题时才试图解决它们。其中大部分处理得很糟糕。在如何管理资金、设备和药品以及赤脚医生的质量控制方面存在很多问题。也许这就是合作医疗服务崩溃的原因。’¹⁵

1973 年，河南省庆阳县的一份政府调查报告显示，许多生产队干部没有看到合作医疗的优势。他们的观点是：“合作医疗不会有任何改变。那些负担得起医疗费用的村民根本不需要任何帮助。对于许多贫困村民来说，合作医疗也帮不了多大的忙，因为它不能[产生足够的钱来]支付他们全部的医疗费用。不如让村民自己支付医疗费用。如果他们负担不起，可以向大队申请帮助。”¹⁶直到 1975 年，由于干部和村民缺乏兴趣以及资金，庆阳县约有 20% 的村办合作医疗服务失败。即使在剩下的 80% 中，也有许多抱怨说，合作医疗使那些最需要帮助的人处于不利地位。据说这个问题在整个省份都非常普遍，以至于 1975 年河南省当局发起了一场运动，以纠正基层的“错误态度和不良管理”¹⁷。整顿，

however, ended with blaming 'corrupt' cadres and Barefoot Doctors. It failed to address the actual obstacles that crippled the RCMS in the first place: instead of utilizing existing local medical resources and motivating communities, the RCMS depended heavily on specialized urban medical facilities such as hospitals, while Barefoot Doctors were supervised and supported by hospital doctors and academics from urban medical schools. In the case of Henan province, as elsewhere in China, the demand for hospital care often exceeded capacity. To extend their services to an increasing number of non-urban patients, many already crowded hospitals charged non-urban patients radically more than the urban patients. This made the RCMS impractical, unviable, and undesirable.¹⁸

Furthermore, grassroots cadres at the village level often prioritized improving agricultural productivity over health equality. When there was a shortage of resources, they would use the Co-operative Medical fund for other purposes, such as water conservation. In the case of one village in Shanxi province's Qin County, after the village cadres misspent the money collected for the Co-operative Medical fund on other purposes, the village could not afford to pay the Barefoot Doctor his work points (wage). In order to support his family, the Barefoot Doctor was forced to leave the village and seek his livelihood elsewhere, and the already crumbling village Co-operative Medical service fell apart completely.¹⁹ The experience of villagers across rural China was so diverse and often so negative that the only force that saw an overall improvement in the health care was the central administration of the CCP, which made claims of continuously improving health care outcomes, outcomes that were based, however, on the manipulation of data rather than on the actual experience of the people. However, the reality seeped out, even into the official accounts, as the local experiences became ever less satisfactory.

'WHY SHOULD I BE PAID LESS?'

Across the entire countryside, according to the official data presented at the National Health Conference in 1973, by the end of 1971 about two-thirds of the Co-operative Medical service had failed to deliver what had been promised.²⁰ Low pay plus heavy workloads and long working hours, as well as poor career prospects and job satisfaction, were identified as some of the major factors contribut-

然而,最终却归咎于“腐败”的干部和赤脚医生。它未能解决最初导致农村合作医疗制度瘫痪的实际障碍:农村合作医疗制度没有利用现有的当地医疗资源和激励社区,而是严重依赖专业的城市医疗设施,如医院,而赤脚医生则由医院医生和城市医学院的学者监督和支持。以河南省为例,与中国其他地方一样,对医院护理的需求往往超过了容量。为了向越来越多的非城市患者提供服务,许多已经拥挤的医院向非城市患者收取的费用远远高于城市患者。这使得农村合作医疗制度不切实际、不可行,也不受欢迎。¹⁸

此外,村级的基层干部往往优先考虑提高农业生产力,而不是医疗平等。当资源短缺时,他们会将合作医疗基金用于其他目的,如水利建设。在山西省沁县的一个村庄,村干部挪用合作医疗基金后,村里无力支付赤脚医生的工分(工资)。为了养家糊口,赤脚医生被迫离开村庄,另谋生路,原本就摇摇欲坠的村合作医疗服务彻底瓦解。¹⁹中国农村村民的经历如此多样,而且往往如此负面,以至于唯一看到医疗保健总体改善的力量是中国共产党的中央政府,它声称医疗保健结果不断改善,但这些结果是基于对数据的操纵,而不是基于人民的实际经历。然而,现实渗透了出来,甚至进入了官方叙述,因为当地的经历变得越来越不令人满意。

“我为什么要少拿钱?”

根据1973年全国卫生会议上提供的官方数据,到1971年底,大约三分之二的合作医疗服务未能兑现承诺,遍及整个农村。²⁰低工资加上繁重的工作量和长时间的工作,以及糟糕的职业前景和工作满意度,被认为是造成这种情况的一些主要因素。

ing to low retention rates for the Barefoot Doctors, undermining the effectiveness of the Co-operative Medical service on the ground.²¹ H was the Barefoot Doctor in a village not far from the Yunnan provincial capital, Kunming. In an interview in 2002, H recalled bitterly how she had always received less food compared to other villagers who did full-time agricultural work: 'For nine years when I was the Barefoot Doctor, I took care of villagers day and night ... in the end I could not earn enough food to feed my children.'²² Even in Chuan-sha County, outside of Shanghai, the national model for the Barefoot Doctors program, various official investigations conducted in the 1970s consistently showed that poor pay and a heavy workload had resulted in a growing number of Barefoot Doctors abandoning their duties and seeking their livelihood elsewhere or engaging in other activities to supplement their income.²³ By 1979, the report produced by the Shanghai Municipal Health Bureau showed that there were only 218 Barefoot Doctors left in Chuansha County compared to 750 prior to 1968. In a huge number of brigades, there was no one on the ground to deliver health services, from prevention to care.²⁴ An earlier investigative report from 1973 by the Chuan-sha County authority stated that a number of Barefoot Doctors in Jiangzhen Commune, the Barefoot Doctors flagship commune, complained that being a Barefoot Doctor made them economically worse off. Some moaned: 'Why should I be paid less than professional doctors? I would get paid more by doing something else.'²⁵ To supplement his income, one Barefoot Doctor at the Xiaoyu brigade was often seen dropping his medical kit by the road to take up some work as a builder or to help other villagers fishing.²⁶ In a more recent interview, one former cadre from a village outside of Qingdao in Shandong province summoned it up in a nutshell: 'Because there was no incentive, the majority of Barefoot Doctors were not proactive, and they showed no interest in learning. For them it was like being a monk in the temple who would go on tolling the bell as long as he is a monk. The only time they would show some enthusiasm was in front of cadres.'²⁷

The problem of poor pay was so severe that it prompted some Barefoot Doctors in Youai brigade in Jiangsu province to write a letter of complaint to the editor of the *People's Daily* (10 November 1972), petitioning the state to intervene by making sure the Barefoot Doctors were paid an amount of work points equal to that of other labourers. The letter revealed that despite their increased political

ing to low retention rates for the Barefoot Doctors, undermining the effectiveness of the Co-operative Medical service on the ground.²¹ H was the Barefoot Doctor in a village not far from the Yunnan provincial capital, Kunming. In an interview in 2002, H recalled bitterly how she had always received less food compared to other villagers who did full-time agricultural work: 'For nine years when I was the Barefoot Doctor, I took care of villagers day and night ... in the end I could not earn enough food to feed my children.'²² Even in Chuan-sha County, outside of Shanghai, the national model for the Barefoot Doctors program, various official investigations conducted in the 1970s consistently showed that poor pay and a heavy workload had resulted in a growing number of Barefoot Doctors abandoning their duties and seeking their livelihood elsewhere or engaging in other activities to supplement their income.²³ By 1979, the report produced by the Shanghai Municipal Health Bureau showed that there were only 218 Barefoot Doctors left in Chuansha County compared to 750 prior to 1968. In a huge number of brigades, there was no one on the ground to deliver health services, from prevention to care.²⁴ An earlier investigative report from 1973 by the Chuan-sha County authority stated that a number of Barefoot Doctors in Jiangzhen Commune, the Barefoot Doctors flagship commune, complained that being a Barefoot Doctor made them economically worse off. Some moaned: 'Why should I be paid less than professional doctors? I would get paid more by doing something else.'²⁵ To supplement his income, one Barefoot Doctor at the Xiaoyu brigade was often seen dropping his medical kit by the road to take up some work as a builder or to help other villagers fishing.²⁶ In a more recent interview, one former cadre from a village outside of Qingdao in Shandong province summoned it up in a nutshell: 'Because there was no incentive, the majority of Barefoot Doctors were not proactive, and they showed no interest in learning. For them it was like being a monk in the temple who would go on tolling the bell as long as he is a monk. The only time they would show some enthusiasm was in front of cadres.'²⁷

工资低的问题非常严重，以至于促使江苏省友爱大队的赤脚医生致信《人民日报》的编辑（1972年11月10日），请求国家干预，确保赤脚医生获得的工作积分与其他劳动者相等。这封信揭示了，尽管他们的政治地位有所提高

status, Barefoot Doctors, as well as other rural health workers, suffered structural inequality as the result of the rural work point system that prioritized agricultural work over health work. In the original design, the idea was that Barefoot Doctors should spend about a third of their time working in the fields; it had not been envisaged as a full-time job. But in the process of implementation, with the increasing political importance of the program as well as to save expenditure on public health activities, in many places being a Barefoot Doctor became a full-time job but without full pay.

The official propaganda that extolled Barefoot Doctors as 'revolutionary', 'working class' health workers, as opposed to the bourgeois medical professionals, further exacerbated this problem. Slogans such as Barefoot Doctors 'wearing no shoes' and 'covering their hands with dirty earth' allowed many brigades to impose heavy labouring rotas that demanded the Barefoot Doctor work in the agricultural fields for a certain number of days each year. In 1971, in order to uphold its national model status, Jiangzhen Commune launched the 'to learn from Wang Guizhen' campaign to prevent Barefoot Doctors turning into bourgeois doctors by spending too much time sitting in the village health station: 'their hands had turned pale and soft, and there was less dirt on their bare feet.' This campaign brought about a rota system, which required all Barefoot Doctors to spend at least one-third of the year doing agricultural work, regardless of the size of the village and the load of their public health duties. This resulted in there often being insufficient time for the Barefoot Doctor to care for villagers or to complete the necessary public health tasks.²⁸ Throughout the country, from Shaanxi province's Bin County in the northwest to Youai brigade in Jiangsu province in the east, a number of Barefoot Doctors complained collectively that the heavy workload, with the farming responsibilities as well as managerial tasks, had prevented them from doing health work. In the case of some villages in Bin County, because Barefoot Doctors were often away attending meetings or working in agricultural fields, sick villagers had to travel to the county hospital or seek help elsewhere. This, in turn, undermined the local Co-operative Medical service.

In addition, many Barefoot Doctors complained that constantly undertaking diverse tasks gave them little or no job satisfaction.²⁹ In Henan province's Minquan County, partly because of the labour shortage problem, the local authority increased the Barefoot Doctor agricultural rota to 240 days a year. For this to work, different

地位, 赤脚医生以及其他农村卫生工作者由于农村工分制度而遭受结构性不平等, 该制度优先考虑农业工作而不是卫生工作。在最初的设计中, 设想赤脚医生应该将大约三分之一的时间用于田间劳动; 它并没有被设想为一份全职工作。但在实施过程中, 随着该计划的政治重要性日益增加, 以及为了节省公共卫生活动的支出, 在许多地方, 当赤脚医生变成了一份全职工作, 却没有全额工资。

官方宣传将赤脚医生誉为“革命”、“工人阶级”卫生工作者, 与资产阶级医疗专业人员相对立, 这进一步加剧了这一问题。诸如赤脚医生“不穿鞋”和“用脏土覆盖双手”之类的口号使许多大队得以实施繁重的劳动轮班, 要求赤脚医生每年在农田中工作一定天数。1971年, 为了维护其国家模范地位, 江镇公社发起了“向王桂珍学习”的运动, 以防止赤脚医生因花太多时间坐在村卫生站而变成资产阶级医生: “他们的手变得苍白而柔软, 赤脚上的污垢也减少了。”这场运动带来了一种轮班制度, 该制度要求所有赤脚医生至少用三分之一的时间从事农业工作, 无论村庄大小和公共卫生任务的负担如何。这导致赤脚医生经常没有足够的时间照顾村民或完成必要的公共卫生任务。²⁸ 在全国范围内, 从陕西省西北部的彬县到江苏省东部的友爱大队, 许多赤脚医生集体抱怨说, 繁重的工作量(包括农业责任和管理任务)使他们无法从事卫生工作。在彬县的一些村庄, 由于赤脚医生经常外出开会或在农田中工作, 生病的村民不得不前往县医院或寻求其他帮助。反过来, 这削弱了当地的合作医疗服务。

此外, 许多赤脚医生抱怨说, 不断承担各种各样的任务让他们几乎没有工作满意度。²⁹ 在河南省民权县, 部分原因是劳动力短缺问题, 地方政府将赤脚医生的农业轮作时间增加到每年240天。为了使这项工作能够顺利进行, 不同的

villagers would have to take turns being the Barefoot Doctor. This not only led to confusion and the neglect of their health duties, but it also prevented them from receiving any consistent health training or even completing the training, since their agricultural work rota often ran into conflict with the training schedule.³⁰ Being a Barefoot Doctor turned out not to be a fulfilling role in a commune that prioritized production, since agricultural production was demanded by the authorities over everything else.

GENDER INEQUALITY

Structural inequality also led to a pay gap between male and female Barefoot Doctors that hampered the enthusiasm of the women in particular in delivering health care. Recently, Fang Xiaoping claimed, based on a case study of a Zhejiang village, that the Barefoot Doctors program granted young women in the countryside equal rights to study medicine and to participate in the rural public health program.³¹ Yet as early as 1973, an official report showed that among eight large brigades in Jiangzhen Commune – the national model for the Barefoot Doctors program – the total pay gap between male and female Barefoot Doctors amounted to more than 1,000 work points.³² Such misalignment had less to do with any policy design at the top but, rather, reflected the deep-seated cultural bias as well as the default of the work points system, which undervalued female labour in general. As with the female mill workers in Great Britain during the Industrial Revolution who were paid less than the male workers, in the PRC during the People's Commune (1958–83) female farmers were paid less work points than the men.³³ This was despite the official propaganda that boasted that women in the PRC enjoyed the same status as men for holding up 'Half of the Sky'.

The pay gap was but one aspect of gender inequality. In the case of the Barefoot Doctors, the inequality was also reflected in the allocation of roles. Based on the social and cultural bias that viewed the midwife and the nurse as roles for women, reinforced by the official policy and discourse, the majority of female Barefoot Doctors were assigned to gendered work such as midwifery and nursing. Some were only given the limited, relevant training. Villagers often treated the female Barefoot Doctor who did midwifery and family planning work, as well as nursing, differently from the male Barefoot Doctor who supposedly practised 'medicine'. In a village outside of Datong

村民们不得不轮流担任赤脚医生。这不仅导致混乱和对他们健康职责的忽视，而且也阻止了他们接受任何持续的健康培训，甚至完成培训，因为他们的农业工作轮班经常与培训时间表发生冲突。³⁰ 事实证明，在一个以生产为先的公社里，做赤脚医生并不是一个令人满意的角色，因为当局要求农业生产高于一切。

GENDER INEQUALITY

结构性不平等也导致男女赤脚医生之间的工资差距，这尤其阻碍了妇女提供医疗保健的热情。最近，方小平根据对浙江省一个村庄的案例研究声称，赤脚医生计划赋予了农村年轻妇女学习医学和参与农村公共卫生计划的平等权利。³¹ 然而早在 1973 年，一份官方报告显示，在江镇公社（全国赤脚医生计划的典范）的八个大队中，男女赤脚医生的总工资差距超过 1000 个工分。³² 这种不一致与任何顶层政策设计关系不大，而是反映了根深蒂固的文化偏见以及工分制度的默认设置，该制度低估了一般女性的劳动价值。正如工业革命时期英国的女性纺织厂工人薪酬低于男性工人一样，在人民公社时期（1958-83 年）的中国，女性农民获得的工分也低于男性。³³ 尽管官方宣传吹嘘中国妇女享有与男子同等的地位，撑起了“半边天”。

工资差距只是性别不平等的一个方面。以赤脚医生为例，这种不平等也体现在角色的分配上。基于认为助产士和护士是女性角色的社会和文化偏见，并得到官方政策和论述的加强，大多数女性赤脚医生被分配到具有性别分工的工作，如助产和护理。有些人只接受了有限的相关培训。村民们对待从事助产、计划生育工作以及护理的女性赤脚医生，通常与对待据称从事“医疗”的男性赤脚医生不同。在大同市外的一个村庄

city in Shanxi province, for instance, the village clinic was staffed with three female Barefoot Doctors and one male one. As the only male, villagers called him 'doctor', and he spent most of his time in the clinic doing 'medical' work. Being the 'doctor', he was also given lighter farming duty, although he received the most work points in comparison to the three female Barefoot Doctors. Of the three female Barefoot Doctors, one did nursing work, another did midwifery, and the third took care of sick children. All three women were given heavier loads of agricultural work for which they received fewer work points. Among the three, the one who did midwifery work received the least work points, for, traditionally, villagers did not regard delivering babies as real 'medical' work.³⁴ Because the system and the social and cultural norms discriminated against certain kinds of health work such as midwifery and nursing, classing them as less important, such work became, as a result, undesirable. Not surprisingly, a number of female Barefoot Doctors were less enthusiastic about undertaking health work, and the increasing number of female Barefoot Doctors quitting impacted on the caregiver/villager ratio and hence the quality of care.

FURTHER IMPEDIMENTS TO PROVIDING LOCAL SERVICES FOR THE COMMUNITY

The ongoing problem of retention of the Barefoot Doctors was also exacerbated by the government's compulsory 're-education' program, which began in 1969 and lasted for more than ten years. The program brought millions of young students (知识青年) from cities to the countryside to 'learn from the masses'. Ling, who nowadays lives and practises acupuncture in London, was one of many hundreds of young students from Beijing who were sent to a remote village in Inner Mongolia in 1969. She had learned some basic acupuncture before leaving Beijing, and upon her arrival in the village, Ling volunteered for the Barefoot Doctor role. With 80 per cent of villagers illiterate and with most having had no previous contact with either allopathic medicine or systemized TCM, her request to volunteer was quickly granted. This was quite common at the time; because of the labour shortage as well as the villagers' lack of interest and education, grassroots cadres would often circumvent the government policy that required the Barefoot Doctor to be from the same village by allocating the role to young students from the cities.

例如, 在山西省的某个城市, 村诊所配备了三名女性赤脚医生和一名男性赤脚医生。作为唯一的男性, 村民们称他为“医生”, 他大部分时间都在诊所里做“医疗”工作。作为“医生”, 他的农活也比较轻松, 尽管与三名女性赤脚医生相比, 他获得了最多的工分。在三名女性赤脚医生中, 一名做护理工作, 另一名做助产工作, 第三名照顾生病的孩子。所有这三位女性都承担了更重的农业劳动, 但获得的工分却更少。在这三人中, 做助产工作的女性获得的工分最少, 因为传统上, 村民们并不认为接生婴儿是真正的

“医疗”工作。³⁴ 由于该体系以及社会和文化规范歧视某些类型的卫生工作, 例如助产和护理, 认为它们不那么重要, 因此, 这些工作变得不受欢迎。毫不奇怪, 许多女性赤脚医生对从事卫生工作不太热情, 而且越来越多的女性赤脚医生退出, 影响了护理人员/村民的比例, 从而影响了护理质量。

进一步阻碍为社区提供本地服务

赤脚医生持续存在的人才保留问题, 也因政府自1969年开始并持续了十余年的强制性“再教育”计划而加剧。该计划将数百万年轻学生(知识青年)从城市带到农村“向群众学习”。如今居住在伦敦并从事针灸的玲, 就是1969年被派往内蒙古偏远村庄的北京数百名年轻学生之一。她在离开北京之前学过一些基本的针灸, 到达村庄后, 玲主动承担了赤脚医生的角色。由于80%的村民是文盲, 而且大多数人之前从未接触过西医或系统化的中医, 因此她自愿的要求很快就被批准了。这在当时非常普遍; 由于劳动力短缺以及村民缺乏兴趣和教育, 基层干部经常通过将赤脚医生的角色分配给来自城市的年轻学生来规避政府要求赤脚医生必须来自同一村庄的政策。

From the village cadres' point of view, this was an efficient way of utilizing human resources, since many of the young students from the cities were more trouble than help in the agricultural fields. For the students from the cities, being the Barefoot Doctor was often seen as a perk for doing less agricultural work. But Ling quickly learned that being a Barefoot Doctor was, in her words, 'very tough ... Villagers came to me for many things. They knocked on my doors day and night. Whenever they called on me, I had to go, even if it was at the other side of the village,' she recalled.³⁵ Although at the time that I interviewed Ling in her comfortable clinic in London she reminisced about her Barefoot Doctor experience in the 1970s as 'spiritually rich', she felt little nostalgia about the harsh life in Inner Mongolia. In 1977, after the universities and colleges resumed, she returned to Beijing to pursue further medical training. Despite her empathy for the poor villagers, the temptations of future career prospects as well as a more comfortable life in the big city was hard to resist.³⁶ Ling was by no means unique. Throughout the country, the red Barefoot Doctor certificate had indeed become the golden passport for many educated youths to return to the cities to undertake further education in medical colleges. Initially, a great number were sponsored by the villages in the false hope that these young students would return to continue to serve the villagers after graduating. Almost none of them did, however.

In today's China, readers were given the account of the exemplary Barefoot Doctor Sun Lize in a 2016 best-selling volume titled *Memories of a Barefoot Doctor*. Like Ling, in 1969 during the Great Proletarian Cultural Revolution, Sun was one of tens of thousands of students from Beijing sent to the countryside to be 'remoulded'. Sun ended up in a village in northern Shaanxi province's Yan'an prefecture, the wartime Red Capital of the CCP, situated at the Yellow Loess Plateau. Villagers there had little to no access to any form of medical or health care. Although there was the commune hospital, health workers rarely turned up at the village. According to one witness's account in the book, these villagers had lived 'under the spell of Yama (the king of hell)' prior to Sun's arrival. Sun arrived with a self-help medical manual and some antibiotics, and he became the village's Barefoot Doctor as soon as he unpacked his luggage. For five years between 1969 and 1975, we are told, Sun 'selflessly' served the villagers with his fine medical skills. Christened the living Norman Bethune, he became a national celebrity. In 1974, after

从村干部的角度来看,这是一种有效利用人力资源的方式,因为许多来自城市的年轻学生在农田里带来的麻烦多于帮助。对于来自城市的学生来说,当赤脚医生通常被视为减少农业工作的一种福利。但玲很快就了解到,用她的话说,当赤脚医生“非常艰难……村民们因为很多事情来找我。他们日夜敲我的门。无论他们什么时候叫我,我都必须去,即使是在村子的另一边,”她回忆道。³⁵虽然我在伦敦舒适的诊所里采访玲时,她回忆起1970年代当赤脚医生的经历,认为那段经历“精神上很丰富”,但她对内蒙古艰苦的生活并没有太多的怀旧之情。1977年,大学和学院恢复后,她回到北京继续接受医学培训。尽管她对贫困的村民们怀有同情,但未来职业前景的诱惑以及大城市更舒适的生活是难以抗拒的。³⁶玲绝不是个例。在全国各地,红色的赤脚医生证书确实成了许多受过教育的青年返回城市,进入医学院接受进一步教育的黄金通行证。最初,有很多学生是由村里资助的,村里虚假地希望这些年轻学生毕业后能回来继续为村民服务。然而,几乎没有人这样做。

在当今中国,读者在2016年畅销书《赤脚医生回忆录》中看到了模范赤脚医生孙立哲的故事。与凌一样,1969年文化大革命期间,孙是从北京派往农村接受“改造”的数万名学生之一。孙最终来到了陕北延安地区的一个村庄,那里是中共战时的红色首都,位于黄土高原。那里的村民几乎无法获得任何形式的医疗保健。虽然有公社医院,但医务人员很少到村里来。根据书中一位目击者的说法,在孙立哲到来之前,这些村民一直“生活在阎罗王(地狱之王)的魔咒下”。孙立哲带着一本自助医疗手册和一些抗生素来到这里,他一卸下行囊就成了村里的赤脚医生。据说,从1969年到1975年的五年里,孙立哲以其精湛的医术“无私地”为村民服务。他被称为活着的白求恩,成为全国名人。1974年,在

Mao personally knighted him as one of the five model students who had successfully 'remoulded' themselves by learning from the masses and who had 'settled' in the countryside, he was escorted by the local Cultural Revolutionary Committee to the Norman Bethune Memorial Hall in Yan'an to give a public lecture propagating the 'Spirit of Norman Bethune'. The next year, as the tale of a national hero, Sun's story was adapted into a graphic novel and a television documentary. He was also appointed a member of the PRC youth delegations visiting Europe and Africa. Yet from the witness accounts of some of his contemporaries, we also learn that Sun had never wanted to 'settle' in the countryside to 'selflessly' serve the poor villagers; he had wanted to return to the big city to pursue further studies in medical school. But as he had been made into a political symbol for upholding Chairman Mao's 'revolutionary approach to health', Sun was initially denied that opportunity. He had to wait until the late 1970s to fulfil his dream of becoming a professional doctor. Following the death of Mao, in 1980 the Barefoot Doctors program was officially condemned as 'the poison seed' that the Gang of Four had planted in health work. Later, Sun and his wife settled in the United States, where he embraced neoliberalism and became a multi-millionaire entrepreneur in the publishing business. Yet in recent years, the romance of the 'selfless' Barefoot Doctor Sun Lize was excavated by some of his contemporaries in order to give a nostalgic veneer to their 'lost youth'. Labelled by his contemporaries as a 'Super Human', the Barefoot Doctor Sun Lize had once again become a symbol, but of a different kind.³⁷

THE BAREFOOT DOCTOR VS THE WITCH DOCTOR

Despite Sun Lize boasting about having saved villagers from 'the spell of Yama (the king of hell)' with the 'Super Human' allopathic medicine, villagers saw him as just another 'magic healer' or 'wizard'. In his detailed study of the Barefoot Doctors program in Zhejiang province's Fuyang County, Fang Xiaoping argued that with the advent of the Communist Liberation in the early 1950s, and with the Barefoot Doctors program in the late 1960s, allopathic medicine – promoted by the state medical system – had effectively replaced the previous multiple medical systems and 'prohibited healers in supernatural and religious sects, and eradicated their practices'.³⁸ Mr Xu, the former Barefoot Doctor in Fuyang County, contested

毛亲自授予他“五好学生”之一的称号，这些学生通过向群众学习并“定居”在农村，成功地“改造”了自己。他被当地文化革命委员会护送到延安的诺尔曼·白求恩纪念馆，公开宣讲“诺尔曼·白求恩精神”。第二年，孙的故事作为一个民族英雄的故事，被改编成了一部图文小说和一部电视纪录片。他还被任命为访问欧洲和非洲的中国青年代表团成员。然而，从他的一些同时代人的证词中，我们也了解到，孙从未想过要“定居”在农村，以“无私”地为贫困的村民服务；他想回到大城市，继续在医学院深造。但由于他被塑造成拥护毛主席“卫生革命路线”的政治符号，孙最初被剥夺了这一机会。他不得不等到20世纪70年代末才实现了成为一名专业医生的梦想。毛去世后，1980年，“赤脚医生”计划被正式谴责为“四人帮”在卫生工作中埋下的“毒瘤”。后来，孙和他的妻子定居美国，在那里他拥抱新自由主义，成为出版业的一位百万富翁企业家。然而近年来，一些同时代人挖掘出了“无私”的赤脚医生孙立哲的浪漫故事，以便给他们“失去的青春”涂上一层怀旧色彩。被同时代人称为“超人”的赤脚医生孙立哲再次成为一种象征，但却是另一种象征。³⁷

赤脚医生 vs 巫医

尽管孙立哲吹嘘自己用“超人”西药将村民从“阎王（地狱之王）的咒语”中解救出来，但村民们认为他只不过是另一个“魔法治疗师”或“巫师”。方小平在对浙江省富阳县赤脚医生项目的详细研究中认为，随着20世纪50年代初的共产主义解放以及20世纪60年代末的赤脚医生项目的出现，国家医疗系统推广的西药有效地取代了以前的多种医疗系统，并“禁止了超自然和宗教教派中的治疗师，并根除了他们的做法”。³⁸富阳县前赤脚医生徐先生对此提出异议

these claims. In a 2015 interview, Xu told me that ‘even after the implementation of the Barefoot Doctor program, many villagers here continued to seek help from the Bodhisattva. That’s what they believed.’³⁹ Contrary to the conventionally held view that Chinese medicine was the ‘traditional medicine’ universally available across China, historically it was predominately only the social elites that had had access to this system. In Zhejiang province, for instance, phenomena such as therapeutic rituals at temples and shrines were a commonplace. Even under socialism, state-sanctioned medicine, in both its allopathic and Traditional Chinese Medicine versions, did not completely replace village rituals for ordinary villagers in Xu’s village. Bowing to the Bodhisattva or lighting some incense helped to manage illnesses that were understood as either misfortune or divine retribution, thus providing a prognosis and a path toward their management.

Despite the official discourse that was highly prejudiced against witchcraft, magic, and other popular healing rituals, labelling them as ‘feudal superstitions’, such practices persisted under socialism in different parts of the Chinese countryside, even with the advent of the RCMS. In Qin County throughout the RCMS era, as I learned from a recent interview with the former Barefoot Doctor, many villagers continued to seek help from the village witch doctor.⁴⁰ Compared to the often erratic Co-operative Medical service, the witch doctor was not only affordable, but, more important, she was always available to help the villagers with their anxieties, losses, and helplessness when the welfare state consistently failed to deliver the promised health care and denied their well-being. As Mr Liu, the former Barefoot Doctor, stated, ‘if the village held a meeting to announce government policy, very few villagers would turn up. If the witch doctor turned up, it would be a very different scene ... You would see villagers busy preparing, cooking, singing, and laughing. A real community fanfare.’ He recalled that when he was the village Barefoot Doctor, he regularly bumped into the witch doctor at sick villagers’ homes. Even though Liu dismissed the witch doctor as ‘unscientific’, most villagers did not privilege Liu, the Barefoot Doctor who claimed to practise ‘scientific medicine’, over the witch doctor. As Liu admitted in the interview, on many occasions he was competing with the witch doctor for the villagers’ affection. ‘I don’t understand why they believe in such superstition,’ he sighed. Many of these villagers did not associate ‘medicine’ with science; what

这些说法。在 2015 年的一次采访中，徐告诉我 “即使在赤脚医生计划实施后，这里的许多村民仍然继续向菩萨寻求帮助。那是他们所相信的。”³⁹ 与普遍认为的中国医药是 “传统医学” 并在中国普遍存在的观点相反，从历史上看，只有社会精英才能获得这种体系。例如，在浙江省，寺庙和神社的治疗仪式是一种司空见惯的现象。即使在社会主义制度下，国家认可的医学，无论是对抗疗法还是传统中医药疗法，都没有完全取代徐村普通村民的乡村仪式。向菩萨鞠躬或点燃一些香，有助于控制被理解为不幸或神罚的疾病，从而提供预后和管理途径。

尽管官方话语高度偏见地反对巫术、魔法和其他流行的治疗仪式，将它们贴上 “封建迷信” 的标签，但即使在人民公社出现后，这些做法仍然在中国农村的不同地区持续存在。正如我最近对一位前赤脚医生的采访中所了解到的，在整个秦县的人民公社时代，许多村民仍然向村里的巫医寻求帮助。⁴⁰ 与经常不稳定的合作医疗服务相比，巫医不仅负担得起，而且更重要的是，当福利国家持续未能提供承诺的医疗保健并否定他们的福祉时，她总是可以帮助村民解决他们的焦虑、损失和无助。正如前赤脚医生刘先生所说， “如果村里召开会议宣布政府政策，很少有村民会来。如果巫医来了，那将是完全不同的景象 …… 你会看到村民们忙着准备、做饭、唱歌和大笑。真正的社区欢腾。” 他回忆说，当他还是村里的赤脚医生时，他经常在生病的村民家中碰到巫医。尽管刘认为巫医是 “不科学的”，但大多数村民并没有优先考虑刘，这位声称 *практикует* “科学医学” 的赤脚医生，而不是巫医。正如刘在采访中承认的那样，在很多情况下，他都在与巫医争夺村民的喜爱。 “我不明白他们为什么相信这种迷信，” 他叹了口气。许多这些村民并没有将 “医学” 与科学联系起来；

mattered to them was convenience, affordability, and efficacy. Their understanding of efficacy and healing differed substantially from the bio-medical concept of 'curing'. While the latter focuses exclusively on eliminating a specific pathogen, for these villagers healing was a process aimed at addressing the social causes of affliction, hence alleviating their harm on the individual patient as well as their family or community by addressing such causes. This is why, when one sick villager was recovering after Liu had treated him with an intravenous injection, the family continued to call on 'the witch doctor to perform magic'.⁴¹

The matter was further complicated by the fact that the official press often did not make distinctions between local practitioners of folk medicine and ritual healers. Many private practitioners of herbal medicine and acupuncture who were not nominated to be the Barefoot Doctors and who practised outside of the Co-operative Medical service were sometimes condemned as 'witch doctors'.⁴² Since many Barefoot Doctors also used similar 'folk' interventions, it is not surprising that many villagers were confused. As one villager from Shandong province put it: 'I don't understand what was different about the Barefoot Doctor. I only know they are from our village. It's convenient to have them.' Quite often, for extra insurance, villagers simultaneously sought help from the local witch doctor, the herbal doctor, and the Barefoot Doctor.

This was even more the case in multi-ethnic regions such as Yunnan province in the southwest. In Chuxiong, as one former Barefoot Doctor recalled, 'most villagers never stopped seeking divine help from witch doctor and sorceress. They believe in them'.⁴³ In Chuxiong, situated on the periphery of the Chinese empire until the CCP took over the region, most of the population had only intermittent contact with both the codified 'traditional medicine', as documented in Chinese medical texts, and modern allopathic medicine. Historically, in these regions sickness had its specific local meanings. As Frances Hsu's detailed ethnographical study of local responses to the 1942 cholera epidemic in western Yunnan documented, ordinary villagers believed that the epidemics were a divine punishment for misbehaviour.⁴⁴ This meant that local therapeutic interventions that were familiar to ordinary people involved locally meaningful ritual components, from correcting behaviours to asking forgiveness or aid from supernatural forces through prayer, magic, or possession. To ease the stress, they may also have sought help

对他们来说,重要的是便利性、可负担性和有效性。他们对有效性和治疗的理解与生物医学的“治愈”概念有很大不同。后者只专注于消除特定的病原体,而对于这些村民来说,治疗是一个旨在解决痛苦的社会原因的过程,从而通过解决这些原因来减轻其对个体患者及其家庭或社区的伤害。这就是为什么当一位生病的村民在刘用静脉注射治疗后正在康复时,他的家人仍然会请“巫医来做法”的原因。⁴¹

更复杂的是,官方媒体通常不区分当地的民间医药从业者和仪式治疗师。许多未被提名为赤脚医生并在合作医疗服务之外执业的草药和针灸私人从业者,有时被谴责为“巫医”。⁴²由于许多赤脚医生也使用类似的“民间”干预措施,因此许多村民感到困惑也就不足为奇了。正如山东省一位村民所说:“我不明白赤脚医生有什么不同。我只知道他们来自我们村。有他们很方便。”通常,为了额外的保障,村民们同时向当地的巫医、草药医生和赤脚医生寻求帮助。

在云南省西南部等多种族地区,情况更是如此。一位前赤脚医生回忆说,在楚雄,“大多数村民从未停止向巫医和女巫寻求神灵的帮助。他们相信他们。”⁴³在楚雄,在中国共产党接管该地区之前,它一直位于中华帝国的边缘,那里的大多数人只是间歇性地接触到中国医学典籍中记载的“传统医学”和现代对抗疗法。从历史上看,在这些地区,疾病具有其特定的地方含义。正如徐烺光的详细民族志研究记录了1942年云南西部霍乱流行期间当地的反应,普通村民认为流行病是对不当行为的神圣惩罚。⁴⁴这意味着普通民众熟悉的当地治疗干预措施涉及具有地方意义的仪式成分,从纠正行为到通过祈祷、魔法或附身请求超自然力量的原谅或帮助。为了缓解压力,他们也可能寻求帮助

from acceptable medical interventions, ranging from herbal medicine to bio-medical interventions such as injections. For them, as Frances Hsu has pointed out, 'the injection and modern measures of medicine were merely more devices against the epidemic, but they were neither the only devices nor the most important ones.'⁴⁵

Although the Barefoot Doctors interviewed were aware that they were different from the other healers and that they sometimes dismissed supernatural healers and other folk healers as practising 'superstitions' rather than 'science', since they were simply another villager, many of them did not see that it was necessary to 'eliminate' therapeutic ritual practices. Indeed, as Mr Liu, the former Barefoot Doctor in Shanxi province's Qin County, admitted, often 'the witch doctor did her stuff, and I did mine. We did not interfere with each other. At most, we might look askance at each other.' At times, when they saw it 'appropriate', some Barefoot Doctors even took part in the 'superstitious' rituals. As Liu recalled, on one occasion a fellow Barefoot Doctor called him to help with his sick uncle. When he arrived at the patient's home, he saw the other Barefoot Doctor burning incense, following the witch doctor's instruction.⁴⁶

BAREFOOT DOCTORS AND BIRTH RITUALS

When it came to delivering babies, however, quite a few Barefoot Doctors, almost all of them female and trained to implement the 'new-style' medicalized childbirth as well as the family planning program, admitted that traditional birth rituals often presented serious challenges. Worldwide, medicalized childbirth has functioned as the public face of the success of allopathic medicine. By focusing solely on the questions of mother and infant mortality and morbidity, success was measured by the ultimate survival of the individual. In modern China, maternal and child health served as indexes to national reconstruction. The training of new-style midwives first began in the late 1920s under the Nationalist government.⁴⁷ After the Communist Liberation of China in 1949, child and maternal health was viewed as integral to the building of the future utopian state. The new Communist state included child and maternal health as one of the Sixty Articles of its provisional constitution, the Common Program of the Chinese People's Consultative Congress.⁴⁸

Concurrently, maternal health became a worldwide priority for the World Health Organization (WHO). Beginning in the early 1950s

来自可接受的医疗干预措施, 范围从草药到生物学干预措施, 如注射。正如许烺光所指出的, 对他们来说, “注射和现代医学措施仅仅是对抗疫情的更多手段, 但它们既不是唯一的手段, 也不是最重要的手段。”⁴⁵

尽管接受采访的赤脚医生意识到他们与其他治疗师不同, 并且有时他们会认为超自然治疗师和其他民间治疗师是在实践“迷信”而不是“科学”, 但由于他们只是另一个村民, 他们中的许多人并不认为有必要“消除”治疗仪式。事实上, 正如山西省沁县的前赤脚医生刘先生承认的那样, 通常“巫医做她的事, 我做我的事。我们互不干涉。最多, 我们可能会互相斜视。”。有时, 当他们认为“合适”时, 一些赤脚医生甚至会参与“迷信”仪式。正如刘回忆的那样, 有一次, 一位赤脚医生打电话给他, 帮助他生病的叔叔。当他到达病人家中时, 他看到另一位赤脚医生正在按照巫医的指示烧香。⁴⁶

赤脚医生与生育仪式

然而, 在接生方面, 不少赤脚医生(几乎都是女性, 并接受过实施“新式”医疗化分娩以及计划生育项目的培训)承认, 传统的生育仪式常常带来严峻的挑战。在世界范围内, 医疗化的分娩已经成为对抗疗法医学成功的公众形象。通过只关注母婴死亡率和发病率的问题, 成功的衡量标准是个体的最终存活。在现代中国, 妇幼保健是国家重建的指标。新式助产士的培训最早于20世纪20年代末在国民政府的领导下开始。⁴⁷ 1949年中国共产党解放后, 儿童和孕产妇的健康被视为未来乌托邦国家建设不可分割的一部分。新的共产主义国家将儿童和孕产妇健康纳入其临时宪法《中国人民政治协商会议共同纲领》的六十条之一。⁴⁸

与此同时, 孕产妇健康成为世界卫生组织(WHO)在全球范围内的首要任务。从20世纪50年代初开始

and aimed at reducing maternal and infant mortality worldwide, the WHO engaged the United Nations International Children's Emergency Fund (UNICEF) to train Traditional Birth Attendants (TBAs). The TBA programs were launched in many newly decolonized Third World countries. In the meantime, in the newly founded PRC, the program for training 'new-style' midwives (the official term in China for midwives who practised medicalized childbirth) that had begun in the Nationalist era grew considerably, with an emphasis on the rural countryside. Overwhelmed by the competing demands made by the state, from agricultural development to disease eradication, such as the Anti-Schistosomiasis Campaign, the training of new-style midwives, as well as the implementation of the new-style medicalized childbirth program in the countryside, remained haphazard until the advent of the Barefoot Doctors program in the 1960s and 1970s. In 1964, Zhimai, against her own wishes, was nominated by the village cadre to undergo training to become a 'new-style' midwife. Her role as the 'new-style' midwife was to mitigate 'risks' by replacing the old-style 'dangerous' childbirth with the 'safe' medicalized child delivery method. After 1968, Zhimai was designated a Barefoot Doctor, although her role remained the same. She remembered that it was hard work persuading labouring women and their families to embrace the new method. Zhimai's village was situated on the Chinese-Burmese border, and the majority of the people in the village were Jinpo by ethnicity. In her village, the traditional belief was that birth and death were two of the core generative functions of human life. As such, the survival of the mother and infant was entwined with the moral fabric of community and family, reaching deep into the past and focusing on the future. Villagers viewed protracted or painful labour as caused by 'ghost affliction'. The ghost was said to be one's ancestor. When that happened, the whole family would make offerings, often in the form of food, such as chickens or eggs, to feed the ghost. On many occasions the expectant mothers refused Zhimai's help. To send her away, they told her: 'No pain yet. It's not happening. Why don't you go home to rest for a bit?' But as soon as she left, they gave birth. '*They did not trust me,*' she said, blaming herself. In her role as the new-style midwife, she was also responsible for the implementation of birth control, yet she recalled that it was an impossible task. Traditionally in her village, fertility was associated with prosperity. Women were often compared to a fruit tree that should bear as much fruit as possible or

为了降低全球孕产妇和婴儿的死亡率,世界卫生组织与联合国儿童基金会 (unicef) 合作,培训传统接生员 (tbas)。TBA 项目在新近获得独立的第三世界国家中启动。与此同时,在新建的中华人民共和国,在国民党时期开始的培训“新式”助产士(中国对实行医疗化分娩的助产士的官方称谓)的项目得到了极大的发展,重点放在农村地区。由于国家提出了农业发展到根除血吸虫病(如防治血吸虫病运动)等相互竞争的需求,对新式助产士的培训,以及在农村地区实施新式医疗化分娩项目,直到20世纪60年代和70年代赤脚医生项目的出现,仍然是零星的。1964年,止迈违背自己的意愿,被村干部提名接受培训,成为一名“新式”助产士。她作为“新式”助产士的角色是通过用“安全”的医疗化分娩方法取代旧式的“危险”分娩来减轻“风险”。1968年以后,止迈被指定为赤脚医生,但她的角色仍然没有改变。她回忆说,说服分娩妇女及其家人接受这种新方法是很困难的。止迈的村庄位于中缅边境,村里的大部分人都是景颇族。在她的村庄里,传统的观念是,出生和死亡是人类生命两个核心的生成功能。因此,母亲和婴儿的生存与社区和家庭的道德结构交织在一起,深入过去,着眼未来。村民们认为,旷日持久或痛苦的分娩是由“鬼缠身”引起的。据说这个鬼是自己的祖先。当这种情况发生时,全家人都会进行祭祀,通常以食物的形式,如鸡或鸡蛋,来喂养鬼。很多时候,准妈妈们拒绝止迈的帮助。为了把她打发走,她们会告诉她:“还不疼。还没开始呢。你为什么回家休息一会儿呢?”但她一离开,她们就生了。“她们不信任我,”她说,责怪自己。作为新式助产士,她还负责实施计划生育,但她回忆说,这是一项不可能完成的任务。在她的村庄里,传统上生育力与繁荣有关。妇女经常被比作一棵应该尽可能多结果的果树,或者

a hen that should produce as many eggs as possible, while infertility was regarded as a misfortune for the entire family. It was believed that infertility could be passed down from mothers to daughters. As such, villagers considered the use of contraception a total taboo.⁴⁹

Further north of Zhimai's village, in Chuxiong region's Nanhua County, Lanfeng, the former midwifery Barefoot Doctor, recounted that she had infuriated many villagers by trying to implement birth control: 'No one turned up to receive contraception devices. I ended up going around houses to persuade them. They were so annoyed with me,' she recalled. Equally, Lanfeng found it a daunting task to ensure that women underwent 'safe' medicalized childbirth in the hospital instead of 'dangerous' home births. In her village, childbirth always took place at home in the presence of the family. On one occasion, a 38-year-old expectant mother was complaining about terrible pain before labour. For the mother's safety, Lanfeng wanted her to go to the hospital to give birth in a 'safe' environment and be assisted by health professionals. The woman refused because she was frightened by the prospect of giving birth outside of her home, and moreover she considered it shameful to give birth in front of strangers. 'How can a woman give birth in the hospital? Even if I were to die, I would stay home,' she told Lanfeng. As expected, there were complications during labour as the umbilical cord broke. 'I did not know what to do. I had never encountered the situation before. I wanted to call the hospital, but there was no telephone,' Lanfeng recalled. In the meantime, some female villagers turned up asking Lanfeng to massage the woman, but Lanfeng turned down their advice because it was the 'old' method and hence involved 'risk'. By ignoring the 'old' village tradition, Lanfeng caused chaos. The female villagers argued with her, and their argument became louder and louder, adding more strain to the already stressed woman and her family. In desperation, Lanfeng put on sterile gloves and gave the woman an injection to speed up the delivery, while the others watched in awe. For the rest of the week, Lanfeng took care of the woman to make sure there was no infection. The incident won her respect and trust in the village. As her reputation grew, more and more women asked her to help in delivering their babies, although most of them continued to refuse to go to the hospital to give birth, often ignoring her warnings of health risks.⁵⁰

Elsewhere in China, as we learn from Gail Hershatter's study of women's memories of the Maoist revolution in rural Shaanxi, until the

母鸡应该尽可能多地产蛋，而无法生育则被认为是整个家庭的不幸。人们认为不孕症可以从母亲传给女儿。因此，村民认为使用避孕措施是完全的禁忌。⁴⁹

在志迈村更北边的楚雄地区南华县，曾经的赤脚医生兼助产士兰凤回忆说，她试图实施计划生育时激怒了很多村民：“没有人来领取避孕器具。我最终挨家挨户去说服他们。他们非常恼火，”她回忆道。同样，兰凤发现要确保妇女在医院进行“安全”的医疗分娩，而不是在家里进行“危险”的分娩，也是一项艰巨的任务。在她的村子里，分娩总是在家里，在家人在场的情况下进行。有一次，一位38岁的准妈妈在分娩前抱怨剧烈的疼痛。为了母亲的安全，兰凤希望她去医院在一个“安全”的环境中分娩，并由健康专业人员协助。这位妇女拒绝了，因为她害怕在自己家外面分娩，而且她认为在陌生人面前分娩是很可耻的。“女人怎么能在医院生孩子？即使我要死了，我也会呆在家里，”她告诉兰凤。不出所料，分娩过程中出现了并发症，脐带断裂。“我不知道该怎么办。我以前从未遇到过这种情况。我想给医院打电话，但没有电话，”兰凤回忆道。与此同时，一些女村民来找兰凤，要求给她按摩，但兰凤拒绝了她们的建议，因为那是“旧”方法，因此存在“风险”。通过无视“旧”的乡村传统，兰凤造成了混乱。女村民和她争吵起来，争吵声越来越大，给已经压力重重的妇女和她的家人带来了更大的压力。在绝望中，兰凤戴上无菌手套，给这位妇女注射了一针，以加速分娩，而其他则敬畏地看着。在那一周的剩余时间里，兰凤照顾这位妇女，以确保没有感染。这件事为她在村里赢得了尊重和信任。随着她的声誉越来越高，越来越多的妇女要求她帮助分娩，尽管她们中的大多数人仍然拒绝去医院分娩，经常无视她对健康风险的警告。⁵⁰

正如我们从盖尔·赫斯哈特的陕西农村妇女对毛泽东时代革命的回忆研究中了解到的那样，在中国其他地方，直到

1970s many women also preferred a homebirth, whether with or without the assistance of midwives. Infant deaths from umbilical tetanus remained common in this rural area well into the 1970s. Hershatter suggests that this had to do with the villagers' indifferent attitude toward antiseptics.⁵¹ In *Purity and Danger* (1966), anthropologist Mary Douglas argued that 'dirt is simply matter out of place'.⁵² What defines dirt is a question of perception; for many women and their families living in different parts of rural China, childbirth was 'dangerous' and 'dirty' because they considered the placental blood 'polluting'. It could cause contamination, harming the mother, the infant, and the family, even impacting on the greater community. In one interview, a woman from rural Sichuan told me that her own grandmother had refused to help cut her umbilical cord after she gave birth for fear of being contaminated by the 'dirty' blood.⁵³ Traditionally in China, to prevent contamination after giving birth, the mother would be confined at home for a whole month of convalescence. This was known as 'doing the month', and it is still practised by some. When she was 'doing the month', the new mother would also be prohibited from cleaning herself, or exposing herself to the wind, or coming into contact with water in order to avoid polluting the environment. In addition, she would follow a strict diet to counter the 'imbalance' induced by the very pregnancy that had made her vulnerable. In contrast, dirt in the natural environment was not associated with 'impurity', nor was it imagined that it had the power to 'pollute'; hence, it was not seen as necessary to wash hands or disinfect the scissors used to cut the umbilical cord. While many infants died of umbilical tetanus and women died of infection, many also survived through relying on local indigenous remedies and methods as well as the restorative postnatal ritual of 'doing the month'. The woman from rural Sichuan quoted above told me that after her grandmother refused to help, she grabbed the scissors and cut the umbilical cord herself, but she suffered an infection as a result. To treat the infection, following the advice given by a local doctor, she washed the wound with an infusion made of mugwort, dried pomelo skin, and Sichuan peppercorns. She recovered, and so did the infant. Subsequently, as this ninety-year-old who was now a grandmother herself proudly told me, she helped to cut the umbilical cords for other villagers: 'I often cut umbilical cords for others. I would cut a very short length to avoid causing too much pain for the mother. In fact, the right length is about this much, but I was afraid of causing the mother and the baby too much pain.'⁵⁴ In 1954, this woman migrated to the

20 世纪 70 年代, 许多妇女也更喜欢在家分娩, 无论是否有助产士的帮助。脐带破伤风导致的婴儿死亡在 20 世纪 70 年代仍然是这个农村地区常见的现象。赫沙特认为, 这与村民们对消毒的漠不关心有关。⁵¹ 在《纯洁与危险》(1966) 一书中, 人类学家玛丽·道格拉斯认为, “污垢只是放错地方的物质”。⁵² 什么定义了污垢是一个感知问题; 对于居住在中国农村不同地区的许多妇女及其家庭来说, 分娩是 “危险的” 和 “肮脏的”, 因为她们认为胎盘血是 “污染的”。它可能导致污染, 伤害母亲、婴儿和家庭, 甚至影响到更大的社区。在一次采访中, 一位来自四川农村的妇女告诉我, 她自己的祖母拒绝在她分娩后帮她剪脐带, 因为害怕被 “脏” 血污染。⁵³ 传统上, 在中国, 为了防止分娩后受到污染, 产妇会在家坐月子整整一个月。这被称为 “坐月子”, 至今仍有人 практикует。当她 “坐月子” 时, 新妈妈也会被禁止清洗自己, 或让自己暴露在风中, 或接触水, 以避免污染环境。此外, 她还会遵循严格的饮食, 以对抗怀孕带来的 “不平衡”, 因为怀孕会使她变得脆弱。相比之下, 自然环境中的污垢与 “不洁” 无关, 也没有人认为它有 “污染” 的能力; 因此, 没有必要洗手或消毒用于剪脐带的剪刀。虽然许多婴儿死于脐带破伤风, 妇女死于感染, 但也有许多人通过依赖当地的土方和方法以及产后恢复性的 “坐月子” 仪式而幸存下来。上文引用的那位来自四川农村的妇女告诉我, 在她祖母拒绝帮忙后, 她抓起剪刀自己剪了脐带, 但结果却感染了。为了治疗感染, 她听从当地医生的建议, 用艾蒿、干柚子皮和四川花椒泡水清洗伤口。她康复了, 婴儿也康复了。后来, 这位当时已经九十岁高龄的祖母自豪地告诉我, 她帮其他村民剪脐带: “我经常帮别人剪脐带。我会剪得很短, 以避免给母亲带来太多的痛苦。事实上, 合适的长度是这么长, 但我害怕给母亲和婴儿带来太多的痛苦。”⁵⁴ 1954 年, 这位妇女移民到了

city where she gave birth to her fourth child at a state-run birth clinic. By this time, any newborn baby not delivered by medicalized birth procedure was denied any legal status. Four years later, in 1958, this woman gave birth to her fifth child at home because she so distrusted the clinic. She also plotted to give the newborn away to a neighbour so that the child could be legally registered through adoption. It was by circumventing state policy in such ways that unassisted home births continued. Many unlucky newborns died at birth; because these babies could not be legally registered, their deaths were never officially recorded. In the following four years, as a result of the famine, the birth rate dropped sharply, which also contributed to the decrease in the infant mortality rate.

By the 1970s, with the advent of the Barefoot Doctors when all villages in rural China were required to have at least one female Barefoot Doctor who performed midwifery duties, unassisted births remained a commonplace in rural China. Even in regions with better access to transport, such as Juntong County in Jiangsu province, official statistics show that neonatal and infant mortality persisted.⁵⁵ In remote mountainous regions such as Guizhou province's Tianzhu County, the situation was considerably worse. There, many villages did not have any Barefoot Doctors or a new-style midwife until as late as 1976. Tianzhu (which literally means 'Pillars Supporting the Heaven') is a multi-ethnic region in southeastern Guizhou bordering Hunan province. More than 98 per cent of the local population is Miao and Dong. Nestled in the high mountains, Yang village had no accessible road until quite recently. But during the heavy rainy season, which happens every summer, the road into the village became dangerous because of mudslides. I experienced this in 2015 when I was conducting field research in the village, and I had to wait for many days until the rain had ceased. But when a baby is due, he or she cannot wait for days for the rain to stop. Yang Mama, the former Barefoot Doctor, was the first new-style midwife in the village. She received her training in 1976 and returned to serve the village in 1977. But even then, some villagers continued to have unassisted homebirths because they could not afford the 1.5 *yuan* fee for a birth assisted by Yang Mama, who, as the village Barefoot Doctor, was supported by the Co-operative Medical service.⁵⁶

By the time I was conducting the research for this book, almost all midwifery Barefoot Doctors I interviewed agreed with the official statistics claiming that the spread of the new-style medicalized childbirth

她曾在市里一家公立妇产医院生下了第四个孩子。当时,任何未经医疗助产程序分娩的新生儿都不被赋予任何法律地位。四年后,在1958年,这位妇女在家里生下了第五个孩子,因为她非常不信任诊所。她还计划把新生儿送给邻居,以便孩子可以通过收养获得合法登记。正是通过这种规避国家政策的方式,无助产士的家庭分娩得以延续。许多不幸的新生儿在出生时就夭折了;由于这些婴儿无法合法登记,他们的死亡从未被正式记录。在接下来的四年里,由于饥荒,出生率急剧下降,这也导致了婴儿死亡率的下降。

到了20世纪70年代,随着赤脚医生的出现,中国农村的所有村庄都被要求至少有一名从事助产工作的女赤脚医生,无助产士的分娩在中国农村仍然很普遍。即使在交通更加便利的地区,如江苏省的浚铜县,官方统计数据显示,新生儿和婴儿死亡率仍然很高。⁵⁵在贵州省天柱县等偏远山区,情况则更为糟糕。在那里,许多村庄直到1976年才拥有赤脚医生或新型助产士。天柱(字面意思是“支撑天堂的柱子”)是贵州省东南部与湖南省接壤的多民族地区。当地人口中,苗族和侗族占98%以上。杨村坐落在高山之中,直到最近才有一条可通行的道路。但在每年夏天都会发生的雨季,由于泥石流,进入村庄的道路变得很危险。2015年,我在村里进行实地调查时,就经历过这种情况,不得不等了好几天雨才停。但是,当婴儿要出生时,他或她不能等几天等雨停。杨妈妈是以前的赤脚医生,也是村里第一位新型助产士。她在1976年接受了培训,并于1977年回到村里服务。但即便如此,一些村民仍然继续进行无助产士的家庭分娩,因为他们负担不起杨妈妈1.5元的助产费用,而作为村里的赤脚医生,杨妈妈得到了合作医疗服务的支持。⁵⁶

在我为本书进行研究时,几乎所有我采访的乡村医生助产士都同意官方统计数据,即新型医学化分娩的普及

has significantly reduced maternity and infant mortality in the countryside. Informants told me in their interviews how the Barefoot Doctor program was instrumental in winning over the women in their villages to embrace medicalized childbirth as the unquestionably 'safe' way of giving birth. As with other bio-medical interventions, medicalized childbirth has undoubtedly helped to save the lives of many mothers and infants. Yet from the accounts of women in different parts of rural China, we also learn that 'safety' is a social and cultural construct, dynamic and always changing. In the eyes of these rural women, anything new and unfamiliar was a 'threat' and thus 'dangerous'. For them, the new-style medicalized childbirth 'ritual' challenged their symbolic cultural values, disrupted their family and community dynamics, and was far from 'safe'. It was simply threatening.⁵⁷ Equally, for many, to give birth in the alienating environment of a hospital, without their family and in the presence of strangers whom they did not trust, was not 'safe' but 'shameful' and stressful.⁵⁸ This may account for why even today the practice of homebirths – most of which are now, however, assisted – remains commonplace in rural southwest China, even though the government has introduced stricter rules aimed at eliminating the purported health 'risks'. During my research for this book, I observed that in different corners of southwest China, there are still many families who are too poor to afford giving birth in hospitals. Aside from affordability, for those living in remote villages such as Danbo village in Barkam in the Himalayas and Yang village in Guizhou, the majority of villagers had no means of travelling to the hospital. For them, homebirth was not a matter of choice. It remained risky for the family and the village doctor (after the Barefoot Doctors program officially ended in the 1980s, many of the Barefoot Doctors were renamed 'village doctors' if they passed the national exam), but today it is a very different kind of risk. If they get caught, both the family and the village doctor would be penalized by the state.⁵⁹ However, many consider it a risk worth taking, since they rely on the individual village doctors more than they trust the state system that often disadvantages them; these village doctors are valued members of their community and highly skilled in delivering babies.

TRUST

For Chen Zhiqian, one of the most eminent figures in the development of public health programs from the Nationalist period through

大大降低了农村地区的孕产妇和婴儿死亡率。受访者在访谈中告诉我，赤脚医生计划在说服村民接受医疗化的分娩方式方面发挥了重要作用，认为这是毋庸置疑的“安全”分娩方式。与其他生物医学干预措施一样，医疗化的分娩无疑有助于挽救许多母亲和婴儿的生命。然而，从中国不同农村地区妇女的叙述中，我们还了解到，“安全”是一种社会和文化建构，是动态的且不断变化的。在这些农村妇女眼中，任何新的和不熟悉的事物都是一种“威胁”，因此是“危险的”。对她们来说，新型的医疗化分娩“仪式”挑战了她们的象征性文化价值观，扰乱了她们的家庭和社区动态，远非“安全”。它简直就是一种威胁。⁵⁷同样，对许多人来说，在陌生的医院环境中，没有家人陪伴，而且在她们不信任的陌生人面前分娩，不是“安全”而是“可耻”和有压力的。⁵⁸这或许可以解释为什么即使在今天，家庭分娩的做法——尽管其中大部分现在都有人协助——在中国西南农村地区仍然很普遍，尽管政府已经出台了更严格的规定，旨在消除所谓的健康“风险”。在为本书进行研究的过程中，我观察到，在中国西南地区的各个角落，仍然有许多家庭过于贫困，无法负担在医院分娩的费用。除了经济承受能力之外，对于居住在偏远村庄的人们，如喜马拉雅山脉巴尔康的丹波村和贵州的杨村，大多数村民没有前往医院的交通工具。对他们来说，家庭分娩不是一种选择。对于家庭和乡村医生来说，这仍然是危险的（在赤脚医生计划于20世纪80年代正式结束后，许多赤脚医生如果通过国家考试，就被更名为“乡村医生”），但今天的风险已经非常不同。如果他们被抓住，家庭和乡村医生都会受到国家的惩罚。⁵⁹然而，许多人认为这种风险是值得承担的，因为他们更依赖于个体乡村医生，而不是信任常常使他们处于不利地位的国家体系；这些乡村医生是他们社区中受重视的成员，并且在接生方面技术娴熟。

信任

对于陈志潜来说，他是从国民党时期到现在的公共卫生项目发展中最杰出的人物之一

to the PRC, developing trust between patients and health care providers was key to the development of local allopathic medical programs. Without the villagers' trust in those assigned to their care, little would have changed, no matter what the *ukase* from above. In 1929, a rural health experiment by the Chinese National Association of the Mass Education Movement began in Ding County in northern China.⁶⁰ It aimed at educating illiterate villagers through public health education and by introducing them to allopathic medicine. According to Dr H.Y. Yao, who headed the public health department of the Mass Education Movement: 'most of these [villagers] are poor, ignorant and superstitious, living in mud huts, blackened by soot and smoke, swamped with flies, mosquitoes, bed bugs, fleas, and rats.'⁶¹ In the late 1980s, reflecting back on his earlier experience in developing the Ding County rural health program, Chen Zhiqian reminded us that the 'Barefoot Doctor' phenomena was nothing new: 'Since time immemorial, Chinese villagers had been seeking medical counsel and obtaining their medication from other villagers whose knowledge of medicine was only slightly greater than their own.' For him, it was 'theoretically plausible to use villagers – who lacked advanced technical knowledge – as the primary health workers: they were already there, and they were apt remain there. Whereas an outsider accustomed to more amenities and less isolation might well be reluctant to suffer the hardship of village life for very long, the inhabitants were accustomed to the local conditions and were bound to their communities by kinship and other ties. Villagers who were trusted by their fellow villagers, moreover, would have an advantage over outsiders, who would have to spend precious time demonstrating their reliability.'⁶²

Zelang Muchu was in her seventies when I interviewed her in 2016. Muchu was one of the very few village doctors struggling to continue providing basic health care to villagers in and near Danbo village in the Himalayas, many of whom have little or no access to, or simply do not trust, the state system and hospital care. In 1973, while still a young girl, Muchu was nominated by her village to be the Barefoot Doctor, and she subsequently received basic medical and midwifery training. Up until 2016, she told me, she had delivered more than seventy babies in her village as well as in surrounding villages:

I went to help villagers whenever and wherever, often without pay. [Under the work point system] I would lose work points if I was found helping women from another village with their

对于中华人民共和国来说，建立患者与医疗保健提供者之间的信任是发展本地西医医疗计划的关键。如果没有村民对受托照料他们的人的信任，无论上面的敕令是什么，都不会有什么改变。1929年，中华平民教育促进会的一个农村卫生实验在华北的定县开始。⁶⁰其目的是通过公共卫生教育和向他们介绍西医来教育文盲村民。据平民教育促进会公共卫生部主任姚和云博士说：“这些[村民]大多贫穷、无知和迷信，住在泥土小屋里，被烟灰熏黑，到处都是苍蝇、蚊子、臭虫、跳蚤和老鼠。”⁶¹ 20世纪80年代后期，在回顾他早期发展定县农村卫生计划的经验时，陈志潜提醒我们，“赤脚医生”现象并不新鲜：“自古以来，中国村民一直在向其他村民寻求医疗建议并获得药物，这些村民的医学知识仅比他们自己略高。”对他来说，使用缺乏先进技术知识的村民作为主要卫生工作者在理论上是可行的：他们已经在那里，而且他们很可能留下来。而一个习惯了更多便利设施和更少孤立的局外人可能不愿意忍受村庄生活的艰辛太久，居民习惯了当地的条件，并通过亲属关系和其他联系与他们的社区联系在一起。此外，受到村民信任的村民比局外人更有优势，局外人将不得不花费宝贵的时间来证明他们的可靠性。⁶²

我在2016年采访泽朗木初时，她已经七十多岁了。木初是为数不多的仍在努力为喜马拉雅山丹波村及其附近村民提供基本医疗保健的乡村医生之一，他们中的许多人几乎没有或根本无法获得国家系统和医院的医疗服务，或者根本不信任它们。1973年，当她还只是个女孩时，木初被她的村庄提名为赤脚医生，随后她接受了基本的医疗和助产培训。她告诉我，直到2016年，她已经在她的村庄以及周围的村庄里接生了70多个婴儿：

我随时随地去帮助村民，经常是无偿的。[在工分制度下]，如果我被发现帮助其他村庄的妇女

delivery. So, I often took the risk to help them at night. I never refused anyone. They were our neighbours. They came for my help because they trusted me. I could not let them down ... This applied to all sick villagers too. They asked for my help because they trusted me. I could not betray their trust ... I was born and raised here. My family had lived here for a long time. Villagers knew me well. They chose me to be the [Barefoot Doctor] because they trusted me. Ever since I finished my Barefoot Doctor training I have been here and I am always around to help. For forty years, the only time I left the village was to buy drugs in Barkam. Villagers knew they would always find me whenever they needed my help ... When I turned sixty-five, I told villagers I would retire. They begged me to continue [being the village doctor]. They trust me and my skill more than they trust doctors and the hospital ... They knew I would never charge them high fees like in the hospital. Villagers from other villages also asked me to stay on.⁶³

Trust remained the defining quality of her presence in the village.

Similarly, in northern China's Shanxi province, Wanping, the former Barefoot Doctor from Qin County who stayed on to become the village doctor despite the low pay, told the interviewer that money was much less important than winning the affection of the villagers:

Villagers liked me because I always tried my best to help them. I never cared to bargain for money. Everybody was poor, why should I bother about money? ... I treated those villagers who could afford it, and I treated those who could not ... On one occasion, I charged one old villager less. It made her very happy. After that, she often asked for my help. We were all poor at the time. I never wanted to charge villagers more money than they could afford ... I slept at the health station so I could be available when villagers needed me ... It wasn't hard work for me. I took on the job because villagers trusted me ... When I saw villagers cry, I became very anxious. I feel for these people. Their problem was mine too. I wanted them to trust me.⁶⁴

The familiarity and comfort that is echoed as a positive feature of the local doctors and midwives is certainly not limited to rural China. In the extensive work of medical sociologists on allopathic medical

递送。所以，我经常冒险在晚上帮助他们。我从不拒绝任何人。他们是我们的邻居。他们来找我帮忙是因为他们信任我。我不能让他们失望……这也适用于所有生病的村民。他们寻求我的帮助是因为他们信任我。我不能背叛他们的信任……我在这里出生和长大。我的家人在这里住了很长时间。村民们很了解我。他们选择我当[赤脚医生]是因为他们信任我。自从我完成赤脚医生培训以来，我就一直在这里，我总是在身边提供帮助。四十年来，我唯一一次离开村子是在马尔康买药。村民们知道他们需要帮助时总能找到我……当我六十五岁时，我告诉村民们我要退休了。他们恳求我继续[当村医]。他们信任我和我的技术，胜过信任医生和医院……他们知道我永远不会像医院那样向他们收取高额费用。其他村庄的村民也要求我留下来。⁶³

信任仍然是她在这个村庄里存在的决定性品质。同样，在中国北方山西省，尽管工资很低，但仍留在秦县当乡村医生的前赤脚医生万平告诉采访者，钱远不如赢得村民的喜爱重要：

村民们喜欢我，因为我总是尽力帮助他们。我从不计较钱。大家都穷，我为什么要为钱烦恼呢？……我给那些付得起钱的村民看病，也给那些付不起钱的村民看病……有一次，我少收了一位老村民的钱。她非常高兴。从那以后，她经常寻求我的帮助。我们当时都很穷。我从来不想向村民收取他们负担不起的费用……我睡在卫生站，这样村民需要我的时候，我就可以随叫随到……这对我来说并不难。我接受这份工作是因为村民们信任我……当我看到村民哭泣时，我变得非常焦虑。我为这些人感到难过。他们的问题也是我的问题。我希望他们信任我。⁶⁴

当地医生和助产士所体现出的熟悉感和舒适感，被认为是他们的一个积极特点，当然这并非中国农村地区所独有。在医学社会学家对对抗疗法医学所做的大量研究中，

delivery, it can be seen that trust in the physician has a curative function in health care.⁶⁵ Trusting the physician, no matter what the therapeutic intervention, is at the core of effective health care, and this is just as true in rural settings in China as in a Western medical context.

As we learned from Muchu and Wanping, above, in small rural villages where virtually everyone was a friend, neighbour, or relative, winning the villagers' trust and respect was precious. Such moral bonds, either by kinship or social ties, increased the health workers' commitment and empathy. Alu, the former Barefoot Doctor from Yunnan's multi-ethnic Ninger County, is a Yi by ethnicity. Although he was illiterate, he was nominated by the villagers to be the Barefoot Doctor because he had some knowledge of herbal medicine. Villagers respected him, he recalled in a soft and slow voice, 'more than they did cadres from the big brigade ... Villagers always pretended they were poor and had no food when cadres from the big brigade turned up. But whenever I went to villagers' home, even if they had hardly anything to eat, they would use the food they saved for their children to feed me ... Some villagers also sent me chickens and eggs ... In our village, I did not earn work points [when I was the Barefoot Doctor]. Villagers donated their food and money to me ... The food I ate, and the money I earned came from villagers. It did not matter how late it was ... or how heavy the rain, I always went to visit villagers as soon as being called. I was always ready for their call.' The younger villagers called him 'Daddy' because he had helped to deliver them. Being respected by villagers made him proud: 'I was very proud when I was chosen to be the Barefoot Doctor ... So was my parent ... When I proposed to the girl I like ... she agreed to marry me. It's partly because I was the Barefoot Doctor. It gave me a face [status] in the community.'⁶⁶ As seen in this instance, trust in the practitioner results in the establishment of the social capital that defines status.

Being the Barefoot Doctor brought status. For He Yongjin, the village doctor in Yunnan province's Nanhua County, this was far more important than financial gain. Yongjin only took up the Barefoot Doctor's role in 1983 just as other Barefoot Doctors were abandoning their duty because of the low pay and the state's lack of interest in continuing the national program. Prior to being officially nominated as the Barefoot Doctor, Yongjin had been treating villagers privately with herbal remedies. He had learned herbal medicine from his grandfather, who was a folk herbalist. Although Yongjin would have made

通过医疗服务的提供, 我们可以看到, 对医生的信任在医疗保健中具有治疗功能。⁶⁵ 无论采取何种治疗干预措施, 信任医生都是有效医疗保健的核心, 这在中国农村地区与西方医疗背景下同样适用。

正如我们从上面的木楚和万平那里了解到的那样, 在几乎每个人都是朋友、邻居或亲戚的小村庄里, 赢得村民的信任和尊重是宝贵的。这种通过血缘关系或社会关系建立的道德纽带, 增强了卫生工作者的责任感和同理心。阿鲁是云南省多民族的宁洱县的前任赤脚医生, 是彝族人。尽管他不识字, 但他因懂一些草药知识而被村民推选为赤脚医生。他用柔和而缓慢的声音回忆说, 村民们尊重他, “胜过尊重大队的干部……大队的干部来的时候, 村民们总是装穷, 说没吃的。但是, 每当我到村民家时, 即使他们几乎没有吃的, 他们也会用留给孩子的食物来喂我……有些村民还送我鸡和鸡蛋……在我们村, 我当赤脚医生的时候没有挣工分 [。村民们把他们的食物和钱捐给我……我吃的食物和我挣的钱都来自村民。无论多晚……或者雨有多大, 只要有人叫我, 我总是去拜访村民。我随时准备着他们的召唤。”年轻的村民们叫他“爸爸”, 因为他曾帮助他们接生。受到村民的尊重让他感到自豪: “当我被选为赤脚医生时, 我感到非常自豪……我的父母也是如此……当我向我喜欢的女孩求婚时……她同意嫁给我。部分原因是我是一名赤脚医生。这给了我一张脸 [在社区中的地位]。”⁶⁶从这个例子可以看出, 对从业者的信任导致了社会资本的建立, 而社会资本定义了地位。

作为赤脚医生能带来地位。对于云南省南华县的乡村医生何永金来说, 这远比经济利益更重要。1983年, 当其他赤脚医生因工资低和国家对继续实施这项国家计划缺乏兴趣而放弃职责时, 永金才开始担任赤脚医生的角色。在被正式提名为赤脚医生之前, 永金一直在用草药为村民私下治疗。他从一位民间草药医生——他的祖父那里学到了草药医学。尽管永金本来可以

more money by continuing his private practice, he wanted to become a Barefoot Doctor to win the respect of villagers: 'In our village, the [village] doctor is the most respected person. Villagers respect [the village doctor] more than they respect the village cadres ... Whenever a villager slaughtered a pig, they would invite me for dinner ... When villagers have difficulties, they would always talk to the village doctor.' Respect is reciprocal. Because villagers respected and trusted him, Yongjin cared more for the villagers: 'When villagers have difficulties or if they are bothered about something, they always come to tell me ... I always listen to them and try to understand ... If I have prescribed medicine to sick villagers or treated them with acupuncture, I always follow up with them. Whenever I run into them, I always ask how they got on with the medicine or treatment, whether I could do more for them. Because I care for them, that's why they respect me.' Yongjin understood that giving care is more than just giving out pills. He was also empathetic and always willing to spend time with his patients and listen to their troubles. For more than thirty years, the village clinic has been the heart of the village and Yongjin the advocate for the community. He recalls:

Once, two families got into a dispute over a small plot of land ... it escalated into a serious fight. One person was hurt and damaged the soft tissue around his waist. He came to me and told me he needed to go to hospital and he would file a lawsuit against the other family. I comforted him and explained that he did not need hospital treatment for soft tissue injury. I also had a long talk with him about taking a step back and try making peace with the other family. Gradually he began to see my point and retreated. The man from the other family got frightened. The fear of being asked to pay compensation troubled him all night so that he could not sleep. The next day he came to see me about his insomnia. After I gave him some medicine, I had a long talk with him trying to persuade him to make peace with the family. There are many incidents like that.⁶⁷

Having status in the community made Yongjin more sensitive to individual villager's needs and thus a more effective healer. By being part of the community, Yongjin better understood the local worlds and the values embedded in them, which would have been alien to an outsider. Although Nanhua was designated by the Communist state as a

他本可以通过继续他的私人诊所赚更多的钱，但他想成为一名赤脚医生，以赢得村民的尊重：“在我们的村庄里，[村]医是最受尊敬的人。村民对[村医]的尊重超过了对村干部的尊重……每当有村民宰猪时，他们都会邀请我吃饭……当村民遇到困难时，他们总是会 and 村医谈谈。尊重是相互的。因为村民们尊重和信任他，永金更关心村民：“当村民们遇到困难或为某些事情烦恼时，他们总是来告诉我……我总是倾听他们并试图理解……如果我给生病的村民开了药或用针灸治疗了他们，我总是会跟踪他们。每当我遇到他们，我总是会问他们药物或治疗的效果如何，我是否可以为他们做更多的事情。因为我关心他们，所以他们尊重我。”永金明白，给予关怀不仅仅是给药。他还富有同情心，总是愿意花时间和病人在一起，倾听他们的烦恼。三十多年来，村诊所一直是村庄的中心，永金一直是社区的倡导者。他回忆道：

从前，有两个家庭因为一小块土地发生争执……事态升级为一场严重的争斗。其中一人受伤，腰部周围的软组织受损。他来找我，告诉我他需要去医院，并会起诉对方家庭。我安慰了他，并解释说软组织损伤不需要住院治疗。我还和他长谈，劝他退一步，尝试与对方家庭和解。渐渐地，他开始明白我的意思，退缩了。另一家人的人害怕了。被要求赔偿的恐惧让他整夜不安，无法入睡。第二天，他来找我看失眠。我给他开了一些药后，和他长谈，试图说服他与那家人和解。像这样的事件有很多。⁶⁷

在社区中拥有地位使永金对村民的个人需求更加敏感，从而成为一个更有效的治疗者。通过成为社区的一份子，永金更好地理解当地的世界以及其中蕴含的价值观，这些价值观对于外人来说可能是陌生的。虽然南华被共产主义国家指定为

Yi ethnic region, in real terms it is a multi-ethnic society. Being a Bai himself, Yongjin is sensitive to the diverse needs of this multi-ethnic community. Although he was originally trained as a herbalist by his grandfather, being attuned to the villagers' different needs and economic capacities, he is always willing to be eclectic in his therapeutic interventions. When he saw that younger villagers were reluctant to take herbal medicine because they disliked the taste, he would prescribe Western drugs for them. For those villagers who could not afford the standard treatment, he would try his best to provide a combination of therapies to lower the costs. Whenever he prescribed a medicinal herb that grew locally, he would tell the villager where to harvest the herb so that they did not have to spend the money to buy it. 'I would make less money this way. But being a doctor, I want to be able to help villagers with their pains. I don't mind if it means less financial gains for myself ... On occasions when some villagers had no money to pay for their treatment or drug, I would write down what they owe in a book ... If they have chicken or eggs, I would pay them market price for the goods. With the money I paid them, they would pay off what they owed for the treatment ... I don't like the method, but there is no other means. Villagers don't have enough cash.'⁶⁸ The social capital acquired by the medical practitioner outweighed any diminishment of their financial status.

After 1983, with the marketization of health care delivery, there arose the perception of a widespread problem of overcharging by health care providers, from individual doctors to hospitals. As part of the community, very few Barefoot Doctors would dream of cheating or overcharging villagers. In such a close community setup, dishonesty was not socially acceptable. Like Yongjin, Shimeng was a former Barefoot Doctor, in his case from Daotian village in Shandong province in eastern China. He also learned herbal medicine from his grandfather. 'Today, there are so many medicines on offer. They [doctors at hospitals] just want to fool us in order to get our money. In the old days, when Shimeng was the Barefoot Doctor, he would give us some herbs, it was cheap, and it always worked,' one villager noted, speaking of him fondly.

For this villager, as for many others in different parts of rural China, the other thing that made them trust the Barefoot Doctor in their village more than they would the current state system with its focus on hospital care was that the Barefoot Doctors were always there when villagers needed them. He or she was also both pleasant and

彝族地区, 实际上是一个多民族社会。作为一名白族人, 永金对这个多民族社区的各种需求非常敏感。尽管他最初是由祖父培养成为一名草药医生, 但他能够敏锐地了解到村民的不同需求和经济能力, 因此他总是愿意在治疗干预方面采取折衷主义的态度。当他看到年轻的村民因为不喜欢草药的味道而不愿意服用时, 他会为他们开西药。对于那些无力负担标准治疗的村民, 他会尽力提供结合疗法来降低成本。每当他开出当地生长的药草时, 他都会告诉村民在哪里采摘, 这样他们就不必花钱购买。 “这样做我会少挣钱。但作为一名医生, 我希望能够帮助村民减轻痛苦。我不介意这意味着我自己的经济收益减少 …… 有时, 当一些村民没有钱支付治疗或药物费用时, 我会把他们欠的钱记在一本书上 …… 如果他们有鸡或鸡蛋, 我会按市场价格支付给他们。用我支付给他们的钱, 他们可以还清治疗费用 …… 我不喜欢这种方法, 但没有其他办法。村民没有足够的现金。’⁶⁸ 医疗从业人员获得的社会资本超过了其财务状况的任何减损。

1983 年之后, 随着医疗保健服务的市场化, 人们普遍认为医疗保健服务提供者存在过度收费的问题, 从个体医生到医院都存在。作为社区的一份子, 很少有赤脚医生会想到欺骗或过度收费村民。在如此紧密的社区环境中, 不诚实的行为在社会上是不可接受的。像永金一样, 史孟也曾是一名赤脚医生, 他来自中国东部山东省道田村。他还从祖父那里学习了草药医学。 “今天, 有这么多的药物可供选择。他们 [医院里的医生] 只想骗我们, 为了得到我们的钱。过去, 当史孟还是赤脚医生的时候, 他会给我们一些草药, 既便宜又有效, ”一位村民深情地说起他。

对于这位村民来说, 就像中国农村许多其他地方的村民一样, 让他们比信任目前专注于医院护理的国家体系更信任赤脚医生的另一个原因是, 赤脚医生总是在村民需要他们的时候出现。而且, 他们既令人愉快又

approachable: 'In those days, in addition to Shimeng, there was a lady Barefoot Doctor ... There were two of them in the village clinic. Whenever they were called, they always went to [the villager in need] as fast as they could. They were very approachable, and they had a good attitude. Nowadays, if we go to hospital, the doctors charge us a lot of money. In addition, the nurses always treat us badly,' one interviewee lamented.⁶⁹ Agreeing with him, in a village from the other side of Korean peninsula, outside of Qingdao, one inhabitant told the interviewer: 'Because the Barefoot Doctor was in the village, if I had a problem I'd go to see her. If she could sort out my problem, that's great. Since we had no money, and I had nowhere else to go, it's good to have her around. Day in or day out, we saw each other all the time. She was always approachable and pleasant, and she was honest if she could not solve my problem. After all it doesn't really matter if she could not solve my problem because she was helpful to me. I wish the doctor today would be as kind-hearted and less obsessed with making money.'⁷⁰ Trust, as becomes evident in such comments, is an inherent component in the efficacy of the practitioner.

Social contexts define medical practice. Trust in the local and the therapy tends to be more effective, even if it is simply because of a willingness to undertake it. Distrust, whether caused by distance or cost, leads inevitably to poorer health outcomes. Hence, the Barefoot Doctors (and subsequently village doctors) were often more efficacious because they slowly assumed the reliable and locally recognized roles of the priest, the witch doctor, and the herbalist. Indeed, by integrating various modalities of intervention beyond the state-sanctioned allopathic medicine or TCM into their practice, they heightened their trustworthiness. These patterns could not be, and were not, directed from above. Their wide variation from region to region, from village to village, from individual to individual meant that the very syntheses were local and personal. Yet trust is not a stable concept. In times of radical political and social change, such as collectivization under Mao and the post-Mao economic reform, traditional family life and community structure were eroded. The existing systems of trust were at risk. In times of extreme scarcity, as during the time of the Great Leap famine (1958-62), selfishness became a means of survival. Under such conditions, even members of the same family did not trust each other, while, on the other hand, when self-worth is measured only in monetary value, as with the marketization in post-Mao China, connectedness and trust give way to competition and distrust.

平易近人：一位受访者感叹道：“在那些日子里，除了石门，还有一位赤脚医生……村里的诊所两个。每当他们被叫到时，他们总是尽快去帮助[需要帮助的村民]。他们非常平易近人，态度也很好。现在，如果我们去医院，医生会收取我们很多钱。此外，护士总是对我们不好。”⁶⁹在韩国半岛另一边的村庄，青岛郊外，一位居民同意他的说法，并告诉采访者：“因为赤脚医生在村里，如果我有问题，我会去看她。如果她能解决我的问题，那就太好了。因为我们没有钱，而且我无处可去，所以有她在身边真好。日复一日，我们总是见面。她总是平易近人、和蔼可亲，如果她解决不了我的问题，她也会诚实地告诉我。毕竟，她是否能解决我的问题并不重要，因为她对我有帮助。我希望今天的医生也能像她一样善良，少一些对赚钱的执着。”⁷⁰正如这些评论所表明的那样，信任是执业者效力的一个内在组成部分。

社会环境定义了医疗实践。对当地和疗法的信任往往会更有效，即使仅仅是因为愿意接受它。不信任，无论是由于距离还是成本造成的，都不可避免地导致更差的健康结果。因此，赤脚医生（以及后来的乡村医生）通常更有效，因为他们慢慢地承担了牧师、巫医和草药师的可靠和当地认可的角色。事实上，通过将国家批准的对抗疗法或中医以外的各种干预方式融入到他们的实践中，他们提高了他们的可信度。这些模式不可能，也未曾从上面进行指导。它们从一个地区到另一个地区，从一个村庄到另一个村庄，从个人到个人之间存在着广泛的差异，这意味着这些综合体本身是地方性和个人性的。然而，信任并不是一个稳定的概念。在激烈的政治和社会变革时期，如毛泽东时代的集体化和后毛泽东时代的经济改革，传统的家庭生活和社区结构受到了侵蚀。现有的信任体系正面临风险。在极端匮乏的时期，如大跃进饥荒时期（1958-62年），自私成为一种生存手段。在这种情况下，即使是同一家庭的成员也不信任彼此，另一方面，当自我价值仅仅用金钱来衡量时，就像后毛泽东时代的中国市场化一样，联系和信任让位于竞争和不信任。

DELIVERING HEALTH WITH
'A BUNCH OF HERBS AND ONE ACUPUNCTURE
NEEDLE' AS WELL AS ANTIBIOTICS

In converting the Barefoot Doctor scheme into a national program, its designers neglected the local. As a result, instead of motivating the community to care for their environment or the health providers becoming advocates for the community, the Barefoot Doctors were forced to act on their own with minimal support from urban experts, while redistributing the scarce national resources in delivering the promised 'Health Care for All'. On the ground, 'Health Care for All' was often reduced to mean 'one cure for all'. Such a reduction not only undermined community responsibility but also weakened the notion that care was a person-to-person exchange. The hegemonic identity of the Barefoot Doctor as the non-professional health worker who was *of* as well as *for* the rural masses, encapsulated by the propaganda image of Barefoot Doctors delivering health care with 'a Bunch of Herbs and One Acupuncture Needle' that adorned village health centres, further hindered individual Barefoot Doctors' competence to deliver care that addressed villagers' actual health needs. In the case of Tonbai County in Henan province, we learn from an official source that the growing popularity of the 'herbal panacea' saved local Barefoot Doctors from 'wasting' time on one-to-one encounters with patients. Although the source praised this as a positive, 'time efficient' health strategy, it is hard to imagine how it would generate a better health outcome than if the Barefoot Doctors spent more time addressing the health needs of individual villagers. In reality, it often facilitated Barefoot Doctors slacking.⁷¹ Many of them had only minimal or no knowledge of herbal medicine.⁷² Unlike the herbal medicines prescribed by traditional healers, who had tested the herbs and had years of experience in administering them, the 'herbal brews' or 'honey pills' decocted by many Barefoot Doctors in mass batches to prevent seasonal flu and malaria or to combat tuberculosis outbreaks were often untested. Yet, as part of Co-operative Medicine, entire villages were obliged to consume them collectively, regardless of the known assumption that individuals responded to different medicinal herbs differently. As with the old Chinese saying that 'all medicine contains some poison', it was not surprising that such intervention measures sometimes caused more harm than the cure or prevention they were deemed to effect.

提供 带有 的卫生保健
“一堆草药 和一根针灸
针头’ 以及抗生素

在将赤脚医生计划转变为国家项目时，其设计者忽略了当地情况。结果，赤脚医生没有激励社区关心他们的环境，也没有使卫生保健提供者成为社区的倡导者，而是被迫在城市专家的极少支持下自行行动，同时重新分配稀缺的国家资源，以提供承诺的“人人享有卫生保健”。在基层，“人人享有卫生保健”通常被简化为“一种疗法适用于所有人”。这种简化不仅削弱了社区责任，而且削弱了护理是人与人之间交流的观念。赤脚医生作为非专业卫生工作者的霸权身份，既属于农村大众，又服务于农村大众，体现在宣传图片中，赤脚医生用“一堆草药和一根针灸针”提供卫生保健，装饰着村卫生所，进一步阻碍了赤脚医生提供满足村民实际健康需求的护理的能力。在河南省桐柏县的案例中，我们从官方来源了解到，“草药万灵药”的日益普及使当地赤脚医生免于将时间“浪费”在与患者的一对一接触上。尽管该消息来源称赞这是一种积极的“时间高效”的卫生保健策略，但很难想象它会比赤脚医生花更多时间解决村民的健康需求产生更好的健康结果。实际上，它通常会助长赤脚医生的懈怠。⁷¹ 他们中的许多人对草药的知识很少或根本没有。⁷² 与传统治疗师开出的草药不同，传统治疗师已经测试过草药并在管理方面拥有多年的经验，许多赤脚医生批量煎制的“草药”或“蜂蜜丸”用于预防季节性流感和疟疾或对抗结核病爆发，通常未经测试。然而，作为合作医疗的一部分，整个村庄都被迫集体食用它们，而不管个人对不同药草的反应不同的已知假设。正如中国古语所说“一切药物都有毒”，令人惊讶的是，这种干预措施有时造成的危害大于它们本应产生的治疗或预防效果。

To an extent, this problem of a 'herbal panacea' was little different from the 'magic pill' [placebo] problem in allopathic medicine.

More worrying was that Barefoot Doctors were often expected to perform medical miracles using traditional interventions such as herbal medicine or acupuncture. While this worked for a few who had been experienced practitioners prior to being appointed as Barefoot Doctors, others ended up carrying out 'revolutionary' medical experiments that could be potentially risky. For instance, many Barefoot Doctors who had no previous experience or training in acupuncture experimented by sticking needles into their own body while holding an acupuncture manual in their hands. Others competed to test newly decocted herbal medicine on themselves, sometimes leading to injury. In some cases, Barefoot Doctors lacerated their own body in order to test whether a particular herb could be used to treat wounds.⁷³ To help Barefoot Doctors in developing new herbal medicine, it was reported that one army medical worker ingested a poisonous herb and that he cut open an artery in his own jaw to test whether a herb could stop external bleeding. Despite awareness of the potential risks, the official propaganda encouraged such 'revolutionary' experiments. In one case, when a team of army medical workers experimented with some herbs on themselves, it was reported that 'some made them dizzy, others made them vomit or caused diarrhoea, but nothing stopped them. Finally, they found an effective treatment.'⁷⁴ Self-experimentation is nothing new; there is a long history of it in allopathic medicine. But historians of medicine have focused more on those cases of self-experimentation that were successful.⁷⁵ These have tended to be based on 'good science', such as Max von Pettenkofer's 1894 ingestion of cholera bacteria or David Pritchard's self-experimentation in 2004 with hookworm and asthma, rather than those that stemmed from false or ineffective theories and resulted only in health catastrophes. The Chinese cases were as mixed as might be expected, given the politics of health care in the PRC.

Rather than promoting Chinese or traditional medicines, these 'revolutionary' medical experiments were often amalgamated under the official label of 'Traditional Chinese Medicine', and encouraging the Barefoot Doctor to deliver a 'cure', rather than having one-on-one encounters, also paved the way for the long-term problem of antibiotic over-prescribing (and eventual resistance) in rural primary care in China. Although the state encouraged Barefoot Doctors to

在某种程度上, 这种“草药万灵药”的问题与对抗疗法中[安慰剂]的“神奇药丸”问题并没有什么不同。

更令人担忧的是, 人们常常期望赤脚医生使用传统干预措施

(如草药或针灸)来创造医学奇迹。虽然这对一些在被任命为赤脚医生之前已经有经验的从业者来说是有效的, 但其他人最终进行了可能具有潜在风险的“革命性”医学实验。例如, 许多以前没有针灸经验或训练的赤脚医生, 在拿着针灸手册的同时, 尝试将针扎入自己的身体。还有一些人争相在自己身上测试新煎好的草药, 有时会导致受伤。在某些情况下, 赤脚医生会撕裂自己的身体, 以测试某种草药是否可以用于治疗伤口。⁷³ 为了帮助赤脚医生开发新的草药, 据报道, 一名军队医务人员摄入了一种有毒草药, 并切开自己下巴上的一条动脉, 以测试一种草药是否可以止住外出血。尽管意识到了潜在的风险, 但官方宣传鼓励这种“革命性”的实验。在一个案例中, 当一个军队医务人员团队在自己身上试验一些草药时, 据报道, “有些人感到头晕, 另一些人感到呕吐或腹泻, 但什么也阻止不了他们。最后, 他们找到了一种有效的治疗方法。”⁷⁴ 自我实验并不是什么新鲜事; 在对抗疗法中, 有着悠久的自我实验历史。但医学史学家更关注那些成功的自我实验案例。⁷⁵ 这些实验往往是基于“好的科学”, 例如马克斯·冯·佩滕科弗尔(Max von Pettenkofer)在1894年摄入霍乱细菌, 或戴维·普里查德(David Pritchard)在2004年用钩虫和哮喘进行自我实验, 而不是那些源于错误或无效的理论, 最终只导致健康灾难的实验。鉴于中华人民共和国的医疗保健政治, 中国的案例正如人们预期的那样错综复杂。

而是 than 推广 中国的 or 传统 药物, 这些“革命性”的医疗实验通常被合并在“中医药”的官方标签下, 并且鼓励赤脚医生提供“治疗”, 而不是进行一对一的接触, 这也为中国农村初级保健中长期存在的抗生素过度处方问题(以及最终的耐药性)铺平了道路。尽管国家鼓励赤脚医生

integrate Western medicine with Chinese medicine, in reality many Barefoot Doctors preferred delivering allopathic medicine, often complying with the demand of the patient. As Fang Xiaoping's study demonstrates, in the 1970s an increasing number of rural villagers came to prefer allopathic medicine as being quicker in treating acute illnesses and hence more effective than herbal medicine.⁷⁶ In addition, for some Barefoot Doctors, allopathic as well as patent Chinese medicine was more convenient to obtain and use: 'Initially, we were trained to use a combination of Chinese medicine and Western medicine. When I was younger, I used Chinese medicine a lot. But as I am getting older, my foot began to give me a lot trouble. It's difficult for me to go up to the hills to pick herbs. Thus I only use Western and patent Chinese medicine now. Western medicine is very effective,' explained Muchu, who lives in a mountainous village in the Himalayas.⁷⁷ Both types of medication could be found in regional pharmacies, and while patent Western and Chinese medicines were clearly more expensive, they bore the imprimatur of 'scientific' (e.g., urban) medical practice. Such changing perceptions have evolved over the past forty years with the erosion of traditional and local methods and the increased prominence and knowledge of both allopathic medicine and systemized TCM.

The problem was not, however, the preference for allopathic medicine or Chinese medicine. As Chen Zhiqian rightly pointed out, the risk lay in the many Barefoot Doctors – who were only given practical training on how to carry out some basic medical procedures and provide immunizations – who were impelled to do clinical work, from diagnosis to treatment, at the same level as a well-trained physician. Chen's daughter Fujun recalled that 'he often talked about the fact that this could lead to serious risk.'⁷⁸ And there were indeed many accidents. In the case of one village in Ningxiang County, Hunan province, the village Barefoot Doctor was also the head of the village Women's Association. This woman had neither medical nor public health training and knew nothing about medicine. She was given the Barefoot Doctor role because she was a relative of the brigade party secretary. Soon after she took the role, accidents began to happen – for example, by mistaking arsenic for talcum powder, she accidentally caused nine deaths.⁷⁹ Fujun's own experience in rural Sichuan also confirmed her father's concern. For several years in the early 1970s, Fujun was sent to the Sichuan countryside to assist with rural health work. She recalled that many Barefoot Doctors

将西医与中医相结合，但实际上许多赤脚医生更喜欢提供对抗疗法药物，通常是遵从患者的要求。正如方小平的研究表明，在1970年代，越来越多的农村村民开始喜欢对抗疗法药物，认为它在治疗急性病方面更快，因此比草药更有效。⁷⁶ 此外，对于一些赤脚医生来说，对抗疗法药物以及中成药更容易获得和使用：“最初，我们接受的培训是使用中西医结合的方法。当我年轻的时候，我经常使用中药。但是随着我年龄的增长，我的脚开始给我带来很多麻烦。我很难上山采药。因此，我现在只使用西药和中成药。西药非常有效，”住在喜马拉雅山区村庄的木楚解释说。⁷⁷ 这两种类型的药物都可以在区域药房找到，虽然西药和中成药显然更贵，但它们带有“科学”（例如，城市）医疗实践的印记。过去四十年来，随着传统和地方方法的衰落，以及对抗疗法药物和系统化中医的日益突出和普及，这种不断变化的观念也在不断演变。

然而，问题不在于对对抗疗法或中医药的偏好。正如陈志潜正确指出的那样，风险在于许多赤脚医生——他们只接受过如何进行一些基本医疗程序和提供免疫接种的实践培训——却被驱使从事临床工作，从诊断到治疗，达到与训练有素的医生相同的水平。陈的女儿福君回忆说，“他经常谈到这样一个事实，即这可能会导致严重的风险。”⁷⁸ 事实上，也确实发生了很多事故。在湖南省宁乡县的一个村庄里，村里的赤脚医生同时也是村妇联主任。这名妇女既没有接受过医疗培训，也没有接受过公共卫生培训，对医学一无所知。她之所以被赋予赤脚医生的角色，是因为她是生产队党支部书记的亲戚。在她担任这个角色后不久，事故就开始发生了——例如，她误将砒霜当成滑石粉，意外地导致了九人死亡。⁷⁹ 福君自己在四川农村的经历也证实了她父亲的担忧。在1970年代初的几年里，福君被派往四川农村协助农村卫生工作。她回忆说，许多赤脚医生

treated patients with injections. One model Barefoot Doctor did not understand that hypodermic needles and syringes needed thorough sterilization after each use, and instead of delivering the promised cures, this Barefoot Doctor had helped the spread of Hepatitis A.⁸⁰ Such cases as this left the Barefoot Doctors with the reputation of being 'shoddy' doctors.⁸¹

Fujun was most troubled by the problem of unrestricted and unsupervised use of antibiotics amongst the Barefoot Doctors: 'I witnessed many Barefoot Doctors in rural Sichuan doing things they were not qualified to do. One thing I noticed was that they randomly gave out pills. They often randomly gave antibiotics to any villager who presented with a fever. This was because they did not know the cause [of the fever]. They were simply not trained to do diagnosis.'⁸² The problem of the Barefoot Doctors, the rural primary caregiver, over-prescribing or misusing antibiotics was by no means limited to rural Sichuan. A 2012 joint study by researchers at Kunming Medical University and the Medical Faculty of Prince Songkla University showed that in many parts of rural Yunnan, the problem of over-prescribing antibiotics to paediatric patients was still widespread amongst rural primary caregivers, almost all of them former Barefoot Doctors. In addition, the research found that the level of supportive care was poor.⁸³ Undoubtedly, as Fujun pointed out, the Barefoot Doctors' inconsistent and inadequate training had contributed to their clinical incompetence and heightened patient risk, hence the problem of their overreliance on antibiotics as a cure-all. On the other hand, the official promise of 'Health Care for All' that came with the Co-operative Medical service, as advertised by slogans such as 'It's Free to See a Barefoot Doctor', created a demand for a therapeutic 'cure'. This was made worse by the propaganda that exaggerated the Barefoot Doctors' 'infallible abilities' in 'curing' many severe illnesses, including removing tumours. Burdened by the demand and the heightened expectations, and in addition to their clinical incompetence, many Barefoot Doctors, as well as other commune hospital doctors, resorted to antibiotics, the 'magic bullet' of their time. For instance, in Chonming Island, just outside of Shanghai, an official report in 1975 showed that primary care doctors at the Xinmin commune hospital prescribed antibiotics as an 'insurance policy' to every sick villager they saw.⁸⁴

The Long 1970s were globally the 'Golden Era' of antibiotic innovation and broadening use. In the excitement surrounding their

为病人注射治疗。有个赤脚医生模式是不明白皮下注射针头和注射器每次使用后都需要彻底消毒的，结果不但没有带来承诺的疗效，反而帮助传播了甲型肝炎。⁸⁰ 这类事件让赤脚医生落下了“马虎”医生的名声。⁸¹

最让福建感到困扰的是赤脚医生对抗生素的无限制和不受监督的使用问题：“我亲眼目睹了四川农村的许多赤脚医生做着他们没有资格做的事情。我注意到的一件事是，他们随意发药。他们经常随意给任何发烧的村民开抗生素。这是因为他们不知道发烧的原因[。]他们根本没有接受过诊断训练。”⁸² 赤脚医生，即农村的主要护理人员，过度开药或滥用抗生素的问题绝不仅限于四川农村。昆明医科大学和博仁大学医学院的研究人员在2012年进行的一项联合研究表明，在云南农村的许多地区，农村初级保健人员（几乎都是以前的赤脚医生）向儿科患者过度开抗生素的问题仍然普遍存在。此外，研究发现，支持性护理水平很差。⁸³ 毫无疑问，正如福建指出的那样，赤脚医生前后矛盾且不足的培训导致了他们的临床无能，并增加了患者的风险，因此导致他们过度依赖抗生素作为包治百病的药物。另一方面，合作医疗服务承诺的“人人享有医疗保健”的官方承诺，正如“看赤脚医生是免费的”等口号所宣传的那样，创造了对治疗性“治疗”的需求。宣传夸大了赤脚医生在“治疗”许多严重疾病（包括切除肿瘤）方面的“万无一失的能力”，这使得情况变得更糟。由于需求的负担和期望的提高，以及他们的临床无能，许多赤脚医生以及其他公社医院的医生都诉诸于抗生素，这是他们那个时代的“灵丹妙药”。例如，在上海郊外的崇明岛，1975年的一份官方报告显示，新民公社医院的初级保健医生给他们看过的每一位生病的村民都开抗生素作为一种“保险政策”。⁸⁴

从全球范围来看，20世纪70年代的“漫长十年”是抗生素创新和广泛应用的“黄金时代”。人们对抗生素感到无比兴奋，

successes in treating a range of previously incurable diseases and serious infections, a broad range of antibiotics were made readily available to the general public, first in the West and then around the world. In China, the mass production of penicillin had begun in 1952 with the outbreak of the Korean War, and that created the initial demand. Until the early 1960s, however, domestic production remained low, and as a result the price was high. The prohibitively high cost meant that antibiotics were generally out of reach for ordinary people.⁸⁵ By the mid-1960s, China successfully obtained the technology and the licensing to produce semi-synthetic antibiotics. This resulted in a huge increase in quantity, driving down the price. By August 1969, the price of more than 1,230 kinds of patented medicines had been slashed significantly, including a whole range of domestically produced antibiotics. The price of domestically produced penicillin, for instance, was 90 per cent lower than in 1952 and 46 per cent lower than the month before! Sulphanilamide tablets, too, became 63 per cent cheaper than in 1952 and 13.3 per cent cheaper than the previous month. This radical reduction in the price of drugs, according to the *People's Daily*, had reduced the health care burden of the masses living in mountainous, rural, and ethnic regions: 'It helped to promote and consolidate the rural Co-operative Medical service and hence the development of rural health care.'⁸⁶ Following this banner article was a news report about Barefoot Doctors in rural Shanghai delivering 'revolutionary' health care to villagers. The increased presence of the Barefoot Doctors as well as the increased affordability of Western drugs – with antibiotics showing the most significant drop in price – were portrayed as the victories of Mao's revolutionary approach to health for the rural masses. The imperative became for the Barefoot Doctor to deliver antibiotics whatever the need or the diagnosis.

Unrestricted and unsupervised use of antibiotics by the Barefoot Doctors, endorsed by the experts in the hospitals and sanctioned by the state, helped to create a drug dependency in rural China. As Chen Zhiqian observed, the majority of villagers in the rural countryside 'depended [on the Barefoot Doctor as their primary caregiver] on health matters and respected their judgement'.⁸⁷ Yet prior to the RCMS, the majority of villagers in rural China had had minimal or no contact with most allopathic medicine. This was even more the case with antibiotics due to their prohibitively high price and limited availability. Even in the early years of the RCMS, antibiotics

在治疗一系列以前无法治愈的疾病和严重感染方面取得成功后, 种类繁多的抗生素开始向普通大众广泛普及, 首先在西方, 然后在世界各地。在中国, 青霉素的大规模生产始于 1952 年朝鲜战争爆发, 这产生了最初的需求。然而, 直到 20 世纪 60 年代初, 国内产量仍然很低, 因此价格也很高。高得令人望而却步的价格意味着普通人根本无法负担抗生素。⁸⁵ 到 20 世纪 60 年代中期, 中国成功地获得了生产半合成抗生素的技术和许可。这导致产量大幅增加, 价格下降。到 1969 年 8 月, 包括一系列国产抗生素在内的 1230 多种专利药品的售价大幅下调。例如, 国产青霉素的价格比 1952 年低 90%, 比前一个月低 46%! 磺胺药片的价格也比 1952 年便宜 63%, 比上个月便宜 13.3%。根据《人民日报》的报道, 这种药品价格的急剧下降减轻了居住在山区、农村和少数民族地区的群众的医疗负担: “这有助于促进和巩固农村合作医疗服务, 从而促进农村卫生保健事业的发展。”⁸⁶ 在这篇重要文章之后, 是一篇关于上海农村的赤脚医生为村民提供“革命性”医疗服务的新闻报道。赤脚医生的不断涌现以及西方药品可负担性的提高——其中抗生素的价格降幅最为显著——被描绘成毛泽东农村群众卫生革命性方针的胜利。当务之急是赤脚医生无论有何需求或诊断, 都要提供抗生素。

赤脚医生不受限制且不受监督地使用抗生素, 得到了医院专家的认可和国家批准, 这助长了中国农村地区对药物的依赖。正如陈志谦观察到的, 农村地区的大多数村民“在健康问题上[依赖]赤脚医生作为他们的主要护理人员, 并尊重他们的判断”。⁸⁷ 然而, 在农村合作医疗制度(rcms)实施之前, 中国农村地区的大多数村民很少或根本没有接触过大多数对抗疗法药物。由于抗生素价格过高且供应有限, 情况更是如此。即使在农村合作医疗制度的早期, 抗生素

remained a 'luxury' reserved for cadres and those who had good relations with the cadres or the Barefoot Doctors. This made them more desirable when they became readily available, since they had the prestige of status associated with their use. The craving for antibiotics was made worse by the little or no access to hospital care as a result of distance, unavailability, or unaffordability. For many rural villagers, antibiotics were a more affordable and accessible alternative. For the majority of them, the practice of self-medication was nothing new. Until the late nineteenth century, the rural masses in China smoked opium to manage a whole range of illnesses, from severe diarrhoea to malaria, or as a prophylactic measure against diseases. However, after the more powerful opiate, morphine, was introduced to China by medical missionaries in the late nineteenth century to 'cure' the Chinese of their opium 'addiction', opium gradually lost its importance as medicine.⁸⁸ By the end of the 1970s, as Fang Xiaoping has shown, many of the younger generation preferred allopathic medicine or patent Chinese medicine over herbal medicine. Antibiotics dominated the consumption of allopathic medicine for their perceived effectiveness in treating many common diseases.⁸⁹ Like opium in the eighteenth and nineteenth centuries, antibiotics gradually became the panacea for the masses in China in the late twentieth century.

In the early 1970s, villagers in different parts of China, often supported by their cadres, opened illicit factories, established black markets, and rented out collective plots for private use. This was partly driven by the official campaign to 'Learn from Dazai' by practising 'self-reliance', but more importantly by the human instinct to 'save oneself' (自救) from danger, in this case the extreme poverty imposed upon them by collectivization. An increasing number of villagers roamed around the country illicitly trading anything they could get their hands on, or which they had produced themselves, often with forged documents. Some Barefoot Doctors abused their power during this furtive move to a quasi-market economy by charging villagers higher fees or peddling medicines on the black market to supplement their income. Many grassroots cadres simply turned a blind eye to such activities, since they also profited from it. In northern parts of rural Zhejiang, where about 18 per cent of the skilled labour had left to start businesses elsewhere, agricultural collectivization gradually collapsed in the late 1970s without any policy changes from above.⁹⁰ In the years immediately following Mao's

仍然是为干部以及那些与干部或赤脚医生关系良好的人保留的“奢侈品”。这使得它们在变得容易获得时更受欢迎，因为它们具有与其使用相关的地位声望。由于距离、不可用或负担不起等原因，很少或根本无法获得住院治疗，这使得人们对抗菌素的渴望更加强烈。对于许多农村村民来说，抗生素是一种更实惠且更容易获得的替代品。对于他们中的大多数人来说，自我用药的做法并不新鲜。直到 19 世纪末，中国农村的大众吸食鸦片来治疗各种疾病，从严重腹泻到疟疾，或者作为预防疾病的措施。然而，在 19 世纪末，更强效的鸦片类药物吗啡被医疗传教士引入中国，以“治愈”中国人的鸦片“瘾”后，鸦片作为药物的重要性逐渐丧失。⁸⁸正如方小平所表明的那样，到 20 世纪 70 年代末，许多年轻一代更喜欢西医或中成药而不是草药。由于抗生素在治疗许多常见疾病方面的公认疗效，它在西医消费中占据主导地位。⁸⁹就像 18 世纪和 19 世纪的鸦片一样，抗生素在 20 世纪末逐渐成为中国大众的万灵药。

在 20 世纪 70 年代初，中国不同地区的村民，通常在干部的支持下，开设非法工厂，建立黑市，并出租集体土地供私人使用。这部分是受到官方“学习大寨”通过实践“自力更生”运动的推动，但更重要的是人类“自救”的本能，在这种情况下，集体化给他们带来了极度贫困。越来越多的村民在全国各地非法交易他们能弄到手的任何东西，或者他们自己生产的东西，而且经常使用伪造的文件。一些赤脚医生在向准市场经济偷偷转移的过程中滥用权力，向村民收取更高的费用，或在黑市上兜售药品以增加收入。许多基层干部对这些活动睁一只眼闭一只眼，因为他们也能从中获利。在浙江北部农村地区，约有 18% 的熟练劳动力外出创业，农业集体化在 20 世纪 70 年代末逐渐崩溃，而上面没有任何政策变化。⁹⁰在毛泽东去世后的几年里

death, more and more villagers in different parts of China joined forces to push open the door for economic reform. From Sichuan province's Guanghan County to Anhui province's Fengyang County, local authorities gradually abandoned the People's Communes, introduced their own local policies, and allowed household contracting. By 1981, as Chen Tingyuan, the party secretary at Fengyang County, admitted: 'Household contracting was like an irresistible wave, spontaneously topping the limits we had placed, and it could not be suppressed or turned around.'⁹¹ Despite its initial reluctance, as Kate Xiao Zhou has argued, the CCP Central Committee gradually came to the understanding that it had little power to stop the widespread illegal transfer of land to households. In Anhui's Fengyang County, previously famous for its huge number of beggars, it was very evident that the household responsibility system had improved grain outputs and increased household income. Villages throughout the country demanded that the same system be applied to them. To retain its legitimacy to power, the CCP Central Committee had to give people what they wanted, and in 1983 it officially abandoned agricultural collectivization.⁹² With the People's Communes dissolved, the RCMS officially came to an end. Health centres as well as commune clinics were closed down, and Barefoot Doctors lost their livelihood.⁹³ In order to make a living, those with good skills opened private practices and charged high prices. Many also joined the illicit drug trade.

The problem of antibiotics abuse escalated with the post-Mao decentralization and marketization of health care. As of the early 1980s, the number of private drug manufacturers – many of which were illicit – mushroomed, coupled to a new network of drug distribution in the countryside. In the coastal province of Fujian, for instance, using forged approval documents and more than 100,000 boxes of fake trademarks, fifty-eight fake drug factories were built between 1983 and 1985. More than 700 peddlers were employed to promote the sale of these illicit products to thousands of hospitals or other health delivery units in twenty-nine provinces and cities using unethical measures such as undercutting costs. The total sales value of the counterfeit drugs was more than 35 million *yuan*. In Jingjiang County's Hankou village alone, twenty-two fake drug factories were built, and 80 per cent of villagers were employed in the business of producing counterfeit drugs.⁹⁴ Partly for convenience, partly for more profit, because many unlicensed private producers

越来越多中国不同地区的村民联合起来,为经济改革打开了大门。从四川省广汉县到安徽省凤阳县,地方政府逐渐放弃了人民公社,推出了自己的地方政策,并允许家庭承包。正如凤阳县委书记陈庭元在1981年承认的那样:“家庭承包就像一股不可抗拒的浪潮,自发地突破了我们设置的限制,无法压制或扭转。”⁹¹正如周凯迪所论证的那样,尽管最初有些不情愿,但中国共产党中央委员会逐渐认识到,它几乎没有权力阻止土地向家庭的大范围非法转移。在安徽省凤阳县,这个以前以乞丐众多而闻名的地方,家庭联产承包责任制显然提高了粮食产量,增加了家庭收入。全国各地的村庄都要求对它们采用同样的制度。为了保持执政的合法性,中国共产党中央委员会不得不给人民他们想要的,并在1983年正式放弃了农业集体化。⁹²随着人民公社的解散,农村合作医疗制度正式结束。卫生中心和公社诊所被关闭,赤脚医生失去了生计。⁹³为了谋生,那些有良好技能的人开设了私人诊所并收取高价。许多人还参与了非法毒品交易。

随着后毛泽东时代医疗保健的权力下放和市场化,抗生素滥用问题日益严重。到20世纪80年代初,私人制药厂商的数量激增,其中许多都是非法的,同时乡村地区也出现了一个新的药品分销网络。例如,在沿海省份福建,1983年至1985年间,使用伪造的批准文件和超过10万箱的假冒商标,建立了58家假药厂。雇佣了700多名小贩,利用诸如削减成本等不道德手段,向29个省市的数千家医院或其他医疗单位推销这些非法产品。假冒药品的总销售额超过3500万元。仅在靖江县的汉口村,就建立了22家假药厂,80%的村民从事假药生产。⁹⁴部分原因是出于便利,部分原因是出于更高的利润,因为许多未经许可的私人生产商

offered cheaper prices than those for the drugs bought through official channels, as well as further incentives, many rural primary caregivers (many of whom were former Barefoot Doctors) began to purchase drugs directly from these private producers. This new network of production, distribution, and marketing 'had troublesome implications,' warned Chen Zhiqian, since they were difficult for the government to monitor, 'posing the danger that quality of the private[ly] produced pharmaceuticals fell significant[ly] below that of those produced by the government'.⁹⁵ Chen was right. Between 1980 and 1987, official drug quality test results had shown that about 22 per cent (the range was 6.7 per cent to 38.5 per cent) of drugs tested failed to meet the quality criteria.⁹⁶ In addition to the huge number of low-quality counterfeits that flooded the market, interviews conducted by researchers from the School of Public Health at Fudan University have suggested that primary health providers in rural China often 'prescribed more drugs than necessary because of marketing activities or patient demand'.⁹⁷ Increased purchasing power and the easier accessibility created by this new network of production, distribution, and marketing also contributed to the prevalence of antibiotic overuse. Until recently, anyone in China could walk into a local pharmacy and buy antibiotics over the counter without a prescription. Yet the government was slow in addressing the problem. It was only in 2012 that the government introduced a regulation to prohibit pharmacies from the unrestricted selling of antibiotics. In the countryside, where demand remains high, the problem has persisted, partly because there are unlimited supplies of relatively cheap, low-quality, or ineffectual counterfeits but equally because in some remote, impoverished regions, villagers continue to have limited or no access to hospital care. Self-medication now relies not on herbal medications, as was earlier the case, but on allopathic interventions such as antibiotics, which may well have greater efficacy but can also lead to long-term health problems on the widest possible scale.

Since adopting the neoliberal market economy, and with a large number of rural villagers migrating into coastal cities, the PRC government began to opt for a market model for financing health services. It moved away from the former model that provided inexpensive and basic health services to a selective primary health model with an emphasis on expertise defined as excellence, with specialized (tertiary) health care and a focus on short-term economic gain. Health care services were reduced or denied following claims that

由于价格比通过官方渠道购买的药品更便宜, 以及进一步的激励措施, 许多农村基层医护人员 (其中许多是以前的赤脚医生) 开始直接从这些私人生产商处购买药品。陈志谦警告说, 这种新的生产、分销和营销网络 “带来了麻烦的影响”, 因为政府很难对其进行监控, “存在私人 [生产的药品质量大大] 低于政府生产的药品质量的危险”。⁹⁵ 陈是对的。在 1980 年至 1987 年期间, 官方药品质量检测结果显示, 约有 22% (范围为 6.7% 至 38.5%) 的受检药品未能达到质量标准。⁹⁶ 除了大量涌入市场的劣质假冒产品外, 复旦大学公共卫生学院的研究人员进行的访谈表明, 中国农村的基层医疗服务提供者经常 “由于营销活动或患者需求而开出超过必要的药品”。⁹⁷ 购买力的提高以及这种新的生产、分销和营销网络带来的更容易的可及性也导致了抗生素的过度使用。直到最近, 中国的任何人都可以在当地药店购买非处方抗生素。然而, 政府在解决这个问题上行动迟缓。直到 2012 年, 政府才出台了一项法规, 禁止药店无限制地销售抗生素。在农村地区, 需求仍然很高, 问题依然存在, 部分原因是相对便宜、低质量或无效的假冒产品供应无限, 但同样也是因为在一些偏远、贫困地区, 村民仍然很少或根本无法获得医院护理。现在的自我用药不再像以前那样依赖草药, 而是依赖西医干预, 如抗生素, 这些抗生素可能具有更大的疗效, 但也可能在最大范围内导致长期健康问题。

自从中国采纳新自由主义市场经济以来, 大量农村村民涌入沿海城市, 中国政府开始选择市场模式来为医疗服务融资。它放弃了以前提供廉价和基本医疗服务的模式, 转而采用选择性的初级医疗模式, 强调被定义为卓越的专业知识, 以及专业的 (三级) 医疗保健, 并注重短期经济效益。在声称以下情况后, 医疗保健服务被减少或拒绝:

they could not be sustained or afforded. Millions were disadvantaged in the process.⁹⁸ The unprecedented scale of urbanization further exacerbated the problem. For those who could neither afford nor had access to hospital care, antibiotics were affordable and readily available. For millions of migrant workers living in squalid conditions, antibiotics have often proved lifesaving, while simultaneously creating the sort of drug resistance that haunts allopathic medicine in the West. At the same time, in those remote rural corners of the PRC where villagers continued to rely on primary health providers for their health care, given the pragmatics of distance as well as cultural reasons of trust, more and more providers left because the system did not incentivize them sufficiently to enable them to continue. As Muchu, the former Barefoot Doctor and currently the village doctor who struggles with how to deliver care and health to her local village, told me (with a big sigh):

Most younger generation village doctors don't want to stay in the village because of the poor subsidy from the state. The amount of money they earn as the village doctor is not enough to take care of their families as well as to pay for their children's education. Anyone with good skills would look for opportunities to be transferred to work in the regional hospital. Even those who did not seek their livelihood elsewhere spent much of their time up in the mountain digging cordyceps sinensis or other precious medicinal herbs to supplement their income. You rarely see them in their villages ... They often complain about the drawbacks of being a village doctor. I think this is a serious issue ... I have been a village doctor for nearly forty years. If I stop being a village doctor, I'd receive no pension. Not even a penny. At one time, the [government] mentioned about introducing an old age insurance system for people like me. But until now, this has not been realized. I am very worried about the future. At the moment, my health is good enough to allow me continuing working as a village doctor ... When I become too old or too sick to continue, I don't have any family to pay for my living expenses. I don't have a pension, what could I live on?⁹⁹

One factor that played, and continues to play, a role in the failure of primary health care systems such as the Maoist Barefoot Doctor program is that they were conceived of initially as aimed at

它们无法维持或负担。在这个过程中，数百万人处于不利地位。⁹⁸前所未有的城市化规模进一步加剧了这个问题。对于那些既负担不起也无法获得医院护理的人来说，抗生素是负担得起且容易获得的。对于数百万生活在肮脏环境中的农民工来说，抗生素常常被证明是救命的，同时也产生了困扰西方对抗疗法的耐药性。与此同时，在中华人民共和国那些偏远的农村角落，村民仍然依赖初级保健提供者来获得医疗保健，考虑到距离的实际情况以及信任的文化原因，越来越多的提供者离开了，因为该系统没有充分激励他们继续下去。正如前赤脚医生、目前正在努力向当地村庄提供护理和健康的乡村医生穆初（Muchu）告诉我（叹了口气）：

大多数年轻一代的乡村医生都不愿意留在村里，因为国家补贴太少。他们作为乡村医生挣的钱不足以照顾家人和支付孩子的教育费用。任何有良好技能的人都会寻找机会调到地区医院工作。即使那些没有在其他地方谋生的人，也花大量时间上山挖掘冬虫夏草或其他珍贵药材来补贴收入。你很少在他们的村子里看到他们……他们经常抱怨当乡村医生的缺点。我认为这是一个严重的问题……我当乡村医生已经快四十年了。如果我不当乡村医生，我就拿不到养老金。一分钱都没有。曾经，[政府]提到要为像我这样的人引入老年保险制度。但直到现在，这还没有实现。我非常担心未来。目前，我的健康状况还允许我继续当乡村医生……当我老了或病得无法继续工作时，我没有任何家人来支付我的生活费用。我没有养老金，我靠什么生活？⁹⁹

在像毛泽东时代的赤脚医生项目这样的初级卫生保健系统失败的原因中，一个发挥作用并且持续发挥作用的因素是，它们最初被认为是针对

the underserved rural population. Yet it was the movement of rural populations into large urban areas, and the increased speed of this movement during this period, that also characterized what is now called the 'Fourth Phase' of public health. Such movements had been inherent to the rise of industrialization, as in Great Britain in the eighteenth century. With the gradual end of colonization and the rise of twentieth-century nationalism, the speed of such movements of populations into urban areas in India, Mexico, and southern Africa, as well as in the PRC, meant that there was a gradual reduction in the actual numbers of underserved people living in rural areas. The Barefoot Doctor scheme was never imagined as functioning in the new megapolis, and the simple expansion of urban health care to include this new population never occurred. The problem of the people's health quickly became one of urban access to health care, where decentralized systems were inappropriate and centralized systems expensive and hence unaffordable for the displaced rural population in the cities.

服务不足的农村人口。然而，农村人口向大型城市地区的流动，以及这一时期这种流动速度的加快，也构成了现在所谓的公共卫生“第四阶段”的特征。正如 18 世纪的英国一样，这种流动是工业化兴起的内在因素。随着殖民化的逐渐结束和 20 世纪民族主义的兴起，印度、墨西哥和南部非洲以及中国农村地区的人口向城市地区流动的速度意味着生活在农村地区的服务不足的人口实际数量逐渐减少。“赤脚医生”计划从未被设想为在新大都市中发挥作用，城市医疗保健的简单扩展以覆盖这一新人口也从未发生。人民健康问题迅速演变为城市获得医疗保健的问题，分散的系统不适用，集中式的系统昂贵，因此城市中流离失所的农村人口负担不起。

China in Global Health: From the ‘Sick Man of Asia’ to the ‘Chinese Approach to Health’

THE SOCIALIST ROAD TO THE PEOPLE’S HEALTH?

In 1948, John Black Grant, an ‘old China hand’ and a self-styled ‘Rockefeller Bolshevik’, writing for the Rockefeller Foundation, argued forcefully that the discipline of medicine must extend ‘beyond clinical diagnosis to social pathology and therapy, thereby making it the task of medicine to assure a mentally and physically healthful community’.¹ For him, the ‘full development and maintenance of mental and physical capacity’ depended ‘not only on adequate medical services but also on minimum standards of nutrition, housing, recreation, education, and social security’. The provision of these ‘necessary non-medical factors’ could not be attained by individual effort but solely ‘by community efforts and government aid’. Grant stressed that the state had a role to create those welfare policies and legislation, which would guarantee that health was a right to be enjoyed by its citizens. Enthused by the post-war ‘revolutionary’ developments of health care in countries he visited, in particular in South Africa, Grant was full of optimism: ‘The pump is primed ... The universal establishment of this pattern of health care as a “social science in the service of society” would usher in a new and momentous era in human welfare.’² Change was going to come, but through which model?

However, one year later, in April 1949, Grant lamented the fact that ‘nowhere up to the present time, has the concept [of extending medical care into health care] been experimented with, much less demonstrated.’ Although in post-war Britain, the National Health Act 1946 had come into effect in July 1948, bringing together a wide range of medical services under one organization and establishing

中国在全球健康领域：从 “亚洲病夫” 到 “中国健康模式”

社会主义通往人民健康的道路？

1948 年，约翰·布莱克·格兰特（John Black Grant）是一位 “中国通”，也是一位自诩的 “洛克菲勒布尔什维克”，他为洛克菲勒基金会撰文，强烈主张医学的范围必须 “超越临床诊断，扩展到社会病理学和治疗，从而使医学的任务是确保一个身心健康的社区”。¹ 他认为，“充分发展和保持身心能力” 不仅取决于充分的医疗服务，还取决于最低标准的营养、住房、娱乐、教育和社会保障。这些 “必要的非医疗因素” 不能通过个人努力来实现，而只能 “通过社区努力和政府援助” 来实现。格兰特强调，国家有责任制定那些福利政策和立法，以保证健康是公民应该享有的权利。格兰特对他在访问过的国家，特别是南非，战后医疗保健的 “革命性” 发展感到兴奋，他充满了乐观：“水泵已经启动 …… 普遍建立这种作为 “服务于社会的社会科学” 的医疗保健模式，将迎来人类福利的新纪元。”² 变革即将到来，但通过哪种模式？

然而，一年后的 1949 年 4 月，格兰特感叹道：“迄今为止，[将医疗保健扩展到健康保健] 的概念从未被试验过，更不用说得证到证实了。” 尽管在战后的英国，1946 年《国民健康法案》已于 1948 年 7 月生效，将范围广泛的医疗服务集中在一个组织下并建立

a comprehensive health service for England and Wales, Grant also noted the continued instability of international health policy. He pondered whether ‘in the evolution of the welfare of communities, are the democratic or the totalitarian countries going to be the first to provide adequate health care?’³ This was the dilemma of the post-war world: divided into ‘totalitarian’ and ‘democratic’ camps, the reality of the Cold War was that it was not clear which was the best way to ensure the improvement of the people’s health, at least as seen from the perspective of internationalist NGOs.

At this precise moment, despite the complication of a schistosomiasis outbreak that affected thousands of Communist soldiers, the Chinese Communist Party’s People’s Liberation Army succeeded in crossing the Yangtze River. After ten years of civil war, peace if not stability had finally been achieved. The newest communist state – the People’s Republic of China (the PRC) – was founded. China was once more united, this time under the leadership of a strong and centralized Communist government. The new Communist authority of China pledged to transform the ‘Sick Man of Asia’ into a prosperous country, a healthy nation that would eventually be free of disease. To reach this nation’s ‘common goal’, it would expand public health and medical work by involving the masses⁴ (as was examined in detail in chapters 1 and 2). It looked as if Grant’s ‘China project’ would finally be realized and the CCP would be the one to turn off the never-ceasing cascade of disease and affliction that Grant and other advocates of social medicine had earlier tried so hard to accomplish but failed.

John Black Grant was born in Ningbo, eastern China, to what he called a ‘sainted’ Canadian Baptist medical missionary, James Skiffington Grant. According to Grant senior, trying to use medicine to save the suffering masses in a country like China was like ‘trying to mop the water off the overflowing sink while the tap is still on’.⁵ Yet despite, or perhaps because of, his father’s warning, J.B. Grant had spent nearly twenty-five years in China trying to turn off that tap by rethinking how medicine should be organized and practised in this vast and impoverished country with multitudes of health problems. Between 1920 and 1921, before returning to China to take up a permanent post at the Peking Union Medical College (PUMC) (the Rockefeller Foundation’s ‘Johns Hopkins for China’), Grant did a master’s degree in public health at the Johns Hopkins’ School of Hygiene and Public Health.⁶ There he met and studied with Sir Arthur Newsholme, a retired medical officer from England who believed that

格兰特在为英格兰和威尔士提供全面的卫生服务时，也注意到国际卫生政策持续的不稳定性。他思考着“在社区福利的演变过程中，民主国家还是极权国家会率先提供充分的医疗保健？”，这就是战后世界的困境：世界分为“极权主义”和“民主”阵营，冷战的现实是，至少从国际非政府组织的视角来看，并不清楚哪种方式最能确保人民健康的改善。

就在此时，尽管数千名共产主义士兵受到血吸虫病爆发的影响，情况复杂，但中国共产党的人民解放军成功渡过长江。经过十年的内战，和平（如果不是稳定的话）终于实现了。最新的共产主义国家——中华人民共和国（the prc）成立了。中国再次统一，这次是在一个强大而集中的共产主义政府的领导下。中国新的共产主义政权承诺将“东亚病夫”转变为一个繁荣的国家，一个最终摆脱疾病的健康国家。为了实现这个国家的“共同目标”，它将通过动员群众来扩大公共卫生和医疗工作⁴（正如在第1章和第2章中详细 examined 的那样）。格兰特的“中国项目”似乎终于要实现了，中国共产党将成为那个能够阻止疾病和痛苦永无止境地蔓延的人，而格兰特和其他社会医学的倡导者早些时候曾努力实现但未能成功。

约翰·布莱克·格兰特出生于中国东部宁波，他的父亲是加拿大浸信会医疗传教士詹姆斯·斯基芬顿·格兰特，他称其为“圣洁的”传教士。根据老格兰特的说法，试图用医学来拯救像中国这样的国家中受苦受难的民众，就像“在水龙头还开着的时候试图擦干溢出的水槽”。⁵然而，尽管如此，或者也许正是因为他父亲的警告，J.B. 格兰特在中国花了近25年的时间，试图通过重新思考如何在这个拥有众多健康问题的庞大而贫困的国家组织和实践医学来关掉那个水龙头。1920年至1921年间，在返回中国在北京协和医学院（pumc）（洛克菲勒基金会的“中国约翰·霍普金斯”）担任永久职位之前，格兰特在约翰·霍普金斯卫生与公共卫生学院获得了公共卫生硕士学位。⁶在那里，他遇到了并师从了来自英国的退休医疗官员亚瑟·纽斯霍姆爵士，他认为

it was primarily poverty that exacerbated the spread and intensity of disease and hence advocated improving health through improvements in living and working conditions as well as better nutrition.⁷ A champion of an organized system of state medicine as an effective tool to prevent and curtail ‘a large mass of illness’, Newsholme would also become an admirer of the new centralized comprehensive health system that combined medicine and public health developed in the Soviet Union under Stalin. In their co-authored book *Red Medicine: Socialized Medicine in the Soviet Union* (1934), Newsholme and John Adams Kingsbury commended the Soviet system as the ‘vast and fascinating experiment of socialized health ... that the rest of the world cannot afford to ignore’.⁸ Kingsbury was the director of the Milbank Foundation, as well as a leader of social work in the United States, who advocated for improving the conditions of the poor.⁹ In their preface to *Red Medicine*, the two authors began with the following passage: ‘When a Russian becomes ill the Government does something about it. In fact, the Government has already done something about it, for Soviet Russia has decided that the health of the individual is the concern of society as a whole. Indeed, the Soviet Union is the one nation in the world which has undertaken to set up and operate a complete organization designed to provide preventive and curative medical care for every man, woman, and child within its borders.’¹⁰

The new USSR, seen through the idealistic model of state engagement promulgated in the West by John Reed immediately after the Bolshevik Revolution, was to be the model for improving people’s health.

After completing his studies at Johns Hopkins, in 1921 J.B. Grant returned to China, seconded by the Rockefeller Foundation to build a brand new, modern public health program.¹¹ In addition to advocating the teaching of preventive medicine as a core of the medical curriculum, he designed the new public health program at the PUMC in such a way ‘that the fundamental techniques in preventative medicine and public health in the community could be practiced and taught’.¹² He also collaborated with the Peking Municipal government to establish the first Public Health Experimental Station in China. A community-based public health experiment such as this was not only new for China; in the early 1920s, such community health undertakings were new and relatively rare throughout the world. Having concluded that ‘the low economic standards with their resultant bad social conditions’, including overcrowding and

主要是贫困加剧了疾病的传播和强度，因此主张通过改善生活和工作条件以及改善营养来改善健康。⁷ 纽斯霍姆提倡建立有组织的国家医疗体系，将其作为预防和减少“大量疾病”的有效工具，他还成为了苏联在斯大林领导下发展起来的结合了医学和公共卫生新型中央集权综合卫生体系的仰慕者。在他与约翰·亚当斯·金斯伯里合著的《红色医学：苏联的社会化医学》（1934年）一书中，纽斯霍姆和约翰·亚当斯·金斯伯里赞扬苏联的制度是“社会化的伟大而引人入胜的实验……世界其他国家不能忽视”。⁸ 金斯伯里是米尔班克基金会的主任，也是美国社会工作的领导者，他主张改善穷人的生活条件。⁹ 在《红色医学》的序言中，两位作者以以下段落开头：“当一个俄罗斯人生病时，政府会采取一些措施。事实上，政府已经采取了一些措施，因为苏俄已经决定，个人的健康是整个社会的事情。事实上，苏联是世界上唯一一个着手建立和运营一个完整组织，旨在为其境内所有男女老少提供预防性和治疗性医疗服务的国家。”¹⁰

通过约翰·里德在布尔什维克革命后立即在西方提出的理想化的国家参与模式来看，新的苏联将成为改善人民健康的典范。

在约翰·霍普金斯大学完成学业后，1921年，J.B. 格兰特回到中国，在洛克菲勒基金会的支持下，建立了一个全新的、现代的公共卫生项目。¹¹ 除了倡导将预防医学的教学作为医学课程的核心外，他还以这样的方式设计了协和医学院的新公共卫生项目，即“预防医学和社区公共卫生的基本技术可以得到实践和教学”。¹² 他还与北京市政府合作，建立了中国第一个公共卫生实验站。这种基于社区的公共卫生实验不仅对中国来说是新的；在1920年代初，这种社区卫生事业在世界范围内也是新的且相对罕见的。他总结说，“低下的经济水平及其导致的恶劣社会条件”，包括过度拥挤和

malnutrition, were 'important factors in the high incidence of [GI TB] and other diseases',¹³ Grant as well as his Chinese collaborators – who included Robert Kho-Seng Lim (more widely known in the West as Bobby Lim), Lim Kho-Seng (an eminent physiologist and the first Chinese professor for the PUMC), and Chen Zhiqian – envisaged that the sole solution to the complex health needs of the Chinese was a system of state-supervised medicine. It would be similar to the Soviet system in that it would '[render] available for every member of the community, irrespective of any necessary relationship to the conditions of individual payment, of all the potentialities of preventative and curative medicine'.¹⁴ Such a system would also encompass the development of a social machinery to ensure standards of living adequate for the maintenance of health.¹⁵ In order for such a system to sustain itself, Grant believed it was also necessary to extensively involve members of local communities. To engage these communities entailed organizing special courses to train health delivery 'auxiliaries', including vaccinators, sanitary inspectors, and other semi-professional groups of health personnel.¹⁶

The American-trained public health expert Selskar Gunn, who had been the vice president of European operations for the Rockefeller Foundation and was an expert in infectious diseases, arrived in China in 1931. Grant and Gunn then worked together to develop a public health experiment in China for the foundation that would form a part of a larger plan of social reconstruction, with an emphasis on raising the economic and social level of the rural masses. They envisaged China becoming 'a vast laboratory in the social science, with implications that would be international in scope'.¹⁷ Their pioneering social experiment in rural China did eventually fail, however. Many later scholars attributed this failure to the Japanese occupation of China after 1937, which had resulted in the termination of all of the social medicine experiments undertaken by the Rockefeller Foundation. However, on a close study of Chinese sources from this period, it could be argued that the failure of these social medicine experiments in China was largely due to Chiang Kai-shek's Nationalist government's increasing orientation toward a model of positive eugenics as the central vehicle for tackling the country's health problems. With the advent of the New Life Movement as of 1934, rather than addressing rural health needs, Chiang's Nationalist government advocated that the Chinese peasantry 'participate' in creating a new rural lifestyle, from eradicating their 'barbaric and

营养不良是「肺结核」和其他疾病高发的重要因素」，¹³ 格兰特和他的中国合作者——包括林可胜（在西方更广为人知的名字是林子豪）、林可胜（一位杰出的生理学家，也是协和医学院的第一位中国教授）和陈志潜——设想解决中国人复杂健康需求的唯一方案是由国家监管的医疗体系。它将类似于苏联的体系，即

「[使] 社区中的每个成员，无论其个人支付条件有任何必要的关系，都能获得预防和治疗医学的所有潜力」。¹⁴ 这样的系统还将包括发展一种社会机制，以确保维持健康的充分生活水平。¹⁵ 为了使这样一个系统能够自我维持，格兰特认为还必须广泛地让当地社区成员参与进来。为了让这些社区参与进来，需要组织专门的课程来培训卫生服务“辅助人员”，包括疫苗接种员、卫生检查员和其他半专业的卫生人员群体。¹⁶

曾在美国受过公共卫生训练的专家塞尔斯卡·冈恩于 1931 年抵达中国，他曾担任洛克菲勒基金会欧洲业务副总裁，并且是传染病专家。随后，格兰特和冈恩共同为该基金会在中国开展了一项公共卫生实验，这项实验将构成一项更大的社会重建计划的一部分，重点是提高农村大众的经济和社会水平。他们设想中国成为“一个广阔的社会科学实验室，具有国际范围的影响”。¹⁷ 然而，他们在华乡村地区的开创性社会实验最终失败了。许多后来的学者将这一失败归因于 1937 年后日本对中国的占领，这导致了洛克菲勒基金会开展的所有社会医学实验的终止。然而，通过对这一时期的中国资料进行深入研究，可以认为，这些社会医学实验在中国的失败，很大程度上是由于蒋介石的国民党政府越来越倾向于将优生学模式作为解决国家健康问题的中心手段。随着 1934 年新生活运动的到来，蒋介石的国民党政府并没有解决农村的健康需求，而是倡导中国农民“参与”创造一种新的农村生活方式，从根除他们的“野蛮和

superstitious' customs to changing their 'unclean' habits and practising 'superior birth' (优生 *yousheng*) or 'race reform'. It was believed that the latter would facilitate those 'retarded' peasants to 'breed out' their 'inferior' and 'unhealthy' genes.¹⁸ Social medicine, as in Germany during National Socialism, gave way to positive and negative eugenic solutions to the people's health. Fascism was not the sole origin of ideas of eugenic improvement, however; in Germany such ideas had already been present in the 1890s, as they had been in Britain, and it became party policy in the late Weimar republic.

With the founding of the People's Republic of China (the PRC), the new Communist authority's commitment to improving the well-being of the common people, as well as its seeming willingness to work with non-communist political groups and individuals in a democratic united front to bring about socialist progress, including public health to China, won many supporters at home and abroad. As Chen Zhiqian, Grant's former Chinese collaborator, recalled: 'I looked forward to contribute to health improvement in the new socialist society.'¹⁹ Among many others, Madame Li Dequan, who became the first minister of health of the new PRC, wholeheartedly supported the new Communist authority's pledge to transform the 'Sick Man of Asia' into a healthy and prosperous socialist country.

The founding of the PRC also came at the intersection of competing global models for the people's health. According to the United Nations Relief and Rehabilitation Administration (UNRRA) in 1948, China presented 'the greatest and most intractable public health problem of any nation in the world'.²⁰ At this exact moment, the UN's newly founded World Health Organization (WHO) rectified and enacted its constitution and gave expression to many of the ideas of social medicine that had been advocated by public health advocates of the left during the interwar period. After World War II, with a mounting problem of worldwide reconstruction and its attendant health crises, a number of these advocates pushed forward their earlier ideas through the new post-war international organizations such as UNRRA and the WHO. Dr Andrija Štampar, the Croatian health reformer and a crusader for agrarian reform who was also an 'old China hand', was elected chairman of the first World Health Assembly held in June–July 1948. Before conflict had broken out in Asia, Štampar had spent several years in China as the League of Nations Health Organization (LNHO)'s representative. While in China, he announced to his mentor Julius Tandler, the Viennese

从改变 '不洁' 的习惯和实行 '优生' (*yousheng*) 或 '种族改良' 等 '迷信' 习俗。人们认为, 后者将有助于那些 '迟钝' 的农民 '消除' 其 '劣等' 和 '不健康' 的基因。¹⁸ 正如国家社会主义时期的德国一样, 社会医学让位于对人民健康的积极和消极的优生解决方案。然而, 法西斯主义并非优生改良思想的唯一来源; 在德国, 早在 1890 年代就已经出现了这种思想, 英国也是如此, 并且在魏玛共和国后期, 它成为了党的政策。

随着中华人民共和国 (prc) 的成立, 新的共产主义政权致力于改善普通民众的福祉, 以及似乎愿意与非共产主义政治团体和个人在民主统一战线中合作, 以实现社会主义进步, 包括中国的公共卫生, 这赢得了国内外许多支持者。正如葛兰特的前中国合作者陈志潜回忆说: '我期待为新社会主义社会的健康改善做出贡献。'¹⁹ 在众多人士中, 李德全夫人成为了新中国的第一任卫生部长, 她全心全意地支持新的共产主义政权将 '东亚病夫' 转变为健康繁荣的社会主义国家的承诺。

中华人民共和国的成立也正值相互竞争的全球人民健康模式的交汇点。根据联合国善后救济总署 (unrra) 1948 年的报告, 中国是 "世界上任何国家中最严重、最棘手的公共卫生问题"。²⁰ 就在此时, 联合国新成立的世界卫生组织 (who) 批准并颁布了其章程, 并表达了两次世界大战期间左翼公共卫生倡导者所倡导的许多社会医学思想。二战后, 随着全球重建问题的日益严重以及随之而来的健康危机, 许多倡导者通过联合国善后救济总署和世界卫生组织等新的战后国际组织推进了他们早期的想法。克罗地亚卫生改革家 Andrija Štampar 博士是一位主张土地改革的斗士, 同时也是一位 "中国通", 他当选为 1948 年 6 月至 7 月举行的第一届世界卫生大会主席。在亚洲冲突爆发之前, Štampar 曾作为国际联盟卫生组织 (lnho) 的代表在中国工作了数年。在中国期间, 他向他的导师维也纳人 Julius Tandler 宣布,

Jewish physician and social reformer who was instrumental in creating a new welfare and health care system for Vienna in the 1920s and 1930s (and who was also simultaneously a eugenicist), that 'my sympathies lie in China, not with the rulers but with the oppressed.'^{*} Štampar was convinced that in order to build a successful health system it was necessary to first remove 'social grievances, such as the sense of exploitation by the landlord'.²¹ After China, Štampar also visited the Soviet Union. The trip was said to have imbued him with enthusiasm for communism. He envisaged that communism would bring about a state health system that would meet the agrarian needs in his native Yugoslavia as well as in China. To his great disappointment, however, the post-war USSR under Stalin had moved further toward a rapid industrialization and urbanization that was accompanied by a more centralized health system. In the meantime, Tito's new communist regime in Yugoslavia, while defying Soviet political hegemony in 1948, adopted a health strategy that was parallel to that of the Soviet Union but which failed to address the radical regional differences and complex agrarian needs of the multi-ethnic state.²² At the same time, as Randall Packard has argued, the WHO's initial efforts to integrate disease control and broad development programs, including the UN's Food and Agricultural Organization's rural welfare program, fell victim to Cold War politics. The model of social medicine integrating rural health with agrarian reform advocated by Štampar and Ludwig Rajchman and their supporters at the 1937 LNHQ's Conference on Rural Hygiene in Bandoeng in Dutch Indonesia (also known as the Bandoeng Conference) was pushed to the margins. Supporters of the vector control method that relied on new bio-medical technologies (for instance, the massive spraying of DDT for malaria control) dominated the field of public health.²³ While these advocates of vector control discounted the social and economic variables that shaped the local epidemiology of diseases such as malaria and schistosomiasis, they trumpeted Western bio-medical expertise and continued to deny local participation.²⁴ In contrast, the new Communist authority of China's agrarian reform policies, such as the Land Reform, the collectivization of agricultural

* After a lecturing trip to China in 1933-34, Tandler then immigrated to China in 1936 because the fascists came into power in Austria. While in China, he was invited by the Soviet government to help reorganize Moscow's sanitary system. He died within a few months of arriving in Moscow.

犹太裔医生和社会改革家，在 20 世纪 20 年代和 30 年代为维也纳创建新的福利和医疗保健体系发挥了重要作用（同时他也是一位优生学家），他表示“我的同情心在中国，不在统治者一方，而在受压迫者一方”。Štampar 坚信，为了建立一个成功的医疗卫生体系，首先必须消除“社会不满，例如被地主剥削的感觉”。²¹ 在中国之后，Štampar 还访问了苏联。据说这次旅行使他对共产主义充满热情。他设想共产主义将带来一个国家医疗卫生体系，该体系能够满足他的祖国南斯拉夫以及中国的农业需求。然而，令他非常失望的是，斯大林领导下的战后苏联进一步走向快速工业化和城市化，同时伴随着更加集中的医疗卫生体系。与此同时，铁托在南斯拉夫的新共产主义政权，虽然在 1948 年反抗了苏联的政治霸权，但采取了一项与苏联平行的卫生战略，但未能解决多民族国家中激烈的区域差异和复杂的农业需求。²² 与此同时，正如 Randall Packard 所认为的那样，世卫组织最初将疾病控制和广泛发展计划（包括联合国粮农组织的农村福利计划）相结合的努力，也成为了冷战政治的牺牲品。Štampar 和 Ludwig Rajchman 及其支持者在 1937 年荷属印度尼西亚万隆举行的国联卫生组织农村卫生会议（也称为万隆会议）上倡导的社会医学模式，即将农村卫生与农业改革相结合，被推到了边缘地位。依赖于新的生物医学技术（例如，大规模喷洒滴滴涕以控制疟疾）的媒介控制方法的支持者主导了公共卫生领域。²³ 虽然这些媒介控制的倡导者低估了影响疟疾和血吸虫病等疾病的当地流行病学的社会和经济变量，但他们大肆宣扬西方生物医学的专业知识，并继续拒绝当地的参与。²⁴ 相比之下，中国农业改革政策的新共产主义权威，例如土地改革、农业集体化

* 在 1933-34 年对中国进行讲学旅行后，坦德勒于 1936 年移民到中国，因为法西斯分子在奥地利掌权。在中国期间，他应苏联政府的邀请，帮助重组莫斯科的卫生系统。抵达莫斯科几个月后，他便去世了。

production, which aimed at the rapid modernization and social transformation of rural China, and the CCP's efforts to engage the masses in its socialist transformative project in which public health undertakings were a core part, stood out as a promising alternative model of a socialist road to public health.* Yet, as we have seen in earlier chapters, many of the CCP's methods to involve the masses were noticeably 'imperialistic' and coercive.

Among the PRC's purported achievements in public health, the nationwide campaign to eradicate schistosomiasis by involving the masses became the most celebrated showcase for the efficacy of the new Chinese approach to the people's health. As Wei Wenbo, the deputy party head in charge of this mass public health campaign proudly announced at the 1959 Nine-Man Subcommittee's Enlarged Meeting for all grassroots managers of the campaign held in anticipation of the tenth anniversary celebration of the founding of the PRC, 'visitors from capitalist Europe, who have nothing good to say about industrial construction in our country, are impressed by our Anti-Schistosomiasis Campaign.'²⁵ Such undertakings came to mark the unique successes claimed by the young state as well by as global public health authorities.

'POLITICAL COMMITMENT' AND 'COMMUNITY PARTICIPATION'

As of the second half of the 1950s, in many of the newly decolonized countries in Latin America and Africa and in countries of the industrialized West, a growing number of advocates of social medicine in the field of public health and medicine – most of them anti-capitalist and anti-imperialist – became attracted by and willing to accept the PRC's claim to have transformed the 'Sick Man of Asia' into a healthy nation without any dependence on Western aid. The PRC was seen to have accomplished this through the political commitment of a strong and centralized socialist government and with the enthusiastic mass participation of local communities.

Among the first Western medical visitors to the PRC was a group of British physicians. In August 1957, a couple of months before Mao

* A growing number of studies in recent years have shown that both programs undertaken in the PRC under Mao were disastrous in terms of the loss of individual life and a true failure of agricultural expansion.

生产, 旨在快速实现中国农村的现代化和社会转型, 以及中国共产党动员群众参与其社会主义改造项目的努力 (其中公共卫生事业是核心组成部分), 成为了通往公共卫生的社会主义道路上一个充满希望的替代模式。^{*} 然而, 正如我们在前面的章节中看到的, 中国共产党动员群众的许多方法都明显带有“帝国主义”和强制的色彩。

在中华人民共和国宣称的公共卫生成就中, 通过动员群众在全国范围内开展的血吸虫病防治运动, 成为展示中国人民健康新方法有效性的最受赞誉的范例。正如负责这项群众性公共卫生运动的党的副组长魏文伯在 1959 年血吸虫病防治九人小组扩大会议上自豪地宣布的那样, 这次会议是为了迎接中华人民共和国成立十周年而为所有基层管理者举办的, ‘来自资本主义欧洲的参观者, 他们对我们国家的工业建设没有什么好话可说, 但对我们的血吸虫病防治运动印象深刻。’²⁵ 这些事业逐渐成为这个年轻国家以及全球公共卫生当局所宣称的独特成功标志。

“政治承诺”和“社区参与”

从 20 世纪 50 年代下半叶开始, 在拉丁美洲和非洲许多新近非殖民化的国家以及工业化的西方国家, 越来越多的公共卫生和医学领域的社会医学倡导者——他们中的大多数是反资本主义和反帝国主义者——被中华人民共和国声称已将“亚洲病夫”转变为一个健康的国家, 而无需依赖西方援助所吸引, 并愿意接受这一说法。人们认为, 中华人民共和国通过强大而集中的社会主义政府的政治承诺以及当地社区的热情参与实现了这一目标。

最早访问中华人民共和国的西方医学人士中, 有一群英国医生。1957 年 8 月, 在毛

* 近年来, 越来越多的研究表明, 毛泽东领导下的中国所实施的各项计划, 在个人生命损失方面是灾难性的, 并且在农业扩张方面是真正的失败。

marched China into the disastrous Great Leap Forward and at the invitation of the Chinese Medical Association (CMA), nine prominent figures in the field of public health and medicine from Britain visited the PRC on a 'medical tour'. The delegation, led by Brian Gilmore Maegraith – the dean of the Liverpool School of Tropical Medicine (LSTM) and one of the most outspoken critics of the imperial model of tropical medicine in the post-war West – included Sir Theodore Fox (more widely known as T.F. Fox), the editor of *The Lancet* and a Quaker, as well as Sir Francis Avery Jones, the renowned gastroenterologist who was also an ardent supporter of the new National Health Service (NHS) in Britain. During their three-and-a-half weeks of carefully choreographed stops in the PRC, restricted to larger cities and the more developed eastern rural regions, the British visitors conceded the innate superiority of the PRC's health system under the CCP's leadership: 'The standard of hygiene reached already is ... most impressive ... the successful control of flies, the litterless streets, and fanatic household cleanliness are having profound effect on the spread of gastro-intestinal infections, especially in children, in whom, we are told, bacillary dysentery is much less common than it was,' marvelled Maegraith and his fellow British travellers.²⁶ For them, it augured well for this strong, centralized government, as T.F. Fox put it explicitly in his lengthy report of their visit: 'This defeat of the flies, which astonishes every traveller in China, illustrates what can be done by a regime which can so easily enlist man, woman, and child against a common enemy.'²⁷

Before World War II had broken out in Europe, T.F. Fox had visited the Soviet Union. He had been deeply impressed by how the Soviet experiment had reallocated medical services 'to the better advantage of the community as a whole' by 'abolishing the common idea that medical care was a commodity on sale for a fee'.²⁸ In post-war Britain, in his capacity as editor of *The Lancet*, T.F. Fox advocated for an organized state medical service for Britain that would free medicine from the marketplace. He was thus actively involved in the discussion about setting up the NHS. He was also an advocate of birth control and primary health care as well as its equitable delivery.* In October 1965, during the Harveian oration delivered before

* Theodore Fox's father was a Quaker doctor who served a rural community in the Scottish Highlands at the turn of the nineteenth and twentieth century. It was a regional community health innovation to serve the poor communities in the rural Scottish Highlands in the early twentieth century (it became known as the Highlands and Islands

在中国医学会 (cma) 的邀请下, 中国迈入了灾难性的大跃进时期, 来自英国公共卫生和医学领域的九位著名人物对中国进行了“医疗之旅”。该代表团由利物浦热带医学院 (lstm) 院长布莱恩·吉尔摩·马格雷思 (Brian Gilmore Maegraith) 率领, 他是战后西方热带医学帝国模式最直言不讳的批评者之一, 成员包括《柳叶刀》的编辑兼贵格会教徒西奥多·福克斯爵士 (更为人所知的名字是 T.F. Fox), 以及著名的肠胃病学家弗朗西斯·艾弗里·琼斯爵士, 他也是英国新的国家医疗服务体系 (nhs) 的坚定支持者。在中国经过精心安排的三周半的访问中, 他们只能在较大的城市和较发达的东部农村地区停留, 英国访客承认了中国共产党领导下中国卫生系统的内在优越性: “已经达到的卫生标准……令人印象深刻……对苍蝇的成功控制、没有垃圾的街道和狂热的家庭清洁对胃肠道感染的传播产生了深远的影响, 尤其是在儿童中, 我们得知, 细菌性痢疾的发生率远低于过去,” 马格雷思和他的英国旅行伙伴惊叹道。²⁶ 对他们来说, 这对这个强大的中央集权政府来说是个好兆头, 正如 T.F. Fox 在他关于访问的长篇报告中明确指出的那样: “这种击败苍蝇的做法让每一位来中国的旅行者都感到惊讶, 它说明了一个政权能够如此轻易地动员男人、女人和儿童来对抗共同的敌人。”²⁷

在第二次世界大战在欧洲爆发之前, T.F. 福克斯曾访问过苏联。苏联的实验给他留下了深刻的印象, 他们通过“废除医疗保健是按收费出售的商品的普遍观念”, 将医疗服务“更好地分配给整个社区”。²⁸ 在战后的英国, T.F. 福克斯以柳叶刀编辑的身份, 倡导为英国建立一个有组织、有组织的国家医疗服务体系, 使医疗摆脱市场。因此, 他积极参与了关于建立英国国民医疗服务体系的讨论。他还提倡节育和初级卫生保健, 以及公平地提供这些服务。*

1965 年 10 月, 在哈维演讲中

* 西奥多·福克斯的父亲是一位贵格会医生, 在十九世纪末和二十世纪初在苏格兰高地一个农村社区服务。这是一个区域性的社区卫生创新, 旨在为二十世纪初苏格兰高地农村地区的贫困社区提供服务 (它被称为高地和岛屿)

the Royal College of Physicians, Fox famously proclaimed that medicine must serve a moral purpose. The primary role of doctors was to help people rather than to advance science, he proclaimed.²⁹ Three years prior to his China visit, Fox had revisited the USSR. This time he had found the more advanced Soviet health service less exciting, partly because his post-war Britain had achieved the provision of a free medical service to the entire British population. But like an increasing number of contemporary Western observers, he found the Soviet authoritarian system morally objectionable. Despite the fact that ‘it is capable of promoting rapid progress up to a certain point’, he asked ‘whether medical practice and medical science can reach their full stature under an authoritarian regime’. Furthermore, Fox was disappointed to see that Western materialism had become ‘part of a new gospel’ in Stalin’s USSR.³⁰ In contrast, Fox was enthused by the visit to the less developed and newly founded Communist China. This, according to him, was partly because he found the enthusiasm of his Chinese hosts for their new country ‘infectious’ and ‘persuasive’, but he was equally enthused by the Communist authority’s ability to manage this vast nation and to ‘motivate’ the 600 million Chinese.³¹ Contrary to the Victorian view of a diseased, filthy, poverty-stricken, and retrograde China, Fox’s impression of the new China, which was echoed by a growing number of China observers in the post-war West, was noticeably more positive. While China was enchanting and timeless, as imagined by Westerners from Marco Polo to Fox’s Quaker missionary father, it was substantially more than this. The new China was, according to Fox, a progressive and independent modern nation, which was undergoing ‘rapid’ and

Medical Services after 1913) that provided the basic structure of the NHS as adopted in 1948. At the time, Fox was known for his support of the NHS. He was also known for his part in the publication of the highly critical Collings Report (1950) of general practitioner (GP) practices in post-war Britain and the subsequent creation of the College of General Practitioners that aimed at improving the standard of general practice as well as generating a growing awareness of the importance of primary care. For a history of the Highlands and Islands Medical Services, see M. McCrae, *The National Health Service in Scotland: Origins and Ideals, 1900–1950* (Edinburgh: Tuckwell Press 2003); D. Hamilton, ‘Highlands and Islands Medical Services’, in Gordon McLachlan, ed., *Improving the Common Wealth, Aspects of Scottish Health Services 1900–1984* (Edinburgh: Edinburgh University Press for Nuffield Trust 1987): 481–90. For an evaluation of the Collings Report, see Roland Petchey, ‘Collings Report on General Practice in England in 1950: Unrecognised, Pioneering Piece of British Social Research?’ *British Medical Journal*, 311 6996, 1 July 1995): 40–2.

福克斯在皇家内科医学院发表著名演讲，宣称医学必须服务于道德目的。他宣称，医生的首要任务是帮助人们，而不是推进科学。²⁹ 在访问中国的三年前，福克斯曾重访苏联。这一次，他发现更先进的苏联医疗服务没有那么令人兴奋，部分原因是他的战后英国已经实现了向全体英国人民提供免费医疗服务。但与越来越多的当代西方观察家一样，他发现苏联的威权体制在道德上令人反感。尽管“它有能力在一定程度上促进快速进步”，但他质疑“在威权政权下，医疗实践和医学科学是否能达到充分的高度”。此外，福克斯失望地看到，西方唯物主义已经成为斯大林苏联“新福音”的一部分。³⁰ 相比之下，福克斯对访问欠发达和新成立的共产主义中国感到兴奋。他认为，这部分是因为他发现他的中国主人对他们的新国家的热情具有“感染力”和“说服力”，但他也同样对共产党当局管理这个庞大国家和“激励”6亿中国人的能力感到兴奋。³¹ 与维多利亚时代对一个疾病缠身、肮脏、贫困和倒退的中国看法相反，福克斯对新中国的印象——战后西方越来越多的中国观察家也对此表示赞同——明显更加积极。虽然中国像马可·波罗到福克斯的贵格会传教士父亲等西方人想象的那样，既迷人又永恒，但它远不止于此。根据福克斯的说法，新中国是一个进步和独立的现代国家，正在经历“快速”的

Medical Services after 1913) that provided the basic structure of the NHS as adopted in 1948. At the time, Fox was known for his support of the NHS. He was also known for his part in the publication of the highly critical Collings Report (1950) of general practitioner (GP) practices in post-war Britain and the subsequent creation of the College of General Practitioners that aimed at improving the standard of general practice as well as generating a growing awareness of the importance of primary care. For a history of the Highlands and Islands Medical Services, see M. McCrae, *The National Health Service in Scotland: Origins and Ideals, 1900–1950* (Edinburgh: Tuckwell Press 2003); D. Hamilton, ‘Highlands and Islands Medical Services’, in Gordon McLachlan, ed., *Improving the Common Wealth, Aspects of Scottish Health Services 1900–1984* (Edinburgh: Edinburgh University Press for Nuffield Trust 1987): 481–90. For an evaluation of the Collings Report, see Roland Petchey, ‘Collings Report on General Practice in England in 1950: Unrecognised, Pioneering Piece of British Social Research?’ *British Medical Journal*, 311 6996, 1 July 1995): 40–2.

'astonishing' transformation by following the socialist path. While he was pleased to note that Western medical missionaries' efforts to 'save' China, as well as the Rockefeller Foundation's earlier initiative to modernize and improve medical education in the country, seemed to have had some lasting impact, in the post-war world such endeavours came increasingly under criticism as the 'white man's burden' in public health. In the search for a new and positive model to improve people's health, he was enthused by the Chinese Communist state's 'great effort to prevent diseases and improve medical services'. Equally, he was excited that the PRC had supposedly achieved this without the help of the West: 'the feet of these 600 million people have been set on the concrete path of hygiene and mechanization that is to give us all long and happy lives in a utopia of nuclear physics.'³² Although he cautioned that the new Communist China was an authoritarian state like the Soviet Union and Nazi Germany, and indeed that in the latter medicine had been a tool in the genocide of the Jews, he was willing to accept the claim that the CCP was exercising its power for the good of the people: 'This power, which can be used for good as well as for ill, is the chief reason for its remarkable achievements, in medicine as in so much else.'³³ Sharing Fox's view, Avery Jones, too, was impressed by how the authoritarian system of the new China had enabled the Communist authority 'to bring its policies most effectively to the attention of each individual and enabled the anti-fly campaign, for example, to be so effective'.³⁴

If both Avery Jones and T.F. Fox had some reservations about the PRC's authoritarian system, Brian Gilmore Maegraith applauded it. As one of the most prominent figures in the field of tropical medicine in the post-war West, Maegraith was a fervent opponent of the prevailing view that diseases can be eradicated by simply eliminating the parasites. During the war, Maegraith spent several years in Freetown, Sierra Leone, heading the Royal Army Medical Corps' mobile pathology laboratory.³⁵ This 'real experience' in the tropics, according to him, broadened his views of tropical medicine. It made him increasingly critical of the imperialistic approach that viewed the local population as inherently inferior (such views were held, for example, by many in the LSTM's rival institute, the London School of Hygiene and Tropical Medicine [LSHTM], such as Patrick Manson (whom we encountered in chapter 1), and therefore denied the locals any participation in the planning and implementation of medical aid. Calling the 'official attitude' in Britain that defined tropical medicine merely an 'ordinary

通过遵循社会主义道路实现的“惊人”转型。虽然他很高兴地注意到西方医疗传教士“拯救”中国的努力，以及洛克菲勒基金会早期为实现该国医疗教育现代化和改进所做的努力，似乎产生了一些持久的影响，但在战后世界，这类努力越来越受到批评，被认为是公共卫生领域中的“白人的负担”。在寻找一种新的、积极的模式来改善人民健康的过程中，他对中国共产党政权为“预防疾病和改善医疗服务”所做的“巨大努力”感到兴奋。同样，令他兴奋的是，中华人民共和国据说在没有西方帮助的情况下实现了这一目标：“这6亿人民已经踏上了卫生和机械化的具体道路，这将使我们所有人都能在核物理学的乌托邦中获得长寿和幸福的生活。”³² 尽管他警告说，新的共产主义中国像苏联和纳粹德国一样是一个专制国家，而且在后者，医学一直是犹太人种族灭绝的工具，但他愿意接受中共为了人民的利益而行使其权力的说法：“这种权力，既可以用于善，也可以用于恶，这是它在医学以及其他许多方面取得显著成就的主要原因。”³³ 艾弗里·琼斯也和福克斯一样，对新中国的专制制度如何使共产主义当局“最有效地将政策传达给每个人，并使灭蝇运动等活动如此有效”印象深刻。³⁴

如果说艾弗里·琼斯（Avery Jones）和 T.F. 福克斯（T.F. Fox）对中国专制的政治体制持保留态度，那么布赖恩·吉尔摩·梅格雷斯（Brian Gilmore Maegraith）则对此表示赞赏。作为战后西方热带医学领域最杰出的人物之一，梅格雷斯强烈反对当时流行的“仅通过消除寄生虫就能根除疾病”的观点。战争期间，梅格雷斯在塞拉利昂的弗里敦待了几年，领导皇家陆军医疗队的流动病理实验室。³⁵ 他认为，在热带地区的这种“真实经验”拓宽了他对热带医学的看法。这使他越来越批判那种将当地居民视为天生低等的帝国主义方式（例如，伦敦卫生与热带医学院 [LSHTM]，的竞争对手机构 LSTM 中的许多人，如我们在第一章中遇到的帕特里克·曼森，就持有这种观点），因此拒绝当地人参与医疗援助的规划和实施。他称英国定义热带医学的“官方态度”仅仅是一种“普通的”

medicine' as transplanted to the tropics a fallacy, which at its worst was truly 'dangerous', Maegraith argued that tropical diseases such as malaria and schistosomiasis were not merely caused by the specific parasites but were also the result of social and economic conditions in the tropics, in particular communal poverty, low standards of living, and a lack of health education. He also opposed the view 'cherished' by many economists in the post-war West, particularly in the United States, that socio-economic 'progress' would inevitably lead to better health. Maegraith warned that 'unless the health aspects have been included from the beginning, the socio-economic development may well become a menace to health not only in the building areas but in the contiguous population.' He argued that 'a healthy economy demands a healthy community' and that 'health aspects must be included at all stages of major socioeconomic developments as an integral part of the complex organization of the planning.'³⁶ 'Modern tropical medicine', as opposed to the established imperial tropical medicine in the United Kingdom, was for him 'above all, a multidisciplinary subject, taking into account social, economic, and environmental factors which influence the whole pattern of disease in the community, as well as in the individual'. Maegraith advocated for local involvement, from local governments and the impacted communities, in the planning and delivery of health care. Importantly, the latter included training medical auxiliaries.³⁷ According to him, local people, with their experience and knowledge, were the 'essential building blocks' in designing a sustainable and efficient health care system, and they could 'take the available medical care to the periphery where professionals [could] seldom operate'.³⁸ Driven by his motto, 'Our impact on the tropics must be in the tropics', Maegraith worked closely with his Thai colleague Chamlong Harinasuta to build the Faculty of Tropical Medicine at Mahidol University in Bangkok, a research and teaching institute to provide 'ordinary medical undergraduates training towards rural and urban community health and to provide trained scientists to work in the field problems'.³⁹ He had hoped that 'one day the Bangkok project would extend beyond the national boundaries into the [Southeast Asian] region' to 'countries bordering on the South China Sea that might be interested in collaborating in graduate medical training programs and in research into the local endemic diseases which were depressing living standards in the region'.⁴⁰ The seeds for local knowledge, if not local control, in providing care for the people's health were already sown.

Maegraith 认为, 将 “现代医学” 移植到热带地区是一种谬论, 最糟糕的情况下是真正 “危险的”。他认为, 疟疾和血吸虫病等热带疾病不仅仅是由特定的寄生虫引起的, 也是热带地区社会和经济条件的产物, 特别是普遍贫困、低生活水平和缺乏健康教育。他还反对战后西方, 特别是美国, 许多经济学家 “珍视” 的观点, 即社会经济 “进步” 必然会带来更好的健康。Maegraith 警告说, “除非从一开始就考虑到健康方面, 否则社会经济发展很可能对健康构成威胁, 不仅在建筑区域, 而且在毗邻的人群中。” 他认为, “健康的经济需要健康的社区”, 并且 “健康方面必须作为规划复杂组织的一个组成部分, 纳入重大社会经济发展的各个阶段”。³⁶ 对于他来说, “现代热带医学” 与英国既定的帝国热带医学不同, “首先是一门多学科的学科, 它考虑到影响社区乃至个人疾病整体模式的社会、经济和环境因素”。Maegraith 主张地方政府和受影响社区参与医疗保健的规划和提供。重要的是, 后者包括培训医疗辅助人员。³⁷ 他认为, 当地人民凭借他们的经验和知识, 是设计可持续和高效医疗保健系统的 “基本组成部分”, 他们可以将现有的医疗服务带到专业人员 [很少] 能够运作的外围地区”。³⁸ 在 “我们对热带地区的影响必须在热带地区” 这一座右铭的驱动下, Maegraith 与他的泰国同事 Chamlong Harinasuta 密切合作, 在曼谷玛希隆大学建立了热带医学学院, 这是一个研究和教学机构, 旨在为 “普通医学本科生提供农村和城市社区卫生培训, 并为训练有素的科学家提供在实地问题中工作”。³⁹ 他曾希望 “有一天, 曼谷项目将超越国界, 扩展到 [东南亚] 地区”, 扩展到 “可能对合作研究生医学培训项目和研究当地流行病感兴趣的南海周边国家, 这些流行病正在降低该地区的生活水平”。⁴⁰ 在为人民健康提供护理方面, 如果不是地方控制, 那么地方知识的种子已经播下。

Maegraith visited the PRC while he was working on this joint venture of 'new' medical education. On this visit, the British were shown 'the nation's all-out totalitarian attempt to "liquidate" the threat of schistosomiasis to the future of the new China'. They were also invited to join 'the hordes of village children picking up roasted snails'.⁴¹ Persuaded by the official rhetoric, Maegraith felt the urge to broadcast revolutionary China's national Anti-Schistosomiasis Campaign to the Western medical and public health world: 'This monumental modern struggle against an ancient disease will interest and excite you as much as it did me.'⁴² For him, the Anti-Schistosomiasis Campaign as well as the PRC government's public health efforts were perfect cases of how keeping the farming communities healthy would, in turn, improve agricultural productivity and hence people's well-being – at least in the rural areas.⁴³ According to Maegraith, not only had China achieved 'nutritional independence' in producing sufficient food to feed an 'astonishing number of 800 million souls' without any Western aid, by controlling diseases and keeping its population healthy, 'this happy state of affair is likely to continue for some time'.⁴⁴ Equally, for Maegraith, the PRC's Anti-Schistosomiasis Campaign was a perfect example of how 'the local community is acting for itself in its own interests and cooperating at the same time in the wider national scheme.'⁴⁵ Yet, as my earlier chapters have shown, while the PRC's Anti-Schistosomiasis Campaign did involve the community, it was a centrally planned and expert-led campaign. It was also hastily implemented, often encountering local resistance. Far from a case of local communities acting in their own interests, to a great extent many of the methods used to involve the masses were noticeably 'imperialistic'. Furthermore, Maegraith also lauded the PRC government's intensive public health propaganda campaign as 'essential' to ensuring the cooperation of the communities: 'The Chinese Government quite rightly regards propaganda as an essential technique in the antischistosomal scheme and has organized public information services in the endemic areas on an impressive scale, with the twofold object of achieving control of the disease and of obtaining a more general improvement in social health and cleanliness, and, even more important, a genuine desire for an improving standard of living.'⁴⁶ It is ironic that Maegraith, the expert voice known for advocating that the locals were the 'essential building blocks' in designing a sustainable and efficient health care system, also endorsed the view, held by Chinese health officials and

梅格雷思在从事“新”医学教育的合资企业时访问了中国。在这次访问中,英国人看到了“国家全力以赴的极权主义,试图‘消灭’血吸虫病对新中国未来的威胁”。他们还应邀参加了“成群结队的乡村儿童捡拾烤蜗牛”的活动。⁴¹在官方言论的劝说下,梅格雷思感到有必要向西方医学和公共卫生界宣传革命中国的全国血吸虫病防治运动:“这场与古老疾病作斗争的伟大现代斗争将像我一样引起你们的兴趣和兴奋。”⁴²对他来说,血吸虫病防治运动以及中国政府的公共卫生工作是完美的案例,说明保持农业社区的健康反过来将提高农业生产力,从而改善人们的福祉——至少在农村地区是这样。⁴³据梅格雷思称,中国不仅在生产足够的食物以养活“惊人的8亿人口”而没有西方援助方面实现了“营养独立”,而且通过控制疾病并保持人口健康,“这种幸福的状态可能会持续一段时间”。⁴⁴同样,对梅格雷思来说,中国的血吸虫病防治运动是“地方社区如何为自身利益采取行动,同时在更广泛的国家计划中进行合作”的完美典范。⁴⁵然而,正如我之前的章节所表明的那样,虽然中国的血吸虫病防治运动确实涉及社区,但它是一项集中计划和专家主导的运动。它也是仓促实施的,经常遇到当地的抵制。许多用于动员群众的方法在很大程度上明显带有“帝国主义”色彩,远非地方社区为自身利益采取行动。此外,梅格雷思还称赞中国政府的密集公共卫生宣传运动对于确保社区的合作“至关重要”:“中国政府非常正确地将宣传视为血吸虫病防治计划中的一项基本技术,并在地方病地区组织了大规模的公共信息服务,其双重目标是控制疾病,更普遍地改善社会健康和清洁,更重要的是,真正渴望提高生活水平。”⁴⁶具有讽刺意味的是,梅格雷思这位以倡导当地人是设计可持续和高效医疗保健系统的“基本组成部分”而闻名的专家,也认可了中国卫生官员的观点,并且

earlier medical missionaries such as Barlow, that local communities were ‘ignorant’ or ‘under-educated’ and that they needed to be ‘enlightened’ about important health concerns such as hygiene and disease control based on a Western bio-medical model. So, what was the local health experience and knowledge worth if he considered the locals inherently ‘unenlightened’?

Maegraith was in favour of the equally, if not greater, top-down authoritarian model in the PRC, seeing it as providing an advantage in coordinating the public health authorities and other allied services, from agriculture to forestry and water conservancy as well as communities, in a what he called a ‘cooperative effort’ to control schistosomiasis: ‘The remarkable degree of integrated cooperation already achieved, perhaps more easily in China than in nontotalitarian countries, augurs well for the future success of the whole scheme. The Chinese have set as their target the practical control of schistosomiasis within twelve years from now. Their cooperative enthusiasm, skill, and dedication to the task should enable them to achieve this objective.’⁴⁷ Yet, as we have seen throughout this book, this highly centralized top-down campaign was very far from a ‘cooperative effort’. The local responses to it had proved fragmented and often contradictory; there was also a fair amount of resistance at different levels, from various state ministries to local authorities and in the impacted communities. Equally troubling was that Maegraith and his fellow travellers cheered as they watched local children take part in snail control activities without protective measures. As an expert on tropical medicine, he might have been expected to know that such activities carried a potential health risk because it would expose these children to the disease.

In the meantime, the British doctors’ positive ‘medical’ impressions of the PRC were propagated to a much wider audience by pro-China journalists in the West, notably Felix Greene. With the exception of Edgar Snow, widely known as the main advocate for the CCP in the West, Greene was the most read journalist and broadcaster reporting on Mao’s China during the Cold War. His book, *What’s Really Happening in China?* published in 1959 by City Lights Books (the San Francisco publishers known for advocating radical left-wing politics and social justice), was one of the first volumes of eyewitness reports on the PRC published in the West. The book portrayed the CCP and its new China in a highly favourable light. With growing criticism of the USSR in the West following the gradual revelation of

早期的医学传教士，如巴洛，认为当地社区是“无知的”或“未受过教育的”，他们需要根据西方的生物医学模式“启迪”关于卫生和疾病控制等重要的健康问题。那么，如果他认为当地人天生就是“不开化的”，那么当地的健康经验和知识又有什么价值呢？

梅格雷思赞成在中国采取同样甚至更加自上而下的威权模式，认为这在协调公共卫生部门和其他相关服务部门（从农业到林业和水利以及社区）方面具有优势，他称之为控制血吸虫病的“合作努力”：“已经实现的卓越的综合合作程度，在中国可能比在非极权主义国家更容易实现，这预示着整个计划未来会取得成功。中国人已经设定了目标，即在未来12年内实际控制血吸虫病。他们对这项任务的合作热情、技能和奉献精神应该能够使他们实现这一目标。”⁴⁷然而，正如我们在本书中所看到的，这种高度集中的自上而下的运动远非“合作努力”。当地对此的反应被证明是分散的，而且经常是矛盾的；从各个州政府部门到地方当局以及受影响的社区，在不同的层面都存在相当大的阻力。同样令人不安的是，梅格雷思和他的同伴们欢呼雀跃地看着当地儿童在没有保护措施的情况下参加灭螺活动。作为一名热带医学专家，人们可能期望他知道这些活动会带来潜在的健康风险，因为它会将这些儿童暴露于这种疾病。

与此同时，英国医生对中国的高度“医学”印象通过西方亲华记者传播给更广泛的受众，尤其是费利克斯·格林。除了广为人知的西方主要中共倡导者埃德加·斯诺之外，格林是冷战期间阅读量最高、报道毛泽东中国的记者和广播员。他的书《中国到底发生了什么？》于1959年由城市之光出版社出版（这家旧金山出版社以倡导激进左翼政治和社会正义而闻名），是西方出版的关于中国的第一批目击报告之一。这本书以非常有利的方式描绘了中国共产党及其新中国。随着苏联逐渐暴露，西方对苏联的批评日益增多

the horrors under Stalin's rule and the suppression of the Hungarian uprising by Soviet armies after Stalin's death, Greene was eager to assure his Western readers that Mao's new China was different from other totalitarian states, which for him included Hitler's Germany as well as the USSR. The new PRC, according to Greene, 'is being led by a group of historically conscious, strong, and enormously competent men who identified themselves with the people, who knew the changes the people wanted and sponsored those changes. They won their revolution because they voiced the demands of a people driven to extremities of suffering, who were determined to end the disease, hunger and corruption which had held them in subjection for centuries. If ever in history there was a people's revolution – this was it.' For Greene, the PRC's achievement in public health was a fruit of this 'people's revolution': 'the [Communist government of China] has succeeded, despite the violence of the civil war, in bringing unity, order, honesty, food, education, health, and above all hope, to the Chinese masses ... No one visiting China today fails to be impressed by the degree of cleanness and order which is found now throughout China.' To prove his claim, he turned to the accounts of Maegraith, Fox, and Avery Jones: 'top-flight British doctors who visited China in 1957 to see what had happened to medicine in China since the revolution'.⁴⁸

Remember that Maegraith had argued that the 'disease situation in the community is further influenced by ... the background of malnutrition and overall shortage of protein, the agricultural and veterinary products available.'⁴⁹ In the four years following the British visit, the PRC's all-out totalitarian attempt to transform the countryside into a 'communist paradise' turned it, rather, into a living hell. Existing rural communities were destroyed; millions died of starvation in the worst famine in history that lasted four years between 1958 and 1962.⁵⁰ In the meantime, diseases such as schistosomiasis were to remain an intrinsic part of the daily experiences of millions of villagers living in the endemic regions.⁵¹

'THE SICK MAN OF ASIA HAS BECOME THE HEALTHIEST MAN IN THE WORLD'

In the early 1960s, while a number of editorials in the Western press, as well as documents from CIA files, suggest that the West was aware of the widespread outbreak of diseases associated with malnutrition (particularly oedema and hepatitis) and the resulting sharp increase

在斯大林统治下的恐怖以及斯大林去世后苏联军队对匈牙利起义的镇压之后, 格林渴望向西方读者保证, 毛泽东的新中国与其他极权主义国家不同, 在他看来, 这些国家包括希特勒的德国以及苏联。格林认为, 新的中华人民共和国 “正在由一群具有历史意识、坚强且极有能力的领导人领导, 他们认同人民, 了解人民想要的变化并支持这些变化。他们之所以赢得革命, 是因为他们表达了一个遭受极端痛苦、决心结束几个世纪以来一直奴役他们的疾病、饥饿和腐败的人民的的要求。如果历史上曾经有过人民革命, 那就是这次革命。” 对于格林来说, 中华人民共和国在公共卫生方面的成就是这场 “人民革命” 的成果: “[中国共产党政府] 成功地, 尽管内战非常暴力, 但它给中国人民带来了团结、秩序、诚实、食物、教育、健康, 以及最重要的希望……今天访问中国的任何人都会对中国各地现在的清洁和秩序程度印象深刻。” 为了证明他的说法, 他求助于梅格雷思、福克斯和艾弗里·琼斯的说法: ‘1957 年访问中国以了解自革命以来中国医学发生了什么变化的顶尖英国医生’。

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请记住, 梅格雷思曾争辩说, “社区的疾病状况进一步受到……营养不良和蛋白质总体短缺, 以及可获得的农业和兽医产品等背景因素的影响。”⁴⁹ 在英国访问后的四年里, 中华人民共和国试图将农村转变为 “共产主义天堂” 的全面极权主义尝试, 反而将其变成了人间地狱。现有的农村社区被摧毁; 在 1958 年至 1962 年期间持续四年的历史上最严重的饥荒中, 数百万人死于饥饿。⁵⁰ 与此同时, 血吸虫病等疾病仍然是生活在地方病流行地区的数百万村民日常经历中不可分割的一部分。⁵¹

“亚洲病夫” 已经成为 “世界上最健康的人”

在 20 世纪 60 年代初期, 西方媒体的一些社论以及中央情报局的文件表明, 西方已经意识到与营养不良相关的疾病 (特别是水肿和肝炎) 的广泛爆发以及由此导致的急剧增加

in the mortality rate in China in 1960 and 1961, news of the Chinese famine and the high mortality rate was dismissed or ignored by many China sympathizers in the West as anti-communist propaganda.⁵² Instead, they turned to Edgar Snow, who visited China at the height of the famine in 1960, and then to Dr Joshua Horn, an English surgeon who was living in China during that time, to argue in favour of the PRC's superior primary health care system under Maoist communism.*

Edgar Snow, an 'anti-colonial, anti-imperialist, pro-independence' radical leftist from Missouri and the author of the earlier global bestseller *Red Star over China* (1937), was one of the most widely read propagandists for the CCP in the West, as well as, according to John S. Service, the 'chief contact' between the United States and the PRC during the first two decades of the Cold War.⁵³ In his first-hand account of his 1960 visit to Maoist China, *The Other Side of the River*, Snow denied that there was a famine or any signs of mass starvation: 'Whatever he was eating, the "average Chinese" maintained himself in good health, as far as anyone could see.'⁵⁴ To further prove the 'average Chinese' was in good health, he cited the 'authoritative' voices of the British doctors who visited China in 1957 as well as the 'authentic' voice of his old friend Shafick George Hatem, the Lebanese-American physician who became the first foreigner to acquire Chinese nationality after the CCP came to power and was another 'brilliant apologist for the Chinese Communists' during the Cold War.⁵⁵ Originally from Buffalo, New York, Hatem went to Shanghai in 1933 during the Great Depression as a medical student looking for an 'Eastern adventure' as well as a livelihood. While in Shanghai, he came to know Madame Sun Yat-sen (Song Qingling), the widow of China's revolutionary leader and the most influential fellow traveller of the CCP after the Nationalist party broke their alliance with the Communists in 1927.[†]

* In the 1960s and 1970s, Chinese communism, often branded as 'Maoism' as opposed to 'bureaucratic' communism in the West and the Soviet Union, appealed to a huge number of radical young leftists (born in the interwar period) in the West as well as in Latin America and Africa.

† After the success of the Northern Expedition, in April 1927 Chiang Kai-shek, who secured his power in the Nationalist party during the Northern Expedition, broke the alliance with the Communist Party and executed a bloody purge of Communists and Communist supporters in Shanghai. Three months later, in July 1927, Wang Jingwei, the Nationalist party's leftist leader, who was seen by many as Sun Yat-sen's anointed heir, followed Chiang Kai-shek's lead by turning on the Communists in Wuhan. Angered by the massacres of thousands of Communists in Shanghai and Wuhan, Song Qingling

关于 1960 年和 1961 年中国死亡率的消息，以及中国饥荒和高死亡率的消息，被西方许多同情中国的人士当作反共宣传而否定或忽视。⁵² 相反，他们求助于埃德加·斯诺（Edgar Snow），他在 1960 年饥荒最严重的时候访问了中国，然后求助于当时居住在中国的英国外科医生约书亚·霍恩（Joshua Horn）博士，以支持毛泽东共产主义领导下中国优越的初级卫生保健系统。^{*}

埃德加·斯诺（Edgar Snow）是来自密苏里州的“反殖民、反帝国主义、支持独立”的激进左派，也是早期全球畅销书《西行漫记》（1937 年）的作者，他是西方最为人知的中国共产党宣传家之一，也是约翰·S·谢伟思（John S. Service）所说的冷战时期美国和中华人民共和国之间的“主要联络人”。⁵³ 在他 1960 年访问毛泽东中国的亲身记述《大河彼岸》中，斯诺否认存在饥荒或任何大规模饥饿的迹象：“无论他吃什么，‘普通中国人’都保持着良好的健康，至少就任何人所能看到的而言。”⁵⁴ 为了进一步证明“普通中国人”的健康状况良好，他引用了 1957 年访问中国的英国医生的“权威”声音，以及他的老朋友沙菲克·乔治·哈特姆（Shafick George Hatem）的“真实”声音，这位黎巴嫩裔美国医生成为新中国成立后第一位获得中国国籍的外国人，也是冷战时期另一位“中国共产党的杰出辩护者”。⁵⁵ 哈特姆最初来自纽约州布法罗市，1933 年大萧条时期前往上海，作为一名医科学生，寻求“东方冒险”和生计。在上海期间，他结识了孙中山夫人（宋庆龄），她是中国的革命领袖的遗孀，也是 1927 年国民党与共产党决裂后中国共产党最具影响力的同路人。[†]

* In the 1960s and 1970s, Chinese communism, often branded as 'Maoism' as opposed to 'bureaucratic' communism in the West and the Soviet Union, appealed to a huge number of radical young leftists (born in the interwar period) in the West as well as in Latin America and Africa.

† After the success of the Northern Expedition, in April 1927 Chiang Kai-shek, who secured his power in the Nationalist party during the Northern Expedition, broke the alliance with the Communist Party and executed a bloody purge of Communists and Communist supporters in Shanghai. Three months later, in July 1927, Wang Jingwei, the Nationalist party's leftist leader, who was seen by many as Sun Yat-sen's anointed heir, followed Chiang Kai-shek's lead by turning on the Communists in Wuhan. Angered by the massacres of thousands of Communists in Shanghai and Wuhan, Song Qingling

In the 1935 'August First Declaration', the CCP strategically shifted from its former position in what had amounted to a Chinese civil war against the Nationalists to be willing to form a new United Front in the wake of the Japanese invasion. Madame Sun, who wholeheartedly supported this new United Front and the 'sacred' cause of resisting the Japanese and saving China, began to actively recruit sympathetic foreigners. This included finding medical personnel as well as raising funds for the International Peace Hospital at the CCP's revolutionary base at Yan'an in the northwest of China. Hatem became increasingly attracted to communism during this period. In 1936, partly in response to Madame Sun's appeal, Hatem decided to go to the Communist base at Yan'an to support their wartime medical work. Madame Sun, who organized the trip, introduced him to Edgar Snow, who was also on the way there. In Yan'an, Hatem subsequently joined the CCP and became involved in its health planning and after 1949 took an active role in the PRC's public health work, in particular the prevention and treatment of venereal diseases and leprosy.⁵⁶ During their reunion in 1960 in Hatem's beautiful but modest little courtyard house in the back of the former imperial palace, the two old 'China hands', Snow and Hatem, bemoaned American racism, McCarthyism, and the appalling state of public health in the United States, comparing the latter with the PRC's achievements in 'eradicating', or the 'near eradication', of a raft of diseases, from cholera and plague to typhus and smallpox, as well as 'controlling' venereal diseases and malaria.⁵⁷ Yet a year after Snow's visit, in 1962 many provinces along or south of the Yangtze experienced a serious pandemic of paracholera caused by a particular strain of the bacterium known as *vibrio cholerae*. At the same time, many of these provinces also experienced some of the worst malaria epidemics in recent memory.*

publicly withdrew from active political work in the Nationalist party. In the following years, she sided with the international leftists and actively supported the Communist underground movement in Shanghai as well as the Communist rural reform programs and its anti-Japanese cause.

* While there were no incidents of cholera and plague reported after 1951 until the time of Snow's visit, the actual vaccination program for both diseases had begun during the Nationalist period. Furthermore, as we know, diseases such as cholera and plague as well as malaria, while there might not be any pandemic for a number of years or in certain seasons in the case of malaria, hence zero or low morbidity, this might have more to do with the condition of the environment and the movement of people and animals than with the actual disease control program.

在1935年的《八一宣言》中，中共从其先前在中国内战中对国民党的立场，战略性地转变为愿意在日本入侵后组建新的统一战线。孙夫人全心全意地支持这个新的统一战线，以及抵抗日本和拯救中国的“神圣”事业，开始积极招募同情中国的外国人。这包括寻找医疗人员，以及为中共在华北西北延安的革命根据地的国际和平医院筹集资金。哈特姆在此期间越来越被共产主义所吸引。1936年，部分是响应孙夫人的呼吁，哈特姆决定前往延安的共产党根据地，支持他们的战时医疗工作。孙夫人组织了这次旅行，并将他介绍给了也正在前往那里的埃德加·斯诺。在延安，哈特姆随后加入了中共，并参与了中共的卫生规划，并在1949年后在中华人民共和国的公共卫生工作中发挥了积极作用，特别是预防和治疗性病和麻风病。⁵⁶在1960年他们重聚时，在哈特姆位于前皇宫后面的美丽而简朴的小四合院里，这两位老“中国通”斯诺和哈特姆感叹美国的种族主义、麦卡锡主义和美国糟糕的公共卫生状况，并将后者与中华人民共和国在“根除”或“接近根除”一系列疾病方面的成就进行比较，这些疾病包括霍乱和鼠疫到斑疹伤寒和天花，以及“控制”性病和疟疾。⁵⁷然而，在斯诺访问一年后，1962年，长江沿线或以南的许多省份经历了由一种特殊的细菌菌株引起的副霍乱严重流行病，这种细菌被称为霍乱弧菌。与此同时，这些省份中的许多省份也经历了近代记忆中最严重的疟疾流行病之一。

*

公开退出了国民党的积极政治工作。在随后的几年里，她站在国际左派一边，积极支持上海的共产主义地下运动以及共产主义农村改革计划和抗日事业。*虽然在1951年之后直到斯诺访问期间，没有报告霍乱和鼠疫事件，但两种疾病的实际疫苗接种计划始于国民党时期。此外，我们知道，霍乱、鼠疫和疟疾等疾病，虽然可能在数年内或在疟疾的某些季节没有大流行，因此发病率可能为零或较低，但这可能与环境状况以及人畜流动有关，而不是与实际的疾病控制计划有关。

Echoing Hatem and Snow, Dr Joshua Horn, in his widely read biographical account of his time in China, *Away with All Pests* (1969), proudly presented to the world the story of ‘how China, once the so-called “Sick Man of Asia”, became the first country in the world to conquer syphilis’ thanks to the political will of the CCP and the ‘voluntary’ effort of the Chinese people.⁵⁸ Joshua Samuel Horn was born in east London to Jewish parents during World War I. In the interwar years, while still a medical student at University College Hospital (UCH), he became actively involved in the struggles against fascism and unemployment, and he subsequently joined the Socialist Medical Association and the Communist Party of Britain. In 1936, he went to China as a ship’s doctor. During his short stay in China, he was appalled by what he saw: ‘China was truly the Sick Man of Asia, rampant with poverty, disease and corruption,’ and he believed that this was caused by feudalism, imperialist oppression, and social evils such as concubinage. ‘China needed a revolution’ was the thought that occupied his mind at the time. In 1949, when the People’s Republic of China was founded, Horn was among the British leftists to embrace its social claims. In 1954, Horn, by then an established surgeon in Britain, took the first opportunity to return to the new China of which he had dreamt. This time, he moved with his family, and they lived there for fifteen years. What he saw, or tended to see, this second time was markedly different. Horn’s new China was an equal and ‘democratic’ communist society where ‘doctors, nurses, orderlies, boiler-men, administrators, Party functionaries, maintenance workers and gardeners, nearly 900 of us all eat together in one huge dining room’ and where ‘rotten’ diseases of the feudal past such as syphilis had been wiped out. According to Horn, by the same political will of the CCP and the ‘voluntary’ and ‘cooperative’ effort of the people, China was on its way to becoming the first country in the world to ‘wipe out’ schistosomiasis – ‘one of the world’s greatest scourges’ and one of the ‘most difficult diseases to eradicate’.⁵⁹ The ‘Sick man of Asia’ had been transformed into ‘the most healthy man in the world’, and politics was the driving force behind China’s remarkable public health achievement, Horn argued forcefully in a public lecture he delivered in New York in 1971, which was subsequently broadcast in May 1972.⁶⁰ In another lecture at the University of Hong Kong a year earlier, Horn had marvelled at China’s Barefoot Doctor program: ‘There are more than one million health workers [in China], and most of them live in the

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countryside. This means one health worker for every six hundred Chinese. For the first time in the five thousand years of Chinese history, China has successfully achieved to deliver health services to its people no matter where they live. China is the first one in the world to achieve this. Yet in wealthy countries such as the United States millions of people have no access to health care.’⁶¹ This last sentence was endorsed by many leftists in the United States who advocated for China as the bastion of a new health care model and who, like Snow and Hatem, were of the generation born and raised during the Great Depression. As Birn and Brown have argued, the impact of McCarthyism and racism in the United States and the Cold War had driven this generation of ‘health-leftists’ to channel ‘much of their political energy beyond U.S. borders, subsumed under the promotion of other bold and worthy causes’. Thus, while Jack Geiger became attracted by the anti-racist, community-oriented primary health care experiments he encountered in South Africa, Victor and Ruth Sidel promoted the Barefoot Doctors they found in Maoist China.⁶² From the Sidel’s first visit to the PRC in the summer of 1971 to the WHO’s Alma Ata Conference in September 1978, the global enthusiasm for the ‘Chinese approach to health’ increased exponentially. What represented the successes of this system and stood for China’s ‘supreme’ health care system were the examples of the Barefoot Doctor program and the Anti-Schistosomiasis Campaign. However, the entire edifice was a Potemkin’s village created to be attractive to visitors passing through the PRC, and the gold-star attractions came to be these two exemplary programs.

THE ‘CHINESE APPROACH TO HEALTH’

Fourteen years after the British doctors’ ‘medical mission’ and ten years after Snow’s trip to the PRC, in the summer of 1971, in the midst of the Great Proletarian Cultural Revolution, a group of prominent physicians and leaders in the field of public health from the United States, again at the invitation of the CMA, visited the PRC. The visit had been initiated by none other than Edgar Snow, who by this point was in political exile from the United States and living in Switzerland – but for Chinese political leaders such as Premier Zhou Enlai, Snow remained a central conduit to the Americans. Between 1970 and 1971, after his second trip to the PRC at the invitation of the CCP Central Committee to observe and report on the Chinese Great Proletarian

乡村。这意味着平均每六百个中国人就有一名卫生工作者。在中国五千年的历史中，中国首次成功地将其卫生服务覆盖到所有人民，无论他们居住在哪里。中国是世界上第一个实现这一目标的国家。然而，在像美国这样的富裕国家，数百万人民却无法获得医疗保健。⁶¹ 最后一句话得到了美国许多左派人士的赞同，他们提倡将中国作为一种新的医疗保健模式的堡垒，并且像斯诺和哈特姆一样，都是在大萧条时期出生和长大的一代人。正如伯恩和布朗所说，麦卡锡主义和种族主义在美国的影响以及冷战驱使这一代 “健康左派” 将 “他们的大部分政治精力转移到美国境外，投入到其他大胆而有价值的事业的推广中”。因此，当杰克·盖格被他在南非遇到的反种族主义、以社区为中心的初级卫生保健实验所吸引时，维克多和露丝·西德尔则推广了他们在毛泽东时代的中国发现的赤脚医生。⁶² 从西德尔夫妇 1971 年夏天首次访问中国到 1978 年 9 月世界卫生组织的阿拉木图会议，全球对 “中国卫生方法” 的热情呈指数级增长。赤脚医生计划和血吸虫病防治运动代表了该制度的成功，并代表了中国 “至高无上” 的医疗保健体系。然而，整个建筑都是一个波将金村，旨在吸引通过中国的游客，而最吸引人的景点是这两个示范项目。

“中国卫生之道”

在英国医生 “医疗使命” 十四年后，斯诺访问中华人民共和国十年后，1971 年夏天，正值无产阶级文化大革命时期，一群来自美国的著名医生和公共卫生领域的领导者，再次应中国医学学会的邀请访问了中华人民共和国。这次访问是由埃德加·斯诺发起的，此时的他正流亡瑞士，远离美国政治中心——但对于中国政治领导人，如周恩来总理来说，斯诺仍然是与美国人沟通的重要渠道。1970 年至 1971 年间，在应中共中央邀请第二次访问中国，观察和报道中国无产阶级

Cultural Revolution, Snow published numerous articles about his trip and his interviews with Chinese leaders in *Life* magazine as well as in its popular Italian counterpart *Epoca*. In several articles for *Epoca*, he detailed the PRC's achievements in medicine and public health, in particular the birth control efforts and the Barefoot Doctor program.⁶³ While it stirred up a great deal of interest in the United States, his reporting also attracted criticism. A number of critics discredited Snow's account on the grounds that he had no professional training in medicine or public health. To defend himself, Snow arranged for his friend E. Grey Dimond, a respected cardiologist, medical educator, and innovator who had founded the University Missouri-Kansas City School of Medicine, to go to the PRC and make his own professional observations and report on them.⁶⁴

This was the first time in twenty-four years that health professions from both countries had made 'official' contact. It has often been referred to as the 'historical visit'.⁶⁵ Dimond was accompanied by Paul Dudley White, the distinguished cardiologist and President Eisenhower's personal physician; Samuel Rosen, the internationally acclaimed ear surgeon; Victor Sidel, the chair of Social Medicine at Montefiore Medical Centre and the professor of Social Medicine at the Albert Einstein College of Medicine; and Ruth Sidel, a social worker at a primary care paediatric health centre at Einstein College who was also committed to the cause of women's and children's rights and well-being. During their twelve-day guided tour, the Americans were taken to see twelve model medical and health facilities and had thirty hours of scheduled discussions with officially chosen Chinese physicians. While none of the Americans had been to China before and while they spoke not a word of Chinese, according to Dimond the *lingua medica* such as stethoscope, ophthalmoscope, electrocardiogram, and laboratory data 'bridged whatever other language gap existed'. The trip impressed Dimond deeply: 'Eight hundred million people, one-fourth of the world's population, are hard at work, disciplined, honest, courteous, intelligent, healthy, and give every evidence of enthusiastically learning and practicing a Communistic way of life.' Writing about the trip for the *Saturday Review* – a widely circulated weekly magazine that promoted liberal causes such as nuclear disarmament and world peace – Dimond warned the American public not to be blinded by the 'Bamboo Curtain' that 'has only obscured our view, including that of medicine ... I think we Americans do ourselves harm, blind ourselves to facts,

文化大革命期间，斯诺发表了大量关于他的旅行以及他在生活杂志和广受欢迎的意大利同行 *Epoca* 中对中国领导人的采访的文章。在为 *Epoca* 撰写的几篇文章中，他详细介绍了中华人民共和国在医学和公共卫生方面的成就，特别是计划生育工作和赤脚医生计划。⁶³ 虽然这引起了美国极大的兴趣，但他的报道也引来了批评。许多批评家以斯诺没有医学或公共卫生方面的专业培训为由，否定了他的说法。为了为自己辩护，斯诺安排了他的朋友 E. Grey Dimond

（一位受人尊敬的心脏病专家、医学教育家和创新者，他创立了密苏里大学堪萨斯城医学院）前往中华人民共和国，进行他自己的专业观察并报告结果。⁶⁴

这是二十四年来两国卫生专业人员首次进行“官方”接触。它通常被称为“历史性访问”。⁶⁵ Dimond 的陪同人员包括杰出的心脏病专家、艾森豪威尔总统的私人医生 Paul Dudley White；国际知名的耳科外科医生 Samuel Rosen；Montefiore 医疗中心社会医学系主任、阿尔伯特·爱因斯坦医学院社会医学教授 Victor Sidel；以及爱因斯坦学院初级保健儿科卫生中心的社工 Ruth Sidel，她也致力于妇女和儿童的权利和福祉事业。在他们为期十二天的导游陪同参观中，美国人参观了十二个模范医疗卫生设施，并与官方挑选的中国医生进行了三十个小时的预定讨论。虽然这些美国人以前都没有去过中国，而且他们一句中文也不会说，但根据 Dimond 的说法，诸如听诊器、检眼镜、心电图和实验室数据等 *lingua medica* “弥合了其他任何语言障碍”。这次旅行给 Dimond 留下了深刻的印象：“8 亿人，占世界人口的四分之一，正在努力工作，有纪律，诚实，有礼貌，聪明，健康，并且充分证明了他们正在热情地学习和实践共产主义的生活方式。” Dimond 为《星期六评论》（一本广泛发行的周刊，提倡核裁军和世界和平等自由主义事业）撰写了关于这次旅行的文章，警告美国公众不要被“竹幕”蒙蔽双眼，因为“竹幕只会模糊我们的视线，包括对医学的看法……我认为我们美国人正在伤害自己，对事实视而不见，”

and perpetuate myths.’ To support his argument, he compared the Chinese ‘achievements’ in public health and the PRC government’s ability to provide ready, inexpensive, and convenient health care to its rural population of 700 million to the failure of the American government to provide adequate health care to the American people. ‘China is more than Communism,’ he concluded forcefully.⁶⁶

Echoing Dimond, his fellow delegate Victor Sidel also felt strongly that China’s ‘revolutionary experience’ in health and medicine could offer valuable lessons for public health and community health work in the United States as well as for the rest of the world. Also born during the Great Depression, Sidel’s own commitment to community primary care grew as a reaction to McCarthyism and Cold War politics. Like many in the American left of their generation, Victor Sidel and his wife Ruth were strongly committed to causes for greater social and economic justice both at home and abroad. As a physician, Victor Sidel channelled much of his political energy into public health work, in particular community health undertakings, and he co-founded Physicians for Social Responsibility, the largest physician-led organization, aimed at ‘creating a healthy, just and peaceful world’. In preparation for their first China visit, the Sidels read Edgar Snow and Joshua Horn, as well as William Hinton’s *Fanshen*.⁶⁷ Their writings presented health care in the PRC in the most favourable light possible, and according to the Sidels, the ‘detailing [of] the extraordinary changes in China during and after the revolution, were particularly useful’.⁶⁸ It was from these works that they developed their questions about health care delivery and community primary care in rural China. In another words, it was through these clearly propagandistic texts that China’s public health achievement as a major medical and political success was to be approached and evaluated.

Their ‘historical visit’ to the PRC also antedated Nixon’s visit and the diplomatic opening to China in 1972, both of which had been brokered, in an odd way, by Edgar Snow’s 1970 discussions with Mao and Zhou Enlai, reported in *Time* magazine, where the idea of Nixon’s trip was first broached. Suddenly, China became a ‘hot topic’. Upon returning to the United States, the Sidels and Dimond, as well as Joshua Horn (who had by then left the PRC), spent much time lecturing and writing to promote the PRC’s purported public health achievements. Their lectures and publications were enthusiastically received by a wider audience, from the FBI agents who

并使神话永存。’ 为了支持他的论点，他将中国在公共卫生方面的“成就”以及中国政府向7亿农村人口提供现成、廉价和便捷的医疗保健的能力与美国政府未能向美国人民提供足够的医疗保健进行了比较。他有力地总结说：‘中国不仅仅是共产主义。’⁶⁶

与迪蒙德一样，他的同事维克多·西德尔也强烈认为，中国在卫生和医学方面的“革命经验”可以为美国以及世界其他地区的公共卫生和社区卫生工作提供宝贵的经验。西德尔也出生于大萧条时期，他对社区基层医疗的承诺是对麦卡锡主义和冷战政治的一种回应。与他们那一代美国左翼人士一样，维克多·西德尔和他的妻子露丝都坚定地致力于为国内外更大的社会和经济正义事业而奋斗。作为一名医生，维克多·西德尔将他的大部分政治精力投入到公共卫生工作中，特别是社区卫生事业，他共同创立了“医生促进社会责任”组织，这是由医生领导的最大组织，旨在“创造一个健康、公正和和平的世界”。在他们首次访问中国之前，西德尔夫妇阅读了埃德加·斯诺和约书亚·霍恩的作品，以及韩丁的翻身。⁶⁷ 他们的作品尽可能地以最有利的光芒呈现了中华人民共和国的医疗保健，并且根据西德尔夫妇的说法，“详细描述了[中国在革命期间和革命后发生的非凡变化，]特别有用”。⁶⁸ 正是从这些作品中，他们对中国农村的医疗保健服务和社区基层医疗产生了疑问。换句话说，正是通过这些明显的宣传文本，中国作为一项重大的医学和政治成就的公共卫生事业才得以被接近和评估。

他们在尼克松访华和1972年中美外交关系解冻之前，就对中国进行了“历史性访问”。而尼克松访华和中美外交关系解冻，在某种奇怪的程度上，是由埃德加·斯诺1970年与毛泽东和周恩来的谈话促成的，这一谈话在《时代》杂志上发表，尼克松访华的想法首次被提出。突然，中国成为一个“热门话题”。回到美国后，西德尔夫妇、迪蒙德以及约书亚·霍恩（当时已离开中国）花了大量时间进行演讲和写作，以宣传中国所谓的公共卫生成就。他们的讲座和出版物受到了更广泛受众的热烈欢迎，包括联邦调查局的特工。

shadowed them to the general public who were fascinated to hear that ‘starving children in China’ had turned into happy, healthy, and ‘cooperative human beings’.⁶⁹

Amongst the Sidels’ ‘new audience’ were also critical voices of the ‘medicationization of life’ who were pushing for the ‘deprofessionalization’ of medicine. In the West, the scientific and technological revolution in medicine that had begun with the end of World War II had seemingly peaked by the late 1960s. Great changes in medical care and access were, however, slower in coming. There was a growing consensus that medicine in the Western world was in a state of crisis. Despite huge efforts and wealth being invested into health care and medicine, the reality was that the general health of the population was deteriorating. In his highly controversial but influential work, *Medical Nemesis* (1975), Ivan Illich, the Viennese-born radical thinker, famously argued that modern allopathic medicine and hospital-centred health care had become a major threat to health in the world: ‘After a century of pursuit of medical Utopia, and contrary to current conventional wisdom, medical services have not been important in producing the changes in life expectancy that have occurred. A vast amount of contemporary clinical care is incidental to the curing of disease, but the damage done by medicine to the health of individuals and populations is very significant.’⁷⁰

This was ‘the closest I ever came to a religious experience,’ said Richard Smith, who later became the editor of the *British Medical Journal* (*BMJ*) and ‘a pillar of the British medical establishment’, recalling the effect of Illich’s words on him at the time: ‘I dropped out of medical school that day [after hearing Illich]. Three days later I dropped back in again, unsure what else to do.’⁷¹ Smith’s anxiety was shared by a great number of people in medicine and public health in the West. There was a strong sense that ‘new ideas are needed, new systems of health care have to be explored.’⁷² Accounts by Joshua Horn and George Hatem, as well as other Western health observers of the PRC – particularly in the popular works by the Sidels and echoed in the 1972 documentary film *Barefoot Doctor of Rural China* produced by a group of scholars from Stanford University – claimed that the health care delivery system in the PRC that relied on non-professional health workers (the Barefoot Doctors) offered a radical new approach to the people’s health. This ‘Chinese approach’, understood as a draconian government measure aimed at providing ready, convenient, and inexpensive health care to the 700

让对听到 “中国饥饿的儿童” 已经变成快乐、健康和 “合作的人” 感到着迷的公众，对他们投下了阴影。⁶⁹

在西德尔夫妇的 “新听众” 中，也有对 “生活药物化” 持批评态度的声音，他们正在推动医学的 “去专业化”。在西方，二战结束后开始的医学科学技术革命似乎在 20 世纪 60 年代末达到顶峰。然而，医疗保健和获取方面的重大变革来得比较缓慢。人们越来越一致认为，西方世界的医学正处于危机之中。尽管在医疗保健和医学方面投入了巨大的努力和财富，但现实情况是，人口的总体健康状况正在恶化。在极具争议但又颇具影响力的著作《医学的报应》（1975 年）中，维也纳出生的激进思想家伊万·伊里奇（Ivan Illich）提出了著名的论点，即现代对抗疗法医学和以医院为中心的医疗保健已成为世界健康的主要威胁：“经过一个世纪对医学乌托邦的追求，与当前传统的观点相反，医疗服务在产生预期寿命的变化方面并不重要。大量的当代临床护理与疾病的治愈无关，但医学对个人和人群健康造成的损害非常严重。”⁷⁰

理查德·史密斯（Richard Smith）回忆起伊利奇当时对他的影响时说，这是 “我最接近宗教体验的一次”，他后来成为英国医学杂志（*BMJ*）的编辑，也是 “英国医学界的栋梁”：“那天我听了伊利奇的话后退出了医学院。三天后，我又回来了，不知道该做什么。”⁷¹ 史密斯的焦虑在西方医学和公共卫生领域引起了许多人的共鸣。人们强烈地感觉到 “需要新的想法，必须探索新的医疗保健系统。”⁷² 约书亚·霍恩（Joshua Horn）和乔治·海德姆（George Hatem）以及其他西方卫生观察员对中国所作的描述——特别是在西德尔夫妇的受欢迎的作品中，并在斯坦福大学的一群学者制作的 1972 年纪录片中国农村的赤脚医生中得到呼应——声称中国依赖于非专业卫生工作者（赤脚医生）的医疗保健服务体系为人民健康提供了一种全新的方法。这种 “中国方法” 被理解为一项严厉政府措施，旨在为 700

million people who lived in the rural countryside, became increasingly attractive to Western health professionals and policy-makers who were seeking a way out of the perceived Western health crisis. One needs to note here that the health care crises in the United States at the time were perceived as centred in the deteriorating urban landscapes. The fascination earlier in the century with rural poverty and health care, such as that represented by Joseph Goldberger's work on pellagra, had given way to the perception that greater urbanization had created even greater risks to the people's health.⁷³

In the next decade, a growing number of American health professionals were offered opportunities to visit the PRC. Like Dimond and the Sidels, they were impressed by the PRC's purported achievements in public health, particularly how China had managed to resolve the problem of providing health care to its 700 million people living in the countryside, whereas the United States had failed to do anything like this. One such individual was Phillip Lee, professor of social medicine at the University of California's School of Medicine in San Francisco. In 1965, Lee had participated in the first White House Conference on Health when the Secretary of State, Dean Rusk, famously declared the export of American health capacity as one of the strongest aspects of US foreign policy.⁷⁴ For the next decade, in his capacity as a medical educator, Lee advocated for health professionals to work in concert with the United States government to meet the growing demands of a changing society and globally. In 1974, after spending several months in China as the guest of the CMA, Lee joined the Sidels and Dimond in promoting the PRC's health care delivery system back home:

It is clear that there has been a pronounced decline in the death rate, particularly infant mortality. Major epidemic diseases have been controlled, and in some cases apparently eradicated. Nutritional status has been improved. Massive campaigns of health education and environmental sanitation have been carried out. Large numbers of health workers have been trained, and a system has been developed that provides some health service for the great majority of the people ... How, in so short a time, did the Chinese manage to transform a country where the death rate was among the highest in the world, epidemic diseases were widespread, sanitation was primitive and modern medical care was essentially limited to the urban elite, into a country where

居住在农村地区的数百万人越来越吸引西方卫生专业人员和政策制定者，他们正在寻找摆脱西方健康危机的方法。这里需要注意的是，当时美国医疗保健危机被认为是集中在恶化的城市景观中。本世纪初对农村贫困和医疗保健的迷恋，例如约瑟夫·戈德伯格对糙皮病的研究，已经让位于这样一种观念，即更大的城市化给人民的健康带来了更大的风险。⁷³

在接下来的十年里，越来越多的美国卫生专业人员获得了访问中华人民共和国的机会。像戴蒙德和西德尔夫妇一样，他们对中华人民共和国在公共卫生方面取得的成就印象深刻，特别是中国如何设法解决了向居住在农村地区的 7 亿人民提供医疗保健的问题，而美国未能做到这一点。菲利普·李就是其中一位，他是加州大学旧金山医学院的社会医学教授。1965 年，李参加了第一届白宫健康会议，当时国务卿迪安·腊斯克公开宣布，美国对外输出医疗能力是美国外交政策最强大的方面之一。⁷⁴ 在接下来的十年里，李以医学教育家的身份，倡导卫生专业人员与美国政府合作，以满足不断变化的社会和全球日益增长的需求。1974 年，在作为中国医学会（CMA）的客人访问中国几个月后，李加入了西德尔夫妇和戴蒙德的行列，在国内推广中华人民共和国的医疗保健服务体系：

显而易见的是，死亡率，特别是婴儿死亡率，已经显著下降。主要的流行病得到了控制，在某些情况下甚至被根除。营养状况得到了改善。大规模的健康教育和环境卫生运动已经开展。大量的卫生工作者已经接受培训，并且已经建立了一个为绝大多数人提供某种卫生服务的系统……在如此短的时间内，中国人是如何将一个死亡率居世界前列、流行病蔓延、卫生条件落后、现代医疗基本上仅限于城市精英的国家，转变为一个

the health status of the population now ranks well ahead of that of most developing nations, most of the people have an appreciation of sanitation and the great majority have access to health care? Is there anything Americans can learn from the Chinese experience in medicine and public health?⁷⁵

To learn from the Chinese experience, Lee joined others in pushing the United States government to prioritize public health programs at home and globally, centred on community health programs. In the meantime, modelled on the Chinese Barefoot Doctor program as well as on parallel programs in Cuba and Chile, the Sidels helped to build a Community Health Participation Program in the Bronx, New York, that recruited, trained, and supervised neighbourhood health workers.⁷⁶ The Bronx, in the 1970s, was very poor and underserved but also very urban. The Sidels' focus had moved from rural China to urban America.

HEALTH DIPLOMACY

While medicine had been part of European 'soft power' in China before the CCP's seizure of power in 1949, as of the late 1950s the PRC government skilfully utilized Western fascination with its mass public health achievements, in particularly the Anti-Schistosomiasis Campaign, to win support internationally. By the second half of the 1960s, an increasing number of Western critics of existing health disparities in industrialized nations and advocates of social medicine came to view the PRC's Barefoot Doctor program, fashioned as Mao's 'revolutionary approach' to the people's health, as able to deliver ready, convenient, and inexpensive health care to the millions living in rural China as a true alternative model for primary care and health care delivery. This complemented increasingly positive images of the PRC and its totalitarian government purveyed by its cohort of Western sympathizers, who contrasted Communist China with the American aggression in Vietnam that was seen as aimed at suppressing a popular revolution. In developing countries and the United Nations, the rise of the 'Third Way', championed by Yugoslavia and India, offered China a global space to effect its political goals through exporting its model of health care.

In 1972, a year after their first visit, the Sidels were invited by the CMA to return to China and undertake more detailed and extensive

现在，中国的人口健康状况远超大多数发展中国家，大多数人对卫生有认识，并且绝大多数人都能获得医疗保健？美国人能从中国的医药和公共卫生经验中学到什么？⁷⁵

为了学习中国的经验，李和其他人一起推动美国政府优先考虑国内和全球的公共卫生项目，重点是社区卫生项目。与此同时，西德尔夫妇以中国的赤脚医生项目以及古巴和智利的类似项目为蓝本，在纽约布朗克斯区建立了一个社区健康参与项目，该项目招募、培训和监督社区卫生工作者。⁷⁶ 20 世纪 70 年代的布朗克斯区非常贫穷且服务不足，但也非常城市化。西德尔夫妇的关注点已经从中国农村转移到美国城市。

卫生外交

虽然在 1949 年中国共产党夺取政权之前，医学一直是欧洲在华

“软实力”的一部分，但从 20 世纪 50 年代末开始，中华人民共和国政府巧妙地利用西方对其大规模公共卫生成就的迷恋，特别是血吸虫病防治运动，以赢得国际支持。到 20 世纪 60 年代后半期，越来越多的西方批评家认为工业化国家现有的卫生差距以及社会医学的倡导者，开始将中华人民共和国的赤脚医生计划（被塑造为毛泽东“革命性方法”的人民健康）视为能够为居住在中国农村的数百万人民提供现成、便利且廉价的医疗保健，作为初级保健和医疗保健服务的真正替代模式。这补充了西方同情者对中华人民共和国及其极权政府日益积极的形象，他们将共产主义中国与美国在越南的侵略进行了对比，后者被视为旨在镇压一场受欢迎的革命。在发展中国家和联合国，由南斯拉夫和印度倡导的“第三条道路”的兴起，为中国提供了一个全球空间，通过输出其医疗保健模式来实现其政治目标。

1972 年，在他们首次访问一年后，西德尔夫妇应中国医学学会的邀请重返中国，进行了更详细和广泛的

observations about China's public health innovations. In the same year, Dimond brought the first PRC medical delegation to the United States. From then on, together with Joshua Horn, they became the PRC's most useful 'international friends' in the arena of international public health. While the Sidels acted as spokespeople for the PRC, Dimond took many groups, both medical and lay, to visit China to promote cultural exchanges between the West and the PRC. As a reward for his 'friendship', he was given honorary professorships at several medical schools in China as well as rare access to many key events in the PRC. The Chinese Minister of Foreign Affairs Huang Hua was among one of his 'close friends' in China.

At the same time, governments in postcolonial Africa, Southeast Asia, and Latin America, where healing and indigenous medicines played an integral part in the political struggle for a new postcolonial identity, had become impressed by China's reported ability to redress health care disparity. They were particularly interested in the way the Chinese had managed to reduce the health burden of the state by incorporating existing or traditional medical practices into its national health service.⁷⁷ In 1967, for example, a medical delegation from Tanzania visiting China was said to be 'impressed by the stage of development of health services, which have been revolutionized and transformed by the new China'.⁷⁸ The training of Barefoot Doctors, as well as the integration of indigenous methods and allopathic medicine – the latter was often branded 'modern' – were seen by these Tanzanian officials as a useful model by which to develop a new national health care delivery system that would cater to its rural populations.⁷⁹ Partly driven by their commitment to non-aligned socialism, a number of African countries, as well as socialist countries in Southeast Asia and Latin America, approached China for health collaboration that included training and health aid.

The PRC leadership quickly capitalized on this, viewing health cooperation with these Third World countries as 'inexpensive but profitable' undertakings that could help its effort to promote a new international order – a 'people's revolutionary movement in Asia, Africa and Latin America' against 'colonialism, imperialism and hegemonism'.⁸⁰ After learning from Luis Echeverría Álvarez – the president of Mexico who had just embarked on a populist political and economic reform that included massive state spending on improving the people's health – that he was planning to promote the PRC's Barefoot Doctor program in Mexico, Mao, for instance,

对中国公共卫生创新的一些观察。同年，戴蒙德率领第一个中国代表团访问美国。从那时起，他们与约书亚·霍恩一起，成为中国在国际公共卫生领域最有用的“国际友人”。西德尔夫妇担任中国发言人的同时，戴蒙德带领许多医疗和非医疗团体访问中国，以促进西方与中国之间的文化交流。作为对其“友谊”的回报，他被授予中国几所医学院的名誉教授职位，并罕见地获准参加中国许多重要活动。中国外交部长黄华是他在中国的“亲密朋友”之一。

与此同时，后殖民时期的非洲、东南亚和拉丁美洲的政府，其中治疗和本土医学在争取新的后殖民身份的政治斗争中发挥了不可或缺的作用，对中国报告的纠正医疗保健差距的能力印象深刻。他们对中国人如何通过将现有或传统的医疗实践纳入其国家卫生服务来减轻国家的健康负担的方式特别感兴趣。⁷⁷ 例如，1967年，来自坦桑尼亚的医疗代表团访问中国时，据说“对卫生服务的发展阶段印象深刻，卫生服务已被新中国彻底改变和改造”。⁷⁸ 赤脚医生的培训，以及本土方法和对抗疗法（后者通常被称为“现代”）的结合，被这些坦桑尼亚官员视为开发一种新的国家医疗保健提供系统的有用模式，该系统将迎合其农村人口。⁷⁹ 部分原因是出于对不结盟社会主义的承诺，一些非洲国家以及东南亚和拉丁美洲的社会主义国家与中国进行了包括培训和卫生援助在内的卫生合作。

中国领导人迅速利用了这一点，将与这些第三世界国家的卫生合作视为“廉价但有利可图”的事业，这有助于其推动建立新的国际秩序——一场反对“殖民主义、帝国主义和霸权主义”的“亚洲、非洲和拉丁美洲人民的革命运动”。⁸⁰ 在向路易斯·埃切维里亚·阿尔瓦雷斯（墨西哥总统，他刚刚开始进行包括大量国家支出来改善人民健康的民粹主义政治和经济改革）了解到他计划在墨西哥推广中国的赤脚医生计划后，例如毛泽东，

advised Xiong Xianghui, the PRC's first ambassador to Mexico, to spend some time in the Chinese countryside to study the Barefoot Doctors.⁸¹ A couple of years earlier, on 5 June 1971, in a conversation with the government delegation from Somalia, Zhou Enlai had carefully emphasized that health collaborations between the two countries were intended to serve the local interests and thus to promote friendship. Chinese medical teams had to avoid the 'super-power chauvinism' associated with the Americans.⁸²

Beginning with sending a medical team to 'aid' newly independent Algeria in 1963, the PRC continuously dispatched medical teams to more than twenty-two countries in Africa over the next decades. Different provinces in China were twinned with African nations. For instance, Sichuan was responsible for developing malaria and schistosomiasis control projects in Mozambique, whereas teams of doctors of both allopathic medicine and traditional Chinese medicine (TCM) from Shandong went to Tanzania and other African countries.⁸³ In addition, China provided medical and health training to a large number of students from Africa, Southeast Asia, and Latin America, competing with the extensive parallel programs of training such physicians in the Soviet Union, such as that at the Lumumba Friendship University in Moscow. China also regularly sent 'friendship delegations' to these countries; the delegations always included at least one health worker as a model 'Barefoot Doctor'. With this expansion, China's influence on health care delivery in Asia, Latin America, and Africa grew exponentially. China's 'friendship', centred in health collaborations, also paved way for the re-emergence of traditional medical practice and a fast-growing herbal market in postcolonial settings such as Tanzania.⁸⁴ In Mozambique, on the other hand, acupuncture imported from the PRC as part of the newly created 'Traditional Chinese Medicine' became widely popular.⁸⁵ At the same time, China's Barefoot Doctor program worked particularly well in the local rural conditions in many African countries.⁸⁶ In contrast, the centralized Soviet model was perceived as less effective.

Contemporary sources that have recently emerged from Chinese archives also show that politics was always an integral part of Chinese medical activities and its selling of Sino-African friendship. Chinese medical teams were fully aware that their mission was to spread

* In September 2017, the Ministry of Health in Mozambique announced the introduction of acupuncture into main provincial hospitals in parts of the country.

中国首任驻墨西哥大使熊向晖被建议花一些时间在中国农村研究赤脚医生。⁸¹ 几年前, 1971年6月5日, 周恩来在与索马里政府代表团的谈话中, 仔细强调了两国之间的卫生合作旨在服务于当地利益, 从而促进友谊。中国医疗队必须避免与美国人有关的“超级大国沙文主义”。⁸²

从1963年向新独立的阿尔及利亚派遣医疗队“援助”开始, 中国在随后的几十年里不断地向非洲的二十多个国家派遣医疗队。中国不同的省份与非洲国家结成对子。例如, 四川负责在莫桑比克开发疟疾和血吸虫病控制项目, 而来自山东的西医和中医(tcm)医生团队则前往坦桑尼亚和其他非洲国家。⁸³ 此外, 中国还为来自非洲、东南亚和拉丁美洲的大量学生提供医疗和卫生培训, 与苏联在莫斯科卢蒙巴友谊大学等机构培训此类医生的广泛平行项目竞争。中国还定期向这些国家派遣“友好代表团”; 代表团总是至少包括一名卫生工作者作为模范“赤脚医生”。随着这种扩张, 中国对亚洲、拉丁美洲和非洲的医疗保健服务的影响呈指数级增长。中国的“友谊”以卫生合作为中心, 也为后殖民背景下传统医学实践的重新出现以及草药市场的快速增长铺平了道路, 例如在坦桑尼亚。⁸⁴ 另一方面, 在莫桑比克, 从中国进口的针灸作为新创建的“中医药”的一部分, 广受欢迎。⁸⁵ 与此同时, 中国的赤脚医生项目在许多非洲国家的当地农村条件下运作得特别好。⁸⁶ 相比之下, 集中的苏联模式被认为效果较差。

最近从中国档案馆中涌现出来的当代资料也表明, 政治始终是中国医疗活动及其推销中非友谊不可分割的一部分。中国医疗队充分意识到他们的使命是传播

* 2017年9月, 莫桑比克卫生部宣布在该国部分地区的主要省级医院引入针灸。



Figure 9.1 Africans revering a Mao portrait.



图 9.1 非洲人敬仰毛泽东画像。

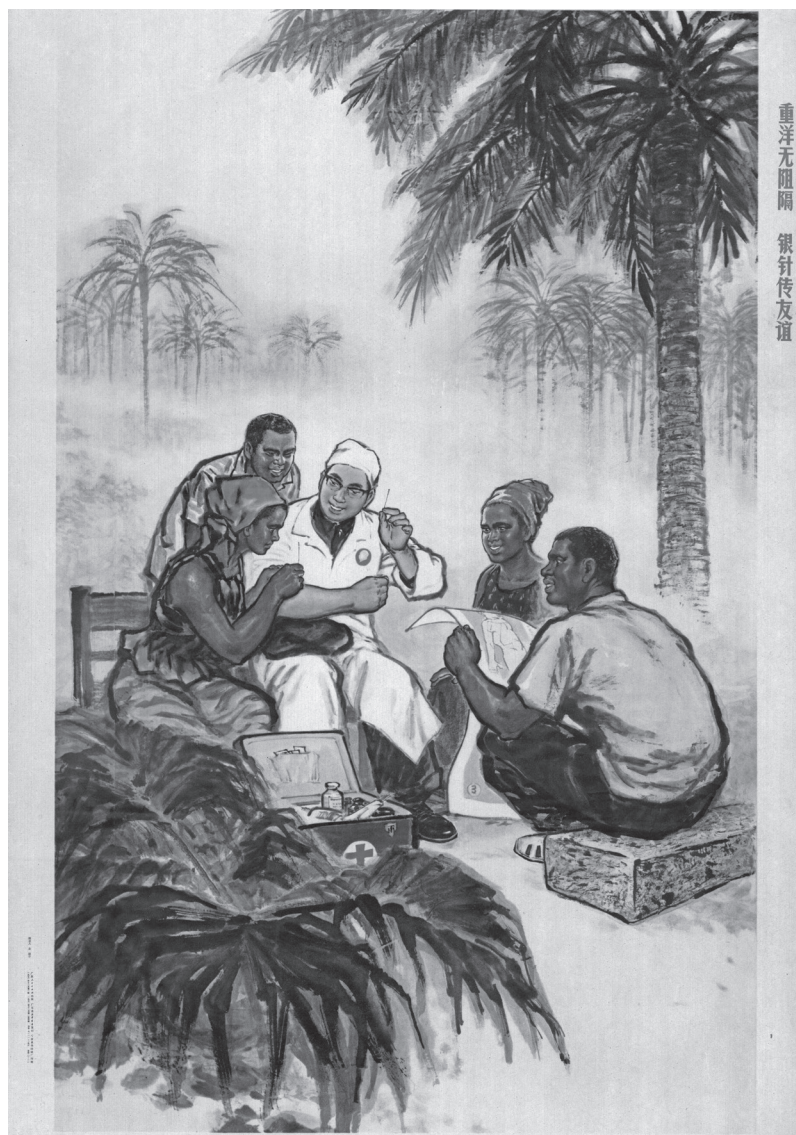


Figure 9.2 Chinese doctor performing acupuncture in Africa.

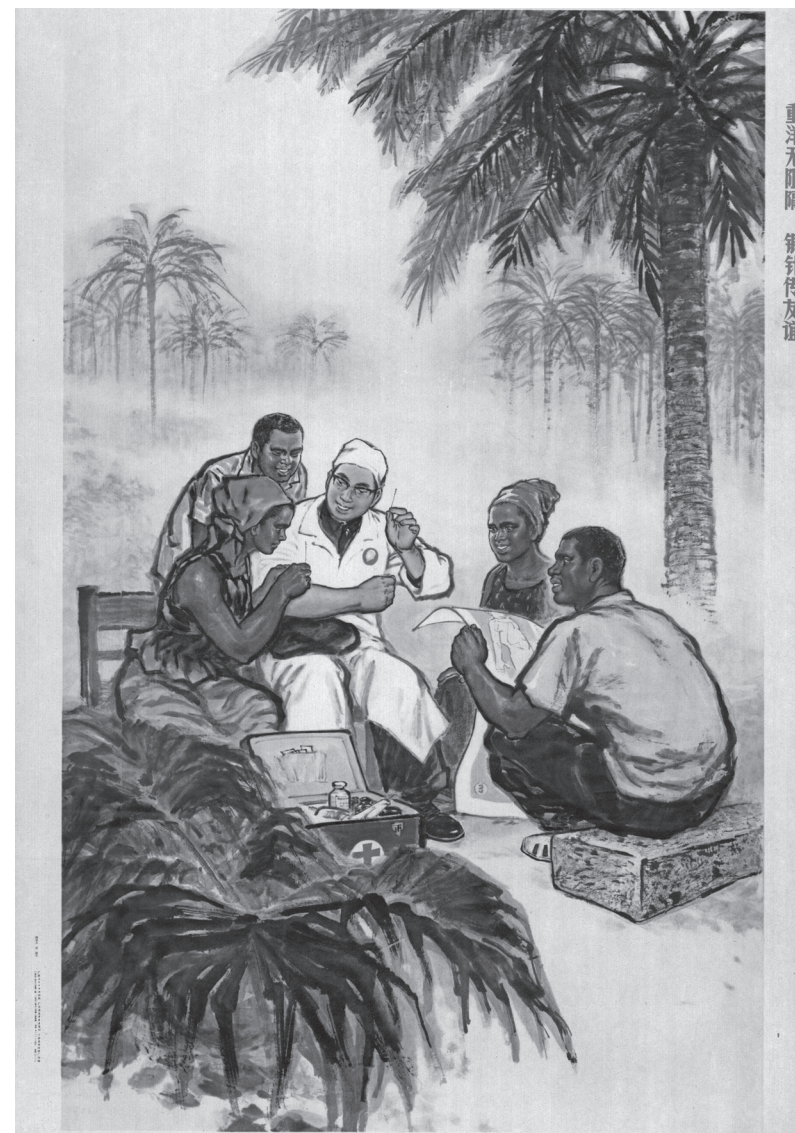


图 9.2 中国医生在非洲进行针灸。

'The Thought of Mao Zedong' (毛泽东思想 or Maoism) in addition to their medical work. For example, while researching the African variety of schistosomiasis in Somalia, the Chinese medical team put up an exhibition showing how under the leadership of Mao, China had successfully eradicated this deadly disease. Those invited to the exhibition included workers, rural residents, officials, middle school students, and policemen. Not only was the medical and public health propaganda rampant, the villagers even learnt to sing 'The East Is Red'. This medical and cultural propaganda bore fruit. After seeing the exhibition, one local official went to the Chinese team and said: 'Thank you. Compared to the American, the British, and the Italian doctors, the Chinese are our most loyal friends.' Another government technician was convinced that the 'Thought of Mao Zedong' was the only guarantee that could bring happiness to the people of the world. During the Somalian New Year celebration in 1971, a group of workers and rural villagers sang 'The East Is Red' while holding Mao's portrait aloft. After singing and dancing, they also shouted: 'We thank the Chinese doctors for helping us', 'The Chinese bring us food and cure us from diseases', 'China brings us life. Don't invade China'.⁸⁷ Whether the medical interventions were as successful as the propaganda claimed is questionable, but the end effect was to establish China as the model for improving the people's health.

At the UN, the PRC's increasing medical 'humanitarian' activities and the bonds of friendship created through such undertakings helped the PRC win its battle against the Republic of China (Taiwan) for the permanent 'Chinese' seat at the Security Council and membership in the General Assembly. At the 1971 UN General Assembly meeting, twenty-six African nations voted in favour of legitimating the status of the People's Republic of China in the UN.⁸⁸ At the same time, the PRC was seen as a crucial player in the so-called 'African issues': 'China could outdo the Soviet Union rhetorically and also sponsor action that is too rich for Moscow's blood,' argued American television correspondent Richard Hottelet, covering the United Nations at the time.⁸⁹ In placing the PRC in the UN, the sense was that it would prevent China from supplying military aid to the various national liberation movements in Africa, given the Chinese support for such activities in Southeast Asia. In comparison, Taiwan was considered too marginal a player in this matter.⁹⁰

With its entrance into the UN in 1973, the PRC was invited to rejoin the World Health Assembly.⁹¹ The PRC began to use the World

除了医疗工作之外, 还有“毛泽东思想”(毛泽东思想或毛泽东主义)。例如, 在索马里研究非洲血吸虫病时, 中国医疗队举办了一个展览, 展示了在中国共产党的领导下, 中国是如何成功根除这种致命疾病的。受邀参观展览的人包括工人、农村居民、官员、中学生和警察。不仅医疗和公共卫生宣传盛行, 村民甚至学会了唱《东方红》。这种医疗和文化宣传取得了成果。一位当地官员在参观完展览后对中国队说: “谢谢你们。与美国、英国和意大利的医生相比, 中国是最忠实的朋友。”另一位政府技术人员确信, “毛泽东思想”是能够给世界人民带来幸福的唯一保证。1971年索马里新年庆祝活动期间, 一群工人和村民高举毛泽东画像, 唱着《东方红》。唱歌跳舞之后, 他们还高呼: “我们感谢中国医生帮助我们”, “中国人给我们带来食物, 治愈我们的疾病”, “中国给我们带来生命。不要侵略中国”。⁸⁷ 医疗干预是否像宣传的那样成功值得怀疑, 但最终的效果是确立了中国作为改善人民健康的榜样。

在联合国, 中华人民共和国日益增多的医疗“人道主义”活动以及通过这些活动建立的友谊, 帮助中华人民共和国赢得了与中华民国(台湾)争夺安理会常任“中国”席位和大会成员资格的斗争。在1971年联合国大会上, 26个非洲国家投票赞成中华人民共和国在联合国的合法地位。⁸⁸与此同时, 中华人民共和国被视为所谓的“非洲问题”中的关键参与者: “中国可以在言辞上胜过苏联, 也可以资助莫斯科负担不起的行动”, 当时在美国联合国电视台记者理查德·霍特莱特这样认为。⁸⁹ 将中华人民共和国置于联合国, 人们认为这可以防止中国向非洲各民族解放运动提供军事援助, 因为中国支持东南亚的此类活动。相比之下, 台湾被认为在这件事上作用太小。⁹⁰

1973年, 随着中华人民共和国加入联合国, 中国被邀请重新加入世界卫生大会。⁹¹ 中国开始利用世界

Health Assembly meetings as the diplomatic arena to promote its new Third World policy and to combat the First World ‘superpowers’, particularly the USSR. For instance, in May 1974, a few days after the PRC’s vice prime minister Deng Xiaoping presented the Third World policy at the UN General Assembly, the model Barefoot Doctor, Wang Guizhen, was briefed by the new minister of health to use the Twenty-Seventh World Health Assembly meeting to defeat the PRC’s enemies by propagating Mao’s ‘revolutionary’ foreign policy, in addition to disseminating the ‘revolutionary’ approach to health preached by the PRC with the Barefoot Doctors and the integration of Chinese and Western medicines. On her return, Wang proudly reported that the meeting was a great triumph for the PRC and for Chairman Mao. According to her, many Third World countries, as well as the WHO officials, welcomed the PRC’s new Third World policy and praised the PRC’s cooperative health system and the Barefoot Doctor program for being truly grassroots, while criticizing Soviet medical training and its aid to the Third World as not applicable to these countries.⁹²

FROM ‘HEALTH FOR ALL’ TO THE DECLARATION OF ALMA ATA

The post-war scientific and technological revolution, as well as the collapse of the remaining colonial empires of Great Britain, France, and the Netherlands, also brought about major changes in the world, presaging what is now called globalization. This led to greater awareness of growing global disparities, especially in health care and access. The emergence of decolonized nations and the spread of national and anti-imperialist socialist movements in the less developed Third World countries (nowadays labelled the ‘Global South’), with more limited economic resources, also demanded a new health care delivery model. In the meantime, the International Health Division of the Rockefeller Foundation had more or less retired from the active global scene in direct health care interventions. The people’s health had begun to enter a new post-imperial stage, now labelled as the ‘Fourth Phase’ of public health. China’s rural health delivery system – the Barefoot Doctor program – which involved a massive training program and a major mobilization and organization of health services, linked with mass political campaigns, soon became a major inspiration for the burgeoning, worldwide Primary Health Care movement.

世界卫生大会是其推行新的第三世界政策并与第一世界 “超级大国”（特别是苏联）作斗争的外交舞台。例如，1974年5月，在中国国务院副总理邓小平在联合国大会上提出第三世界政策几天后，新任卫生部长向模范赤脚医生王桂珍汇报，要求她利用第27届世界卫生大会会议，通过宣传毛泽东的 “革命” 外交政策，并传播中国赤脚医生所宣扬的 “革命” 卫生方法以及中西医结合，来击败中国的敌人。回国后，王自豪地报告说，这次会议是中国和毛主席的伟大胜利。据她说，许多第三世界国家以及世卫组织官员都欢迎中国新的第三世界政策，并赞扬中国合作医疗制度和赤脚医生计划是真正扎根基层的，同时批评苏联的医疗培训及其对第三世界的援助不适用于这些国家。⁹²

从 “人人享有卫生保健” 到《阿拉木图宣言》

战后科学技术革命以及英国、法国和荷兰剩余殖民帝国的崩溃也给世界带来了重大变化，预示着现在所谓的全球化。这导致人们更加意识到全球差距日益扩大，尤其是在医疗保健和获取方面。非殖民化国家的出现以及不发达的第三世界国家（现在被称为 “全球南方”）中民族主义和反帝国主义社会主义运动的蔓延（这些国家经济资源有限）也需要一种新的医疗保健服务模式。与此同时，洛克菲勒基金会的国际卫生部门或多或少地退出了直接医疗保健干预的积极全球舞台。人民的健康开始进入后帝国主义的新阶段，现在被称为公共卫生的 “第四阶段”。中国的农村卫生服务体系——赤脚医生计划——涉及大规模的培训计划以及卫生服务的大规模动员和组织，并与大规模的政治运动相结合，很快成为蓬勃发展的全球基层卫生保健运动的主要灵感来源。

In 1967, John Bryant published *Health and the Developing World*, based on a collaborative project sponsored by the Rockefeller Foundation to assess health care delivery in developing countries. Bryant had been one of the foremost leaders of international public health over the previous three decades and a member of the Christian Medical Commission – a member of the World Council of Churches and the Lutheran World Federation, created in the late 1960s by medical missionaries, which advocated primary health care. While warning that ‘large numbers of the world’s people, perhaps more than half, have no access to health care at all’, Bryant argued against transplanting hospital-based health care from the West to the developing countries: ‘for many of the rest, the care they receive does not answer the problems they have ... the most serious health needs cannot be met by teams with spray guns and vaccinating syringes.’⁹³ Also around this time, medical missionaries working in developing countries, notably those of the Christian Medical Commission, were pushing for primary health care, with an emphasis on training grass-roots health workers and adequate but inexpensive care.⁹⁴

At the WHO headquarters in Geneva, the perceived failure of single-disease programs, also known as the vertical health approach, as promoted by the WHO and many US agencies in the 1950s and 1960s, provided a further context for moving away from hospital-centred health care and expert-based medicine toward primary health care and preventive medicine. In 1967, Kenneth W. Newell, an epidemiologist at Tulane University in New Orleans, was appointed by the WHO to lead the new Research in Epidemiology and Communications Science Division. Newell began his career as a medical officer at the Te Araroa Maori region in New Zealand where he had been actively involved in developing community health projects. The Ngati Porou tribe there sold their cattle to raise money to fund his further training in public health in the UK. For many years, Newell had been arguing for an integrated horizontal approach to control zoonotic diseases such as *salmonellae*.^{*} He stressed that the control measures for the developing countries where ‘there was lack of safe water supplies and methods of excreta disposal’ had to be

* In recent years, such an approach has been rebranded and modified as the One Health approach and adopted by a number of global institutions such as the World Bank. See World Bank, *People, Pathogens and Our Planet, Volume 1: Towards a One Health Approach for Controlling Zoonotic Diseases* (Washington: World Bank 2010).

1967年, 约翰·布莱恩特出版了《健康与发展中国家》, 该书基于洛克菲勒基金会赞助的一个合作项目, 旨在评估发展中国家的医疗保健服务。在过去的三十年中, 布莱恩特一直是国际公共卫生领域最重要的领导者之一, 也是基督教医疗委员会的成员——该委员会是世界基督教协进会和路德宗世界联盟的成员, 由医疗传教士在20世纪60年代后期创建, 倡导初级卫生保健。布莱恩特警告说, “世界上有大量的人口, 也许超过一半, 根本无法获得医疗保健”, 但他反对将西方的以医院为基础的医疗保健移植到发展中国家: “对于其余许多人来说, 他们得到的医疗保健并不能解决他们的问题……最严重的健康需求不能通过携带喷枪和疫苗注射器的团队来满足。”⁹³ 大约在同一时间, 在发展中国家工作的医疗传教士, 特别是基督教医疗委员会的传教士, 正在推动初级卫生保健, 重点是培训基层卫生工作者和提供充分但廉价的护理。

⁹⁴

在日内瓦的世界卫生组织总部, 人们认为单一疾病项目(也称为垂直卫生方法)的失败, 而这种方法是由世界卫生组织和许多美国机构在20世纪50年代和60年代推广的, 这为从以医院为中心的医疗保健和以专家为基础的医学转向初级卫生保健和预防医学提供了进一步的背景。1967年, 新奥尔良杜兰大学的流行病学家肯尼斯·W·纽厄尔被世界卫生组织任命领导新的流行病学和传播科学部。纽厄尔的职业生涯始于新西兰 Te Araroa 毛利地区的医疗官, 在那里他积极参与开发社区卫生项目。那里的 Ngati Porou 部落出售了他们的牛来筹集资金, 以资助他在英国接受公共卫生方面的进一步培训。多年来, 纽厄尔一直主张采用综合的横向方法来控制人畜共患疾病, 例如沙门氏菌。^{*} 他强调, 对于发展中国家来说, 控制措施必须是, 因为这些国家 “缺乏安全供水和排泄物处理方法”。

* 近年来, 这种方法经过重新包装和修改, 被称为 “同一健康” 方法, 并被世界银行等多个全球机构采用。参见世界银行, 《人民、病原体 and 我们的星球, 第一卷: 迈向控制人畜共患病的 “同一健康” 方法》(华盛顿: 世界银行, 2010年)。

different from those used in the developed countries.⁹⁵ After joining the WHO, Newell began to advocate for wider attention to primary health care and became interested in China's Barefoot Doctor program and in comparable developments in Cuba and Tanzania, which in the latter had been carried out with Chinese help. Consequently, he invited the Sidels to the WHO headquarters in Geneva to consult on a WHO and UNICEF joint project entitled Alternative Approaches to Meeting Basic Health Needs in Developing Countries. They were to contribute a case study on the health system in the PRC. The project was both organizations' attempt to 'develop fresh policies and approaches' to assist developing countries in improving people's health by 'examining successful or promising systems of delivery of primary health care'.⁹⁶ The project also led to a landmark publication, *Health by the People*, edited by Newell, which included a chapter on the PRC's health care delivery system contributed by the Sidels.⁹⁷ Among the nine countries studied, two communist countries, the PRC and Cuba, stood out as the 'really successful' examples of achieving 'better health care coverage, and improve levels of health' with limited resources. Drawing on their 'positive' experiences, the project identified that the government's 'political commitment', 'the integrated approach to health and development', 'mass mobilization', and 'community participation' were keys to building a successful primary health care system that would meet the basic health needs in the developing world. The project report recommended that 'successful' programs such as those in the PRC and Cuba should be 'fostered, extended, adapted and used as examples for a large-scale global program'.⁹⁸

To take the recommendation further, Halfdan Mahler, the director general of the WHO at the time and another driving force behind this joint project, proposed 'health for all citizens of the world'.⁹⁹ Before becoming assistant director general in the summer of 1970, Mahler had served as a WHO senior officer attached to the National Tuberculosis Program in India where he had been committed to causes for social justice. During his tenure as the assistant director general, Mahler worked closely with Newell to build the new Division of Strengthening of Health Services. 'Both men were visionaries who shared common values that included dedication to the pursuit of human development and justice,' recalled Socrates Litsios, the former senior scientist at the WHO's Division of the Control of Tropical Diseases.¹⁰⁰ Like Newell, Mahler responded enthusiastically to the widely cited example of the

与发达国家使用的不同。⁹⁵ 加入世卫组织后, 纽维尔开始倡导更多地关注基层医疗保健, 并对中国的赤脚医生项目以及古巴和坦桑尼亚的类似发展产生了兴趣, 后者是在中国的帮助下进行的。因此, 他邀请西德尔夫妇到日内瓦的世卫组织总部, 就世卫组织和联合国儿童基金会的一个联合项目 “发展中国家满足基本健康需求的替代方法” 进行咨询。他们将贡献一个关于中国卫生系统的案例研究。该项目是这两个组织试图 “制定新的政策和方法”, 通过 “审查成功或有希望的基层医疗保健提供系统” 来帮助发展中国家改善人民的健康。⁹⁶ 该项目还促成了一本具有里程碑意义的出版物, *Health by the People*, 由纽维尔编辑, 其中包括西德尔夫妇撰写的关于中国医疗保健提供系统的章节。⁹⁷ 在研究的九个国家中, 两个共产主义国家, 中国和古巴, 脱颖而出, 成为以有限的资源实现 “更好的医疗保健覆盖率, 并提高健康水平” 的 “真正成功” 的例子。该项目借鉴了他们 “积极” 的经验, 认为政府的 “政治承诺”、“健康与发展的综合方法”、“群众动员” 和 “社区参与” 是建立一个成功的基层医疗保健系统的关键, 该系统将满足发展中国家的基本健康需求。该项目报告建议, 像中国和古巴这样的 “成功” 项目应该 “培养、扩展、调整并用作大规模全球项目的榜样”。⁹⁸

为了进一步推进这项建议, 当时的世卫组织总干事哈夫丹·马勒 (Halfdan Mahler) 以及这一联合项目的另一推动力, 提出了 “人人享有健康” 的口号。⁹⁹ 在 1970 年夏天担任助理总干事之前, 马勒曾担任世卫组织高级官员, 隶属于印度国家结核病项目, 在那里他一直致力于社会正义事业。在他担任助理总干事期间, 马勒与纽维尔密切合作, 建立了新的加强卫生服务司。 “两人都是有远见的人, 他们拥有共同的价值观, 包括致力于追求人类发展和正义,” 世卫组织热带病控制司前高级科学家苏格拉底·利特西奥斯 (Socrates Litsios) 回忆说。¹⁰⁰ 像纽维尔一样, 马勒对被广泛引用的例子做出了积极回应, 即

PRC's successful experience in tackling health problems with limited financial, technological, and human resources and thought it should be promoted around the world. During his speech at the Twenty-Ninth World Health Assembly in May 1976, Mahler spoke of the need for radical change: 'Many social evolutions and revolutions have taken place because the social structures were crumbling. There are signs that the scientific and technical structures of public health are also crumbling ... we are on the threshold of a new era in community health ... it is our duty and our sacred privilege to shape this new era and to chart the ways that lead to it.'¹⁰¹

'Health for All' was formally adopted by the Twenty-Ninth World Health Assembly as its goal, to be achieved by the year 2000, and this became the rallying call for the worldwide Primary Health Care movement during the Long 1970s.

While major Western powers accepted the Mahlerian notion of Primary Health Care, the USSR, still very much in favour of centralized health care, opposed it, condemning it as a step backward from the advances in science and technology that had been made since the beginning of the Cold War. In addition, from their perspective, the notion of Primary Health Care was seen as a victory for the developing or 'Third World'. This would, in turn, undermine the USSR's claim to be the world leader in health care delivery and the people's health provider. This was impacted by, and became an intrinsic part of, the open and growing hostility between the PRC and the USSR since Khrushchev's denunciation of Stalin in 1956. Since the goal of 'Health for All' drew its inspiration heavily from the Chinese experience, particularly the Barefoot Doctor program, Moscow felt strongly that it could not 'permit Peking a victory within the Third World'.¹⁰²

After the PRC delegation to the WHO first proposed the idea of an international conference on Primary Health Care, the Soviet Union began to lobby hard to host the conference.¹⁰³ In January 1976, the day before the WHO Executive Board meeting, the Soviet Minister of Health Dimitri Venediktov turned up at David A. Tejada de Rivero's home (the assistant director general of the WHO at the time) in Geneva and made an offer: 'I will give you US\$2 million for an international conference on Primary Health Care.' Despite Tejada de Rivero's reluctance to accept the deal, at the Executive Board meeting Venediktov once again presented his proposal. It was only after Venediktov agreed that the conference would not be held in Moscow but instead in a

中华人民共和国以有限的财政、技术和人力资源成功解决健康问题的经验，并认为应该在世界范围内推广。在 1976 年 5 月第二十九届世界卫生大会上的讲话中，马勒谈到了彻底变革的必要性：“许多社会演变和革命之所以发生，是因为社会结构正在崩溃。有迹象表明，公共卫生的科学和技术结构也在崩溃……我们正处于社区卫生新时代的门槛……塑造这个新时代并规划通往新时代的道路是我们的责任和神圣的特权。”¹⁰¹

“人人享有卫生保健”被第二十九届世界卫生大会正式采纳为目标，目标是在 2000 年实现，这成为 20 世纪 70 年代漫长时期内全球初级卫生保健运动的号召。

虽然主要的西方大国接受了马勒提出的初级卫生保健概念，但苏联仍然非常支持中央集权式的医疗保健，并对此表示反对，谴责这是自冷战开始以来科学技术进步的倒退。此外，从他们的角度来看，初级卫生保健的概念被视为发展中或“第三世界”的胜利。这反过来会削弱苏联作为世界医疗保健提供者和人民健康提供者的领导地位的主张。自赫鲁晓夫于 1956 年谴责斯大林以来，中苏之间公开且日益增长的敌意影响了这一点，并成为其内在组成部分。由于“人人享有健康”的目标在很大程度上受到了中国经验的启发，特别是赤脚医生计划，莫斯科强烈认为它不能“允许北京在第三世界取得胜利”。¹⁰²

在中国代表团首次向世卫组织提出召开国际初级卫生保健会议的想法后，苏联开始大力游说以主办该会议。¹⁰³ 1976 年 1 月，在世卫组织执行委员会会议召开的前一天，苏联卫生部长德米特里·韦内季克托夫出现在日内瓦的大卫·A·特哈达·德·里韦罗（当时是世卫组织助理总干事）的家中，并提出了一个提议：“我将为召开一次关于初级卫生保健的国际会议提供 200 万美元。”尽管特哈达·德·里韦罗不愿接受这笔交易，但在执行委员会会议上，韦内季克托夫再次提出了他的建议。只有在韦内季克托夫同意会议不在莫斯科举行而是在一个

developing country that the Executive Board accepted his offer and the conference was scheduled for 1978.¹⁰⁴

The search for a suitable location in the ‘Third World’ was a difficult one. Since the USSR was sponsoring the conference, the PRC – the world’s putative leader in and certainly a major inspiration for the Primary Health Care movement – had to be excluded. Other countries favoured by the WHO were Costa Rica, Egypt, and Iran, but none could successfully secure the additional US \$1 million needed for the conference. After some extensive travel around the USSR, Venediktov and Tejada de Rivero agreed on Alma Ata (today’s Almaty), the capital of the Kazakh Soviet Socialist Republic (today’s Kazakhstan) as a suitable location for the conference. According to Tejada de Rivero, Alma Ata was chosen for two reasons: it had a very impressive conference hall and a very dynamic minister of health, Professor Turgeldy S. Sharmanov.¹⁰⁵ For the Soviets, however, Alma Ata was also significant because it was geographically proximate to China. Furthermore, it had become the USSR’s showcase of socialist achievement in providing health care for the ‘backward’ Kazakhs through a centralized, state-run health delivery system and bringing modern bio-medicine to rural villagers.¹⁰⁶ (It should also be noted that it was the site of the politically inspired famine of 1932–33, known today as the Goloshchekin genocide, which caused the deaths of approximately two million people.¹⁰⁷ While the Chinese famine was a recent public health horror, the Soviet famine had become a distant memory, occluded by the new ideal of a Kazakh health program.)

Back at the WHO headquarters in Geneva, Kenneth Newell was put in charge of drawing up documents for the Alma Ata conference. There were extensive conflicts over the concept of ‘Health for All’ as well as the wording of the official text of the declaration. According to Tejada de Rivero, with ‘Health for All’ Mahler ‘made it clear that he was referring to the need to provide a level of health that would enable all people without exception to live socially and economically productive lives ... It was a social and political goal, but above all a battle cry to incite people to action.’¹⁰⁸ Yet, as Mahler pointed out in a 2008 interview, ‘Primary Health Care will not succeed unless we can generate participation from individuals, families and communities, but this community participation will not work unless there is support from the health system.’¹⁰⁹ Not only the USSR and many WHO member states supported a centralized health system

发展中国家，执行委员会接受了他的提议，会议安排在 1978 年举行。¹⁰⁴

在“第三世界”寻找合适的地点非常困难。由于苏联是会议的赞助者，因此中华人民共和国——世界公认的初级卫生保健运动的领导者，并且无疑是其灵感来源——必须被排除在外。世界卫生组织青睐的其他国家包括哥斯达黎加、埃及和伊朗，但没有一个国家能够成功获得会议所需的额外 100 万美元。在苏联境内进行了一些广泛的旅行之后，韦尼迪克托夫和特哈达·德·里韦罗同意将阿拉木图（今天的阿拉木图）作为会议的合适地点，阿拉木图是哈萨克苏维埃社会主义共和国（今天的哈萨克斯坦）的首都。根据特哈达·德·里韦罗的说法，选择阿拉木图有两个原因：它有一个非常令人印象深刻的会议厅和一位非常有活力的卫生部长，图格尔德·S·沙尔马诺夫教授。¹⁰⁵ 然而，对于苏联人来说，阿拉木图也具有重要意义，因为它在地理位置上靠近中国。此外，它已成为苏联通过集中的、国家运营的卫生服务系统为“落后”的哈萨克人提供医疗保健并将现代生物医学带给农村村民的社会主义成就的展示。¹⁰⁶（还应该指出的是，这里是 1932 年至 1933 年政治性饥荒的发生地，今天被称为戈洛谢金种族灭绝，造成约 200 万人死亡。¹⁰⁷ 虽然中国饥荒是最近发生的公共卫生恐怖事件，但苏联饥荒已成为遥远的记忆，被哈萨克卫生计划的新理想所掩盖。）

回到日内瓦的世卫组织总部，肯尼斯·纽维尔负责起草阿拉木图会议的文件。关于“人人享有卫生保健”的概念以及宣言官方文本的措辞存在广泛冲突。根据特哈达·德·里韦罗的说法，马勒通过“人人享有卫生保健”“明确表示他指的是需要提供一种卫生水平，使所有人都能毫无例外地过上具有社会和经济生产力的生活……这是一个社会和政治目标，但首先是激励人们采取行动的口号”。¹⁰⁸ 然而，正如马勒在 2008 年的一次采访中指出的那样，“除非我们能够让个人、家庭和社区参与进来，否则基层卫生保健就不会成功，但如果没有卫生系统的支持，这种社区参与就不会奏效”。¹⁰⁹ 不仅苏联和许多世卫组织成员国支持集中式卫生系统

approach, but a number of delegates also 'fought to include details that had more to do with medical specialties than with health'.¹¹⁰ 'It wasn't easy,' as Mahler remembered it. 'There was a lot of fighting during the months of preparation and at the conference itself ... For example, to include "family planning" alongside "maternal and child health care" in the declaration virtually caused the whole thing to break down ... But the Secretariat was anxious about getting a consensus, which was vital. That did not mean trying to convince our adversaries they were wrong, but trying to unite ourselves with them at a higher level of insight. This was exactly what happened in Alma Ata. It was almost a spiritual atmosphere, not in the religious sense, but in the sense that people wanted to accomplish something great.'¹¹¹ The Chinese model came to be redefined by the debates but remained at their core.

Between 6 and 12 September 1978 at Alma Ata, 3,000 delegates from 134 member states, sixty-seven international organizations, and a dozen NGOs from around the world attended the world's first Primary Health Care conference. At the conference, the Final Declaration of Alma Ata was read aloud to all delegates. This declaration made it clear that health is 'a fundamental human right and that the attainment of the highest possible level of health is a most important worldwide social goal whose realization requires the action of many other social and economic sectors in addition to the health sector'.¹¹² 'Lots of people had tears in their eyes,' Mahler recalled fondly in an interview twenty years later. 'We never thought we would come that far. That was a sacred moment. The 1970s was a warm decade for social justice. That's why after Alma-Ata in 1978, everything seemed possible.'¹¹³ Yet the reality of the next decades was to be quite different from the emotional sentimentality of the declaration. 'Health for All' remained a distant promise for most of the globe. The end result of the popular acceptance of this 'mirage of health', as labelled by Rene Dubois, based on the Chinese model, was not only its failure in those nations attending the conference but also in the one conspicuous nation missing at the conference: the People's Republic of China.

Medicine and health always have a political dimension, and it is no wonder that it came to have an explicit one during the Cold War, especially in regard to the PRC and its relationship to the First as well as the Third World. Standing in opposition to the perceived inadequacies of American health care and the Soviet centralized health

的方法, 但许多代表也 “努力纳入更多与医学专业而非健康有关的细节”。¹¹⁰ 正如马勒回忆的那样, “这并不容易”。 “在准备的几个月和会议本身期间, 有很多争斗……例如, 在宣言中将 “计划生育” 与 “妇幼保健” 并列, 几乎导致整个事情崩溃……但秘书处渴望达成共识, 这至关重要。这并不意味着试图说服我们的对手他们是错的, 而是试图在更高的洞察力层面上将我们与他们团结起来。这正是在阿拉木图发生的事情。这几乎是一种精神氛围, 不是在宗教意义上, 而是在人们想要完成一些伟大的事情的意义。”¹¹¹ 中国模式通过辩论得到重新定义, 但仍然是辩论的核心。

1978年9月6日至12日, 在阿拉木图, 来自134个成员国、67个国际组织和来自世界各地的十几个非政府组织的3000名代表参加了世界首次初级卫生保健会议。在会议上, 《阿拉木图最终宣言》向所有代表宣读。该宣言明确指出, 健康是 “一项基本人权, 实现尽可能高的健康水平是一项最重要的全球社会目标, 其实现除了卫生部门外, 还需要许多其他社会和经济部门的行动”。¹¹² 马勒在二十年后的访谈中深情地回忆说: “很多人都热泪盈眶。我们从未想过我们会走到那一步。那是一个神圣的时刻。20世纪70年代是社会正义的温暖十年。这就是为什么在1978年阿拉木图会议之后, 一切似乎皆有可能。”¹¹³ 然而, 未来几十年的现实与宣言的情感化截然不同。 “人人享有健康” 对全球大多数地区来说仍然是一个遥远的承诺。正如勒内·杜博斯 (Rene Dubois) 所说, 这种基于中国模式的 “健康海市蜃楼” 被广泛接受的最终结果, 不仅是参加会议的那些国家的失败, 而且是唯一一个在会议上引人注目的缺席国家: 中华人民共和国。

医药卫生始终具有政治层面, 毫不奇怪, 在冷战期间, 它开始具有明确的政治层面, 尤其是在中华人民共和国及其与第一世界和第三世界的关系方面。它与人们认为的美国医疗保健的不足和苏联集中式医疗保健体系相对立。

care structure, the PRC held out the promise of a true alternative – one that proved as ephemeral as the success falsely attributed to the Barefoot Doctor program and the Anti-Schistosomiasis Campaign. The Long 1970s was the pivotal decade in which this model seemed at the verge of success, only to collapse into the morass of global neoliberal capitalism by the late twentieth century when health care became less and less an exportable model of cultural propaganda. The promise held out by the PRC health care model to deal with the people's health turned out to falter on a number of factors, primarily on the false perception of the coherence of the system itself and its radical focus on rural health in an age of increasing urbanization. Here, we can contrast these promises to the reality, no matter how partial and incomplete in the 1970s, of the access that improved the people's health through the National Health Service in Great Britain. Mortality and morbidity in Great Britain and the PRC radically decreased beginning in the 1950s.¹¹⁴ Yet the overwhelming initial decreases in the PRC were certainly countered by the mass deaths during the famine years, even though they rebounded thereafter.¹¹⁵

Foreign advocates of public health innovations in the PRC were, in the end, much less interested in health care as an end in itself than as a means of achieving social and political reform. To them, success in one dimension – the people's health – demonstrated success in the other: the political realm. The reality was that in attempting to define the improvement of the people's health as an indicator of the utopian success of the political system that furthered it, for good or for ill, they accepted its putative success without question. In the details were, of course, the problems that caused the system to fail or the inability to implement it. Ideas may well have the power to transform. Karl Marx did not think so, however: Marx firmly believed that the material basis of society was the source of all ideology. Ideas and visions may well have an influence on the social beyond being a means of justifying a material structure. But ideas are plastic: they can be twisted and misused easily simply because they do not have a material presence. The health policy of the PRC and its reception is a classic case of this dilemma. The desire for 'Health for All' remained after the death of Mao, as well as the Alma Ata Conference, a goal unequally sought and rarely accomplished across the globe.

关怀体系方面，中国曾展现出真正替代方案的前景——但事实证明，它如同被错误地归功于赤脚医生计划和血吸虫病防治运动的成功一样，转瞬即逝。20世纪70年代是关键十年，这个模式似乎一度濒临成功，但到了20世纪末，却最终崩溃陷入全球新自由主义资本主义的泥潭，此时的医疗保健越来越不像一种可输出的文化宣传模式。中国医疗模式在处理人民健康问题所展现的前景，最终由于多种因素而动摇，主要是由于对该体系本身连贯性的错误认知，以及在城市化日益加剧的时代，其对农村卫生的极端关注。在此，我们可以将这些承诺与英国国家医疗服务体系（National Health Service）在20世纪70年代所实现的改善人民健康的现实进行对比，无论这种改善多么不全面和不完整。自20世纪50年代开始，英国和中国的死亡率和发病率都大幅下降。¹¹⁴然而，中国最初的压倒性下降，无疑被饥荒年代的大规模死亡所抵消，尽管此后有所反弹。¹¹⁵

最终，在中华人民共和国倡导公共卫生创新的外国人士对医疗保健本身的兴趣远不如将其作为实现社会和政治改革的手段。对他们来说，一个维度（人民的健康）的成功证明了另一个维度（政治领域）的成功。实际情况是，他们试图将人民健康的改善定义为推动它的政治制度的乌托邦式成功的指标，无论好坏，他们都毫无疑问地接受了其所谓的成功。当然，细节中存在导致系统失败或无法实施的问题。想法很可能具有改变的力量。然而，卡尔·马克思并不这么认为：马克思坚信社会物质基础是所有意识形态的根源。思想和愿景可能对社会产生影响，而不仅仅是作为证明物质结构合理的手段。但思想具有可塑性：它们很容易被扭曲和滥用，仅仅因为它们没有物质存在。中华人民共和国的卫生政策及其接受情况就是这种困境的典型例子。在毛泽东逝世后，以及阿拉木图会议之后，“人人享有健康”的愿望仍然存在，这是一个在全球范围内不平等地追求且很少实现的目标。

Afterword

THE AFTERMATH OF ALMA ATA

The jubilation at Alma Ata that enthroned the People's Republic of China (PRC) as the model for improving the people's health most appropriate for the 'Third World' quickly dissipated. Despite the euphoria at the time, merely a year after the Declaration of Alma Ata, Kenneth Warren, who had been appointed director of health science at the Rockefeller Foundation in 1977, and his colleague Julia Wash proposed a radical alternative that vitiated the utopian promises made at Alma Ata. Their Selective Primary Health Care model introduced 'low-cost' but 'high-impact' targeted interventions as an 'interim' strategy for disease control in developing countries.¹ A number of UN agencies quickly adopted this selective approach, seeing it as less costly and simply more pragmatic than the more integrated approach advocated by Halfdan Mahler and Kenneth Newell. Even UNICEF moved away from its earlier commitment to the integrated and holistic model of Primary Health Care in favour of Warren's selective approach.

The background of this decision is important. Kenneth Warren was an expert in schistosomiasis research and grasped the complex reasons for the failure to curb the disease. According to him, it was an earlier 'transformative' experience of watching infants and children die by the roadside in Kenya that gave him the great impetus of using scientific mechanisms to alleviate the health problems in poor countries.² Sometimes described by those who knew him well as 'contemptuous' and 'dynamic', Warren was obsessed with health

后记

alma ata 的后果

阿拉木图会议将中华人民共和国（prc）奉为改善人民健康的最适合“第三世界”的典范，但欢欣鼓舞之情很快就消散了。尽管当时一片欢腾，但在《阿拉木图宣言》发表仅仅一年后，1977年被任命为洛克菲勒基金会健康科学主任的肯尼斯·沃伦和他的同事朱莉娅·沃什提出了一种激进的替代方案，该方案破坏了阿拉木图会议所作的乌托邦式承诺。他们的选择性初级卫生保健模式引入了“低成本”但“高影响力”的定向干预措施，作为发展中国家疾病控制的“临时”战略。¹许多联合国机构迅速采纳了这种选择性方法，认为它比哈尔夫丹·马勒和肯尼斯·纽厄尔提倡的更综合的方法成本更低，也更务实。甚至联合国儿童基金会也放弃了其早期对综合和整体初级卫生保健模式的承诺，转而支持沃伦的选择性方法。

这一决定的背景非常重要。Kenneth Warren 是血吸虫病研究方面的专家，他掌握了遏制该疾病失败的复杂原因。据他说，早年他在肯尼亚亲眼目睹婴儿和儿童死在路边的“变革性”经历，给了他巨大的动力，促使他利用科学机制来缓解贫困国家的健康问题。²认识他的人有时形容 Warren “蔑视一切”和“充满活力”，他痴迷于健康问题。

quality and information.*³ Under his leadership and financially backed by the Rockefeller Foundation, the Great Neglected Disease of Mankind Programme (GND) was established with the intention to create ‘a network of high-quality investigators who would constitute a critical mass in this field, attract brightest students, and conduct research of excellence’.⁴ No longer was the panacea of universal health care seen as possible.

This unexpected shift was a great disappointment to Mahler and Newell and their supporters: ‘That brought us right back to square one,’ Mahler lamented in a 2008 interview. ‘We had started with selective health-care programmes, single diseases such as malaria and tuberculosis in the 1950s and 1960s. Then we had this spiritual and intellectual awakening that came out of Alma-Ata, and suddenly some proponents of primary health care went back to the old selective approach again. Perhaps, paradoxically, Alma-Ata had in such instances the opposite effect to the one intended, as it made people think too much about selection, rather than following the Alma-Ata gospel of health for all.’⁵ Reflecting on why their ‘ambitious and worthy’ dream of ‘Health for All by 2000’ failed to realize, David Tejada de Rivero’s view was that the concept of ‘Health for All’ had often been misunderstood, confused with a simple concept of programming that is technical and bureaucratic rather than social and political, whereas ‘the conceptualization of “primary health care” was based on erroneous and biased perceptions of the experiences of Third World countries in providing health care with limited resources. In particular, the Chinese experience with “barefoot doctors” was interpreted simplistically and superficially.’⁶

If the PRC had been excluded from the Alma Ata conference as the result of Cold War politics, it was, however, by choice that at virtually the same moment that Warren proposed the Selective Primary Health Care model, the post-Mao government began to gradually abandon the Barefoot Doctor program. The program was officially ended in 1983 as the post-Mao PRC government under Deng Xiaoping’s leadership abandoned the Maoist collective economic system in favour of neoliberal market capitalism – labelled as ‘socialism with a Chinese face’. As the PRC government opted for a

* Interestingly, ‘information-sharing’, as advocated by Warren, is also the core part of the One Health approach, currently advocated by the WHO.

质量与信息。³ 在他的领导下，并在洛克菲勒基金会的资助下，成立了“人类被忽视的重大疾病计划”（gnd），其目的是创建一个“高质量的研究人员网络，他们将构成该领域的一个临界质量，吸引最聪明的学生，并进行卓越的研究”。⁴ 不再将普及医疗保健视为可能实现的灵丹妙药。

这种出乎意料的转变让马勒和纽厄尔以及他们的支持者感到非常失望：“这让我们回到了原点，”马勒在2008年的一次采访中感叹道。“我们从选择性的医疗保健项目开始，比如20世纪50年代和60年代的疟疾和肺结核等单一疾病。然后，我们在阿拉木图进行了一次精神和思想上的觉醒，突然一些基层医疗保健的支持者又回到了旧的选择性方法。也许，矛盾的是，阿拉木图在这种情况下产生了与预期相反的效果，因为它让人们过多地思考选择，而不是遵循阿拉木图的“人人享有健康”的福音。⁵ 大卫·特哈达·德·里维罗在反思他们“雄心勃勃且有价值”的“2000年人人享有健康”的梦想为何未能实现时，认为“人人享有健康”的概念经常被误解，与一个简单的、技术性和官僚而非社会性和政治性的规划概念混淆，而“‘基层医疗保健’的概念是基于对第三世界国家在有限资源下提供医疗保健的经验的错误和有偏见的看法。特别是，对中国“赤脚医生”的经验解读过于简单和肤浅。⁶

如果说由于冷战政治，中华人民共和国被排除在阿拉木图会议之外，那么，具有讽刺意味的是，几乎就在沃伦提出选择性初级卫生保健模式的同时，后毛泽东政府开始逐步放弃赤脚医生计划。该计划于1983年正式结束，当时在邓小平领导下的后毛泽东中华人民共和国政府放弃了毛主义集体经济体系，转而支持被贴上“具有中国特色的社会主义”标签的新自由主义市场资本主义。由于中华人民共和国政府选择了

* 有趣的是，沃伦所倡导的“信息共享”也是世界卫生组织目前倡导的“同一个健康”方法的核心组成部分。

Selective Primary Health Care model, it began to privatize providers. Barefoot Doctors were reconfigured as village doctors, and they opened private village clinics. To earn a living as well as to sustain their services, they charged the patients fees. Those who could not afford the fees were denied health care. Although the government tried to remedy this by introducing measures to regulate prices for services, drugs, and equipment, it resulted in many providers focusing their activities on high-margin services by investing in high-tech equipment and expensive drugs. This meant health care became even less affordable for many villagers as the providers moved their efforts away from low-margin activities such as day-to-day patient care. These villagers turned to relatively cheaper local indigenous remedies or to cheap antibiotics and other products with unknown side effects. In the 1990s, there were efforts by some local authorities to revive the Rural Co-operative Medical System (RCMS), but most were unsuccessful because the majority of villagers had lost their trust in the co-operative system. Some were simply too poor to make the RCMS economically feasible.⁷ While researching for this book, I also learned that after the government had abandoned the program, in one village not far from Hangzhou in Zhejiang province, the local villagers opted to keep the RCMS. This village had a small but profitable hardware factory, and the villagers collectively decided to use the profit it generated to set up a village health fund to finance the health care costs of those villagers who would need it. 'This had nothing to do with the government policy. It's what villagers wanted,' one villager told me.⁸ This case as well as the other examples I have given in this book suggest that, to a great extent, if the PRC's Rural Cooperative Health System was ever successful, it had more to do with local agency spurred on by the shared aspiration to improve the communities' health care that contributed to that success.

From the early 1990s, in a drive to modernize China once again, the PRC government embarked on an epic-scale urbanization project. Millions of Chinese farmers were uprooted and displaced into coastal cities where employment in the new quasi-capitalist industries provided better economic incentives than the rural communities could provide. However, the rigid, state Hukou system of household registration meant that they had little to no access to health care and other urban welfare provisions because a person's local identity

选择性初级卫生保健模式开始将服务提供商私有化。“赤脚医生”被重新配置为乡村医生，他们开设了私人乡村诊所。为了谋生和维持服务，他们向患者收取费用。那些负担不起费用的人被拒绝医疗保健。尽管政府试图通过出台措施来规范服务、药品和设备的价格来补救这种情况，但这导致许多服务提供商将活动重点放在高利润服务上，投资于高科技设备和昂贵的药品。这意味着，由于服务提供商将精力从日常患者护理等低利润活动中转移出来，许多村民更难以负担医疗保健费用。这些村民转而使用相对便宜的当地土方或廉价抗生素和其他副作用不明的产品。20 世纪 90 年代，一些地方当局曾努力恢复农村合作医疗制度（rcms），但大多数都未能成功，因为大多数村民已经失去了对合作制度的信任。有些人只是太穷，无法使农村合作医疗制度在经济上可行。⁷在为本书做研究时，我还了解到，在政府放弃该项目后，在浙江省杭州附近的一个村庄，当地村民选择保留农村合作医疗制度。这个村庄有一个小型但盈利的五金厂，村民们集体决定用它产生的利润建立一个村庄卫生基金，以资助那些需要医疗保健的村民的医疗费用。“这与政府政策无关。这是村民想要的，”一位村民告诉我。⁸本案例以及我在本书中给出的其他例子表明，在很大程度上，如果中国农村合作医疗制度曾经成功过，那更多的是因为当地机构受到改善社区医疗保健的共同愿望的刺激，从而促成了这一成功。

从 20 世纪 90 年代初开始，为了再次实现中国的现代化，中国政府启动了一项史诗般的城市化项目。数百万中国农民被连根拔起，迁移到沿海城市，在那里，新的准资本主义工业提供的就业机会比农村社区提供的经济激励更好。然而，僵化的国家户口登记制度意味着他们几乎无法获得医疗保健和其他城市福利，因为一个人的本地身份

defined their access to the faltering state system of health care.* You had to be treated where you were officially registered – a fact that excluded millions.

The new Selective Primary Health Care model – but even more importantly the large-scale urbanization – also impacted the control of schistosomiasis in the PRC. In 1981, two years after Warren proposed Selective Primary Health Care as ‘an interim strategy for disease control in developing countries’, he led an international team to China to conduct a joint study on schistosomiasis mortality in rural Anhui province, a marshy region along the Yangtze River. His resulting evaluation discredited the earlier Chinese strategy of eradication as not only too costly and ineffective but also harmful to the people’s health.⁹ Although Su Delong, Warren’s Chinese co-investigator, argued that to effectively control the disease it was still necessary to eliminate the snails, the Ministry of Health, which had had its power reinstated to manage disease control in 1986, agreed with Warren’s evaluation that using newer single-dose, oral, nontoxic chemotherapeutic agents in treating the disease was more cost-effective than trying to eliminate snails.¹⁰ This also coincided with, in 1984, the World Health Organization (WHO) expert consultation committee changing its strategy and objective from controlling snails and the environment to reducing morbidity and mortality by focusing on chemotherapy and health education.¹¹ In subsequent years, the earlier integrated eradication strategy was abandoned in the PRC, and the morbidity control strategy intended to reduce the prevalence and intensities among local populations and livestock was implemented in the lake regions along the Yangtze.¹²

Between 1992 and 2001, with a loan from the World Bank, the chemotherapeutic intervention favoured by Warren was introduced in the PRC as the key to reducing the rate of human infection and the intensity of schistosomiasis. An official statistic suggests that as a consequence of this, the number and morbidity of cases of the disease were reduced by half in endemic regions.¹³ In the next decade, the PRC government published the Schistosomiasis Prevention and

* Hukou is a comprehensive family registration system to control population mobility first introduced in 1958. This system discourages migration by limiting individuals’ eligibility for regular urban welfare benefits or social services (e.g., urban pension plans, public housing, and medical and health care) – a means of preventing the rural population from moving into cities.

决定了他们获得日渐衰落的国家医疗保健体系的途径。* 你必须在你正式注册的地方接受治疗——这一事实排除了数百万人。

新的选择性初级卫生保健模式——但更重要的是大规模的城市化——也影响了中国血吸虫病的控制。1981年，在沃伦提出选择性初级卫生保健作为“发展中国家疾病控制的临时战略”两年后，他率领一个国际团队前往中国，对安徽省农村地区（长江沿岸的沼泽地区）的血吸虫病死亡率进行联合研究。他的评估结果否定了中国早期的根除策略，认为它不仅成本过高、效果不佳，而且对人民的健康有害。⁹ 尽管沃伦的中国合作研究员苏德隆认为，为了有效控制该疾病，仍然有必要消灭钉螺，但1986年恢复权力管理疾病控制的卫生部同意沃伦的评估，即使用更新的单剂量、口服、无毒的化学治疗药物来治疗该疾病比试图消灭钉螺更具成本效益。¹⁰ 这与1984年世界卫生组织（WHO）专家咨询委员会改变其战略和目标，从控制钉螺和环境转变为通过关注化疗和健康教育来降低发病率和死亡率也相吻合。¹¹ 在随后的几年里，中国放弃了早期的综合根除战略，并实施了旨在降低长江沿岸湖泊地区当地居民和牲畜的患病率和强度的疾病控制战略。¹²

1992年至2001年间，在世界银行的贷款支持下，沃伦倡导的化学疗法干预措施被引入中国，作为降低人类感染率和血吸虫病强度的关键。一项官方统计数据显示，由于这项措施，地方病流行地区的病例数量和发病率减少了一半。¹³ 在接下来的十年里，中国政府发布了《血吸虫病防治》

* 户口是一种综合性的家庭登记制度，旨在控制人口流动，于1958年首次推出。该系统通过限制个人获得常规城市福利或社会服务（如城市养老金计划、公共住房以及医疗保健）的资格来阻止移民——这是一种阻止农村人口迁入城市的方式。

Control Regulation, which resulted in the implementation of a more comprehensive strategy for disease control. Such controls ranged from the construction of lavatories with running water to the institution of drug treatment and health education.¹⁴ The introduction of this new national schistosomiasis control program also coincided with the modernization of agriculture, with the reduction in the number of agricultural workers through mechanization and with the simultaneous urbanization and the moving of displaced rural workers into the ever-expanding industrial centres that has occurred since the end of the twentieth century. With the coterminous shift of agricultural land being contracted to individual households, a greater number of villagers also switched from growing labour- and water-intensive crops such as rice to cash crops like tea, medicinal herbs, fruits, or vegetables. While a combination of sanitary interventions and Praziquantel drug therapy have undoubtedly contributed to the decrease in the number of infected people in the endemic regions, the shifts in the means and modes of agricultural production had a much greater effect.¹⁵ These shifts in agricultural practice radically decreased the proportion of people and farm animals engaged in agricultural activities as well as the acreage taken up by rice paddies. The farmers' working hours were also shortened, and the resulting overall improvements in the standard of living were at the core of the upgrading of the people's health.

The depopulation of rural regions and the mass abandonment of agriculture also radically reduced the incidence and prevalence of what were actually primarily rural diseases, such as schistosomiasis. By 2015, official published statistics showed that all endemic regions had achieved control of the infection, with the number of infected people falling by 91 per cent.¹⁶ Just as malaria disappeared from the East Anglia Fens in England in the nineteenth century after the local population moved away, in many regions of China where schistosomiasis had been endemic the greatest challenge for those in charge of local treatment today is finding sufficient patients, since virtually all male labourers have migrated to the coastal cities for their livelihood.¹⁷ The occasional acute outbreaks are mostly among rural children who become infected after playing in contaminated water.¹⁸ However, many infected villagers from endemic regions of rural Anhui, Hunan, and Hubei moved to coastal cities seeking employment in factories or work on construction sites, and as a result there was an increased danger of acute outbreaks of the disease in these

控制条例, 从而实施了更全面的疾病控制策略。这些控制措施包括建造带自来水的厕所, 以及实施药物治疗和健康教育。¹⁴ 这项新的全国血吸虫病控制计划的推出, 也恰逢农业现代化, 通过机械化减少了农业工人数量, 同时伴随着城市化以及自 20 世纪末以来流离失所的农村工人涌入不断扩张的工业中心。随着农业用地同时转移到个体家庭, 更多的村民也从种植劳动和用水密集型作物 (如水稻) 转变为种植茶叶、药材、水果或蔬菜等经济作物。虽然卫生干预措施和吡喹酮药物治疗无疑对减少地方病流行地区感染人数做出了贡献, 但农业生产方式的转变产生了更大的影响。¹⁵ 农业实践的这些转变大大减少了从事农业活动的人口和牲畜比例, 以及稻田所占的面积。农民的工作时间也缩短了, 由此带来的生活水平的全面提高是人民健康改善的核心。

农村地区的人口减少和大规模放弃农业也从根本上降低了血吸虫病等主要发生在农村地区的疾病的发病率和流行率。到 2015 年, 官方发布的统计数据显示, 所有流行区域都已控制了感染, 感染人数下降了 91%。¹⁶ 正如 19 世纪疟疾在英格兰东安格利亚芬区随着当地居民的迁离而消失一样, 在中国血吸虫病流行的许多地区, 当地治疗负责人今天面临的最大挑战是找到足够的病人, 因为几乎所有的男性劳动力都已迁移到沿海城市谋生。¹⁷ 偶尔发生的急性爆发主要发生在受污染水中玩耍而感染的农村儿童中。¹⁸ 然而, 来自安徽、湖南和湖北农村流行地区的许多受感染村民搬到沿海城市, 在工厂或建筑工地寻求就业, 因此, 在这些地区爆发急性疾病的危险性增加

urbanized areas where the infection had previously disappeared precisely because of urbanization and the consequent draining of fields and swamps.¹⁹ Many of these displaced rural workers lived in crowded accommodation with appalling sanitary conditions. If the water supply became contaminated, there was a real danger of schistosomiasis quickly becoming endemic again in these urbanized areas, especially as an increasing number of snails were now found in many of the reclaimed wetlands in the lower reaches of the Yangtze River near the coast because of the rise of water levels and the absence of a budget as well as cheap labour for snail control.²⁰ Adding to the existing complications has been the increased Sino-African cooperation in recent years. There have been reports of Chinese contract workers, as well as travellers, returning to the PRC from African countries carrying variant forms of schistosomiasis after becoming infected in their host countries.²¹ At the same time, after the PRC adopted the Selective Primary Health Model and in the wake of the SARS pandemic in 2003, a number of local schistosomiasis control institutions were simply dissolved and restructured into local centres for disease control.²² Just as antibiotic overuse has led to the reappearance of a wide range of infectious diseases long since considered under control, so, too, have patterns of diseases such as schistosomiasis continued to shift – from its spread by the movement of Communist troops immediately after World War II to the displacement of rural workers into the cities, as well as the PRC's economic and political expansion into Africa half a century later.

With radical system change and concomitant social upheaval, insecurity prevailed, and the basic trust necessary for health care was at risk. Local trust had vanished with increased urbanization and the dispersal of ever-greater numbers of people who were now crowded into what have become massive urban slums with increased rates of infectious disease and malnutrition. Such was the unintended consequence of urbanization as well as the utopian projects to modernize China. Urban health care, relying on centralized medical expertise, never earned the trust of the new migrant patients. Indeed, the urban system even lost whatever trust their original urban patients had had in it as it became overloaded, inefficient, expensive, and chaotic. Violence directed against doctors, euphemistically called 'medical disturbances' (医闹), has become a regular feature of urban health care.²³ Distrust of the state's primary care system has resulted in increasing rates of self-medication, which relies on cheap counterfeit drugs

城市化地区，这些地区的感染 *панее* 已经消失，正是因为城市化以及随之而来的田地和沼泽排水。¹⁹ 许多流离失所的农村工人都住在拥挤的住所，卫生条件恶劣。如果供水受到污染，血吸虫病确实有可能在这些城市化地区迅速再次成为地方病，特别是由于现在在长江下游沿海附近许多开垦的湿地中发现了越来越多的蜗牛，这是由于水位上升以及缺乏预算和廉价劳动力来控制蜗牛。²⁰ 近年来，中非合作的增加使现有情况更加复杂。据报道，中国合同工以及旅行者从非洲国家返回中国，携带了变异形式的血吸虫病，他们在各自所在国感染了这种疾病。²¹ 与此同时，在中国采用选择性初级卫生模式以及 2003 年非典大流行之后，许多地方血吸虫病控制机构被简单地解散并重组为地方疾病控制中心。²² 正如抗生素的过度使用导致了早已被认为得到控制的各种传染病重新出现一样，血吸虫病等疾病的模式也在不断变化——从二战结束后共产主义军队的流动传播到农村工人流离失所到城市，以及半个世纪后中国的经济和政治扩张到非洲。

随着激进的系统变革和随之而来的社会剧变，不安全感普遍存在，而医疗保健所需的基本信任也面临风险。随着城市化进程的加快，以及越来越多的人分散开来，他们现在拥挤在已经成为大规模城市贫民窟的地方，传染病和营养不良的发生率也在增加，当地的信任感已经消失。这就是城市化以及中国现代化乌托邦项目无意造成的后果。依靠集中医疗专业知识的城市医疗保健，从未赢得新移民患者的信任。事实上，随着城市系统的超负荷运转、效率低下、费用昂贵和混乱，它甚至失去了原有城市患者对它的信任。针对医生的暴力行为，委婉地称为“医闹”，已成为城市医疗保健的常见现象。²³ 对国家初级保健系统的不信任导致了自我用药率的上升，而自我用药依赖于廉价的假冒药品

smuggled in from India or on out-of-date clandestine remedies.²⁴ In 2018, Wen Muye's film *Dying to Survive* was the top box office success in Chinese cinema that year. The film tells the true story of how the leukaemia patient Lu Yong became a folk hero when he imported unapproved anti-cancer drugs from India to supply fellow patients in China suffering from chronic myeloid leukaemia (CML) who could not afford the cost of the medication on the domestic market.²⁵ At the same time, allopathic medicine is undermined by corruption at all levels, from the pharmaceutical industry to the hospitals. Folk remedies have also been rendered unsafe because practitioners employ equally questionable remedies with little or no training in their application or are driven by the profit motive. Repeated unsafe vaccine scandals have further undermined people's trust of the state: 'If a government can't deliver safe vaccines for children, is it fit to rule?' one asks.²⁶ People's anger has prompted the government to again attempt to alter the health care system by tinkering at its edges. The sense on the ground is of chaos in the health care system; the public response is wide-scale distrust and anxiety.

The Mao era has long since passed. China is now the world's second largest economy. The previously dirty and diseased countryside, 'the Sick man of Asia', has given way to megapolises with hyper-modern facilities. Yet China is long way from being the disease-free 'socialist garden' imagined in Mao's utopian plans. The cities create continuously greater health risks; air pollution and flu pandemics have become the largest threats to people's health in urban areas. In the meantime, health disparity continues and expands across the board. Although the divide imagined by Mao in the 1960s between the urban and rural populations has vanished, health disparity is everywhere.

THE LEGACY OF THE PEOPLE'S HEALTH IN THE PRC

By the end of the twentieth century, the trajectory of the wide range of undertakings to improve the people's health in the PRC had taken a markedly downward turn. What had begun after the successful seizure of power by the Communists in 1949 in linking health care to their revolutionary agenda ended precipitously after Mao's death in the late 1970s. As I have shown, social upheaval, political change, and health care were, and are, intimately linked in the PRC. Health care delivery, public health interventions, and the changes in medical

从印度走私进来的，或已过期的秘密疗法。²⁴ 2018年，文牧野的电影《我不是药神》成为当年中国电影票房最高的电影。该片讲述了白血病患者陆勇成为民间英雄的真实故事，他从印度进口未经批准的抗癌药物，以供应中国患有慢性粒细胞白血病（cml）的同胞，他们无力承担国内市场上的药物费用。²⁵ 与此同时，对抗疗法的医学因各层面的腐败而受到损害，从制药业到医院。民间疗法也变得不安全，因为从业者采用同样可疑的疗法，但对其应用几乎没有或根本没有培训，或者受到逐利动机的驱使。反复发生的不安全疫苗丑闻进一步削弱了人们对国家的信任：有人问道：“如果政府不能为儿童提供安全疫苗，它还适合统治吗？”²⁶ 人们的愤怒促使政府再次尝试通过修补边缘来改变医疗保健系统。人们普遍感觉医疗保健系统一片混乱；公众的回应是大规模的不信任和焦虑。

毛泽东时代早已过去。中国现在是世界第二大经济体。以前肮脏和充满疾病的乡村，即“亚洲病夫”，已经让位于拥有超现代化设施的大都市。然而，中国离毛泽东乌托邦计划中想象的无疾病“社会主义花园”还差得很远。城市不断产生更大的健康风险；空气污染和流感大流行已成为城市地区人们健康的最大威胁。与此同时，健康差距持续存在并在各个方面扩大。尽管毛泽东在20世纪60年代设想的城乡人口之间的差距已经消失，但健康差距无处不在。

THE LEGACY OF THE PEOPLE'S HEALTH IN THE PRC

到二十世纪末，中华人民共和国为改善人民健康而进行的各种努力的轨迹发生了明显 downward 转。在共产党1949年成功夺取政权后，将医疗保健与他们的革命议程联系起来的做法，在20世纪70年代末毛泽东去世后戛然而止。正如我已经表明的那样，社会动荡、政治变革和医疗保健在中华人民共和国中密切相关。医疗保健服务、公共卫生干预以及医疗方面的变化

practice occasioned by shifts in political needs at the centre and on the periphery have changed health care in the PRC, sometimes for the better but often, because of the unintended and secondary effects of other state policies, for the worse. The legacy of the ‘Chinese approach’ to improving people’s health is thus as complex as all of the other tales of improving health care. While many of the indicators of the people’s health, such as the gross statistics of morbidity and mortality, tended to improve over time, the way individuals experienced such changes was often radically different from the official claims. The experience of those engaged in these processes, both the experts from above and the grassroots cadres, the health workers and villagers from below, as presented in this book, points to the importance of undertaking careful examination of the many complexities involved in the implementation of large-scale public health initiatives. China serves as a good example of the complexity of creating a monolithic health system for a vast and varied country.

By moving away from a narrow focus on policy formulation and the published official accounts, and by examining the reality of health care and medicine at the local level, this book has provided a fuller and more nuanced account of how and whether the PRC’s internationally acclaimed mass public health undertakings – the Anti-Schistosomiasis Campaigns and the Barefoot Doctor program – worked on the ground and the many difficulties and contradictions that emerged as they were implemented over time and in various geographic and cultural locations. By contrasting the views of those impacted, either as patients or as experts and health cadres, with the officially stated goals to improve the people’s health, a better sense can be achieved of the accuracy of the claims of these programs, as well as the attitudes toward them, for the first time. By bringing those directly impacted by the campaign into my account of this complex history and seeing them as active participants in the state public health campaigns, it can be seen how great designs on paper often turned into makeshift solutions, with little resemblance to the original project, as soon as they encountered human reality.

What is vital in this account is that the progress of both campaigns varied from province to province, from location to location, and over time. They depended less on the political vision and the policy from above than on a whole range of local factors: from the precise nature of the relationship between agencies at various levels of local authority to the vagaries of the local economy, the

由于中央和地方政治需求的转变而引起的实践，改变了中国医疗保健，有时会更好，但通常由于其他国家政策的意外和次要影响而变得更糟。“中国式”改善人民健康遗产的复杂性，与其他改善医疗保健的故事一样。虽然许多人民健康指标（如发病率和死亡率的总体统计数据）随着时间的推移而趋于改善，但个人体验这些变化的方式通常与官方说法截然不同。正如本书所介绍的，参与这些过程的人们的经验，包括来自上级的专家和来自基层的干部、卫生工作者和村民，都表明必须认真审查大规模公共卫生举措实施过程中涉及的诸多复杂性。中国很好地例证了为一个广阔而多样的国家创建一个统一的卫生系统的复杂性。

本书通过摆脱对政策制定和已发布的官方报告的狭隘关注，并考察地方层面医疗保健的现实情况，对中国在国际上广受赞誉的大众公共卫生事业——血吸虫病防治运动和赤脚医生计划——在实践中如何运作，以及在不同时间和不同地理文化背景下实施过程中出现的诸多困难和矛盾，提供了一个更全面、更细致的描述。通过将作为患者或专家和卫生干部的受影响者的观点与改善人民健康的官方目标进行对比，可以首次更好地了解这些项目的主张的准确性以及对这些项目的态度。通过将受运动直接影响的人纳入我对这段复杂历史的叙述中，并将他们视为国家公共卫生运动的积极参与者，我们可以看到，纸上谈兵的伟大设计一旦遭遇现实，往往会变成权宜之计，与最初的项目几乎没有相似之处。

在这个叙述中，至关重要的是，这两项运动的进展因省而异，因地而异，并且随着时间的推移而变化。它们较少依赖于自上而下的政治愿景和政策，而是更多地依赖于一系列地方因素：从各级政府机构之间关系的精确性质到当地经济的变幻莫测。

range of resources, and the cultural sensibilities of both villagers and grassroots health workers, as well as other more 'practical' considerations. To overcome these obstacles, the frontline workers or the CCP cadres managing the campaigns on the ground understood the need to be persuasive rather than authoritarian in their approach. This was reflected in the efficacy of the public health propaganda as well as in local organizational work. It was not national norms but local conditions that defined success.

Of equal importance to the vagaries of local experience is the simple fact that health planners at the centre never understood how communities at the periphery perceived their health needs. The state's attempt to improve the people's health with modern allopathic medicine and systemized Traditional Chinese Medicine (TCM), by stamping out existing popular healing practices as 'feudal practices', was regularly contested on the ground. A wide range of vernacular healing rituals, from witchcraft to exorcism as well as other forms of collective, everyday healing rituals and self-help strategies, continued unabated under socialism.

As the promise to improve the people's health had been, from the beginning of the new state, a central tool utilized by the CCP in establishing political legitimacy, the state's failure to deliver it nearly cost the CCP its legitimacy. When bureaucracy hindered Mao and the CCP from exercising power from the centre, slogans such as 'To Serve the People' and 'Revolutionary Humanitarianism' were evoked as a means of papering over the difficulties on the ground and of easing tensions between the people and the party. Villains who thwarted the party's (and simultaneously the people's) desires – in this case the 'revisionists' and 'bourgeois' experts – were sought out, identified, and overcome, no matter their culpability. While Mao's political vision and the leadership's ability to mobilize the masses may have contributed to an improved health care system and ultimately to overall better health, political factors had unintended yet truly negative consequences for the people's health. The political instability of the Mao era often meant that the public health policies and campaigns could not be sustained or carried out over time, and the result was radical fluctuations in the trajectory of any overall change.

Thus, the utopian goal of turning the Chinese countryside into a 'disease free socialist garden' through health improvements also had unplanned and lasting negative consequences on the environment and hence on the people's health. The application of molluscicides,

一系列资源, 以及村民和基层卫生工作者的文化敏感性, 以及其他更“实际”的考虑因素。为了克服这些障碍, 在前线工作人员或负责管理基层运动的中共干部理解到, 他们的做法需要具有说服力, 而不是独断专行。这体现在公共卫生宣传的有效性以及当地的组织工作中。决定成功的不是国家规范, 而是当地情况。

与当地经验的多变性同等重要的是一个简单的事实: 中央的卫生规划人员从未理解外围社区如何看待他们的健康需求。国家试图通过现代对抗疗法和系统化的中医药 (tcm) 来改善人民的健康, 并通过取缔现有的流行疗法作为“封建习俗”, 这在基层经常受到质疑。从巫术到驱魔, 以及其他形式的集体、日常治疗仪式和自助策略等各种本土治疗仪式在社会主义制度下继续有增无减。

由于改善人民健康的承诺从新中国成立之初就一直是中共建立政治合法性的核心工具, 因此, 国家未能兑现这一承诺几乎使中共丧失了合法性。当官僚机构阻碍了毛泽东和中共从中央行使权力时, “为人民服务”和“革命人道主义”等口号被用来掩盖实际困难, 缓和人民与党之间的紧张关系。阻挠党的 (同时也阻挠人民的) 愿望的恶棍——在本例中是“修正主义者”和“资产阶级”专家——无论其罪责如何, 都会被找出、识别和克服。虽然毛泽东的政治愿景和领导层动员群众的能力可能有助于改善医疗保健系统, 并最终改善整体健康状况, 但政治因素对人民的健康产生了意想不到但却真正负面的影响。毛泽东时代的政治不稳定常常意味着公共卫生政策和运动无法长期维持或开展, 结果是任何整体变革的轨迹都会出现剧烈波动。

因此, 通过改善健康状况将中国农村变成“无疾病的社会主义花园”的乌托邦式目标, 也对环境以及人民的健康产生了计划外且持久的负面影响。杀软体动物剂的应用,

as well as the use of radical engineering interventions such as land reclamation, to kill snails damaged the natural ecosystem. Such physical changes in the environment also conflicted with the system for water conservation, contributing to the severe flooding that is still haunting the population who live in the impacted regions along the Yangtze River and its flood plain. At the same time, human exploitation of the environment, as part of the agricultural expansion that was aimed at ending rural poverty and conquering diseases such as schistosomiasis, exposed the greater population to the disease and increased the number and geographic distribution of those becoming ill from it.

One further and important lesson that can be extrapolated from the Chinese experiences in dealing with the people's health during the first three decades of the PRC is that utopian solutions linking health to absolute political goals are bound to frustrate and disappoint those most directly affected. The reality of change, if not improvement, in people's health relies on multiple variables, which often shift from moment to moment in a kaleidoscope of causations and intended and unintended consequences. Utopian solutions, from the left and from the right, focus on specific relationships and causes – whether macroeconomic, 'racial', caste, geographic, or cultural – that are always shifting and indeed which are often invented and reinvented for purposes that have little or nothing to do with people's health. Who would have imagined that the radical success in combatting infectious diseases, beginning (at least in the West) with Edward Jenner and smallpox, continuing through the first age of biological thinking with Louis Pasteur (rabies) and Robert Koch (cholera), would have foundered in the twenty-first century, not because of war and poverty but on the economic success of a middle class oblivious to the horrors of infectious diseases and exposed to anti-vaccination claims through the new technology of the worldwide web?²⁷ Or that the promises of eradication of the vectors of other diseases – of snails, of mosquitos, of rats – would be shattered because of the vagaries of changes as radically different as cheap air travel and climate change? And that the powerful 'wonder cure' of antibiotics would lead to equally powerful drug resistance? If Mao's claims linking the people's health to the politics of his state were simultaneously supported and undermined by changes he attributed to the politics of the day, how much more can we claim that his utopian goals destroyed the lives of many more than it saved? In a speech

激进的工程干预措施（如土地开垦）以及为杀死蜗牛而采取的措施破坏了自然生态系统。环境的这种物理变化也与水利系统相冲突，导致严重的洪水灾害，至今仍困扰着居住在长江及其洪泛区沿岸受影响地区的人们。与此同时，作为旨在消除农村贫困和征服血吸虫病等疾病的农业扩张的一部分，人类对环境的开发使更多的人口暴露于该疾病中，并增加了因此患病的人数和地理分布。

从中国在前三十年处理人民健康的经验中可以推断出的另一个重要教训是，将健康与绝对政治目标联系起来的乌托邦式解决方案注定会让那些受到最直接影响的人感到沮丧和失望。人们健康的改变，即使不是改善，也依赖于多个变量，这些变量常常在一系列因果关系以及有意的和无意的后果中不断变化。来自左派和右派的乌托邦式解决方案都关注特定的关系和原因——无论是宏观经济、‘种族’、种姓、地理还是文化——这些关系和原因总是在变化，而且实际上常常是为了与人民健康几乎或根本无关的目的而发明和重新发明的。谁会想到，从爱德华·詹纳和天花开始（至少在西方），并经由路易·巴斯德（狂犬病）和罗伯特·科赫（霍乱）的生物学思维的第一个时代继续发展起来的在抗击传染病方面的巨大成功，会在 21 世纪因中产阶级的经济成功而失败，他们对传染病的恐怖视而不见，并通过万维网的新技术接触到反疫苗接种的主张？²⁷ 或者，消灭其他疾病的媒介——蜗牛、蚊子、老鼠——的承诺会因为廉价的航空旅行和气候变化等截然不同的变化的反复无常而破灭？而强大的“神奇疗法”抗生素会导致同样强大的耐药性？如果毛泽东将人民的健康与他的国家的政治联系起来的主张同时受到他归因于当时政治的变化的支持和破坏，那么我们更可以声称，他的乌托邦式的目标摧毁的生命比拯救的生命还要多吗？在一次演讲中

to the Second Session of the Eighth Communist Party Congress on 8 May 1958, Mao announced that the PRC would make the superhuman breakthrough to achieve the Great Leap Forward by ‘mak[ing] the high mountain bow its head; mak[ing] the river yield the way’. This goal ended with the worst famine in human history, claiming tens of millions of lives. Such destructive and often unintended consequences to people’s health of such utopian goals remain today. The crisis of health care in the PRC today as seen in the response to the COVID-19 virus is, to a great extent, the result of this complex and contradictory history of the Maoist model to improve people’s health. Through a nuanced understanding of different health systems’ components, the contexts in which these systems exist, and the sequencing of actions, it can be seen how the claims to design better interventions to strengthen the health system, to increase health care coverage, and ultimately to improve people’s health can have long-term consequences, intentional or unintended.

1958年5月8日，在中共八届二次会议上，毛泽东宣布中华人民共和国将通过“叫高山低头，叫河水让路”的“超人”突破来实现大跃进。这一目标最终导致了人类历史上最严重的饥荒，夺走了数千万人的生命。这种具有破坏性的，且往往是无意的后果，即这种乌托邦式的目标对人民健康的影响，至今仍然存在。今天，在中国对新冠病毒的反应中所看到的医疗保健危机，很大程度上是毛泽东模式改善人民健康的复杂和矛盾历史的结果。通过对不同卫生系统组成部分、这些系统存在的背景以及行动顺序的细致理解，可以看出，设计更好的干预措施以加强卫生系统、扩大医疗保健覆盖面并最终改善人民健康的说法，可能产生长期的后果，无论是故意的还是无意的。

Notes

LIST OF ARCHIVES

Archives in China

BEIJING – Beijing Municipal Archive:

Beijing Municipal Bureau of Health and Hygiene (135)

GANSU – Gansu Provincial Archive:

Gansu Provincial Party Committee (91)

GUIZHOU – Chishui County Archive:

Chishui County Party Committee (1-A)

HUNAN – Hunan Provincial Archive:

Hunan Provincial Party Committee (140)

Hunan Province Bureau of Health and Hygiene (163)

HUNAN – Yueyang City Archive:

Yueyang Region Revolutionary Committee (100)

JIANGSU – Kunshan City Archive (Kunshan)

JIANGXI – Jiangxi Provincial Archive: Jiangxi Provincial Health Bureau
(X111)

SHANGHAI – Shanghai Municipal Archive:

Shanghai Municipal Party Committee Socialist Education Committee (A21)

Shanghai Municipal Party Committee Rural Work Department (A70)

Shanghai Municipal Party Committee Rural Work Committee (A72)

Shanghai Municipal Public Service Office (A59)

注释

档案馆列表

中国档案馆 beijing – 北京市档案馆: 北京市卫生局
(135)

gansu – 甘肃省档案馆: 甘肃省委
(91)

guizhou – 赤水县档案馆: 赤水县委
(1-A)

hunan – 湖南省档案馆: 湖南省委 (140) 湖南省
卫生厅 (163)

hunan – 岳阳市档案馆: 岳阳地区革命委员会
(100)

jiangsu – 昆山市档案馆 (昆山)

jiangxi – 江西省档案馆: 江西省卫生厅 (X111)

shanghai – 上海市档案馆: 上海市委社会主义教育委员会 (A21) 上
海市委农村工作部 (A70) 上海市委农村工作委员会 (A72) 上海市市
政服务办公室 (A59)

Shanghai Municipal Health Bureau (B242)
 Shanghai Municipal Committee on Agricultural Work (B250)
 Shanghai Association of Medical Workers (C3)
 The Communist China Youth League Shanghai Branch (C21)
 The United Front Shanghai Branch (C44)
 Tongde Medical College (Q249)

SHANGHAI – Fudan University's School of Public Health Archive:
 Professor Su Delong's Academic Papers (RM008-082)

SICHUAN – Sichuan Provincial Archive:
 Sichuan Province Bureau of Health and Hygiene (JC133)
 Western Sichuan Region Party Committee (JX1)
 Xikang Province Health Department (JK32)
 Xikang Province Party Committee (JK1)

SICHUAN – Xichang City Archive:
 Xichang County Health Authority (62)

YUNNAN – Yunnan Provincial Archive:
 Yunnan Province Bureau of Grain (120)
 Yunnan – Dali Region Schistosomiasis Control Research Institute

ZHEJIANG – Zhejiang Provincial Archive:
 Zhejiang Province Health and Disease Prevention Department (J166)

ZHEJIANG – Jiaxing City Archive:
 Jiaxing Municipal Government (143)
 Jiaxing County Authority (016)
 Jiaxing Party History Research Office (191)
 Jiaxing County Health Bureau (053)
 Jiaxing Disease Prevention Station (078)
 Jiaxing Schistosomiasis Control Office (008)
 Jiaxing Agricultural Bureau (031)
 Jiaxing Jingxiang Commune Schistosomiasis Control Work Team (102)

ZHEJIANG – Jiaxing City Library 'Schistosomiasis Control in Jiaxing
 Oral History' Archive

上海市卫生局 (B242) 上海市农业工作委员会 (B250) 上海市
 医务工作者协会 (C3) 中国共产主义青年团上海分部 (C21)
 上海统战部 (C44) 同德医学院 (Q249)

上海 – 复旦大学公共卫生学院档案馆：
 苏德隆教授的学术论文 (RM008-082)

四川 – 四川省档案馆：四川省卫生厅 (jc133) 川西区党
 委 (jx1) 西康省卫生厅 (jk32) 西康省委 (jk1)

四川 – 西昌市档案馆：西昌县卫生局
 (62)

云南 – 云南省档案馆：云南省粮食局 (120) 云南省大理地区血吸
 虫病防治研究所

浙江 – 浙江省档案馆：浙江省卫生防疫 भाग (J166)

浙江 – 嘉兴市档案馆：嘉兴市政府 (143) 嘉兴县政府 (016) 嘉兴市委
 党史研究室 (191) 嘉兴县卫生局 (053) 嘉兴市疾病预防站 (078) 嘉兴市
 血吸虫病防治办公室 (008) 嘉兴市农业局 (031) 嘉兴景巷公社血吸虫
 病防治工作队 (102)

浙江 – 嘉兴市图书馆 ‘嘉兴血吸虫病防治口述史’ 档案

Non-Chinese Archives

ROCKEFELLER FOUNDATION ARCHIVES

The Rockefeller Foundation Annual Reports

Projects (RG1.1)

General Correspondence (RG2)

China Medical Board (CMB)

Claude H. Barlow Papers, 1919–64

DIGITAL ARCHIVES FOR THE WHO AND THE LEAGUE OF NATIONS DOCUMENTS:

Official Records of the World Health Organization at the WHO digital library (IRIS: <https://apps.who.int/iris/handle/10665/85537> /[browse?type=dateissued](https://apps.who.int/iris/handle/10665/85537/browse?type=dateissued))

Reports on the Work of the League at the Northwestern University

Library's digital collection of League of Nations Statistical and

Disarmament Documents ([https://wayback.archive-it.org/6321/](https://wayback.archive-it.org/6321/20160901163206/http://digital.library.northwestern.edu/league/search.html)[20160901163206/http://digital.library.northwestern.edu/league/search.html](https://wayback.archive-it.org/6321/20160901163206/http://digital.library.northwestern.edu/league/search.html))

INTRODUCTION

- 1 I am using the term 'the people's health', *Volksgesundheit*, as an ideological construct of the latter half of the nineteenth century that encompassed the widest range of interventions from quarantine to vaccination, to sewers to eugenics, to social hygiene. It also spawned multiple academic subfields, from social medicine, to public health, to epidemiology tropical medicine and beyond allopathic medicine into a wide range of arenas now labelled as alternative and complementary medicine. It took various political forms, from Bismarck's anti-socialist interventions in 1884 to Beveridge's idea of the nation's health in 1946. See, for example, Francis B. Smith, *The People's Health* (New York: Holmes and Meier 1979); Robert Proctor, *Racial Hygiene: Medicine under the Nazis* (Cambridge, MA: Harvard University Press 1988); Jürgen Reulecke, Adelheid Gräfin zu Castell Rüdenhausen, eds, *Stadt und Gesundheit: Zum Wandel von 'Volksgesundheit' und Kommunalen Gesundheitspolitik im 19. und frühen 20. Jahrhundert* (Stuttgart: Franz Steiner Verlag 1991); Steven Feierman and John M. Janzen, eds, *The Social Bases of Health and Healing in Africa* (Berkeley: University of California Press 1992); David Arnold, *Colonizing*

非中文档案馆洛克菲勒基金会档案馆 洛克

菲勒基金会年度报告项目 (rg1.1) 通用信

件 (rg2) 中国医学委员会 (cmb)

Claude H. Barlow Papers, 1919–64

世卫组织和国际联盟文件的数字档案馆: 世界卫生组织数字图书馆中的世界卫生组织正式记录 (iris:

<https://apps.who.int/iris/handle/10665/85537/browse?type=dateissued>) 西北大学图书馆的国际联盟统计和裁军文件数字馆藏中的国际联盟工作报告 ([https://wayback.archive-it.org/6321/](https://wayback.archive-it.org/6321/20160901163206/http://digital.library.northwestern.edu/league/search.html)

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导论

- 1 我使用“人民健康” *Volksgesundheit* 一词, 作为 19 世纪后半叶的一种意识形态建构, 它涵盖了从隔离到疫苗接种, 从下水道到优生学, 再到社会卫生等最广泛的干预措施。它还催生了多个学术子领域, 从社会医学到公共卫生, 再到流行病学热带医学, 并超越了对抗疗法医学, 进入了现在被称为替代医学和补充医学的广泛领域。它采取了各种政治形式, 从俾斯麦在 1884 年采取的反社会主义干预措施到贝弗里奇在 1946 年提出的国家健康理念。例如, 参见 Francis B. Smith, 《人民的健康》(纽约: Holmes and Meier 1979); Robert Proctor, 《种族卫生: 纳粹统治下的医学》(马萨诸塞州剑桥: 哈佛大学出版社 1988); Jürgen Reulecke, Adelheid Gräfin zu Castell Rüdenhausen 编辑, 《城市与健康: 19 世纪和 20 世纪初“人民健康”与地方卫生政策的变迁》(斯图加特: Franz Steiner Verlag 1991); Steven Feierman 和 John M. Janzen 编辑, 《非洲健康与治疗的社会基础》(伯克利: 加利福尼亚大学出版社 1992); David Arnold, 《殖民化》

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CHAPTER ONE

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CHAPTER TWO

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CHAPTER THREE

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CHAPTER FOUR

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CHAPTER FIVE

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CHAPTER SEVEN

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CHAPTER EIGHT

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CHAPTER NINE

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第九章

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后记

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